

NCLEX-PN

Bonus Full-Length Simulation Tests

27 Additional Full-Length Practice Tests

Timed practice with a complete Answer Key after each test.

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This bonus content is provided as a companion to the printed Study Guide.

Simulation Test 1

100 questions

1. A practical nurse is organizing care for four clients on a medical unit. Which client should the nurse attend to first?

- A. A client with pneumonia who is suddenly restless and has a respiratory rate of 32/min
- B. A client with pancreatitis who reports nausea after breakfast
- C. A client with peripheral vascular disease who is due for a routine dressing change
- D. A client with insomnia who asks for a prescribed bedtime sedative

2. A practical nurse is assigning tasks to an assistive personnel (AP) worker on a rehabilitation unit. Which task is most appropriate to delegate to the AP?

- A. Teach a client how to transfer safely from the bed to a wheelchair using a gait belt
- B. Assist a stable client with a scheduled shower and report any skin redness noted during care
- C. Decide whether a client who feels weak should participate in physical therapy
- D. Evaluate whether a client's pain medicine was effective before morning exercises

3. A hospitalized adult client who is alert and oriented says to the nurse, "I do not want my sister to receive updates about my condition anymore." Later that afternoon, the sister calls the unit and asks, "How is she doing after her tests?" What should the nurse do?

- A. Provide a brief update because the sister was involved earlier in the hospitalization
- B. Ask another nurse to speak with the sister to avoid conflict
- C. Tell the sister that the nurse cannot share information without the client's permission
- D. Give only general information, such as whether the client is stable

4. A practical nurse is updating a client communication board in a shared hospital room. Which information is most appropriate to write on the board to support care while protecting privacy?

- A. The client's full diagnosis and pending test results
- B. The client's medication schedule and allergy list
- C. The client's first name and the names of assigned care team members
- D. The client's insurance provider and discharge destination

5. A practical nurse is caring for a client who is a local business owner. At the end of the shift, the nurse's neighbor, who works in dietary services and is not assigned to the client, says, "I heard he was admitted. Is he really here?" Which response by the nurse is best?

- A. "Yes, but I can't discuss why he was admitted."
- B. "You should ask the charge nurse if you need to know."
- C. "I can't confirm whether someone is a client here unless sharing that information is part of your role in the client's care."
- D. "I can tell you only if the client gave general consent for treatment."

6. An adult client being admitted for elective surgery says, "I already have an advance directive, but I left it at home." Which nursing action is most appropriate?

- A. Ask the client to have a copy brought to the facility for the medical record
- B. Tell the client the directive is invalid unless the original document is present
- C. Have the client complete a new advance directive immediately before surgery
- D. Document that advance directives cannot be used for surgical clients

7. A client with metastatic cancer is no longer able to make decisions. The chart contains a durable power of attorney for health care naming the client's niece as the decision-maker. The client's adult son tells the nurse, "I'm the closest family member here, so I will be making the decisions." Which response by the nurse is best?

- A. The son should decide because immediate family always has priority over other relatives
- B. The provider should choose the decision-maker because the family disagrees
- C. The named health care agent should make decisions for the client while the client lacks decision-making capacity
- D. Both the son and the niece must agree before any treatment decisions can be made

8. A client who is scheduled for a liver biopsy tells the nurse, "The provider explained the procedure earlier, but I forgot most of it. Can you remind me why I need it?" Which response by the nurse is most appropriate?

- A. I'll explain the purpose of the procedure to the best of my ability so you can sign now.
- B. You already met with the provider, so it is not necessary to review it again.
- C. I will ask the provider to return and explain the procedure again before you decide.
- D. You can read the consent form again because it contains all the information you need.

9. A client is admitted for a scheduled knee arthroscopy. The consent form was signed at the surgeon's office yesterday. This morning, the client receives an IV dose of an opioid analgesic for severe pain. Fifteen minutes later, transport arrives to take the client to surgery, and the nurse notices the consent form is missing from the chart. The client says, "That's okay, I'll just sign another one now." Which action should the nurse take?

- A.** Have the client sign a new consent form because the client already agreed yesterday.
- B.** Delay obtaining the signature now and notify the provider because the client recently received opioid medication.
- C.** Ask another nurse to witness the signature so the procedure is not delayed.
- D.** Document that verbal consent is enough since the written consent was previously signed.

10. A client is ready for discharge 24 hours after treatment for a deep partial-thickness burn to the dominant hand. The provider prescribes daily dressing changes at home. During teaching, the client correctly repeats the wound care steps but says, "I don't think I can do this with one hand." Which nursing action is most appropriate?

- A.** Ask the client to sign the discharge papers and review the written instructions again
- B.** Request a referral for home health services to assist with wound care needs
- C.** Tell the client to wait until the next clinic visit to have the dressing changed
- D.** Advise the client to remove the dressing only if the hand becomes more painful

11. A client with newly diagnosed glaucoma is preparing for discharge with prescribed eye drops. The client speaks limited English, and the adult grandchild has been interpreting during the hospitalization. When the nurse begins discharge teaching, the grandchild says, "I'll explain everything later at home." Which action by the nurse is best?

- A.** Provide the written instructions to the grandchild and continue the discharge process
- B.** Ask the provider to postpone discharge until the client can speak English well enough to understand
- C.** Use a qualified medical interpreter to review the discharge instructions and medication schedule with the client
- D.** Teach only the grandchild how to give the eye drops because the grandchild will be helping at home

12. A home health agency is trying to reduce medication errors during admission visits. Before changing its workflow, the team asks the practical nurse to help identify when and where the errors are most likely to occur. Which action best supports quality improvement at this stage?

- A.** Collect data on the time, circumstances, and types of medication errors during recent admission visits
- B.** Immediately require all nurses to use a new medication form for every client
- C.** Counsel the nurse involved in the most recent medication error
- D.** Stop scheduling high-risk clients for admission visits until errors decrease

13. A surgical unit launched a checklist intended to reduce delays in postoperative pain reassessment. Three months later, audit results show that nurses documented use of the checklist in 97% of eligible cases, but the percentage of clients reassessed within the required time frame changed only from 61% to 63%. Which interpretation is most appropriate?

- A.** The checklist should be considered fully effective because staff compliance was high
- B.** The team should conclude that postoperative clients did not need timely reassessment
- C.** The checklist was implemented consistently, but it did not meaningfully improve the desired client-care process
- D.** The audit results prove that documentation accuracy, not reassessment timing, is the only important outcome

14. A practical nurse is contributing to an interdisciplinary care conference for a client with Parkinson disease who has had repeated coughing during meals. Which information is most important for the nurse to share with the team?

- A.** The client prefers to eat dinner later in the evening
- B.** The client has been coughing with thin liquids and takes much longer to finish meals
- C.** The client's spouse usually brings in the client's favorite soup from home
- D.** The client says the hospital food does not taste good

15. A practical nurse is preparing to notify the provider and pharmacist about a client who is prescribed warfarin and has developed new gum bleeding and large bruises. Which additional information is most important for the nurse to gather before making the report?

- A.** The client's favorite foods from the weekly menu
- B.** The client's most recent INR result and any newly started medications
- C.** The number of visitors the client had that day
- D.** The date of the client's last influenza vaccine

16. A practical nurse is caring for a client with active tuberculosis. The client's adult neighbor arrives and says, "I'm like family, so just tell me how bad it is." The client is resting and has not given permission to share information. Which response by the nurse is most appropriate?

- A.** Provide a brief update without using the client's name
- B.** Ask the neighbor to wait until the provider can explain the condition
- C.** Explain that client information cannot be shared without the client's permission
- D.** Share only the diagnosis because tuberculosis is a public health concern

17. A practical nurse gave an oral medication at 0800 but discovers at 1000 that the medication had been discontinued at 0700. The client has no apparent adverse effects. Which action by the nurse is most appropriate?

- A.** Document the dose as given and monitor the client, since no harm occurred
- B.** Notify the charge nurse and provider, assess the client, and complete an incident report per policy
- C.** Ask the oncoming nurse whether the medication should simply be omitted from the record
- D.** Wait until the next scheduled dose to see whether the client develops symptoms

18. A nurse in a rehabilitation unit notices that a paraplegic client has a new pressure injury, poor hygiene, and repeated missed follow-up appointments after weekend visits home. The client's spouse says, "We're doing the best we can," and the nurse is unsure whether the situation meets the threshold for mandatory reporting. Which principle should guide the nurse's action?

- A.** Report only after collecting enough evidence to prove neglect occurred
- B.** Report suspected neglect when reasonable concern exists, according to facility policy and law
- C.** Wait to see whether the client's condition worsens before making a report
- D.** Discuss the concern only with the spouse to avoid violating confidentiality

19. A nurse in an outpatient dialysis center overhears an assistive personnel member tell a cognitively impaired adult client, "If you keep moving, I'll leave you here all day." The client appears frightened and later whispers, "Please don't say anything." Which action by the nurse is most appropriate?

- A.** Confront the staff member privately and take no further action if the behavior stops
- B.** Document the incident factually and follow the required reporting process for suspected abuse of a vulnerable adult

- C.** Honor the client's request for secrecy because the client is upset and fearful
- D.** Wait for a second staff member to confirm the behavior before reporting

20. A practical nurse is caring for four clients in a skilled nursing facility with help from one assistive personnel (AP). Which task is most appropriate for the nurse to assign to the AP?

- A.** Reinforce teaching to a client newly prescribed compression stockings
- B.** Perform passive range-of-motion exercises for a client with stable contractures
- C.** Assess a client's report of new burning pain at an incision site
- D.** Evaluate the effectiveness of a PRN analgesic given 45 minutes earlier

21. A practical nurse in a public health clinic is scheduling a client for prenatal follow-up. The client says, "I can only come after 6 p.m. because I work two jobs, and I do not get paid if I miss work." Which response by the nurse best supports equitable access to care?

- A.** Explain that the client should request unpaid leave because prenatal visits are important
- B.** Ask the client to arrive early one morning before work so the clinic schedule is not disrupted
- C.** Explore evening or alternative appointment options and connect the client with available scheduling or community resources
- D.** Advise the client to go to the emergency department if a prenatal concern comes up between regular clinic hours

22. A nurse in an outpatient clinic is preparing to give an intramuscular vaccine to a client who has been called from the waiting room. The client answers to the name "Ms. Lee." Which action best verifies the correct client before administering the vaccine?

- A.** Ask the client to state her full name and date of birth, and compare both with the medical record and identification band
- B.** Confirm the client's identity by checking the room assignment and asking why she is in the clinic
- C.** Use the client's last name only because she responded when called from the waiting room
- D.** Ask another staff member if this is the client scheduled for the vaccine

23. A nurse is about to use a heating pad for a client with chronic back pain. The client has diabetic neuropathy and says, "Turn it up high so I can really feel it." Which action by the nurse is best?

- A.** Set the heating pad on high and leave it in place until the client reports relief

- B.** Apply the heating pad over a damp towel so the heat will penetrate more deeply
- C.** Use the lowest effective setting, place a barrier between the pad and skin, and check the skin frequently
- D.** Avoid using the heating pad only if the client's temperature is above normal
- 24.** A nurse is about to administer the first dose of amoxicillin to a client. During the medication check, the client states, "I had a rash and trouble breathing after taking penicillin years ago." The medication administration record does not list any allergies. Which action should the nurse take?
- A.** Administer the medication because there is no documented allergy in the record
- B.** Hold the medication and notify the prescribing provider of the client's reported reaction
- C.** Ask another nurse to give the medication and observe the client closely
- D.** Give the medication with food to reduce the chance of a reaction
- 25.** The practical nurse is checking client rooms at the start of the shift. Which situation presents the greatest environmental safety risk and should be corrected first?
- A.** A call light cord is hanging off the side of the bed and touching the floor
- B.** A bedside table is placed slightly out of the client's reach
- C.** A portable space heater brought from home is plugged in beside oxygen equipment
- D.** A client's clean linens are stacked on a chair near the window
- 26.** A tornado warning is announced for a long-term care facility. Which action by the practical nurse is most appropriate?
- A.** Move ambulatory clients to rooms with large windows so staff can monitor outside conditions
- B.** Keep clients in the dining room so they can be observed together in one open area
- C.** Close client room doors and relocate clients away from windows to an interior hallway or sheltered area
- D.** Transport all clients outside immediately before the storm reaches the building
- 27.** A client with moderate dementia becomes agitated during mealtime, stands up suddenly, and wanders toward other clients' trays. The client is not physically aggressive and calms when spoken to slowly. Which nursing action best follows the principle of least-restrictive restraint use?
- A.** Apply a vest restraint to keep the client seated through the meal
- B.** Move the client to a quieter area and provide one-to-one redirection during the meal
- C.** Request an as-needed sedative so the client will remain at the table
- D.** Place all four side rails up after returning the client to bed
- 28.** A nurse preparing an IV antibiotic notices that the pharmacy label lists the correct client name and medication but shows a dose of 2 g instead of the prescribed 1 g. The medication has not been administered. Which action is most appropriate?
- A.** Administer 1 g by estimating half of the prepared dose, then notify the pharmacy
- B.** Return the medication to the pharmacy, obtain the correct dose, and complete an incident report according to facility policy
- C.** Discard the medication and document that the client refused the antibiotic
- D.** Hold the antibiotic for the shift because no client harm occurred
- 29.** A nurse is planning to move a client from a bed to a chair. The client can bear some weight, follow directions, and hold onto a standing aid, but cannot safely pivot with only a gait belt. Which transfer device is the most appropriate for this client?
- A.** Transfer board
- B.** Sit-to-stand lift
- C.** Full-body mechanical lift
- D.** Friction-reducing sheet
- 30.** A practical nurse is changing the linens of a client whose wound is draining purulent fluid. The old top sheet is damp with drainage. Which action by the nurse is best to reduce the spread of microorganisms during linen removal?
- A.** Hold the sheet away from the uniform and roll it inward before placing it in the laundry bag
- B.** Shake the sheet gently to locate the most soiled area before removing it from the bed
- C.** Place the sheet on the overbed table while finishing the bed change
- D.** Carry the sheet uncovered to the soiled utility room after removing it from the bed
- 31.** A nurse is preparing to change the dressing on a client with a draining abdominal wound. The old dressing is saturated, and irrigation may be needed. Which personal protective equipment should the nurse use?
- A.** Sterile gloves only
- B.** Clean gloves and a gown
- C.** Gloves, gown, mask, and eye protection
- D.** Mask only

32. A practical nurse is adding sterile instruments to an already prepared sterile field. While placing the last item, the nurse's sterile-gloved hand drops below waist level for a moment. Which action should the nurse take next?

- A.** Continue the procedure because the glove did not touch any visible surface
- B.** Ask another nurse to watch the field and continue if no contamination is seen
- C.** Consider the glove contaminated and replace it before touching the sterile field again
- D.** Clean the glove with antiseptic solution and continue the setup

33. A nurse is preparing a client with varicella (chickenpox) for transfer to another department for a necessary diagnostic test. Which action is most appropriate?

- A.** Have the client wear a surgical mask during transport
- B.** Place a clean gown on the client and leave the room door open
- C.** Transport the client only after giving an antipyretic medication
- D.** Ask the receiving department to place the client in a semiprivate waiting area

34. A practical nurse is preparing to change the dressing on a client's surgical incision. The incision is clean and dry, but the old dressing has a small amount of serosanguineous drainage. Which action best demonstrates use of standard precautions?

- A.** Put on clean gloves before removing the old dressing and perform hand hygiene after glove removal
- B.** Wear sterile gloves from the start because any surgical incision requires full sterile precautions
- C.** Skip gloves because the drainage is minimal and the incision shows no signs of infection
- D.** Apply a mask only after drainage is seen on the old dressing

35. A nurse is assessing a 5-year-old child during a well-child visit. Which task should the nurse expect the child to be able to perform?

- A.** Build a tower of 3 blocks
- B.** Tie shoelaces independently
- C.** Skip on alternate feet
- D.** Use a spoon without spilling

36. A practical nurse is reinforcing teaching with the parent of a first-year college student who moved away from home 2 weeks ago. The parent says, "She calls every night crying and says she wants to quit and come home. I'm worried something is terribly wrong." Which response by the nurse is most appropriate?

- A.** This means your daughter is not emotionally ready for college and should probably withdraw now.
- B.** Homesickness and stress can occur during this transition, so encourage her to build routines and use campus support services.
- C.** Students usually adjust only if parents stop all contact for a few weeks.
- D.** Feeling this upset after moving away almost always indicates a serious mental illness.

37. At a senior center, a practical nurse is reinforcing teaching about screening and prevention. Which statement by a client shows correct understanding of the purpose of screening tests?

- A.** Screening tests are used only after a disease has already been diagnosed.
- B.** A normal screening result means no follow-up care will ever be needed.
- C.** Screening can help find health problems early, even before symptoms appear.
- D.** Screening is most useful only for clients who feel sick.

38. A nurse is obtaining a health history from an adult client who recently immigrated and speaks limited English. The client nods yes to several questions but avoids eye contact and gives brief answers. Which action by the nurse is best?

- A.** Ask a family member to interpret so the interview can be completed more quickly
- B.** Continue the interview and assume the client understands because the client is nodding
- C.** Use a qualified medical interpreter to clarify the client's responses and improve communication
- D.** Postpone the interview until the client is more comfortable speaking English

39. A practical nurse is admitting an adult client for treatment of pneumonia. To compare future findings with the client's starting status, which assessment is most important to complete and document first as baseline physical data?

- A.** The client's preferred visiting hours
- B.** The client's usual pharmacy location
- C.** The client's lung sounds, respiratory rate, and oxygen saturation
- D.** The client's opinion about the hospital food

- 40.** A nurse is screening clients at a community clinic before giving scheduled vaccines. Which client finding is the highest priority to report to the provider before administration?
- A.** A 4-month-old infant who cried after the last injection and was fussy for several hours
 - B.** A 19-year-old client with a history of severe allergy to eggs who is scheduled for an influenza vaccine
 - C.** A 7-year-old child who is taking oral amoxicillin for otitis media and is afebrile
 - D.** A 65-year-old client who reports mild soreness at the injection site after a previous pneumococcal vaccine
- 41.** A nurse is reinforcing teaching with the family of an 82-year-old client who is moving into an assisted-living community. Which response by the family shows understanding of a common developmental need in older adulthood?
- A.** We should make most decisions for the client to reduce stress.
 - B.** We should encourage the client to stay involved in choices and maintain social relationships.
 - C.** We should limit the client's activities until adjustment to the move is complete.
 - D.** We should avoid discussing past experiences so the client can focus on the present.
- 42.** A nurse is giving diet instructions to a client with diabetes. The client keeps glancing at the clock, sighing, and says, "I need to get downstairs before my ride leaves." Which action by the nurse is best to reduce the communication barrier?
- A.** Continue the teaching quickly so all of the information is covered before discharge
 - B.** Ask the client to sign the teaching form now and read the instructions later at home
 - C.** Pause the teaching, address the client's immediate concern, and arrange a better time to continue the education
 - D.** Give the teaching only to the family member who is waiting in the room
- 43.** A nurse plans to teach a client how to perform colostomy care before discharge. The client received prescribed pain medication 15 minutes ago and says, "I'm still too uncomfortable to pay attention right now." Which action by the nurse is best?
- A.** Continue the teaching now so discharge is not delayed
 - B.** Ask a family member to learn the procedure instead of the client
 - C.** Delay teaching until the client's pain is better controlled
 - D.** Provide the client with written instructions to read later
- 44.** A client with a new colostomy tells the practical nurse, "I don't want my spouse to see me like this." Which response by the nurse is most therapeutic?
- A.** You should let your spouse help, because you will need support at home.
 - B.** Many people feel that way at first, so try not to worry about it.
 - C.** It sounds like you are feeling embarrassed and unsure about how this may affect your relationship.
 - D.** Your spouse probably will not be bothered by the colostomy.
- 45.** A client who is being treated for alcohol use disorder 18 hours after the last drink becomes tremulous, irritable, and nauseated. The vital signs are temperature 37.1°C (98.8°F), pulse 112/min, respirations 22/min, and blood pressure 168/96 mm Hg. Which prescription should the nurse expect to help prevent progression to severe alcohol withdrawal?
- A.** Administer a benzodiazepine as prescribed
 - B.** Restrict oral fluids until nausea resolves
 - C.** Give naloxone as needed for respiratory depression
 - D.** Place the client in four-point restraints for safety
- 46.** A client who is 2 days postoperative becomes confused in the evening, tries to climb out of bed, and says, "I need to catch the bus home." The client becomes more restless when staff repeatedly explain that hospitalization is necessary. Which action should the nurse take first?
- A.** Apply wrist restraints to prevent the client from getting out of bed
 - B.** Firmly tell the client to stay in bed and stop arguing
 - C.** Escort the client to the nurses' station and speak calmly using short, simple statements
 - D.** Ask several orientation questions to determine how confused the client is
- 47.** A client whose infant is being treated in the neonatal intensive care unit says to the nurse, "I keep thinking this is my fault." Which response by the nurse is most therapeutic?
- A.** You should not feel guilty because the staff is doing everything possible for your baby.
 - B.** Tell me more about what has been making you feel responsible.
 - C.** Many parents feel this way, so try not to dwell on it.
 - D.** I know exactly how you feel; my sister went through the same thing.

48. A client on an inpatient unit becomes visibly upset after being told that visiting hours are over. The client begins pacing near the doorway and says, "You can't tell me what to do. I'm not staying in this room." Which response by the nurse is most appropriate?

- A.** "If you do not return to your room now, security will be called."
- B.** "You need to calm down so we can talk about this."
- C.** "I can see that you're upset. Let's step over here where it's quieter and talk about what you need."
- D.** "The rules apply to everyone, so arguing will not change visiting hours."

49. A client with early dementia is becoming increasingly forgetful and sometimes misplaces personal items. During a home visit, the spouse says, "I keep testing my partner's memory so it won't get worse, but that usually leads to arguments." Which response by the practical nurse is most appropriate?

- A.** "Correct every wrong answer right away so your partner stays oriented to reality."
- B.** "Ask complex questions several times a day to improve memory through repetition."
- C.** "Use simple reminders and a regular routine instead of quizzing, because repeated testing can increase frustration."
- D.** "Avoid mentioning forgotten events, and make all decisions without involving your partner."

50. A client says, "Ever since I started caring for my father at home, I feel tense all day. I keep telling myself I just need to push through it alone." Which nursing response is most appropriate to support effective stress management?

- A.** Try to avoid thinking about your stress so you can stay focused on caregiving tasks.
- B.** It may help to identify one trusted person or community resource you can ask for specific help this week.
- C.** You should wait until you feel less emotional before making any changes to your routine.
- D.** If caregiving is important to you, feeling this stressed is something you need to accept.

51. A client recovering from abdominal surgery tells the practical nurse, "My faith teaches me to fast on certain days. Tomorrow is one of those days, and I do not want to eat." The client is on a prescribed postoperative diet. Which action by the nurse is most appropriate?

- A.** Tell the client that fasting is not allowed while hospitalized and remove meal trays without further discussion
- B.** Ask the client to describe the importance of the fast and notify the RN and provider so the care team can discuss safe options

- C.** Encourage the client to eat anyway because nutrition is more important than spiritual practices after surgery
- D.** Arrange for the dietary department to skip all meals the next day without informing the rest of the care team

52. A nurse is obtaining a health history from a client who recently moved from another country. When asked about current treatments, the client says, "I use a healing paste from my community healer," and asks whether it can be used with the prescribed medication. Which response by the nurse is most appropriate?

- A.** Stop using the paste until the provider says it is acceptable.
- B.** Tell me what is in the paste and how you use it so we can evaluate it safely with your treatment plan.
- C.** Traditional remedies are not reliable, so it is best to use only the prescribed medication.
- D.** You may continue the paste because cultural practices should always be respected.

53. A client tells the nurse, "I identify as nonbinary, and I get anxious every time I come to a clinic because people make assumptions about me." Which response by the nurse is best?

- A.** "Try not to focus on that right now so we can finish your assessment."
- B.** "I understand. Tell me what name and pronouns you would like us to use, and let me know if there are other ways we can help you feel more comfortable."
- C.** "Your chart already lists your sex, so we will just use that for this visit."
- D.** "Most staff members mean well, so it is best not to take it personally."

54. A client who developed persistent tinnitus after a workplace accident tells the nurse, "The noise never stops. I can't focus, and I'm starting to snap at everyone." Which nursing action is best?

- A.** Tell the client that most people adjust over time and should try to ignore the sound
- B.** Acknowledge the client's frustration and explore coping strategies that have reduced stress in the past
- C.** Suggest avoiding discussion of the tinnitus so the client does not dwell on it
- D.** Explain that the priority is protecting the ears from further injury rather than discussing emotions

55. A client receiving end-of-life care says to the nurse, "Please don't call my daughter yet. She lives out of state, and I want to tell her myself when I'm ready." The daughter's phone number is listed in the chart, and she has been calling the unit for updates. Which nursing action is most appropriate?

- A.** Call the daughter and explain that the client is declining so she can make travel plans

- B.** Ask the provider to speak with the client about why family should be notified immediately
- C.** Respect the client's request and withhold updates from the daughter unless the client gives permission
- D.** Encourage another family member to contact the daughter since the nurse cannot do so directly
- 56.** A nurse is flushing a client's gastrostomy tube after a feeding and notes increasing resistance. The tube is still patent, but the flush is moving slowly. Which nursing action is most appropriate?
- A.** Use firm pressure on the syringe plunger to force the tube open
- B.** Attempt to clear the tube by flushing gently with warm water
- C.** Insert a sterile cotton-tipped applicator into the tube opening
- D.** Add the next feeding to the tube to help move the blockage through
- 57.** A nurse is reinforcing teaching with a client who will use a standard walker at home after hip surgery. Which client action indicates correct understanding of walker use?
- A.** The client advances the walker, steps forward with the affected leg first, and then brings the unaffected leg forward.
- B.** The client keeps the walker still, steps forward with the unaffected leg, and then moves the affected leg to meet it.
- C.** The client lifts the walker and places it several feet ahead to encourage a longer stride.
- D.** The client pulls up on the walker while rising from a chair to improve balance.
- 58.** A client with peripheral vascular disease reports aching pain in both feet and asks for something other than medication to help. Which nonpharmacologic intervention should the nurse avoid?
- A.** Providing a warm blanket for comfort
- B.** Using guided breathing techniques
- C.** Applying a heating pad directly to the feet
- D.** Repositioning the legs for support
- 59.** A nurse is caring for a client with chronic obstructive pulmonary disease who becomes fatigued while performing morning hygiene. Which intervention best helps the client complete activities of daily living while conserving energy?
- A.** Encourage the client to finish bathing as quickly as possible without rest periods
- B.** Schedule hygiene activities after meals so the client has more energy
- C.** Provide a chair and allow the client to rest between parts of the bath
- D.** Complete the entire bath for the client to prevent shortness of breath
- 60.** A nurse is caring for a client who has chronic constipation and is on a bowel training program. Which instruction should the nurse reinforce to promote regular bowel elimination?
- A.** Ignore the urge to defecate until a more convenient time
- B.** Attempt defecation at the same time each day, especially after meals
- C.** Limit fluid intake to reduce abdominal fullness
- D.** Remain on bed rest for 1 hour before trying to have a bowel movement
- 61.** A nurse is caring for a client who says, "I usually drink several cups of coffee during the day, and I still have one with dessert after dinner." The client reports trouble falling asleep most nights. Which response by the nurse is best?
- A.** Continue the evening coffee if it is part of the client's routine
- B.** Replace the evening coffee with a caffeine-free beverage
- C.** Go to bed earlier to make up for the lost sleep
- D.** Stay in bed longer each morning until sleep improves
- 62.** A practical nurse is caring for a client who uses a wheelchair most of the day and has decreased sensation in the lower body. Which instruction is best to help prevent pressure injury development?
- A.** Massage reddened areas over bony prominences at the end of the day
- B.** Shift weight in the chair at regular intervals throughout the day
- C.** Limit fluid intake to reduce the need for transfers to the toilet
- D.** Place a donut-shaped cushion under the buttocks while sitting
- 63.** A practical nurse is assisting a client who has unilateral hearing loss in the right ear. To communicate most effectively during morning care, where should the nurse position themselves when speaking to the client?
- A.** On the client's right side so the client can turn toward the sound
- B.** Directly behind the client to reduce visual distractions
- C.** On the client's left side and facing the client
- D.** At the foot of the bed while speaking in a louder voice
- 64.** A client on strict intake and output has the following output during the night shift: 500 mL urine in the catheter bag, 150 mL emesis, and 75 mL liquid stool. The client also had a small formed bowel movement. What total output should the practical nurse record?
- A.** 575 mL
- B.** 650 mL

C. 725 mL

D. 800 mL

65. A practical nurse is caring for a client who has been immobilized in bed for several days. Which intervention is most appropriate to help prevent venous stasis?

A. Place a pillow directly under both knees while the client is resting

B. Encourage active or passive leg and ankle exercises at regular intervals

C. Limit fluid intake so the client will need less assistance with toileting

D. Keep the client in high-Fowler position for most of the day

66. A nurse is preparing to administer a scheduled dose of insulin to a client with diabetes. Before giving the medication, the nurse checks the client's blood glucose and finds it is 58 mg/dL. Which action best supports safe medication administration?

A. Administer the insulin as scheduled because it was prescribed

B. Hold the insulin and follow facility protocol for hypoglycemia before notifying the provider

C. Give the insulin with orange juice to prevent the blood glucose from dropping further

D. Ask another nurse to administer the insulin to verify the dose is correct

67. A client with heart failure and symptomatic anemia is prescribed a transfusion. Which blood product should the nurse question if the goal is to improve oxygen-carrying capacity while reducing the risk of fluid overload?

A. Packed red blood cells

B. Leukocyte-reduced red blood cells

C. Whole blood

D. Washed red blood cells

68. A nurse must administer 500 mL of IV fluid over 5 hours by gravity. The available tubing has a drop factor of 15 gtt/mL. How many drops per minute should the nurse regulate?

A. 20 gtt/min

B. 25 gtt/min

C. 30 gtt/min

D. 35 gtt/min

69. A practical nurse administers 0.5 mg of a prescribed controlled medication from a 1 mg single-use vial. Facility policy requires witnessed wasting of any remaining controlled substance. Which action by the nurse is best?

A. Ask the next shift nurse to dispose of the remaining medication later

B. Discard the remaining medication in the sharps container and document the full dose as given

C. Have an authorized witness observe the waste immediately and document the amount administered and wasted

D. Keep the remaining medication in the medication cart in case the client needs another dose soon

70. A nurse is caring for a client 6 hours after orthopedic surgery. The client is receiving pain medication through a patient-controlled analgesia (PCA) pump. Which action by the nurse is appropriate?

A. Encourage the client to press the PCA button before pain becomes severe

B. Set the pump to deliver an extra dose whenever the client rates pain above 5

C. Ask a family member to remind the client to press the button every hour

D. Stop the PCA when the client becomes drowsy after receiving pain relief

71. A nurse is preparing to administer oral acetaminophen to a client who reports mild pain 1 hour after receiving a prescribed hydrocodone/acetaminophen tablet. Which pre-administration information is most important for the nurse to review?

A. The client's total acetaminophen intake during the last 24 hours

B. The client's most recent oral temperature

C. The time of the client's last bowel movement

D. The client's daily fluid intake

72. A client with chronic stable angina starts a prescribed nitroglycerin transdermal patch. Which finding best indicates the medication is producing the intended therapeutic response?

A. The client reports fewer episodes of chest pain during usual activity

B. The client's heart rate increases from 72/min to 96/min

C. The client reports a mild headache after patch application

D. The client's oxygen saturation rises from 97% to 99%

73. A client has been receiving heparin therapy for treatment of a deep vein thrombosis. Which finding should the nurse report to the provider immediately as the most likely adverse response to the medication?

- A. Platelet count decreases from 220,000/mm³ to 88,000/mm³
- B. Small bruise noted at a previous venipuncture site
- C. Mild soreness at the abdominal injection site
- D. Heart rate of 88/min after ambulation

74. A patient taking warfarin for a history of deep vein thrombosis tells the nurse, "I've been drinking a large glass of cranberry juice every day this week because I heard it's healthy." Which response by the nurse is best?

- A. Continue the cranberry juice because it does not affect warfarin therapy
- B. Stop taking warfarin until the prescribing provider can be contacted
- C. Avoid sudden increases in cranberry intake and report any unusual bleeding
- D. Drink the cranberry juice only at bedtime to reduce the chance of interaction

75. A client with atrial fibrillation is prescribed warfarin 5 mg by mouth each evening. Before administering the scheduled dose, the nurse reviews the laboratory results and notes an INR of 4.8. Which action is most appropriate?

- A. Administer the dose as prescribed and recheck the INR in 24 hours
- B. Hold the warfarin and notify the provider of the elevated INR
- C. Give the warfarin with food to reduce the risk of bleeding
- D. Administer the dose and instruct the client to avoid foods high in potassium

76. A nurse is teaching a client who is starting metformin for newly diagnosed type 2 diabetes. Which statement by the client indicates correct understanding of the teaching?

- A. I should take this medication on an empty stomach for the best effect.
- B. I will stop taking this medication as soon as my blood sugar is normal for a few days.
- C. I should take this medication with meals to help decrease stomach upset.
- D. If this medication gives me diarrhea, I should double the next dose to make up for it.

77. A nurse removes an ampule of lorazepam injection from the medication drawer and notices that the solution is darker than usual and contains small particles. The expiration date has not passed. Which action should the nurse take?

- A. Administer the medication because it is still within the expiration date
- B. Shake the ampule gently, then administer the medication
- C. Discard the medication and obtain a new dose from the pharmacy
- D. Dilute the medication in normal saline to reduce the discoloration

78. A nurse must administer furosemide 0.04 g by mouth. The available tablets contain 20 mg each. How many tablets should the nurse administer?

- A. 1 tablet
- B. 2 tablets
- C. 3 tablets
- D. 4 tablets

79. A practical nurse is collecting data on a client who will receive a first dose of an IV antibiotic. Which finding should the nurse identify as the greatest immediate risk factor for an adverse reaction?

- A. The client reports a history of seasonal pollen allergies
- B. The client states, "I got hives and trouble breathing after penicillin years ago"
- C. The client says the IV site feels cool to the touch
- D. The client has not eaten since early morning

80. A nurse is collecting a specimen for a 24-hour urine test. The client voided at 0800, and the nurse discarded that urine and noted the start time. Which action should the nurse take next to ensure accurate collection?

- A. Save all urine voided by the client for the next 24 hours, including the urine at 0800 the next day
- B. Ask the client to limit fluid intake during the collection period
- C. Send each voided specimen to the laboratory immediately after collection
- D. Add the client's first void after breakfast to begin the collection

81. A client who is 33 weeks pregnant arrives at the labor and delivery unit reporting sudden severe abdominal pain after being in a minor car crash 2 hours ago. The client has a rigid, tender abdomen and says the baby is moving less than usual. Which complication should the nurse suspect first?

- A. Placenta previa
- B. Abruptio placentae
- C. Hyperemesis gravidarum

D. Chorioamnionitis

82. A nurse is preparing to perform a bedside urine ketone test on a client with nausea, vomiting, and diabetes. Which action is most important to ensure the point-of-care result is accurate?

- A.** Use a specimen that was collected at the beginning of the previous shift
- B.** Compare the reagent strip to the color chart at the time specified by the manufacturer
- C.** Dip the strip in urine for 2 full minutes before removing it
- D.** Read the result only after the urine specimen reaches room temperature overnight

83. A practical nurse is obtaining a full set of vital signs for an adult client who has just returned from smoking outside and drinking a cup of coffee. The client also walked quickly back to the room. Which action is most appropriate to help obtain the most accurate blood pressure reading?

- A.** Measure the blood pressure immediately before the client sits down
- B.** Have the client rest quietly for several minutes, then measure the blood pressure
- C.** Use the arm with the strongest pulse, even if an IV is infusing there
- D.** Take the blood pressure over the client's sleeve to avoid delaying the assessment

84. A client is scheduled for a bronchoscopy at 1000. Which statement by the client requires the nurse to follow up before the procedure?

- A.** I removed my dentures this morning.
- B.** I had a cup of coffee with cream at 0800.
- C.** My throat may feel sore afterward.
- D.** Someone is driving me home after the test.

85. A nurse is caring for a client on telemetry. The monitor alarm sounds, and the screen displays a flat line. The client is sitting up in bed, talking with family, and denies any symptoms. Which action should the nurse take first?

- A.** Call the rapid response team immediately
- B.** Assess the client and check the lead connections
- C.** Document artifact due to movement
- D.** Administer oxygen by nasal cannula

86. A nurse is obtaining a routine venous blood specimen from an adult client. After filling the last collection tube, which action should the nurse take first to reduce the risk of bleeding and bruising at the puncture site?

- A.** Release the tourniquet and immediately apply pressure with clean gauze as the needle is withdrawn

- B.** Ask the client to bend the arm sharply at the elbow for 2 minutes

- C.** Label the tubes before removing the needle from the vein

- D.** Massage the site for 30 seconds after removing the needle

87. A practical nurse is caring for a client 6 hours after a transurethral resection of the prostate (TURP) with continuous bladder irrigation. Which finding requires immediate action?

- A.** Light pink urine draining from the catheter
- B.** Reported bladder spasms during irrigation
- C.** No urine output in the drainage bag for 30 minutes
- D.** Blood-tinged urine with a few small clots

88. A nurse is caring for a client who had a nasogastric tube inserted 30 minutes ago for gastric decompression. Which finding best indicates that the tube is in the correct location and can be used safely?

- A.** The client is able to speak clearly after insertion
- B.** A chest x-ray confirms the tube tip is in the stomach
- C.** The client reports mild throat discomfort
- D.** Air is heard over the stomach when the tube is flushed with air

89. A nurse is caring for a client who is 24 hours post-op after open reduction and internal fixation of a tibial fracture. The client reports, "My pain is getting much worse, and my toes feel tingly," even after receiving prescribed pain medication. Which focused assessment should the nurse perform first?

- A.** Assess color, temperature, movement, sensation, and pulses of the affected extremity
- B.** Ask the client to rate the pain again in 30 minutes
- C.** Review the client's preoperative laboratory results
- D.** Reposition the client and apply a warm blanket

90. A client with iron-deficiency anemia began oral ferrous sulfate 3 weeks ago. Which finding best indicates the treatment is effective?

- A.** Hemoglobin has increased from 8.9 g/dL to 10.6 g/dL
- B.** The client reports darker stools since starting the medication
- C.** The client asks to take the medication with meals to reduce nausea
- D.** The client states that constipation has occurred every other day

- 91.** A client has a T-tube in place following biliary surgery. During the shift assessment, the nurse notes the drainage bag is attached to the bed frame and contains 250 mL of greenish-brown fluid collected over the past 8 hours. Which nursing action is most appropriate?
- A.** Clamp the T-tube for 2 hours to reduce fluid loss
 - B.** Reposition the drainage bag below the level of the abdomen
 - C.** Irrigate the T-tube with sterile saline to maintain patency
 - D.** Document the finding as expected and make no change
- 92.** A nurse is caring for a client who has a temperature of 39.4°C (102.9°F). Which additional finding best indicates that the fever is increasing body fluid loss and places the client at risk for dehydration?
- A.** Cool skin with piloerection
 - B.** Urine specific gravity of 1.034
 - C.** Heart rate of 88/min
 - D.** Blood pressure of 122/76 mm Hg
- 93.** A nurse is assessing a client's sacral pressure injury during a dressing change. The skin around the wound is pale, wrinkled, and softened after repeated exposure to drainage. How should the nurse document this finding?
- A.** Eschar
 - B.** Maceration
 - C.** Granulation tissue
 - D.** Undermining
- 94.** A nurse is teaching unlicensed assistive personnel about responding to an adult who suddenly collapses. Which finding most strongly indicates the person is in cardiac arrest rather than just unconscious?
- A.** The person does not open the eyes when the name is called
 - B.** The person has cool, pale skin and weak peripheral pulses
 - C.** The person is unresponsive, is not breathing normally, and has no pulse
 - D.** The person briefly grimaces when the sternum is rubbed
- 95.** A client 12 hours after abdominal surgery is reluctant to cough because of incisional pain. Breath sounds are diminished at the bases, and the client has a weak, nonproductive cough. Which nursing intervention is most appropriate to improve ventilation and help prevent postoperative pulmonary complications?
- A.** Encourage the client to splint the incision with a pillow while coughing and deep breathing
 - B.** Limit activity until the client can cough without discomfort
 - C.** Place the client flat in bed to decrease strain on the incision
 - D.** Offer fluids only after bowel sounds return
- 96.** A nurse is caring for a client with a newly placed tracheostomy 8 hours after surgery. Which bedside setup is most important to maintain airway safety?
- A.** A sterile dressing tray and extra gauze pads
 - B.** An obturator and a tracheostomy tube of the same size and one size smaller
 - C.** A peak flow meter and an incentive spirometer
 - D.** A tongue blade and an oral airway
- 97.** A client receiving mechanical ventilation has a high-pressure alarm sounding repeatedly. The nurse finds the client anxious and coughing, with an oxygen saturation of 95% and equal bilateral breath sounds. Which action should the nurse take first?
- A.** Silence the alarm and continue to monitor the client
 - B.** Assess the ventilator tubing for kinks or obstruction
 - C.** Prepare for immediate chest tube insertion
 - D.** Lower the ventilator's pressure alarm setting
- 98.** A client with symptomatic bradycardia is receiving transcutaneous pacing. The monitor shows pacing spikes followed by wide QRS complexes at the set rate, but the client remains confused, cool, and hypotensive. Which action should the nurse take next?
- A.** Document successful pacing because electrical capture is present
 - B.** Assess for a palpable pulse and other signs of mechanical capture
 - C.** Lower the pacing rate to determine the client's intrinsic rhythm
 - D.** Remove the pacing pads and obtain a 12-lead ECG
- 99.** A nurse is teaching a client how to care for a new ileostomy at home. Which statement by the client indicates correct understanding of pouch care?
- A.** I should wait until the pouch is completely full before emptying it.
 - B.** I should empty the pouch when it is about one-third to one-half full.
 - C.** I should remove the pouch after each meal to keep the skin clean.
 - D.** I should scrub the skin around the stoma with alcohol each time I change the pouch.
- 100.** A nurse is caring for a client 24 hours after a thyroidectomy. Which finding should the nurse recognize as the highest priority change in condition?
- A.** The client reports incisional pain rated 6/10 when swallowing
 - B.** The client has a hoarse voice and frequently clears the throat
 - C.** The client is restless and has noisy inspiratory stridor

D. The client requests an extra blanket because feeling cold is uncomfortable

Simulation Test 1 — Answer Key

1. A	26. C	51. B	76. C
2. B	27. B	52. B	77. C
3. C	28. B	53. B	78. B
4. C	29. B	54. B	79. B
5. C	30. A	55. C	80. A
6. A	31. C	56. B	81. B
7. C	32. C	57. A	82. B
8. C	33. A	58. C	83. B
9. B	34. A	59. C	84. B
10. B	35. C	60. B	85. B
11. C	36. B	61. B	86. A
12. A	37. C	62. B	87. C
13. C	38. C	63. C	88. B
14. B	39. C	64. C	89. A
15. B	40. B	65. B	90. A
16. C	41. B	66. B	91. B
17. B	42. C	67. C	92. B
18. B	43. C	68. B	93. B
19. B	44. C	69. C	94. C
20. B	45. A	70. A	95. A
21. C	46. C	71. A	96. B
22. A	47. B	72. A	97. B
23. C	48. C	73. A	98. B
24. B	49. C	74. C	99. B
25. C	50. B	75. B	100. C

Simulation Test 2

100 questions

1. A practical nurse working in an urgent care center is notified about four newly arrived clients. Which client should the nurse prioritize for assessment?

- A. An adult with a laceration to the palm that has slowed bleeding after direct pressure
- B. An older adult with diabetes who is pale, sweaty, and difficult to arouse in the waiting area
- C. A young adult with a sore throat and a temperature of 100.1°F (37.8°C)
- D. An adult with ankle swelling after a twisting injury who rates pain as 7/10

2. A practical nurse is caring for clients in an assisted-living setting with help from an assistive personnel (AP) worker. Which task is most appropriate for the nurse to delegate to the AP?

- A. Apply a prescribed medicated cream to a new rash
- B. Teach a client how to use a walker safely for the first time
- C. Escort a stable client to the dining room and assist with a routine meal
- D. Decide whether a client's ankle swelling has worsened since yesterday

3. A client who primarily speaks Spanish is being prepared for a bronchoscopy. The provider has explained the procedure in English, and the client appears confused. The client's bilingual teenage son says, "I can explain it to my mother so she can sign the consent now." What should the nurse do?

- A. Ask the son to translate because he knows the client's language and can help avoid delay
- B. Have the client sign the consent form if the son states that the procedure was explained correctly
- C. Request a qualified medical interpreter and delay signing until the client can receive and discuss the information accurately
- D. Witness the signature now and ask the interpreter to review the procedure with the client afterward

4. A practical nurse receives a call from a pharmacist at a client's usual community pharmacy asking to verify the dose of a newly prescribed medication before filling it. Which action by the nurse best protects client privacy while supporting safe care?

- A. Provide the requested information after confirming the caller's identity and that the information is needed for the client's treatment
- B. Refuse to share any information because pharmacy staff are outside the facility

- C. Give the information only if the client's family member gives permission over the phone
- D. Ask a coworker to handle the call because medication information is not the practical nurse's responsibility

5. An alert adult client asks, "If I fill out an advance directive, does that mean I can't change my mind later?" Which response by the nurse is most appropriate?

- A. Once it is signed, an advance directive cannot be changed.
- B. You can change or revoke it later as long as you are able to make your own decisions.
- C. It can be changed only by the health care provider who signed your admission forms.
- D. It applies only during this hospitalization and then expires automatically.

6. A nurse is caring for a client who is alert, oriented, and able to make health care decisions. The client asks to review and complete an advance directive before a scheduled procedure later that day. The client's daughter says, "Don't bring that up right now. It will upset my parent." Which action by the nurse is most appropriate?

- A. Delay the discussion until the daughter agrees it is appropriate.
- B. Ask the provider to decide whether the client should complete the document.
- C. Privately ask the client whether the client wants information and assistance with the advance directive.
- D. Explain that advance directives are completed only when a client becomes unable to decide.

7. A client who is deaf is scheduled for a nonemergency cardiac catheterization. The provider arrives with the consent form, and the client's spouse says, "I can explain everything in sign language for my partner." Which action by the nurse is most appropriate?

- A. Allow the spouse to interpret because family members know the client best
- B. Ask the client to read the form and sign if ready
- C. Request a qualified interpreter so the provider can obtain informed consent directly from the client
- D. Witness the signature if the spouse states the client understands

8. A nurse is preparing a client for elective surgery. The consent form was signed 2 days ago. During the preoperative checklist, the client says, "The surgeon told me this operation would remove the whole thyroid, but today another provider said they might remove only part of it depending on what they find." Which action should the nurse take?

- A. Proceed because the client already signed the consent form

- B.** Explain that the exact extent of surgery is always decided in the operating room
- C.** Notify the surgeon and hold preparation until the client receives clarification and confirms consent
- D.** Ask the client to sign a second consent form after the nurse reviews the procedure
- 9.** A client is being discharged after treatment for cellulitis of the left leg. The provider prescribes an oral antibiotic for 10 days and a follow-up visit in 3 days. The client tells the nurse, "I don't have a primary care provider, and I usually go to urgent care when I get sick." Which nursing action is most appropriate before discharge?
- A.** Ask the client to call a clinic after discharge if the leg gets worse
- B.** Request a referral to a primary care provider or outpatient clinic before the client leaves
- C.** Teach the client to stop the antibiotic if redness improves in 2 to 3 days
- D.** Delay all discharge teaching until a family member arrives
- 10.** A client is preparing for discharge after hospitalization for a diabetic foot ulcer. The wound is improving, but the client must continue dressing changes and pressure-relief measures at home. During discharge planning, the client says, "I live in a rural area, my internet is unreliable, and the clinic is over an hour away. My son can bring me in only once a month." Which action by the nurse best supports a safe discharge plan?
- A.** Arrange home health services for wound assessment and care, if eligible
- B.** Instruct the client to return to the emergency department for all future dressing changes
- C.** Advise the client to stop walking on the foot until the next monthly clinic visit
- D.** Postpone discharge until the client can travel weekly to the clinic
- 11.** A skilled nursing unit wants to reduce missed daily weights for clients with heart failure. Before making any changes, the team asks the practical nurse what information would be most helpful to collect first. Which information is best?
- A.** The times, shifts, and client situations in which daily weights are most often missed
- B.** The names of staff members who forgot to obtain weights last week
- C.** The brand of scale preferred by most nurses on the unit
- D.** The number of clients discharged with heart failure in the past year
- 12.** An outpatient dialysis center is trying to reduce treatment delays. A new previsit equipment check was introduced 1 month ago. Since then, staff documentation shows the check was completed in 98% of cases, but the percentage of treatments starting on time changed from 54% to 55%. Which interpretation is most appropriate?
- A.** The project was successful because staff consistently completed the new check
- B.** The equipment check should be stopped because on-time starts did not reach 100%
- C.** The team improved use of the process, but the intervention has not meaningfully improved the outcome
- D.** The data prove that treatment delays are caused only by client nonadherence
- 13.** A practical nurse is caring for a client with type 2 diabetes who is scheduled for discharge this afternoon. During a routine conversation, the client says, "I can't read the instructions on my insulin pen, and my son works nights so he usually isn't home to help." Which team member should the nurse notify first to support safe discharge planning?
- A.** Dietitian
- B.** Social worker or case manager
- C.** Radiology technician
- D.** Unit secretary
- 14.** A practical nurse cares for a client admitted with a diabetic foot ulcer. Over 24 hours, the wound drainage has increased, the surrounding skin is more red, and the client now reports worsening pain when the dressing is changed. The provider has already seen the client this morning. Which action by the practical nurse best demonstrates effective interdisciplinary information sharing?
- A.** Document the findings in the chart and wait for the next scheduled wound care visit
- B.** Tell the next shift that the wound seems worse so they can keep watching it
- C.** Promptly communicate the specific changes to the appropriate licensed team member or provider and include objective details about drainage, redness, and pain
- D.** Ask the nursing assistant to monitor the wound more closely during linen changes
- 15.** A practical nurse sees a coworker access the electronic health record of a former client who is no longer on the unit. When asked, the coworker says, "I was just curious how she was doing." Which action by the nurse is most appropriate?
- A.** Ignore the situation because no information was shared aloud
- B.** Privately remind the coworker to log out when finished
- C.** Report the privacy violation according to facility policy

D. Ask the former client later whether the access was acceptable

16. A practical nurse is caring for a postoperative client who becomes increasingly restless and reports severe incisional pain. The provider writes a new prescription for IV morphine. Before giving the medication, the nurse notices the prescribed dose is much higher than the usual dose given on the unit for similar clients. Which action should the nurse take first?

- A.** Administer the medication because the provider is responsible for prescribing
- B.** Hold the medication and clarify the prescription with the provider
- C.** Ask another practical nurse to give the medication instead
- D.** Document the prescription as written and wait until the next scheduled round

17. A nurse in a medical office is caring for a 9-year-old child with repeated unexplained absences from follow-up visits. Today, the child arrives with poor hygiene, untreated head lice, and a worsening skin infection. The parent says, "We're having a hard time, but don't involve anyone." What is the nurse's best action?

- A.** Wait to report until the nurse can confirm that neglect is intentional
- B.** Report the suspected neglect according to facility policy and state law
- C.** Teach the parent about hygiene needs and reassess at the next visit before acting
- D.** Document the findings but avoid reporting because the parent asked for privacy

18. A nurse in a long-term care facility notices that an older adult with limited mobility has several new bruises on the inner arms. The client quietly states, "They get rough when I'm slow," but then asks the nurse not to tell anyone. The nurse documents the findings and prepares to make a mandatory report. Which documentation is most appropriate to include?

- A.** Client was definitely abused by staff during morning care
- B.** Suspicious bruising noted; abuse seems likely based on nurse intuition
- C.** Two 3-cm purple bruises noted on inner right arm and one 2-cm yellow-purple bruise on left forearm; client stated, "They get rough when I'm slow"
- D.** Bruises probably resulted from improper handling by an aide who should be disciplined

19. An assistive personnel (AP) reports these findings to the practical nurse after routine morning care. Which report should the nurse follow up on first?

- A.** A resident with chronic arthritis says the usual knee pain is now 4/10 after walking to breakfast

B. A resident with dementia slept only 5 hours overnight and wants a nap before lunch

C. A resident who had a stroke 3 days ago coughs repeatedly while drinking water

D. A resident with diabetes says breakfast was served 20 minutes later than usual

20. A practical nurse is giving directions to a newly hired assistive personnel (AP) who will obtain routine vital signs on stable clients. Which instruction is most appropriate?

- A.** Record the vital signs, and I will review them at the end of the shift
- B.** Let me know right away if a client has abnormal vital signs or seems different from earlier
- C.** Repeat any unusual vital sign after 4 hours and report it only if it is still abnormal
- D.** Ask another AP to verify any reading you think is incorrect before telling me

21. A practical nurse at a rural clinic is reinforcing instructions for a client who was referred to a cardiology specialist 90 miles away. The client says, "I want to go, but I do not have internet at home, my phone minutes are limited, and the specialist's office only accepts online appointment requests." Which action by the nurse best promotes equitable access to care?

- A.** Tell the client to ask a family member to request the appointment online
- B.** Contact the referral office and help arrange an appointment using an available clinic phone or staff-assisted process
- C.** Advise the client to wait until monthly phone minutes reset before scheduling
- D.** Document that the client is noncompliant with the referral plan

22. A nurse is preparing to administer an inhaled bronchodilator to a client in a skilled nursing facility. The client is sitting in a wheelchair outside the room and says, "You can give it now—I use this every day." Which action by the nurse best verifies the client's identity?

- A.** Ask the client to state full name and date of birth, then compare that information with the identification band and medication record
- B.** Confirm the client's identity by checking the room number and asking a nursing assistant to verify the client
- C.** Recognize the client from earlier in the shift and proceed because the medication is routinely prescribed
- D.** Ask the client to state the medication name and dose before administration

23. A nurse plans to use a portable pulse oximeter to assess a client who reports shortness of breath. When the nurse turns on the device, the display is dim and the reading repeatedly jumps between 82% and 96% while the client is sitting still and talking comfortably. Which action is best?

- A.** Document the lowest reading because it may show early hypoxia
- B.** Apply oxygen immediately without further assessment
- C.** Check the probe placement and replace the device or batteries before relying on the reading
- D.** Ask the client to hold the sensor in place for a few minutes and then average the readings

24. A nurse is preparing to insert an indwelling urinary catheter. The client says, "I get itchy and develop a rash whenever I use rubber gloves to clean." Which action should the nurse take first?

- A.** Proceed with catheterization using sterile latex gloves because the reaction was limited to skin symptoms
- B.** Ask the client to describe the reaction further and check the record for a documented latex allergy
- C.** Apply a barrier cream to the client's skin before starting the procedure
- D.** Administer the prescribed analgesic before clarifying the allergy history

25. A practical nurse in a rehabilitation unit sees a staff member place a portable space heater beside a client's bedside chair because the client says the room feels cold. The heater's cord runs in front of the doorway. Which action should the nurse take first?

- A.** Move the heater away from the doorway and continue to observe the client
- B.** Turn off and remove the heater, then report the unsafe equipment use according to facility policy
- C.** Ask maintenance to raise the room temperature when time permits
- D.** Place a blanket over the client's legs and leave the heater in place

26. A client admitted for dehydration becomes confused in the evening and repeatedly tries to remove the telemetry leads. The client is not aggressive and settles when a staff member stays at the bedside and reorients the client. Which action best follows least-restrictive restraint principles?

- A.** Apply soft wrist restraints to prevent lead removal
- B.** Ask the provider for a restraint order in case the behavior continues overnight
- C.** Use one-to-one observation and frequent reorientation before considering restraints
- D.** Place all 4 side rails up and move the client farther from the nurses' station

27. A nurse cares for a client with acute confusion who was placed in soft bilateral wrist restraints earlier in the shift after repeated attempts to remove a newly inserted central venous catheter. Which nursing action best demonstrates ongoing use of the least-restrictive approach?

- A.** Keep the restraints in place until the end of the shift to avoid repeated reassessment
- B.** Reassess the client's behavior and remove the restraints as soon as safer alternatives can protect the line
- C.** Tighten the restraints if the client continues to reach toward the catheter site
- D.** Leave the restraints on while the client is sleeping to prevent future agitation

28. A nurse is about to assist a postoperative client out of bed for the first time when the nurse notices that the sequential compression device is plugged in but not turned on. The client has not yet ambulated and no harm has occurred. Which action is most appropriate?

- A.** Turn on the device, assess the client, and report the near-miss according to facility policy
- B.** Assist the client to ambulate first because early mobility is the priority, then restart the device later
- C.** Document that no injury occurred, so no further follow-up is needed
- D.** Notify the provider immediately before taking any other action

29. A nurse is preparing to reposition a client up in bed using a friction-reducing device with assistance from another staff member. Which action is most appropriate to reduce injury to the staff during the move?

- A.** Reach across the bed and pull the client toward the head of the bed using the drawsheet
- B.** Raise the bed to a comfortable working height before moving the client
- C.** Keep the bed low so the client feels more secure during the repositioning
- D.** Ask the client to cross the arms over the chest after the move is completed

30. A practical nurse is preparing to check a postoperative client's blood pressure and pulse. The nurse sees a portable vital-signs machine in the hallway that was just used for a client on contact precautions. Which action is most appropriate before using the equipment for the postoperative client?

- A.** Use the machine if gloves are worn during the assessment
- B.** Wipe only the blood pressure cuff because it touches the client directly
- C.** Clean and disinfect the equipment according to facility policy before taking it into the room

D. Place a clean sheet over the machine and use it without further action

31. A nurse is preparing to reposition a client who has a large area of uncontrolled diarrhea on the bed linens. The nurse expects direct contact with the client's skin and soiled bedding but does not anticipate splashing. Which personal protective equipment should the nurse use?

- A.** Gloves only
- B.** Gloves and gown
- C.** Gloves, gown, and N95 respirator
- D.** Sterile gloves and face shield

32. A practical nurse is preparing supplies for a sterile tracheostomy dressing change. After opening the outer wrappers, the nurse notices a small tear in the sterile package containing the dressing tray. The tray inside does not appear wet or visibly soiled. Which action is best?

- A.** Use the tray because the contents look clean and dry
- B.** Use the tray only if the tear is on the far corner of the package
- C.** Discard the tray and obtain a new sterile dressing tray
- D.** Open the tray onto the field and avoid touching the area near the tear

33. A nurse is caring for a client diagnosed with scabies. Which intervention should the nurse include in the plan of care?

- A.** Place the client in a negative-pressure room
- B.** Wear a gown and gloves for direct contact with the client
- C.** Wear a fit-tested N95 respirator when entering the room
- D.** Limit the client's fluids to reduce skin drainage

34. A practical nurse is preparing to assist a client with repositioning in bed. The client has no isolation orders, but the nurse notes a leaking Jackson-Pratt drain with fluid on the gown and linens. Which action best follows standard precautions?

- A.** Put on gloves before contact with the drain fluid and perform hand hygiene after glove removal
- B.** Use gloves only if the client is known to have a bloodborne infection
- C.** Wear a sterile gown and sterile gloves for all contact with the client
- D.** Delay repositioning until the provider writes an isolation order

35. A nurse is reinforcing teaching with a client who is 26 weeks pregnant about warning signs to report to the provider. Which client statement indicates correct understanding?

- A.** I should call if I notice swelling in my ankles at the end of the day.

B. I should call if I have vaginal bleeding or fluid leaking from my vagina.

C. I should call if I feel short of breath after climbing stairs.

D. I should call if I have occasional heartburn after meals.

36. A practical nurse is reinforcing teaching with the parent of a 17-year-old who recently got a part-time job and now wants to manage their own schedule, money, and transportation. The parent says, "I think this new independence is making my teen pull away from the family." Which response by the nurse is most appropriate?

A. "This behavior usually means your teen is becoming disrespectful and needs stricter limits right away."

B. "Wanting more independence is expected at this age, and clear rules can help your teen practice responsibility safely."

C. "Teens who ask for more control are often not ready for adult responsibilities yet."

D. "It would be best to make all major decisions for your teen until graduation."

37. A practical nurse is reinforcing teaching for a group of adults about levels of prevention. Which activity is an example of secondary prevention?

A. Teaching clients to use seat belts on every car ride

B. Performing blood pressure checks at a community health fair

C. Providing rehabilitation exercises after a stroke

D. Administering prescribed antibiotics for pneumonia

38. A nurse is obtaining a health history from an adult client who says, "I use herbal supplements and teas because they're natural, so they don't really count as medicine." Which response by the nurse is best?

A. "Natural products are usually safe, so I only need to document prescription medications."

B. "Tell me the names, amounts, and how often you use those products so I can include them in your medication history."

C. "You should stop all herbal products before I continue the interview."

D. "I will note that you prefer home remedies instead of standard treatment."

39. A practical nurse is collecting baseline physical data from an older adult admitted to a long-term care facility. Which finding best reflects the client's baseline mobility status?

A. The client walked 50 feet with a walker and one-person assist before becoming tired

B. The client states, "I hope physical therapy will help me get stronger soon."

C. The client's daughter says the client has always been very independent

- D.** The client reports being worried about falling in the new room
- 40.** A nurse is teaching a pregnant client about vaccines recommended during pregnancy. Which statement by the client indicates correct understanding?
- A.** I should wait until after delivery to receive the Tdap vaccine.
 - B.** Getting the Tdap vaccine during pregnancy can help protect my newborn after birth.
 - C.** If I received Tdap in a previous pregnancy, I do not need it again this pregnancy.
 - D.** The Tdap vaccine is only recommended if I have a puncture wound during pregnancy.
- 41.** A nurse is collecting data during a routine health visit for a 4-year-old child. Which behavior should the nurse identify as an expected finding for this age group?
- A.** Cuts food with a table knife independently
 - B.** Rides a two-wheel bicycle without training wheels
 - C.** Uses four-word sentences and asks many questions
 - D.** Shows readiness to think abstractly about consequences
- 42.** A nurse is explaining newborn safe-sleep guidelines to a client who had a cesarean birth 2 hours ago. The client repeatedly closes her eyes, asks the nurse to repeat information, and says, "I'm so tired right now." Which action by the nurse is best?
- A.** Continue the teaching now so the client receives all discharge information as early as possible
 - B.** Ask the client's support person to listen instead and consider the teaching complete
 - C.** Delay detailed teaching until the client is more rested, and return later to review the information
 - D.** Speak more loudly and provide the information a second time
- 43.** A nurse is preparing to teach a client newly prescribed warfarin about home management. The client says, "The print on that handout is blurry, and I forgot my glasses at home." Which action by the nurse is best?
- A.** Ask a family member to read the handout to the client later
 - B.** Continue the teaching now and speak more loudly
 - C.** Provide teaching materials in larger print and review the information verbally
 - D.** Postpone all teaching until the day of the follow-up appointment
- 44.** A client who was in a house fire yesterday is being treated for minor burns. While the practical nurse changes the dressing, the client says, "I keep seeing the smoke and hearing my kids scream. I can't stop thinking about it." Which response by the nurse best provides emotional support?
- A.** "Try not to think about it right now so you can stay calm during the dressing change."
 - B.** "That sounds frightening. Do you want to tell me more about what keeps coming back to your mind?"
 - C.** "At least everyone survived, so things could have been much worse."
 - D.** "You should focus on getting rest, because these feelings will probably pass soon."
- 45.** A client with newly diagnosed heart failure says, "There is so much to change that I don't even know where to begin." Which nursing response best supports adaptive coping?
- A.** "Try not to think about it so much, because stress will make your condition worse."
 - B.** "Let's focus on one manageable step today, such as making a simple plan for taking your medications."
 - C.** "You need to accept the diagnosis and follow all instructions exactly as prescribed."
 - D.** "Most clients adjust eventually, so you will probably feel better after a few days."
- 46.** A client with paranoid schizophrenia on an inpatient unit says to the nurse in a tense voice, "That man in the corner keeps sending me messages," while staring at another client across the room. The client is breathing rapidly and gripping the arms of the chair. Which action should the nurse take first?
- A.** Tell the client that no one is sending messages and the belief is not real
 - B.** Move the client to a quieter area and speak with the client calmly
 - C.** Ask the other client to explain what was said to prevent misunderstanding
 - D.** Instruct the client to ignore the messages and join the group activity
- 47.** A client who recently learned about a chronic kidney disease diagnosis tells the nurse, "I don't want to talk about it. My family is already worried enough." Which response by the nurse is most therapeutic?
- A.** "You need to discuss this with your family so they can help you."
 - B.** "It sounds like you are trying to protect your family while dealing with a lot yourself."
 - C.** "If you keep your feelings inside, your stress will only get worse."
 - D.** "Why don't you want to talk about your diagnosis?"

48. A nurse is caring for a client who becomes upset after a roommate turns on the television during rest time. The client starts breathing rapidly, speaks in a loud voice, and says, "I can't take this anymore. Everybody back off." The client is standing near the bed and is not making physical contact with anyone. Which response by the nurse is most appropriate?

- A.** "You need to calm down right now before this gets worse."
- B.** "Tell me what is upsetting you, and we can talk about it over here where it is quieter."
- C.** "Your roommate has the right to watch television, so you need to be more cooperative."
- D.** "If you keep talking like that, I will have to call security."

49. The practical nurse is caring for an older adult with dementia who becomes agitated during bathing and clenches both fists when staff try to remove the client's clothing quickly. Which action is most appropriate?

- A.** Continue the bath as fast as possible to finish the task before the agitation worsens
- B.** Explain each step slowly, offer one direction at a time, and allow the client to participate as able
- C.** Ask several staff members to assist so the bath can be completed without delay
- D.** Tell the client that refusing care is inappropriate and may lead to skin problems

50. A client preparing for discharge after a myocardial infarction tells the nurse, "I'm trying to handle stress by keeping busy every minute so I don't think about anything." Which response by the nurse is most appropriate?

- A.** "Staying constantly busy is the best way to prevent stress from building up."
- B.** "It may help to set aside short quiet periods each day for relaxation breathing or another calming activity."
- C.** "You should avoid talking about your stress until you feel completely recovered."
- D.** "If stress gets worse, try cutting back on your heart medications for a few days."

51. A client receiving palliative care says to the practical nurse, "I don't need pain medicine right now. I need to talk to someone from my faith before I can make peace with what's happening." Which action by the nurse is most appropriate?

- A.** Tell the client that pain control should come first and return after the next medication round
- B.** Ask the client whether they would like the nurse to help contact a spiritual adviser or faith representative
- C.** Explain that spiritual concerns are best discussed with the provider during rounds
- D.** Offer reassurance by saying that many clients feel this way and it usually passes

52. A nurse is assessing a client who recently gave birth. The client says, through an interpreter, "In my family, we stay in bed for several weeks after delivery and avoid bathing with cold water." Which action by the nurse is most appropriate?

- A.** Tell the client that prolonged bed rest is unhealthy and should be stopped immediately
- B.** Ask the client to describe the practice in more detail and discuss how to support it safely during recovery
- C.** Document that the client is noncompliant with standard postpartum care
- D.** Instruct the family that hospital routines must take priority over cultural traditions

53. A nurse enters a client's room and hears an unlicensed assistive personnel say, "I don't know what to call this client, so I'm avoiding talking to them." The client looks embarrassed and turns away. Which action by the nurse is most appropriate?

- A.** Tell the assistive personnel to use the name on the medical record until the client brings up another preference
- B.** Ask the client privately what name and pronouns they want used, then communicate that preference to staff
- C.** Reassign the assistive personnel so the client does not have to interact with them again
- D.** Explain to the client that some staff need time to adjust to unfamiliar gender identities

54. A client with newly diagnosed Ménière disease tells the nurse, "When the room starts spinning, I panic and feel like something terrible is happening to me." Which nursing response is best?

- A.** "Try to ignore the sensation, because focusing on it usually makes it worse."
- B.** "You are safe right now. Tell me what the episodes feel like for you, and we can talk about ways to manage the anxiety."
- C.** "Most clients get used to dizziness after a while, so you probably will too."
- D.** "You should avoid talking about the episodes so you do not become more upset."

55. A client receiving end-of-life care says to the nurse, "I know the doctors explained things, but I don't want to talk anymore. I just want someone to sit here with me for a little while." Which action by the nurse is most appropriate?

- A.** Respect the client's wish, remain quietly at the bedside, and offer presence without forcing conversation
- B.** Encourage the client to discuss feelings immediately so unresolved emotions can be addressed
- C.** Leave the room to give the client privacy and return later with written information about coping
- D.** Tell the client that family support is more helpful than staff presence and ask whom to call

56. A nurse is preparing to administer scheduled oral medications to a client who receives nutrition through an enteral feeding tube. Which action is most appropriate?

- A.** Crush all of the medications together, mix them with formula, and instill them through the tube at one time
- B.** Mix each medication with sterile saline and administer them while the client is lying flat
- C.** Administer each medication separately through the tube and flush with water between medications
- D.** Add the medications directly to the feeding bag so they infuse slowly with the enteral formula

57. A nurse is caring for a client who had a below-the-knee amputation 2 days ago. Which positioning action is most appropriate to help prevent a mobility complication?

- A.** Place a pillow under the residual limb to keep the knee slightly flexed
- B.** Encourage the client to keep the residual limb elevated on folded blankets all day
- C.** Assist the client to lie prone for short periods as prescribed
- D.** Keep the client in high-Fowler's position with the residual limb supported on a chair

58. A nurse is reinforcing dietary teaching for a client who has a stage 3 pressure injury. Which food selection by the client shows an understanding of the teaching?

- A.** Apple juice and gelatin
- B.** Grilled chicken, milk, and scrambled eggs
- C.** White toast with jam and tea
- D.** Steamed rice and canned peaches

59. A nurse is caring for a client with early cataracts who wants to continue performing morning grooming independently. Which intervention is most appropriate to support safe completion of this activity of daily living?

- A.** Place grooming supplies in a consistent location and provide bright, non-glare lighting
- B.** Complete the grooming tasks for the client to prevent eye strain
- C.** Arrange the room with dim lighting to reduce visual discomfort
- D.** Move grooming items each day so the client practices locating them

60. A nurse is caring for a client who has reflex urinary incontinence following a spinal cord injury. Which intervention is most appropriate to help the client empty the bladder on a regular schedule?

- A.** Encourage the client to wait until the urge to void is felt
- B.** Use intermittent catheterization at prescribed intervals
- C.** Limit oral fluids to reduce urine production

D. Offer a bedpan only at bedtime

61. A nurse is caring for a client who has been on bed rest for 3 days and reports sleeping poorly at night. Which intervention is most appropriate to help promote the client's sleep?

- A.** Encourage the client to stay in bed and conserve energy during the day
- B.** Offer a large snack and fluids at bedtime
- C.** Assist the client to sit in a chair and participate in daytime activity as tolerated
- D.** Keep the television on until the client falls asleep

62. A practical nurse is changing the dressing on a client's heel and observes a pressure injury with partial-thickness skin loss and a shallow pink wound bed. No slough or eschar is present. How should the nurse document this finding?

- A.** Stage 1 pressure injury
- B.** Stage 2 pressure injury
- C.** Stage 3 pressure injury
- D.** Unstageable pressure injury

63. A practical nurse is caring for a client with hearing impairment who communicates mainly through a hearing aid. Before starting discharge teaching, the nurse notices the client is nodding but answering questions inappropriately. Which action is most appropriate first?

- A.** Speak more loudly from the doorway
- B.** Check whether the hearing aid is turned on and functioning
- C.** Continue teaching and provide a written handout at the end
- D.** Ask a family member to explain the instructions instead

64. A client on strict intake and output has consumed the following since the start of the shift: 6 oz of apple juice, 120 mL of ice cream, and 1 cup of water. The client also had 90 mL of liquid medication by mouth. What total intake should the practical nurse document?

- A.** 510 mL
- B.** 570 mL
- C.** 630 mL
- D.** 690 mL

65. A practical nurse is caring for a client who has been immobilized for several days after a spinal injury. Which intervention is most appropriate to help prevent contractures?

- A.** Place the client in a high-Fowler position for all meals
- B.** Reposition the client every 8 hours
- C.** Perform range-of-motion exercises to the affected joints regularly

D. Massage reddened areas over bony prominences

66. A nurse is preparing to administer furosemide 40 mg PO to a client. After identifying the client, the nurse asks about allergies and learns the client had a severe rash after taking sulfonamide antibiotics in the past. Which action is most appropriate before giving the medication?

- A.** Administer the medication because furosemide is not an antibiotic
- B.** Hold the medication and clarify the order with the provider or pharmacist before administration
- C.** Give the medication with food to reduce the chance of an allergic reaction
- D.** Ask another nurse to administer the medication instead

67. A practical nurse is receiving a client transferred from a rehabilitation facility. The transfer paperwork lists levothyroxine 75 mcg by mouth each morning, but the MAR on the unit does not include this medication. The client says, "I've been taking my thyroid pill every day." What is the nurse's best action?

- A.** Administer the client's own levothyroxine from the bedside drawer until the provider enters an order
- B.** Add levothyroxine to the MAR based on the transfer paperwork
- C.** Hold the medication and notify the provider to clarify and complete medication reconciliation
- D.** Skip today's dose and wait to mention the discrepancy during end-of-shift report

68. A nurse needs to infuse 250 mL of IV fluid over 2 hours using gravity tubing with a drop factor of 12 gtt/mL. What flow rate should the nurse regulate in drops per minute?

- A.** 20 gtt/min
- B.** 25 gtt/min
- C.** 30 gtt/min
- D.** 35 gtt/min

69. A practical nurse is preparing to give a prescribed controlled analgesic to a client who had surgery 30 minutes ago. Which action is most important for the nurse to take immediately before administering the medication?

- A.** Review the client's most recent respiratory rate and level of sedation
- B.** Ask the client whether the medication was effective the last time it was taken
- C.** Document the administration time on the MAR before giving the medication
- D.** Offer the client a snack to reduce gastrointestinal upset

70. A nurse is monitoring a client who is receiving analgesia through an epidural catheter after surgery. Which assessment finding is most important to report immediately?

- A.** Blood pressure 84/50 mm Hg
- B.** Pain rating of 4 on a 0-to-10 scale
- C.** Itching at the tape site
- D.** Temperature of 99.1°F (37.3°C)

71. A nurse is preparing to administer the next dose of phenytoin to a client who has a seizure disorder. Which pre-administration data is most important for the nurse to review?

- A.** The client's most recent serum phenytoin level
- B.** The client's preferred time for taking the medication
- C.** The number of visitors at the bedside
- D.** The client's fluid intake from the previous shift

72. A client with major depressive disorder has been taking sertraline for 5 weeks. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client reports sleeping 10 hours each night
- B.** The client states, "I am starting to enjoy my usual activities again."
- C.** The client has a dry mouth after each dose
- D.** The client reports mild nausea during the first week of therapy

73. A client who received morphine 30 minutes ago after surgery is now difficult to arouse and has a respiratory rate of 8/min. Which adverse response should the nurse identify?

- A.** Orthostatic hypotension
- B.** Respiratory depression
- C.** Paradoxical excitation
- D.** Opioid withdrawal

74. A patient who takes simvastatin each evening says, "I started drinking grapefruit juice every morning because I'm trying to eat healthier." Which response by the nurse is best?

- A.** "That is fine as long as you take the simvastatin at night instead of in the morning."
- B.** "Grapefruit juice can increase the level of simvastatin in your body, so you should avoid it unless the prescriber says it is safe."
- C.** "Grapefruit juice decreases the effect of simvastatin, so you should drink more of it to balance the dose."
- D.** "This combination is only a problem if you also take the medication with food."

75. A client with epilepsy is prescribed phenytoin 100 mg by mouth three times daily. Before giving the next dose, the nurse reviews the laboratory results and notes a serum albumin level of 2.1 g/dL and a total phenytoin level of 18 mcg/mL. Which action is most appropriate?

- A.** Administer the dose as prescribed because the phenytoin level is within the usual therapeutic range
- B.** Hold the dose and notify the provider because low albumin can increase the active phenytoin level
- C.** Give the medication with food to reduce gastric upset and recheck the level later
- D.** Document the finding as expected and continue to monitor for seizure activity only

76. A nurse is teaching a client who has a new prescription for phenytoin to prevent seizures. Which statement by the client indicates correct understanding of the teaching?

- A.** I should stop taking the medication as soon as I have been seizure-free for a few days.
- B.** If my gums start to bleed, I will brush them harder to keep them healthy.
- C.** I need to avoid suddenly stopping this medication and keep my follow-up appointments for blood tests.
- D.** I can safely take this medication with any over-the-counter herbal product.

77. A nurse is preparing to administer an IV antibiotic that was removed from the refrigerator 2 hours ago. The label states, "Store refrigerated. Discard if left at room temperature longer than 1 hour." The solution still looks clear, and the expiration date is next month. Which action should the nurse take?

- A.** Administer the medication because the solution is clear and not expired
- B.** Warm the medication in the hands and then administer it
- C.** Discard the medication and obtain a new dose
- D.** Return the medication to the refrigerator for later use

78. A nurse needs to administer levothyroxine 0.05 mg by mouth. The available tablets are labeled 25 mcg each. How many tablets should the nurse give?

- A.** 1 tablet
- B.** 2 tablets
- C.** 2.5 tablets
- D.** 5 tablets

79. A practical nurse is assisting with care planning for four clients in a long-term care facility. Which client should the nurse identify as being at greatest risk for falling?

- A.** A client with chronic arthritis who uses a walker correctly and asks for help before standing

- B.** A client with Parkinson disease who has a shuffling gait and recently started a sedative at bedtime
- C.** A client with hearing loss who wears bilateral hearing aids and ambulates independently
- D.** A client with well-controlled hypertension who reports occasional fatigue after physical therapy

80. A nurse is obtaining a capillary blood specimen for glucose testing from an adult client. Which action is most appropriate to promote an accurate specimen?

- A.** Puncture the center of the fingertip pad
- B.** Wipe away the first drop of blood before collecting the sample
- C.** Use vigorous milking of the finger to obtain enough blood
- D.** Clean the site with alcohol and collect the sample before the skin dries

81. A nurse is reviewing laboratory results for a client who has been receiving phenytoin for seizure control. Which result should the nurse identify as most concerning?

- A.** White blood cell count 7,800/mm³
- B.** Platelet count 42,000/mm³
- C.** Hemoglobin 13.6 g/dL
- D.** Sodium 138 mEq/L

82. A nurse is using a bedside urine dipstick to test a specimen for protein and glucose. After dipping the strip, the nurse leaves it on the bedside table and returns 3 minutes later to compare the colors with the container chart. Which statement best describes this result?

- A.** The result is reliable because the strip was fully immersed before being removed
- B.** The result is reliable if the urine specimen was freshly collected
- C.** The result may be inaccurate because the strip was not read at the recommended time
- D.** The result may be inaccurate only if the client is taking antibiotics

83. A practical nurse is preparing to measure an adult client's blood pressure. The client has a peripherally inserted central catheter (PICC) in the right arm, and the left arm has an arteriovenous fistula for hemodialysis. Which action is most appropriate?

- A.** Measure the blood pressure in the right arm above the PICC site
- B.** Measure the blood pressure in the left arm below the fistula
- C.** Use a lower extremity or obtain another approved site per facility policy
- D.** Skip the blood pressure and document that it cannot be obtained

84. A client is scheduled for a percutaneous liver biopsy. Which nursing action is most important before the procedure?

- A.** Have the client drink clear fluids up to 2 hours before the biopsy
- B.** Check the client's coagulation studies and platelet count
- C.** Place the client in high-Fowler's position
- D.** Ask the client to empty the bladder after the procedure

85. A nurse is preparing to obtain a 12-lead ECG for a client with a very hairy chest. Which action is most appropriate to help ensure accurate electrode contact?

- A.** Apply the electrodes over the hair and secure them with extra tape
- B.** Shave or clip the hair at the electrode sites before applying the electrodes
- C.** Moisten the hair with alcohol and place the electrodes immediately
- D.** Place the limb leads on the abdomen instead of the chest

86. A nurse is preparing to obtain a venous blood specimen from an adult client. The nurse palpates a bouncy vein in the cephalic area near the wrist and another in the median cubital area. Which site should the nurse choose first to reduce the risk of client injury?

- A.** The cephalic vein near the wrist because it is smaller and less likely to bruise
- B.** The cephalic vein near the wrist because it is easier to stabilize with the nondominant hand
- C.** The median cubital vein because it is farther from major nerves and arteries
- D.** Either site because both veins are equally appropriate for routine venipuncture

87. A practical nurse is preparing a client for surgery. Which client statement requires the nurse to notify the RN or provider before the client is taken to the operating room?

- A.** I drank black coffee with sugar about 2 hours ago.
- B.** I removed my wedding ring and put it in the drawer.
- C.** I used the bathroom right before getting into bed.
- D.** I signed the consent form last night.

88. A client has a chest tube connected to a drainage system after thoracic surgery. During assessment, the nurse notes continuous bubbling in the water-seal chamber. What is the nurse's priority action?

- A.** Increase the wall suction pressure
- B.** Check the tubing and drainage system for an air leak
- C.** Clamp the chest tube near the client's chest
- D.** Strip the tubing to remove possible clots

89. A nurse is caring for a client who had a total hip arthroplasty earlier today. The client suddenly reports shortness of breath and says, "My chest feels strange." Which focused assessment should the nurse perform first?

- A.** Ask the client to rate the incisional pain on a 0 to 10 scale
- B.** Inspect the operative hip dressing for drainage
- C.** Assess respiratory rate, oxygen saturation, and breath sounds
- D.** Check the client's appetite and fluid intake since surgery

90. A client with hypothyroidism started levothyroxine 6 weeks ago. Which finding best indicates the treatment is effective?

- A.** Heart rate decreases from 88/min to 54/min
- B.** The client reports improved energy and less sensitivity to cold
- C.** The client has a 4-lb weight gain in 1 week
- D.** Deep tendon reflexes become more sluggish

91. A nurse cares for an older adult who has hypothyroidism. The room is cool, and the client reports feeling very cold. Which finding should the nurse recognize as the clearest sign of impaired thermoregulation requiring prompt follow-up?

- A.** Oral temperature of 35.8°C (96.4°F)
- B.** Requesting an extra blanket at bedtime
- C.** Heart rate of 88/min after walking to the bathroom
- D.** Drinking a warm beverage with breakfast

92. A nurse assesses a client's wound and notes thick drainage that is green with a foul odor. How should the nurse document this drainage?

- A.** Serous drainage
- B.** Sanguineous drainage
- C.** Purulent drainage
- D.** Serosanguineous drainage

93. A nurse is performing CPR on an adult client when another rescuer takes over and begins giving very rapid, shallow compressions with frequent pauses to check for a pulse. Which outcome is the greatest concern with this technique?

- A.** Decreased coronary and cerebral perfusion during resuscitation
- B.** Increased likelihood of rib fracture from deeper compressions
- C.** Improved oxygen delivery because compressions are given faster
- D.** Better cardiac output because pauses allow the heart to refill

94. A client with asthma is receiving oxygen by simple face mask at 6 L/min. The client says, "This mask feels suffocating," and the nurse notes the oxygen tubing has become disconnected from the wall source. Which action should the nurse take first?

- A.** Reconnect the oxygen tubing to the wall source
- B.** Remove the mask and notify the provider
- C.** Lower the flow rate to 2 L/min to reduce discomfort
- D.** Switch the client to a nasal cannula at 6 L/min

95. A nurse is caring for a client with a long-term tracheostomy who is breathing comfortably and asks why room humidification was recommended at home. Which explanation by the nurse is best?

- A.** Humidification helps prevent secretions from becoming thick and difficult to clear.
- B.** Humidification keeps the tracheostomy stoma from closing too quickly.
- C.** Humidification eliminates the need for routine tracheostomy cleaning.
- D.** Humidification reduces the risk of aspiration by strengthening the cough reflex.

96. A client receiving mechanical ventilation becomes suddenly restless and tachycardic. The ventilator's high-pressure alarm is sounding, and the nurse notes diminished breath sounds on the right side. Which action should the nurse take first?

- A.** Reposition the client and continue to observe
- B.** Administer a prescribed sedative for anxiety
- C.** Disconnect the ventilator and manually ventilate the client with 100% oxygen while calling for help
- D.** Silence the alarm and prepare for routine endotracheal suctioning

97. A client with a demand pacemaker is on telemetry.

The nurse notes pacing spikes occurring at inappropriate times, including within the client's intrinsic beats, and the client reports occasional palpitations. Which pacemaker problem does this finding suggest?

- A.** Failure to capture
- B.** Failure to sense
- C.** Failure to pace
- D.** Normal pacemaker function

98. A nurse is changing the pouch for a client with a stoma. To help prevent leakage and protect the peristomal skin, which action is most appropriate?

- A.** Cut the skin barrier opening about 1/2 inch larger than the stoma
- B.** Apply the pouching system to moist skin so it seals more easily

C. Measure the stoma and cut the skin barrier to fit closely around it

D. Use alcohol to clean the peristomal skin before attaching the pouch

99. A nurse is caring for a client with chronic obstructive pulmonary disease who has been stable on 2 L/min oxygen by nasal cannula. Which change in assessment should the nurse recognize as the most urgent?

- A.** The client reports a dry mouth and asks for ice chips.
- B.** The client becomes increasingly restless and confused.
- C.** The client has a productive cough with thick yellow sputum.
- D.** The client states feeling tired after walking to the bathroom.

100. A nurse is caring for a client admitted with confusion after drinking large amounts of water during a heat wave. The client's laboratory report shows a serum sodium level of 122 mEq/L. Which assessment finding is the nurse's priority to monitor?

- A.** Generalized muscle twitching or seizure activity
- B.** Increased thirst and dry oral mucosa
- C.** Deep tendon reflexes that are diminished
- D.** Blood pressure increasing from 118/70 mm Hg to 124/76 mm Hg

Simulation Test 2 — Answer Key

1. B	26. C	51. B	76. C
2. C	27. B	52. B	77. C
3. C	28. A	53. B	78. B
4. A	29. B	54. B	79. B
5. B	30. C	55. A	80. B
6. C	31. B	56. C	81. B
7. C	32. C	57. C	82. C
8. C	33. B	58. B	83. C
9. B	34. A	59. A	84. B
10. A	35. B	60. B	85. B
11. A	36. B	61. C	86. C
12. C	37. B	62. B	87. A
13. B	38. B	63. B	88. B
14. C	39. A	64. B	89. C
15. C	40. B	65. C	90. B
16. B	41. C	66. B	91. A
17. B	42. C	67. C	92. C
18. C	43. C	68. B	93. A
19. C	44. B	69. A	94. A
20. B	45. B	70. A	95. A
21. B	46. B	71. A	96. C
22. A	47. B	72. B	97. B
23. C	48. B	73. B	98. C
24. B	49. B	74. B	99. B
25. B	50. B	75. B	100. A

Simulation Test 3

100 questions

1. A practical nurse on a surgical unit has these tasks pending 20 minutes before a client's scheduled transfer from the postanesthesia care unit. Which action should the nurse complete first?

- A. Verify that suction and oxygen equipment are set up and working at the bedside
- B. Print the client's medication administration record for the chart
- C. Call dietary services to request a clear-liquid tray for later
- D. Restock the room with extra linens and bath supplies

2. A practical nurse in a clinic is working with an assistive personnel (AP) worker. Which task is most appropriate for the nurse to delegate to the AP?

- A. Teach a client how to collect a clean-catch urine specimen at home
- B. Call a client to determine whether a newly prescribed antibiotic is causing a rash
- C. Obtain a height and weight for a stable adolescent during a routine visit
- D. Review discharge instructions with a client who has a new diagnosis of hypertension

3. An alert adult client with newly diagnosed diabetes tells the nurse, "Please do not discuss my condition while my employer is visiting. I haven't told anyone at work." A few minutes later, the employer arrives and says, "I'm like family. Just tell me what the doctor said so I can help." What is the nurse's best response?

- A. Explain the diagnosis because the employer states a close relationship with the client
- B. Ask the employer to step out briefly, then confirm with the client what information, if any, may be shared
- C. Give a general update without naming the diagnosis so the employer can still be helpful
- D. Tell the employer to wait until discharge, when the client can decide whether to share information

4. A practical nurse receives a handwritten note from a client's coworker that says, "Please tell me if Jordan will be back at work next week. We are worried." The coworker is waiting in the hallway. The client is resting in bed, is alert and oriented, and has not discussed sharing information with the coworker. Which action should the nurse take?

- A. Tell the coworker that the client is stable but discharge plans are not final
- B. Ask the client whether the client wants information shared with the coworker before responding

- C. Explain the diagnosis to the coworker but avoid discussing treatment details
- D. Give the coworker the expected discharge date because it is not as private as the diagnosis

5. An alert adult client preparing for discharge asks, "If I complete an advance directive, will my family be able to override it later if they disagree?" Which response by the nurse is best?

- A. Yes, the family can change the directive if most relatives agree with a different plan.
- B. Yes, but only a spouse can override an advance directive once it is signed.
- C. No, the client's expressed wishes in a valid advance directive should guide care if the client later cannot decide.
- D. No, because an advance directive takes effect immediately and replaces all current treatment decisions.

6. A client is scheduled for insertion of a permanent pacemaker tomorrow morning. During the evening assessment, the client says, "I signed the consent earlier, but now I am not sure I want the procedure anymore." Which action by the nurse is most appropriate?

- A. Tell the client that the provider already explained the procedure and the consent is still valid
- B. Ask the client to wait until the morning to discuss the concern with the procedure team
- C. Notify the provider and document that the client is reconsidering the decision
- D. Have a family member talk with the client about why the procedure is necessary

7. A nurse is preparing four clients for nonemergency procedures. For which client should the nurse identify that informed consent can be obtained directly from the client?

- A. A 16-year-old who is married and scheduled for an endoscopy
- B. An adult who received IV midazolam 20 minutes ago and is scheduled for cardioversion
- C. An adult with advanced dementia who is disoriented and scheduled for a biopsy
- D. An adult declared incompetent by the court who is scheduled for a cystoscopy

8. A client is being discharged after hospitalization for major depression. The client will go home with a new antidepressant prescription and a follow-up appointment at a community mental health clinic in 5 days. During discharge teaching, the client says, "I may not be able to get there because I don't have a car." Which action by the nurse is most appropriate before discharge?

- A. Ask the client to call the clinic later if transportation becomes a problem

- B.** Request a social work referral to help arrange transportation resources
- C.** Tell the client to use the emergency department for follow-up care
- D.** Postpone all discharge teaching until the follow-up appointment is confirmed

9. A client is preparing for discharge after treatment for a new spinal cord injury. The client will return home with an indwelling urinary catheter and bowel program. The spouse says, "I'll do everything," but is tearful and repeatedly performs the steps incorrectly during return demonstration. Which action should the nurse take first?

- A.** Notify the provider that discharge should be delayed until additional planning and caregiver support are arranged
- B.** Provide the spouse with written instructions and encourage practicing at home
- C.** Continue with discharge because the spouse is willing to help
- D.** Arrange the first follow-up visit for 1 month after discharge

10. A practical nurse is part of a team trying to reduce specimen collection errors in a clinic. The team plans to try a new labeling step with one staff member during the afternoon shift for 3 days before using it clinic-wide. What is the best reason for this approach?

- A.** It allows the team to test the change on a small scale before broader implementation
- B.** It guarantees the new process will eliminate all labeling errors
- C.** It replaces the need to collect data after the change is started
- D.** It ensures the change will work the same way on every shift

11. A long-term care facility wants to reduce overnight call-light delays. After reviewing baseline data, the team learns that delays are highest between 6:00 p.m. and 8:00 p.m., especially for clients needing toileting assistance. The practical nurse suggests assigning one assistive personnel staff member to make rounds focused on toileting during that time for 2 weeks. Which outcome would best show whether this change improved client care rather than only staff compliance?

- A.** The percentage of evenings when the rounding assignment sheet was completed
- B.** The number of staff who attended teaching about the new rounding plan
- C.** The average call-light response time for clients during the 6:00 p.m. to 8:00 p.m. period
- D.** The number of reminders the charge nurse gave staff to follow the rounding plan

12. A practical nurse is caring for a client with heart failure. The dietitian asks whether the client is following the low-sodium meal plan. Which response by the nurse best demonstrates effective interdisciplinary information sharing?

- A.** "I think the client understands the diet because teaching was done yesterday."
- B.** "The client ate canned soup brought by family at lunch and said, 'I don't think salt really affects my breathing.'"
- C.** "The client has heart failure, so sodium restriction is important."
- D.** "You can review the chart because all teaching should be documented there."

13. A practical nurse is caring for a client 1 day after abdominal surgery. The physical therapist plans to begin ambulation. Which information is most important for the nurse to communicate to the physical therapist before the session?

- A.** The client prefers to walk in the hallway after visitors leave
- B.** The client needed help choosing dinner from the menu
- C.** The client became lightheaded earlier when moving from bed to chair and had a blood pressure of 88/54 mm Hg
- D.** The client asked when the dressing will be changed again

14. A practical nurse is caring for a client who has decision-making capacity and a written advance directive refusing artificial tube feedings. After the client becomes unable to speak, the client's adult son tells the nurse, "Start the feeding anyway. My mother would want every chance to live." Which action by the nurse is most appropriate?

- A.** Insert the feeding tube because the son is the next of kin and can override the document
- B.** Follow the advance directive and notify the charge nurse and provider of the son's concerns
- C.** Ask another family member to decide whether the advance directive should still apply
- D.** Delay all care until the ethics committee meets and makes a final decision

15. Near the end of a shift, a practical nurse realizes that a sterile dressing change performed 2 hours earlier was never charted. The wound is now covered with a clean dry dressing, and the nurse accurately remembers the assessment findings and care provided. Which documentation action is most appropriate?

- A.** Add the documentation to the earlier time slot so the record appears complete
- B.** Ask the oncoming nurse to document the dressing change since the wound is currently intact

- C.** Enter a late note with the current time and include the actual time the care was performed
- D.** Leave the dressing change undocumented because late entries can create legal problems

16. A newly licensed practical nurse suspects that a hospitalized 6-year-old child may be experiencing physical abuse. The nurse tells the charge nurse, who says, "I'll handle it later." Which action should the practical nurse take next?

- A.** Wait for the charge nurse to decide whether a report is necessary
- B.** Document the concern and file the report according to agency policy
- C.** Ask the child's teacher to verify whether abuse has occurred
- D.** Question the caregiver repeatedly until a confession is obtained

17. A nurse in a home health agency visits a 34-year-old client with a developmental disability. The client has a healing wrist fracture, several small burns on the forearm, and appears fearful when a paid caregiver enters the room. The caregiver explains, "Accidents happen because he is clumsy," and asks the nurse not to "cause trouble." The client cannot clearly describe what happened. Which action is most appropriate?

- A.** Delay reporting until the nurse can collect more evidence from several visits
- B.** Report suspected abuse of the vulnerable adult to the appropriate authority and document objective findings
- C.** Confront the caregiver immediately and tell the caregiver that abuse has been proven
- D.** Respect the caregiver's request for privacy because the client did not directly disclose abuse

18. A practical nurse is delegating tasks to an assistive personnel (AP) in an assisted-living facility. Which task is most appropriate for the nurse to assign to the AP?

- A.** Teach a client how to use an incentive spirometer after pneumonia
- B.** Reinforce discharge instructions about a new insulin regimen
- C.** Measure and record the weight of a stable client with chronic heart failure
- D.** Assess a client who reports new burning pain with urination

19. A practical nurse asks an assistive personnel (AP) to help a client transfer from the bed to a chair for the first time after surgery. The AP says, "The client almost fell because the legs buckled, but I got the client into the chair." Which action should the nurse take first?

- A.** Instruct the AP to use a gait belt the next time the client transfers

- B.** Assess the client for injury and current status
- C.** Document the event in the medical record
- D.** Complete an incident report

20. A practical nurse in a school-based health clinic is helping with vaccine follow-up for four adolescents. Which situation should the nurse identify as the clearest equity-related barrier to care?

- A.** A parent asks for another printed copy of the vaccine information sheet
- B.** A student says the family recently moved and misplaced the appointment card
- C.** A parent says the family cannot attend the only follow-up clinic because it is held during the parent's work shift and no evening option is available
- D.** A student says an injection made the arm sore for 1 day after a previous vaccine

21. A practical nurse working in a free community clinic notices that many clients with chronic illnesses do not return for monitoring after the first visit. Review of missed appointments shows that most of these clients rely on public transportation, many have changing work schedules, and reminder calls often go unanswered because phone service is interrupted. Which clinic change would best promote equitable access to ongoing care?

- A.** Require clients to confirm appointments by phone 24 hours in advance or be removed from the schedule
- B.** Offer walk-in follow-up blocks during extended hours and provide reminder options such as mailed notices or in-person scheduling before clients leave
- C.** Schedule all clients with chronic illnesses for the earliest morning appointments to reduce waiting times
- D.** Give each client a detailed handout explaining why missed follow-up visits can worsen disease control

22. A nurse is preparing to change the dressing on a client with contact precautions. The client is in bed and says, "I know why you're here—you changed this dressing yesterday." The medication administration record is not being used for this task. Which action should the nurse take to verify the client's identity before starting the procedure?

- A.** Ask the client to state their full name and date of birth, and compare the information with the identification band
- B.** Confirm the client's identity by checking the room number and the isolation sign on the door
- C.** Rely on the client's recognition of the nurse because the same nurse provided care the day before
- D.** Ask another staff member whether this is the correct client for the dressing change

23. A nurse is preparing to use a bed alarm for an older adult client who is confused and tries to get up without assistance. After the alarm pad is placed and connected, which action is most important before the nurse leaves the room?

- A.** Lower the bed and place the call light within reach
- B.** Test the alarm to make sure it activates correctly
- C.** Explain to the client that the alarm will remind them to stay in bed
- D.** Ask assistive personnel to check on the client every 2 hours

24. A nurse is preparing to apply telemetry leads to a newly admitted client. The client says, "Bandages and tapes make my skin blister badly." Which action should the nurse take first?

- A.** Apply the standard adhesive leads and monitor the skin every 4 hours
- B.** Ask the client to describe the reaction and notify the provider if adhesive sensitivity is suspected
- C.** Shave the client's chest to improve lead contact before applying the leads
- D.** Place the leads over a thin layer of gauze to protect the skin

25. During evening rounds in an assisted-living facility, the practical nurse sees a frayed electrical cord on a client's bedside lamp. The lamp is plugged in, and the client says, "It still works fine." Which action should the nurse take first?

- A.** Unplug the lamp and remove it from use
- B.** Wrap the damaged area with tape so the cord is covered
- C.** Move the lamp farther from the bed so the client will not touch it
- D.** Tell the client to avoid using the lamp until maintenance can inspect it

26. A hospitalized client with alcohol withdrawal is awake, anxious, and repeatedly tries to pull off the cardiac monitor leads. The client is not attempting to strike staff and becomes calmer when the nurse speaks in a low voice. Which action by the nurse best follows the principle of using the least-restrictive intervention?

- A.** Apply bilateral wrist restraints to prevent removal of the leads
- B.** Ask the provider for a restraint order so restraints are available if needed later
- C.** Stay with the client, reduce stimulation, and use calm verbal reorientation
- D.** Raise all 4 side rails to keep the client from moving around in bed

27. A nurse preparing a client for transport to radiology notices that the stretcher's wheel lock on one side does not engage. The client has not been moved and no injury has occurred. Which action should the nurse take first?

- A.** Use the stretcher carefully for this one trip and report the problem afterward
- B.** Move the client to the stretcher with help from another nurse
- C.** Remove the stretcher from use and report the equipment problem according to facility policy
- D.** Document that no harm occurred and continue with transport

28. A nurse finds an unlabeled syringe containing a clear liquid on the medication cart after morning medications have been passed. No client has reported symptoms, and the nurse cannot verify the syringe's contents or intended client. Which action is most appropriate?

- A.** Ask nearby staff whether the syringe belongs to them and use it if someone identifies it
- B.** Discard the syringe safely and report the incident per facility policy
- C.** Set the syringe aside until the next shift can investigate
- D.** Label the syringe as 'unknown' and send it to the pharmacy

29. A nurse is caring for a client who has severe right-sided weakness after a stroke and is being moved from a wheelchair to the bed with a full-body mechanical lift. Which action by the nurse is most appropriate during the transfer?

- A.** Lock the wheels of both the bed and the wheelchair before moving the client
- B.** Lower the bed completely to the floor after the client is lifted
- C.** Place the client's stronger side away from the bed during the transfer
- D.** Remove the lift sling as soon as the client's body clears the wheelchair

30. A practical nurse is preparing to administer insulin to a client using a pen device that was found in the medication cart without a name label. Another client in the same room also receives insulin. Which action should the nurse take?

- A.** Use the pen for the scheduled dose after changing the needle
- B.** Ask the clients which one usually uses the pen, then give the dose
- C.** Set the pen aside and obtain a client-specific pen or a new insulin source per facility policy
- D.** Clean the pen with disinfectant and use it for either client

- 31.** A nurse is preparing to place a peripheral IV in a cooperative client who is not on isolation precautions. The nurse expects contact with blood at the insertion site but does not anticipate splashing or spraying. Which personal protective equipment is most appropriate?
- A.** Gloves only
 - B.** Gloves and gown
 - C.** Gloves, mask, and eye protection
 - D.** Sterile gloves, gown, mask, and eye protection
- 32.** A practical nurse is preparing a sterile field for a bedside wound procedure. After the sterile drape is opened, the nurse notices that a corner of the drape is hanging below the level of the overbed table. Which action should the nurse take?
- A.** Pull the drape back up onto the table without touching the top surface
 - B.** Continue setup because only the hanging portion is contaminated
 - C.** Replace the drape and set up a new sterile field
 - D.** Cover the hanging corner with a sterile towel and proceed
- 33.** A practical nurse is caring for a client who has suspected *Clostridioides difficile* infection and is on contact precautions. Which action is most appropriate for the nurse to take after removing gloves and gown and leaving the client's room?
- A.** Clean hands with an alcohol-based hand rub
 - B.** Wash hands thoroughly with soap and water
 - C.** Put on a new pair of sterile gloves before charting
 - D.** Use a disinfectant wipe on the stethoscope, then perform hand hygiene later
- 34.** A practical nurse is preparing to irrigate a client's colostomy. The client has no isolation orders, but the nurse expects liquid stool may splash during the procedure. Which action best reflects standard precautions?
- A.** Put on gloves and a face shield before starting the irrigation
 - B.** Wear sterile gloves because the procedure involves a body opening
 - C.** Use only a gown because stool is not considered an infectious body fluid
 - D.** Skip personal protective equipment unless the client is known to have an infection
- 35.** A nurse is reinforcing teaching with a client who is 12 weeks pregnant about expected body changes during pregnancy. Which client statement indicates correct understanding?
- A.** I should expect to feel my baby move sometime this week.
 - B.** A small amount of vaginal spotting after exercise is a normal change in pregnancy.
 - C.** My breasts may feel tender and become fuller during the first trimester.
 - D.** Swelling of my face in the morning is a common early sign of a healthy pregnancy.
- 36.** A laboring client at 40 weeks' gestation is receiving oxytocin. The contraction pattern is every 1.5 minutes, lasting 95 seconds, and the fetal heart rate shows recurrent late decelerations. Which action should the nurse take first?
- A.** Discontinue the oxytocin infusion
 - B.** Assist the client to begin pushing with the next contraction
 - C.** Increase the rate of the primary IV infusion
 - D.** Document the findings and continue to observe
- 37.** A practical nurse is helping with a community education program on breast health. Which statement by a participant shows correct understanding of screening and prevention?
- A.** "If I do monthly self-checks, I do not need routine screening tests."
 - B.** "Screening can help find a problem before I notice any symptoms."
 - C.** "Screening is only useful for people who already have pain or a lump."
 - D.** "A normal screening result means I will never develop the disease later."
- 38.** A nurse is obtaining a health history from an adult client who reports frequent headaches. Which question is best to assess factors that may trigger the headaches?
- A.** "When did you first begin having headaches?"
 - B.** "How would you rate the headache pain on a scale of 0 to 10?"
 - C.** "What seems to happen before the headaches start?"
 - D.** "Where is the headache usually located?"
- 39.** A practical nurse is collecting baseline physical data from a newly admitted adult client. Which information best represents the client's baseline respiratory status?
- A.** The client currently has crackles in both lower lobes after walking from the stretcher to the bed.
 - B.** The client states, "I usually get short of breath only after climbing one flight of stairs."
 - C.** The client reports chest tightness that began 20 minutes ago.
 - D.** The client has an oxygen saturation of 88% on room air at admission.

40. A nurse is reinforcing teaching for an older adult who will receive the herpes zoster vaccine. Which statement by the client indicates correct understanding of the purpose of this immunization?

- A.** It will cure shingles if I get the rash later.
- B.** It lowers my risk of developing shingles and related nerve pain.
- C.** It replaces the need for the yearly influenza vaccine.
- D.** It is mainly used to treat chickenpox in adults.

41. A nurse is reinforcing teaching with the parent of a 6-month-old infant. Which observation should the nurse identify as an expected developmental finding?

- A.** The infant pulls to a standing position while holding onto furniture
- B.** The infant sits briefly leaning forward on both hands
- C.** The infant uses 2-word phrases to request a toy
- D.** The infant hops on one foot without support

42. A nurse is teaching a client how to perform pin-site care at home. The client says, "I can read, but these instructions don't make much sense to me." Which action by the nurse is best to reduce this communication barrier?

- A.** Ask the client to sign the teaching form and review the instructions again later
- B.** Provide detailed medical terminology so the client can become familiar with the correct words
- C.** Demonstrate the skill step by step and ask the client to show it back
- D.** Give the client several pamphlets on bone healing to read before discharge

43. A nurse is teaching a client who was newly diagnosed with glaucoma how to instill prescribed eye drops at home. During the teaching session, the client repeatedly asks the nurse to repeat instructions and responds to questions with unrelated answers. Which action by the nurse is best?

- A.** Speak more loudly and continue the teaching session
- B.** Ask the client to read the printed instructions independently
- C.** Assess for factors interfering with learning, such as hearing difficulty or distraction
- D.** Postpone all teaching until the day of the follow-up appointment

44. A client who recently had a below-the-knee amputation says to the practical nurse, "I don't even want to look at myself anymore." Which response by the nurse best provides emotional support?

- A.** You need to stay positive and focus on your recovery.
- B.** Many clients feel this way at first, so try not to think about it.

C. It sounds like this change is very upsetting for you.

Would you like to talk about what feels hardest right now?

D. Once you get your prosthesis, you will feel much better.

45. A client who is 2 days post-myocardial infarction tells the nurse, "I keep thinking about everything that could go wrong after I go home." Which nursing response best promotes effective coping?

A. "Try not to think about that right now. Stress will only make things worse."

B. "Tell me which part of going home worries you most so we can talk through it together."

C. "Most clients feel better after a few days, so you probably won't need to worry."

D. "You should focus on following instructions exactly and not dwell on negative thoughts."

46. A nurse in a psychiatric unit is leading a small activity group when one client abruptly stands up, kicks a chair, and says, "Nobody here better touch my things again." The client is breathing rapidly and glaring at another client. Which action should the nurse take first?

A. Ask the other clients to explain what happened

B. Tell the client that threatening behavior is unacceptable

C. Guide the client to a quieter area and speak in a calm, clear manner

D. Administer a prescribed sedative medication immediately

47. A client who has just been told that discharge will be delayed says, "This always happens to me. Nothing ever works out." Which response by the nurse is most therapeutic?

A. "You should try to stay positive so you do not make this harder on yourself."

B. "Tell me more about what is most upsetting about the delay."

C. "At least the delay means the provider wants to be extra careful."

D. "There is no reason to be so discouraged because it is only one more day."

48. During a home health visit, a client becomes increasingly agitated when discussing missed pain medication doses. The client begins speaking in a loud voice, paces across the room, and says, "You people never do anything right." Which action by the nurse is most appropriate?

A. Stand directly in front of the client and firmly instruct the client to sit down

B. Speak calmly, allow personal space, and ask the client what is most upsetting right now

C. Explain in detail why the medication schedule was delayed and remind the client to stay respectful

D. Tell the client that the visit will end unless the client stops raising their voice

49. A client with moderate dementia becomes anxious in the late afternoon, paces near the exit, and says, "I need to get to work before I'm late." Which action by the practical nurse is most appropriate?

- A.** Explain firmly that the client is retired and must stop trying to leave
- B.** Ask the client to describe the job, then redirect the client to help fold towels in a quiet area
- C.** Place the client in the room alone until the pacing stops
- D.** Tell the client that leaving is not allowed and apply restraints if the client continues

50. A client says to the nurse, "When I start to feel stressed, I scroll on my phone for hours and skip my usual walk because I don't have the energy." Which response by the nurse is most appropriate?

- A.** Try to ignore the stress so it does not get worse.
- B.** Replace some screen time with a short daily walk or another regular physical activity.
- C.** Stay busy with your phone until the stressful feelings pass.
- D.** Avoid all activity until your stress is completely gone.

51. A client who is being treated for a new cancer diagnosis tells the practical nurse, "I am not sure what I believe anymore, but I keep wondering why this is happening to me." Which response by the nurse is most appropriate?

- A.** Everything happens for a reason, so try to stay positive.
- B.** Many people feel this way when they are very sick. What thoughts have been on your mind?
- C.** You should ask your healthcare provider to explain your diagnosis again.
- D.** Would you like me to tell you about my own beliefs about illness?

52. A nurse is preparing to perform a urinary catheterization on a client. The client states, "In my culture, intimate care should be given by someone of the same sex whenever possible." A staff member of the same sex is available on the unit but will not be free for 20 minutes. Which action by the nurse is most appropriate?

- A.** Explain that hospital staff assignments cannot be changed for cultural preferences and proceed now
- B.** Ask the client's family member to assist with the procedure to maintain the client's cultural preference
- C.** Delay the procedure briefly and arrange for a same-sex staff member if the client's condition is stable
- D.** Tell the client that the request may interfere with care and document refusal of treatment

53. A nurse is preparing to perform a focused assessment on a client who identifies as a transgender woman. Before beginning, the nurse notes that the electronic record lists the client's sex assigned at birth, while the client introduced herself with a different name and she/her pronouns. Which nursing action is most appropriate to promote respectful, individualized care during the assessment?

- A.** Ask the client which name and pronouns she wants used, and use those while obtaining any anatomy-specific information needed for care
- B.** Use only the information listed in the electronic record to avoid documentation errors
- C.** Delay the assessment until the provider updates the chart with the client's current gender identity
- D.** Avoid discussing gender-related information unless the client brings it up later in the visit

54. A client with newly diagnosed glaucoma tells the nurse, "I'm afraid I'll go blind and become a burden to my family." Which response by the nurse is best?

- A.** Most people adjust quickly once treatment starts, so you should try not to worry.
- B.** Let's talk about what concerns you most and what support may help you manage these changes.
- C.** If you take your medications exactly as prescribed, blindness can be completely prevented.
- D.** You need to stay positive so your family will not become more upset.

55. A client receiving end-of-life care says to the nurse, "I don't want any visitors today. I know my family means well, but I am exhausted and I just want quiet." Several relatives arrive on the unit and ask to go into the room immediately. Which nursing action is most appropriate?

- A.** Tell the family they may visit briefly because family presence is important at the end of life
- B.** Ask the client to explain the reason for refusing visitors before limiting access
- C.** Honor the client's request and explain to the family that the client wants privacy and rest at this time
- D.** Delay the family in the waiting area until the health care provider decides whether visits should be restricted

56. A nurse is caring for a client receiving continuous enteral feeding through a gastrostomy tube. The feeding bag is almost empty, and the next container of formula is ready to hang. Which action by the nurse is most appropriate to reduce the client's risk for bacterial contamination?

- A.** Add the new formula to the remaining formula already in the bag
- B.** Rinse the feeding bag with sterile water before adding the new formula

- C. Hang the fresh formula in a clean feeding bag according to agency policy
- D. Leave the current bag in place and refill it each time the formula runs low

57. A nurse is assisting an older adult who has left-sided weakness to climb stairs during rehabilitation. Which instruction should the nurse give the client?

- A. Lead with the weaker leg when going up the stairs
- B. Keep both feet on one step before moving to the next if needed
- C. Hold the handrail with the weaker arm for better support
- D. Place the cane on the step above before moving either leg

58. A nurse is reinforcing dietary teaching for a client who has celiac disease. Which food choice by the client indicates an understanding of the teaching?

- A. Scrambled eggs with sliced oranges
- B. Whole-wheat toast with peanut butter
- C. Barley soup with crackers
- D. Pasta salad with bottled soy sauce

59. A client with a minor burn to the forearm that occurred 30 minutes ago reports burning pain at the site. Which nonpharmacologic intervention should the nurse implement first?

- A. Apply a warm, moist compress to the area
- B. Place the burned area under cool running water
- C. Massage the tissue surrounding the burn
- D. Encourage range-of-motion exercises of the arm

60. A nurse is reinforcing teaching for a client who has a neurogenic bladder and performs intermittent self-catheterization at home. Which client statement indicates correct understanding of how to reduce the risk of bladder overdistention?

- A. "I will catheterize myself at regular scheduled intervals, even if I do not feel the urge to void."
- B. "I will wait to catheterize until I feel lower abdominal discomfort."
- C. "I will drink very little fluid during the day so my bladder does not fill quickly."
- D. "I will try to hold urine as long as possible between catheterizations to improve bladder tone."

61. A nurse is reviewing the medication administration record for four clients before bedtime. Which medication should the nurse recognize as most likely to interfere with the client's ability to sleep?

- A. Furosemide given at 2100
- B. Docusate sodium given at 2100
- C. Acetaminophen given at 2100

- D. Heparin subcutaneous injection given at 2100

62. A practical nurse is caring for a client who wears a rigid leg cast. Which finding requires the nurse's immediate follow-up to protect skin and tissue integrity?

- A. The client reports itching under the cast
- B. The skin around the cast edges appears slightly dry
- C. The client states there is a burning pain under one area of the cast
- D. The client asks when the cast can get wet during bathing

63. A practical nurse is preparing to ambulate a client who is newly blind after eye surgery. Which action is most appropriate when assisting the client to walk in the hallway?

- A. Walk a few steps ahead and tell the client to follow the sound of the nurse's voice
- B. Place the client's hand on the nurse's elbow and describe upcoming turns or obstacles
- C. Support the client from behind by holding both shoulders during ambulation
- D. Ask the client to hold onto the wall while the nurse carries the IV pole

64. A client on strict intake and output has had 1,260 mL of total intake and 980 mL of total output so far this shift. Which fluid balance should the practical nurse document?

- A. +280 mL
- B. -280 mL
- C. +240 mL
- D. -240 mL

65. A practical nurse is caring for a client who has been confined to bed for 5 days. Which action is most appropriate to reduce the risk of renal calculi related to immobility?

- A. Encourage increased fluid intake if not contraindicated
- B. Limit oral fluids after the evening meal
- C. Keep the client in a side-lying position most of the day
- D. Offer a high-calorie snack between meals

66. A nurse is preparing to administer a scheduled dose of metoprolol to a client and notes the apical pulse is 54/min. The prescription includes a hold parameter to withhold the medication if the pulse is below 60/min. Which action is most appropriate?

- A. Administer the medication because it is a scheduled cardiac drug
- B. Withhold the medication and notify the provider of the finding
- C. Ask another nurse to give the medication instead
- D. Delay the medication for 30 minutes and then give it without reassessment

67. A practical nurse is reviewing the MAR before the 1800 medication pass for a client newly admitted after surgery. The MAR lists insulin lispro 6 units subcutaneously before meals. The client has been NPO since admission and says, "I haven't eaten anything today." What is the nurse's best action?

- A.** Administer the insulin as scheduled because it is listed on the MAR
- B.** Hold the insulin and document that the client refused the dose
- C.** Clarify the prescription with the provider before administering the medication
- D.** Ask another nurse to co-sign the dose, then give half of it

68. A nurse is caring for a patient with severe nausea who is ordered ondansetron for quick symptom relief. The patient is alert but is strict NPO before surgery. Which route of administration should the nurse expect to use if the medication is available in that form?

- A.** Transdermal
- B.** Intravenous
- C.** Topical
- D.** Enteral feeding tube

69. During routine narcotic count at shift change, the practical nurse and the offgoing nurse find that one dose of a controlled medication is missing from the count record. Which action should the practical nurse take first?

- A.** Document the discrepancy at the end of the shift after all clients have been assessed
- B.** Notify the charge nurse or follow facility procedure for an immediate controlled-substance discrepancy investigation
- C.** Replace the missing dose from unit stock so the count will match
- D.** Ask the offgoing nurse to sign the count sheet anyway because the cause is probably a charting error

70. A nurse is caring for a client 4 hours after surgery who is receiving morphine by patient-controlled analgesia (PCA). The client says, "My daughter has been pushing the button when I fall asleep so I won't wake up in pain." Which action by the nurse is best?

- A.** Instruct the daughter to press the button only if the client rates pain above 5 on a 0-to-10 scale
- B.** Turn off the PCA pump until the provider can be notified
- C.** Explain that only the client should press the PCA button and reinforce the reason for this safety rule
- D.** Ask the daughter to keep a written record of each time she presses the PCA button

71. A nurse is preparing to administer the first dose of allopurinol to a client with gout. Which pre-administration information is most important for the nurse to review?

- A.** The client's serum uric acid level
- B.** The client's allergy history, especially any previous rash with this medication
- C.** The client's fluid intake during the previous shift
- D.** The client's last pain rating

72. A client with newly diagnosed hypothyroidism begins taking levothyroxine. Which assessment finding best indicates the medication is producing the intended therapeutic response?

- A.** The client's heart rate decreases from 88/min to 60/min
- B.** The client reports improved energy and less sensitivity to cold
- C.** The client reports increased appetite and a 4-lb weight gain in 1 week
- D.** The client's deep tendon reflexes become slower than baseline

73. A client who began taking isoniazid 1 month ago for tuberculosis returns to the clinic. Which new finding should the nurse identify as the most important adverse response to report to the provider?

- A.** Tingling and numbness in the hands
- B.** Mild headache after the morning dose
- C.** Urine that is darker yellow than usual
- D.** Increased appetite

74. A patient who takes levothyroxine every morning asks whether it is okay to start taking an over-the-counter calcium supplement at breakfast. Which response by the nurse is best?

- A.** Take both together with food to prevent stomach upset.
- B.** Take the calcium supplement at least several hours after the levothyroxine.
- C.** Stop taking levothyroxine on days when calcium is taken.
- D.** Crush the levothyroxine tablet and mix it with the calcium supplement.

75. A client with heart failure is prescribed digoxin 0.125 mg by mouth daily. Before administering the scheduled dose, the nurse reviews the laboratory results and notes a serum digoxin level of 2.6 ng/mL. Which action is most appropriate?

- A.** Administer the dose as prescribed and recheck the level in 24 hours
- B.** Hold the medication and notify the provider of the elevated level
- C.** Give the medication with food to reduce gastrointestinal upset

D. Administer half the dose and document the adjustment

76. A nurse is teaching a client who is starting isoniazid for treatment of tuberculosis. Which statement by the client indicates correct understanding of the teaching?

- A.** I should stop taking this medication as soon as my cough improves.
- B.** I will report numbness or tingling in my hands or feet to my provider.
- C.** I can drink alcohol in moderation while taking this medication.
- D.** I should expect my urine to turn red-orange while taking this medication.

77. A nurse is preparing to give a dose of liquid ferrous sulfate through a client's feeding tube. The client is also receiving a continuous enteral formula. To help reduce the risk of decreased medication absorption, which action should the nurse take?

- A.** Mix the ferrous sulfate directly into the enteral formula bag
- B.** Administer the ferrous sulfate without interrupting the feeding
- C.** Pause the feeding, flush the tube, give the medication, and flush again before restarting
- D.** Dilute the ferrous sulfate with formula before instilling it through the tube

78. A nurse is preparing an intermittent IV antibiotic. The prescription is ceftriaxone 1.5 g IV. The vial available after reconstitution is labeled 250 mg/mL. How many milliliters should the nurse draw up for one dose?

- A.** 4 mL
- B.** 5 mL
- C.** 6 mL
- D.** 7.5 mL

79. A practical nurse is collecting data in a clinic on four clients. Which client should the nurse identify as being at greatest risk for infection?

- A.** A client with type 2 diabetes who has a fasting blood glucose of 248 mg/dL
- B.** A client with a healed ankle sprain who reports mild stiffness in the morning
- C.** A client with seasonal allergies who has clear nasal drainage
- D.** A client with tension headaches relieved by rest

80. A nurse is collecting a stool specimen for ova and parasite testing from a client who has diarrhea. Which instruction should the nurse give the client?

- A.** Place toilet tissue in the specimen container with the stool sample
- B.** Collect the stool after it mixes with urine in the toilet

C. Avoid getting urine or toilet water into the stool specimen

D. Store the specimen at the bedside until the end of the shift

81. A nurse reviews laboratory results for a client who takes lithium for bipolar disorder and reports new hand tremors, nausea, and increasing confusion. Which laboratory finding requires the nurse's priority action?

- A.** Lithium level 0.9 mEq/L
- B.** Lithium level 1.8 mEq/L
- C.** Sodium 138 mEq/L
- D.** Magnesium 2.0 mg/dL

82. A client who is 36 weeks pregnant tells the nurse, "My baby has hardly moved today compared with usual." Which instruction should the nurse give first?

- A.** Drink a glass of cold juice, lie on the left side, and count fetal movements
- B.** Take a warm bath and reassess movement in the morning
- C.** Massage the abdomen for several minutes to stimulate fetal activity
- D.** Schedule the next available prenatal visit within 1 week

83. A practical nurse is obtaining vital signs for an adult client who has an irregular radial pulse. Which action is most appropriate to obtain the most accurate heart rate?

- A.** Count the radial pulse for 15 seconds and multiply by 4
- B.** Count the radial pulse for 30 seconds and multiply by 2
- C.** Auscultate the apical pulse for 1 full minute
- D.** Use the pulse oximeter reading instead of counting manually

84. A client is scheduled for a renal biopsy. Which action should the nurse take first before the procedure?

- A.** Check the client's coagulation studies
- B.** Encourage the client to drink extra fluids
- C.** Place the client in high-Fowler's position
- D.** Ask the client to empty the bladder after the procedure

85. A nurse is preparing a client for a 12-lead ECG. The client says, "I'm nervous. Will this test send electricity into my chest?" Which response by the nurse is best?

- A.** Yes, but the amount of electricity is very small and should not be painful.
- B.** No, the ECG only records the heart's electrical activity and does not deliver electricity to the body.
- C.** No, because the test takes pictures of the heart muscle instead of measuring electrical activity.
- D.** Yes, but only if the machine detects an abnormal rhythm during the test.

86. A nurse is obtaining a venous blood specimen from an adult client. As the first tube is filling, the client reports sudden sharp pain radiating down the arm. Which action should the nurse take first?

- A. Advance the needle slightly to stabilize it in the vein
- B. Release the tourniquet and withdraw the needle immediately
- C. Ask the client to open and close the fist until the pain subsides
- D. Finish filling the current tube quickly before stopping the procedure

87. A practical nurse is reinforcing preoperative teaching for a client who is scheduled for abdominal surgery later that morning. Which client statement shows that the teaching about postoperative leg exercises was effective?

- A. "I will stay in bed and keep my legs still so my incision does not open."
- B. "I will bend my knees once after surgery, and that should be enough."
- C. "I will flex and point my feet and move my legs as instructed after surgery."
- D. "I should avoid moving my legs until the provider says I can walk to the bathroom."

88. A client receiving total parenteral nutrition through a central venous catheter is prescribed a new IV tubing set and bag. Which nursing action is most appropriate to reduce the client's risk for catheter-related infection?

- A. Change the tubing only when the current bag is completely empty
- B. Use sterile technique for the catheter hub and tubing connection during the change
- C. Flush the catheter with sterile water before connecting the new tubing
- D. Disconnect the tubing for several minutes before attaching the new bag

89. A nurse is caring for a client who is 18 hours postpartum and reports, "I have a really bad headache that gets worse when I sit up, but it feels a little better when I lie flat." The client received epidural anesthesia during labor. Which focused assessment should the nurse perform first?

- A. Assess the client's headache pattern in relation to position changes
- B. Inspect the client's perineal pad for the amount of lochia
- C. Palpate the client's fundus for firmness and location
- D. Ask the client when she last voided

90. A client with acute pancreatitis has been receiving prescribed IV fluids, bowel rest, and pain management for 24 hours. Which finding best indicates the client is responding to treatment?

- A. Urine output of 15 mL/hr
- B. Serum amylase remains higher than the reference range
- C. Abdominal pain decreased from 8/10 to 3/10
- D. Abdominal distention is increasing

91. A nurse is reinforcing discharge teaching for a client with a clean healing incision. Which client statement indicates an understanding of nutrition that supports wound healing?

- A. I will increase foods like eggs, chicken, and beans in my meals.
- B. I should avoid citrus fruits until the incision is fully closed.
- C. I will limit fluids so the dressing stays dry longer.
- D. I should mainly eat bread and pasta so my body can rest.

92. A nurse enters a client's room and finds the client collapsed on the floor. The nurse notes a strong smell of oxygen in the room and sees the client's nasal cannula tubing still attached to the wall oxygen source. The client is unresponsive and not breathing normally. Which action should the nurse take first?

- A. Begin chest compressions immediately
- B. Turn off the oxygen source or move it away from the client
- C. Apply the AED pads to the client's bare chest
- D. Give 2 rescue breaths with a bag-mask device

93. A client with emphysema is prescribed oxygen at 28% by Venturi mask. During rounds, the nurse finds that the mask has been replaced with a nonrebreather mask at 15 L/min by an unlicensed assistive person who says, "The client looked short of breath, so I gave more oxygen." The client is sleepy but arousable, respirations are 10/min, and oxygen saturation is 95%. Which action should the nurse take first?

- A. Lower the oxygen concentration by reapplying the Venturi mask as prescribed
- B. Encourage the client to cough and deep breathe
- C. Document the change in oxygen delivery and continue to monitor
- D. Raise the head of the bed and reassess in 30 minutes

94. A nurse is providing tracheostomy care for a client with a cuffed tracheostomy tube. To help prevent accidental decannulation while changing the tracheostomy ties, which action is best?

- A. Remove the old ties before applying the new ties
- B. Ask the client to flex the neck forward during the tie change

- C.** Have an assistant stabilize the tracheostomy tube while the nurse secures the new ties
- D.** Loosen the ties enough to allow the nurse to place three fingers underneath

95. A client who is receiving mechanical ventilation is being monitored for readiness to wean. Which finding best suggests the client may be ready for a spontaneous breathing trial?

- A.** The client requires an FiO₂ of 80% to maintain oxygen saturation
- B.** The client is hemodynamically stable and initiating adequate spontaneous breaths
- C.** The client has increasing thick tracheal secretions and frequent suctioning needs
- D.** The client remains deeply sedated with minimal respiratory effort

96. A client with a temporary transvenous pacemaker is being monitored after insertion. The ECG shows regular pacer spikes, each immediately followed by a QRS complex. The client is awake, warm, and has a blood pressure of 84/50 mm Hg with dizziness. Which action should the nurse take next?

- A.** Document that the pacemaker is functioning properly and continue routine monitoring
- B.** Increase the ECG alarm limits because electrical capture is present
- C.** Assess for mechanical capture by checking the client's pulse and hemodynamic response
- D.** Turn off the pacemaker because the electrical activity is already adequate

97. A nurse is reinforcing discharge teaching for a client with a new ascending colostomy. Which instruction is most appropriate to include?

- A.** Irrigate the colostomy at the same time each day to regulate stool output.
- B.** Increase fluid intake and watch for signs of dehydration because the stool will be more liquid.
- C.** Expect little to no gas because the stool bypasses most of the large intestine.
- D.** Change the skin barrier only when drainage begins leaking from the pouch.

98. A nurse is caring for a client who is 8 hours postoperative after a bowel resection. Which change in the client's condition should the nurse recognize as the priority to report immediately?

- A.** Urine output of 25 mL/hr for the last 2 hours
- B.** Temperature of 37.8 C (100.0 F)
- C.** Heart rate increase from 88/min to 124/min with cool, pale skin
- D.** Incisional pain rated 7 on a scale of 0 to 10 before the next scheduled analgesic dose

99. A nurse is caring for a client who has had diarrhea for 2 days. The client is weak and reports palpitations. Which laboratory result should the nurse report to the provider first?

- A.** Potassium 2.9 mEq/L
- B.** Sodium 133 mEq/L
- C.** Chloride 96 mEq/L
- D.** Magnesium 1.9 mg/dL

100. A client receiving peritoneal dialysis tells the nurse, "My abdomen feels very full, and now I'm getting short of breath." Which action should the nurse take first?

- A.** Slow or stop the infusion and assess the client's respiratory status
- B.** Raise the head of the bed and encourage the client to take deep breaths
- C.** Obtain a prescribed pain medication for abdominal discomfort
- D.** Document that abdominal fullness is expected during peritoneal dialysis

Simulation Test 3 — Answer Key

1. A	26. C	51. B	76. B
2. C	27. C	52. C	77. C
3. B	28. B	53. A	78. C
4. B	29. A	54. B	79. A
5. C	30. C	55. C	80. C
6. C	31. A	56. C	81. B
7. A	32. C	57. B	82. A
8. B	33. B	58. A	83. C
9. A	34. A	59. B	84. A
10. A	35. C	60. A	85. B
11. C	36. A	61. A	86. B
12. B	37. B	62. C	87. C
13. C	38. C	63. B	88. B
14. B	39. B	64. A	89. A
15. C	40. B	65. A	90. C
16. B	41. B	66. B	91. A
17. B	42. C	67. C	92. B
18. C	43. C	68. B	93. A
19. B	44. C	69. B	94. C
20. C	45. B	70. C	95. B
21. B	46. C	71. B	96. C
22. A	47. B	72. B	97. B
23. B	48. B	73. A	98. C
24. B	49. B	74. B	99. A
25. A	50. B	75. B	100. A

Simulation Test 4

100 questions

1. The practical nurse is halfway through a morning medication pass when an assistive personnel team member reports several client concerns. Which situation should the nurse address first?

- A.** A client with pneumonia needs help to the bathroom and is becoming impatient
- B.** A client who started a blood transfusion 10 minutes ago reports chills and low back pain
- C.** A client with chronic arthritis asks when the next PRN pain medication is due
- D.** A client scheduled for discharge says the ride home will arrive 30 minutes early

2. The practical nurse in a long-term care facility has 15 minutes before a scheduled provider call about resident status changes. Which action should the nurse take first?

- A.** Administer a scheduled stool softener to a resident with chronic constipation
- B.** Document that a resident's morning bath was refused
- C.** Assess a resident with diabetes who is sweaty, shaky, and newly confused before lunch
- D.** Refill the supply cart with incontinence products

3. A practical nurse is coordinating care for four clients with assistance from an assistive personnel (AP). Which activity is most appropriate for the nurse to delegate to the AP?

- A.** Collect a sputum specimen from a client who just developed a productive cough and fever
- B.** Perform finger-stick blood glucose testing for a client who is pale, trembling, and difficult to arouse
- C.** Help a client with chronic arthritis get dressed after the nurse has already assessed the client's mobility and pain level for the morning
- D.** Teach a client how to use a newly prescribed walker before discharge

4. An adult client who is alert and oriented tells the nurse, "I don't want to be examined by that clinician because the comments made earlier felt disrespectful." The appointment is not an emergency. What is the nurse's best advocacy action?

- A.** Explain that the clinician is busy and encourage the client to continue with the exam to avoid delays
- B.** Tell the client that refusing the clinician could be considered noncompliance with treatment
- C.** Acknowledge the client's concern and help the client request another qualified clinician according to facility policy

D. Document the complaint after the exam so the client's care is not interrupted

5. A practical nurse is caring for a client who is a well-known teacher in the community. During lunch in the hospital cafeteria, the nurse hears two unlicensed assistive personnel discussing the client's diagnosis and prognosis, even though neither is assigned to that client. What should the nurse do first?

- A.** Join the conversation and correct any inaccurate details so misinformation is not spread
- B.** Privately tell the assistive personnel to stop the discussion because it violates client privacy
- C.** Wait until the end of the shift and report the incident to the nurse manager
- D.** Ask the client's provider to explain confidentiality expectations to the assistive personnel

6. An alert adult client says, "My sister and I completed our advance directives years ago in another state. I'm being admitted today, and I want mine followed if I become unable to decide." Which response by the nurse is most appropriate?

- A.** "Advance directives from another state are not recognized, so you will need to complete a new one immediately."
- B.** "Your provider must decide first whether the document is valid before it can be placed in the record."
- C.** "Let's place a copy in your record and notify the provider so the document can be reviewed and honored according to facility policy and state law."
- D.** "Because you are able to make decisions now, the advance directive should not be included in your chart."

7. A nurse is caring for a client who is scheduled for an elective cardiac stress test. The client is alert and oriented. As the nurse witnesses the signature on the consent form, the client asks, "If my heart rhythm becomes abnormal during the test, what will they do about it?" Which action should the nurse take?

- A.** Explain the emergency equipment and medications that are commonly kept available during the test
- B.** Tell the client that minor rhythm changes are expected and usually do not need treatment
- C.** Pause the signing process and notify the provider that the client has additional questions about the procedure
- D.** Have the client sign the form first and ask the provider to answer questions afterward

8. A client is being discharged after treatment for a venous leg ulcer. The provider prescribes compression therapy and weekly follow-up visits. The client says, "I can afford my medicines, but I do not have a way to get to the wound clinic every week." Which action by the nurse is most appropriate before discharge?

- A.** Ask the client to call the clinic if the ulcer gets worse

- B.** Arrange referral to case management or social services to address transportation needs
- C.** Tell the client to skip follow-up visits if the dressing stays clean and dry
- D.** Delay all discharge teaching until transportation is guaranteed
- 9.** A client is scheduled for discharge after a total knee replacement. The client is able to transfer safely and take prescribed medications but needs continued strengthening and gait training. The client lives at home with a partner who can assist in the evenings. Which referral should the nurse anticipate as most appropriate?
- A.** Speech therapy
- B.** Occupational health services
- C.** Physical therapy
- D.** Hospice care
- 10.** A practical nurse on an adult medical unit notices that several clients are missing their scheduled morning blood glucose checks before breakfast. The team wants to improve this problem. Which action by the practical nurse best supports quality improvement?
- A.** Begin checking glucose at random times during the shift when staff are available
- B.** Collect data for several days on which clients miss checks, when the misses occur, and possible contributing factors
- C.** Ask the provider to change all glucose checks to after breakfast
- D.** Remind staff to work faster during the morning routine
- 11.** A rehabilitation unit wants to reduce delays in dressing changes for clients with complex wounds. For 4 weeks, the team tracks two measures after introducing a supply cart stocked with common wound-care items. Audit results show the cart was stocked correctly on 96% of days, and the percentage of dressing changes completed within the ordered time increased from 58% to 81%. Which conclusion is most appropriate?
- A.** The project should be considered unsuccessful because the cart was not stocked correctly every day
- B.** The improved timeliness suggests the change may be helping client care, although continued monitoring is still needed
- C.** The results prove that stocking the cart is the only factor affecting dressing-change delays
- D.** The unit should stop measuring outcomes now that performance improved
- 12.** A practical nurse is caring for a client with a new colostomy. During an interdisciplinary care meeting, which information is most important for the nurse to share to help the team plan safe discharge teaching?
- A.** The client prefers to rest after lunch before doing any teaching sessions
- B.** The client says, "I don't think I can empty or change this pouch by myself."
- C.** The client asked whether spicy foods will still be allowed at home
- D.** The client's spouse visited for 20 minutes this morning
- 13.** A practical nurse is caring for a client receiving furosemide for heart failure. Before the provider arrives for rounds, the nurse wants to give the most useful update to the interdisciplinary team. Which report is best?
- A.** The client is stable and seems to be breathing a little better today.
- B.** The client has heart failure, is on a low-sodium diet, and is taking prescribed medications.
- C.** The client voided 1,900 mL since 0700, lost 1.1 kg (2.4 lb) since yesterday, has fewer crackles at the lung bases, and reports less shortness of breath when walking to the chair.
- D.** The client asked whether discharge will happen soon and said breakfast was not very good.
- 14.** A practical nurse is giving end-of-shift report when a close friend texts, "I heard your cousin was admitted to your unit. Is she okay?" The nurse is not assigned to that client. Which response is most appropriate?
- A.** Reply with a brief update because the friend is also a relative
- B.** Check the chart first so the information shared will be accurate
- C.** State that the nurse cannot share any information about clients
- D.** Ask the charge nurse whether it is acceptable to give a general update
- 15.** A practical nurse receives a written prescription that appears to use an unsafe abbreviation and could be interpreted two different ways. The client is stable and waiting for the medication. Which action by the nurse is most appropriate?
- A.** Give the lower of the two possible doses to avoid overdosing the client
- B.** Ask another nurse what the provider probably meant and proceed with that interpretation
- C.** Hold the medication and contact the provider to clarify the prescription before administering it
- D.** Administer the medication as written because the provider is responsible for the order

16. A practical nurse in a pediatric clinic notices that a 4-year-old child has multiple cigarette-burn scars on the forearm and flinches when the parent reaches toward the child. The parent says, "Those marks are from a rash." What is the nurse's priority action?

- A.** Ask the parent to sign a statement explaining the injuries before taking further action
- B.** Report the suspected abuse according to facility policy and state law
- C.** Wait to see whether another nurse agrees with the suspicion before acting
- D.** Confront the parent directly and state that the injuries appear intentional

17. A practical nurse in an emergency department cares for an alert adult client who states privately, "My roommate at the group home hits me when I'm slow getting dressed." The client has a fading bruise on the jaw and asks the nurse not to tell anyone because "it will only get worse." Which action is most appropriate?

- A.** Respect the client's request for secrecy because the client is an adult and alert
- B.** Delay action until the nurse can interview the roommate for both sides of the story
- C.** Document the client's exact statement and findings, then report the suspected abuse per policy
- D.** Advise the client to move out of the group home before the nurse takes any further steps

18. A practical nurse is coordinating care with an assistive personnel (AP) on a pediatric unit. Which task is most appropriate for the nurse to assign to the AP?

- A.** Reinforce discharge teaching to a parent about home nebulizer use
- B.** Measure and record the weight of a preschool child admitted for observation
- C.** Assess a child with a new rash after the first dose of an antibiotic
- D.** Determine whether a toddler's pain medicine was effective 30 minutes after administration

19. A practical nurse asks an assistive personnel (AP) to help with afternoon care for several clients. Which assignment is most appropriate?

- A.** Offer thickened liquids to a client newly placed on swallowing precautions after a stroke
- B.** Change the sterile dressing on a client's central line insertion site
- C.** Accompany a stable client who uses a cane to the bathroom after the nurse has already assessed the client's mobility
- D.** Collect data from a client who reports new chest pressure after walking in the hallway

20. A practical nurse in a family clinic is preparing printed after-visit instructions for a client who has heart failure and reads at a very limited level. Which action best promotes equitable access to care?

- A.** Provide the usual written packet and ask the client to call later with questions
- B.** Review the instructions using plain language and ask the client to explain the plan back in their own words
- C.** Give the client a detailed booklet that includes medical terms for future reference
- D.** Ask a family member in the waiting room to read all instructions to the client at home

21. A practical nurse in an oncology clinic is reinforcing a treatment schedule with a client who has newly diagnosed cancer. The client says, "I want treatment, but I am Deaf, and the infusion center keeps telling me to call if I need to change appointments. I use text and video relay, and sometimes they hang up when the relay operator starts speaking." Which action by the nurse best promotes equitable access to care?

- A.** Advise the client to have a family member make all future calls to avoid confusion
- B.** Ask the client to come to the clinic in person whenever an appointment change is needed
- C.** Notify the care team and help arrange communication methods the client can reliably use, such as secure messaging, text-capable contact options, or accepted relay services
- D.** Explain that the clinic uses the same phone policy for all clients, so no changes can be made

22. A nurse is preparing to obtain a capillary blood sample for bedside glucose testing from a client who is confused and unable to state their name. The electronic chart is open in the room, and the client's daughter says, "This is my father, James Turner." Which action should the nurse take to identify the client correctly before performing the test?

- A.** Use the daughter's statement and proceed with the glucose test
- B.** Confirm the client's identity using the identification band and another facility-approved identifier
- C.** Rely on the electronic chart displayed in the room because it matches the assigned bed
- D.** Ask another nurse who cared for the client earlier to verify that this is James Turner

23. A nurse is preparing to use a portable floor lamp to improve lighting while performing a sterile dressing change in a client's home. After plugging in the lamp, the nurse notices the cord stretches across the walkway between the bed and the door. Which action is best?

- A.** Tape the cord securely to the floor or reposition the lamp so the cord is out of the walking path

- B.** Ask the client to stay in bed until the dressing change is finished
- C.** Continue with the procedure because the lamp is needed for adequate visualization
- D.** Unplug the lamp and perform the dressing change using only the room's overhead light
- 24.** A nurse is preparing to administer the first dose of cefazolin to a client. During the preadministration check, the client says, "I had an anaphylactic reaction to cephalexin a few years ago." What should the nurse do first?
- A.** Administer the medication slowly while closely monitoring the client
- B.** Hold the medication and clarify the prescription with the provider
- C.** Give the medication after administering an antihistamine
- D.** Ask another nurse to verify the dose before administration
- 25.** On a pediatric unit, the practical nurse notices that a bottle of alcohol-based hand sanitizer has leaked onto the floor near an electrical outlet strip used for a warming device. Several children and family members are walking through the area. Which action should the nurse take first?
- A.** Move people away from the area and secure the hazard
- B.** Unplug the warming device from the outlet strip
- C.** Wipe up the sanitizer spill with paper towels
- D.** Report the leaking bottle to the charge nurse
- 26.** A client with acute delirium is trying to scratch at a fresh abdominal incision and intermittently swings at staff during care. The provider writes an order for wrist restraints. Which nursing action best supports the principle of least-restrictive restraint use after the restraints are applied?
- A.** Keep the restraints in place until the client is fully oriented and no longer delirious
- B.** Remove one restraint at a time at scheduled intervals to assess whether a less restrictive measure can be used safely
- C.** Tighten the restraints if the client continues to move the arms during care
- D.** Limit staff interaction to avoid increasing the client's agitation
- 27.** A nurse reviewing a client's chart notices that a potassium supplement scheduled for 0800 was documented as given, but the medication cup is still full in the client's room. The client has no symptoms and says, "I haven't taken any pills yet this morning." Which action is most appropriate?
- A.** Remove the medication cup, administer the potassium, and make no further report because no harm occurred
- B.** Assess the client, notify the provider for a new prescription, and complete an incident report after the end of the shift
- C.** Withhold the medication until the next scheduled dose and document that the client refused it
- D.** Verify the finding, hold the medication until the discrepancy is clarified, notify the appropriate nurse or charge nurse, and follow facility policy for reporting the near miss
- 28.** A nurse is transferring a client from the bed to a bedside commode with a sit-to-stand lift. Which client finding makes this device inappropriate to use?
- A.** The client becomes tired after standing for 1 minute
- B.** The client can follow simple commands
- C.** The client is unable to bear weight on either leg
- D.** The client needs help grasping the lift handles
- 29.** A nurse is preparing to move a client from a stretcher to a bed using a mechanical lateral transfer aid. The client is drowsy after sedation and has a drainage tube and an IV infusion. Which action should the nurse take first?
- A.** Raise the bed to the height of the stretcher
- B.** Assess and secure the client's lines and tubes
- C.** Place the transfer aid under the client's shoulders and hips
- D.** Ask the client to cross both arms over the chest
- 30.** A practical nurse is preparing to irrigate a client's eye. The nurse performs hand hygiene, opens a sterile irrigation tray, and pours sterile solution into the sterile basin. While adjusting the overhead light, the nurse reaches across the sterile field with a bare forearm. Which action should the nurse take next?
- A.** Continue with the procedure because the nurse's hands did not touch the supplies
- B.** Ask an assistive personnel staff member to finish the procedure using the same setup
- C.** Discard the contaminated supplies and set up a new sterile field
- D.** Wipe the top of the sterile field with antiseptic and proceed
- 31.** A nurse is preparing to clean and disinfect a bedside stand after a client vomited onto it. The nurse expects contact with the contaminated surface and cleaning solution but does not expect splashing. Which personal protective equipment is most appropriate?
- A.** Gloves only
- B.** Gloves and a sterile gown
- C.** Gloves, gown, mask, and goggles
- D.** N95 respirator and gloves

32. A practical nurse is preparing a sterile field for a bedside procedure. After the field is set up, the nurse reaches across it to adjust the overbed light. Which action is most appropriate?

- A.** Continue with the procedure because the nurse did not touch any sterile items
- B.** Replace only the sterile item closest to where the nurse reached across
- C.** Consider the sterile field contaminated and set up a new sterile field
- D.** Ask another staff member to observe the field and decide whether it is still sterile

33. A nurse is caring for a client with measles who is hospitalized in an airborne infection isolation room. Which action by the nurse is most appropriate?

- A.** Keep the door to the client's room open to allow easier observation
- B.** Wear an N95 respirator before entering the client's room
- C.** Apply sterile gloves before touching any item in the client's room
- D.** Place the client on droplet precautions during routine care in the room

34. A practical nurse is preparing to empty a client's emesis basin. The client has no isolation orders, but the nurse notes that the basin is nearly full and could splash while being emptied. Which action best follows standard precautions?

- A.** Put on gloves and wear a face shield or mask with eye protection before emptying the basin
- B.** Wear a gown only because vomit is not considered potentially infectious
- C.** Empty the basin carefully with bare hands, then perform hand hygiene afterward
- D.** Ask another staff member to empty the basin because standard precautions do not apply without an isolation order

35. A nurse is reinforcing teaching with a client who is 24 weeks pregnant about comfort measures for back pain during pregnancy. Which statement by the client indicates understanding of the teaching?

- A.** I should wear low-heeled supportive shoes and avoid heavy lifting.
- B.** I should stay on bed rest most of the day until the pain goes away.
- C.** I should apply a heating pad to my abdomen for long periods.
- D.** I should start doing sit-ups each morning to strengthen my stomach muscles.

36. A client in labor at 6 cm dilation has been walking in the hallway and suddenly says, "I need to have a bowel movement right now." Which action should the nurse take first?

- A.** Assist the client to the bathroom
- B.** Perform a vaginal examination if permitted by agency policy and provider orders
- C.** Encourage the client to use slow-paced breathing
- D.** Administer a prescribed analgesic for increasing discomfort

37. A nurse is caring for a client 18 hours after an uncomplicated vaginal birth. The client says, "I haven't had a bowel movement yet, and I'm worried that straining will tear something." Which response by the nurse is best?

- A.** Limit fluid intake until bowel function returns to normal
- B.** Avoid walking today so the perineal area can rest
- C.** Increase fluids and fiber, and use the prescribed stool softener if needed
- D.** Delay any bowel movement for several days to prevent pain

38. A nurse is collecting a health history from an adult client who states, "I haven't been sleeping well lately." Which follow-up question is best to further assess this concern?

- A.** What time do you usually go to bed and wake up?
- B.** How many hours of exercise do you get each week?
- C.** Did either of your parents have trouble sleeping?
- D.** Have you ever had surgery for a sleep problem?

39. A practical nurse is documenting baseline physical data for a client newly admitted from home. Which entry best describes the client's baseline ability to perform activities of daily living?

- A.** Needs assistance from one person to transfer from bed to chair and uses a walker to reach the bathroom
- B.** Skin warm and dry with no open areas noted
- C.** Reports abdominal pain rated 6 on a 0 to 10 scale
- D.** Blood pressure 128/76 mm Hg, pulse 82/min, respirations 18/min

40. A nurse is reinforcing teaching for the parent of an infant who is scheduled to receive immunizations today. Which parent statement indicates correct understanding about expected reactions after routine vaccination?

- A.** A mild fever and soreness at the injection site can occur for a day or two.
- B.** Any redness at the injection site means the child is having a serious allergic reaction.
- C.** The infant should be kept awake for several hours after vaccination for observation.

D. If the infant cries after the injection, the next scheduled vaccines should be delayed.

41. A nurse in a clinic is collecting data from a parent about a 3-year-old child. Which statement by the parent describes an expected finding for this age?

- A.** My child can ride a two-wheel bicycle without help.
- B.** My child usually speaks in short sentences of three to four words.
- C.** My child consistently ties shoes without assistance.
- D.** My child prefers cooperative team sports with other children.

42. A nurse is preparing to teach a client about home blood pressure monitoring. The client speaks clearly and answers questions appropriately, but frequently asks the nurse to repeat statements and says, "People mumble a lot in here." Which action by the nurse is best to reduce this communication barrier?

- A.** Provide the client with a longer written handout to review independently
- B.** Speak loudly into the client's ear while giving the instructions
- C.** Face the client directly and speak slowly in a normal tone in a quiet area
- D.** Ask a family member to stay and explain the instructions after discharge

43. A nurse is preparing to teach a client who was newly diagnosed with chronic kidney disease about dietary changes. When the nurse begins, the client says, "I left school in eighth grade, so just tell me the main thing I need to do." Which nursing action is best?

- A.** Provide a detailed written booklet and ask the client to review it independently
- B.** Use plain language, focus on a few key points, and ask the client to explain the plan back in their own words
- C.** Delay all teaching until a family member can come and receive the instructions
- D.** Use medical terms consistently so the client becomes familiar with the diagnosis

44. A client who was diagnosed with multiple sclerosis 2 weeks ago says to the practical nurse, "Some days I act like nothing is wrong, and other days I feel angry at everyone." Which response by the nurse best provides emotional support?

- A.** You need to stay positive so your family does not worry.
- B.** Those feelings can happen after a diagnosis like this. Tell me more about what has been hardest for you.
- C.** Anger will only make your symptoms worse, so try to calm down.
- D.** You should focus on learning about your medications right now.

45. A client with newly diagnosed rheumatoid arthritis tells the nurse, "My hands hurt all the time now, and I keep thinking I'll end up unable to do anything for myself." Which response by the nurse best promotes effective coping?

- A.** You should try not to think about the future until you know more.
- B.** Many people adjust with time, so you will probably feel better soon.
- C.** Tell me which part of this change feels hardest for you right now.
- D.** Let me explain everything about rheumatoid arthritis so you know what to expect.

46. A nurse on a medical-surgical unit is caring for a client who has become increasingly angry after being told that discharge will be delayed. The client is pacing, jaw is tight, and the client says, "If that doctor comes in here again, somebody is going to get hurt." Which action should the nurse take first?

- A.** Ask the client to describe the reasons for the anger in detail
- B.** Administer a PRN sedative medication immediately
- C.** Remain calm and move the client to a quiet area away from others
- D.** Explain that threats of violence are unacceptable and may delay discharge further

47. A client who has been taking high doses of alprazolam for several months tells the nurse, "I ran out of pills yesterday, but I'll be fine. I just need to sleep it off." Which response by the nurse is most appropriate?

- A.** "You may have some discomfort, but withdrawal from this medication is usually not dangerous."
- B.** "Stopping this medication suddenly can be serious, so you need prompt medical evaluation for withdrawal."
- C.** "Try drinking extra fluids and avoiding caffeine until the shakiness passes."
- D.** "If you feel anxious, take an over-the-counter sleep aid tonight."

48. A nurse is caring for a client who has become increasingly upset after receiving bad news from a provider. The client is in the hallway, speaking loudly, glaring at staff, and saying, "Just leave me alone before I lose it." Which action by the nurse is most appropriate?

- A.** Stand directly in front of the client and explain that this behavior is unacceptable
- B.** Guide the client to a quieter area, keep a calm voice, and allow personal space while offering to listen
- C.** Ask several direct questions at once to find out exactly why the client is angry
- D.** Touch the client's arm reassuringly and tell the client to calm down

49. A client with moderate dementia becomes suspicious during medication pass and says to the practical nurse, "You're trying to poison me." The client pushes the medication cup away and begins to tremble. Which action by the practical nurse is most appropriate?

- A.** Tell the client the accusation is false and insist that the medication must be taken now
- B.** Ask another nurse to administer the medication without further explanation
- C.** Acknowledge the client's fear, speak calmly, and offer to stay while the client looks at each medication one at a time
- D.** Leave the room immediately and document that the client is being noncompliant

50. A client whose parent was admitted to hospice says, "I feel overwhelmed, and my mind keeps racing at night. I made a list of everything I need to do, but it only makes me more anxious." Which nursing recommendation is most appropriate to support effective stress management?

- A.** Focus on finishing the entire list before taking any breaks
- B.** Choose one or two priority tasks and schedule short periods for rest and relaxation
- C.** Avoid talking about the situation so emotions do not become harder to control
- D.** Stay busy with television until falling asleep to keep from thinking about the stress

51. A client who recently had a stroke becomes tearful during morning care and tells the practical nurse, "I can't use my right hand, so I can't perform my usual prayer ritual the way I always have. It feels wrong." Which response by the nurse is most appropriate?

- A.** Your recovery should be the priority right now, so try not to focus on rituals.
- B.** Would you like to talk about what part of the ritual is most important to you so we can look for a way to support it?
- C.** You can skip the ritual until your arm strength returns.
- D.** I will ask your family to explain that the ritual is not necessary while you are hospitalized.

52. A nurse is assessing a client who was admitted with shortness of breath. When asked to rate pain, the client pauses and says, "In my family, we do not complain." The client is breathing rapidly and holding the chest tightly. Which action by the nurse is most appropriate?

- A.** Document that the client denies pain and continue with the assessment
- B.** Explain that all clients must use the same pain scale and ask again later
- C.** Recognize that pain may be expressed differently across cultures and further assess pain using additional cues

D. Ask a family member to answer questions about the client's pain experience

53. A client who identifies as genderqueer is admitted for treatment of pneumonia. During report, the nurse learns that several staff members have been unsure what terms to use when discussing the client's body parts during care. Which nursing action is best to support safe, respectful care?

- A.** Ask the client which name, pronouns, and anatomy terms they prefer staff to use during care and document this information for the team
- B.** Use the terminology listed in the medical record so all staff members remain consistent
- C.** Avoid discussing anatomy unless the client brings it up first to prevent embarrassment
- D.** Limit communication to yes-or-no questions until the primary provider clarifies the client's gender status

54. A client with newly diagnosed diabetic retinopathy tells the nurse, "I stopped going to the dining room because I spilled my drink in front of everyone yesterday. I'd rather just eat alone in my room." Which response by the nurse is best?

- A.** "It may help to stay in your room until your vision improves."
- B.** "You should ask your family to bring all your meals so you do not feel embarrassed."
- C.** "That sounds upsetting. Let's talk about ways to make meals easier so you can stay involved with others if you want to."
- D.** "Most people adjust quickly, so try not to focus on it."

55. A client with metastatic cancer who is receiving end-of-life care tells the nurse, "I don't want my family to see me like this anymore, but they keep saying they need to be here all day." Which nursing response is most appropriate?

- A.** "They mean well, so it may be best to let them stay as long as they want."
- B.** "Tell me more about what feels hardest about having them here right now."
- C.** "I will explain to them that visiting is no longer allowed for the rest of the day."
- D.** "You should try to spend as much time with them as possible while you still can."

56. A nurse is caring for a client who has been receiving intermittent enteral feedings through a gastrostomy tube for 3 days. Before the next feeding, the nurse aspirates 180 mL of gastric residual. The prescription says to hold the feeding and notify the provider if the residual is greater than 200 mL. Which action should the nurse take?

- A.** Discard the residual and administer the feeding as scheduled

- B.** Return the aspirated residual, delay the feeding, and recheck the residual according to facility policy
- C.** Administer the feeding now because any residual under 200 mL is always expected
- D.** Flush the tube with carbonated beverage and begin the feeding slowly
- 57.** A nurse is assisting an immobile client to move up in bed with the help of another staff member. Which action by the nurse is most appropriate to reduce the risk of injury to both the client and staff?
- A.** Place the bed in a low position and pull the client upward by the arms
- B.** Lower the side rail, lock the bed wheels, and use a draw sheet to lift and reposition the client
- C.** Ask the client to bend at the waist and slide upward while the nurse pulls from the shoulders
- D.** Raise the head of the bed to high-Fowler's position before moving the client upward
- 58.** A nurse is assisting with meal planning for a client who has difficulty chewing because of poorly fitting dentures. Which meal selection is most appropriate to promote adequate nutrition?
- A.** Turkey sandwich on toasted bread, carrot sticks, and an apple
- B.** Baked chicken, brown rice, and steamed broccoli
- C.** Scrambled eggs, oatmeal, and yogurt
- D.** Pork chop, corn on the cob, and a dinner roll
- 59.** A client with sickle cell disease is hospitalized for a vaso-occlusive pain crisis and reports severe pain in both legs. In addition to prescribed analgesics, which nonpharmacologic intervention should the nurse implement?
- A.** Apply cold packs to the painful areas
- B.** Encourage the client to walk frequently to distract from the pain
- C.** Place warm compresses on the painful areas as tolerated
- D.** Massage the legs firmly every 2 hours
- 60.** A nurse is caring for a client who has right-hand weakness after surgery and wants to feed themselves at lunch. Which intervention best promotes independence with this activity of daily living?
- A.** Ask the family member to feed the client to prevent spills
- B.** Provide a plate guard and built-up utensils for use with the left hand
- C.** Cut all food into very small pieces and feed the client quickly
- D.** Encourage the client to wait until right-hand strength returns before self-feeding
- 61.** A nurse is preparing a client for discharge after treatment for chronic insomnia. Which instruction should the nurse include to help the client reset the sleep-wake cycle?
- A.** Go to bed earlier each night until sleep improves
- B.** Use the bed for sleeping rather than for watching television or eating
- C.** Stay in bed longer on weekends to catch up on missed sleep
- D.** Exercise vigorously right before bedtime to feel more tired
- 62.** A practical nurse is caring for a client who has a dry, flaky skin tear on the forearm after bumping into a bedside table. The surrounding skin is fragile, and the client reports the area stings when adhesive tape is removed. Which action is most appropriate when applying the new dressing?
- A.** Secure the dressing with strong adhesive tape directly on the surrounding skin
- B.** Apply a dry gauze dressing and change it frequently to keep the wound exposed to air
- C.** Use a nonadherent dressing and avoid placing adhesive directly on the fragile skin
- D.** Clean the area vigorously to remove loose surface tissue before covering it
- 63.** A practical nurse is reinforcing home-safety teaching for a client who has decreased sensation in both feet from peripheral neuropathy. Which instruction is most appropriate?
- A.** Test bath water temperature with the elbow before getting in
- B.** Wear elastic socks at night to improve circulation
- C.** Soak the feet in hot water for 20 minutes each evening
- D.** Walk barefoot at home to strengthen the feet
- 64.** A client on strict intake and output receives a continuous enteral feeding at 50 mL/hr from 8:00 a.m. to 2:00 p.m. The nurse also administers 60 mL of water to flush the tube at 10:00 a.m. and again at 2:00 p.m. How much intake should the nurse record for this time period?
- A.** 300 mL
- B.** 360 mL
- C.** 420 mL
- D.** 480 mL
- 65.** A practical nurse is caring for an immobilized client whose heels rest against the mattress most of the day. Which action is most appropriate to help prevent pressure injury to the heels?
- A.** Place a donut-shaped cushion under each heel
- B.** Massage the heels for 5 minutes every shift

C. Float the heels off the bed with a pillow under the lower legs

D. Apply powder to the heels to keep the skin dry

66. A nurse is preparing to administer a liquid anticonvulsant to a client through an enteral feeding tube. After verifying the provider's prescription and the client identity, the nurse notes the medication label states, "Do not crush or give via feeding tube." Which action is most appropriate?

A. Administer the medication through the tube because it is a liquid and the prescription is verified

B. Mix the medication with the tube feeding to reduce the chance of tube irritation

C. Hold the medication and contact the provider or pharmacist for a safe alternative route or formulation

D. Dilute the medication with 30 mL of water and give it slowly through the tube

67. A practical nurse is preparing to administer a client's 2100 medications. On the MAR, clonazepam 0.5 mg PO is listed as scheduled for 2100. The nurse checks the chart and sees a provider prescription from that afternoon stating, "Discontinue clonazepam." No one has documented that the 2100 dose was given yet. What is the nurse's best action?

A. Administer the clonazepam because it is still listed on the MAR

B. Hold the medication and clarify the discrepancy according to facility policy before administering

C. Ask the client whether clonazepam is usually taken at home and give it if the client says yes

D. Document the medication as refused so the dose is not missed

68. A nurse is preparing to administer morphine to a postoperative patient who needs continuous pain control over the next 24 hours without repeated injections. Which route of administration best meets this goal?

A. Transdermal

B. Intramuscular

C. Intravenous infusion

D. Topical

69. A client with a gastrointestinal bleed has received several units of packed red blood cells and now reports tingling around the mouth and muscle cramping. The nurse notes a prolonged QT interval on the cardiac monitor. Which transfusion-related complication should the nurse suspect?

A. Hyperkalemia from hemolyzed blood

B. Hypocalcemia from citrate binding calcium

C. Circulatory overload from rapid infusion

D. Febrile nonhemolytic reaction

70. A nurse is caring for a client 8 hours after surgery who is receiving analgesia through a patient-controlled analgesia (PCA) pump. The client tells the nurse, "I'm not sure this thing is working because my pain gets bad when I wait too long to press the button." Which instruction should the nurse provide?

A. Press the button as soon as pain begins instead of waiting until the pain becomes severe.

B. Turn off the pump for 30 minutes if the medication does not seem strong enough.

C. Have a family member press the button when you are resting to prevent pain from returning.

D. Press the button several times in a row to override the pump's time limits.

71. A nurse is preparing to administer the next dose of spironolactone to a client with heart failure. Which pre-administration data is most important for the nurse to review?

A. Serum potassium level

B. Apical pulse rate

C. Hemoglobin level

D. White blood cell count

72. A client with atrial fibrillation began taking metoprolol 3 days ago. Which finding best indicates the medication is producing the intended therapeutic response?

A. Apical pulse decreased from 128/min to 82/min

B. Urine output increased from 25 mL/hr to 40 mL/hr

C. Oxygen saturation increased from 92% to 96%

D. Fasting blood glucose decreased from 168 mg/dL to 110 mg/dL

73. A client with heart failure has been taking spironolactone for 10 days. Which finding should the nurse recognize as the most important adverse response to report to the provider?

A. Serum potassium of 5.9 mEq/L

B. Urine output of 1,400 mL in 24 hours

C. Mild thirst after walking in the hallway

D. Blood pressure of 118/72 mm Hg

74. A patient taking digoxin for heart failure tells the nurse, "I've been using an over-the-counter herbal product to help with my mood and energy." Which additional statement by the patient requires the nurse's immediate follow-up for a possible drug interaction?

A. "It's St. John's wort, and I started it about 2 weeks ago."

B. "It's melatonin, and I only take it at bedtime."

C. "It's psyllium fiber, and I mix it with water each morning."

D. "It's echinacea, and I take it when I feel a cold coming on."

75. A client with bipolar disorder is prescribed lithium carbonate 300 mg by mouth three times daily. Before administering the next dose, the nurse reviews the laboratory results and notes a serum lithium level of 1.9 mEq/L. Which action is most appropriate?

- A.** Administer the dose as prescribed and encourage the client to drink more fluids
- B.** Hold the medication and notify the provider of the critical laboratory result
- C.** Give the medication with food to reduce gastrointestinal upset
- D.** Administer half the prescribed dose and recheck the level in 24 hours

76. A nurse is teaching a client who is starting a nitrofurantoin prescription for a urinary tract infection. Which statement by the client indicates correct understanding of the teaching?

- A.** I should stop taking this medication as soon as my burning with urination goes away.
- B.** I will take this medication with food if it upsets my stomach.
- C.** I should expect my urine to stay clear while I am taking this medication.
- D.** I need to limit fluids so the medication stays in my bladder longer.

77. A nurse is preparing to administer a dose of insulin glargine and notices that another nurse has drawn the medication into a syringe along with regular insulin to reduce the number of injections. Which action should the nurse take?

- A.** Administer the insulin mixture because both medications are insulins
- B.** Inject the mixture within 5 minutes to prevent loss of potency
- C.** Discard the syringe and prepare the insulin glargine in a separate syringe
- D.** Add sterile normal saline to the syringe to improve compatibility

78. A nurse is preparing to administer phenytoin 125 mg by mouth. The oral suspension available is labeled 250 mg/5 mL. How many milliliters should the nurse administer for one dose?

- A.** 1.25 mL
- B.** 2.5 mL
- C.** 5 mL
- D.** 10 mL

79. A practical nurse is collecting data on several clients at the start of the shift. Which client should the nurse identify as being at greatest risk for bleeding?

- A.** A client with a platelet count of 48,000/mm³ who reports new gum bleeding when brushing teeth
- B.** A client with a hemoglobin of 10 g/dL who reports fatigue with activity
- C.** A client with a white blood cell count of 13,000/mm³ who has a temperature of 37.8°C (100°F)
- D.** A client with a potassium level of 3.2 mEq/L who reports leg cramps

80. A nurse needs to obtain a blood specimen for coagulation studies from a client who has a continuous IV infusion running in the left forearm. Which action is most appropriate to help ensure an accurate specimen?

- A.** Draw the specimen from the left forearm below the IV insertion site
- B.** Stop the infusion briefly and draw the specimen from the IV tubing
- C.** Obtain the specimen from a vein in the right arm
- D.** Ask the client to flex and relax the left hand while the blood is drawn near the infusion site

81. A nurse is reviewing laboratory results for a client who has pancreatitis and now reports tingling around the mouth and muscle cramps. Which laboratory result best explains these findings?

- A.** Calcium 7.1 mg/dL
- B.** Sodium 148 mEq/L
- C.** Magnesium 2.1 mg/dL
- D.** Platelets 220,000/mm³

82. A client who is 27 weeks pregnant calls the prenatal clinic and says, "I've had burning when I urinate since yesterday, and now I feel feverish and have pain in my lower back on the right side." Which response by the nurse is most appropriate?

- A.** Advise the client to increase oral fluids and keep the next routine prenatal appointment
- B.** Instruct the client to go to the health care provider or emergency department for evaluation today
- C.** Tell the client that urinary discomfort is common in pregnancy and usually resolves on its own
- D.** Recommend limiting activity and using a heating pad on the lower back for the rest of the day

83. A nurse is preparing to perform a bedside urine pregnancy test for a client in the emergency department. Which action is most important to help ensure the point-of-care result is reliable?

- A.** Read the result within the time frame listed in the manufacturer's instructions

- B.** Collect the urine specimen from the toilet hat after it has been standing for 1 hour
- C.** Ask the client to drink extra fluids before providing the specimen
- D.** Place the test device near a sunny window so the lines are easier to see

84. A client is scheduled for a cardiac catheterization in 1 hour. Which finding is most important for the nurse to report to the provider before the procedure?

- A.** The client ate a full breakfast 3 hours ago
- B.** The client reports an allergy to iodine-based antiseptics
- C.** The client takes acetaminophen for headaches
- D.** The client has a blood pressure of 138/84 mm Hg

85. A nurse is reinforcing teaching for a client who will have a 12-lead ECG. Which statement by the client indicates correct understanding of the test?

- A.** The test will show how well my lungs are exchanging oxygen.
- B.** The test records the electrical activity of my heart.
- C.** The test will measure the strength of blood flow through my arteries.
- D.** The test uses sound waves to create a picture of my heart valves.

86. A nurse is preparing to obtain a blood specimen by venipuncture from an adult client. The client states, "I had a mastectomy on my left side," and inspection shows a functioning arteriovenous fistula in the right forearm. Which action is most appropriate?

- A.** Use the left arm because it does not contain a fistula
- B.** Use the right arm distal to the fistula
- C.** Stop and clarify an alternative collection plan with the provider or facility policy
- D.** Perform a capillary fingerstick instead of venipuncture for all ordered tests

87. A practical nurse is caring for a client who is 24 hours postoperative after a total hip arthroplasty. Which action is most appropriate when turning the client in bed?

- A.** Place a pillow between the client's legs and keep the affected leg in proper alignment
- B.** Flex the hip and knee sharply to make repositioning easier
- C.** Turn the client onto the operative side without support devices
- D.** Cross the client's ankles to maintain body alignment during the turn

88. A nurse is preparing to administer an IV dose of phenytoin through a client's peripheral IV site. Before giving the medication, which action is most appropriate?

- A.** Flush the IV with dextrose 5% in water to maintain compatibility
- B.** Verify patency with normal saline and assess the site for blood return and irritation
- C.** Mix the medication with the client's maintenance IV solution to reduce vein irritation
- D.** Administer the medication by rapid IV push to limit contact with the vein

89. A nurse is caring for a client who is 2 hours postpartum after a vaginal birth. The client reports, "I feel pressure, and I need to push again." Which focused assessment should the nurse perform first?

- A.** Inspect the perineum for bulging or a hematoma
- B.** Ask when the client last voided
- C.** Check the fundal height and firmness
- D.** Review the amount of lochia on the perineal pad

90. A client with acute gout received the first doses of prescribed anti-inflammatory medication yesterday. Which finding best indicates the treatment is effective?

- A.** The client reports less pain and can bear weight on the affected joint more easily
- B.** The client asks for a diet sheet listing foods high in purines
- C.** The affected joint remains red, swollen, and warm to touch
- D.** The serum uric acid level is unchanged from the value drawn before treatment

91. A client who had a thyroidectomy 4 hours ago tells the nurse, "I feel like my throat is getting tight." Which finding requires the nurse's immediate action?

- A.** Hoarse voice when answering questions
- B.** Frequent swallowing and a visible swelling at the incision site
- C.** Incisional pain rated 6 out of 10
- D.** Blood pressure of 148/88 mm Hg

92. A nurse witnesses an adult client suddenly collapse while walking in the hallway. The client is unresponsive. After confirming the scene is safe, the nurse notes the client is gasping irregularly and cannot feel a pulse within 10 seconds. Which action should the nurse take next?

- A.** Place the client in the side-lying recovery position and continue to observe breathing
- B.** Begin chest compressions and have someone activate the emergency response system and obtain the AED
- C.** Provide 2 rescue breaths before starting any compressions

D. Reassess the pulse for a full minute to confirm cardiac arrest

93. A client with rib fractures has shallow respirations because of pain. The provider prescribes incentive spirometry every hour while awake. Which finding best indicates that the intervention is effective?

- A.** The client can hold the breath for several seconds after a slow deep inhalation
- B.** The client coughs less often during the shift
- C.** The client reports that the device is easier to use while lying flat
- D.** The client breathes more rapidly immediately after each use

94. A nurse is caring for a client with a cuffed tracheostomy tube who is receiving tube feedings. Which finding most strongly suggests that cuff pressure should be checked?

- A.** The client needs suctioning twice during the shift
- B.** The client has a hoarse voice and food-colored secretions are noted around the tracheostomy
- C.** The client reports mild dryness of the mouth
- D.** The oxygen saturation is 97% while receiving humidified oxygen

95. A client receiving mechanical ventilation through an endotracheal tube suddenly develops a heart rate of 48/min. The monitor shows no other rhythm change, and the oxygen saturation remains 95%. The nurse is suctioning the airway at the time this occurs. Which action should the nurse take first?

- A.** Continue suctioning until all visible secretions are removed
- B.** Stop suctioning and provide oxygenation before reassessing the client
- C.** Increase the ventilator rate to improve oxygen delivery
- D.** Prepare to administer atropine immediately

96. A client with a permanent pacemaker is being monitored on telemetry. The nurse notes a heart rate of 58/min with clear pacemaker spikes visible before some beats, while other beats occur without spikes. The client denies dizziness or chest pain, and blood pressure is stable. Which interpretation is most appropriate?

- A.** The pacemaker is failing to capture and requires immediate intervention
- B.** The pacemaker is functioning as an on-demand device and is sensing intrinsic beats
- C.** The pacemaker is firing inappropriately and competing with the client's natural rhythm
- D.** The pacemaker has lost sensitivity and is unable to detect electrical activity

97. A nurse is reinforcing teaching for a client with a newly created ostomy. Which instruction is best to help reduce odor from the pouch?

- A.** Empty the pouch only once each day to keep the seal intact
- B.** Avoid all fluids between meals to reduce ostomy output
- C.** Check the pouch for leaks and empty it when it is about one-third to one-half full
- D.** Clean the stoma vigorously with scented soap at each pouch change

98. A nurse is caring for a client who had a total hip arthroplasty yesterday. Which change in the client's condition should the nurse recognize as the priority to report immediately?

- A.** The client rates incisional pain as 7 on a 0-to-10 scale 1 hour before the next analgesic dose is due
- B.** The client has not had a bowel movement since surgery
- C.** The client becomes suddenly short of breath and restless, with an oxygen saturation of 88% on room air
- D.** The client reports difficulty sleeping because of noise in the hallway

99. A nurse is caring for a client with chronic kidney disease who has missed the last two dialysis treatments. The client reports muscle weakness and nausea. Which finding requires the nurse to notify the provider immediately?

- A.** Serum potassium of 6.2 mEq/L
- B.** Serum sodium of 133 mEq/L
- C.** Blood urea nitrogen of 34 mg/dL
- D.** Phosphorus level of 5.0 mg/dL

100. A client with chronic kidney disease is receiving hemodialysis through a newly created arteriovenous fistula. Which assessment finding should the nurse report immediately?

- A.** A palpable thrill is present over the fistula site
- B.** The client says the access arm feels slightly sore
- C.** No bruit is heard over the fistula with a stethoscope
- D.** A small dry dressing is intact over the surgical site

Simulation Test 4 — Answer Key

1. B	26. B	51. B	76. B
2. C	27. D	52. C	77. C
3. C	28. C	53. A	78. B
4. C	29. B	54. C	79. A
5. B	30. C	55. B	80. C
6. C	31. A	56. B	81. A
7. C	32. C	57. B	82. B
8. B	33. B	58. C	83. A
9. C	34. A	59. C	84. B
10. B	35. A	60. B	85. B
11. B	36. B	61. B	86. C
12. B	37. C	62. C	87. A
13. C	38. A	63. A	88. B
14. C	39. A	64. C	89. A
15. C	40. A	65. C	90. A
16. B	41. B	66. C	91. B
17. C	42. C	67. B	92. B
18. B	43. B	68. C	93. A
19. C	44. B	69. B	94. B
20. B	45. C	70. A	95. B
21. C	46. C	71. A	96. B
22. B	47. B	72. A	97. C
23. A	48. B	73. A	98. C
24. B	49. C	74. A	99. A
25. A	50. B	75. B	100. C

Simulation Test 5

100 questions

1. The practical nurse is planning the next hour of care for four clients on a rehabilitation unit. Which client should the nurse see first?

- A.** A client with Parkinson disease who needs assistance getting to the bathroom
- B.** A client 2 days after a stroke whose speech is slightly more slurred than it was at breakfast
- C.** A client with chronic low back pain requesting a scheduled heat pack
- D.** A client with diabetes who asks when lunch trays will arrive

2. A practical nurse in a clinic has four actions to complete at the same time. Which action should the nurse prioritize first?

- A.** Call back a client taking furosemide who reports new muscle weakness and palpitations
- B.** Provide routine teaching to a client newly prescribed iron tablets
- C.** Fax a completed insurance form for a client referral
- D.** Document a resolved dressing change from earlier in the morning

3. A practical nurse delegates collection of a clean-catch urine specimen to an assistive personnel (AP) for a client with dysuria. Thirty minutes later, the AP says, "The client forgot the instructions and urinated into the toilet. I told the client to drink more water and we'll try again later." Which response by the practical nurse is best?

- A.** Thank the AP for handling the situation and ask for an update at the end of the shift
- B.** Tell the AP to document that the specimen could not be collected and cancel the order
- C.** Reinstruct the client on the procedure and clarify with the AP to report problems immediately rather than making independent decisions about the plan
- D.** Ask another AP to obtain the specimen because the first AP did not complete the task correctly

4. At the beginning of a shift in a long-term care facility, a practical nurse is working with an assistive personnel (AP). Which task is most appropriate for the nurse to delegate to the AP?

- A.** Teach a newly admitted client how to use a walker safely
- B.** Change the sterile dressing on a client's leg ulcer
- C.** Observe a client for increasing confusion after a fall
- D.** Help a stable client get dressed for breakfast

5. A client with limited vision is being discharged with four new prescriptions. The nurse notices that all instructions were printed in small font, and the client says, "I'll figure it out when I get home." What is the nurse's best advocacy action?

- A.** Ask the client's neighbor to review the medication instructions at home
- B.** Request teaching materials in an accessible format and review the medications using a method the client can use safely
- C.** Tell the client to call the pharmacy later if any questions come up
- D.** Document that the client accepted the printed instructions and proceed with discharge

6. A practical nurse is caring for a client who speaks limited English. The client's adult son says, "Just tell me what the doctor said, and I'll explain it to my mother later." The client is awake, appears attentive, and has not indicated that information may be shared with the son. Which action should the nurse take?

- A.** Explain the diagnosis to the son because he is helping with communication
- B.** Ask the client directly whether she wants her son involved in the discussion before sharing information
- C.** Provide the son with a written summary to translate for the client at home
- D.** Refuse to discuss anything further until the physician returns to the room

7. An adult client who is alert and has decision-making capacity asks, "What is the main purpose of an advance directive?" Which response by the nurse is best?

- A.** It allows the health care team to begin treatment without the client's consent in an emergency.
- B.** It documents the client's care preferences and may name someone to make decisions if the client cannot.
- C.** It automatically becomes active whenever the client is admitted to a hospital.
- D.** It gives family members equal authority to make medical decisions for the client.

8. A client is scheduled for an elective angiography.

When the nurse arrives with the consent form, the client says, "My son has power of attorney, so he signs all my medical papers." The client is alert, oriented, and able to explain the reason for the procedure in their own words. Which action should the nurse take?

- A.** Have the son sign because a power of attorney always signs consent forms
- B.** Ask the provider to obtain consent from the son instead of the client
- C.** Have the client sign the consent form after confirming the provider has explained the procedure

D. Delay the procedure until the facility's legal department determines who may sign

9. A client is preparing for discharge after hospitalization for newly diagnosed hypertension. The provider prescribes a new antihypertensive medication and a follow-up visit in 2 weeks. During teaching, the client says, "I can pay for the medicine, but I don't have anyone to watch my 4-year-old during appointments." Which nursing action is most appropriate before discharge?

- A.** Ask the client to call the clinic later if missing the appointment becomes a problem
- B.** Document the concern and continue discharge because the client can obtain the medication
- C.** Notify case management or social services to help identify community resources for childcare or appointment support
- D.** Tell the client to bring the child to the appointment so follow-up is not delayed

10. A practical nurse is helping with a quality improvement project on a busy clinic unit after several blood pressure readings were documented incorrectly. Before the team changes the workflow, which action is most appropriate?

- A.** Counsel the staff members who made the incorrect entries
- B.** Collect baseline data about when and how the documentation errors occur
- C.** Require every nurse to repeat blood pressure training immediately
- D.** Begin a new chart-audit form and expect error rates to drop within the week

11. A rehabilitation clinic wants to reduce clients' missed first physical therapy sessions after hospital discharge. The team starts a new process in which staff call each client within 24 hours of referral to confirm the appointment, review transportation needs, and answer questions. After 2 months, records show that staff completed the calls for 94% of referred clients, but the percentage of clients who attended their first therapy session changed from 62% to 64%. Which interpretation is most appropriate?

- A.** The project was successful because staff followed the new process consistently
- B.** The clinic should stop measuring because enough data have been collected
- C.** The intervention improved the process measure, but it has not yet meaningfully improved the client outcome
- D.** The data prove that missed therapy visits are caused only by client noncompliance

12. A practical nurse is caring for a client with dysphagia after a traumatic brain injury. At lunch, the nurse observes repeated throat clearing, wet-sounding voice, and pocketing of food. Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the findings at the end of the shift if the client finishes the meal
- B.** Immediately report the observed swallowing problems to the appropriate licensed nurse/provider and notify speech-language pathology according to facility process
- C.** Ask the nursing assistant to continue feeding the client more slowly and report back later
- D.** Switch the client to clear liquids until the next scheduled team meeting

13. A practical nurse is caring for a client with a sacral pressure injury. During the dressing change, the nurse notes a strong odor, a larger amount of thick drainage than yesterday, and a temperature increase from 37.1°C (98.8°F) this morning to 38.2°C (100.8°F) this afternoon. Which report to the wound care nurse best demonstrates effective interdisciplinary information sharing?

- A.** The wound looks worse today, and the client may need a different dressing.
- B.** The client has a pressure injury and seems uncomfortable during care.
- C.** The sacral wound now has increased thick drainage with strong odor, and the client's temperature has risen to 38.2°C; the drainage was smaller in amount and without odor yesterday.
- D.** Can you come see this client when you have time? I am concerned about the wound.

14. A practical nurse is floated to a rehabilitation unit. The charge nurse asks the nurse to complete the initial admission assessment for a newly arrived client because the unit is busy. The nurse has not been oriented to the unit's admission process and is unsure whether this task is allowed by state law and facility policy. Which action is most appropriate?

- A.** Complete the assessment to avoid delaying care, then ask later whether it was permitted
- B.** Ask the charge nurse to clarify the assignment and verify the nurse's role under facility policy before accepting the task
- C.** Refuse the entire assignment and leave the unit until a supervisor arrives
- D.** Ask an unlicensed assistive person to gather the admission information instead

15. A practical nurse is caring for an adolescent client who quietly tells the nurse, "Please don't tell anyone, but my stepfather hit me last night." The client has visible bruising on the upper arms. The client then says, "If you report this, I'll never trust you again." Which action by the nurse is most appropriate?

- A.** Promise to keep the disclosure secret to preserve the therapeutic relationship
- B.** Encourage the client to tell the provider later and take no further action unless the client agrees
- C.** Explain that the nurse must report suspected abuse through the required process and then follow facility policy
- D.** Confront the stepfather directly to determine whether the allegation is true

16. A practical nurse in a school clinic evaluates a 13-year-old student who reports frequent headaches and asks for ice for a sore shoulder. While changing into a gown for assessment, the student is found to have several parallel bruises across the back. The student quietly says, "Please don't call home. I get in trouble for talking." What is the nurse's best action?

- A.** Report the suspicion according to mandatory reporting policy and document the student's statements and physical findings
- B.** Wait to report until the student agrees to disclosure
- C.** Call the parent first to ask for an explanation of the bruising
- D.** Refer the student to the school counselor and reassess in a few days

17. A practical nurse in an assisted living facility suspects financial exploitation of an older adult after noticing unpaid pharmacy bills, repeated requests for cash withdrawals by a family member, and the client's statement, "I sign whatever my grandson puts in front of me because he gets angry if I ask questions." Which action by the nurse is most appropriate?

- A.** Ask the grandson to provide receipts before deciding whether to act
- B.** Discuss the concern only at the next routine care conference
- C.** Report the suspicion through the required channels and document specific observations and the client's quoted statement
- D.** Delay reporting unless the client signs a written complaint

18. A practical nurse is planning care for clients on a medical unit with help from one assistive personnel (AP). Which task is best to assign to the AP?

- A.** Reinforce teaching about how to use an incentive spirometer after the nurse's initial instruction
- B.** Measure and record the urine output of a client with an indwelling urinary catheter

C. Assess a client who reports new pain at an IV site

D. Decide whether a client's PRN laxative should be given based on bowel pattern

19. A practical nurse assigned an assistive personnel (AP) to obtain a clean-catch urine specimen from a client with weakness on the right side after a stroke. Twenty minutes later, the AP says, "The client could not hold the container well, so I collected urine from the bedpan after the client voided. I labeled it and set it on the counter." Which response by the nurse is most appropriate?

- A.** Send the specimen to the laboratory because it was collected right away
- B.** Ask the AP to add a note that the specimen came from a bedpan
- C.** Discard the specimen and re-explain the correct collection technique before obtaining a new sample
- D.** Document that the client was unable to provide a specimen and cancel the test

20. A practical nurse at a community clinic reviews the following situations during morning huddle. Which client is experiencing the clearest equity-related barrier that requires nursing advocacy to ensure fair access to care?

- A.** A client with newly prescribed antibiotics asks whether the medication should be taken with food
- B.** A client with chronic kidney disease says the clinic portal is the only way to view lab results, but the client does not own a computer and cannot read English
- C.** A client with a sprained ankle reports pain of 6 on a 0-to-10 scale while waiting to be seen
- D.** A client with seasonal allergies says over-the-counter medicine has not worked this week

21. A practical nurse in a pediatric clinic is helping coordinate care for a child with asthma. The parent says, "We recently moved, and I do not understand the bus route to the pharmacy or to the clinic. I missed the last visit because I got lost, and I am almost out of the inhaler." Which action by the nurse best promotes equitable access to care?

- A.** Tell the parent to call back after finding transportation independently
- B.** Provide the parent with written bus-route directions, confirm the preferred language, and connect the family with clinic staff who can help arrange follow-up
- C.** Advise the parent to wait until the next scheduled appointment to discuss refill options
- D.** Suggest that the parent purchase an over-the-counter cough medicine until transportation is available

22. A nurse in a dialysis center is preparing to start treatment for a client who has significant hearing loss and speaks very little English. The client nods when the nurse says a name, but several family members are answering questions for the client. Which action best identifies the client before care begins?

- A.** Ask a family member to confirm the client's identity and proceed if the family agrees
- B.** Use the client's room or chair assignment along with the client's appearance to confirm identity
- C.** Compare the client's identification band with the treatment record and verify a second identifier using an approved source such as date of birth
- D.** Accept the client's nod as confirmation because the client responded when the name was spoken

23. A nurse is preparing to use a home glucometer to check a client's capillary blood glucose. The meter displays a message that the test strips are expired. The client says, "It's probably close enough. Just use them so I don't have to wait." Which action by the nurse is best?

- A.** Use the expired strips if they have been stored in a sealed container
- B.** Use the expired strips for this reading, then replace them before the next check
- C.** Obtain unexpired test strips before performing the blood glucose test
- D.** Perform the test with the expired strips and confirm only if the result seems unusual

24. A nurse is preparing to administer an influenza vaccine in a clinic. Before the injection, the client says, "The last time I got a vaccine, I started wheezing and my face swelled within minutes." The current record does not describe the reaction. What is the nurse's priority action?

- A.** Administer the vaccine and observe the client for 30 minutes
- B.** Hold the vaccine and clarify the allergy history with the client and provider before proceeding
- C.** Give the vaccine in a smaller dose to reduce the chance of a reaction
- D.** Ask another nurse to administer the vaccine to confirm the technique is correct

25. A practical nurse is caring for clients in a long-term care facility when the fire alarm sounds. After moving the clients in immediate danger away from the smoke-filled area, which action should the nurse take next?

- A.** Close doors and windows near the fire area
- B.** Return to client rooms to gather medications
- C.** Use the elevator to move nonambulatory clients more quickly
- D.** Open the nurses' station windows to clear the smoke

26. A client who had a stroke yesterday is trying to slide out of the chair without calling for help. The client is weak on the left side, becomes frustrated easily, and says, "I need to go now." Which nursing action best follows the principle of using the least-restrictive intervention first?

- A.** Apply a vest restraint to keep the client in the chair
- B.** Move the client closer to the nurses' station and offer scheduled toileting assistance
- C.** Raise all four side rails while the client remains in the chair
- D.** Ask the provider for an order for bilateral wrist restraints

27. A nurse opens a package of prefilled insulin syringes for morning administration and notices that one syringe is labeled with the correct medication but has no client identifier on it. The nurse has not given the insulin and cannot verify who the syringe was prepared for. Which action is most appropriate?

- A.** Administer the insulin after checking the medication administration record for the next client due to receive it
- B.** Ask another nurse whether they remember which client the syringe was intended for, then give it if they agree
- C.** Set the syringe aside, obtain a properly labeled dose, and report the near-miss according to facility policy
- D.** Relabel the syringe with the intended client's name after comparing the dose with the prescription

28. A nurse is deciding how to move a postoperative client from a wheelchair back to bed. The client is alert, follows directions, has good upper-body strength, cannot bear weight on either leg, and became short of breath during a previous assisted pivot transfer. Which transfer device is the safest choice?

- A.** Transfer board
- B.** Gait belt
- C.** Sit-to-stand lift
- D.** Full-body mechanical lift

29. A practical nurse enters a client's room to assist with repositioning. The nurse is wearing gloves after handling the client's soiled dressing. The client's call light falls onto the bed, and the nurse needs to place it back within reach. Which action is most appropriate?

- A.** Place the call light within reach while still wearing the gloves, then remove the gloves afterward
- B.** Ask the client to wait, leave the room, and return later with clean gloves
- C.** Remove the gloves, perform hand hygiene, and then place the call light within reach
- D.** Wipe the gloved hands on a towel before touching the call light

30. A practical nurse is preparing to change a sterile dressing on a central line site. After opening the sterile dressing kit, the nurse notices that one corner of the sterile drape is hanging below waist level. Which action should the nurse take?

- A.** Continue the procedure because only the corner of the drape is below waist level
- B.** Reposition the drape onto the bedside table and use the rest of the field
- C.** Consider only the hanging corner contaminated and avoid placing supplies on that area
- D.** Discard the setup and prepare a new sterile field

31. A nurse is preparing to assist a client with oral care after the client has been coughing up thick secretions into a basin. The nurse expects close contact with the client's mouth and possible spray of secretions during rinsing and spitting. Which personal protective equipment should the nurse use?

- A.** Gloves only
- B.** Gloves and a gown
- C.** Gloves, mask, and eye protection
- D.** Sterile gloves, mask, and shoe covers

32. A practical nurse is preparing to assist with a sterile bedside procedure. After performing hand hygiene, the nurse opens the sterile tray and begins arranging supplies. Which action contaminates the sterile field?

- A.** Keeping the sterile tray above waist level during setup
- B.** Placing a sterile item onto the center of the field without reaching over it
- C.** Turning away from the sterile field to answer a question from another nurse
- D.** Opening the outer flap of the sterile wrapper away from the body first

33. A practical nurse is reinforcing teaching for an unlicensed assistive personnel (UAP) who will deliver a meal tray to a client with active pulmonary tuberculosis. Which statement by the UAP indicates correct understanding of isolation precautions?

- A.** I should wear an N95 respirator before entering the client's room.
- B.** I only need gloves because I will not be touching the client.
- C.** I can leave the door open briefly as long as I stay near the doorway.
- D.** I should put on a surgical mask after I enter the room.

34. A practical nurse is preparing to transport a client to radiology. The client has no isolation orders. Before leaving the room, the nurse notices that the client's wheelchair armrest has a smear of dried blood from a previous skin tear. Which action best follows standard precautions?

- A.** Place a clean towel over the armrest and transport the client immediately
- B.** Wipe the blood off with a dry paper towel, then perform hand hygiene
- C.** Clean and disinfect the armrest with appropriate equipment-cleaning supplies before use
- D.** Ask radiology staff to clean the wheelchair after the transport is complete

35. A nurse is reinforcing teaching with a client who is 30 weeks pregnant about receiving immunizations during pregnancy. Which statement by the client indicates correct understanding?

- A.** I should avoid all vaccines until after my baby is born.
- B.** I can receive an inactivated influenza vaccine during pregnancy if it is recommended.
- C.** Vaccines are only safe during the first trimester.
- D.** If I had childhood vaccines, I should not need any immunizations during pregnancy.

36. A client in active labor has been receiving IV fluids and opioid pain medication. Thirty minutes later, the nurse notes the fetal heart rate baseline has dropped from 140/min to 110/min with minimal variability. Maternal vital signs are blood pressure 86/52 mm Hg, pulse 118/min, and respirations 20/min. Which action should the nurse take first?

- A.** Place the client in a side-lying position
- B.** Perform a sterile vaginal examination
- C.** Administer the next dose of opioid pain medication
- D.** Prepare the client for immediate vacuum-assisted birth

37. A nurse is reinforcing self-care teaching for a client who is 1 day postpartum after an uncomplicated vaginal birth. Which instruction is most appropriate to help prevent infection during the postpartum period?

- A.** Use tampons until vaginal bleeding stops
- B.** Clean the perineal area from front to back after voiding
- C.** Delay showering for several days to protect the perineum
- D.** Apply powder to the perineal area after each pad change

38. A nurse observes several children in a pediatric waiting room. Which behavior is most consistent with expected development for a 24-month-old toddler?

- A.** Uses a spoon well and speaks in two-word phrases
- B.** Hops on one foot and draws a person with six body parts

- C. Sits without support and transfers an object from one hand to the other
- D. Prints first name and skips alternating feet

39. A practical nurse is obtaining baseline physical data for a client newly admitted from home after a mild stroke. Which question best helps establish the client's baseline communication status?

- A. Are you having pain anywhere right now?
- B. How long have you been feeling weak on your left side?
- C. Before this illness, did you usually speak clearly and understand conversations without difficulty?
- D. Would you like your family to stay during the assessment?

40. A nurse is teaching the parent of a child who received an inactivated vaccine at a well-child visit. Which statement by the parent indicates correct understanding?

- A. My child could develop the disease from this vaccine for a few days.
- B. This vaccine uses a killed form of the germ and cannot cause the infection it protects against.
- C. If my child has soreness at the injection site, it means the vaccine should not be given again.
- D. This type of vaccine gives lifelong protection after one dose.

41. A nurse is reinforcing teaching with the parent of a 9-year-old child who asks why the child suddenly cares so much about fitting in with friends. Which response by the nurse is most appropriate?

- A. Children this age usually prefer solitary play over spending time with peers.
- B. Wanting approval from friends is common during the school-age years.
- C. This behavior suggests a delay in emotional development.
- D. Interest in peer acceptance usually does not begin until late adolescence.

42. A nurse is obtaining a health history from a client who recently had a stroke. The client understands questions but answers slowly with single words and becomes frustrated when rushed. Which action by the nurse is best to reduce this communication barrier?

- A. Ask the client's family member to answer most of the questions
- B. Repeat each question several times until the client responds more quickly
- C. Use short, simple questions and allow extra time for the client to respond
- D. Speak loudly and continuously rephrase the questions in different ways

43. A nurse is teaching a client newly prescribed insulin how to draw up and inject the medication. The client avoids eye contact, wrings their hands, and says, "My spouse always handled medical stuff. I don't think I can do this." Which nursing action is best?

- A. Continue the full teaching session so the client receives all needed information before discharge
- B. Ask the client what part of giving insulin feels most difficult, then provide teaching in small steps
- C. Give the client a detailed written packet to review alone before the next injection is due
- D. Tell the client that insulin injections are simple and there is no reason to be nervous

44. A client who recently lost vision in one eye after an accident says to the practical nurse, "Everyone keeps telling me to stay positive, but I just feel frustrated and useless." Which response by the nurse best provides emotional support?

- A. You should try to focus on the fact that your other eye still works.
- B. It sounds like this change has been very hard for you. Tell me more about what has been most frustrating.
- C. Many people adjust over time, so you will probably feel better soon.
- D. You do not need to feel useless because your family can help with what you cannot do.

45. A client who recently began hemodialysis tells the nurse, "I keep thinking about everything I won't be able to do now, so I just stay in bed and avoid talking about it." Which response by the nurse best promotes adaptive coping?

- A. "You should focus on being grateful that dialysis is available."
- B. "Try not to think negatively. Most clients adjust if they stay positive."
- C. "Tell me which part of starting dialysis feels hardest for you right now."
- D. "It is best to rest until you feel emotionally ready to deal with this."

46. A nurse in an acute care clinic is assessing a client who appears increasingly agitated during the visit. Which finding should the nurse identify as the strongest warning sign of possible violence?

- A. The client avoids eye contact and answers questions with brief responses
- B. The client wrings both hands and asks when the appointment will end
- C. The client begins shouting, clenches the jaw, and repeatedly steps into the nurse's personal space
- D. The client reports feeling tired after several nights of poor sleep

- 47.** A client arrives at the emergency department reporting daily heavy alcohol use and says the last drink was 12 hours ago. The client is restless, has visible hand tremors, and asks, "Can I just go home and sleep this off?" Which nursing action is the priority?
- A.** Place the client in a quiet room and encourage rest
 - B.** Assess vital signs and history of prior withdrawal complications
 - C.** Explain the long-term effects of alcohol misuse
 - D.** Offer oral fluids and ask the client to return if symptoms worsen
- 48.** A client in a long-term care facility with mild cognitive impairment becomes frustrated during lunch, throws a spoon onto the tray, and says, "Leave me alone!" The client is breathing quickly but remains seated. Which action should the nurse take first?
- A.** Tell the client that throwing objects is unacceptable behavior
 - B.** Stay with the client, speak calmly, and reduce stimulation at the table
 - C.** Ask another staff member to remove the tray immediately and escort the client to the room
 - D.** Explain in detail why the client needs to finish the meal
- 49.** A client with mild dementia is admitted to a long-term care facility. The client becomes frustrated each morning because clothing choices feel overwhelming and says, "I can't figure out what to wear." Which nursing action is most appropriate?
- A.** Lay out two simple outfit choices and allow the client to choose between them
 - B.** Open the closet and encourage the client to dress independently without assistance
 - C.** Choose the client's clothing without explanation so the task is completed quickly
 - D.** Ask the client to describe what was worn yesterday to help improve memory
- 50.** A client says, "By the end of the day, I feel so tense that I snap at people. I want something I can do the moment I notice myself getting stressed." Which nursing recommendation is most appropriate?
- A.** Practice slow deep-breathing exercises when stress begins
 - B.** Wait to discuss the stress until the client feels calmer later
 - C.** Avoid thinking about the source of stress as much as possible
 - D.** Use caffeinated drinks to improve energy during stressful periods
- 51.** A client admitted for uncontrolled diabetes tells the practical nurse, "I am supposed to take medication with food at sunrise and after sunset during this holy month. I don't want to break my practice, but I also don't want to get sicker." Which nursing action is most appropriate?
- A.** Tell the client that prescribed treatment must be followed exactly as ordered while hospitalized
 - B.** Ask the client to explain the practice and notify the supervising nurse and provider so the plan of care can be reviewed
 - C.** Advise the client to skip the medication until the holy month is over
 - D.** Reassure the client that spiritual practices are less important than controlling blood glucose during illness
- 52.** A nurse is teaching a client about a newly prescribed low-sodium diet. The client smiles, nods, and says, "Yes, yes," throughout the discussion. When the nurse asks what foods the client plans to avoid, the client repeats the nurse's last sentence word for word but cannot give any examples. Which action by the nurse is most appropriate?
- A.** Repeat the instructions more slowly using louder speech
 - B.** Ask the client to explain the diet in their own words and use familiar food examples
 - C.** Document that the client understood the teaching because the client agreed throughout the session
 - D.** Postpone teaching until a family member is present to receive the information instead
- 53.** A client who identifies as a transgender woman is admitted for abdominal pain. Before the nurse begins the assessment, the client says, "Please don't use certain words for my body. It makes me really uncomfortable." Which response by the nurse is best?
- A.** "I need to use the standard medical terms so the assessment stays accurate."
 - B.** "Tell me what terms you prefer, and I will use those when possible during your care."
 - C.** "We can skip the questions about those areas if they are upsetting to you."
 - D.** "I will ask another nurse to do the assessment if this is too difficult for you."
- 54.** A client with progressive vision loss tells the nurse, "Since I can't recognize people easily anymore, I stopped going to the recreation room. I just sit here so I don't make a fool of myself." Which nursing action is best?
- A.** Tell the client that avoiding activities will only worsen the problem and encourage immediate participation
 - B.** Arrange the environment so the client can identify people more easily and offer to accompany the client to the recreation room

- C.** Suggest that the client wait to join activities until the vision loss has been fully treated
- D.** Reassure the client that the staff will explain the situation to others so the client does not need to worry

55. A client who is receiving end-of-life care tells the nurse, "I keep thinking about all the mistakes I made. Maybe I deserve this." Which response by the nurse is most therapeutic?

- A.** "You should try not to think about the past right now."
- B.** "Many people feel this way when they are very ill."
- C.** "Tell me more about what you have been thinking about."
- D.** "You do not deserve this, so you need to stop blaming yourself."

56. A nurse is caring for a client with a newly placed gastrostomy tube. Which finding should the nurse report to the provider immediately?

- A.** A small amount of dried drainage at the insertion site
- B.** Mild tenderness around the stoma during routine care
- C.** The external bolster is tightly pressing into the skin
- D.** A pink, moist stoma with intact surrounding skin

57. A nurse is teaching a client to use a quad cane because of weakness in the right leg. Which client action demonstrates correct cane use while walking on a level surface?

- A.** Moves the right leg forward, then the cane, then the left leg
- B.** Moves the cane and right leg forward together, then steps through with the left leg
- C.** Moves the cane forward, then the left leg, then the right leg
- D.** Moves the left leg and cane forward together, then brings the right leg forward

58. A nurse is caring for a client who has nausea and a poor appetite after surgery. Which intervention is most appropriate to help promote oral intake?

- A.** Offer small, frequent meals that include the client's preferred foods
- B.** Encourage the client to drink large amounts of fluid with each meal
- C.** Serve foods with strong aromas to stimulate the appetite
- D.** Delay meals until the client reports being completely free of nausea

59. An older adult with chronic neck muscle tension reports mild aching pain and says, "I would like to try something without medicine first." Which nonpharmacologic intervention is most appropriate for the nurse to implement?

- A.** Apply a warm, moist compress to the neck muscles

- B.** Place the client flat in bed with one pillow removed
- C.** Massage the painful area vigorously for 10 minutes
- D.** Apply an ice pack directly to the skin over the neck

60. A nurse is teaching a client who has left-sided weakness from a recent stroke how to put on pants with as much independence as possible. Which instruction should the nurse give?

- A.** Put the stronger leg into the pants first, then the weaker leg
- B.** Put both legs into the pants at the same time while standing
- C.** Put the weaker leg into the pants first, then the stronger leg
- D.** Have another person pull the pants up to avoid fatigue

61. A nurse is reinforcing teaching for a client who has stress urinary incontinence. Which instruction should the nurse give to help reduce urine leakage episodes?

- A.** Void every 8 hours during the day
- B.** Perform pelvic floor muscle exercises several times each day
- C.** Limit fluid intake to less than 1 L per day
- D.** Drink caffeinated beverages to stimulate bladder emptying

62. A practical nurse is helping transfer a client from the bed to a stretcher. The client cannot assist and has fragile skin. Which action best reduces the risk of skin injury during the move?

- A.** Pull the client across the bed by grasping the drawsheets at the shoulders and hips
- B.** Lift the client with a friction-reducing device or lift sheet rather than dragging
- C.** Raise the head of the bed to high Fowler's position before moving the client
- D.** Move the client quickly in one motion to shorten the transfer time

63. A practical nurse is caring for a client with decreased sensation after a stroke. The client says, "I like to use a heating pad on my weak arm when it feels stiff." Which response by the nurse is most appropriate?

- A.** Use the heating pad on the highest setting for 10 minutes at a time.
- B.** Wrap the heating pad tightly around the arm so the heat stays in place.
- C.** Avoid applying heat to the arm unless the provider specifically approves it, because reduced sensation increases the risk for burns.
- D.** Place the heating pad on the arm only at bedtime so movement will not disturb it.

- 64.** A client is on strict intake and output. At breakfast, the client drinks 4 oz of orange juice and 6 oz of coffee, eats 1/2 cup of yogurt, and receives 30 mL of liquid medication by mouth. Later, the client has 8 oz of ice chips. If ice chips count as half their measured volume when melted, how much intake should the practical nurse record?
- A. 510 mL
 - B. 570 mL
 - C. 630 mL
 - D. 690 mL
- 65.** A practical nurse is caring for a client who has been immobile in bed for several days. During morning care, which finding should the nurse report first as most concerning for a complication of immobility?
- A. A reddened area over the sacrum that blanches with pressure
 - B. A small decrease in appetite at breakfast
 - C. A warm, swollen lower leg with new tenderness
 - D. Stiffness in the joints after waking
- 66.** A nurse is preparing to administer a scheduled dose of levothyroxine to a client. After comparing the medication label with the MAR, the nurse notices the MAR lists the dose as 0.125 mg, while the package label reads 125 mcg. Which action is most appropriate?
- A. Administer the medication because 0.125 mg and 125 mcg are equivalent doses
 - B. Hold the medication and contact the pharmacy because the units are written differently
 - C. Ask another nurse to decide whether the medication should be given
 - D. Document the discrepancy as a medication error before administering the dose
- 67.** A practical nurse is preparing a client's 0900 medications and notices the MAR lists furosemide 40 mg PO daily. The provider's prescription entered last night reads furosemide 20 mg PO daily, and the outgoing nurse documented that the 0600 dose was not given. The client asks, "Am I still getting my water pill this morning?" What is the nurse's best action?
- A. Administer 40 mg because the MAR is the document used during medication pass
 - B. Hold the medication and notify the provider that the client refused the dose
 - C. Clarify the discrepancy with the charge nurse or provider and withhold the medication until the order and MAR match
 - D. Administer 20 mg based on the provider's prescription and correct the MAR after giving it
- 68.** A nurse is teaching a patient how to use a prescribed fentanyl patch at home for chronic pain management. Which patient statement indicates correct understanding of the route of administration?
- A. I should place the patch on broken skin so the medicine absorbs faster.
 - B. I can cut the patch in half if the dose feels too strong.
 - C. I will apply the patch to intact skin and replace it as prescribed.
 - D. I should remove the patch whenever pain improves and reapply the same patch later.
- 69.** A client with severe burns is receiving albumin. Which assessment finding best indicates the therapy is having the intended effect?
- A. Decreased peripheral edema and improved blood pressure
 - B. Increased hemoglobin level
 - C. Shorter activated partial thromboplastin time (aPTT)
 - D. Higher platelet count
- 70.** A nurse needs to administer 180 mL of IV antibiotic over 90 minutes using tubing calibrated at 10 gtt/mL. How many drops per minute should the nurse regulate?
- A. 12 gtt/min
 - B. 20 gtt/min
 - C. 30 gtt/min
 - D. 45 gtt/min
- 71.** A nurse is preparing to administer the next dose of metformin to a client with type 2 diabetes. Which pre-administration data is most important for the nurse to review?
- A. The client's most recent serum creatinine level
 - B. The client's preferred meal schedule
 - C. The client's daily weight from the past 3 days
 - D. The client's bowel movement pattern
- 72.** A client with generalized tonic-clonic seizures has been taking phenytoin for 3 months. Which finding best indicates the medication is producing the intended therapeutic response?
- A. The client reports no seizure episodes during the past 2 months
 - B. The client has a serum phenytoin level of 18 mcg/mL
 - C. The client reports mild gingival tenderness when brushing teeth
 - D. The client has a heart rate of 88/min and blood pressure of 124/76 mm Hg

73. A client who started taking carbamazepine 3 weeks ago for trigeminal neuralgia calls the clinic. Which new symptom should the nurse instruct the client to report to the provider immediately as a possible serious adverse response?

- A.** Mild drowsiness after each dose
- B.** Sore throat with fever
- C.** Occasional nausea when taking the medication without food
- D.** Dry mouth at bedtime

74. A patient with chronic back pain takes oxycodone as prescribed. During a follow-up call, the patient says, “I started taking a new allergy medicine, and now I feel unusually sleepy.” Which over-the-counter medication should the nurse identify as the most likely cause of this drug interaction?

- A.** Loratadine
- B.** Diphenhydramine
- C.** Guaifenesin
- D.** Calcium carbonate

75. A client with a mechanical heart valve is receiving a continuous IV infusion of unfractionated heparin. Before hanging a new bag, the nurse reviews the morning laboratory results and notes an activated partial thromboplastin time (aPTT) of 124 seconds. Which action is most appropriate?

- A.** Hang the new bag as prescribed because clients on heparin are expected to have an elevated aPTT
- B.** Slow the infusion rate slightly and recheck the aPTT in 24 hours
- C.** Hold the infusion and notify the provider of the critical aPTT result
- D.** Administer vitamin K and then continue the infusion at the prescribed rate

76. A nurse is teaching a client who has a new prescription for levothyroxine to treat hypothyroidism. Which statement by the client indicates correct understanding of the teaching?

- A.** I will take this medication with breakfast so it does not upset my stomach.
- B.** I should stop taking this medication once my energy level improves.
- C.** I will take this medication at the same time each morning on an empty stomach.
- D.** If I miss a dose, I should take two tablets together at my next scheduled time.

77. A nurse is organizing medications in a clinic refrigerator and notes the following storage locations. Which finding requires immediate correction?

- A.** A box of prefilled syringes stored on the center shelf with space between packages for air circulation
- B.** Several medication vials placed in the refrigerator door compartment for easy access
- C.** A labeled bin of refrigerated eye drops stored away from food and specimens
- D.** Unopened insulin cartons stored in a dedicated medication refrigerator

78. A nurse is preparing to administer enoxaparin 30 mg subcutaneously. The prefilled syringe available is labeled 40 mg/0.4 mL. How many milliliters should the nurse administer?

- A.** 0.2 mL
- B.** 0.3 mL
- C.** 0.4 mL
- D.** 0.75 mL

79. A practical nurse is collecting data on four clients in an outpatient clinic. Which client should the nurse identify as being at greatest risk for medication-related injury?

- A.** A client with hypertension who reports taking an herbal sleep aid along with a newly prescribed antihypertensive medication
- B.** A client with seasonal allergies who forgot one dose of an over-the-counter antihistamine yesterday
- C.** A client with acne who started a topical antibiotic 2 days ago and reports mild skin dryness
- D.** A client with hypothyroidism who takes levothyroxine every morning and asks whether it should be taken before breakfast

80. A nurse needs to collect a urine specimen for routine urinalysis from a client who is menstruating and can follow instructions independently. Which action is most appropriate to help reduce contamination of the specimen?

- A.** Collect the first urine that the client voids without any perineal cleansing
- B.** Obtain the specimen from the urine remaining in the bedside commode after voiding
- C.** Ask the client to cleanse the perineal area and collect a midstream urine specimen
- D.** Delay the specimen collection until the menstrual period has completely ended

81. A nurse is reviewing morning laboratory results for a client who is receiving trimethoprim-sulfamethoxazole for a urinary tract infection. Which result should the nurse report to the provider first?

- A.** Platelet count 185,000/mm³

- B. White blood cell count 2,100/mm³
- C. Hemoglobin 11.2 g/dL
- D. Sodium 134 mEq/L

82. A client who is 10 weeks pregnant comes to the clinic reporting nausea and vomiting for the past 3 days. Today the client says, "I can't even keep water down, and I feel dizzy when I stand up." Which finding is most important for the nurse to report to the provider?

- A. Weight loss of 5 lb since the last prenatal visit
- B. Urine ketones are present on dipstick testing
- C. The client says crackers help a little in the morning
- D. The client reports increased sensitivity to food odors

83. A nurse is about to perform a bedside fecal occult blood test using a stool specimen collected from a bedpan. The specimen is mixed with urine. What should the nurse do?

- A. Perform the test because urine does not affect the result
- B. Test the specimen but document that it was contaminated
- C. Discard the specimen and obtain a new stool sample that is not contaminated with urine
- D. Add tap water to the specimen to dilute the urine before testing

84. A practical nurse is preparing to obtain an adult client's tympanic temperature. The client has been lying on the affected side and just finished using a heating pad on that ear for comfort. Which action is most appropriate?

- A. Measure the temperature in the same ear immediately because tympanic readings are not affected by local heat
- B. Use the other ear or wait before measuring to avoid an inaccurate elevated reading
- C. Switch to an axillary temperature because ear temperatures should never be taken in adults
- D. Pull the pinna down and back before inserting the probe

85. A nurse is obtaining a 12-lead ECG for a client who reports intermittent palpitations. After the leads are attached, the tracing shows a wandering baseline and unclear waveforms. The client is lying still and denies feeling cold. Which action should the nurse take next to improve the tracing?

- A. Ask the client to take slow deep breaths during the recording
- B. Check that the lead wires and electrodes are securely connected
- C. Place the client in high-Fowler's position
- D. Offer the client water and repeat the ECG in 15 minutes

86. A nurse is collecting a venous blood specimen from an adult client. After inserting the needle, blood begins to flow into the tube, but then the specimen suddenly stops flowing before the tube is filled. The client reports no pain, and the site shows no swelling. Which action should the nurse take first?

- A. Pull the needle out slightly to search for the vein
- B. Release the tourniquet and remove the needle immediately
- C. Gently reposition the tube in the holder to ensure the tube stopper is engaged
- D. Ask the client to pump the fist several times

87. A practical nurse is receiving a client from the postanesthesia care unit 45 minutes after an open appendectomy. Which finding is most important to report to the RN immediately?

- A. Temperature of 99.1°F (37.3°C)
- B. Blood pressure of 88/54 mm Hg
- C. Incisional pain rated 6 on a 0-to-10 scale
- D. Small amount of serosanguineous drainage on the dressing

88. A client is receiving IV potassium chloride through a peripheral line. About 10 minutes after the infusion begins, the client reports burning at the IV site. The nurse notes redness and tenderness along the vein, but no coolness or swelling. Which action is most appropriate?

- A. Slow the infusion rate and continue to monitor the site
- B. Discontinue the IV site and restart the infusion in a new vein
- C. Apply a cold compress and continue the prescribed infusion
- D. Flush the catheter with normal saline to check for patency

89. A nurse is caring for a client who is 6 hours after a laparoscopic cholecystectomy. The client says, "I feel my heart racing, and my shoulder pain is suddenly much worse." Which focused assessment should the nurse perform first?

- A. Inspect the abdominal dressings and assess the abdomen for distention and firmness
- B. Ask the client which foods usually trigger abdominal discomfort
- C. Review the client's preoperative consent form
- D. Reposition the client and dim the room lights

90. A client with hypokalemia is receiving IV potassium chloride supplementation. Which finding best indicates the treatment is effective?

- A. The client reports increased appetite

- B.** The client's serum potassium rises from 2.9 to 3.8 mEq/L
- C.** The client's urine output decreases from 40 mL/hr to 20 mL/hr
- D.** The client reports mild burning at the IV site

91. A client receiving an IV antibiotic suddenly reports itching and dizziness. Assessment findings include widespread hives, inspiratory stridor, blood pressure 82/48 mm Hg, and heart rate 126/min. Which action should the nurse take first?

- A.** Stop the IV antibiotic infusion
- B.** Place the client in high-Fowler position
- C.** Administer epinephrine as prescribed for anaphylaxis
- D.** Apply oxygen at 2 L/min by nasal cannula

92. A client with chronic heart failure is prescribed furosemide 40 mg by mouth daily and digoxin 0.125 mg by mouth daily. Before administering the morning medications, the practical nurse notes an apical pulse of 56/min, reports of nausea, and new blurred vision with yellow halos around lights. Which action is best?

- A.** Administer both medications as prescribed and reassess the client in 1 hour
- B.** Hold the digoxin and notify the provider of the findings
- C.** Give the digoxin and hold the furosemide because the pulse is low
- D.** Encourage the client to eat breakfast first, then give both medications

93. A client with community-acquired pneumonia has coarse crackles and a weak cough. The client asks, "What can I do to help get this mucus up?" Which nursing instruction is most appropriate?

- A.** Take rapid, shallow breaths, then cough several times in a row
- B.** Limit fluid intake so secretions do not become heavier
- C.** Sit upright, take a deep breath, hold it briefly, then cough forcefully
- D.** Lie flat for 30 minutes before coughing to conserve energy

94. A nurse is caring for a client with a tracheostomy who suddenly becomes anxious and tachypneic. Assessment reveals diminished air movement through the tube and the client is unable to clear secretions by coughing. After quickly applying oxygen, which action should the nurse take next?

- A.** Remove and clean the inner cannula
- B.** Suction the tracheostomy using sterile technique
- C.** Deflate the cuff to improve airflow
- D.** Change the tracheostomy ties

95. A client who is receiving mechanical ventilation is prescribed enteral feedings through a nasogastric tube. Which nursing action is most appropriate to reduce the risk of aspiration during feeding?

- A.** Place the client flat in bed during the feeding to reduce fatigue
- B.** Elevate the head of the bed to at least 30 degrees during the feeding
- C.** Suction the endotracheal tube every hour while the feeding is infusing
- D.** Stop oral care until the feeding is completed

96. A client with a permanent pacemaker is admitted after a fall. During the assessment, the nurse notices a strong, regular twitching of the client's left pectoral muscle near the generator site. The client says, "My chest keeps jumping." Telemetry shows an underlying sinus rhythm at 76/min with occasional pacer spikes. Which action should the nurse take first?

- A.** Document the finding as expected because the client has a pacemaker
- B.** Assess the client for pacemaker lead displacement or malfunction and notify the provider
- C.** Apply oxygen at 2 L/min by nasal cannula and continue to observe
- D.** Instruct the client to limit arm movement for 24 hours and reassess later

97. A nurse is reinforcing teaching for a client with a transverse colostomy. Which instruction is most appropriate for helping the client avoid loosening of the pouch seal during routine care?

- A.** Clean the skin around the stoma with an oil-based lotion before applying the pouch
- B.** Apply the pouching system to skin that is clean and completely dry
- C.** Cut the skin barrier opening much larger than the stoma to prevent pressure
- D.** Change the pouch immediately after eating a full meal

98. A nurse is caring for a client with a small-bore feeding tube who has been receiving continuous enteral feedings without difficulty. Which change in condition should the nurse recognize as the priority to address immediately?

- A.** The client reports dry lips and requests ice chips
- B.** The client has a residual volume of 110 mL before the next bag is hung
- C.** The client develops sudden coughing, oxygen saturation drops from 97% to 89%, and breath sounds are coarse
- D.** The client has a soft, loose stool after receiving a liquid formula

99. A nurse is caring for a client who received extensive thermal burns 12 hours ago. Which finding is most important for the nurse to report to the provider immediately?

- A.** Urine output of 20 mL over the past hour
- B.** Serum potassium level of 6.1 mEq/L
- C.** Generalized edema at the burn sites
- D.** Hematocrit of 52%

100. A client with chronic kidney disease returns from hemodialysis and reports feeling dizzy. Which assessment finding most strongly suggests the dizziness is related to fluid removal during dialysis?

- A.** Blood pressure decreased from 148/86 mm Hg before dialysis to 98/60 mm Hg after dialysis
- B.** Temperature increased from 98.4°F to 99.1°F
- C.** Urine output increased from 100 mL to 180 mL over 8 hours
- D.** Serum potassium increased from 4.2 mEq/L to 4.5 mEq/L

Simulation Test 5 — Answer Key

1. B	26. B	51. B	76. C
2. A	27. C	52. B	77. B
3. C	28. D	53. B	78. B
4. D	29. C	54. B	79. A
5. B	30. D	55. C	80. C
6. B	31. C	56. C	81. B
7. B	32. C	57. B	82. B
8. C	33. A	58. A	83. C
9. C	34. C	59. A	84. B
10. B	35. B	60. C	85. B
11. C	36. A	61. B	86. C
12. B	37. B	62. B	87. B
13. C	38. A	63. C	88. B
14. B	39. C	64. B	89. A
15. C	40. B	65. C	90. B
16. A	41. B	66. A	91. C
17. C	42. C	67. C	92. B
18. B	43. B	68. C	93. C
19. C	44. B	69. A	94. A
20. B	45. C	70. B	95. B
21. B	46. C	71. A	96. B
22. C	47. B	72. A	97. B
23. C	48. B	73. B	98. C
24. B	49. A	74. B	99. B
25. A	50. A	75. C	100. A

Simulation Test 6

100 questions

1. Near the end of the shift, a practical nurse must decide which newly reported change needs attention first. Which situation is the highest priority?
 - A. A client with pneumonia becomes restless and has an oxygen saturation decrease from 95% to 89%
 - B. A client with gout asks for the next scheduled dose of anti-inflammatory medication
 - C. A client 2 days after appendectomy reports incisional pain rated 5/10
 - D. A client with iron-deficiency anemia requests help walking to the bathroom
2. A practical nurse is creating the care sequence for the next 30 minutes. Which action should the nurse perform first?
 - A. Reinforce teaching for a client newly prescribed a low-sodium diet before discharge this afternoon
 - B. Obtain a repeat blood pressure for a client whose reading is 86/54 mm Hg and who now reports dizziness
 - C. Call the pharmacy about a routine multivitamin dose missing from the medication drawer
 - D. Document intake and output totals for a client with an indwelling urinary catheter
3. A practical nurse delegated routine morning hygiene to an assistive personnel (AP) for a stable older adult. Ten minutes later, the AP reports, "While I was helping the client wash up, I noticed a new red area over the coccyx." What is the nurse's best action?
 - A. Ask the AP to apply a foam dressing and continue hygiene care
 - B. Assess the area promptly and determine whether the care plan needs to be updated
 - C. Tell the AP to document the finding because the AP discovered it
 - D. Instruct the AP to reposition the client and report the skin change at the end of the shift
4. A practical nurse is supervising an assistive personnel (AP) worker in a postoperative unit. Which task is most appropriate for the nurse to delegate to the AP?
 - A. Help a stable client who had surgery 2 days ago use the bedside commode after the nurse has assessed the client's mobility
 - B. Teach a client how to use a patient-controlled analgesia pump
 - C. Assess a client's incision after the client reports increased drainage
 - D. Decide whether a client's nausea requires contacting the provider
5. An alert adult client is admitted for pneumonia. While reviewing the plan of care, the nurse asks whether the client would like information about completing an advance directive. The client replies, "No, I don't want one, and I don't want anyone asking me about it again." What is the nurse's best response?
 - A. Document the client's decision and continue care without pressuring the client to complete one
 - B. Explain that an advance directive is required before further treatment can be provided
 - C. Ask the client's closest relative to complete the form on the client's behalf
 - D. Delay nonurgent medications until the client agrees to discuss the form again
6. A nurse is caring for a client who has mobility limitations after a stroke. The client is scheduled for discharge the next morning to a second-floor apartment with no elevator. During teaching, the client says, "My daughter works nights, and I will be alone most of the day. I don't think I can get up those stairs safely." What is the nurse's best advocacy action?
 - A. Tell the client to limit stair use until strength improves
 - B. Proceed with discharge because the provider has already written the order
 - C. Notify the care team about the safety concern and request discharge planning review for appropriate support or placement
 - D. Advise the client to ask neighbors for help after returning home
7. A practical nurse is preparing a client for transfer to a rehabilitation facility. The client's spouse is at the bedside and says, "I need a copy of the medication list and discharge plan so I can help at home." The client is alert and says, "Please give my spouse whatever is needed." What should the nurse do next?
 - A. Provide the information needed for continuity of care because the client gave permission
 - B. Refuse to share any information until the spouse shows legal power of attorney
 - C. Ask the spouse to wait until after transfer because discharge information cannot be shared on the unit
 - D. Share only the medication names, but not the discharge plan, because treatment details are always private

8. An adult client admitted with sepsis is now confused and unable to make treatment decisions. The chart contains an advance directive stating the client does not want CPR if cardiac arrest occurs, but there is no provider prescription addressing resuscitation status. The client's partner asks, "Doesn't the advance directive mean you won't do CPR?" Which response by the nurse is most appropriate?

- A.** Yes; the advance directive alone is enough, so the nurse should withhold CPR if the client arrests.
- B.** No; an advance directive is only used after the family gives permission for it to be followed.
- C.** The advance directive should be reviewed with the provider because a current prescription is needed to direct resuscitation status.
- D.** The nurse should ask the partner to sign a consent form confirming the client's wishes about CPR.

9. A nurse is getting a client ready for an elective amniocentesis. The client reads the consent form and says, "I understand why the test is being done, but no one told me there was a chance it could harm the pregnancy." Which action should the nurse take?

- A.** Ask the client to sign the form because the procedure is already scheduled
- B.** Explain the possible complications of the procedure to the client
- C.** Hold the procedure and notify the provider that the client needs more information before consenting
- D.** Have the client's partner sign the consent form instead

10. A client is preparing for discharge after hospitalization for a pelvic fracture treated without surgery. The client will go home with a walker and instructions to avoid climbing stairs without assistance. During discharge planning, the client says, "My bedroom is upstairs, and I don't think I can get up there yet." Which action by the nurse is most appropriate?

- A.** Ask the client to practice stair climbing again before discharge
- B.** Suggest that the client sleep in a recliner until the fracture heals
- C.** Notify the case manager or provider team to address home safety needs before discharge
- D.** Teach the client to limit fluid intake so trips upstairs are less frequent

11. A long-term care unit wants to reduce resident falls during transfers from bed to chair. Before starting any new intervention, the practical nurse is asked to help the team choose a baseline measure. Which data would be most useful to collect first?

- A.** The number of residents currently receiving physical therapy
- B.** The percentage of staff who say transfers feel rushed

- C.** The number of falls during bed-to-chair transfers by shift and location
- D.** The average length of time residents have lived in the facility

12. A practical nurse is caring for a client with a new below-knee amputation. During the afternoon assessment, the nurse notes that the client has refused physical therapy twice, says, "I don't want to look at my leg," and has eaten only a few bites of lunch. Which team member should the nurse update first to address the client's most immediate psychosocial need?

- A.** Dietitian
- B.** Occupational therapist
- C.** Social worker
- D.** Unlicensed assistive personnel

13. A practical nurse is preparing to call the provider about a client with chronic kidney disease whose morning laboratory results show potassium 6.1 mEq/L. Which additional information is most important for the nurse to gather before making the report?

- A.** The client's most recent cardiac rhythm and electrocardiogram changes
- B.** How many visitors the client had overnight
- C.** The date of the client's last bowel movement
- D.** Whether the client prefers morning or evening medications

14. A practical nurse is caring for a client who speaks limited English. The client's adult daughter says, "I can translate everything. Just explain the surgery to me, and I'll tell my mother so she can sign." The client is awake and looking uncertain. Which action by the nurse is most appropriate?

- A.** Ask the daughter to translate because family members know the client best
- B.** Proceed with teaching as long as the daughter agrees to explain it accurately
- C.** Request a qualified interpreter and delay further teaching until accurate communication can occur
- D.** Have the client sign the form first and provide additional explanation after surgery

15. A practical nurse accidentally administers the wrong dose of a prescribed antihypertensive medication to a client. The client remains alert and has stable vital signs. The nurse notifies the provider and charge nurse, completes required incident reporting, and continues close monitoring. When documenting in the medical record, which entry is most appropriate?

- A.** Document the medication given, the client's response, assessments performed, and the provider notification, but do not mention the incident report

- B.** Document that an incident report was completed so the chart reflects all legal steps taken
- C.** Wait until the end of the shift to document, so the full outcome is known before entering the event
- D.** Record only the client's stable condition to avoid creating a record that could be used against staff

16. A practical nurse in a medical-surgical unit is caring for an alert 76-year-old client who quietly says, "My daughter leaves me alone for days and takes my pain pills." The client has no acute injuries and says, "Please don't tell anyone. I just wanted to talk." What is the nurse's best action?

- A.** Keep the information confidential because the client requested privacy
- B.** Ask the daughter for her explanation before taking further action
- C.** Report the suspected abuse according to facility policy and state law
- D.** Wait to report until the client agrees in writing

17. A practical nurse in a pediatric clinic notes that a 5-year-old child has a patterned bruise on the lower back. The caregiver states, "He fell off his bike," but the injury pattern does not match the explanation. The nurse immediately ensures the child is safe and notifies the RN. Which additional action is most appropriate to support mandatory reporting?

- A.** Document the caregiver's explanation as abuse because the story seems false
- B.** Record objective findings, including the size, location, and appearance of the bruise and the exact statements made
- C.** Question the child repeatedly until the child gives a clear explanation
- D.** Call the caregiver later to see whether the story changes

18. A practical nurse is supervising an assistive personnel (AP) on a medical unit. Which activity is appropriate for the nurse to assign to the AP?

- A.** Reinforce teaching about how to use an incentive spirometer
- B.** Measure and record the weight of a stable client before breakfast
- C.** Assess a client's pain 30 minutes after analgesic administration
- D.** Decide whether a client is ready for discharge after meeting goals

19. An assistive personnel (AP) tells the practical nurse, "I emptied 900 mL from the client's Foley bag in 2 hours, and the urine looks much darker than it did this morning." Which response by the nurse is best?

- A.** "Document the amount and color, and I will assess the client now."

B. "Offer the client more oral fluids the next time you enter the room."

C. "Clamp the tubing for 30 minutes and then recheck the output."

D. "This can happen after the bag gets full, so continue routine care."

20. A practical nurse in a clinic helps with referrals after hospitalization. Which client situation most strongly indicates a need for nursing advocacy to reduce inequitable access to care?

- A.** A client with private insurance asks whether a follow-up visit can be moved to a different day because of a vacation
- B.** A client says the referred specialty office requires an online patient portal, but the client has no computer, no email address, and no one to help create an account
- C.** A client wants to delay outpatient physical therapy until feeling more rested at home
- D.** A client reports mild anxiety about meeting a new provider after discharge

21. A practical nurse in a community mental health clinic is helping a client who was prescribed a new antidepressant. The client says, "I can read English a little, but I understand health information best in Spanish. I usually just nod because I do not want to hold people up." Which action by the nurse best promotes equitable access to care?

- A.** Ask the client's bilingual teenage son to explain the medication instructions at home
- B.** Provide the teaching with a qualified medical interpreter and offer written instructions in Spanish if available
- C.** Speak more slowly and loudly in English while repeating the most important points
- D.** Give the client the pharmacy phone number and suggest calling later with any questions

22. A nurse in a long-term care facility is preparing to administer a scheduled oral medication to a client with mild dementia. The client smiles and says, "I'm in room 214, and everyone here knows me." Which action by the nurse is best to verify the client's identity before giving the medication?

- A.** Ask the client to state their full name and date of birth, then compare that information with the medication record and identification band
- B.** Confirm the room number and ask a nursing assistant whether this is the correct client
- C.** Check the name posted on the client's door and compare it with the medication cup label
- D.** Recognize the client from previous shifts and administer the medication as scheduled

23. A nurse in a long-term care facility is preparing to help a client shower using a shower chair. While setting up the equipment, the nurse notices one of the chair's wheel locks does not stay engaged. Which action is best?

- A.** Use the chair anyway and ask an assistive personnel to hold it steady during the shower
- B.** Place the chair against the wall to keep it from moving during use
- C.** Remove the chair from service and obtain equipment that is functioning properly
- D.** Lock the other wheels and transfer the client carefully while avoiding sudden movement

24. A nurse is preparing to administer a dose of heparin drawn from a prefilled syringe. During the safety check, the client says, "I once had a severe allergic reaction after using pork products in a medication." What is the nurse's priority action?

- A.** Administer the medication because heparin is not given by mouth
- B.** Hold the medication and verify the allergy and product source before administration
- C.** Give the medication slowly while observing for flushing or itching
- D.** Ask another nurse to administer the medication to confirm the reaction is not anxiety-related

25. A practical nurse on a skilled nursing unit notices that a red biohazard bag filled with used dressings has been left on the floor in the corridor outside a client's room. Residents and staff frequently use this hallway. Which action should the nurse take first?

- A.** Move the bag to the designated disposal area and then report the hazard
- B.** Wait for housekeeping staff to remove the bag during rounds
- C.** Place a wet-floor sign near the bag to warn others
- D.** Ask the client in the room whether anyone was injured

26. A client with postoperative confusion repeatedly grabs at a newly placed nasogastric tube during short periods of agitation. When the nurse speaks calmly and offers the client a washcloth to hold, the client stops reaching for the tube for several minutes. Which action should the nurse take next to follow least-restrictive restraint principles?

- A.** Apply bilateral wrist restraints now because the client has already touched the tube
- B.** Ask the provider for a restraint order to prevent any chance of tube removal
- C.** Continue close observation and use simple nonrestraint measures such as reorientation and distraction
- D.** Secure the client more tightly in bed with all side rails raised

27. A nurse removes a vial from an automated dispensing cabinet for a client's evening medication and notices before preparation that the vial strength is higher than the provider's prescribed dose. The medication has not been given, and the client has no adverse effects. How should the nurse classify this event?

- A.** An expected variance that does not require follow-up because no medication was administered
- B.** A near miss because the error was caught before reaching the client
- C.** A sentinel event because the medication could have caused serious harm
- D.** An adverse event because the wrong medication strength was available

28. A nurse is assisting a client from the toilet to a wheelchair. The client can follow directions and bear weight briefly but has poor balance and becomes fatigued during transfers. Which transfer device is the safest choice?

- A.** Transfer belt
- B.** Sit-to-stand lift
- C.** Transfer board
- D.** Full-body mechanical lift

29. A practical nurse is emptying a client's urinary drainage bag. After removing the drain spout cap, the nurse notices the tip of the spout accidentally touched the outside of the graduate container. Which action should the nurse take next?

- A.** Wipe the spout with an alcohol pad and continue draining the urine
- B.** Replace the drainage bag and tubing with a new closed system using sterile technique
- C.** Drain the urine as planned, then document the break in technique
- D.** Rinse the graduate container and use a different container to finish emptying the bag

30. A nurse dons a gown, gloves, mask, and face shield before caring for a client on combined contact and droplet precautions. Which action is most appropriate when removing the PPE?

- A.** Remove the face shield in the hallway after leaving the room
- B.** Remove the gloves first because they are usually the most contaminated
- C.** Remove the mask first to make breathing easier during the rest of removal
- D.** Remove all PPE at the bedside before performing hand hygiene

- 31.** While preparing to remove a soiled isolation gown, a nurse should loosen the neck and waist ties and pull the gown away from the body by touching which area?
- A.** The front of the gown
 - B.** The sleeves near the wrists
 - C.** The inside surface of the gown only
 - D.** Any area as long as gloves are still on
- 32.** A practical nurse is preparing to irrigate a wound using sterile technique. After opening the sterile tray, the nurse needs to place a bottle of sterile normal saline onto the field. Which action is most appropriate?
- A.** Place the bottle cap face down on the bedside table while pouring
 - B.** Hold the bottle 1 to 2 inches above the sterile container and pour without touching the field
 - C.** Rest the bottle lip on the inside edge of the sterile basin to avoid spilling
 - D.** Pour a small amount of solution into the basin, then return the remaining solution to the bottle
- 33.** A practical nurse is caring for a client who is admitted with suspected bacterial meningitis. Which type of isolation precautions should the nurse implement first?
- A.** Droplet precautions
 - B.** Airborne precautions
 - C.** Protective environment precautions
 - D.** Contact precautions
- 34.** A practical nurse is preparing to assist a client with shaving at the bedside. The client has no isolation orders but has several acne lesions on the face, and the nurse expects there could be minor nicks from the razor. Which action best reflects standard precautions?
- A.** Wear clean gloves while providing the shave and perform hand hygiene after glove removal
 - B.** Use a mask because all personal care activities require droplet protection
 - C.** Skip gloves because the client has no known infection
 - D.** Place the client on contact precautions until the shave is completed
- 35.** A nurse is collecting data at a routine prenatal visit for a client who is 29 weeks pregnant. Which finding should the nurse report to the provider immediately?
- A.** Blood pressure 148/92 mm Hg
 - B.** Mild ankle edema at the end of the day
 - C.** Urinary frequency
 - D.** Heartburn after meals
- 36.** A client in labor has an external fetal monitor in place. The fetal heart rate baseline is 138/min with moderate variability. The nurse notes abrupt drops to 90/min that vary in timing with contractions and return quickly to baseline. Which action is most appropriate?
- A.** Reposition the client and perform a vaginal examination for possible cord compression
 - B.** Increase the oxytocin infusion to improve uteroplacental perfusion
 - C.** Prepare the client for immediate operative birth due to fetal distress
 - D.** Document this as an expected early-deceleration pattern from fetal head compression
- 37.** A nurse is reinforcing teaching for a client who is 4 days postpartum after an uncomplicated vaginal birth. The client says, "I've been feeling tired and overwhelmed, and yesterday I cried a little for no clear reason, but today I feel better." Which response by the nurse is most appropriate?
- A.** These feelings are common in the first several days after birth, but call your provider if they worsen, last longer than about 2 weeks, or you have thoughts of harming yourself or your baby.
 - B.** This means you are developing postpartum depression and should begin limiting time alone with your baby.
 - C.** This is caused by blood loss after birth and will go away once your iron level returns to normal.
 - D.** You should avoid talking about these feelings so you can focus on bonding with your baby.
- 38.** A nurse is assessing an 8-month-old infant during a well-child visit. Which behavior is most consistent with expected development at this age?
- A.** Sits steadily without support and transfers a toy from one hand to the other
 - B.** Walks up steps with one hand held
 - C.** Uses a spoon well without spilling
 - D.** Draws a person with three body parts
- 39.** A practical nurse is reinforcing teaching with the family of an 82-year-old client who moved in with relatives 1 month ago after no longer being able to live alone safely. The family says, "He gets upset when we rearrange his things and insists on following the same routine every day." Which response by the nurse is most appropriate?
- A.** "This behavior shows that he is developing severe confusion and needs immediate memory testing."
 - B.** "A strong need for familiar belongings and routines can be a normal response to a major life change."
 - C.** "You should avoid letting him make choices until he adjusts to the new home."
 - D.** "It is best to discourage discussion about his old home so he can move on more quickly."

40. A nurse is reinforcing teaching for the parent of a child who is scheduled to receive the measles, mumps, and rubella (MMR) vaccine. Which information should the nurse include?

- A.** The vaccine should be avoided if the child is currently taking high-dose corticosteroids.
- B.** A mild stomachache after the vaccine indicates a severe adverse reaction.
- C.** The child will need antibiotic prophylaxis after receiving the vaccine.
- D.** The vaccine provides immediate immunity within a few hours.

41. A nurse is reinforcing teaching with the parent of a 7-year-old child who is starting to spend more time in team activities and clubs. Which behavior should the nurse identify as an expected developmental characteristic of this age?

- A.** Prefers parallel play and has little interest in rules
- B.** Thinks mainly in abstract terms about future goals
- C.** Enjoys cooperative play and wants approval from peers and adults
- D.** Shows no concern about success in school or completing tasks

42. A nurse is providing instructions about infant immunizations to a client. During the conversation, the client repeatedly looks away, fidgets with the blanket, and says, "I don't want to ask a dumb question." Which response by the nurse is best to reduce this communication barrier?

- A.** "The instructions are simple, so just follow the schedule I gave you."
- B.** "Save your questions for the provider, since immunizations can be complicated."
- C.** "Many people have questions about vaccines. What concerns do you have?"
- D.** "If you are unsure later, read the handout again at home."

43. A nurse is teaching a client who was prescribed a new seizure medication. The client says, "I can't read these instructions because the words keep moving around," and squints at the medication label. Which action by the nurse is best?

- A.** Ask the client to sign the teaching form and review the instructions later at home
- B.** Continue the teaching session by reading the handout aloud at a normal pace
- C.** Assess the client's vision needs and provide teaching materials in a format the client can see clearly
- D.** Speak more loudly so the client can better understand the medication instructions

44. A client who recently began hemodialysis tells the practical nurse, "I feel like my whole life revolves around this machine now." Which response by the nurse best provides emotional support?

- A.** You will get used to the dialysis schedule after a few weeks.
- B.** At least dialysis is helping keep you alive and stable.
- C.** It sounds as if this treatment has changed a lot for you. Tell me more about what has been hardest.
- D.** You should focus on the positive and try not to think about the machine so much.

45. A client who was told today that they will need a below-knee amputation says to the nurse, "I can't deal with this right now. I need to know what I should focus on first." Which response by the nurse best promotes effective coping?

- A.** "Try not to think about the surgery until the provider explains everything again."
- B.** "Let's break this into smaller steps and talk about what concerns you most right now."
- C.** "You need to stay positive so you do not make this harder on yourself."
- D.** "Many clients adjust well after amputation, so you should be less worried."

46. A nurse in the emergency department is caring for a client who was informed that a visitor would not be allowed into the treatment area. The client is now pacing near the nurses' station, speaking in a low, tense voice, and repeatedly saying, "You people always push me too far." Which action should the nurse take first?

- A.** Tell the client that threatening language will not be tolerated
- B.** Move the client to a quieter area and speak calmly with the client
- C.** Ask security to stand nearby while the nurse continues charting
- D.** Explain the visitor policy in greater detail

47. A client is 24 hours into an inpatient stay after abruptly stopping daily use of a sedative-hypnotic medication. The client is anxious, reports severe insomnia, and has a coarse hand tremor. Which finding should alert the nurse to the greatest risk for a life-threatening withdrawal complication?

- A.** Dilated pupils and yawning
- B.** Generalized seizure activity
- C.** Muscle aches and abdominal cramping
- D.** Depressed mood and social withdrawal

48. A client with moderate intellectual disability becomes upset in the clinic waiting area when the appointment is delayed. The client begins rocking, covering both ears, and repeating, "Too loud, too loud." Which action should the nurse take first?

- A.** Move the client to a quieter area and speak in short, calm sentences
- B.** Firmly tell the client to stop the behavior because others are waiting
- C.** Ask several questions to determine exactly why the client is upset
- D.** Explain that the delay is unavoidable and the client must remain seated

49. A client sits quietly and avoids eye contact after being told that further testing is needed. When the nurse asks how the client is feeling, the client shrugs and says, "It doesn't matter." Which response by the nurse is most therapeutic?

- A.** You should try to stay positive until the test results are back.
- B.** Why do you think it doesn't matter?
- C.** It sounds as though this news may feel overwhelming right now.
- D.** There is no reason to worry until you know more.

50. A client with chronic back pain tells the nurse, "When my stress builds up, I cancel plans, stop doing anything enjoyable, and just sit thinking about everything that is wrong." Which nursing suggestion is most appropriate to promote effective stress management?

- A.** Set aside a short time each day for a pleasant activity, even if stress is still present
- B.** Avoid discussing the stressors so the client does not become more upset
- C.** Wait to restart activities until all major stressors are resolved
- D.** Focus on identifying every possible problem before trying any coping strategy

51. A client admitted for treatment of pneumonia tells the practical nurse, "My belief requires my body to remain covered as much as possible. I get very anxious when too many people come in and expose me during care." Which action by the nurse is most appropriate?

- A.** Explain that hospital routines must take priority over personal beliefs during treatment
- B.** Ask the client to describe specific preferences and plan care to maintain privacy whenever possible
- C.** Tell the client that only family members can help with spiritual concerns
- D.** Delay hygiene and assessment until the client is less anxious about the hospitalization

52. A nurse is preparing to teach a client how to perform daily wound care at home. The client says, "Before making health decisions, I always discuss them with the head of my family." Which action by the nurse is most appropriate?

- A.** Proceed with the teaching immediately and remind the client that self-care decisions must be made independently
- B.** Ask the client whom they would like included in the teaching and, with permission, involve that person in the discussion
- C.** Delay all teaching until the family arrives, even if the client is ready to learn now
- D.** Explain that the health care team cannot include family in education because of confidentiality rules

53. A client who identifies as gender nonconforming is scheduled for urinary catheterization. Before the procedure, the client says, "Please explain exactly what you need to do and what words you will use. I get very uncomfortable during exams like this." Which nursing response is best?

- A.** "I understand. Tell me what name for body parts feels most respectful to you, and I will explain each step before I begin."
- B.** "The procedure is routine, so it will be easier if we just get it done quickly."
- C.** "I can ask another nurse to do it if this topic makes you uncomfortable."
- D.** "We have to use standard medical terms, but I will try to be gentle."

54. A client with newly diagnosed bilateral hearing loss tells the nurse, "At church and family dinners, I miss so much that I stopped going. Then everyone thinks I'm being rude." Which nursing response is best?

- A.** "You should avoid large gatherings until your hearing improves."
- B.** "Tell your family to speak louder so you do not have to work so hard to listen."
- C.** "It sounds upsetting to feel left out. Let's talk about ways to stay involved, such as choosing quieter seating and asking others to face you when they speak."
- D.** "If you ignore the problem, people will eventually stop bringing attention to it."

55. A client receiving end-of-life care tells the nurse, "I haven't prayed in years, but now I keep wondering whether God is punishing me." Which response by the nurse is most therapeutic?

- A.** "You should try not to think that way because it will only make you more upset."
- B.** "Many people feel this way near the end of life. Would you like to talk more about those feelings or speak with someone from your faith tradition?"

- C. "God is not punishing you, and you need to stay positive."
- D. "I will ask your family what religion you used to practice so we can decide what to do next."

56. A nurse is caring for a client who receives nocturnal enteral feedings through a gastrostomy tube. In the morning, the nurse notes the client has been lying flat since the feeding ended 30 minutes ago. Which action is most appropriate?

- A. Place the client in high-Fowler's position for at least 30 to 60 minutes
- B. Flush the tube with 30 mL of air to clear residual formula
- C. Clamp the tube and leave the client flat to promote rest
- D. Remove the dressing from the site and leave the tube open to air

57. A nurse is caring for a client who is weak after several days of illness and needs to be turned in bed. Which action is best to promote proper body alignment and reduce pressure during repositioning?

- A. Place a pillow directly under the client's knee joints while the client is supine
- B. Use a drawsheet to turn the client and place supportive pillows to maintain alignment
- C. Pull the client across the bed by the arms to avoid friction from the linens
- D. Position the client flat on the back without supportive devices after each turn

58. A nurse is reinforcing nutrition teaching for a client who has type 2 diabetes mellitus and asks how to help keep blood glucose more stable throughout the day. Which client choice shows an understanding of the teaching?

- A. Skipping lunch to avoid a rise in blood sugar
- B. Eating meals at regular times and including a consistent amount of carbohydrates
- C. Drinking fruit juice instead of eating breakfast
- D. Choosing candy for quick energy between meals every day

59. A client with a new left-arm cast reports itching and mild discomfort under the cast. The nurse has verified that circulation, movement, and sensation are intact. Which nonpharmacologic action is most appropriate?

- A. Use a hair dryer on a cool setting to blow air into the cast
- B. Insert a cotton-tipped applicator under the cast to scratch the area
- C. Apply a heating pad over the cast for 20 minutes
- D. Encourage the client to keep the arm dependent to reduce pressure

60. A nurse is assisting a client who has hemiparesis following a recent stroke with using a bedside commode. Which action by the nurse best promotes safety and independence during this activity of daily living?

- A. Place the commode on the client's weaker side so the stronger side can turn away from it
- B. Encourage the client to pull on the nurse's neck for support during the transfer
- C. Allow the client to push up with the stronger leg and pivot toward the stronger side
- D. Complete the entire transfer for the client to reduce effort and prevent fatigue

61. A nurse is caring for a client who has a new prescription for a timed-voiding program to manage functional urinary incontinence. Which action should the nurse take?

- A. Encourage the client to wait until the urge to void is strong before using the toilet
- B. Assist the client to the toilet at regularly scheduled intervals during the day
- C. Limit oral fluids to reduce the number of incontinence episodes
- D. Insert an indwelling urinary catheter until bladder control improves

62. A nurse is caring for a hospitalized client who says, "I slept almost all day, and now I don't feel sleepy at bedtime." Which nursing action is best to help the client rest during the night?

- A. Offer the client a large snack and a caffeinated drink before bed
- B. Encourage the client to stay awake during the day and participate in daytime activities
- C. Keep the television on to provide background noise until the client falls asleep
- D. Suggest going to bed earlier the next evening whether or not the client feels tired

63. A practical nurse is reinforcing discharge teaching for a client with diabetic retinopathy who has trouble reading medication labels. Which instruction is most appropriate?

- A. Store all medications together in one container so they are easier to find
- B. Ask the pharmacy to place large-print labels on prescription bottles
- C. Memorize each medication by bottle shape and size alone
- D. Keep medications in a dimly lit cabinet to reduce eye strain

- 64.** A client is on strict intake and output after surgery. By 2:00 p.m., the practical nurse notes the following: 240 mL of IV fluid infused, 8 oz of milk consumed, 1/2 cup of soup eaten, and 150 mL of urine output. Which amount should the nurse record as the client's total intake by 2:00 p.m.?
- A. 480 mL
 - B. 600 mL
 - C. 630 mL
 - D. 750 mL
- 65.** A practical nurse is caring for a client who has remained mostly supine for 4 days after a pelvic fracture. Which nursing action is most appropriate to help prevent skin breakdown over the sacrum?
- A. Massage the reddened sacral area at each shift
 - B. Reposition the client at regular intervals and keep the skin clean and dry
 - C. Place a donut-shaped ring under the sacrum
 - D. Limit fluid intake to reduce the need for movement during linen changes
- 66.** A nurse is administering morning medications in a long-term care facility. One client says, "I take my heart pill after breakfast, not before." The MAR lists the medication as due now, and the nurse has not yet given it. Which action is most appropriate?
- A. Tell the client the MAR is the legal record and the medication must be taken now
 - B. Hold the medication until later in the shift without further review
 - C. Review the prescription and facility schedule, then clarify the timing before administering the medication
 - D. Ask another nurse to give the medication so the medication pass is not delayed
- 67.** A practical nurse is admitting a client who brought several medication bottles from home. The client says, "I take these every day, and I also use an inhaler when I feel short of breath." The inhaler is not listed on the admission medication list, and one bottle has a different dose on its label than the dose documented in the chart. Which action should the nurse take first?
- A. Document the medications exactly as they appear in the chart and update the list after the first scheduled doses are given
 - B. Hold all home medications until discharge teaching is completed
 - C. Compare the medication bottles and the client's report with the admission list, then clarify discrepancies with the provider
 - D. Ask the client which medications seem most important and record only those
- 68.** A nurse is preparing to give an analgesic to a patient who is receiving the medication through a gastrostomy tube. Which route of administration should the nurse document on the MAR?
- A. Oral
 - B. Enteral
 - C. Intramuscular
 - D. Topical
- 69.** A client with disseminated intravascular coagulation (DIC) is oozing blood from venipuncture sites. Laboratory results show a fibrinogen level of 70 mg/dL. Which blood product should the nurse anticipate administering to specifically raise the client's fibrinogen level?
- A. Cryoprecipitate
 - B. Packed red blood cells
 - C. Platelets
 - D. Albumin
- 70.** A nurse is preparing to administer 840 mL of IV solution over 7 hours using tubing calibrated at 15 gtt/mL. How many drops per minute should the nurse regulate?
- A. 25 gtt/min
 - B. 30 gtt/min
 - C. 35 gtt/min
 - D. 40 gtt/min
- 71.** A practical nurse is reviewing the medication administration record before giving a prescribed controlled opioid to a client. The MAR shows the dose was administered 1 hour ago by another nurse, but the client reports continued severe pain and asks for more medication now. Which action should the practical nurse take first?
- A. Administer the medication because the client still reports severe pain
 - B. Hold the medication and verify the timing and prescription before giving another dose
 - C. Ask the client to wait until the next shift to request more pain medication
 - D. Document the client's pain rating and waste the medication with another nurse
- 72.** A client with benign prostatic hyperplasia has been taking tamsulosin for 2 weeks. Which finding best indicates the medication is producing the intended therapeutic response?
- A. The client reports a stronger urine stream and less difficulty starting urination.
 - B. The client reports mild dizziness when standing up quickly.
 - C. The client has a blood pressure of 104/64 mm Hg.

D. The client reports decreased nocturia after limiting evening fluids.

73. A client began taking allopurinol 1 week ago for gout prevention. Which new finding should the nurse recognize as the most important adverse response to report to the provider immediately?

- A.** Mild drowsiness in the evening
- B.** A diffuse rash with blistering of the skin
- C.** Increased urination during the day
- D.** Temporary decrease in appetite

74. A patient takes metformin for type 2 diabetes and is scheduled for a computed tomography (CT) scan with iodinated contrast media this afternoon. Which action by the nurse is most appropriate?

- A.** Administer the metformin as scheduled because contrast media does not affect this medication
- B.** Hold the metformin and clarify the order with the prescribing provider before the procedure
- C.** Give the metformin with food immediately before the scan to reduce stomach upset
- D.** Administer half the usual metformin dose after the scan is completed

75. A client with rheumatoid arthritis is prescribed methotrexate 15 mg by mouth once weekly. Before administering the scheduled dose, the nurse reviews the laboratory results and notes a platelet count of 62,000/mm³. Which action is most appropriate?

- A.** Administer the medication and monitor for therapeutic response
- B.** Hold the medication and notify the provider
- C.** Give the medication with food to reduce gastrointestinal upset
- D.** Request that the dose be changed to a daily schedule

76. A nurse is teaching a client who has a new prescription for oral alendronate to treat osteoporosis. Which instruction should the nurse include?

- A.** Take the medication at bedtime with a snack.
- B.** Remain upright for at least 30 minutes after taking the medication.
- C.** Crush the tablet and mix it with applesauce if swallowing is difficult.
- D.** Take the medication with milk to improve absorption.

77. A nurse is preparing to hang an IV infusion of amphotericin B. While checking the medication against the MAR, the nurse sees a bag of 5% dextrose in water (D5W) and a bag of 0.9% sodium chloride available at the bedside. The nurse recalls that amphotericin B is not compatible with normal saline. Which action is most appropriate?

- A.** Infuse the amphotericin B with 0.9% sodium chloride because isotonic fluids are preferred for IV medications
- B.** Administer the amphotericin B with D5W as prescribed and avoid mixing it with normal saline
- C.** Mix the amphotericin B with both solutions to dilute the medication and reduce vein irritation
- D.** Delay the dose until the next shift because incompatible fluids can be used if infused slowly

78. A nurse is preparing to administer azithromycin 0.5 g by mouth. The available tablets are labeled 250 mg each. How many tablets should the nurse administer?

- A.** 1 tablet
- B.** 2 tablets
- C.** 2.5 tablets
- D.** 5 tablets

79. A practical nurse is collecting data on a client who is 8 hours postoperative after a bowel resection. Which finding should the nurse identify as the strongest indicator of risk for opioid-induced respiratory depression?

- A.** Respiratory rate of 10/min and difficult to arouse after receiving morphine
- B.** Pain rating of 7 on a 0 to 10 scale before the next scheduled analgesic dose
- C.** Urinary output of 35 mL/hr with dry oral mucous membranes
- D.** Blood pressure of 146/88 mm Hg while turning in bed

80. A nurse is preparing to collect a urine specimen for culture from a client with an indwelling urinary catheter. Which action is most appropriate?

- A.** Disconnect the catheter from the drainage tubing and collect urine in a sterile container
- B.** Obtain urine from the drainage bag using clean technique
- C.** Clamp the tubing briefly, cleanse the sampling port, and withdraw urine with a sterile syringe
- D.** Empty the first urine from the drainage bag, then collect the next specimen that drains into the bag

81. A nurse is caring for a client 2 days after a total thyroidectomy. The client reports tingling in the fingers and around the mouth. Which laboratory value would best support the nurse's suspicion about the cause of these findings?

- A.** Serum calcium 7.1 mg/dL

- B.** Serum sodium 133 mEq/L
C. Serum potassium 5.1 mEq/L
D. Serum magnesium 2.0 mg/dL
- 82.** A client who is 9 weeks pregnant comes to the clinic reporting spotting and mild lower abdominal cramping. Which additional finding should the nurse identify as most concerning for an ectopic pregnancy?
A. Unilateral shoulder pain and dizziness
B. Breast tenderness and fatigue
C. Urinary frequency without burning
D. Mild nausea in the morning
- 83.** A nurse is preparing to obtain a bedside capillary blood glucose level for a client who just returned from the cafeteria. The client used an alcohol-based hand sanitizer and then handled a packet of sugar for coffee. Which action should the nurse take first to help prevent an inaccurate point-of-care result?
A. Wipe away the first drop of blood and use the second drop for testing
B. Wash and dry the client's hands thoroughly before obtaining the sample
C. Massage the finger from base to tip to increase blood flow
D. Use the same finger that was used for the previous glucose check
- 84.** A practical nurse is obtaining routine vital signs on an adult client. The client has a regular pulse, is breathing comfortably, and is talking with the nurse throughout the assessment. Which technique is best for measuring the respiratory rate accurately?
A. Tell the client to take several deep breaths, then count respirations for 15 seconds and multiply by 4
B. Count respirations while appearing to continue the pulse assessment, observing chest movement for a full 30 to 60 seconds
C. Ask the client to breathe normally, then place a hand on the abdomen and count each exhalation for 10 seconds
D. Measure the respiratory rate immediately after the client finishes speaking and walking to the bathroom
- 85.** A client is scheduled for a myelogram with intrathecal contrast later today. Which information obtained during the preprocedure assessment is most important for the nurse to report to the provider?
A. The client drank coffee with cream 4 hours ago
B. The client reports taking metformin for type 2 diabetes
C. The client has a history of seizure disorder
D. The client states feeling anxious about lying still during the test
- 86.** A nurse is collecting a venous blood specimen from an adult client. Shortly after the needle is inserted, the area around the puncture site begins to swell and darken. Which action should the nurse take first?
A. Advance the needle slightly to re-enter the vein
B. Release the tourniquet and remove the needle
C. Apply a warm compress while the needle remains in place
D. Ask the client to open and close the fist
- 87.** A practical nurse is caring for a client who is scheduled for surgery later today. Which action is appropriate when completing preoperative preparation?
A. Ask the client to sign the surgical consent form after the nurse explains the procedure
B. Allow the client to keep dentures in place until arrival in the operating room
C. Verify that allergies are documented and any allergy alert is in place
D. Give the client a full glass of water with preoperative oral medication
- 88.** A client with an enteral feeding tube suddenly begins coughing during a feeding. The nurse notes the client's oxygen saturation has dropped from 97% to 90%. Which action should the nurse take first?
A. Slow the feeding rate and continue to monitor the client
B. Stop the feeding and assess tube placement and the client's respiratory status
C. Flush the tube with 30 mL of water to check patency
D. Place the client flat to reduce abdominal discomfort
- 89.** A nurse is caring for a client who had a below-knee cast applied 4 hours ago for a fractured ankle. The client reports increasing pain that is not relieved by the prescribed analgesic. Which focused assessment should the nurse perform first?
A. Check the toes for color, temperature, capillary refill, movement, and sensation
B. Ask the client to rate the pain again on a 0 to 10 scale
C. Assess the client's understanding of cast care instructions
D. Inspect the skin above the cast for dryness and irritation
- 90.** A client with symptomatic hypoglycemia was treated with 15 g of fast-acting oral carbohydrate. Which finding 15 minutes later best indicates the treatment was effective?
A. Blood glucose increased from 52 mg/dL to 88 mg/dL, and the client reports less shakiness
B. The client says, "I feel hungry," and asks for a snack
C. Blood glucose increased from 52 mg/dL to 60 mg/dL, and the client is drowsy

D. The client is no longer diaphoretic, but blood glucose has not been rechecked

91. A client 24 hours after a long-bone fracture suddenly becomes confused and says, "I can't catch my breath." The nurse notes petechiae on the chest, a temperature of 38.5°C (101.3°F), heart rate 112/min, respiratory rate 30/min, and oxygen saturation 88% on room air. Which complication should the nurse suspect?

- A.** Fat embolism syndrome
- B.** Deep vein thrombosis
- C.** Compartment syndrome
- D.** Acute kidney injury

92. A client with long-standing type 2 diabetes is admitted to a long-term care unit. Which finding should the practical nurse report first?

- A.** A fasting blood glucose of 168 mg/dL
- B.** A new sore on the foot with black tissue at the center
- C.** The client states, "I have been more hungry than usual this week."
- D.** A hemoglobin A1C result of 8.2%

93. A client with bacterial meningitis becomes increasingly restless and difficult to arouse. The nurse notes a widened pulse pressure, irregular respirations, and bradycardia. Which interpretation is most appropriate?

- A.** The client is developing neurogenic shock
- B.** The client is showing signs of increased intracranial pressure
- C.** The client is experiencing a medication-related allergic reaction
- D.** The client is entering the diuretic phase of acute kidney injury

94. A nurse is assessing a client with a tracheostomy 2 days after placement. Which finding should the nurse report to the provider immediately?

- A.** Small amount of thin mucus around the stoma
- B.** Client asks for suctioning after coughing several times
- C.** Bright red blood oozing from the tracheostomy site
- D.** Mild discomfort during routine tracheostomy care

95. A client receiving mechanical ventilation suddenly becomes difficult to ventilate. The ventilator alarm sounds, the client's oxygen saturation drops to 86%, and the nurse cannot pass the suction catheter through the endotracheal tube. Which action should the nurse take first?

- A.** Increase the ventilator's tidal volume setting
- B.** Remove the endotracheal tube and prepare for reintubation

C. Disconnect the client from the ventilator and manually ventilate with 100% oxygen

D. Reposition the client onto the side to improve secretion drainage

96. A client with a temporary transvenous pacemaker is being monitored after a myocardial infarction. The ECG shows the client's intrinsic ventricular rate at 72/min, but pacer spikes appear after each native QRS complex instead of being inhibited. The client reports intermittent palpitations. Which pacemaker malfunction does the nurse suspect?

- A.** Failure to capture
- B.** Failure to sense
- C.** Failure to pace
- D.** Battery depletion

97. A nurse is reinforcing home care teaching for a client with a descending colostomy. Which instruction is most appropriate about when to empty the ostomy pouch?

- A.** Empty the pouch when it is about one-third to one-half full
- B.** Empty the pouch only once each morning before breakfast
- C.** Wait to empty the pouch until the adhesive seal begins to loosen
- D.** Keep the pouch completely full to avoid frequent opening of the system

98. A nurse is caring for a client with a history of heart failure who was admitted for worsening edema. During the shift, the client becomes increasingly restless and says, "I can't catch my breath." Which additional finding should the nurse recognize as the most urgent change in condition?

- A.** Oxygen saturation decreases from 95% to 86%
- B.** Urine output totals 250 mL over 8 hours
- C.** Bilateral ankle edema increases from 2+ to 3+
- D.** The client reports no bowel movement for 2 days

99. A nurse is caring for a client who has been receiving continuous nasogastric tube feedings for 4 days. The client becomes lethargic, reports intense thirst, and has dry mucous membranes. Which laboratory result most strongly supports the nurse's suspicion of hypernatremia related to inadequate free water intake?

- A.** Serum sodium 151 mEq/L
- B.** Serum potassium 3.1 mEq/L
- C.** Serum calcium 8.4 mg/dL
- D.** Serum magnesium 1.6 mg/dL

100. The nurse is caring for a client with end-stage kidney disease who has an arteriovenous fistula for hemodialysis. Which finding indicates the fistula is functioning properly?

- A.** A palpable vibration over the access site
- B.** Cool, pale skin distal to the access
- C.** Redness and tenderness at the insertion site
- D.** Absence of a bruit when auscultating the fistula

Simulation Test 6 — Answer Key

1. A	26. C	51. B	76. B
2. B	27. B	52. B	77. B
3. B	28. B	53. A	78. B
4. A	29. B	54. C	79. A
5. A	30. B	55. B	80. C
6. C	31. C	56. A	81. A
7. A	32. B	57. B	82. A
8. C	33. A	58. B	83. B
9. C	34. A	59. A	84. B
10. C	35. A	60. C	85. C
11. C	36. A	61. B	86. B
12. C	37. A	62. B	87. C
13. A	38. A	63. B	88. B
14. C	39. B	64. B	89. A
15. A	40. A	65. B	90. A
16. C	41. C	66. C	91. A
17. B	42. C	67. C	92. B
18. B	43. C	68. B	93. B
19. A	44. C	69. A	94. C
20. B	45. B	70. B	95. C
21. B	46. B	71. B	96. B
22. A	47. B	72. A	97. A
23. C	48. A	73. B	98. A
24. B	49. C	74. B	99. A
25. A	50. A	75. B	100. A

Simulation Test 7

100 questions

1. A practical nurse on a telemetry unit is notified of these client developments at the same time. Which client should the nurse assess first?
 - A. A client with atrial fibrillation whose heart rate is now 132/min and who reports new dizziness
 - B. A client with stable angina who wants assistance to the bathroom
 - C. A client 1 day after pacemaker insertion who reports mild incisional soreness rated 3/10
 - D. A client with hypertension whose scheduled breakfast tray has not arrived
2. A practical nurse has just finished receiving report in a pediatric unit and must decide which prescription to carry out first. Which action is the priority?
 - A. Administer oral acetaminophen to a child with a temperature of 101.3°F (38.5°C)
 - B. Begin seizure precautions for a child admitted after a febrile seizure yesterday
 - C. Obtain a clean-catch urine specimen from a child with suspected urinary tract infection
 - D. Start a prescribed IV bolus of normal saline for a child with dry mucous membranes, tachycardia, and delayed capillary refill
3. A practical nurse asks an assistive personnel (AP) worker to obtain a pulse oximetry reading on a stable client receiving oxygen by nasal cannula. The AP reports, "The reading is 86%, but the client keeps taking the probe off because it is bothering her." Which action by the practical nurse is best?
 - A. Tell the AP to document the reading and recheck it in 1 hour
 - B. Ask the AP to increase the oxygen flow rate and then repeat the reading
 - C. Assess the client promptly and determine whether the reading is accurate and whether the client is in distress
 - D. Notify the provider immediately that the client's oxygen saturation is low
4. A practical nurse is supervising an assistive personnel (AP) worker in a skilled nursing facility. Which task is most appropriate to delegate to the AP?
 - A. Teach a client how to use a newly prescribed walker
 - B. Apply a warm compress to a stable client using a treatment plan already established by the nurse
 - C. Assess a client's swallowing ability before the first meal after a stroke
 - D. Decide whether a client's PRN pain medication is effective
5. An alert adult client is about to be discharged with a new colostomy. The client says, "I understand the steps when you show me, but I feel overwhelmed and I live alone." Which action by the nurse best advocates for this client?
 - A. Ask the client to sign the discharge papers and review the instructions again quickly
 - B. Arrange for a wound-ostomy nurse or home health referral before discharge
 - C. Tell the client that anxiety is common and confidence will improve at home
 - D. Ask a family member to learn the care even though the client lives alone
6. A nurse is caring for an adult client who is alert, oriented, and scheduled for an elective blood transfusion. The client states, "I changed my mind. I do not want the transfusion." The provider says, "The consent is already signed, and the blood is ready. Start it now." What should the nurse do?
 - A. Begin the transfusion because the signed consent remains valid
 - B. Ask the client's family to persuade the client to accept the transfusion
 - C. Hold the transfusion and notify the provider that the client is refusing treatment
 - D. Delay the transfusion briefly, then start it if the client does not mention refusal again
7. A practical nurse is giving end-of-shift report at the nurses' station. A volunteer delivering flowers is standing nearby and can hear the conversation. Which action should the nurse take?
 - A. Continue the report because the volunteer is part of the facility team
 - B. Lower the voice and finish the report quickly
 - C. Move the report to a more private location
 - D. Ask the volunteer not to repeat what was heard
8. A practical nurse receives a message through the electronic health record from a physical therapist asking for the latest pain medication time and the client's current activity restrictions before a mobility session. What is the most appropriate action by the nurse?
 - A. Provide the requested information through the secure record because it supports the client's care
 - B. Refuse to share any information because only the prescribing provider may release client details
 - C. Call the client's family first to obtain permission for the therapist to review the information
 - D. Print the medication administration record and leave it at the therapy desk for pickup

9. A nurse is caring for an adult client who is alert, oriented, and preparing for discharge after treatment for heart failure. The client says, "I want to complete an advance directive, but I only want my primary provider, my spouse, and the hospital to know what's in it." Which nursing response is most appropriate?

- A.** "Once an advance directive is signed, all adult family members have the right to review it."
- B.** "The document is part of the medical record, and information about it should be shared only with those involved in the client's care as appropriate."
- C.** "Advance directives are public legal documents, so confidentiality does not apply after discharge."
- D.** "The hospital must provide a copy to the client's employer and insurance company for it to be valid."

10. A nurse is preparing a client for an elective cardiac catheterization. The client signed the consent form this morning. Just before transport arrives, the client says, "I only agreed because my daughter kept insisting. I really don't want this test today." Which action should the nurse take first?

- A.** Notify the provider that the client is withdrawing consent
- B.** Ask the daughter to step out and have the client sign a new consent form
- C.** Explain that the procedure can still be done because the form is already signed
- D.** Send the client to the procedure area so the team can discuss the concern there

11. A client is preparing for discharge after hospitalization for newly diagnosed Parkinson disease. The client will return home with a spouse and has a new prescription schedule that includes medications at different times during the day. During teaching, the spouse says, "I'm worried we'll forget the midday dose because we're both busy." Which nursing action is most appropriate to support a safe discharge?

- A.** Ask the provider to change all medications to once-daily dosing
- B.** Recommend using a written medication schedule and setting phone or clock alarms for dosing times
- C.** Tell the spouse to give the missed dose with the next scheduled dose if they forget
- D.** Delay discharge until the spouse can repeat the medication names from memory

12. An urgent care clinic wants to reduce the number of clients who leave without being seen. For 6 weeks, the team tests a quick registration form at check-in. At the end of the test period, 95% of clients used the new form, but the percentage of clients who left without being seen stayed about the same. Which interpretation is most appropriate?

- A.** The project was successful because staff used the new form consistently
- B.** The new form improved an outcome measure even though staff compliance was low
- C.** The change was implemented as planned, but it did not meaningfully improve the target outcome
- D.** The clinic should stop collecting data because the intervention clearly failed

13. A practical nurse is caring for a client admitted with cellulitis who is receiving IV antibiotics. During the afternoon assessment, the nurse notes that the area of redness has spread 3 cm beyond the morning skin marker, the client rates pain as 8/10 instead of 4/10, and the temperature has increased from 37.4°C (99.3°F) to 38.3°C (100.9°F). Which communication to the provider best demonstrates effective interdisciplinary information sharing?

- A.** The client seems worse this afternoon and may need a different antibiotic.
- B.** I'm calling about the client with cellulitis. Since this morning, the redness has extended 3 cm beyond the marked border, pain increased from 4/10 to 8/10, and temperature rose to 38.3°C. The client is receiving IV antibiotics as ordered.
- C.** The cellulitis is still red and painful, and the client has a fever now.
- D.** Could you come look at this client's leg when you get a chance?

14. A practical nurse is preparing to give a newly prescribed medication. The client says, "I changed my mind. I don't want to take that drug until I talk to the provider again." The nurse believes the medication would likely help the client. Which action is most appropriate?

- A.** Tell the client the medication is necessary and administer it as prescribed
- B.** Document the refusal and notify the registered nurse or provider according to facility policy
- C.** Ask a family member to convince the client to take the medication
- D.** Crush the medication and mix it into food so the client still receives treatment

15. A practical nurse is working in a long-term care facility. A resident with mild dementia says, "I don't want my niece making my decisions anymore." The chart includes a durable power of attorney for health care naming the niece, but the nurse is unsure whether it is currently active. Which action by the nurse is most appropriate?

- A.** Immediately stop sharing all information with the niece and remove her from the chart
- B.** Ask the resident to sign a new legal form naming a different decision-maker
- C.** Report the resident's statement to the supervising nurse and review the chart and facility policy to clarify the legal authority in effect
- D.** Continue to take direction from the niece because a durable power of attorney always overrides the resident's wishes

16. A practical nurse in a clinic is caring for a 10-year-old child with a forearm fracture. The caregiver states the child fell while skating. When the nurse speaks with the child alone, the child says, "I got in trouble and was pushed," then asks the nurse not to tell anyone. What is the nurse's best action?

- A.** Keep the disclosure confidential because the child asked the nurse not to tell
- B.** Report the suspected abuse according to facility policy and state law
- C.** Wait to report until the child repeats the statement in front of the caregiver
- D.** Question the caregiver again until the explanation matches the injury

17. A practical nurse in an emergency department cares for an alert 29-year-old client who reports that a spouse caused the client's bruises during an argument at home. The client has decision-making capacity, declines police involvement, and asks for treatment only. Which action is most appropriate by the nurse?

- A.** Respect the client's wishes, document the findings carefully, and follow facility policy regarding available resources and required reporting
- B.** Immediately file a mandatory abuse report in every case of intimate partner violence involving a competent adult
- C.** Refuse discharge until the client agrees to notify law enforcement
- D.** Avoid detailed documentation because it could be used against the spouse later

18. A practical nurse in a clinic asks an assistive personnel (AP) to measure and record the height and weight of several stable adult clients. At the end of the morning, the AP says, "One client's weight is 6 lb higher than last week, but I figured the scale was off and wrote down the same weight as before." Which response by the nurse is most appropriate?

- A.** Thank the AP for trying to avoid confusion and continue with the next client
- B.** Tell the AP to leave the chart as it is because the nurse will assess the client later
- C.** Explain that observations must be reported and documented accurately, then reweigh the client and assess as needed
- D.** Instruct the AP to ask the client whether the weight gain is expected before changing the record

19. A practical nurse is working with an assistive personnel (AP) on a medical-surgical unit. Which task is appropriate for the nurse to assign to the AP?

- A.** Reinforce teaching to a client newly prescribed insulin about rotating injection sites
- B.** Apply a sterile dressing to a stage 2 pressure injury after the nurse assesses the wound
- C.** Offer fluids and document intake for a stable client who needs encouragement to drink
- D.** Evaluate whether a client's pain medicine was effective 30 minutes after administration

20. A practical nurse in a women's health clinic is calling clients who missed screening appointments. One client says, "I didn't come because I am undocumented, and I was afraid the clinic would share my information with immigration authorities." Which nursing action best supports equitable access to care?

- A.** Tell the client that screening is optional and she can return when she feels ready
- B.** Reassure the client that the clinic protects private health information and explain how to schedule a new appointment
- C.** Ask the client to bring a family member to the next visit to confirm the information is accurate
- D.** Advise the client to go to the emergency department instead because identification requirements are usually different

21. A practical nurse in a dialysis center reviews patterns among missed treatments. Which finding most strongly suggests a system-level equity problem that should be brought forward for quality improvement?

- A.** One client missed a treatment after forgetting the appointment time
- B.** A client transferred to another center after moving to a different county

- C.** Clients from one neighborhood miss treatments more often because the center's first available appointment is before the local paratransit service begins running
- D.** Several clients ask for extra blankets during treatment because the room feels cold

22. A nurse enters a semiprivate room to administer a scheduled subcutaneous anticoagulant injection. One client points to the other bed and says, "She's Mrs. Carter—the shot is for her." The nurse sees that the medication is due for Mrs. Evelyn Carter. Which action should the nurse take to verify the correct client before giving the injection?

- A.** Ask the roommate to confirm Mrs. Carter's full name and date of birth
- B.** Check the room assignment and give the injection to the client in the bed labeled Carter
- C.** Ask the intended client to state her full name and date of birth, then compare that information with the identification band and medication record
- D.** Use the client's diagnosis and scheduled medication time as the two identifiers

23. A nurse is preparing to use an incentive spirometer with a postoperative client. The device was previously used by another client, and the mouthpiece has been wiped off but not replaced. What is the nurse's best action?

- A.** Use the device because incentive spirometers are noninvasive equipment
- B.** Attach a new mouthpiece and use the spirometer for the client
- C.** Obtain clean client-specific equipment before use
- D.** Ask the client to rinse the mouth after using the spirometer

24. A nurse is preparing to send a client for a contrast-enhanced imaging study. During the preprocedure interview, the client says, "After contrast dye once, I broke out in hives and felt my chest get tight." The provider's order is active, and the reaction is not documented in the current record. Which action should the nurse take first?

- A.** Administer the contrast as ordered and monitor the client closely during the study
- B.** Document the client's statement, hold the procedure, and notify the provider and radiology team
- C.** Ask the client whether the previous study involved iodine from shellfish
- D.** Send the client to radiology and request that emergency medications be kept nearby

25. A practical nurse enters a client's room and finds a small puddle of clear liquid on the floor beneath an overflowing portable humidifier. The humidifier is plugged into a wall outlet, and the client is walking barefoot toward the bathroom. Which action should the nurse take first?

- A.** Unplug the humidifier and remove the client from the area
- B.** Place a towel over the puddle and finish the assessment
- C.** Ask housekeeping to bring supplies for cleanup
- D.** Teach the client to call for help before getting out of bed

26. A client 1 day after cataract surgery becomes mildly confused in the evening and keeps lifting a hand toward the operative eye. The client is calm, responds to redirection, and stops when the nurse places the client's hands on top of the blanket and explains, "Your eye needs protection while it heals." Which action is most appropriate?

- A.** Apply soft wrist restraints to prevent accidental injury to the eye
- B.** Request a sedative so the client will sleep through the night
- C.** Continue frequent observation and use repeated calm redirection to keep the client's hands away from the eye
- D.** Raise all four side rails to keep the client from moving in bed

27. A nurse preparing to insert an indwelling urinary catheter opens the sterile kit and notices that the sterile drape has a small tear in the wrapper that was not seen before opening. The catheter has not been inserted, and the client has not been harmed. How should the nurse respond?

- A.** Continue with the procedure because the contents appear clean and no contact has occurred
- B.** Replace the kit, obtain intact sterile supplies, and report the near miss according to facility policy
- C.** Use the current kit but add extra antiseptic solution during cleansing
- D.** Ask another nurse to observe the procedure and proceed if the field still looks sterile

28. A nurse is caring for a client who needs to be moved from a wheelchair into a car for discharge. The client is alert, cooperative, and can sit upright, but cannot safely stand or bear weight on either leg. Which transfer device is most appropriate to use?

- A.** Transfer board
- B.** Gait belt
- C.** Sit-to-stand lift
- D.** Standing aid

29. A practical nurse is preparing to suction a client's oropharynx. After opening the sterile catheter kit and putting on sterile gloves, the nurse coughs and feels droplets land on one gloved hand. Which action should the nurse take?

- A.** Wipe the glove with sterile saline and continue the procedure
- B.** Continue because the gloves are still in place and the catheter remains unopened
- C.** Remove the contaminated gloves, set up new sterile supplies as needed, and restart the procedure
- D.** Ask an assistive personnel staff member to hold the catheter while the nurse finishes the suctioning

30. A nurse is wearing gloves to remove a client's used dressing. While pulling off the first glove, the nurse's bare fingers accidentally touch the contaminated outside surface of the second glove. Which action should the nurse take next?

- A.** Continue removing the second glove and perform hand hygiene after both gloves are off
- B.** Remove the second glove carefully, then wash the contaminated fingers with soap and water only
- C.** Stop, perform hand hygiene immediately on the exposed fingers, then remove the remaining glove and perform hand hygiene again
- D.** Put a clean glove over the exposed fingers, remove the second glove, and continue care

31. A practical nurse is setting up supplies for a sterile dressing change. After opening a sterile package, the nurse notices the package's expiration date passed 2 days ago. The wrapper is dry and intact. Which action is best?

- A.** Use the item because the package is unopened and shows no visible damage
- B.** Use the item only if the charge nurse approves it
- C.** Discard the item and obtain a new sterile package
- D.** Place the item on the field but avoid using it near the wound

32. A practical nurse is assisting with a sterile dressing change at the bedside. To create more workspace, the nurse places a sterile drape on an overbed table that has just been wiped with disinfectant and still feels damp. Which action should the nurse take?

- A.** Proceed because the disinfectant reduces microorganisms on the table surface
- B.** Proceed if a second sterile drape is placed over the first one
- C.** Move the supplies to the damp area only after sterile gloves are on
- D.** Dry the table completely and set up a new sterile field

33. A practical nurse is preparing to enter the room of a client who has herpes zoster (shingles) with lesions confined to one area of the trunk and an intact immune system. Which isolation precaution should the nurse expect to use?

- A.** Standard precautions only
- B.** Contact precautions only
- C.** Droplet precautions
- D.** Airborne and contact precautions

34. A practical nurse is preparing to insert a rectal suppository for a client who has no isolation orders. The client has diarrhea, and the nurse expects contact with feces during the procedure. Which action best reflects standard precautions?

- A.** Put on clean gloves before the procedure and perform hand hygiene after glove removal
- B.** Wear a sterile gown and sterile gloves because stool is always considered sterile-site contamination
- C.** Use no gloves if the client's skin is intact and there are no visible open areas
- D.** Apply a surgical mask because standard precautions require a mask for all medication administration

35. A nurse is reinforcing teaching with a client who is 16 weeks pregnant about danger signs that can occur before the next prenatal visit. Which client statement indicates a need for further teaching?

- A.** I should call my provider if I have burning when I urinate.
- B.** I may notice some mild nasal stuffiness during pregnancy.
- C.** If my ankles are a little swollen at the end of the day, I should go to the emergency department right away.
- D.** I should report vaginal bleeding even if it seems light.

36. A client in labor has just received an epidural anesthetic. Five minutes later, the nurse notes maternal blood pressure has decreased from 122/76 mm Hg to 88/50 mm Hg, and the fetal heart rate has a baseline of 170/min. Which action should the nurse take first?

- A.** Increase the oxytocin infusion rate
- B.** Place the client in a lateral position
- C.** Perform a vaginal examination
- D.** Prepare the client for immediate delivery

37. A nurse is reinforcing discharge teaching for a client who is 1 week postpartum after an uncomplicated vaginal birth. Which client statement shows correct understanding of postpartum warning signs?

- A.** I should expect my vaginal bleeding to return to bright red after it has been getting lighter.
- B.** I should call my provider if I develop a temperature of 100.4°F (38°C) or higher.

- C.** I should stay in bed most of the day to help my body recover.
- D.** I should avoid drinking extra fluids until my swelling is gone.

38. A nurse is assessing a 9-month-old infant at a routine visit. Which finding is most consistent with expected development?

- A.** Uses a mature pincer grasp to pick up small objects
- B.** Pulls to a standing position while holding onto furniture
- C.** Speaks in 2-word phrases such as "more juice"
- D.** Hops on one foot without losing balance

39. A practical nurse is reinforcing teaching with the parent of a 6-year-old child who recently started first grade. The parent says, "Since school started, my child wants to finish homework alone and gets very upset when I correct every small mistake." Which response by the nurse is most appropriate?

- A.** This behavior suggests the child is not ready for the demands of school.
- B.** Wanting to do tasks independently is a common part of this stage of development.
- C.** Children this age usually depend more on parents and should not be expected to work alone.
- D.** This reaction most likely means the child is having serious difficulty adjusting to peers.

40. At a workplace wellness event, a practical nurse is reinforcing teaching about blood pressure screening. Which statement by an employee indicates a need for further teaching?

- A.** "High blood pressure can be present even if I feel fine."
- B.** "If one reading is high, I may need follow-up checks before any diagnosis is made."
- C.** "A normal reading today means I will never need my blood pressure checked again."
- D.** "Screening can help find problems early so treatment can start sooner if needed."

41. A nurse is contributing to a teaching plan for the caregiver of a 17-year-old client. Which task is most consistent with normal psychosocial development during adolescence?

- A.** Establishing independence from parents while developing a personal identity
- B.** Learning to trust caregivers to meet basic needs
- C.** Gaining satisfaction from sharing and cooperating in group play
- D.** Adjusting to retirement and reflecting on life achievements

42. A nurse is discussing smoking cessation with a client. The television is on loudly, two visitors are talking at the bedside, and the client keeps asking, "What did you say?" Which action by the nurse is best to reduce the communication barrier?

- A.** Speak more quickly so the teaching can be finished before more interruptions occur
- B.** Turn off the television and move the conversation to a quieter setting
- C.** Ask one of the visitors to repeat the instructions to the client later
- D.** Continue the teaching and provide a printed handout at the end

43. A nurse is preparing to teach a client about care of a newly applied removable cast. As the nurse begins, the client says, "I left my hearing aids at home, so I can only catch a few words when people talk." Which action by the nurse is best?

- A.** Provide the client with a detailed written handout and continue the teaching session as planned
- B.** Face the client, reduce background noise, and speak clearly in a normal tone
- C.** Ask a family member to listen to the teaching and explain it to the client later
- D.** Speak loudly into the client's ear so the instructions are easier to hear

44. A client who had a stroke 3 days ago becomes tearful during morning care and says, "I hate needing help with everything. I feel embarrassed when my daughter sees me like this." Which response by the practical nurse best provides emotional support?

- A.** You should focus on getting stronger instead of feeling embarrassed.
- B.** Many people feel this way after a stroke, so try not to worry about it.
- C.** It sounds like losing independence is very upsetting for you right now.
- D.** Your daughter knows you cannot help it, and she probably does not mind.

45. A client whose partner moved out 1 month ago tells the nurse, "After work I go straight home, scroll on my phone for hours, and ignore the bills piling up on the table. It helps me not think about anything." Which nursing response is most appropriate?

- A.** "Avoiding stressful reminders for a while is usually the healthiest way to cope with a breakup."
- B.** "Tell me which part of the situation feels hardest right now, and we can look at one small step you feel able to take today."
- C.** "You should force yourself to finish all of the bills tonight so the problem does not get worse."

D. "The best way to move on is to stay busy and not talk about your feelings anymore."

46. A nurse in a rehabilitation unit is admitting a client who is becoming increasingly irritable. The client is restless, speaks through clenched teeth, and says, "People need to stop messing with me before I snap." Which nursing action is the priority?

- A.** Tell the client that threatening language is unacceptable and must stop immediately
- B.** Move the client to a quiet area and speak in a calm, nonthreatening manner
- C.** Ask the client to explain why the admission process is causing frustration
- D.** Leave the room to give the client time alone to calm down

47. A client is admitted to the medical unit after a minor injury. During the history, the client says, "I drink every day, but I haven't had anything since this morning." Eight hours later, the client is restless, sweating, and pacing in the room. Which nursing action is most appropriate at this time?

- A.** Limit stimulation and continue routine observation because these findings are expected to resolve on their own
- B.** Assess the client for withdrawal severity and notify the provider of suspected alcohol withdrawal
- C.** Offer strong coffee to improve alertness and decrease irritability
- D.** Explain that the symptoms are caused by anxiety and encourage deep breathing only

48. A client with borderline personality disorder becomes angry after being told that visiting hours are over. The client shouts, "You nurses don't care about me at all!" and throws a cup of water onto the floor. Which response by the nurse is most appropriate?

- A.** "If you keep acting this way, you will lose all visiting privileges tomorrow."
- B.** "I can see that you are upset. We can talk here quietly, but throwing things is not acceptable."
- C.** "You need to go to your room now until you can calm down by yourself."
- D.** "The rules are the rules, and arguing about them will not change anything."

49. A client who was in a house fire 2 days ago tells the nurse, "I keep hearing the alarms and seeing the smoke every time I close my eyes." Which response by the nurse is most therapeutic?

- A.** Try not to think about it, because reliving it will only make you feel worse.
- B.** You are safe now, so these feelings should go away soon.
- C.** That sounds very upsetting. Can you tell me more about what happens when you try to sleep?

D. Many people have experiences like this after a frightening event.

50. A client in an outpatient infusion center becomes upset after being told an appointment was entered for the wrong day. The client begins cursing, slams a bag onto the chair, and says, "This place is useless." Which action by the nurse is most appropriate?

- A.** Tell the client that this behavior is unacceptable and security will be called if it continues
- B.** Ask the client to explain the problem while the nurse stands at a safe distance and uses a calm, low voice
- C.** Touch the client's arm and reassure the client that everything will be fixed right away
- D.** Explain that the scheduling error was made by another department and is not the nurse's responsibility

51. A client admitted for severe burns tells the practical nurse, "My hair must not be cut for religious reasons, but I heard it may need to be shaved around the wound. I am very upset about that." Which response by the nurse is most appropriate?

- A.** Explain that treatment needs are more important than religious practices during hospitalization.
- B.** Tell the client that the provider will decide what is necessary and there is nothing else to discuss.
- C.** Acknowledge the client's concern and notify the nurse in charge so the care team can discuss possible alternatives while protecting wound care needs.
- D.** Reassure the client that no hair will be removed until discharge.

52. A nurse is caring for a client who is scheduled for surgery the next morning. The client says, "In my culture, major family decisions are discussed with my older brother before I agree." The client is alert, oriented, and able to make decisions. Which action by the nurse is most appropriate?

- A.** Tell the client that medical decisions must be made independently without family involvement
- B.** Ask the client whether they would like the older brother included in the discussion before making a decision
- C.** Contact the older brother and ask him to provide consent for the surgery
- D.** Delay all teaching until the older brother arrives, even if the client wants information now

53. A client who identifies as transgender is admitted for treatment of dehydration. During the admission interview, the nurse asks, "What name and pronouns would you like us to use?" The client answers, "Jordan, and they/them." Later, the nurse hears another staff member refer to the client with different pronouns in the hallway. Which action by the nurse is best?

- A.** Ignore the comment because the staff member was not speaking directly to the client

- B.** Privately correct the staff member and reinforce using the client's stated name and pronouns consistently
- C.** Ask the client to remind staff personally each time the wrong pronouns are used
- D.** Wait until the end of the shift and document that some staff were confused

54. A client with new partial hearing loss becomes quiet during a family teaching session. After the family leaves, the client tells the nurse, "I just smiled and nodded because I was tired of everyone correcting me. They probably think I'm confused." Which nursing response is best?

- A.** "You should tell your family not to talk unless they are directly next to you."
- B.** "It sounds frustrating to feel misunderstood. Let's talk about ways to make conversations easier and help your family communicate more clearly with you."
- C.** "If you keep practicing, your hearing will likely return to normal soon."
- D.** "Try not to worry about what your family thinks. They know you are doing your best."

55. A client who is receiving end-of-life care says to the nurse, "My family keeps telling me to stay positive, but I need to talk about the fact that I am dying." Which response by the nurse is most therapeutic?

- A.** You should try to focus on the good days instead of talking about dying.
- B.** Your family means well, so it may be best not to upset them with this topic.
- C.** It sounds like you want an honest conversation about what you are facing. Tell me what has been on your mind.
- D.** Many clients feel this way, but these thoughts usually pass if you keep busy.

56. A nurse is caring for a client who is receiving continuous enteral feeding through a nasogastric tube. During assessment, the nurse notes abdominal distention and the client reports nausea. Which action should the nurse take first?

- A.** Slow the feeding rate and continue to monitor the client
- B.** Stop the feeding and assess tube placement and feeding tolerance
- C.** Administer an antiemetic as prescribed and resume the feeding
- D.** Irrigate the tube with sterile water to improve flow

57. A nurse is reinforcing teaching for a client who will use crutches with a three-point gait after a left ankle injury. Which client action indicates correct understanding of this gait pattern?

- A.** Moves the left foot forward first, then both crutches, then the right foot

- B.** Advances both crutches and the left leg together, then shifts weight to the crutches and steps through with the right leg
- C.** Moves the right crutch forward, then the left foot, then the left crutch
- D.** Advances both legs forward together, then swings both crutches forward

58. A nurse is caring for a client who has persistent vomiting and has been unable to tolerate solid food for 24 hours. Which snack selection is most appropriate when the client is ready to begin oral intake again?

- A.** Dry toast and applesauce
- B.** Sausage and scrambled eggs
- C.** Raw vegetables with ranch dip
- D.** Chili topped with shredded cheese

59. A client who developed a large hematoma at an IV insertion site 15 minutes ago reports tenderness and throbbing in the area. Which nonpharmacologic intervention should the nurse implement first?

- A.** Apply a warm, moist compress to the site
- B.** Elevate the extremity and apply a cold pack
- C.** Massage the area gently to improve circulation
- D.** Encourage active range-of-motion exercises of the limb

60. A nurse is caring for a client with multiple sclerosis who has hand tremors and becomes frustrated while trying to eat breakfast independently. Which intervention is most appropriate to support the client's ability to perform this activity of daily living?

- A.** Feed the client to prevent spills and reduce meal time
- B.** Provide weighted utensils and allow extra time for the meal
- C.** Ask the client to wait until tremors stop before attempting to eat
- D.** Offer only finger foods and discourage use of utensils

61. A nurse is caring for a client who has a spinal cord injury and is on a bowel evacuation program for a reflex bowel. Which action should the nurse take to promote regular elimination?

- A.** Schedule bowel care at the same time each day, preferably after a meal
- B.** Limit fluid intake before bowel care to reduce cramping
- C.** Keep the client on bed rest for 1 hour before the procedure
- D.** Vary the time of bowel care to match the client's daily activity level

62. A nurse is admitting a client who reports needing a bedtime routine at home to fall asleep. Which action by the nurse is best to support the client's rest in the facility?

- A.** Offer the client a large snack and caffeinated soda at bedtime
- B.** Encourage the client to stay awake until very late so sleep will come faster
- C.** Ask about the client's usual bedtime habits and incorporate them when possible
- D.** Schedule routine care activities throughout the night to avoid waking the client in the morning

63. A practical nurse is caring for an older client with a stage 2 pressure injury on the sacrum. The client eats only small amounts at meals and has a serum albumin level below the expected range. Which intervention best supports skin and tissue healing?

- A.** Offer a high-protein snack or supplement between meals as prescribed
- B.** Limit fluids to reduce moisture exposure at the wound site
- C.** Massage the skin around the pressure injury twice daily
- D.** Clean the wound with full-strength hydrogen peroxide at each dressing change

64. A client is being monitored for fluid balance. During the shift, the client drank 150 mL of water, received 240 mL of IV fluids, and had 520 mL of urinary output. The client also had a medium formed stool. Which fluid balance should the practical nurse document at the end of the shift?

- A.** +130 mL
- B.** -130 mL
- C.** +370 mL
- D.** -370 mL

65. A practical nurse is assisting with care for a client who has been immobile in bed for 6 days. Which assessment finding most likely indicates atelectasis related to immobility?

- A.** Crackles heard at the lung bases and a low-grade fever
- B.** Sudden sharp chest pain and hemoptysis
- C.** Absent bowel sounds and abdominal distention
- D.** Warmth, redness, and swelling of one calf

66. A nurse is preparing to administer the first dose of an antibiotic to a client. After verifying the prescription, the nurse enters the room and asks the client to state their full name and date of birth. The client becomes upset and says, "You already know who I am. Why are you asking me again?" Which response by the nurse is best?

- A.** I need to make sure the pharmacy sent the correct antibiotic for this room.
- B.** I ask every client to state two identifiers so I can confirm the medication is being given to the right person.
- C.** The policy requires me to ask, even when I already recognize the client.
- D.** I am checking whether you remember your personal information before I give the medication.

67. A practical nurse is preparing a client's 1000 medications and notices that the MAR lists potassium chloride 20 mEq PO once daily. The provider's prescription entered 2 hours earlier states, "Hold potassium chloride today; serum potassium 5.6 mEq/L." The previous shift did not document giving the dose. What should the nurse do?

- A.** Administer the potassium because it is still listed on the MAR and no dose was documented
- B.** Hold the medication and clarify the discrepancy according to facility procedure before administration
- C.** Ask the client whether potassium was taken at home this morning and give it if the client says no
- D.** Document the dose as refused so the MAR will match the current prescription

68. A nurse is caring for a patient who has painful oral ulcers and should avoid swallowing unnecessary medications. The provider prescribes a medication intended to act locally on the mouth lining. Which route of administration should the nurse use?

- A.** Buccal
- B.** Enteral
- C.** Intramuscular
- D.** Subcutaneous

69. A nurse is preparing to transfuse packed red blood cells to a client with symptomatic anemia. Which action is appropriate when setting up the transfusion?

- A.** Administer the blood through tubing primed with 0.9% sodium chloride
- B.** Add dextrose 5% in water to the blood to prevent hypoglycemia
- C.** Infuse the blood through the same tubing used for total parenteral nutrition
- D.** Warm the blood in a microwave if the client reports feeling cold

70. A nurse needs to infuse 325 mL of IV fluid over 2.5 hours using an infusion pump. What rate in mL/hr should the nurse program?

- A. 120 mL/hr
- B. 130 mL/hr
- C. 150 mL/hr
- D. 162.5 mL/hr

71. A practical nurse is preparing to administer a prescribed controlled opioid to a client who is alert and reporting pain. The nurse removes the medication from the automated dispensing system, but before going to the room, another client in the assignment suddenly requires urgent assistance with shortness of breath. Which action by the nurse is best?

- A. Place the controlled medication in a pocket and give it after assisting with the urgent situation
- B. Ask unlicensed assistive personnel to hold the medication until the nurse returns
- C. Secure the controlled medication according to facility procedure until it can be administered safely
- D. Leave the medication at the client's bedside so it is ready when the nurse comes back

72. A nurse is preparing to transfer a postoperative client from the PACU to the surgical unit. The client is receiving IV opioid analgesia through a patient-controlled analgesia (PCA) pump and reports good pain relief. Which action is most important for the nurse to take before transfer?

- A. Ask the client to rate pain again after arriving on the surgical unit
- B. Verify the PCA settings with another licensed nurse according to agency policy
- C. Encourage the client to use nonpharmacologic pain-relief methods instead of the PCA pump
- D. Document that the client understands the pain scale being used

73. A client with bipolar disorder has been taking lithium for 1 week. Which new finding should the nurse report to the provider immediately as a possible toxic adverse response?

- A. Fine hand tremor and mild thirst
- B. Weight gain of 1 lb (0.45 kg)
- C. Slurred speech and unsteady gait
- D. Occasional nausea after meals

74. A patient takes isosorbide mononitrate for chronic angina. During discharge teaching, the patient says, "I also use sildenafil sometimes." Which response by the nurse is best?

- A. Take the sildenafil only at bedtime so it does not overlap with the nitrate dose.

B. Use both medications only if chest pain is mild and your blood pressure feels normal.

- C. Do not take sildenafil with this nitrate because the combination can cause a dangerous drop in blood pressure.
- D. Skip the nitrate on days you plan to take sildenafil, then restart it the next morning.

75. A client with type 2 diabetes is scheduled to receive metformin 1,000 mg by mouth with breakfast. The nurse reviews the morning laboratory results and notes an estimated glomerular filtration rate (eGFR) of 24 mL/min/ 1.73 m^2 . Which action is most appropriate?

- A. Administer the medication with food as prescribed
- B. Hold the medication and notify the provider
- C. Give half the prescribed dose and document the change
- D. Administer the medication and encourage increased oral fluids

76. A nurse is reinforcing teaching for a client who is starting oral ciprofloxacin for a respiratory infection. Which instruction should the nurse include?

- A. Take the medication with milk to prevent stomach upset.
- B. Stop taking the medication as soon as your fever is gone.
- C. Avoid taking the medication at the same time as antacids.
- D. Limit fluid intake while taking this medication.

77. A nurse is preparing to administer an IV dose of phenytoin through a peripheral line. The client is currently receiving 5% dextrose in water (D5W) at 75 mL/hr. Which action should the nurse take first?

- A. Administer the phenytoin through the line without changes because the maintenance fluid is infusing slowly
- B. Mix the phenytoin into the bag of D5W to dilute the medication before infusion
- C. Stop the D5W infusion and verify compatibility before administering the phenytoin
- D. Flush the line with sterile water and then administer the phenytoin

78. A nurse is preparing to administer potassium chloride 20 mEq by mouth. The oral solution available is labeled 10 mEq per 15 mL. How many milliliters should the nurse administer for one dose?

- A. 15 mL
- B. 20 mL
- C. 30 mL
- D. 45 mL

79. A practical nurse in a rehabilitation unit is collecting data on four clients before the evening meal. Which client should the nurse identify as being at greatest risk for aspiration?

- A.** A client with a recent stroke who coughs while drinking thin liquids
- B.** A client with osteoarthritis who needs help opening food containers
- C.** A client with heart failure who reports decreased appetite
- D.** A client with a healed wrist fracture who has difficulty cutting meat

80. A nurse needs to collect a sputum specimen for culture from a client who has pneumonia. Which instruction should the nurse give the client to obtain the best specimen?

- A.** Rinse the mouth with water, then cough deeply and expectorate sputum into the sterile container
- B.** Collect the specimen immediately after drinking juice to loosen secretions
- C.** Spit saliva from the mouth into the container until it is half full
- D.** Use the specimen that has been sitting in the bedside tissue since the night before

81. A nurse is reviewing laboratory results for a client with cirrhosis who has become increasingly confused and restless. Which laboratory finding best supports the nurse's suspicion that the client's change in mental status is related to hepatic encephalopathy?

- A.** Serum ammonia 92 mcg/dL
- B.** Total bilirubin 2.8 mg/dL
- C.** Aspartate aminotransferase (AST) 76 units/L
- D.** Albumin 3.1 g/dL

82. A client who is 24 weeks pregnant calls the prenatal clinic and says, "For the last several hours, my vision has had flashing spots, and I have a constant pain high on the right side under my ribs." Which response by the nurse is most appropriate?

- A.** Advise the client to rest on the left side and call back if the symptoms continue tomorrow
- B.** Instruct the client to come to the clinic the next day for a routine blood pressure check
- C.** Tell the client to go to the emergency department or labor and delivery unit for immediate evaluation
- D.** Recommend taking an antacid because the pain is most likely related to heartburn

83. A nurse performs a bedside capillary blood glucose test on a client who is receiving an IV infusion of dextrose in the left hand. The client has warm fingers and good circulation. Which action is best to help obtain the most accurate point-of-care result?

- A.** Obtain the sample from a finger on the left hand after stopping the IV for 1 minute
- B.** Use a finger on the right hand for the capillary sample
- C.** Massage the left hand vigorously before obtaining the sample
- D.** Collect blood from the finger with the largest visible callus

84. A practical nurse is obtaining an oral temperature on an adult client. Which situation is most likely to make the reading inaccurate if the temperature is taken immediately?

- A.** The client drank a glass of ice water 5 minutes ago
- B.** The client has been resting quietly in bed for 20 minutes
- C.** The client is wearing a light cotton gown
- D.** The client had blood pressure measured in the opposite arm

85. A client is scheduled for a colonoscopy tomorrow morning. Which statement by the client indicates a need for the nurse to notify the provider before the procedure?

- A.** I finished the bowel preparation as instructed this evening.
- B.** I took my warfarin dose last night like I usually do.
- C.** I signed the consent form after the provider explained the test.
- D.** I arranged for someone to drive me home after the procedure.

86. A nurse obtains a 6-second ECG rhythm strip that shows 9 QRS complexes. Based on this strip, what is the client's heart rate?

- A.** 54/min
- B.** 72/min
- C.** 84/min
- D.** 90/min

87. A practical nurse is caring for a client 8 hours after abdominal surgery. The client has not voided since the indwelling urinary catheter was removed 6 hours ago. Which action is most appropriate for the nurse to take first?

- A.** Encourage the client to drink fluids and attempt to void
- B.** Insert a straight catheter using sterile technique
- C.** Notify the provider immediately about postoperative urinary retention
- D.** Document the finding and recheck in 4 hours

88. A client has a central venous catheter and is receiving total parenteral nutrition. The current infusion bag is nearly empty, but the next TPN bag has not arrived from the pharmacy. Which action should the nurse take?

- A. Hang 0.9% sodium chloride at the same rate until the new TPN bag arrives
- B. Slow the current TPN rate to make the bag last longer
- C. Stop the infusion and flush the catheter with heparin
- D. Hang 10% dextrose in water at the prescribed rate until the new TPN bag arrives

89. A nurse is caring for a client who is 6 hours postpartum after a vaginal birth. The client says, "I suddenly feel very weak and my heart is pounding." Which focused assessment should the nurse perform first?

- A. Inspect the perineal pad and palpate the uterine fundus
- B. Ask the client how many hours she slept last night
- C. Review the client's prenatal laboratory results
- D. Assess the infant's most recent feeding pattern

90. A client with bacterial conjunctivitis began prescribed ophthalmic antibiotic drops 48 hours ago. Which finding best indicates the treatment is effective?

- A. The client reports less eye drainage and redness
- B. The client says the drops sting briefly after administration
- C. The affected eye remains crusted shut each morning
- D. The client asks whether the medication can be stopped now

91. A client with chronic obstructive pulmonary disease becomes suddenly drowsy while receiving oxygen at 6 L/min by simple face mask. Which assessment finding most strongly suggests carbon dioxide retention?

- A. Bounding pulse and flushed skin
- B. Productive cough with thick yellow sputum
- C. Blood pressure 148/88 mm Hg
- D. Bilateral ankle edema

92. A client with chronic liver disease has increasing forgetfulness and is awake but confused about the date. The practical nurse also notes asterixis when the client extends both arms. Which action is most appropriate to take first?

- A. Encourage the client to increase protein intake with the next meal
- B. Report the change in mental status to the registered nurse or provider promptly
- C. Reorient the client and document the findings at the end of the shift
- D. Place the client flat in bed and limit activity for the rest of the day

93. A client with type 1 diabetes reports nausea and abdominal pain. The nurse notes deep, rapid respirations, dry mucous membranes, and a fruity odor to the breath. Which finding is the priority to report immediately because it suggests the client is developing a life-threatening complication?

- A. Blood glucose of 356 mg/dL
- B. Serum potassium of 6.2 mEq/L
- C. Urine positive for ketones
- D. History of missing the morning insulin dose

94. A client has a newly created ileal conduit with a urinary drainage bag attached. Which finding requires the nurse to intervene first?

- A. The stoma is moist and pink-red in color
- B. Urine output has been 20 mL over the last 4 hours
- C. Mucus strands are visible in the urine
- D. The pouch seal needs reinforcement at one edge

95. A client on mechanical ventilation suddenly becomes restless, and the pulse oximeter reading falls from 97% to 89%. The ventilator disconnect alarm is sounding. Which action should the nurse take first?

- A. Reconnect the ventilator tubing to the endotracheal tube
- B. Increase the oxygen flow rate to the ventilator
- C. Administer a prescribed sedative medication
- D. Call the rapid response team

96. A client with a temporary transvenous pacemaker suddenly becomes lightheaded. On the monitor, the nurse sees no visible pacer spikes for several seconds, and the heart rate drops to 32/min. The pacemaker generator is on, and the battery indicator is normal. Which problem should the nurse suspect first?

- A. Failure to capture
- B. Failure to sense
- C. Failure to pace
- D. Pacemaker-induced ventricular tachycardia

97. A nurse is reinforcing teaching for a client with a new ileostomy who asks how to protect the skin around the stoma. Which instruction is most appropriate?

- A. Clean the peristomal skin with warm water, dry it well, and make sure the pouch opening fits closely around the stoma.
- B. Apply a thick layer of lotion to the skin around the stoma before attaching the pouch.
- C. Scrub the skin around the stoma with soap each time the pouch is changed.
- D. Cut the pouch opening larger than the stoma to prevent pressure on the tissue.

98. A nurse is caring for a client 36 hours after an ischemic stroke. The client was alert and able to swallow crushed medications with applesauce earlier in the shift. Which new finding should the nurse recognize as the most urgent change in condition?

- A.** The client has a blood pressure of 148/86 mm Hg
- B.** The client develops a wet-sounding voice and coughs while taking sips of water
- C.** The client reports a dull headache rated 3 on a 0-to-10 scale
- D.** The client sleeps more between care activities than during the previous shift

99. A nurse is caring for a client who has been taking hydrochlorothiazide for hypertension. The client reports new fatigue and leg cramps. Which additional finding most strongly suggests hypokalemia?

- A.** Shallow respirations and diminished bowel sounds
- B.** Irregular pulse and muscle weakness
- C.** Warm flushed skin and bounding pulses
- D.** Facial edema and increased deep tendon reflexes

100. A client receiving peritoneal dialysis reports new abdominal pain, and the nurse notes that the dialysate outflow is cloudy. Which complication should the nurse suspect first?

- A.** Peritonitis
- B.** Hypokalemia
- C.** Pulmonary edema
- D.** Dialysis disequilibrium syndrome

Simulation Test 7 — Answer Key

1. A	26. C	51. C	76. C
2. D	27. B	52. B	77. C
3. C	28. A	53. B	78. C
4. B	29. C	54. B	79. A
5. B	30. C	55. C	80. A
6. C	31. C	56. B	81. A
7. C	32. D	57. B	82. C
8. A	33. A	58. A	83. B
9. B	34. A	59. B	84. A
10. A	35. C	60. B	85. B
11. B	36. B	61. A	86. D
12. C	37. B	62. C	87. A
13. B	38. B	63. A	88. D
14. B	39. B	64. B	89. A
15. C	40. C	65. A	90. A
16. B	41. A	66. B	91. A
17. A	42. B	67. B	92. B
18. C	43. B	68. A	93. B
19. C	44. C	69. A	94. B
20. B	45. B	70. B	95. A
21. C	46. B	71. C	96. C
22. C	47. B	72. B	97. A
23. C	48. B	73. C	98. B
24. B	49. C	74. C	99. B
25. A	50. B	75. B	100. A

Simulation Test 8

100 questions

1. A practical nurse on a medical unit is deciding which task to do next. Which situation should the nurse prioritize?
 - A. Check a fasting blood glucose before breakfast for a client with diabetes
 - B. Evaluate a client who suddenly becomes restless and says, "I can't catch my breath"
 - C. Bring a warm blanket to a client who feels cold after a bath
 - D. Answer a family member's request for an update on visiting hours
2. A practical nurse in a rehabilitation facility receives these updates at the same time. Which client should the nurse assess first?
 - A. A client with a new ileostomy whose pouch leaked and needs a linen change
 - B. A client 1 day after hip repair who reports calf pain and new shortness of breath
 - C. A client with Parkinson disease who is 30 minutes late receiving a scheduled dose of carbidopa-levodopa
 - D. A client recovering from pneumonia who wants help walking to the bathroom
3. A practical nurse delegated collection of a 24-hour urine specimen to an assistive personnel (AP) for a stable client. Near the end of the collection period, the AP reports, "The client accidentally voided in the toilet once this morning, but I saved the rest." Which action by the practical nurse is best?
 - A. Send the specimen to the laboratory because most of the urine was collected
 - B. Document the missed void and continue the collection as planned
 - C. Discard the specimen and restart the 24-hour collection
 - D. Ask the AP to estimate the amount of the missed void and add that amount to the record
4. A practical nurse is caring for a stable adult client who had a simple dressing change yesterday. The provider writes an order for the dressing to be changed today. Which action is most appropriate for the nurse to delegate to an assistive personnel (AP)?
 - A. Remove the old dressing and assess the wound for redness and drainage
 - B. Teach the client how to keep the wound clean at home
 - C. Reposition the client and gather clean supplies for the dressing change
 - D. Decide whether the wound is healing as expected
5. A nurse is caring for an alert adult client who says, "I do not want my roommate in the room while we talk about my test results." The roommate is sitting nearby and says, "It's fine. I hear everything anyway." What should the nurse do?
 - A. Continue the discussion because the roommate has already heard prior conversations
 - B. Ask the roommate to step out or move the discussion to a private area
 - C. Lower the voice and give only the most important test information
 - D. Ask the roommate to promise not to repeat the information
6. A client with mild expressive aphasia after a stroke is being asked to choose between two discharge rehabilitation facilities. The provider says, "He can't explain himself well, so his brother should decide." The client becomes frustrated, points to one brochure repeatedly, and nods when the nurse asks, "Is this the place you want?" What is the nurse's best action?
 - A. Support the brother's decision because communication deficits mean the client lacks decision-making ability
 - B. Ask the provider to select the facility because discharge planning is a medical decision
 - C. Advocate for the client's preference and use communication supports to confirm the choice
 - D. Delay discharge until the client can speak clearly enough to discuss every option in detail
7. A practical nurse is caring for a client admitted after a suicide attempt. The client's close friend arrives at the unit and says, "The client told me yesterday that I could always be involved in care. Please tell me how the client is doing." The client is sleeping and no written permission is documented. Which action should the nurse take?
 - A. Provide a brief update because the friend states the client previously gave permission
 - B. Ask the friend to return after the client wakes so the client can decide what information may be shared
 - C. Share only the client's room number and general condition
 - D. Discuss the client's condition with the friend because the friend may help support recovery
8. A practical nurse is leaving a voicemail for a client to confirm tomorrow's clinic appointment. Which message best protects the client's privacy?
 - A. Hello, this is the diabetes clinic calling to remind you about your insulin teaching visit tomorrow at 9 a.m.
 - B. This is your nurse from the medical center. Please call us back at 555-0184 regarding your appointment.

C. Your test results are back, and the provider wants to discuss your abnormal findings at tomorrow's appointment.

D. This is the oncology office calling to confirm your chemotherapy follow-up for tomorrow morning.

9. A client with advanced illness asks, "What is the difference between a living will and a health care proxy?" Which response by the nurse is best?

A. A living will names the person who pays medical bills, and a health care proxy lists the client's current medications.

B. A living will gives written instructions about care the client would or would not want, and a health care proxy names someone to make decisions if the client cannot.

C. A living will is used only in the hospital, and a health care proxy is used only after discharge.

D. A living will cannot be changed, but a health care proxy can be updated at any time.

10. An adult client is being admitted for treatment of injuries after a motor vehicle crash. The client is alert and states, "I don't want to complete any advance directive forms." The client's spouse tells the nurse, "You need to keep pushing. Make sure this gets done tonight." Which action by the nurse is most appropriate?

A. Ask the provider to explain that completing an advance directive is required before treatment can continue.

B. Have the spouse sign the forms because the spouse will likely be involved in decisions later.

C. Document the client's refusal and continue care while respecting the client's decision.

D. Return later with the forms and request that the spouse remain present to encourage the client.

11. A client is being prepared for a nonemergency paracentesis. The provider has explained the procedure, and the nurse is present when the client signs the consent form. Immediately afterward, the client asks the nurse, "So this procedure will cure my liver disease, right?" Which action by the nurse is most appropriate?

A. Witness the signature and continue preparation because the consent has already been signed

B. Explain the expected results of the procedure so the client feels reassured

C. Hold the procedure preparation and notify the provider that the client may not understand the procedure

D. Ask a family member to confirm that the provider explained the procedure earlier

12. A client is preparing for discharge after

hospitalization for a new diagnosis of seizure disorder. The provider prescribes an anticonvulsant and instructs the client not to drive until cleared. The client says, "I work the night shift at a warehouse, and driving myself is the only way I get there." Which nursing action is most appropriate before discharge?

A. Ask the provider to remove the driving restriction so the client can return to work

B. Refer the client to case management or social services to explore transportation and community resource options

C. Advise the client to take the medication only on days off from work to reduce drowsiness while driving

D. Delay discharge until the client can guarantee a family member will drive to every shift

13. A pediatric clinic wants to improve completion of routine childhood immunizations by 24 months of age. Review of records shows that many children miss vaccines after their 12-month visit because follow-up appointments are not scheduled before families leave the clinic. The team tests a new process in which staff schedule the next vaccine visit before checkout. Which measure is the best process measure for this quality improvement project?

A. The percentage of eligible children who leave the clinic with their next vaccine appointment scheduled

B. The number of children in the community who develop vaccine-preventable illnesses

C. The percentage of children fully immunized by 24 months of age

D. The number of parents who say they are satisfied with the clinic overall

14. A practical nurse is caring for a client admitted for a COPD exacerbation. During the evening assessment, the nurse notes the client becomes short of breath while walking to the bathroom, needs to stop twice to rest, and says, "I don't think I can manage all those stairs at home." The client is expected to be discharged in 2 days. Which action best demonstrates effective interdisciplinary information sharing?

A. Document the findings in the chart and wait for the discharge planner to review the record

B. Tell the client to discuss concerns about stairs with the provider during morning rounds

C. Notify the appropriate team members that the client has exertional dyspnea and may have difficulty managing stairs at home

D. Ask the nursing assistant to observe the client walking again later and report whether the shortness of breath improves

15. A practical nurse is assigned to care for a client receiving a continuous IV infusion. The nurse notices that the infusion pump is malfunctioning and delivering the medication at an inconsistent rate. Another nurse says, "Just estimate the rate manually until biomedical staff can look at it." The practical nurse has not been trained or checked off on manual regulation of this high-alert medication. Which action is most appropriate?

- A.** Regulate the infusion manually to avoid interrupting the prescribed therapy
- B.** Turn off the infusion and leave the client unattended while searching for replacement equipment
- C.** Stop using the malfunctioning pump, stay with the client, and notify the charge nurse or provider according to facility policy
- D.** Ask the other nurse to document the malfunction so the practical nurse does not become involved

16. A practical nurse in a prenatal clinic is caring for a 15-year-old client who is 12 weeks pregnant. During the visit, the client states that the baby's father is 24 years old and says, "Please don't tell anyone. I chose this." Which action by the nurse is most appropriate?

- A.** Keep the information confidential because the client states the relationship is voluntary
- B.** Encourage the client to tell a parent and wait to see what the family decides
- C.** Follow agency policy and state law for mandatory reporting of suspected abuse involving a minor
- D.** Report the situation only if the client also has physical injuries

17. A practical nurse suspects neglect of an older adult client seen in a community clinic. Which statement best reflects the nurse's responsibility for mandatory reporting?

- A.** The nurse should report only after personally proving that neglect occurred
- B.** The nurse should make a report based on reasonable suspicion according to policy and law
- C.** The nurse should wait until the client agrees to the report
- D.** The nurse should report only if a provider gives a written order

18. A practical nurse delegates morning blood glucose checks to an assistive personnel (AP) for stable clients who have standing orders for finger-stick monitoring. One hour later, the nurse notices that the AP documented a glucose result of 42 mg/dL for a client and then moved on to the next room. Which action by the nurse is most appropriate?

- A.** Ask the AP to repeat the test after finishing all assigned glucose checks

B. Instruct the AP to give the client orange juice and recheck the glucose in 15 minutes

C. Assess the client immediately and address the low glucose result without delay

D. Remind the AP at the end of the shift that abnormal values should be reported

19. A practical nurse is working with an assistive personnel (AP) in a rehabilitation unit. Which delegated task requires the nurse to provide the most specific follow-up instructions before the AP begins?

- A.** Offer fresh water to clients who do not have fluid restrictions
- B.** Transport a stable client to the therapy gym by wheelchair
- C.** Record the amount of breakfast eaten by each assigned client
- D.** Assist a client with left-sided weakness to transfer from bed to chair for the first time after admission

20. A practical nurse in a clinic is preparing follow-up care for four clients. Which client is facing the clearest barrier to equitable access to care?

- A.** A client who prefers morning appointments and is scheduled for 10 a.m.
- B.** A client who asks for written instructions to review again at home
- C.** A client whose prescribed medication costs more than the client can afford
- D.** A client who says the wait time in the lobby is longer than expected

21. A practical nurse working with an interdisciplinary clinic team reviews appointment data from the last 6 months. Clients from one neighborhood have the highest rate of missed visits. Many of these clients rely on buses, several bus routes were recently reduced, and the clinic's only diabetes education class is offered at 2 p.m. on weekdays. Which recommendation by the nurse best promotes equitable access to care?

- A.** Encourage clients to take greater responsibility for arranging transportation before making appointments
- B.** Offer additional diabetes education options, such as evening sessions or remote participation, and connect clients with transportation assistance resources
- C.** Discharge clients from the program after two missed visits so appointment slots stay available
- D.** Shorten each diabetes education class so clients can return home earlier

22. A nurse in the emergency department is preparing to administer an IV antiemetic to a client who has just been moved from a hallway stretcher to a treatment room. The client says, "I'm so nauseated—please give it quickly." Which action by the nurse is best to verify the correct client before administering the medication?

- A.** Ask the client to state their full name and date of birth, then compare that information with the medication record and identification band
- B.** Confirm the client's room assignment and chief complaint with the unit clerk before giving the medication
- C.** Ask the client's family member to identify the client and confirm the medication is intended for them
- D.** Check the name printed on the treatment room door and proceed if it matches the medication record

23. A nurse is preparing to use a portable electric suction machine for a client who may need airway suctioning during transport to radiology. The machine turns on, but the tubing is disconnected from the collection canister and no suction is felt at the catheter port. Which action should the nurse take first?

- A.** Reconnect the tubing and verify that suction pressure is functioning before transport
- B.** Send the machine with the client and troubleshoot it if suction is needed later
- C.** Proceed with transport and use wall suction in radiology if necessary
- D.** Insert the suction catheter now to determine whether the machine is working

24. A nurse is preparing to cleanse and dress a postoperative wound. The client says, "I had a serious reaction when a doctor used chlorhexidine on my skin before a procedure." Which action should the nurse take first?

- A.** Hold the chlorhexidine, verify the details of the reaction, and check the record for documentation before proceeding
- B.** Apply a small amount of chlorhexidine to intact skin to see whether the client reacts again
- C.** Proceed with chlorhexidine because skin-prep reactions are usually mild and temporary
- D.** Administer the dressing change as planned, then document the allergy afterward

25. A practical nurse in an older adult clinic notices that a client waiting for transportation is smoking in a courtyard area directly beneath a wall sign marked "Oxygen in Use" near a storage alcove for portable oxygen cylinders. Which action should the nurse take first?

- A.** Ask the client to stop smoking and move away from the oxygen storage area immediately
- B.** Report the incident to facility maintenance so the oxygen cylinders can be relocated later

- C.** Remove the wall sign because it may be confusing clients in the courtyard
- D.** Wait for security staff to arrive before approaching the client

26. A client with acute confusion keeps trying to walk to the bathroom alone despite weakness and an unsteady gait. The client is cooperative, answers simple questions, and agrees when reminded to call for help. Which action by the nurse best follows the principle of least-restrictive restraint use?

- A.** Apply a vest restraint to prevent the client from leaving the bed
- B.** Place the client in a room near the nurses' station and offer scheduled toileting assistance
- C.** Raise all four side rails to keep the client from getting up
- D.** Request an as-needed sedative each time the client attempts to stand

27. A nurse removes a client's breakfast tray and notices a cup containing several tablets still on the bedside table, even though the electronic record shows the 0730 medications were documented as administered by the previous nurse. The client says, "I didn't take those yet." The client has no symptoms. Which action is most appropriate?

- A.** Discard the tablets, document that the client refused the medications, and continue the medication pass
- B.** Assess the client, hold the medications until they can be identified, notify the charge nurse or provider per facility policy, and complete an incident report
- C.** Give the tablets immediately so that the documentation matches what was charted
- D.** Ask the client which medications are usually taken in the morning and document based on the client's report

28. A nurse is preparing to move a client from a bed to a stretcher using an air-assisted lateral transfer device. Which action is most appropriate?

- A.** Lower the side rail on the far side of the stretcher to allow easier movement
- B.** Keep the client covered and move the client only after inflating the device fully
- C.** Ask the client to reach across to the stretcher during the transfer
- D.** Place the stretcher several inches lower than the bed to let gravity assist the move

29. A practical nurse is preparing to administer oral medications to a client who is in transmission-based isolation. Before entering the room, the nurse reviews the provider's orders and gathers the needed supplies. Which action is most important to reduce the spread of infection to other clients?

- A.** Take only the medications into the room and leave the medication cup on the bedside table for later retrieval

- B.** Bring the entire medication cart into the room so all supplies remain with the nurse during administration
- C.** Keep nonessential supplies outside the room and bring in only the items needed for that client's care
- D.** Set clean medication supplies on the overbed table so they can be used again if they do not touch the client
- 30.** A nurse is preparing to administer an intramuscular injection to a restless client who has previously coughed suddenly during close care. The nurse expects routine contact with the client's skin and a small amount of blood at the injection site but also recognizes a chance of secretions reaching the face if the client coughs unexpectedly. Which personal protective equipment should the nurse wear?
- A.** Gloves only
- B.** Gloves and a surgical mask
- C.** Gloves and eye protection
- D.** Gloves, mask, and eye protection
- 31.** A practical nurse is preparing to perform sterile suctioning through a tracheostomy. After opening the sterile kit, the nurse realizes the sterile gloves in the kit are the wrong size. A coworker places the correct-size sterile gloves on the bedside table and offers to open them onto the field. Which action by the coworker best preserves sterility?
- A.** Drop the glove package from several inches above the field into the center of the sterile setup
- B.** Open the glove package so the contents fall gently onto the sterile field without the coworker reaching over the field
- C.** Place the unopened glove package on the edge of the sterile field for the nurse to open later
- D.** Hand the sterile glove package directly to the nurse, who is already touching nonsterile equipment
- 32.** A practical nurse is caring for a client admitted from a long-term care facility with profuse diarrhea and suspected *Clostridioides difficile* infection. Which intervention is most appropriate to include in the client's care plan?
- A.** Use alcohol-based hand rub after contact with the client
- B.** Place the client in a negative-pressure room
- C.** Use dedicated equipment for the client whenever possible
- D.** Wear a surgical mask for all care provided in the room
- 33.** A practical nurse is reinforcing teaching for the family of a hospitalized client who is on droplet precautions for pertussis. Which statement by a family member indicates a need for further teaching?
- A.** I should put on a mask before I go into the room.
- B.** The client can be transported if it is necessary for care.
- C.** The room door needs to stay closed at all times.
- D.** I should follow staff instructions about protective equipment.
- 34.** A practical nurse is preparing to flush a client's feeding tube with water before medication administration. The client has no isolation orders, but the nurse notes that the client is nauseated and begins gagging as the syringe is attached. Which action best follows standard precautions?
- A.** Continue the procedure because standard precautions are needed only for clients with known infections
- B.** Apply gloves and a face shield before flushing because body fluid splashing may occur
- C.** Ask another staff member to hold the client upright so personal protective equipment is unnecessary
- D.** Place the client on contact precautions before giving the medication
- 35.** A nurse is collecting data from a client who is 10 weeks pregnant at the first prenatal visit. Which finding is most important to report to the provider?
- A.** Urinary frequency
- B.** Blood pressure of 138/88 mm Hg
- C.** Fetal heart rate not heard with a handheld Doppler
- D.** Reports of mild fatigue in the afternoon
- 36.** A client in labor is lying supine for continuous fetal monitoring and says, "I feel lightheaded and nauseated." The nurse notes the client's skin is pale, blood pressure is 92/56 mm Hg, and the fetal heart rate has decreased from 140/min to 118/min. Which action should the nurse take first?
- A.** Reposition the client to a side-lying position
- B.** Apply oxygen at 10 L/min by face mask
- C.** Increase the primary IV infusion rate
- D.** Perform a vaginal examination
- 37.** A nurse is reinforcing teaching for a client who is 12 hours postpartum after an uncomplicated vaginal birth. Which action should the client be encouraged to do first to reduce the risk of urinary retention and postpartum hemorrhage?
- A.** Void at regular intervals, even if the urge to urinate is mild
- B.** Limit oral fluids for the first 24 hours
- C.** Remain on bed rest until lochia becomes scant
- D.** Delay ambulation until perineal swelling is gone
- 38.** A nurse is providing anticipatory guidance to the parent of a 4-month-old infant. Which finding should the nurse identify as an expected developmental milestone?
- A.** The infant rolls from back to abdomen
- B.** The infant sits without support

C. The infant laughs aloud and reaches for objects

D. The infant uses a pincer grasp to pick up cereal

39. A practical nurse is reinforcing teaching with the parent of a 4-year-old child whose new baby sibling was born 2 weeks ago. The parent says, "Ever since the baby came home, my child wants a bottle again and has started having toileting accidents." Which response by the nurse is most appropriate?

A. This behavior suggests a developmental disorder and should be reported right away.

B. These behaviors can be a temporary response to stress from a major family change.

C. Your child is probably too young to understand that the baby needs extra attention.

D. You should avoid giving attention to these behaviors so they stop more quickly.

40. A practical nurse is reinforcing teaching with a 39-year-old client who says, "I skipped my last health visit because I felt fine." Which response by the nurse is best?

A. Screening tests are mainly useful after a disease has already been diagnosed.

B. Routine screening can detect some health problems early, even before symptoms appear.

C. If you have no symptoms, you only need health screening every few years.

D. Screening is most important only for older adults with chronic illness.

41. A nurse is collecting a health history from an adult client who says, "I sometimes feel pressure in my chest when I walk up stairs." Which follow-up question is most appropriate to help assess what relieves the symptom?

A. How long does the pressure usually last?

B. Can you rate the pressure on a scale from 0 to 10?

C. What do you do that makes the pressure go away?

D. Does the pressure stay in your chest or move anywhere else?

42. A nurse is reviewing cast care with a client who recently moved from another country. The client smiles and nods throughout the teaching session. When the nurse asks, "How will you keep the cast dry at home?" the client gives an unrelated answer in broken English. Which action by the nurse is best to reduce this communication barrier?

A. Repeat the same instructions more slowly and more loudly

B. Ask a family member who is visiting to explain the instructions

C. Use a qualified medical interpreter to continue the teaching

D. Provide the client with written instructions to read later

43. A nurse is preparing to teach a client about a newly prescribed low-fat diet after gallbladder surgery. The client keeps looking at the door and says, "My ride is waiting downstairs, and I need to leave now." Which action by the nurse is best?

A. Provide the full teaching quickly so all instructions are covered before discharge

B. Ask the client to sign the discharge papers and review the diet instructions later by mail

C. Identify the client's immediate concern, give the most essential teaching in brief segments, and arrange follow-up reinforcement

D. Delay discharge until the client agrees to stay for the entire teaching session

44. A client who was told today that chemotherapy will begin next week says to the practical nurse, "I keep thinking about losing my hair and being sick all the time. I don't know if I can handle this." Which response by the nurse best provides emotional support?

A. You need to stay positive because many people do well with chemotherapy.

B. Tell me more about what worries you most about starting treatment.

C. The provider can explain the side effects better than I can.

D. Try not to think about next week until it actually gets here.

45. A client who learned today that they have hypertension tells the nurse, "I can't stop thinking about all the worst-case scenarios. I feel too overwhelmed to do anything." Which nursing response is most appropriate to promote effective coping?

A. "Try not to think about it so much. Stress will only make your blood pressure worse."

B. "Let's focus on one step you can take today, such as writing down your medications and follow-up appointment."

C. "You need to accept that this condition will require major lifestyle changes right away."

D. "Most people get used to this diagnosis quickly, so you will probably feel better by tomorrow."

46. A nurse in a community crisis center is interviewing a client who was brought in by a family member after a heated dispute at home. The client is sitting forward in the chair, has a flushed face, and keeps tapping one foot rapidly. Which additional finding should the nurse recognize as the strongest indicator of increasing risk for violence?

A. The client avoids eye contact and answers questions with brief responses

B. The client states, "Sometimes I stay up late when I'm stressed"

- C.** The client begins speaking in a loud voice and clenches both fists while saying, "He'd better stay away from me"
- D.** The client asks to call a friend before continuing the interview
- 47.** A nurse is caring for a client who is withdrawing from alcohol. Which intervention is most appropriate to help reduce stimulation and promote safety during the withdrawal period?
- A.** Place the client in a quiet, well-lit room near the nurses' station
- B.** Encourage frequent visitors to keep the client oriented and distracted
- C.** Keep the television on to provide continuous background noise
- D.** Move the client to a dark room and limit all staff contact
- 48.** A client in the emergency department is intoxicated, shouting at staff, and slamming a hand against the wall after being told there will be a wait for evaluation. The client is standing near the nurses' station, and other clients are watching. Which action should the nurse take first?
- A.** Tell the client that this behavior is unacceptable and security will be called if it continues
- B.** Move the client to a quieter area and speak in a calm, concise manner
- C.** Ask several staff members to surround the client and explain unit expectations
- D.** Immediately restrain the client to protect the safety of the unit
- 49.** A client who recently had a below-the-knee amputation tells the nurse, "I don't want my grandchildren to see me like this." Which response by the nurse is most therapeutic?
- A.** They are your family, so they will accept you no matter what.
- B.** You should let them visit now so you can get used to it.
- C.** It sounds as though you are worried about how your grandchildren will react.
- D.** Many clients feel better once they start physical therapy.
- 50.** A client who has been waiting for discharge transportation starts slamming a bedside table drawer and says in a rising voice, "I've been ignored for hours." The client is standing, breathing fast, and looking directly at the nurse. Which nursing action is most appropriate?
- A.** Stand directly in front of the client and explain that this behavior is unacceptable
- B.** Ask the client to describe what is most frustrating while the nurse maintains a calm tone and personal space
- C.** Tell the client to sit down immediately so the nurse can continue the discharge process
- D.** Leave the room at once without speaking so the client can calm down alone
- 51.** A hospitalized older adult with mild dementia is awake most of the night and becomes more confused after being moved to a different room. Which intervention by the practical nurse is most appropriate to help reduce the client's disorientation?
- A.** Keep the television on through the night to provide constant stimulation
- B.** Ask several staff members to reorient the client each hour
- C.** Place a clock and calendar in clear view and maintain a consistent routine
- D.** Limit family visits until the client adjusts to the new room
- 52.** A nurse is reinforcing teaching with a client who has heart failure about taking a prescribed diuretic each morning. The client says, "In my community, medicines are usually stopped once swelling goes down, because taking them too long can weaken the body." Which response by the nurse is most appropriate?
- A.** Tell the client that this belief is incorrect and the medication must be taken exactly as prescribed
- B.** Ask the client to describe the belief in more detail and discuss how the prescribed medication works before planning next steps
- C.** Document that the client is noncompliant and notify the provider that the client refuses treatment
- D.** Advise the client to alternate the prescribed medication with traditional remedies to respect the client's culture
- 53.** A nurse is reinforcing teaching before a genital examination for a client who identifies as nonbinary. The client says, "Medical exams like this are stressful for me." Which nursing action is most appropriate to support the client's psychosocial safety during the examination?
- A.** Tell the client the examination will be brief and encourage them not to focus on their discomfort
- B.** Ask the client what terms they prefer for body parts and explain each step before it is performed
- C.** Delay the examination until a provider of the same gender identity is available
- D.** Limit conversation during the examination to reduce embarrassment for the client
- 54.** A client with newly diagnosed peripheral vision loss bumps into objects on the affected side and says, "I feel stupid when this happens, so I just stay in bed unless someone makes me get up." Which nursing action is best?
- A.** Tell the client that limiting activity will prevent further embarrassment

- B.** Encourage the client to move independently without assistance to build confidence quickly
- C.** Acknowledge the client's feelings and work with the client to practice safe scanning and mobility strategies
- D.** Explain that many clients adjust eventually and redirect the conversation to the treatment plan

55. A client with end-stage chronic obstructive pulmonary disease who is receiving end-of-life care tells the nurse, "I don't want to keep talking. Can you help me record a short message for my grandchildren while I still have the energy?" Which nursing action is most appropriate?

- A.** Tell the client that discussing emotions with the nurse is more therapeutic than making recordings
- B.** Assist the client in making the recording and provide privacy as desired
- C.** Explain that the family should arrange personal recordings after discharge
- D.** Redirect the client to rest because emotional conversations may increase fatigue

56. A nurse is caring for a client with a mature gastrostomy tube who receives intermittent feedings. Before the feeding, the nurse notes the skin around the stoma is reddened and moist, with a small amount of formula-like drainage on the dressing. Which nursing action is most appropriate?

- A.** Cleanse the area, apply a skin barrier, and check that the external bolster is not too loose
- B.** Rotate the tube forcefully and tighten the external bolster firmly against the skin
- C.** Hold the feeding permanently and remove the tube
- D.** Cover the site with a thick occlusive dressing and avoid further assessment until the next shift

57. A nurse is reinforcing teaching with a client who is using crutches after a lower-leg injury. The client must avoid bearing weight on the injured leg while walking. Which crutch gait should the nurse teach?

- A.** Four-point gait
- B.** Two-point gait
- C.** Three-point gait
- D.** Swing-through gait

58. A nurse is caring for a client who has chronic kidney disease and is prescribed a low-protein diet. Which client meal selection indicates an understanding of the diet?

- A.** Grilled chicken breast, yogurt, and milk
- B.** Scrambled eggs, sausage, and cheese toast
- C.** Rice, steamed carrots, and a dinner roll with jelly
- D.** Turkey sandwich with peanut butter crackers

59. A client with a right ankle sprain that occurred earlier in the day reports pain and swelling at the injury site. Which nonpharmacologic intervention should the nurse implement?

- A.** Apply a warm compress to the ankle
- B.** Elevate the ankle and apply a cold pack
- C.** Massage the ankle for 10 minutes
- D.** Encourage the client to bear weight to reduce stiffness

60. A nurse is caring for a client with mild dementia who can usually complete grooming with verbal reminders. The client becomes confused when several hygiene items are placed at the bedside and says, "I don't know what to do first." Which nursing action best supports the client's ability to perform this activity of daily living?

- A.** Complete the grooming tasks for the client to avoid frustration
- B.** Offer one item at a time and give simple step-by-step directions
- C.** Place all supplies within reach and allow extra time without cueing
- D.** Ask the client to describe the entire grooming routine before starting

61. A nurse is caring for a client who is recovering from a stroke and has aphasia, limited mobility, and episodes of urinary incontinence because the client cannot get to the toilet quickly enough. Which intervention is most appropriate to include in the plan of care?

- A.** Limit oral fluids during the day to reduce the need to void
- B.** Offer toileting assistance at regular intervals throughout the day
- C.** Insert an indwelling urinary catheter for convenience
- D.** Encourage the client to wait to void until the urge becomes strong

62. A nurse is caring for a client who reports, "I wake up every night after I use the bathroom, and then I can't get back to sleep." Which nursing suggestion is most appropriate to help improve the client's sleep?

- A.** Drink most of the day's fluids in the 2 hours before bedtime
- B.** Limit fluids in the evening while maintaining adequate intake earlier in the day
- C.** Stay in bed longer each morning to make up for lost sleep
- D.** Do light housework whenever unable to fall back asleep at night

- 63.** A practical nurse is caring for a client who has been lying in a semi-Fowler position for several hours. The client begins sliding down in bed and must be repositioned often. Which problem is the client at greatest risk for because of this movement?
- A.** Shear injury to the skin and underlying tissue
 - B.** Maceration caused by excess moisture
 - C.** Allergic dermatitis from bed linens
 - D.** Bruising related to decreased platelet count
- 64.** A practical nurse is caring for a client who has chronic tinnitus and reports difficulty falling asleep at night because the ringing seems louder in a quiet room. Which intervention is most appropriate for the nurse to suggest?
- A.** Keep the room completely silent to reduce sensory stimulation
 - B.** Use soft background sound, such as a fan or low-volume music, at bedtime
 - C.** Drink coffee in the evening to stay relaxed until sleep comes
 - D.** Insert cotton into both ears before going to sleep
- 65.** A practical nurse is caring for a client who has been immobilized in bed for several days. To help prevent constipation related to immobility, which nursing action is most appropriate?
- A.** Encourage fluid intake and provide foods high in fiber if not contraindicated
 - B.** Limit oral fluids to decrease abdominal distention
 - C.** Keep the client on complete bed rest after meals
 - D.** Offer a bedpan only at the end of the day
- 66.** A nurse is preparing to administer a scheduled dose of enoxaparin subcutaneously. After verifying the prescription and identifying the client, the nurse notes that the prefilled syringe contains an air bubble. Which action is most appropriate to support safe medication administration?
- A.** Expel the air bubble before injection to ensure the full dose is accurate
 - B.** Administer the medication with the air bubble left in the prefilled syringe
 - C.** Discard the syringe and obtain a vial form of the medication instead
 - D.** Hold the medication and contact the provider to clarify the dose
- 67.** A practical nurse is reviewing the MAR at the start of the shift for a client with postoperative pain. The provider prescribed morphine 2 mg IV every 4 hours PRN for pain rated 7 to 10. The MAR shows the dose was given 30 minutes ago by the previous nurse. The client now reports pain of 8/10 and asks for more morphine. What is the nurse's best action?
- A.** Administer the morphine because the client still rates the pain as severe
 - B.** Wait until the next dose is due, monitor the client, and use the MAR to verify the last administration time
 - C.** Ask the client whether the previous nurse actually gave the medication, then decide
 - D.** Document the pain rating and remove the morphine from the MAR
- 68.** A nurse is caring for a patient who is wheezing and short of breath during an asthma flare. The provider prescribes albuterol for the fastest direct effect on the airways. Which route of administration should the nurse use?
- A.** Oral
 - B.** Inhalation
 - C.** Transdermal
 - D.** Subcutaneous
- 69.** A client with chronic kidney disease has fatigue, shortness of breath with activity, and a hemoglobin level of 7.1 g/dL. The provider prescribes 1 unit of packed red blood cells. Before obtaining the blood from the blood bank, which information is most important for the nurse to review?
- A.** The client's most recent blood type and crossmatch results
 - B.** The client's daily calcium intake
 - C.** The time of the client's last bowel movement
 - D.** The client's preferred meal schedule
- 70.** A nurse is administering 1,200 mL of IV fluid over 10 hours using an infusion pump. After 4 hours, 780 mL remains in the bag. At what rate in mL/hr should the nurse set the pump for the remaining infusion time so the full amount is delivered as prescribed?
- A.** 95 mL/hr
 - B.** 100 mL/hr
 - C.** 110 mL/hr
 - D.** 130 mL/hr
- 71.** A practical nurse is documenting administration of a prescribed controlled opioid after giving it to a client with postoperative pain. Which action is most appropriate?
- A.** Document the medication on the MAR immediately after administration according to facility procedure

- B.** Wait until the end of the shift so pain relief can be charted at the same time
- C.** Ask the unlicensed assistive personnel to record the medication because the dose was already given
- D.** Document the dose only in the nurse's progress notes because controlled substances should not be entered on the MAR until the next count

72. A nurse is caring for a postoperative client who is receiving opioid analgesia by patient-controlled analgesia (PCA). Which assessment finding indicates the PCA should be withheld and the nurse should take immediate action?

- A.** Respiratory rate of 8/min
- B.** Pain rating of 6 on a 0-to-10 scale
- C.** Client request for instruction about the lockout interval
- D.** Mild nausea after ambulation

73. A nurse is preparing to administer the next dose of regular insulin to a client with type 1 diabetes. The client is scheduled to eat lunch in 15 minutes. Which pre-administration data is most important for the nurse to review first?

- A.** The client's most recent blood glucose level
- B.** The client's potassium level from yesterday morning
- C.** The client's weight from admission
- D.** The client's hemoglobin level from last week

74. A patient taking phenelzine for depression tells the nurse, "I've had a bad cold, so I bought a decongestant at the pharmacy." Which over-the-counter medication should the nurse identify as the greatest concern for a serious drug interaction?

- A.** Pseudoephedrine
- B.** Guaifenesin
- C.** Saline nasal spray
- D.** Dextromethorphan lozenges

75. A client with a deep vein thrombosis is prescribed rivaroxaban 15 mg by mouth twice daily. Before administering the morning dose, the nurse reviews the laboratory results and notes hemoglobin 7.1 g/dL, decreased from 11.8 g/dL two days ago. Which action is most appropriate?

- A.** Administer the medication as prescribed and document the laboratory result
- B.** Hold the medication and notify the provider of the significant drop in hemoglobin
- C.** Give the medication with food to reduce gastrointestinal irritation
- D.** Request that the dose be reduced by half before administration

76. A nurse is teaching a client who has a new prescription for sublingual nitroglycerin for stable angina. Which instruction should the nurse include?

- A.** Swallow the tablet with water as soon as chest discomfort begins
- B.** Store the tablets in the original dark container and replace them as directed
- C.** Take one tablet every morning to prevent chest pain during the day
- D.** Lie flat for 30 minutes after taking the medication

77. A nurse is preparing a dose of nystatin oral suspension for an infant and finds that the bottle has been stored in the medication refrigerator. Which action is most appropriate before administering the medication?

- A.** Shake the bottle well and administer the medication as scheduled
- B.** Warm the medication in the microwave for 10 seconds before use
- C.** Hold the dose and ask the pharmacy to dispense a new bottle
- D.** Mix the medication with formula to improve tolerance

78. A nurse is preparing to administer clindamycin 450 mg by mouth. The oral solution available is labeled 75 mg/5 mL. How many milliliters should the nurse administer for one dose?

- A.** 15 mL
- B.** 20 mL
- C.** 30 mL
- D.** 35 mL

79. A practical nurse is collecting data on four clients in an urgent care clinic. Which client should the nurse identify as being at greatest risk for injury from orthostatic hypotension?

- A.** A client with allergic rhinitis who reports nasal congestion after mowing the lawn
- B.** A client with type 2 diabetes who reports increased thirst and urination for 2 days
- C.** A client who started an antihypertensive medication 3 days ago and becomes dizzy when standing
- D.** A client with a wrist sprain who rates pain as 4 on a scale of 0 to 10

80. A nurse is collecting a stool specimen to test for fecal occult blood. Which action is most appropriate to help obtain an accurate specimen?

- A.** Collect the specimen from stool that is mixed with urine in the toilet
- B.** Use tissue from the toilet bowl to transfer a small amount of stool into the specimen container

- C. Obtain the specimen from a clean, dry bedpan or collection hat before it contacts toilet water
- D. Add tap water to the stool specimen if the sample appears too thick to spread

81. A nurse is reviewing laboratory results for a client receiving long-term total parenteral nutrition (TPN). The client is now weak and reports new muscle twitching. Which laboratory result should the nurse identify as the priority finding?

- A. Magnesium 1.1 mg/dL
- B. Sodium 133 mEq/L
- C. Albumin 3.3 g/dL
- D. BUN 24 mg/dL

82. A client who is 12 weeks pregnant comes to the clinic reporting persistent vomiting, a 5-lb (2.3-kg) weight loss in 1 week, and very dark urine. Which prescription should the nurse question?

- A. Obtain urine ketone testing
- B. Start IV fluids as ordered
- C. Encourage small, frequent bland meals as tolerated
- D. Restrict oral fluids for the next 24 hours

83. A nurse performs a bedside urine dipstick test on a client with dysuria. The reagent strips were removed from their original container and stored in a clear plastic bag in a sunny windowsill. What is the nurse's best action?

- A. Use the strips if the urine specimen is fresh
- B. Use two strips and compare the results for consistency
- C. Discard the strips and obtain a new bottle stored as recommended
- D. Proceed with testing but document that the strips may be less sensitive

84. A practical nurse is measuring blood pressure on an adult client with obesity. The first reading is 168/94 mm Hg using a standard adult cuff, but the cuff appears too narrow to wrap properly around the upper arm. Which action should the nurse take next?

- A. Record the reading because the client should rest before any repeat measurement
- B. Repeat the measurement using a correctly sized larger cuff
- C. Use the same cuff on the forearm and document the result as equivalent
- D. Switch to a wrist device because upper-arm readings are unreliable in clients with obesity

85. A client is scheduled for an upper gastrointestinal (GI) series with barium at 0900. Which statement by the client requires the nurse to follow up before the test?

- A. I stopped eating and drinking after midnight.

- B. I removed my necklace and earrings this morning.
- C. I used a dose of milk of magnesia late last night for constipation.
- D. I expect my stools may look lighter after the procedure.

86. A nurse is reviewing a client's ECG tracing and notes a PR interval of 0.24 second on several consecutive beats. The client is alert, reports no chest pain, and has stable vital signs. Which interpretation is most appropriate?

- A. First-degree atrioventricular block
- B. Atrial fibrillation
- C. Ventricular tachycardia
- D. Normal sinus rhythm

87. A nurse is preparing to collect a venous blood specimen for routine laboratory testing. After selecting the site, the nurse cleans the skin with antiseptic. Which action is most appropriate next to reduce contamination and support a successful specimen collection?

- A. Fan the site with a gloved hand until it feels dry, then insert the needle
- B. Palpate the vein again with an ungloved finger just before insertion
- C. Allow the antiseptic to air-dry completely before inserting the needle
- D. Wipe the site once more with dry gauze immediately before insertion

88. A client with a newly placed feeding tube is prescribed medications by the enteral route. Before giving the medication, which action is most appropriate to verify safe tube placement?

- A. Inject 30 mL of air into the tube and listen over the stomach
- B. Check the color of aspirated contents and compare it with the previous assessment
- C. Confirm tube placement by x-ray before the first use
- D. Place the end of the tube in water and look for bubbling

89. A nurse is caring for a client with a new arteriovenous fistula for hemodialysis in the left forearm. The client reports, "My hand feels cold and numb." Which focused assessment should the nurse perform first?

- A. Palpate for a thrill and auscultate for a bruit over the fistula
- B. Assess the color, temperature, capillary refill, and pulses of the left hand
- C. Measure the client's oral fluid intake for the past 24 hours
- D. Ask the client to rate the discomfort on a 0 to 10 pain scale

90. A client with heartburn and epigastric discomfort began pantoprazole therapy 2 weeks ago for a gastric ulcer. Which finding best indicates the treatment is effective?

- A.** The client reports less burning pain after meals
- B.** The client has a hemoglobin level of 9 g/dL
- C.** The client reports a decreased appetite
- D.** The client has black, tarry stools

91. A client with a chest tube connected to a water-seal drainage system suddenly becomes short of breath after turning in bed. The nurse notes rapid, shallow respirations and sees continuous bubbling in the water-seal chamber that was not present earlier. Which action should the nurse take first?

- A.** Clamp the chest tube near the client's chest
- B.** Assess the tubing and drainage system for an air leak
- C.** Raise the drainage system above chest level to improve flow
- D.** Empty the drainage chamber immediately

92. A client with chronic obstructive pulmonary disease (COPD) is being discharged home. Which statement by the client indicates correct understanding of breathing techniques to reduce shortness of breath?

- A.** I will breathe in quickly through my mouth and out through my nose when I feel short of breath.
- B.** I will hold my breath for 10 seconds before exhaling to keep my lungs open.
- C.** I will inhale through my nose and exhale slowly through pursed lips.
- D.** I will take several deep, rapid breaths to bring in more oxygen.

93. A client 2 days after abdominal surgery suddenly becomes restless and reports feeling "very weak." The nurse finds cool, clammy skin; a blood pressure of 78/44 mm Hg; a heart rate of 132/min; and a rigid, distended abdomen. Which action should the nurse take first?

- A.** Increase the client's oral fluid intake
- B.** Place the client flat in bed and reassess in 30 minutes
- C.** Activate the rapid response team and stay with the client
- D.** Administer the prescribed PRN opioid analgesic for abdominal pain

94. A nurse is caring for a client who had abdominal surgery yesterday and has a Jackson-Pratt drain. Which finding should the nurse report to the provider first?

- A.** The drain contains 40 mL of serosanguineous fluid over 8 hours
- B.** The bulb is compressed and suction is maintained
- C.** The insertion site is slightly pink and tender
- D.** The drainage changes from serosanguineous to thick, foul-smelling fluid

95. A nurse is caring for a client who developed a fever 2 hours ago. Which additional finding is most consistent with the chill phase of fever?

- A.** Warm flushed skin with diaphoresis
- B.** Complaints of feeling cold and visible shivering
- C.** Decreased heart rate and dry mucous membranes
- D.** Pale skin with slow, regular respirations after sweating

96. A client with a newly implanted permanent pacemaker is being discharged. Which statement by the client indicates a need for further teaching?

- A.** I will check my pulse regularly and report dizziness or fainting.
- B.** I should carry my pacemaker identification card with me.
- C.** If I notice redness, swelling, or drainage at the incision, I should call my provider.
- D.** I can lean directly over a running car engine as long as I do not touch it.

97. A nurse is reinforcing teaching for a client with a new ileostomy who is concerned about becoming dehydrated. Which instruction is most appropriate for the nurse to include?

- A.** Limit fluid intake between meals to reduce ostomy output.
- B.** Report dizziness, dry mouth, or decreased urine output to the provider.
- C.** Expect the ostomy output to become firm within the first week after surgery.
- D.** Avoid foods containing salt to help prevent fluid retention.

98. A nurse is caring for a client with bacterial meningitis who was awake and answering questions appropriately 1 hour ago. The client is now difficult to arouse, has a temperature of 39.4°C (102.9°F), blood pressure of 86/50 mm Hg, heart rate of 128/min, and cool, mottled skin. Which change in condition should the nurse recognize as the priority?

- A.** The client may be developing septic shock
- B.** The client is experiencing expected fatigue from infection
- C.** The client needs an antipyretic for fever reduction
- D.** The client is showing anxiety related to hospitalization

99. A nurse is caring for a client 1 day after transsphenoidal hypophysectomy. The client has had 1,200 mL of urine output over the last 3 hours and reports extreme thirst. Which finding should the nurse expect if the client is developing diabetes insipidus?

- A.** Urine specific gravity of 1.002
- B.** Bounding peripheral pulses and crackles in the lungs
- C.** Serum sodium level of 128 mEq/L

D. Decreased serum osmolality

100. A client with end-stage kidney disease has an arteriovenous fistula in the right forearm for hemodialysis. Which nursing action is appropriate when caring for this client?

- A.** Use the right arm for routine blood draws to preserve veins in the left arm
- B.** Check the fistula site daily for a bruit and thrill
- C.** Apply a tight elastic bandage over the fistula after each dialysis treatment
- D.** Place the blood pressure cuff on the right arm only if the left arm is unavailable

Simulation Test 8 — Answer Key

1. B	26. B	51. C	76. B
2. B	27. B	52. B	77. A
3. C	28. B	53. B	78. C
4. C	29. C	54. C	79. C
5. B	30. D	55. B	80. C
6. C	31. B	56. A	81. A
7. B	32. C	57. C	82. D
8. B	33. C	58. C	83. C
9. B	34. B	59. B	84. B
10. C	35. B	60. B	85. C
11. C	36. A	61. B	86. A
12. B	37. A	62. B	87. C
13. A	38. C	63. A	88. C
14. C	39. B	64. B	89. B
15. C	40. B	65. A	90. A
16. C	41. C	66. B	91. B
17. B	42. C	67. B	92. C
18. C	43. C	68. B	93. C
19. D	44. B	69. A	94. D
20. C	45. B	70. D	95. B
21. B	46. C	71. A	96. D
22. A	47. A	72. A	97. B
23. A	48. B	73. A	98. A
24. A	49. C	74. A	99. A
25. A	50. B	75. B	100. B

Simulation Test 9

100 questions

1. A practical nurse on a mental health unit is deciding which client to check immediately after hearing several reports from staff. Which client should the nurse prioritize first?

- A. A client with depression who has refused breakfast and remains in bed
- B. A client with generalized anxiety who asks for a PRN medication before group therapy
- C. A client with alcohol withdrawal who is newly confused and diaphoretic
- D. A client with insomnia who says the room is too noisy to nap

2. The practical nurse is preparing to administer a scheduled morning dose of digoxin to a client with heart failure. Before the medication is given, the nurse receives four pieces of new information. Which finding should the nurse address first?

- A. The client's apical pulse is 52/min
- B. The client says breakfast arrived late and wants to eat first
- C. The client's potassium level from yesterday was 3.6 mEq/L
- D. The client asks whether the medication will need to be taken at home

3. A practical nurse delegated routine oral intake documentation to an assistive personnel (AP) for a stable client receiving strict intake and output monitoring. At the end of the shift, the AP says, "I forgot to measure the client's soup and coffee at lunch, so I left that section blank." Which action by the practical nurse is best?

- A. Estimate the missing amounts based on the meal tray and document them
- B. Ask the AP to fill in likely amounts so the record appears complete
- C. Document the intake that is known, note that the record is incomplete, and report the missing information as needed
- D. Leave the entire intake and output record undocumented for the shift

4. A practical nurse is organizing care for a stable client who had a stroke 3 days ago and has mild right-sided weakness. Which task is most appropriate to assign to assistive personnel (AP)?

- A. Evaluate whether the client is swallowing safely during breakfast
- B. Assist the client with a preplanned transfer from bed to chair using a gait belt

- C. Teach the client how to use the affected arm during dressing
- D. Determine whether the client's weakness has improved since yesterday

5. A client with hearing loss is being discharged with a new insulin regimen. The nurse notices the client smiles and nods during teaching but incorrectly demonstrates the dose when asked to show how it will be prepared. What is the nurse's best advocacy action?

- A. Ask a family member to review the instructions with the client at home
- B. Repeat the teaching more loudly and continue with the discharge plan
- C. Delay discharge teaching completion and arrange communication methods the client can use effectively, then reassess understanding
- D. Have the client sign the discharge paperwork to document that teaching was completed

6. A nurse in a long-term care facility observes an unlicensed assistive personnel (UAP) speaking sharply to a resident and pulling the resident's wheelchair roughly into place. The resident looks frightened but has no visible injury. The UAP says, "She never listens, and we are behind." What should the nurse do first?

- A. Privately counsel the UAP at the end of the shift about respectful communication
- B. Immediately protect the resident from further mistreatment and report the incident according to facility policy
- C. Ask the resident later whether a formal complaint is desired before taking action
- D. Document the resident's emotional response only if bruising appears during the next assessment

7. A practical nurse is preparing to fax a client's wound care progress note to a home health agency involved in the client's follow-up care. Before sending the fax, which action is most important to protect the client's privacy?

- A. Call the receiving office to verify the fax number and confirm that an authorized person is available to receive it
- B. Remove the nurse's name from the progress note before faxing it
- C. Wait until the end of the shift so fewer faxes are being sent
- D. Fax the document in two separate parts instead of one complete transmission

8. A practical nurse is caring for a client who is a minor. The client's parent asks to review the chart. The adolescent client then quietly tells the nurse, "I don't want my parent to know I asked the provider about birth control." Which response by the nurse is most appropriate?

- A.** Tell the client that parents always have unlimited access to every part of the chart
- B.** Refuse to speak with the parent about any part of the client's care
- C.** Explain that access to some information may depend on applicable law and facility policy, and consult the charge nurse or supervisor before sharing that part of the record
- D.** Ask the client to decide alone whether the parent can review the full chart

9. A client with chronic kidney disease is alert and says, "I named my friend as my health care agent years ago, but now I want my brother to make decisions for me if I can't." Which action by the nurse is most appropriate?

- A.** Tell the client that the original choice cannot be changed once the document has been signed
- B.** Ask the client to discuss the change with family members before any update is made
- C.** Inform the client that advance directives can usually be revised while the client has decision-making capacity and provide information on how to update the document
- D.** Document the client's verbal request and treat the brother as the decision-maker immediately

10. A nurse is caring for a client who is unconscious after a severe head injury. The chart contains both a living will and a durable power of attorney for health care. The living will states the client does not want prolonged life support in an irreversible terminal condition. The client's appointed health care agent asks the team to continue all life-sustaining treatment indefinitely, saying, "I know the document says that, but I want everything done no matter what." Which response by the nurse is best?

- A.** Follow the health care agent's request because the agent's decisions replace the written directive
- B.** Explain that the client's expressed wishes in the advance directive should guide decisions and notify the provider of the conflict
- C.** Delay any discussion until more family members can agree on a plan
- D.** Ask the ethics committee to take over all decisions immediately without notifying the provider

11. A client is scheduled for an elective endoscopy this afternoon. When the nurse brings the client to the procedure area, the provider says, "We need consent right away," and asks the nurse to explain the procedure and obtain the client's signature because the provider is running behind. Which action by the nurse is most appropriate?

- A.** Explain the procedure, answer the client's questions, and have the client sign the consent form
- B.** Ask another nurse to witness the signature after the nurse explains the procedure
- C.** Tell the provider that the provider must give the explanation needed for informed consent before the nurse can witness the signature
- D.** Have the client sign now and ask the provider to answer questions after the procedure

12. A nurse is preparing a client for a scheduled lumbar puncture. The client signed the consent form earlier in the day. Just before the procedure, the nurse notes the client received an IV benzodiazepine 10 minutes ago for severe anxiety. The client is now drowsy but arousable and says, "That's fine, I'll sign whatever you need again." Which action should the nurse take?

- A.** Allow the client to sign a new consent form because the client is still able to speak
- B.** Proceed with the existing consent because it was signed before the sedative was given, and report the client's current condition if questions arise
- C.** Ask the client's spouse to sign a new consent form because the client is drowsy
- D.** Cancel the procedure because any sedative automatically voids all previous consent

13. A client is preparing for discharge after surgical repair of a wrist fracture. The provider prescribes a new opioid pain medication for 3 days and instructs the client to return to the orthopedic clinic in 1 week. The client says, "I understand how to take the medicine, but I can't read the prescription labels well and I sometimes mix up my pills." Which action by the nurse is most appropriate before discharge?

- A.** Ask the client to sign the discharge papers and rely on the pharmacist to explain the labels
- B.** Request a pill organizer and review the medication schedule using large-print, clearly labeled instructions
- C.** Tell the client to stop all home medications until the follow-up appointment
- D.** Recommend that the client take the opioid only when the pain becomes severe

14. A practical nurse in an ambulatory surgery center joins a quality improvement project to reduce incomplete discharge instruction forms. The team plans to compare results before and after a new discharge workflow is introduced. Which measure best reflects the client care outcome of the project rather than staff completion of the new process?

- A.** The percentage of nurses who use the revised discharge form
- B.** The number of staff members trained on the new workflow
- C.** The percentage of clients who return calls because they are unsure about home care instructions
- D.** The number of discharge packets printed each week

15. A practical nurse is caring for a client admitted after an ischemic stroke. During the morning medication pass, the nurse notices the client crushes pills into applesauce with the unaffected hand, then coughs twice and says, "It feels like it's sticking." The client is scheduled for occupational therapy in 30 minutes and has not yet been seen by speech therapy. Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the coughing episode in the chart and continue giving the remaining oral medications
- B.** Notify the speech-language pathologist and provider of the coughing with medication administration, hold additional oral intake as directed by facility policy, and update occupational therapy before the session
- C.** Ask occupational therapy to assess swallowing during the scheduled visit because the client will already be working with that discipline
- D.** Offer more applesauce and encourage the client to swallow carefully before deciding whether to report the finding

16. A practical nurse is caring for a client who is scheduled for a minor bedside procedure. The provider asks the nurse to witness the client's signature on the consent form because the unit is busy. The client quietly asks the nurse, "I signed because they told me to, but I still don't really understand what the procedure is for." Which action by the nurse is most appropriate?

- A.** Witness the signature because explaining the procedure is the provider's responsibility
- B.** Ask the nursing assistant to stay with the client while the nurse signs as a witness
- C.** Delay witnessing the consent and notify the provider that the client has questions about the procedure
- D.** Briefly explain the procedure and then witness the client's signature

17. A practical nurse in an urgent care clinic is assessing a 17-year-old client with a deep laceration to the hand. While the parent is out of the room, the client says, "My stepfather cut me with a knife during a fight last night, but please don't report it." Which action by the nurse is most appropriate?

- A.** Respect the client's request because the client disclosed the injury in confidence
- B.** Wait to see whether the client repeats the statement to the provider before taking further action
- C.** Report the suspected abuse according to facility policy and state law
- D.** Tell the parent what the client said and ask for an explanation before making a report

18. At the start of the shift, a practical nurse asks an experienced assistive personnel (AP) to measure and record intake and output for several clients. At midday, the AP reports these findings. Which client should the nurse assess first?

- A.** A client with heart failure who drank 240 mL at breakfast and has not voided since 0600
- B.** A client who is 1 day postoperative and has 120 mL of emesis after lunch
- C.** A client receiving a blood transfusion who reports new chills while the AP is helping with lunch setup
- D.** A client with chronic kidney disease whose urine output for 4 hours is 100 mL

19. A practical nurse in a long-term care facility is supervising an assistive personnel (AP) who is providing evening care. Which task is most appropriate for the nurse to assign to the AP?

- A.** Teach a client how to use an incentive spirometer after pneumonia
- B.** Apply a prescribed medicated cream to a new rash
- C.** Offer toileting assistance to a client with chronic urinary incontinence
- D.** Assess a client's pain 30 minutes after an analgesic was given

20. A practical nurse in an outpatient clinic is reinforcing care instructions for a client with newly diagnosed hypertension. The client nods during teaching but then quietly says, "I did not finish school, so I usually have my grandson read medical papers to me." Which nursing action best promotes equitable access to care?

- A.** Ask the grandson to explain all future instructions to the client at home
- B.** Provide plain-language instructions, review them verbally, and use teach-back to confirm understanding
- C.** Document that the client accepted the printed materials and appeared cooperative

D. Give the client several detailed pamphlets to review before the next visit

21. A practical nurse working with a mobile primary care program reviews outreach data for three neighborhoods. Neighborhood A has the highest rate of uncontrolled diabetes, but most residents live within walking distance of a clinic. Neighborhood B has moderate disease rates and high attendance at appointments. Neighborhood C has the lowest average income, no nearby pharmacy, limited bus service, and the highest rate of missed follow-up after new prescriptions are written. Which outreach plan best supports equitable access to care?

- A.** Send the mobile unit primarily to Neighborhood A because disease severity should be the only factor in deciding services
- B.** Keep the current schedule because each neighborhood already has some form of available care
- C.** Prioritize regular visits to Neighborhood C and include medication-access coordination during those visits
- D.** Offer a general health fair in all three neighborhoods once each year

22. A nurse is preparing to deliver a breakfast tray to a client who is scheduled for a fasting laboratory test later that morning. Another staff member says, "That's Mr. Nolan in bed 2," and the client nods when the nurse approaches. Which action should the nurse take to identify the client correctly before deciding whether to leave the tray?

- A.** Ask the client to state their full name and date of birth, then compare that information with the identification band
- B.** Confirm the client's identity with the staff member who pointed out the bed
- C.** Use the bed number and room assignment to verify the client
- D.** Ask the client whether they are waiting for laboratory testing this morning

23. A nurse is preparing to place a client on a portable cardiac monitor for transport to diagnostic imaging. During the equipment check, the nurse notes the monitor powers on, but the battery indicator shows 5% remaining and no charging cable is available. The client has a history of dysrhythmias. Which action should the nurse take?

- A.** Proceed with transport because the monitor is currently functioning
- B.** Transport the client and plan to check the battery if an alarm sounds
- C.** Obtain a monitor with an adequate battery charge before transport
- D.** Turn off nonessential monitor features to conserve the remaining battery

24. A nurse is preparing to start a maintenance IV infusion using a prefilled flush syringe and transparent dressing kit. During the room introduction, the client says, "Avocados and bananas make my mouth itch, and I once got hives after blowing up party balloons." Which action should the nurse take first?

- A.** Proceed with the IV start but observe closely for a reaction
- B.** Ask the provider to prescribe a premedication before the IV start
- C.** Hold the procedure and obtain latex-free supplies before continuing
- D.** Document the food reactions after the IV is inserted to avoid delay

25. A practical nurse is completing rounds in a long-term care facility when a carbon monoxide detector begins sounding in a lounge area near a gas fireplace. Several residents are sitting in the room, and one resident says, "It's probably a false alarm." Which action should the nurse take first?

- A.** Open the windows in the lounge and continue observing the residents
- B.** Move the residents out of the area and notify the appropriate emergency response personnel
- C.** Turn off the alarm to reduce resident anxiety while maintenance checks the fireplace
- D.** Ask the residents if any of them have headache or dizziness before deciding what to do

26. A client with delirium after a severe infection keeps trying to remove an indwelling urinary catheter. The client is frightened, not combative, and repeatedly asks where they are. Which action by the nurse best follows least-restrictive restraint principles?

- A.** Apply mitt restraints to both hands to prevent catheter removal
- B.** Request a sedative medication so the client can rest
- C.** Stay with the client, provide reorientation, and use distraction while assessing for causes of discomfort
- D.** Raise all four side rails to keep the client from moving around in bed

27. A nurse enters a client's room to administer a scheduled subcutaneous anticoagulant and notices that the medication syringe in hand has the correct drug and dose but a label with another client's name. The injection has not been given, and the client has not been harmed. Which action is most appropriate?

- A.** Administer the medication because the drug and dose are correct, then notify the charge nurse
- B.** Set the syringe aside, obtain the correctly labeled medication, and complete an incident report according to facility policy

- C.** Ask the client whether the medication is usually received at this time, then give the injection if the client agrees
- D.** Discard the syringe and continue to the next task without reporting because no harm occurred
- 28.** A nurse is preparing to transfer a client from a wheelchair to the toilet using a gait belt. The client follows directions and usually bears weight well. After standing, the client suddenly says, "My legs are giving out." Which action should the nurse take?
- A.** Lower the client to the floor while supporting with the gait belt
- B.** Pull the client upright and continue the transfer quickly
- C.** Release the gait belt and step back to avoid injury
- D.** Turn the client toward the wheelchair and try to seat the client immediately
- 29.** A practical nurse is preparing to perform a sterile dressing change on a postoperative abdominal incision. After opening the sterile supplies, the nurse realizes that the client needs an extra pillow for positioning. Which action should the nurse take?
- A.** Ask the client to hold still while the nurse quickly gets the pillow and then continues the procedure
- B.** Keep the sterile gloves on, get the pillow, and avoid touching the sterile tray
- C.** Leave the sterile field, obtain the pillow, perform hand hygiene, and set up a new sterile field before continuing
- D.** Cover the sterile field with a clean towel, get the pillow, and resume the dressing change
- 30.** A nurse enters a client's room wearing a gown, gloves, mask, and goggles. After completing care, the nurse removes the gloves and gown inside the room and then steps into the hallway still wearing the mask and goggles. Which action should the charge nurse take?
- A.** Tell the nurse to remove the goggles first and then remove the mask after hand hygiene
- B.** Tell the nurse to keep both items on until the next client care task begins
- C.** Tell the nurse to remove the mask first and leave the goggles on until the end of the shift
- D.** Tell the nurse that the sequence is acceptable because the most contaminated items were already removed
- 31.** A practical nurse is assisting with a sterile wound dressing change. After the sterile field is prepared, the client suddenly turns toward the setup and sneezes directly in its direction. No items have been touched yet. Which action should the nurse take?
- A.** Proceed with the dressing change because the sterile items were not touched
- B.** Cover the field with a sterile drape and continue the procedure
- C.** Discard the sterile field and set up a new sterile field
- D.** Remove only the top supplies that were closest to the client and continue
- 32.** A practical nurse is caring for a client with suspected infectious diarrhea. The provider has prescribed contact precautions. Which action by the nurse is most appropriate when using dedicated equipment for this client?
- A.** Use the client's blood pressure cuff for other clients after wiping it with alcohol at the end of the shift
- B.** Keep a stethoscope and blood pressure cuff in the client's room for that client only
- C.** Take the unit's portable pulse oximeter into the room and return it to the clean supply area after use
- D.** Ask the client to bring personal hygiene items and equipment to common treatment areas
- 33.** A practical nurse is making an occupied bed for a client who has no isolation orders. While rolling the client to one side, the nurse notices a wet area on the sheet from urine leakage. Which action best follows standard precautions?
- A.** Put on gloves before handling the wet linens and perform hand hygiene after glove removal
- B.** Continue without gloves because urine is not covered by standard precautions unless it is visibly bloody
- C.** Ask another staff member to change the linens because body fluids always require two caregivers
- D.** Apply a mask before changing the linens because all body fluid exposure requires a mask
- 34.** After providing perineal care, a practical nurse removes gloves and leaves the client's room to document. At the nurses' station, the nurse realizes the bedside privacy curtain was touched with contaminated gloves during care. Which action is most appropriate?
- A.** Wait until housekeeping rounds later because curtains are changed on a routine schedule
- B.** Clean hands again and arrange for the contaminated high-touch surface to be addressed according to facility infection-control procedure
- C.** Return to the room and wipe the curtain with an alcohol pad while wearing no additional PPE
- D.** Place the client on contact precautions because the room environment may now be contaminated
- 35.** A nurse is reinforcing teaching with a client who is 8 weeks pregnant about common discomforts of early pregnancy. Which instruction should the nurse include to help reduce urinary frequency?
- A.** Limit total fluid intake to 1 liter per day
- B.** Drink most fluids earlier in the day and reduce intake before bedtime
- C.** Avoid emptying the bladder completely when voiding
- D.** Drink caffeinated beverages instead of water

36. A client at 39 weeks' gestation is in active labor and is receiving magnesium sulfate for severe preeclampsia. Which assessment finding should the nurse report to the provider immediately?

- A.** Urine output of 20 mL/hr
- B.** Deep tendon reflexes rated 2+
- C.** Respiratory rate of 18/min
- D.** Report of feeling flushed and warm

37. A nurse is reinforcing care for a client who is 6 hours postpartum after an uncomplicated vaginal birth. The client reports mild afterpains that become stronger while breastfeeding. Which explanation by the nurse is best?

- A.** Breastfeeding stimulates oxytocin release, which causes the uterus to contract
- B.** The bladder fills more quickly during breastfeeding and causes uterine spasms
- C.** Milk let-down decreases pelvic blood flow and leads to cramping
- D.** Breastfeeding shifts fluid into the abdomen and increases pressure on the uterus

38. A nurse is teaching the parent of a 6-year-old child about expected developmental milestones. Which behavior should the nurse identify as most appropriate for this age?

- A.** Rides a tricycle and uses mostly 2-word sentences
- B.** Ties shoelaces and understands the concept of right and left
- C.** Builds a tower of 2 blocks and says "mama" and "dada"
- D.** Uses parallel play and has a vocabulary of about 10 words

39. A practical nurse is reinforcing teaching with the parent of a 12-year-old who recently began puberty. The parent says, "My child gets embarrassed easily now, spends more time in front of the mirror, and seems very sensitive when I comment about appearance." Which response by the nurse is most appropriate?

- A.** "This behavior suggests a serious emotional problem and should be corrected right away."
- B.** "Concern about body changes and appearance is common during early adolescence."
- C.** "Children this age should not be focused on appearance if development is on track."
- D.** "This usually means your child is becoming unusually dependent on peer approval."

40. A practical nurse is reinforcing teaching with an older adult at a community clinic about osteoporosis prevention and screening. Which client statement indicates correct understanding?

- A.** I should wait until I have back pain before asking about osteoporosis testing.

- B.** If I do weight-bearing exercise and get enough calcium and vitamin D, I can help lower my risk of bone loss.
- C.** Screening is only useful if someone in my family has already had osteoporosis.
- D.** Once I reach older adulthood, prevention no longer matters because bone loss cannot be slowed.

41. A nurse is obtaining a health history from an adult client who says, "I felt my heart racing twice this week." Which follow-up question is best to assess the duration of this symptom?

- A.** What were you doing when it started?
- B.** How long did each episode last?
- C.** Did anything make it feel better?
- D.** How would you describe the sensation?

42. A practical nurse is collecting baseline physical data from an adult client newly admitted for observation. Which finding is most important to document as part of the client's baseline neurological status?

- A.** Pupils equal, round, and reactive to light
- B.** Client prefers juice instead of water with medications
- C.** Client reports usually eating dinner at 8 p.m.
- D.** Family plans to visit later in the evening

43. A nurse is preparing to teach a client about home care after a new ileostomy. Before teaching begins, the client says, "I don't need any of this. My neighbor had one, and I'll just do what she did." Which response by the nurse is best?

- A.** Ask the client to repeat back the surgeon's instructions instead of providing additional teaching
- B.** Tell the client that following a neighbor's advice could be dangerous and continue the teaching as planned
- C.** Explore the client's current understanding and readiness to learn before continuing the teaching session
- D.** Postpone all teaching until the day of the follow-up appointment so the client has time to adjust

44. A client with Parkinson disease becomes frustrated while buttoning a shirt and says to the practical nurse, "This is ridiculous. I can't do anything for myself anymore." Which response by the nurse best provides emotional support?

- A.** "Let me finish buttoning it for you so you do not get upset."
- B.** "You are still able to do many things, so try not to focus on this."
- C.** "It sounds like losing independence is really upsetting for you."
- D.** "Buttoning can be hard with Parkinson disease because of the tremors."

45. A client with newly diagnosed epilepsy tells the nurse, "Every time I think about having another seizure, I panic and cancel my plans for the day." Which nursing response is most appropriate to promote effective coping?

- A.** "Try not to think about it, because stress can make seizures worse."
- B.** "Tell me what situations make you most likely to cancel your plans so we can identify one step you feel able to manage."
- C.** "You need to continue your normal routine so you do not become dependent on others."
- D.** "Most people adjust quickly once they start medication, so this feeling should pass soon."

46. A nurse in an inpatient behavioral health unit is speaking with a client who has been arguing with peers. The client suddenly steps very close to the nurse, points a finger, and says, "You'd better back off before I do something bad." Which action should the nurse take first?

- A.** Tell the client that threats are unacceptable and will result in seclusion
- B.** Move to maintain personal space and speak to the client in a calm, clear manner
- C.** Turn away to reduce stimulation and give the client time alone
- D.** Ask another client to leave the area while continuing the conversation

47. A client on a medical unit tells the nurse, "I use heroin every day, but I want to stop." Twelve hours after the last use, the client is yawning frequently, has abdominal cramping, watery eyes, and gooseflesh. Which nursing statement is most appropriate?

- A.** These symptoms are expected with opioid withdrawal, and treatment can focus on comfort and monitoring.
- B.** You are likely developing delirium tremens and need immediate seizure precautions.
- C.** These symptoms mean the drug is still in your system and the withdrawal process has not started.
- D.** Opioid withdrawal is usually fatal without emergency airway support.

48. A client with moderate Alzheimer disease becomes agitated during the late afternoon, walks quickly toward the exit, and says, "My children will be home from school soon." The client is not threatening others but ignores repeated statements that the children are adults. Which nursing action is most appropriate?

- A.** Firmly tell the client to sit down and stop trying to leave the unit
- B.** Ask the client to explain why leaving is unnecessary and discuss the current date

- C.** Walk with the client and say, "You sound worried about your children. Let's sit together and talk while we wait."
- D.** Place the client in a quiet room alone until the agitation stops

49. A client who recently experienced a miscarriage tells the nurse, "I don't want to see anyone right now. They all keep saying things that make me feel worse." Which response by the nurse is most therapeutic?

- A.** "You need to let your family support you during this time."
- B.** "At least you know you can become pregnant."
- C.** "It sounds like being around others feels overwhelming right now."
- D.** "Try not to focus on what people say; they are only trying to help."

50. A nurse is caring for a client in a long-term care facility who becomes increasingly upset during a noisy group activity. The client starts yelling, "Turn that off! I can't think!" and covers both ears. The client is standing but not threatening anyone. Which action by the nurse is most appropriate?

- A.** Tell the client that everyone must follow the activity schedule and encourage staying in the room
- B.** Guide the client to a quieter area and speak in a calm, simple manner
- C.** Ask several staff members to stand nearby in case the client becomes violent
- D.** Explain that yelling is inappropriate and ask the client to apologize to the group

51. An older adult with moderate dementia is becoming more confused during meals and often stops eating after a few bites. The practical nurse notices the client is seated in a noisy dining room near a television and several other residents who are talking loudly. Which intervention is most appropriate to support the client's food intake?

- A.** Move the client to a quieter area with fewer distractions during meals
- B.** Offer several different foods at once to increase the client's interest
- C.** Ask different staff members to encourage the client throughout the meal
- D.** Remind the client repeatedly that finishing the meal is necessary

52. A client tells the nurse, "I've been under a lot of stress lately, so I started writing down what situations seem to trigger it and how I react each time." Which response by the nurse is most appropriate?

- A.** "That is a useful first step because it can help you identify patterns and choose better coping strategies."

- B.** "You should stop focusing on the stressors so you do not make them worse."
- C.** "It is better to wait until the stress goes away before trying to analyze it."
- D.** "Tracking stress is only helpful if you also avoid talking to anyone about it."

53. A client who identifies as transgender tells the nurse, "I stopped going to my last clinic because they kept arguing with me about who I am." The client is due for several follow-up appointments after discharge. Which nursing action is best to support the client's psychosocial integrity and ongoing care?

- A.** Tell the client that all clinics must follow the same policies, so future visits should be easier
- B.** Ask the client what helped or made care harder in the past and document preferences the team can use for follow-up planning
- C.** Focus only on the client's physical discharge needs to avoid making the client more uncomfortable
- D.** Advise the client to bring a family member to future appointments to speak on the client's behalf

54. A client with newly diagnosed expressive aphasia after a stroke becomes tearful when unable to answer quickly and pushes away a meal tray. Which nursing action is best to support the client's psychosocial needs?

- A.** Finish the client's sentences to reduce frustration and keep communication moving
- B.** Ask only yes-or-no questions so the client will not have to struggle with speech
- C.** Allow extra time for responses and encourage the client to use alternate communication methods such as pointing or a communication board
- D.** Limit conversation until speech therapy begins to avoid increasing the client's anxiety

55. A client with terminal illness who is receiving end-of-life care tells the nurse, "I don't want any more treatments that just make me tired. I want to spend whatever time I have left awake enough to talk with my family." Which nursing action is most appropriate?

- A.** Tell the client that stopping treatment may cause the family to lose hope
- B.** Explore the client's goals and notify the provider so the plan of care can reflect the client's preferences
- C.** Encourage the client to wait until the next day before making any decisions about care
- D.** Explain that treatment decisions should be discussed only with the client's family

56. A nurse is caring for a client receiving continuous enteral feeding through a nasogastric tube. The client is scheduled for a chest x-ray and will be transported off the unit. Which action by the nurse is most appropriate during transport?

- A.** Clamp the tube and stop the feeding until the client returns
- B.** Lower the head of the bed to make the client more comfortable during movement
- C.** Ask transport staff to monitor the tube placement during the trip
- D.** Increase the feeding rate before transport so the client receives the full prescribed volume

57. A nurse is assisting a client who has weakness of the left leg to walk using a cane. Which instruction should the nurse give the client for correct use of the cane?

- A.** Hold the cane on the left side and move the cane forward with the left leg
- B.** Hold the cane on the right side and move the cane forward with the left leg
- C.** Hold the cane on the right side and move the cane forward after the right leg steps
- D.** Hold the cane on the left side and advance the right leg with the cane

58. A nurse is assisting in the care of a client who has had a stroke and reports that food "gets stuck" in the throat during meals. Which action is most appropriate to promote safer nutrition intake?

- A.** Offer fluids through a straw with each bite of food
- B.** Place the client in a high-Fowler's position while eating
- C.** Encourage the client to tilt the head back when swallowing
- D.** Serve meals quickly to reduce fatigue during eating

59. A client with a stage II pressure injury on the coccyx reports discomfort when lying in bed. Which nonpharmacologic intervention should the nurse implement first to help reduce the client's pain?

- A.** Reposition the client at regular intervals and use pressure-relieving support surfaces
- B.** Massage the reddened skin around the coccyx for 5 minutes
- C.** Place the client flat on the back to avoid movement-related discomfort
- D.** Limit repositioning to once per shift to prevent disturbing the wound

60. A nurse is caring for a client with moderate Alzheimer disease who is able to perform parts of bathing but becomes distracted and stops after washing one arm. Which nursing action is most appropriate to help the client complete this activity of daily living?

- A.** Finish the bath for the client to avoid frustration

- B.** Provide short, step-by-step cues during the bath
 - C.** Explain the entire bathing process before starting
 - D.** Ask the client which bathing supplies should be used first
- 61.** A nurse is caring for a client who has a spinal cord injury with an upper motor neuron bladder. Which intervention is most appropriate to help the client initiate urination?
- A.** Encourage the client to drink less fluid in the evening
 - B.** Use the Crede maneuver over the bladder
 - C.** Stroke the inner thigh or gently tap the suprapubic area
 - D.** Teach the client to perform Kegel exercises several times daily
- 62.** A nurse is caring for a client who states, "I go to bed at 9 p.m., but I usually lie there checking messages on my phone until I fall asleep." The client reports feeling tired each morning. Which teaching should the nurse provide?
- A.** Use the phone in bed only with the screen brightness turned down
 - B.** Move phone use to an earlier time and keep the bed for sleep rather than screen time
 - C.** Stay in bed longer each morning to make up for lost sleep
 - D.** Turn on the television after getting into bed so the room feels less quiet
- 63.** A practical nurse is caring for a client with diabetes who is on bed rest and has a small pressure injury on the lateral ankle. Which finding should the nurse report immediately because it suggests the wound may be becoming infected?
- A.** A small amount of clear drainage on the dressing
 - B.** Pale pink tissue in the wound bed
 - C.** Increasing warmth, swelling, and foul-smelling drainage at the site
 - D.** Mild discomfort when the wound is cleaned
- 64.** A practical nurse is reinforcing foot-care teaching for a client who has peripheral neuropathy and impaired temperature sensation. Which statement by the client indicates a need for further teaching?
- A.** I will test bath water with my elbow before getting in.
 - B.** I will wear well-fitting shoes even when I am indoors.
 - C.** I will use a heating pad on my feet if they feel cold.
 - D.** I will check my feet every day for redness or blisters.
- 65.** A client is on strict intake and output. Over the past 8 hours, the client received 500 mL of IV fluids, drank 3 oz of cranberry juice, ate 1 cup of gelatin, and had 420 mL of urine and 80 mL of emesis. What is the client's fluid balance for this 8-hour period?
- A.** +90 mL
 - B.** +170 mL
 - C.** +330 mL
 - D.** +590 mL
- 66.** A nurse is preparing to administer a scheduled potassium chloride tablet to a client. After checking the MAR, the nurse enters the room and finds the client asleep. The nurse notices a labeled medication cup on the bedside table from the previous shift containing an unlabeled white tablet. Which action is most appropriate?
- A.** Ask the client after awakening whether the tablet is a usual home medication, then give both tablets if the client agrees
 - B.** Compare the tablet's appearance with an online pill identifier and administer it if it matches the MAR
 - C.** Discard the unlabeled tablet according to facility policy and administer only the medication the nurse prepared and verified
 - D.** Leave the newly prepared potassium chloride at the bedside and return later when the client wakes up
- 67.** A practical nurse is preparing a client's 0800 medications and notices that the MAR lists amoxicillin 500 mg PO every 8 hours. The provider's most recent prescription in the chart, entered overnight after a culture result, states, "Discontinue amoxicillin. Start doxycycline 100 mg PO every 12 hours." No doxycycline appears on the MAR yet, and the next antibiotic dose is due now. What is the nurse's best action?
- A.** Administer the amoxicillin because it is the medication currently listed on the MAR
 - B.** Hold the antibiotic dose and notify the charge nurse after the medication pass is finished
 - C.** Clarify the discrepancy and have the MAR updated before administering any antibiotic
 - D.** Administer doxycycline based on the provider prescription even though it is not yet on the MAR
- 68.** A nurse is preparing to administer an antipyretic to a child who is vomiting and refuses to take anything by mouth. The provider wants the medication absorbed systemically without using an injection. Which route of administration is most appropriate?
- A.** Rectal
 - B.** Topical
 - C.** Intradermal
 - D.** Inhalation

69. A client with severe anemia who declines transfusion for personal reasons asks the nurse what benefit albumin would provide instead. Which response by the nurse is most accurate?

- A.** Albumin increases hemoglobin levels by supplying red blood cells
- B.** Albumin expands intravascular volume but does not improve oxygen-carrying capacity
- C.** Albumin provides clotting factors that rapidly correct anemia-related fatigue
- D.** Albumin stimulates the bone marrow to produce platelets and red blood cells

70. A nurse is preparing to infuse 550 mL of IV solution over 11 hours using an infusion pump. What rate in mL/hr should the nurse set?

- A.** 45 mL/hr
- B.** 50 mL/hr
- C.** 55 mL/hr
- D.** 60 mL/hr

71. A practical nurse is caring for a client who received a controlled opioid 20 minutes ago for postoperative pain. The client now says, "That medicine barely helped. Can you give me another dose now?" The prescription is for the medication every 4 hours as needed. Which action should the nurse take?

- A.** Explain that the medication cannot be repeated yet and reassess the client's pain, respirations, and sedation level
- B.** Administer half of the prescribed dose now because the client still reports pain
- C.** Ask another nurse to remove a second dose from the dispensing system for faster relief
- D.** Document that the medication was ineffective and discard the next dose

72. A nurse is caring for a client who had surgery this morning and is receiving IV opioid medication through a patient-controlled analgesia (PCA) pump. The client says, "I'm hurting now, so I'm going to wait until the pain gets really bad before I press the button a few times." Which response by the nurse is best?

- A.** "That is a good plan because the pump works best for severe pain."
- B.** "Press the button when pain begins, because it is usually easier to control pain early than after it becomes severe."
- C.** "Ask a family member to press the button for you if you are starting to feel sleepy."
- D.** "Try not to use the pump unless the provider changes the prescription."

73. A nurse is preparing to administer the next dose of lithium carbonate to a client with bipolar disorder. Which pre-administration data is most important for the nurse to review?

- A.** The client's most recent serum lithium level
- B.** The client's bowel movement pattern for the past 24 hours
- C.** The client's daily protein intake
- D.** The client's pupillary response to light

74. A client with Parkinson disease started taking carbidopa-levodopa 2 weeks ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client reports fewer hand tremors and improved ability to walk
- B.** The client sleeps 10 hours each night without awakening
- C.** The client's blood pressure decreases from 148/86 mm Hg to 118/72 mm Hg
- D.** The client reports a decreased appetite but no nausea

75. A client with a urinary tract infection is prescribed trimethoprim-sulfamethoxazole. Before administering the next dose, the nurse reviews the laboratory results and notes a white blood cell count of 2,100/mm³. Which action is most appropriate?

- A.** Administer the medication as prescribed and recheck the white blood cell count in 24 hours
- B.** Hold the medication and notify the provider of the leukopenia
- C.** Give the medication with food to decrease gastrointestinal upset
- D.** Ask the client whether the medication has caused a rash

76. A nurse is teaching a client who is starting transdermal fentanyl patches for chronic cancer pain. Which instruction should the nurse include?

- A.** Apply a heating pad over the patch if the pain increases
- B.** Cut the patch in half if the prescribed dose feels too strong
- C.** Place the patch on intact skin and rotate application sites
- D.** Remove the patch each night and apply a new one each morning

77. A nurse is preparing medications for a home health client and finds a bottle of sublingual nitroglycerin tablets that the client transferred into a weekly plastic pill organizer for convenience. Which action by the nurse is most appropriate?

- A.** Tell the client to keep using the organizer as long as the tablets stay dry
- B.** Return the tablets to their original container and teach the client to store them there

- C.** Refrigerate the pill organizer between doses to help preserve the tablets
- D.** Discard the tablets only if they are chipped, discolored, or expired
- 78.** A nurse is preparing to administer regular insulin 18 units subcutaneously. The vial available is labeled 100 units/mL. How many milliliters should the nurse draw up?
- A.** 0.18 mL
- B.** 0.8 mL
- C.** 1.8 mL
- D.** 0.35 mL
- 79.** A practical nurse is collecting data on a client who is 2 days postpartum. Which finding should the nurse identify as the strongest risk factor for developing a venous thromboembolism?
- A.** Lochia rubra with a mild earthy odor
- B.** Ambulating in the hallway twice during the shift
- C.** Cesarean birth followed by prolonged bed rest
- D.** Urinary frequency related to increased fluid shifts
- 80.** A nurse is preparing to collect a blood specimen for blood cultures from a client who has newly developed chills and a temperature of 39.2 C (102.6 F). Which action is most appropriate when obtaining the specimen?
- A.** Draw the specimen from the existing peripheral IV tubing after stopping the infusion for 5 minutes
- B.** Clean the tops of the culture bottles and the venipuncture site with antiseptic before collecting the specimen
- C.** Collect one culture bottle now and the second bottle after the first dose of antibiotics is given
- D.** Use sterile gloves only and avoid skin antisepsis so normal flora are not disturbed
- 81.** A nurse is reviewing arterial blood gas results for a client who has had several hours of rapid, deep breathing after a panic episode. The results are pH 7.49, PaCO₂ 30 mm Hg, and HCO₃⁻ 24 mEq/L. How should the nurse interpret these findings?
- A.** Respiratory alkalosis
- B.** Respiratory acidosis
- C.** Metabolic acidosis
- D.** Metabolic alkalosis
- 82.** A client who is 16 weeks pregnant calls the clinic and says, "I've had a fever since last night, and now I'm having pain low in my belly with a foul-smelling vaginal discharge." Which response by the nurse is most appropriate?
- A.** Advise the client to rest, increase fluids, and keep the next prenatal appointment
- B.** Instruct the client to go to the emergency department or provider immediately for evaluation
- C.** Teach the client that these discomforts are common in the second trimester
- D.** Recommend using a heating pad on the lower abdomen and calling back tomorrow if symptoms continue
- 83.** A nurse is performing bedside fecal occult blood testing on stool from a client who has bleeding gums from severe gingivitis. Which action is most important to help avoid a misleading point-of-care result?
- A.** Collect the specimen from stool that is visibly mixed with bright red blood from the gums
- B.** Obtain the stool specimen after the client rinses the mouth and avoids swallowing blood
- C.** Use a larger stool sample than usual to improve test sensitivity
- D.** Delay testing until the stool has dried completely
- 84.** A practical nurse is collecting vital signs on an adult client who is receiving oxygen by nasal cannula. The pulse oximeter repeatedly shows 86%, but the client is sitting up, talking in full sentences, and has warm hands with no cyanosis. Which action should the nurse take first?
- A.** Increase the oxygen flow rate immediately
- B.** Reposition the sensor and check the waveform or signal quality
- C.** Document the oxygen saturation as obtained
- D.** Notify the charge nurse that the client is severely hypoxic
- 85.** A client is scheduled for a pulmonary function test this afternoon. Which statement by the client requires the nurse to notify the provider before the test?
- A.** I used my rescue inhaler about 30 minutes ago because I felt short of breath.
- B.** I had a small sip of water with my morning medications.
- C.** I wore a loose shirt so I can breathe comfortably during the test.
- D.** I brought my list of home medications with me.
- 86.** A nurse is reviewing a client's ECG strip and notes that each QRS complex is preceded by a P wave, the rhythm is regular, and the ventricular rate is 48/min. Which rhythm should the nurse identify?
- A.** Sinus bradycardia

- B. Sinus tachycardia
- C. Atrial fibrillation
- D. Ventricular tachycardia

87. A nurse is preparing to obtain a venous blood specimen for a fasting glucose test from an adult client. The nurse cleans the selected site with alcohol and allows it to dry completely. Which action is most appropriate next to reduce contamination of the site?

- A. Repalpate the vein with a bare gloved finger just before insertion
- B. Insert the needle without touching the cleansed site again
- C. Fan the area with a glove to speed drying and then anchor the vein over the site
- D. Apply the tourniquet tighter to make the vein easier to feel through the cleaned skin

88. A practical nurse is caring for a client 4 hours after an abdominal hysterectomy. The client is awake, reports pain of 6/10, and has a prescription for an as-needed opioid analgesic. Which assessment finding is most important for the nurse to report to the RN before administering the medication?

- A. Blood pressure 138/84 mm Hg
- B. Respiratory rate 10/min
- C. Temperature 99.1°F (37.3°C)
- D. Surgical dressing with a 2-cm area of old drainage

89. A nurse is caring for a client with a chest tube after a spontaneous pneumothorax. The client suddenly becomes restless and says, "It's harder to breathe now." Which focused assessment should the nurse perform first?

- A. Check the client's oxygen saturation and assess breath sounds
- B. Inspect the client's meal tray to estimate fluid intake
- C. Ask the client to rate incisional pain on a 0 to 10 scale
- D. Review the client's smoking history in the chart

90. A client with pulmonary edema receives IV furosemide. Which finding 2 hours later best indicates the treatment has been effective?

- A. Urine output of 900 mL and decreased crackles in the lung bases
- B. Blood pressure decreased from 168/94 mm Hg to 150/88 mm Hg and pulse increased to 116/min
- C. Oxygen saturation remains 90% on 2 L/min by nasal cannula and the client reports fatigue
- D. The client has dry mucous membranes and reports feeling thirsty

91. A client 2 days after a subtotal gastrectomy suddenly reports severe abdominal pain. The nurse finds the abdomen rigid and distended, temperature 38.9°C (102°F), heart rate 124/min, blood pressure 88/54 mm Hg, and respirations 28/min. Which complication should the nurse suspect?

- A. Peritonitis from an anastomotic leak
- B. Dumping syndrome
- C. Postoperative atelectasis
- D. Paralytic ileus

92. A client with long-term hypothyroidism takes levothyroxine each morning. Which client statement indicates correct understanding of how to take this medication?

- A. I will take it with my calcium supplement so I do not forget either one.
- B. I will stop taking it when my energy level returns to normal.
- C. I will take it on an empty stomach at the same time each day.
- D. I will take an extra dose on days when I feel more tired than usual.

93. A client 8 hours after a total laryngectomy becomes restless and signals that breathing is difficult. The nurse notes harsh respirations, decreasing oxygen saturation, and thick secretions visible at the tracheostomy stoma. Which action should the nurse take first?

- A. Suction the tracheostomy using sterile technique
- B. Raise the head of the bed to semi-Fowler position
- C. Administer prescribed IV pain medication
- D. Notify the health care provider immediately

94. A client has a chest tube connected to a drainage system after a pneumothorax. During assessment, the nurse finds the tubing lying in a dependent loop on the bed with drainage collecting in the loop. The client is breathing comfortably and oxygen saturation is unchanged. Which action should the nurse take first?

- A. Milk the chest tube tubing until the fluid moves into the drainage chamber
- B. Reposition the tubing to eliminate the dependent loop and allow drainage to flow by gravity
- C. Clamp the chest tube briefly while straightening the tubing
- D. Raise the drainage system onto the bed to keep the tubing from pulling

95. A nurse is caring for an older adult in a long-term care facility during a winter power outage. The client is wearing a light gown, is sleepy, and has cool skin. Vital signs are temperature 35.2°C (95.4°F), pulse 54/min, respirations 10/min, and blood pressure 100/64 mm Hg. Which action should the nurse take first?

- A. Offer the client warm oral fluids
- B. Apply warm blankets and increase the room temperature
- C. Assist the client to ambulate to stimulate heat production
- D. Massage the client's arms and legs to improve circulation

96. A nurse is assessing a client's wound 3 days after a drain was removed. Which finding should the nurse identify as most concerning and report to the provider?

- A. A small amount of clear pink drainage on the outer dressing
- B. Wound edges that are slightly swollen and tender
- C. A sudden increase in wound drainage with a bulging area beneath the incision
- D. Mild itching at the incision site

97. A nurse is reinforcing teaching for a client with a new colostomy who asks how to remove the ostomy pouch without injuring the skin. Which instruction should the nurse give?

- A. Pull the barrier off quickly to reduce discomfort
- B. Loosen the edge gently while supporting the skin and remove it slowly
- C. Scrub the skin with soap and a washcloth before removing the pouch
- D. Apply powder around the seal before peeling the pouch away

98. A nurse is caring for a client with type 1 diabetes mellitus who received the usual dose of rapid-acting insulin before breakfast. Thirty minutes later, the client is pale, diaphoretic, and says, "My heart is racing." Which additional finding should the nurse recognize as the priority change in condition?

- A. Blood glucose 52 mg/dL
- B. Urine output 200 mL over 4 hours
- C. Oral temperature 37.4°C (99.3°F)
- D. Blood pressure 138/84 mm Hg

99. A nurse is caring for a client with a history of heart failure who has gained 2.3 kg (5 lb) in 3 days. Which additional finding most strongly supports fluid volume excess?

- A. Flat neck veins when sitting upright
- B. Bounding peripheral pulses and crackles in both lung bases

C. Dry mucous membranes and poor skin turgor

D. Blood pressure 88/54 mm Hg with weak, thready pulse

100. A client with end-stage kidney disease is scheduled for hemodialysis this morning. Before transport arrives, the practical nurse notes which finding should be reported to the provider because it may require delaying dialysis until further evaluation?

- A. Blood pressure of 148/86 mm Hg
- B. Potassium level of 5.6 mEq/L
- C. Temperature of 101.8°F (38.8°C)
- D. Weight gain of 2 lb (0.9 kg) since the last treatment

Simulation Test 9 — Answer Key

1. C	26. C	51. A	76. C
2. A	27. B	52. A	77. B
3. C	28. A	53. B	78. A
4. B	29. C	54. C	79. C
5. C	30. A	55. B	80. B
6. B	31. C	56. A	81. A
7. A	32. B	57. B	82. B
8. C	33. A	58. B	83. B
9. C	34. B	59. A	84. B
10. B	35. B	60. B	85. A
11. C	36. A	61. C	86. A
12. B	37. A	62. B	87. B
13. B	38. B	63. C	88. B
14. C	39. B	64. C	89. A
15. B	40. B	65. B	90. A
16. C	41. B	66. C	91. A
17. C	42. A	67. C	92. C
18. C	43. C	68. A	93. A
19. C	44. C	69. B	94. B
20. B	45. B	70. B	95. B
21. C	46. B	71. A	96. C
22. A	47. A	72. B	97. B
23. C	48. C	73. A	98. A
24. C	49. C	74. A	99. B
25. B	50. B	75. B	100. C

Simulation Test 10

100 questions

1. During evening rounds in a skilled nursing facility, the practical nurse receives these reports from assistive personnel. Which report should the nurse respond to first?

- A.** A resident with Parkinson disease is coughing while eating dinner
- B.** A resident with chronic arthritis asks for help repositioning in bed
- C.** A resident with insomnia says the room is too warm
- D.** A resident with venous insufficiency asks when compression stockings can be removed

2. The practical nurse is reviewing four new prescriptions for assigned clients. Which prescription should the nurse question and clarify first because it presents the greatest immediate safety risk?

- A.** Administer a scheduled stool softener to a client who has not had a bowel movement in 2 days
- B.** Apply a heating pad to the lower back of a client with chronic muscle soreness
- C.** Give a beta blocker to a client whose apical pulse is 52/min
- D.** Provide a bedtime snack to a client with type 2 diabetes whose blood glucose is 110 mg/dL

3. A practical nurse is working with an assistive personnel (AP) on a medical unit. Which task is most appropriate for the nurse to assign to the AP at the start of the shift?

- A.** Check a newly admitted client's swallowing ability before breakfast
- B.** Assist a stable client with chronic arthritis to the bathroom using a gait belt
- C.** Teach a client how to use a newly prescribed walker
- D.** Evaluate whether a client's PRN pain medication relieved discomfort

4. A practical nurse delegates routine premeal capillary blood glucose checks to an experienced assistive personnel (AP) for several stable clients. Before lunch, the AP reports, "Mr. Ruiz's blood glucose is 52 mg/dL, but he says he feels fine. I was about to give him his tray." Which action by the practical nurse is best?

- A.** Tell the AP to give the tray and recheck the glucose after the meal
- B.** Ask the AP to document the result because the client has no symptoms
- C.** Assess the client immediately and follow the facility protocol for hypoglycemia

D. Instruct the AP to give the scheduled insulin now to cover the upcoming meal

5. A nurse is preparing an alert adult client for surgery when the client says quietly, "I do not want my spouse to make decisions for me if something happens. I want my brother to do that instead, but I never filled out any papers." What is the nurse's best response?

- A.** Tell the client that the spouse will automatically make all decisions, so there is no need to discuss it further
- B.** Ask the client to write the brother's name on the consent form for surgery
- C.** Provide information about advance directives and notify the appropriate person to help the client complete the needed documents
- D.** Advise the client to wait until after surgery to decide who should make decisions

6. A nurse is caring for an alert adult client with a new spinal cord injury. During rounds, the rehabilitation team discusses discharge to a facility two hours away because a bed is available immediately. After the team leaves, the client says, "My partner does not drive, and my child care depends on my mother nearby. If I go that far away, my family will not be able to participate in my rehab." What is the nurse's best action?

- A.** Document the client's concern and ask the case manager and rehabilitation team to reevaluate the discharge plan with the client's priorities in mind
- B.** Explain that bed availability is the main factor in placement decisions and encourage the client to accept the transfer
- C.** Tell the client to discuss the issue independently with the facility after transfer
- D.** Delay all discharge planning until the family can come to the hospital and make the decision

7. A practical nurse is riding the elevator to lunch when another nurse asks, "How is the client in room 214 doing after that new cancer diagnosis?" Several visitors are also in the elevator. Which response by the practical nurse is best?

- A.** Provide a brief update without using the client's name
- B.** Say that the discussion should wait until they are in a private area
- C.** Whisper the information so the visitors are less likely to hear
- D.** Answer only if the other nurse is assigned to the client

8. A practical nurse receives a printed assignment sheet for the shift that includes client names, room numbers, diagnoses, and precautions. At the end of the shift, the nurse notices the sheet is still in a uniform pocket while leaving the unit. Which action is most appropriate?

- A.** Take the sheet home and bring it back on the next shift for shredding

- B.** Throw the sheet into the regular trash can near the exit
- C.** Return to the unit and place the sheet in the designated secure disposal container
- D.** Keep the sheet if the nurse does not plan to show it to anyone

9. An adult client admitted for treatment of pneumonia is alert and has decision-making capacity. The client says, "I signed an advance directive years ago, but my wishes have changed since then." Which response by the nurse is most appropriate?

- A.** "Once an advance directive is signed, it cannot be changed."
- B.** "You can review and update it to reflect your current wishes."
- C.** "Your family must approve any changes before they can be made."
- D.** "Changes can only be made after discharge from the hospital."

10. A nurse is caring for a client who is scheduled for a high-risk procedure later that day. The client is alert, oriented, and says, "I want to make sure my treatment preferences are followed if something goes wrong. Can you help me complete an advance directive now?" The provider tells the nurse, "We don't have time for that before the procedure." Which action by the nurse is best?

- A.** Tell the client to wait until after the procedure because the schedule cannot be delayed
- B.** Ask the family to complete the form for the client so the procedure can stay on time
- C.** Support the client's request and follow facility process to address the advance directive before the procedure
- D.** Explain that informed consent for the procedure makes an advance directive unnecessary

11. A client is scheduled for an elective colonoscopy.

The consent form has been signed. During preprocedure care, the client says, "My neighbor told me this test can sometimes poke a hole in the bowel. No one mentioned that to me." Which action should the nurse take?

- A.** Ask the client to sign a new consent form after reviewing the instructions for bowel preparation
- B.** Explain that serious complications are rare and continue preparing the client for the procedure
- C.** Hold the procedure preparation and notify the provider that the client has questions about the risks
- D.** Tell the client that the procedure cannot be delayed now that the consent form is already signed

12. A nurse is preparing a client for a scheduled skin graft. The client is alert, oriented, and has already signed the consent form after speaking with the surgeon. The client's adult child pulls the nurse aside and says, "Do not tell my father that the graft might fail. He gets too anxious, and I don't want him changing his mind." Which action by the nurse is most appropriate?

- A.** Respect the adult child's request because family members may limit upsetting information
- B.** Ask the surgeon to speak only with the adult child about any additional risks
- C.** Tell the client only the postoperative instructions and avoid discussing the procedure further
- D.** Advocate for the client's right to receive complete information and notify the provider of the family's request

13. A client is being discharged 2 days after an exacerbation of chronic obstructive pulmonary disease. The client is prescribed inhalers, home oxygen, and a follow-up visit in 1 week. During discharge teaching, the client says, "I understand the medicines, but I'm worried because my electricity gets shut off sometimes." Which action by the nurse is most appropriate before discharge?

- A.** Advise the client to use the oxygen only at night to conserve the tank
- B.** Refer the client to social services or case management to address utility and home oxygen needs
- C.** Instruct the client to keep the oxygen flow rate higher when electricity is unavailable
- D.** Delay discharge teaching until the follow-up visit

14. A client is being discharged after an uncomplicated cesarean birth. The incision is healing well, and the newborn is rooming in. The provider prescribes routine follow-up and iron supplementation. During discharge planning, the client says, "I'm staying with my sister for now because my partner hurt me during the pregnancy, and he has been texting that he knows where I am." Which nursing action is most appropriate?

- A.** Ask the client to block the partner's number and continue with routine discharge
- B.** Tell the client to discuss the situation with the provider at the postpartum visit
- C.** Collaborate with the interdisciplinary team to make a safety-focused discharge plan and provide domestic violence resources
- D.** Request that the sister assume responsibility for all discharge teaching

15. A long-term care facility is trying to reduce delays in notifying providers about residents' new signs of infection. The team introduces a standardized communication form for nurses to use when calling the provider. After 1 month, audits show the form was used in 95% of eligible calls, but the average time from identifying concerning symptoms to receiving a provider response did not improve. Which conclusion is most appropriate?

- A.** The project was successful because staff consistently completed the new form
- B.** The new process was adopted, but it may not be addressing the main cause of the delay
- C.** The audits should be stopped because provider response time is outside the nursing team's control
- D.** The team should immediately require all staff to repeat documentation training

16. A practical nurse is caring for a client who started a new opioid pain medication this morning after hip surgery. By afternoon, the client is very drowsy, needs repeated prompting to stay awake during conversation, and is unable to participate in the scheduled occupational therapy session. Which information is most important for the nurse to communicate to the provider and occupational therapist?

- A.** The client said the room has been noisy since lunch
- B.** The client required assistance choosing a television channel earlier in the day
- C.** The client is difficult to arouse after the new medication and cannot safely participate in therapy
- D.** The client asked whether therapy could be moved to a later time tomorrow

17. A practical nurse is caring for a client whose oxygen saturation suddenly drops to 84%. The nurse finds that the oxygen tubing has become disconnected from the wall source and reconnects it immediately. The client's saturation rises to 95% within 1 minute, and the client says, "I feel much better." Which action should the nurse take next?

- A.** Document the event and the nurse's actions in the medical record, and complete any required incident report according to facility policy
- B.** Complete an incident report only, because the problem was corrected and the client recovered quickly
- C.** Document only the client's current stable condition to avoid creating legal risk for the nurse
- D.** Wait until the end of the shift to decide whether the event needs to be reported if no further symptoms occur

18. A practical nurse in a rehabilitation center notices that a nursing assistant forcefully transfers a client from wheelchair to bed, causing the client to cry out and later develop a fresh skin tear on the forearm. The assistant says, "The client always exaggerates, so don't make this a big issue." What is the nurse's best action?

- A.** Wait to see whether the injury worsens before documenting or reporting the event
- B.** Confront the assistant privately and decide whether to report based on the response
- C.** Ensure the client's immediate safety, document objective findings, and follow facility procedure for mandatory reporting
- D.** Ask the client whether the client wants the nurse to file a report before taking further action

19. A practical nurse is assigning tasks for the shift in a rehabilitation unit. Which task is most appropriate to delegate to an assistive personnel (AP)?

- A.** Reinforce teaching to a client who is learning to use a walker after hip surgery
- B.** Assist a client with chronic left-sided weakness to the bathroom using the client's usual gait belt and transfer technique
- C.** Assess a client who reports new tingling in both feet after a back procedure
- D.** Decide whether a client is ready to progress from a clear liquid diet to a regular diet

20. A practical nurse in a multispecialty clinic reviews interpreter-use data with the team. Clients who prefer a language other than English have a much higher rate of incomplete preprocedure teaching than other clients. Staff often document, "Family interpreted." Which change should the nurse recommend first to best promote equitable access to care?

- A.** Require staff to use qualified medical interpreters during teaching instead of relying routinely on family members
- B.** Translate discharge papers into English only so all staff can review the same forms
- C.** Limit teaching to the day of the procedure so fewer appointments need interpreters
- D.** Ask bilingual housekeeping staff to help when clinical staff are busy

21. A practical nurse in a primary care clinic is helping plan follow-up for clients with newly prescribed medications. Which client situation most clearly reflects a digital equity barrier to care?

- A.** A client says the pharmacy is 2 miles away and asks whether prescriptions can be mailed.
- B.** A client says the clinic posts lab results only in the patient portal, but the client has no internet service and shares one basic phone with the household.

- C.** A client says medication labels are hard to read without glasses.
- D.** A client says morning appointments are difficult because of a part-time job.

22. A nurse is preparing to administer a PRN oral pain medication to a client in a medical-surgical unit. The client says, "I'm Robert Hill, born on May 9, 1958." The identification band reads Robert Hill, 05/09/1958, and the medication administration record matches. Which identifiers did the nurse use to verify the client correctly?

- A.** Client's name and room number
- B.** Client's name and diagnosis
- C.** Client's name and date of birth
- D.** Client's date of birth and physician's name

23. A nurse is preparing to administer a scheduled IV antibiotic to a client who is confused and repeatedly gives the wrong birth year when asked. The client's identification band is present and legible. Which action is best for the nurse to take to verify the client's identity before giving the medication?

- A.** Ask the client's roommate to confirm the client's name
- B.** Use the identification band and compare it with the medication administration record
- C.** Rely on the room assignment because the client cannot answer accurately
- D.** Delay the medication until a family member arrives to identify the client

24. A nurse is preparing to use a PCA (patient-controlled analgesia) pump for a postoperative client. While reviewing the settings, the nurse notices the concentration programmed into the pump does not match the concentration printed on the medication syringe. Which action should the nurse take?

- A.** Start the infusion because the prescribed dose is correct and the client is reporting pain
- B.** Reprogram the pump to match the syringe concentration and begin the infusion
- C.** Stop setup of the pump and resolve the discrepancy according to facility policy before use
- D.** Ask another nurse to monitor the client closely while the current settings are used

25. A nurse is preparing to administer the first dose of a prescribed enteral feeding formula through a nasogastric tube. The client says, "I have a serious soy allergy and carry an epinephrine auto-injector at home." The ingredient list on the formula container includes soy protein isolate. What should the nurse do first?

- A.** Administer a small amount first to test the client's tolerance

- B.** Hold the feeding and clarify the formula order with the provider

- C.** Premedicate the client with an antihistamine before starting the feeding

- D.** Ask dietary services to bring the formula because it was already prescribed

26. A practical nurse on a long-term care unit sees an unlabeled spray bottle sitting on a windowsill in a resident's room. The bottle contains a blue liquid, and the resident's grandchild is visiting in the room. What should the nurse do first?

- A.** Move the bottle to the bedside table so staff can identify it later
- B.** Ask housekeeping whether the liquid is a cleaning product before taking action
- C.** Remove the bottle from the resident's reach and follow facility policy for unidentified chemicals
- D.** Leave the bottle in place because it is not being used at the moment

27. A client 12 hours after abdominal surgery becomes confused and repeatedly tries to pull out a peripheral IV. The client is not combative, settles when spoken to, and keeps saying, "I need my glasses." Which action by the nurse best follows the principle of least-restrictive restraint use?

- A.** Apply soft wrist restraints to prevent accidental IV removal
- B.** Move the IV to the client's dominant arm so it is less noticeable
- C.** Provide the client's glasses and continue calm reorientation and observation
- D.** Request a sedative prescription so the client will sleep

28. A nurse enters a client's room and sees that a small puddle of water has leaked from the sink onto the floor near the doorway. The client is in bed, no one has slipped, and the area has not yet caused harm. Which action by the nurse is most appropriate?

- A.** Wipe up the water, place a wet-floor sign if available, and report the near-miss according to facility policy
- B.** Wait until housekeeping makes rounds because no injury has occurred
- C.** Document only that the client remained safe because an incident did not occur
- D.** Move the client to another room before taking any other action

29. A nurse is preparing to transfer a client from the bed to a chair. The client is alert and cooperative, can bear full weight, and has steady balance. Which transfer method is most appropriate?

- A.** Use a full-body mechanical lift
- B.** Use a sit-to-stand lift

- C. Assist the client with a gait belt during a pivot transfer
- D. Use a transfer board

30. A practical nurse is assigned to care for a client with suspected pulmonary tuberculosis who has just arrived from the emergency department. Which action should the nurse take first to reduce the risk of transmission to others?

- A. Place a surgical mask on the client and transport the client to radiology for a chest x-ray
- B. Move the client to a private room and apply airborne precautions
- C. Put on sterile gloves before taking the client's vital signs
- D. Obtain a sputum specimen before placing the client in a room

31. A nurse is preparing to insert a urinary catheter for a client who is not on isolation precautions. The nurse expects contact with mucous membranes and urine but does not anticipate splashing. Which personal protective equipment is most appropriate?

- A. Sterile gloves only
- B. Sterile gloves and a gown
- C. Sterile gloves, mask, and goggles
- D. Sterile gloves, gown, mask, and face shield

32. A practical nurse is preparing to assist with a sterile dressing change. The nurse opens a sterile tray and places a sterile basin in the center of the field. To moisten gauze, the nurse begins pouring sterile saline from a newly opened bottle held about 6 inches above the basin. After a small amount is poured, the bottle tip touches the inside edge of the basin. Which action should the nurse take?

- A. Continue pouring because both the bottle and basin are sterile
- B. Set the bottle aside and obtain a new sterile bottle of saline
- C. Wipe the bottle tip with sterile gauze and continue using it
- D. Ask another nurse to hold the basin while the nurse finishes pouring

33. A practical nurse is caring for a client with suspected mumps. Which type of isolation precautions should the nurse implement in addition to standard precautions?

- A. Airborne precautions
- B. Droplet precautions
- C. Protective environment precautions
- D. Contact precautions

34. A practical nurse is preparing to obtain a sputum specimen from a client who has no isolation orders. The client is alert but has a frequent productive cough, and the nurse expects secretions may spray during collection. Which action best follows standard precautions?

- A. Wear gloves and a face shield during specimen collection
- B. Wear sterile gloves to prevent contamination of the specimen
- C. Place the client in a negative-pressure room before obtaining the sample
- D. Skip gloves if the specimen cup can be held by the client

35. A nurse is reinforcing teaching with a client who is 32 weeks pregnant about traveling by car for a family trip. Which instruction should the nurse include?

- A. Wear the lap belt high across the abdomen
- B. Limit fluids during the trip to reduce restroom stops
- C. Stop every 2 hours to walk and stretch
- D. Keep the seat in a fully reclined position

36. A client in active labor is dilated to 5 cm and requests IV pain medication. Which assessment finding should cause the nurse to question administering the medication at this time?

- A. The fetal heart rate baseline is 142/min with moderate variability
- B. The client rates contraction pain as 8 on a 0-to-10 scale
- C. The client's respiratory rate is 10/min
- D. The contractions are occurring every 4 minutes

37. A nurse is reinforcing teaching for a client who is 2 weeks postpartum after an uncomplicated vaginal birth. The client asks when it is safe to begin pelvic floor exercises. Which response by the nurse is best?

- A. Begin pelvic floor exercises now if they are comfortable, and gradually do them several times a day
- B. Wait until the postpartum checkup because pelvic floor exercises should not be started before 6 weeks
- C. Avoid pelvic floor exercises until vaginal bleeding has completely stopped
- D. Start pelvic floor exercises only after sexual activity has resumed without discomfort

38. During a well-child visit, a nurse asks the parent of a 4-year-old child about play patterns at home. Which parent statement describes behavior most consistent with expected development?

- A. My child usually plays next to other children but does not really interact with them.
- B. My child likes to make up pretend stories and assign roles when playing with friends.

C. My child mainly enjoys shaking toys and putting them in the mouth.

D. My child prefers simple peekaboo games and laughs when objects reappear.

39. A practical nurse is reinforcing teaching with the spouse of a client who gave birth 10 days ago. The spouse says, "She cries easily, says she feels overwhelmed, and worries she is not doing anything right, but she is still caring for the baby and sleeping when the baby sleeps." Which response by the nurse is most appropriate?

A. These feelings are common early after birth, but she should report symptoms that last more than 2 weeks or include thoughts of self-harm or harming the baby.

B. This behavior shows she is not ready for parenting and should let family members take over infant care completely.

C. These findings mean she has postpartum psychosis and needs immediate inpatient psychiatric treatment.

D. She should avoid talking about these feelings so they do not become worse.

40. A practical nurse is reinforcing teaching with a client after a community cholesterol screening. The client's result was higher than the recommended range, and the client says, "That means I definitely have heart disease." Which response by the nurse is best?

A. A high screening result means heart disease is already present and treatment should begin right away.

B. Screening results can identify possible risk, but the client needs follow-up evaluation to determine what the result means.

C. The result is not important unless the client is having chest pain or shortness of breath.

D. The client should repeat the same screening in several years instead of seeking follow-up now.

41. A nurse is obtaining a health history from an adult client who reports nausea. Which follow-up question best assesses the severity of this symptom?

A. When did the nausea begin?

B. What seems to make the nausea worse?

C. How would you rate the nausea on a scale from 0 to 10?

D. Have you had nausea like this before?

42. A practical nurse is admitting a client with chronic heart failure to a rehabilitation unit. To establish useful baseline physical data for later comparison, which information is most important for the nurse to obtain at admission?

A. The client's favorite foods and meal schedule at home

B. The client's usual daily weight range and presence of ankle swelling

C. The names of family members expected to visit

D. The client's preferred bedtime television program

43. A nurse is reinforcing discharge teaching for the parent of a 15-month-old child who received several routine immunizations at a well-child visit. Which instruction should the nurse include about the injection sites?

A. Apply a warm, moist compress to the sites if they become sore

B. Massage the sites for 5 minutes every hour while the child is awake

C. Cover the sites with airtight dressings for 24 hours

D. Clean the sites with alcohol every 2 hours for the rest of the day

44. A client who recently moved to an assisted-living apartment after repeated falls says to the practical nurse, "I know this place is safer, but I feel like I lost my independence." Which response by the nurse best provides emotional support?

A. You should focus on the fact that this move will prevent more injuries.

B. Many people your age have to make this adjustment, so your feelings are normal.

C. It sounds like this change has been difficult for you, and you are grieving the loss of your independence.

D. Try to stay busy and make friends here so you will not think about it as much.

45. A client who was diagnosed with breast cancer 3 days ago tells the nurse, "I keep replaying the word cancer in my head. I don't think I can handle all of this at once." Which response by the nurse best promotes effective coping?

A. You need to stay positive so you do not make the situation worse.

B. Let's focus on what feels most urgent to you right now and break it into smaller steps.

C. It is too early to worry because the provider has not discussed treatment yet.

D. You should avoid thinking about the diagnosis until you feel calmer.

46. A nurse in a substance-use treatment unit is assessing four clients at the start of the shift. Which client should the nurse identify as having the highest immediate risk for violence?

A. A client who avoids eye contact and says, "I'm upset, but I just want to be left alone."

B. A client who is wringing hands and asks repeatedly when the provider will arrive

C. A client who is standing rigidly, clenching fists, and says, "If he comes near me again, I'll take care of it myself."

D. A client who is crying loudly and states, "Nobody understands what I'm going through."

- 47.** A client tells the nurse, "I drink a fifth of liquor every day, but I want to quit on my own at home." Which client statement most strongly indicates the need for immediate medical evaluation before attempting to stop alcohol use?
- A.** "I usually cannot sleep the first night without drinking."
 - B.** "When I tried to stop before, I had a seizure."
 - C.** "I get very irritable and argue with my family."
 - D.** "I lose my appetite for a couple of days."
- 48.** A client on an inpatient mental health unit suddenly stands up during a community meeting, points at another client, and says loudly, "He keeps laughing at me!" The client is flushed, breathing fast, and taking several steps toward the other client. Which action should the nurse take first?
- A.** Tell the client that the other client was probably not laughing at them
 - B.** Position the nurse between the two clients and calmly direct the client to walk to a quiet area
 - C.** Ask the community group to explain that no one meant any harm
 - D.** Document the incident and notify the provider after the meeting ends
- 49.** A client who recently started hemodialysis says to the nurse, "I hate depending on a machine just to stay alive." Which response by the nurse is most therapeutic?
- A.** You will get used to the routine after a few treatments.
 - B.** It sounds like this treatment makes you feel a loss of control.
 - C.** Try to focus on the fact that dialysis is keeping you alive.
 - D.** Many clients feel this way, so you are not alone.
- 50.** A nurse approaches a client who has become increasingly upset after a phone call. The client is standing in the room, breathing rapidly, and saying in a loud voice, "Stop asking me questions." Which action by the nurse is most appropriate to help de-escalate the situation?
- A.** Stand directly in front of the client and explain that the behavior is unacceptable
 - B.** Lower the voice, give the client personal space, and offer to talk when the client is ready
 - C.** Tell the client that medication will be needed if the client does not calm down immediately
 - D.** Ask several questions at once to quickly identify the reason for the client's anger
- 51.** A client with moderate dementia becomes upset while looking for a wallet and insists, "Someone here stole my money." The client is breathing rapidly and searching through drawers. Which response by the practical nurse is most appropriate?
- A.** "No one stole anything. You must have forgotten where you put it."
 - B.** "You are safe. Let's look together in the places you usually keep it."
 - C.** "If you keep accusing people, you may lose visiting privileges."
 - D.** "Your family already took care of your finances, so you do not need your wallet."
- 52.** A client tells the nurse, "When I get overwhelmed, I keep replaying everything that could go wrong. I need a way to calm down that I can use anywhere." Which nursing recommendation is most appropriate?
- A.** Set aside time each day to review every possible worst-case outcome
 - B.** Practice slow deep-breathing exercises when stress begins to build
 - C.** Avoid talking about the stressful situation until it is completely resolved
 - D.** Wait to address the stress until physical symptoms become severe
- 53.** A client who is scheduled for surgery later today tells the practical nurse, "Before any procedure, my tradition requires a short blessing from a family elder. My daughter is on her way, but I am afraid the team will take me before she gets here." Which action by the nurse is most appropriate?
- A.** Tell the client that hospital routines must take priority over personal rituals
 - B.** Document the request and promptly notify the charge nurse or surgical team so the client's need can be considered in planning care
 - C.** Advise the client to skip the ritual this time because it is not medically necessary
 - D.** Ask another staff member to explain to the client why spiritual practices cannot delay procedures
- 54.** A client with newly diagnosed severe hearing loss becomes visibly tense whenever staff enter the room and says, "I get nervous because I never know who is talking or what they want. I feel on edge all the time." Which nursing action is best to reduce the client's anxiety?
- A.** Approach the client from behind to avoid startling the client, then speak in a louder voice
 - B.** Limit conversation to essential care tasks until the client adjusts to the hearing loss
 - C.** Gain the client's attention before speaking and face the client during communication

D. Encourage the client's family to answer questions so the client does not become overwhelmed

55. A client with terminal cancer is awake and alert during evening care. The client tells the nurse, "My family keeps changing the subject when I try to talk about what will happen after I die. I think they are trying to protect me, but it makes me feel more alone." Which response by the nurse is most therapeutic?

- A.** "They probably mean well, so try not to take it personally."
- B.** "Tell me more about what you would like your family to understand right now."
- C.** "You should focus on staying calm and not upsetting your family."
- D.** "Most families have a hard time discussing death, but this feeling will pass."

56. A nurse is reinforcing teaching for a client who will use a wheelchair at home after a spinal cord injury. Which client action indicates correct understanding of safe wheelchair mobility?

- A.** Locks the wheels before transferring in or out of the chair
- B.** Leans far forward in the chair when reaching for objects on the floor
- C.** Keeps footrests down while standing up from the chair
- D.** Pushes the wheelchair quickly over thresholds to avoid getting stuck

57. A nurse is reinforcing dietary teaching for a client who has acute diarrhea. Which food choice should the nurse recommend?

- A.** Bran cereal with skim milk
- B.** Baked chicken with brown rice and broccoli
- C.** Bananas and dry toast
- D.** Cheeseburger with french fries

58. A client with acute low back strain says, "My pain gets worse when I need to turn in bed." Which nonpharmacologic nursing intervention is most appropriate to reduce pain during repositioning?

- A.** Place a pillow under the client's knees and use proper body alignment while assisting with logrolling
- B.** Encourage the client to remain flat in bed without moving for the rest of the shift
- C.** Massage the painful area vigorously before each position change
- D.** Apply a cold pack directly to bare skin for 30 minutes before turning

59. A nurse is caring for a client who had abdominal surgery yesterday. The client is alert, reports incisional pain of 6 out of 10, and says, "I want to wash my face and brush my teeth myself, but it hurts to move." Which action by the nurse best supports the client's ability to perform these activities of daily living?

- A.** Complete all morning hygiene for the client to prevent discomfort
- B.** Administer prescribed pain medication, then return later to assist the client as needed with self-care
- C.** Tell the client to finish hygiene now before the pain becomes worse
- D.** Ask the client to remain on bed rest and skip hygiene until the next shift

60. A nurse is caring for a client who has a new prescription for a bladder-training program after prolonged use of an indwelling urinary catheter. Which outcome best indicates the program is effective?

- A.** The client voids at scheduled intervals with gradually increasing time between voids
- B.** The client drinks less fluid to avoid the urge to urinate
- C.** The client remains incontinent between voiding attempts
- D.** The client requests catheter reinsertion at bedtime

61. A nurse is caring for a client who reports waking frequently during the night because the room feels too warm and the bed linens feel damp. Which intervention should the nurse implement first to promote rest?

- A.** Offer the client a high-protein snack before bedtime
- B.** Change the bed linens and adjust the room temperature if possible
- C.** Encourage the client to stay awake longer the next evening
- D.** Suggest watching television until the client feels sleepy

62. A practical nurse is caring for a client who has been on bed rest for 1 week. During skin assessment, the nurse finds a dark purple, intact area on the client's heel. The area is painful and feels boggy compared with surrounding tissue. Which interpretation is most appropriate?

- A.** This is a stage 1 pressure injury because the skin is still intact
- B.** This finding is most consistent with a deep tissue pressure injury
- C.** This is a stage 2 pressure injury because heel tissue is fragile
- D.** This finding is caused by moisture-associated skin damage

63. A practical nurse is assisting with discharge planning for an older adult who has moderate hearing loss and mild memory problems. Which strategy is most likely to improve the client's understanding of home-care instructions?

- A. Provide the instructions only as a detailed verbal explanation at the end of the visit
- B. Face the client, speak clearly in short phrases, and give written instructions to review afterward
- C. Speak loudly from the doorway so the client can hear the full explanation without interruption
- D. Ask a family member to listen to the instructions instead of teaching the client directly

64. A client is on strict intake and output after abdominal surgery. During the shift, the client receives 500 mL of packed red blood cells, 80 mL of IV piggyback antibiotic solution, and drinks 120 mL of water. The client has 450 mL of urine in the catheter bag and 90 mL of drainage from a nasogastric tube. Which fluid balance should the practical nurse document for the shift?

- A. +160 mL
- B. +250 mL
- C. -160 mL
- D. -250 mL

65. A practical nurse is caring for an immobilized client who is incontinent of urine and stool. Which action is most important to reduce the client's risk for skin breakdown?

- A. Massage reddened areas after each episode of incontinence
- B. Cleanse the skin promptly and apply a moisture barrier as needed
- C. Limit fluid intake to decrease the number of incontinence episodes
- D. Place the client in a high-Fowler position after each linen change

66. A nurse prepares to give a client's 0900 medications and notices one tablet is not listed on the MAR, even though it was dispensed in the client's medication drawer. Which action is most appropriate?

- A. Administer the tablet because medications in the client's drawer have already been verified by the pharmacy
- B. Ask another nurse whether the client usually takes the tablet and then give it if confirmed
- C. Hold the tablet and clarify the discrepancy before administering any medication not listed on the MAR
- D. Give the tablet if the client recognizes it and says it is part of the usual morning medications

67. A practical nurse is preparing to administer a client's 0700 medications. The MAR lists lisinopril 10 mg PO daily and docusate sodium 100 mg PO daily. The nurse notices the client has a documented allergy to lisinopril that was entered during admission overnight, but the medication is still active on the MAR. The client says, "I told them that medicine made my lips swell before." What is the nurse's best action?

- A. Hold the lisinopril, administer the docusate, and notify the RN or provider about the active lisinopril order and MAR discrepancy
- B. Administer both medications because the provider originally prescribed them and the MAR still lists them as due
- C. Withhold all morning medications until the entire MAR is reviewed and rewritten
- D. Ask the client whether the swelling was severe enough to count as a true allergy before deciding

68. A nurse is preparing to administer a medication for an adult patient who has severe vomiting and confusion. The provider wants the drug absorbed quickly and notes that the patient has poor peripheral circulation from shock. Which route of administration should the nurse expect to be most reliable?

- A. Subcutaneous
- B. Intramuscular
- C. Intravenous
- D. Topical

69. A client with hemophilia A is bleeding after a dental extraction. Which prescribed blood product or blood-derived therapy should the nurse expect to administer to replace the missing clotting factor?

- A. Factor VIII concentrate
- B. Packed red blood cells
- C. Platelet transfusion
- D. Albumin

70. A nurse has 400 mL of IV solution remaining to infuse over the next 5 hours. What rate should the nurse set on the infusion pump?

- A. 60 mL/hr
- B. 75 mL/hr
- C. 80 mL/hr
- D. 100 mL/hr

71. A practical nurse is completing a room transfer and finds a client-controlled analgesia (PCA) pump containing a controlled opioid connected to the client. Which action by the nurse is most appropriate during the transfer?

- A. Move the client first and document the controlled medication count after the transfer is complete

B. Have the receiving nurse verify the controlled medication amount with the transferring nurse according to facility procedure

C. Ask the unlicensed assistive personnel to watch the pump while the nurse transports the client

D. Disconnect the PCA pump during transport and reconnect it after the client arrives in the new room

72. A nurse is caring for four postoperative clients who are using pain-control devices. Which client should the nurse assess first?

A. A client with a transdermal opioid patch who reports mild itching at the application site

B. A client with a PCA pump who reports nausea after receiving several doses

C. A client with epidural analgesia who has new lower-extremity weakness and a blood pressure of 84/50 mm Hg

D. A client with a regional nerve block who reports pain of 6 on a 0-to-10 scale during physical therapy

73. A nurse is preparing to administer the next dose of apixaban to a client who developed atrial fibrillation after surgery. Which pre-administration data is most important for the nurse to review?

A. The client's most recent hemoglobin and hematocrit levels

B. The client's bowel movement pattern over the last 24 hours

C. The client's daily oral fluid intake

D. The client's capillary refill time

74. A client with heart failure has been taking spironolactone for 10 days. Which finding best indicates the medication is producing the intended therapeutic response?

A. The client reports a 4-lb weight loss and decreased ankle edema

B. The client's apical pulse decreases from 96/min to 72/min

C. The client's fasting blood glucose decreases from 168 mg/dL to 110 mg/dL

D. The client reports less burning epigastric pain after meals

75. A client with atrial fibrillation has been taking amiodarone for 2 months. Which new finding should the nurse report to the provider promptly as a possible adverse response to this medication?

A. Increasing shortness of breath with a nonproductive cough

B. Mild constipation after meals

C. Occasional drowsiness in the evening

D. Temporary soreness at a previous IV site

76. A nurse is teaching a client who has a new prescription for warfarin. Which instruction should the nurse include?

A. Increase intake of green leafy vegetables to improve the medication's effect

B. Use a soft toothbrush and report unusual bruising or bleeding

C. Take aspirin for mild pain unless bleeding occurs

D. Stop taking the medication when the prescribed tablets are finished

77. A nurse is preparing to administer IV diazepam through a client's existing peripheral line. The client is currently receiving 0.9% sodium chloride at 75 mL/hr. The nurse checks the compatibility reference and finds that diazepam is incompatible with the current IV solution. Which action is most appropriate?

A. Stop the IV, flush the line per protocol, administer the diazepam using the line as directed, and restart the maintenance fluid afterward

B. Lower the rate of the 0.9% sodium chloride infusion and administer the diazepam slowly through the same line

C. Mix the diazepam into the 0.9% sodium chloride bag to improve compatibility

D. Administer the diazepam through the line because incompatibility matters only if precipitation is visible

78. A nurse is preparing to administer vancomycin 750 mg IV. The pharmacy label states vancomycin 1 g in 200 mL. How many milliliters should the nurse infuse for one dose?

A. 100 mL

B. 150 mL

C. 175 mL

D. 225 mL

79. A practical nurse is collecting data on four clients in a community clinic. Which client should the nurse identify as being at greatest risk for an adverse effect from heat exposure during a summer power outage?

A. A client with hypertension who takes hydrochlorothiazide daily

B. A client with seasonal allergies who takes loratadine as needed

C. A client with gastroesophageal reflux disease who takes omeprazole each morning

D. A client with hypothyroidism who takes levothyroxine each day

80. A nurse is preparing to collect a wound culture from a client's draining pressure injury. Which action is most appropriate to obtain the specimen?

A. Swab the drainage found on the old dressing

- B.** Cleanse the wound with sterile solution, then swab the wound bed
- C.** Collect the specimen after applying topical antibiotic ointment
- D.** Use a swab to collect tissue from the wound edge without cleansing

81. A nurse is reviewing laboratory results for a client who has type 2 diabetes and reports excessive thirst, frequent urination, and fatigue. Which finding should the nurse identify as most consistent with poor long-term glucose control?

- A.** Hemoglobin A1c 9.2%
- B.** Fasting blood glucose 108 mg/dL
- C.** Urine specific gravity 1.010
- D.** White blood cell count 8,000/mm³

82. A client who is 29 weeks pregnant arrives at the prenatal clinic reporting a sudden gush of fluid from the vagina 1 hour ago and continued leaking since then. The client denies contractions. Which instruction should the nurse give first?

- A.** Place a clean perineal pad and come to the hospital for evaluation now
- B.** Lie flat in bed for 2 hours and call back if the leaking continues
- C.** Take a warm bath to see whether the leaking stops
- D.** Insert a tampon to absorb the fluid until the next office appointment

83. A nurse performs a bedside capillary blood glucose test for a client who is diaphoretic and trembling. The meter displays an error message indicating there was not enough blood on the strip. What should the nurse do next?

- A.** Document the result as unreadable and recheck at the next scheduled time
- B.** Repeat the test immediately with a new strip and a fresh capillary sample
- C.** Administer insulin because the result is invalid and hyperglycemia is likely
- D.** Send a urine specimen to the laboratory for glucose testing instead

84. A practical nurse is obtaining routine vital signs on an adult client. During assessment, the nurse counts an apical pulse of 104/min for 1 full minute. The radial pulse at the same time is 92/min. Which interpretation is most appropriate?

- A.** The difference is expected because the apical pulse is usually slightly faster than the radial pulse
- B.** The client has a pulse deficit that should be reported for further evaluation
- C.** The radial pulse is more accurate than the apical pulse and should be documented instead

D. The finding indicates the blood pressure cuff was too large and the pulse should be retaken later

85. A client is scheduled for a thyroid scan that may use an iodine-containing contrast agent. Which statement by the client requires the nurse to notify the provider before the procedure?

- A.** I had toast and coffee about 2 hours ago.
- B.** I stopped taking my multivitamin yesterday.
- C.** I have been taking propylthiouracil for hyperthyroidism.
- D.** I removed my necklace before coming to the department.

86. A nurse is applying telemetry electrodes to a client for continuous ECG monitoring. To place the electrodes correctly, where should the nurse position the client first?

- A.** Supine or semi-Fowler's position
- B.** Prone position
- C.** Trendelenburg position
- D.** Left Sims' position

87. A nurse is preparing to obtain a venous blood specimen from an adult client. To help the vein become more prominent before selecting a site, which action is most appropriate?

- A.** Ask the client to tightly clench and release the fist repeatedly for 30 seconds
- B.** Place the client's arm below heart level for a short time
- C.** Apply the tourniquet and leave it in place while gathering supplies
- D.** Vigorously tap the skin over several veins until one rises

88. A practical nurse is caring for a client who is scheduled for bowel surgery in the morning. During the evening shift, the client says, "I ate a sandwich that my family brought me because I was hungry." What is the nurse's best action?

- A.** Document the intake and continue routine preoperative care
- B.** Tell the client to avoid any more food or fluids and report the intake to the RN or provider
- C.** Administer the prescribed bedtime sedative so the client can rest before surgery
- D.** Proceed with the ordered bowel prep and say nothing unless the client develops nausea

89. A client has a nasogastric feeding tube in place and is receiving continuous enteral feeding. The nurse prepares to administer a liquid medication through the tube and notes that the client is lying flat after a recent dressing change. Which action is most appropriate before giving the medication?

- A.** Stop the feeding, place the client in a side-lying position, and administer the medication immediately

- B.** Raise the head of the bed to at least 30 degrees and verify tube placement per facility policy
- C.** Flush the tube with 60 mL of air to confirm patency before medication administration
- D.** Aspirate gastric contents and return them to the client before giving the medication
- 90.** A client with newly diagnosed primary open-angle glaucoma began prescribed timolol eye drops 1 month ago. Which finding best indicates the treatment is effective?
- A.** Reports less eye redness after instilling the drops
- B.** Intraocular pressure has decreased since therapy began
- C.** Visual acuity improved from 20/60 to 20/40
- D.** Pupils are equal and reactive to light
- 91.** A client 1 day after a hip fracture repair suddenly becomes anxious and reports sharp chest pain that worsens with breathing. The nurse notes heart rate 122/min, respiratory rate 28/min, and oxygen saturation 89% on room air. Which action should the nurse take first?
- A.** Raise the head of the bed and apply oxygen
- B.** Obtain a prescription for a chest x-ray
- C.** Encourage the client to cough and deep-breathe
- D.** Reassure the client that postoperative pain is expected
- 92.** A client with rheumatoid arthritis has had increasing joint stiffness and pain for several weeks. The client says, "I stopped doing my hand exercises because my joints hurt too much to move." Which response by the practical nurse is best?
- A.** "You should avoid using your hands until all pain is gone."
- B.** "Try to do the exercises only on days when your joints do not hurt."
- C.** "Gentle range-of-motion exercises can help maintain function, so let's talk about ways to do them safely."
- D.** "If the exercises cause pain, replace them with complete bed rest for a few days."
- 93.** A client receiving a blood transfusion becomes restless 10 minutes after the infusion starts and reports chills, low back pain, and difficulty breathing. Which action should the nurse take first?
- A.** Slow the transfusion rate and continue to monitor the client
- B.** Stop the transfusion and keep the IV line open with normal saline
- C.** Notify the health care provider after obtaining a complete set of vital signs
- D.** Administer prescribed PRN acetaminophen for the chills
- 94.** A client has a Salem sump tube connected to low intermittent suction after bowel surgery. During assessment, the nurse notes that the blue air vent is taped to the gown and a small amount of clear fluid is leaking from it. The client reports mild nausea. Which action should the nurse take first?
- A.** Irrigate the blue air vent with sterile saline
- B.** Clamp the nasogastric tube for 30 minutes and reassess nausea
- C.** Remove the tape and keep the blue air vent above the level of the stomach
- D.** Advance the tube 2 inches to improve drainage
- 95.** A nurse is caring for a client who is receiving a warming blanket after exposure to cold temperatures. Which finding indicates to the nurse that the client's thermoregulation is improving?
- A.** The client begins to shiver
- B.** The client reports feeling less drowsy and becomes more alert
- C.** The client's skin remains pale and cool
- D.** The client's heart rate decreases from 60/min to 48/min
- 96.** A nurse is assessing a client's wound on the third postoperative day. Which finding is most consistent with normal healing by secondary intention?
- A.** Wound edges are well approximated with minimal scar formation expected
- B.** A pink-red wound bed is filling in gradually from the bottom up
- C.** Visible separation of the incision with drainage of intestinal contents
- D.** A dry black covering over the wound bed with no granulation tissue present
- 97.** A nurse enters a client's room and finds the client slumped in bed and unresponsive. The nurse activates the emergency response system and asks another staff member to bring the crash cart. The client is not breathing normally but has a pulse of 64/min. Which action should the nurse take next?
- A.** Begin chest compressions at a rate of 100 to 120/min
- B.** Provide rescue breaths and recheck the pulse about every 2 minutes
- C.** Place the client in a side-lying position and wait for the code team
- D.** Apply the AED immediately and follow the prompts
- 98.** A nurse is caring for a client who is 2 days postpartum after a vaginal birth. The client tells the nurse, "I suddenly feel very weak and dizzy." Which additional finding should the nurse recognize as the most urgent change in condition?
- A.** Scant lochia rubra with a firm fundus

- B.** Perineal pain rated 6 on a scale of 0 to 10
- C.** A boggy uterus with heavy vaginal bleeding
- D.** Breast engorgement with tenderness during feeding

99. A nurse is caring for a client who has been receiving large-volume IV normal saline for 3 days after surgery. Which finding most strongly suggests fluid volume overload?

- A.** Flat neck veins when lying supine
- B.** Crackles heard at both lung bases
- C.** Poor skin turgor over the sternum
- D.** Urine specific gravity of 1.032

100. A client with end-stage kidney disease is receiving peritoneal dialysis. To promote drainage of the dialysate during the outflow phase, which action is most appropriate?

- A.** Warm the dialysate bag in a microwave before restarting the exchange
- B.** Reposition the client from side to side
- C.** Clamp the tubing for 30 minutes and then reopen it
- D.** Increase the prescribed infusion volume for the next cycle

Simulation Test 10 — Answer Key

1. A	26. C	51. B	76. B
2. C	27. C	52. B	77. A
3. B	28. A	53. B	78. B
4. C	29. C	54. C	79. A
5. C	30. B	55. B	80. B
6. A	31. A	56. A	81. A
7. B	32. B	57. C	82. A
8. C	33. B	58. A	83. B
9. B	34. A	59. B	84. B
10. C	35. C	60. A	85. C
11. C	36. C	61. B	86. A
12. D	37. A	62. B	87. B
13. B	38. B	63. B	88. B
14. C	39. A	64. A	89. B
15. B	40. B	65. B	90. B
16. C	41. C	66. C	91. A
17. A	42. B	67. A	92. C
18. C	43. A	68. C	93. B
19. B	44. C	69. A	94. C
20. A	45. B	70. C	95. B
21. B	46. C	71. B	96. B
22. C	47. B	72. C	97. B
23. B	48. B	73. A	98. C
24. C	49. B	74. A	99. B
25. B	50. B	75. A	100. B

Simulation Test 11

100 questions

1. A practical nurse on a medical-surgical unit is organizing care for four assigned clients. Which client should the nurse assess first?

- A.** A client with pneumonia whose oxygen saturation dropped from 95% to 90% on 2 L/min by nasal cannula
- B.** A client 1 day after a colonoscopy who requests a stool softener
- C.** A client with a healing leg ulcer who needs a routine dressing change
- D.** A client with chronic arthritis pain who asks for a PRN analgesic rated 5/10

2. At the start of the shift in a skilled nursing facility, the practical nurse receives these reports from the night nurse. Which client should the nurse follow up on first?

- A.** A client with Parkinson disease who slept poorly and is more tired than usual this morning
- B.** A client receiving furosemide who gained 2 lb (0.9 kg) since yesterday and now has new bilateral ankle edema
- C.** A client with cataracts who says the breakfast menu print is hard to read
- D.** A client with chronic constipation who had a small bowel movement during the night

3. A practical nurse delegated routine morning vital signs to an assistive personnel (AP) for stable clients on a step-down unit. The AP reports that one client's temperature is 38.9 C (102 F), pulse is 116/min, and blood pressure is 88/54 mm Hg. Which action by the practical nurse is best?

- A.** Assess the client immediately and verify the findings
- B.** Ask the AP to repeat the vital signs after the client rests for 30 minutes
- C.** Document the values and notify the provider after completing medication pass
- D.** Instruct the AP to give the client more oral fluids and recheck the temperature later

4. A practical nurse is assigning morning care on a medical-surgical unit to an assistive personnel (AP). Which task should the practical nurse perform rather than delegate to the AP?

- A.** Help a stable client dress after breakfast
- B.** Record the amount of urine in a bedside drainage bag for a stable client
- C.** Reposition a client who needs turning every 2 hours
- D.** Determine whether a newly admitted client can safely swallow oral medications

5. An alert adult client with chronic kidney disease tells the nurse, "I want to leave the hospital today, even if the provider thinks I should stay." The provider has not yet arrived to discuss the risks. What is the nurse's best action?

- A.** Tell the client that leaving is not allowed until the provider gives discharge orders
- B.** Ask the client to wait quietly until the provider can convince the client to stay
- C.** Explain the client's right to leave, notify the provider, and follow the facility process for leaving against medical advice
- D.** Have the client's family sign a form stating they accept responsibility for the decision

6. A nurse is caring for an adult client who is alert and oriented after a biopsy. The provider tells the nurse, "Do not tell the client the pathology result yet. The family asked that the client not be upset until they arrive tonight." The client then asks, "Did the report come back? I want to know exactly what it says." What should the nurse do?

- A.** Tell the client that the family requested privacy until they can be present
- B.** Avoid answering and tell the client to wait for the provider's next round
- C.** Ask the family whether they want the nurse or provider to explain the result first
- D.** Advocate for the client's right to receive information and promptly notify the provider that the client is requesting the result

7. A practical nurse is documenting in the electronic health record at the nurses' station and is called away to help another client urgently. Which action best protects client confidentiality?

- A.** Leave the chart open because the nurse will return in a few minutes
- B.** Ask a nearby staff member to watch the screen until the nurse returns
- C.** Log off or lock the computer before leaving the workstation
- D.** Minimize the chart window so others cannot read it easily

8. A practical nurse is caring for a client who was admitted after a motor vehicle crash. A police officer comes to the unit and says, "I need the client's blood alcohol result for my report." No consent form or release is on the chart, and the provider is not present. Which action by the nurse is most appropriate?

- A.** Give the result because law enforcement is involved in the case
- B.** Tell the officer the result only if the officer promises to keep it confidential

C. Refer the officer to the nurse manager or facility policy before releasing any information

D. Read the result aloud but avoid giving the officer a printed copy

9. During admission, an alert adult client says, "I never completed an advance directive because I thought only older people needed one." Which response by the nurse is most appropriate?

A. Advance directives are mainly recommended after age 65.

B. Only clients with terminal illness are eligible to complete an advance directive.

C. Any competent adult can complete an advance directive so future wishes are known if decision-making capacity is lost.

D. An advance directive is only needed before surgery or other high-risk procedures.

10. A nurse is caring for a client who is alert and has decision-making capacity. The client says, "My brother and I are arguing about my care, so I want to name one person to make decisions for me only if I become unable to decide later." Which document best addresses this goal?

A. Durable power of attorney for health care

B. Incident report

C. Informed consent form

D. Provider prescription for restraints

11. A client is scheduled for an elective renal biopsy. The client asks the nurse, "What exactly is your job in this consent process? Are you the one who makes sure I understand everything?" Which response by the nurse is most appropriate?

A. "Yes. I am responsible for explaining the procedure, risks, benefits, and alternatives before you sign."

B. "My role is to witness your signature and verify that you are signing voluntarily and appear able to do so."

C. "I only need to make sure the form is in the chart before the procedure begins."

D. "Once you sign the form, it means you cannot change your mind later."

12. A client is scheduled for elective placement of a tunneled central venous catheter. The provider discussed the procedure earlier, and the client signed the consent form. Later, while the nurse is completing preprocedure care, the client says, "I know they told me about the benefits and risks, but I still don't understand what other choices I have besides this." Which action should the nurse take?

A. Ask the client to sign a second consent form to confirm willingness to proceed

B. Explain the available alternatives so the procedure is not delayed

C. Notify the provider that the client needs more information before the procedure continues

D. Document that the client has already signed consent and continue with preparation

13. A client is preparing for discharge after hospitalization for heart failure. The provider prescribes a low-sodium diet, daily weights, and a follow-up visit in 1 week. During teaching, the client says, "I don't own a scale, and I get paid next Friday." Which action by the nurse is most appropriate before discharge?

A. Tell the client to weigh at the follow-up visit instead of at home

B. Ask the client to estimate fluid retention by how tight clothing feels

C. Notify the case manager or social worker to help obtain an affordable or loaner scale

D. Instruct the client to skip daily weights until the scale can be purchased

14. A client is ready for discharge after an ischemic stroke with mild right-sided weakness. The client is alert, can swallow safely, and can take medications by mouth. The client will live alone and says, "Bathing and getting dressed take me a long time now, but I want to manage at home." Which referral should the nurse anticipate as most appropriate?

A. Occupational therapy

B. Respiratory therapy

C. Speech-language pathology

D. Dialysis services

15. A practical nurse is part of a quality improvement team that wants to reduce central supply delivery delays on a unit. The team is deciding how to tell whether a new restocking procedure actually improves client care. Which measure is the best outcome measure?

A. The percentage of staff who signed the new restocking checklist

B. The number of days the supply cart was fully stocked by 7:00 a.m.

C. The percentage of treatments delayed because needed supplies were unavailable

D. The number of staff members trained on the new restocking procedure

16. A clinic wants to improve follow-up after abnormal laboratory results. Review of the last 3 months shows that most delays happen when results arrive late in the day and are not addressed until the next morning. The team is considering two changes: having the practical nurse check the electronic inbox at 4:30 p.m. each day, or adding an automatic alert that flags abnormal results for same-day review. Because staffing is limited, the team can test only one change first. Which action is most appropriate?

- A.** Start both changes at the same time so the clinic can improve results faster
- B.** Test the automatic alert first because it directly targets the identified delay point
- C.** Ask staff which option they prefer and implement that one across the clinic immediately
- D.** Wait to make any change until 12 months of additional baseline data have been collected

17. A practical nurse is caring for a client recovering from pneumonia. During the morning assessment, the nurse notes the client is more fatigued than yesterday, needs help sitting up, and eats only a few bites of breakfast. The unlicensed assistive personnel reports the client was too weak to walk to the bathroom and needed a bedside commode. Which information is most important for the nurse to share with the interdisciplinary team during rounds?

- A.** The client prefers juice instead of milk with meals
- B.** The client says the hospital bed is uncomfortable at night
- C.** The client has shown a decline in strength and activity tolerance since yesterday
- D.** The client's family plans to visit later this afternoon

18. A practical nurse is caring for a client who is alert and oriented after a painful dressing change. The nurse tells the client, "I can ask the provider for something stronger next time." The client replies, "No, please don't. I don't want any more pain medicine." A few minutes later, the client's spouse approaches the nurse in the hallway and says, "He always says that when he's upset. Just get the medication ordered anyway and give it before the next dressing change." Which action by the nurse is most appropriate?

- A.** Request the medication because the spouse is trying to prevent unnecessary pain
- B.** Explain that the client has the right to refuse medication and notify the provider only if the client chooses to discuss pain-management options
- C.** Ask the spouse to sign a consent form allowing the nurse to follow the spouse's request
- D.** Wait until the next dressing change and administer the medication if the client does not object at that time

19. A practical nurse in a community clinic assesses a 72-year-old client who has moderate dementia and arrives with a neighbor. The client appears withdrawn, has poor hygiene, and has lost weight since the last visit. The neighbor says, "I do what I can, but sometimes I just leave food out and lock the door so wandering isn't a problem." What is the nurse's best action?

- A.** Wait to report until the provider confirms that neglect has occurred
- B.** Discuss the concern with the neighbor and report only if the explanation changes
- C.** Make a mandatory report based on reasonable suspicion and document objective findings
- D.** Respect the neighbor's caregiving decisions because the client cannot speak clearly

20. A practical nurse is working with one assistive personnel (AP) on a rehabilitation unit. Which assignment is most appropriate for the nurse to give the AP?

- A.** Reinforce teaching to a client who is learning to self-administer insulin
- B.** Walk a client who used a gait belt safely with physical therapy earlier and now has an order to ambulate in the hall
- C.** Evaluate whether a new pain medication is relieving a client's postoperative discomfort
- D.** Decide whether a client with a newly productive cough should be placed on aspiration precautions

21. A practical nurse in an outpatient specialty clinic notices that clients who use wheelchairs are frequently placed in the last available exam room, which is large enough for mobility devices. On busy days, those clients often wait much longer than other clients because staff use the accessible room for any open appointment. Which action by the nurse best promotes equitable access to care?

- A.** Ask wheelchair users to arrive earlier than other clients so delays are less likely
- B.** Recommend reserving at least one accessible exam room for clients who need it and flagging that need during scheduling
- C.** Continue assigning rooms as available because treating every client exactly the same is the fairest approach
- D.** Encourage clients who use wheelchairs to transfer to standard chairs when possible so other rooms can be used

22. A nurse is preparing to administer a scheduled oral medication to a client who asks, "Why do you keep checking my bracelet every time? You gave me medicine an hour ago." Which response by the nurse is best?

- A.** I only need to check your identification band when giving a new medication for the first time that shift.

B. I verify each client's identity before every medication administration to help prevent errors.

C. Since I remember you from earlier, I can skip the identification check this time.

D. The identification band is checked mainly so the pharmacy can charge the medication correctly.

23. A nurse is preparing to use a wheelchair to take a client to physical therapy. After helping the client into the chair, which action is most important before the nurse asks the client to stand and pivot into it from the bed?

A. Place a blanket over the client's lap

B. Lock the wheelchair brakes

C. Adjust the footrests to the lowest position

D. Move the wheelchair to the foot of the bed

24. A nurse in a rehabilitation unit is preparing to ambulate a client for the first time after surgery using a gait belt and a wheeled walker. The client becomes lightheaded when sitting at the bedside but says, "I'm fine—let's keep going." Which action by the nurse is best?

A. Proceed with ambulation but keep one hand on the gait belt

B. Have the client stand briefly to see whether the dizziness passes

C. Return the client to a safe seated or lying position and reassess before using the walker

D. Ask an assistive personnel member to bring a second walker for added support

25. A nurse is preparing to give the first dose of a prescribed IM vitamin B12 injection. Before administration, the client says, "I had a severe reaction to cobalt in jewelry testing and was told to avoid products containing it." What is the nurse's priority action?

A. Administer the injection slowly while observing for skin changes

B. Hold the medication and verify the ingredient/allergy concern with the provider and pharmacist before giving it

C. Ask the client whether previous reactions were limited to itching rather than breathing problems

D. Premedicate the client with an antihistamine and then give the injection

26. A practical nurse is performing a safety round on a postoperative unit and notices that several full oxygen cylinders are standing unsecured beside a medication cart in a busy hallway. Staff members are moving equipment through the area. Which action should the nurse take first?

A. Move the cylinders immediately to a secured storage area

B. Finish the safety round and report the issue at shift change

C. Place a caution sign near the cylinders and continue rounds

D. Ask the next staff member passing by to watch the cylinders

27. A client admitted with delirium is repeatedly trying to get out of bed and says, "I have to feed my dog." The client is weak, becomes more upset when several staff members talk at once, and settles briefly when one nurse sits nearby and speaks calmly. Which nursing action is most appropriate to follow least-restrictive restraint principles?

A. Apply a vest restraint so the client cannot leave the bed unattended

B. Move the client closer to the nurses' station and assign frequent one-to-one reorientation

C. Raise all four side rails to keep the client from climbing out of bed

D. Ask the provider for an as-needed restraint order in case the client becomes more restless

28. A nurse preparing to check a client's blood glucose notices that the lancet device at the bedside appears to have been left in the room from the previous occupant. The nurse has not used the device, and no client exposure has occurred. Which action is most appropriate?

A. Use the device after wiping it with an alcohol pad because there was no direct client contact

B. Set the device aside, obtain appropriate single-client equipment, and report the near miss according to facility policy

C. Ask the client whether the previous occupant had any infections before deciding what to do

D. Document that no harm occurred and continue care without reporting the situation

29. A nurse is choosing a transfer device for a client who must be moved from the bed to a stretcher. Which client situation is the best indication for using a lateral transfer device rather than a gait belt or standing assist?

A. A client who can stand briefly, pivot with help, and follow directions

B. A client who cannot bear weight but can push up with both arms to slide into a chair

C. A client who must remain in a lying position and is unable to assist with movement

D. A client who is weak but able to grasp a standing aid and support most body weight

30. A practical nurse is preparing to obtain a capillary blood glucose reading for a client. The room has a shared glucometer that was used earlier for another client. Which action is most appropriate to prevent transmission of infection?

- A.** Wipe the outside of the glucometer with a dry paper towel before use
- B.** Clean and disinfect the glucometer according to facility policy before using it for this client
- C.** Apply clean gloves and use the glucometer without further preparation
- D.** Use the same lancet device if the needle was changed after the last client

31. A nurse is removing PPE after caring for a client on contact precautions. Which action is most appropriate to prevent self-contamination?

- A.** Remove the goggles first because they are farthest from the hands
- B.** Touch the front of the gown to pull it off quickly before removing gloves
- C.** Roll the gown inside out while removing it and avoid touching the outside surface
- D.** Keep the contaminated gloves on to remove the mask last

32. A practical nurse is assisting with a sterile dressing change. The nurse opens a sterile kit and places the sterile field on a bedside table. A few minutes later, the nurse notices that a nonsterile blood pressure cuff tubing is resting on one corner of the sterile drape. Which action should the nurse take?

- A.** Move the tubing away and continue using the rest of the sterile field
- B.** Consider only the corner contaminated and avoid using that area
- C.** Discard the sterile field and set up a new sterile field
- D.** Wipe the contaminated area with antiseptic solution and continue

33. A practical nurse is caring for a client who is placed on protective (neutropenic) precautions after chemotherapy. Which action by the nurse is most appropriate?

- A.** Place fresh flowers from the family on the bedside table
- B.** Use a dedicated stethoscope for the client
- C.** Encourage the client to eat raw fruits brought from home
- D.** Leave the room door open to improve air circulation

34. A practical nurse is assigned to give a bed bath to a client who has no isolation orders. The nurse notices several open areas from scratching on the client's lower legs, but there is no active bleeding. Which action best reflects standard precautions?

- A.** Wear gloves while washing the affected areas and perform hand hygiene after glove removal
- B.** Put on an N95 respirator before beginning the bath
- C.** Use sterile gloves for the entire bed bath
- D.** Delay the bath until the client is placed on contact precautions

35. A client at 41 weeks' gestation is in labor with contractions every 3 minutes. The fetal heart rate tracing shows a baseline of 145/min with moderate variability and recurrent decelerations that begin after the peak of each contraction and return to baseline after the contraction ends. Which interpretation is most appropriate?

- A.** Umbilical cord compression
- B.** Fetal head compression
- C.** Uteroplacental insufficiency
- D.** Normal fetal sleep cycle

36. A nurse is reinforcing discharge teaching for a client who is 5 days postpartum after an uncomplicated vaginal birth. Which client statement indicates correct understanding of activity during recovery?

- A.** I should stay on complete bed rest for the first 2 weeks to prevent bleeding.
- B.** I can gradually increase my activity and should stop and rest if I become overly tired.
- C.** I should avoid climbing any stairs until after my 6-week follow-up visit.
- D.** I can return to my full exercise routine now as long as I am no longer in pain.

37. A nurse is assessing a 10-month-old infant during a well-child visit. Which observation is most consistent with expected development?

- A.** Uses a neat pincer grasp to pick up a raisin
- B.** Sits steadily without support and pulls to a standing position
- C.** Rides a tricycle across the room
- D.** Prints the first letter of their name

38. A practical nurse is reinforcing teaching with the parents of a 3-year-old child who recently started attending daycare full-time. The parents say, "At drop-off, our child cries, clings to us, and throws toys. By the end of the day, the staff says our child is playing normally." Which response by the nurse is most appropriate?

- A.** This behavior suggests a developmental delay and should stop immediately.
- B.** This is a common response to separation, and a consistent goodbye routine can help.
- C.** Your child is too young for group care and should stay home until school age.
- D.** This behavior means your child is being mistreated at daycare.

39. A practical nurse is reinforcing teaching with the parent of a 6-year-old child about routine health visits. The parent says, "My child sees well enough to read, so yearly vision screening at school is unnecessary." Which response by the nurse is best?

- A.** Vision screening helps detect problems early, even when a child does not complain of symptoms.
- B.** Children usually need vision testing only after they begin having frequent headaches.
- C.** If a child can read classroom materials, formal screening is no longer needed.
- D.** Routine vision screening is mainly used to diagnose eye infections before they spread.

40. A nurse is obtaining a health history from an adult client who says, "I have to get up to urinate several times every night." Which follow-up question is best to further assess this symptom?

- A.** "About how many times do you wake up to urinate during the night?"
- B.** "Do you usually drink coffee with breakfast?"
- C.** "Has anyone in your family ever had kidney stones?"
- D.** "When was your last complete physical exam?"

41. A practical nurse is collecting baseline physical data for a newly admitted adult client. Which finding best reflects the client's baseline nutritional status?

- A.** The client ate 75% of breakfast this morning.
- B.** The client reports a usual weight loss of 12 lb over the past 2 months.
- C.** The client states that lunch is usually the largest meal of the day.
- D.** The client says the hospital food tastes different from food at home.

42. A nurse is reinforcing teaching for the parent of a child who is scheduled to receive the varicella vaccine. Which statement by the parent indicates correct understanding?

- A.** My child should avoid contact with pregnant family members for 1 week after the vaccine.
- B.** A mild rash can occur after this vaccine, and I should report a severe allergic reaction right away.
- C.** This vaccine will be given if my child is taking high-dose corticosteroids.
- D.** If my child had hives after eating eggs, this vaccine should be avoided.

43. A nurse is reinforcing teaching with the parent of a 13-year-old client. Which behavior is most consistent with expected psychosocial development during early adolescence?

- A.** Preferring to make all decisions based mainly on family approval
- B.** Showing increased concern about body image and peer acceptance
- C.** Choosing solitary play over spending time with friends
- D.** Demonstrating complete independence from parental guidance

44. A client with newly diagnosed rheumatoid arthritis says to the practical nurse, "My hands hurt so much in the morning that I can't even open a jar. I used to do everything for my family, and now I feel useless." Which response by the nurse best provides emotional support?

- A.** You need to stay positive and focus on what you can still do.
- B.** It sounds like this loss of function is very upsetting for you.
- C.** Rheumatoid arthritis treatments have improved a lot, so you should feel better soon.
- D.** Why don't you ask your family to take over the tasks that are hard for you?

45. A client with chronic low back pain tells the nurse, "On days when the pain gets worse, I skip my walk, stay in bed, and tell myself nothing is going to help anyway." Which nursing response best promotes adaptive coping?

- A.** "It may help to identify one manageable action, such as a short walk or relaxation exercise, that you can still do on difficult days."
- B.** "You should avoid activity until the pain is completely gone so you do not make the problem worse."
- C.** "Try not to think about the pain, because focusing on it will usually make it disappear."
- D.** "If you cannot handle the pain on your own, your family should make your decisions for you."

46. A nurse in a primary care office is assessing a client who was asked to wait longer than expected. The client is standing, breathing hard, and repeatedly hitting a rolled-up magazine against a chair. Which finding should the nurse identify as the clearest sign that the client's risk for violence is increasing?

- A.** The client says, "This place is always disorganized."
- B.** The client asks, "How much longer is this going to take?"
- C.** The client begins striking the chair harder and will not respond to the nurse's attempts to speak calmly.
- D.** The client tells another client, "I had a long day before I even got here."

47. A client on a psychiatric unit reports daily heavy alcohol use and says the last drink was 3 days ago. Which finding requires the nurse's immediate attention?

- A.** Fine hand tremors and nausea
- B.** Blood pressure 148/90 mm Hg and anxiety
- C.** New-onset generalized seizure activity
- D.** Insomnia and poor appetite

48. A client with antisocial behavior on an inpatient unit becomes verbally aggressive when told smoking is not allowed on the unit. The client steps close to the nurse, points a finger, and says, "If you don't let me go now, you'll be sorry." Which nursing action is most appropriate?

- A.** Firmly tell the client that threatening staff will result in immediate loss of privileges
- B.** Step back, keep a calm voice, and direct the client to move to a quieter area to talk
- C.** Argue the unit policy and explain why the rule must be followed
- D.** Turn away from the client and leave the area to show that the behavior will not get attention

49. A client who was laid off from work this week tells the nurse, "I haven't told my family yet. I just keep sitting in my car before I go inside." Which response by the nurse is most therapeutic?

- A.** You should tell them tonight so you can get it over with.
- B.** At least you still have your health, and another job will come along.
- C.** It sounds like you're having a hard time figuring out how to face them.
- D.** Why haven't you told your family already?

50. A nurse enters a semi-private room and finds one client becoming increasingly agitated while the roommate and television are adding to the noise. The agitated client is pacing, speaking loudly, and saying, "I need everyone to stop talking right now." The client is not making threats or attempting to strike anyone. Which nursing action is most appropriate?

- A.** Stand directly in front of the client and firmly instruct the client to sit down
- B.** Ask several questions to determine exactly why the client is upset
- C.** Reduce environmental stimulation and speak calmly using short, simple statements
- D.** Tell the client that the behavior is inappropriate and will not be tolerated

51. A client with moderate dementia becomes upset during the evening and repeatedly asks, "When is my mother coming to get me?" The client begins wringing both hands and looking toward the door. Which response by the practical nurse is most appropriate?

- A.** "Your mother is not coming, because you are an adult now."
- B.** "Why do you keep asking that when I already answered you?"
- C.** "You miss your mother. Tell me about her while we sit together."
- D.** "If you stop asking, I will check the schedule later."

52. A client says, "I can tell my stress is building because my shoulders get tight and I start talking faster, but I usually do not notice it until I am already overwhelmed." Which nursing suggestion is most appropriate?

- A.** Wait to address the stress until the client can identify the exact cause
- B.** Practice noticing early physical and behavioral warning signs so coping strategies can be started sooner
- C.** Avoid thinking about the stress to prevent making the feelings worse
- D.** Use the same coping method every time, even if it has not been helpful before

53. A client who was admitted after a myocardial infarction tells the practical nurse, "I don't belong to any religion, but being in here makes me feel empty and alone. I keep wondering what the point of all this is." Which response by the nurse is most appropriate?

- A.** "Many people feel that way in the hospital, so try not to think about it right now."
- B.** "It sounds like you're struggling with questions about meaning and support. Would you like to talk more about that or see someone who can help?"
- C.** "You should focus on getting stronger first, and those feelings will probably go away."

D. "Have you considered joining a religious group after discharge?"

54. A nurse is caring for a client who recently had a stroke. During feeding assistance, the client's spouse says, "In our family, it is disrespectful to question the doctor, so please do not ask my husband to choose between treatment options." The client is alert, able to communicate, and has been making needs known appropriately. Which action by the nurse is most appropriate?

- A.** Avoid discussing choices with the client and direct all treatment questions to the spouse
- B.** Explain that the client must make all decisions alone without family input
- C.** Ask the client how they prefer health information and decisions to be handled, then support that preference
- D.** Tell the spouse that cultural beliefs cannot be considered during hospital care

55. A client receiving end-of-life care tells the nurse, "I am not afraid of being dead. I am afraid of what my body will be like in the last few days, and no one will answer me directly." Which response by the nurse is most appropriate?

- A.** "Try not to think about that right now. We want to keep you comfortable."
- B.** "Many people feel this way. Tell me what changes worry you most, and I will answer honestly and stay with you while we talk."
- C.** "Your provider can explain the physical process better than I can, so you should wait and ask later."
- D.** "Most clients become sleepy and peaceful, so there is usually nothing to worry about."

56. A nurse is teaching an assistive personnel (AP) how to help care for a client with a mature gastrostomy tube receiving intermittent feedings. Which statement by the AP indicates a need for further teaching?

- A.** I should report if the skin around the tube becomes increasingly red or has drainage.
- B.** I should keep the client upright during the feeding and for a while afterward.
- C.** I should use hydrogen peroxide every day to clean around the stoma so it stays sterile.
- D.** I should avoid pulling on the tube when helping the client change position.

57. A nurse is reinforcing teaching for a client who needs to increase potassium intake because of diuretic therapy. Which food should the nurse recommend?

- A.** Banana
- B.** White rice
- C.** Cheddar cheese
- D.** Toast with jelly

58. A client with a migraine reports throbbing head pain and sensitivity to light and sound. The client asks what the nurse can do besides medication to help right now. Which intervention is most appropriate?

- A.** Place the client in a dark, quiet room
- B.** Encourage the client to walk in the hallway
- C.** Apply a heating pad to the client's forehead
- D.** Offer a caffeinated drink and turn on the television for distraction

59. A nurse is caring for a client with a below-the-knee amputation who is learning to bathe and dress independently. Which nursing action best supports the client's ability to perform activities of daily living safely?

- A.** Complete the entire bath for the client to prevent fatigue
- B.** Encourage the client to perform as much care as possible and provide assistive devices as needed
- C.** Ask the client to remain in bed until all self-care can be done without help
- D.** Limit practice with dressing until the prosthesis is permanently fitted

60. A nurse is caring for a client who is at risk for urinary retention after receiving an opioid analgesic. Which nursing action is most appropriate to help promote bladder emptying before using invasive measures?

- A.** Offer privacy and assist the client to a normal voiding position
- B.** Restrict oral fluids until the client voids independently
- C.** Apply an adult brief so the client does not have to get out of bed
- D.** Insert an indwelling urinary catheter as soon as 4 hours have passed without voiding

61. A nurse is caring for a client who says, "I wake up in the middle of the night because my mind starts racing about everything I need to do." Which nursing suggestion is most appropriate to promote sleep?

- A.** Do light housework until feeling tired again
- B.** Keep a notepad by the bed to write down concerns before trying to return to sleep
- C.** Watch television in bed until sleep returns
- D.** Drink a caffeinated beverage to stay awake until the next bedtime

62. A practical nurse is caring for a client receiving oxygen by nasal cannula. During assessment, the nurse notes redness and soreness behind both ears where the tubing rests. Which action is best to help protect the client's skin?

- A.** Remove the oxygen for 30 minutes every 4 hours
- B.** Pad the tubing contact areas and reposition the tubing as needed

- C.** Tighten the cannula to prevent it from rubbing the skin
- D.** Apply lotion under the tubing every shift without further changes
- 63.** A practical nurse is caring for a client with Ménière disease who reports that the room suddenly feels like it is spinning. Which action should the nurse take first?
- A.** Help the client lie down and reduce sudden movement
- B.** Offer the client a full glass of water
- C.** Encourage the client to walk slowly to improve balance
- D.** Open the curtains to increase lighting in the room
- 64.** A practical nurse is recording intake and output for a client with an indwelling urinary catheter. At 8:00 a.m., the drainage bag is emptied. At 12:00 p.m., the bag contains 325 mL. The nurse empties the bag again. At 3:00 p.m., the client voids 175 mL around the catheter, and the drainage bag contains 140 mL. What total urinary output should the nurse record from 8:00 a.m. to 3:00 p.m.?
- A.** 465 mL
- B.** 500 mL
- C.** 640 mL
- D.** 825 mL
- 65.** A practical nurse is caring for a client who has been immobilized in bed for 1 week. Which action is most appropriate when repositioning the client to protect the skin from friction and shear?
- A.** Pull the client up in bed without lifting to avoid disturbing alignment
- B.** Raise the head of the bed to 60 degrees before moving the client
- C.** Use a drawsheet with assistance to lift the client during repositioning
- D.** Massage reddened areas after turning to improve circulation
- 66.** A nurse is preparing to administer an oral opioid analgesic to a client who is alert and reports pain of 8 out of 10. After the nurse identifies the client and verifies the medication against the MAR, the client asks, "What medication is this, and what is it for?" Which action by the nurse best supports the client's medication rights?
- A.** Tell the client that the medication can be discussed after it is taken so pain relief is not delayed
- B.** Ask the client to trust the provider's prescription and take the medication as ordered
- C.** Explain the name of the medication, its purpose, and expected effect before administration
- D.** Document that teaching will be completed later because the medication is already due
- 67.** A practical nurse is reviewing a newly admitted client's home medication history with the client and family. The client takes prescribed medications, an over-the-counter antacid as needed, and a weekly injectable medication for diabetes. The family adds that the client also uses a nicotine patch at home. None of the nonprescription items are documented on the current medication list. Which action by the nurse is best?
- A.** Document only the prescribed medications because nonprescription products are not part of medication reconciliation
- B.** Add all medications and products the client uses, including over-the-counter, patch, and injectable therapies, and report the complete list per agency process
- C.** Wait until discharge teaching to update the medication list so the provider can compare old and new medicines at one time
- D.** Ask the family to bring in the medication containers later and avoid documenting any additional items until then
- 68.** A nurse is preparing to administer a routine medication to a patient with dysphagia after a recent stroke. The patient is awake, follows commands, and has a feeding tube in place. Which route of administration is most appropriate for this medication if it is available in liquid form and approved for tube administration?
- A.** Oral
- B.** Enteral via feeding tube
- C.** Sublingual
- D.** Intramuscular
- 69.** A client with cirrhosis is scheduled for a paracentesis later today. Morning laboratory results show an INR of 2.4. The provider prescribes a blood product to reduce the risk of bleeding before the procedure. Which product should the nurse expect to administer?
- A.** Packed red blood cells
- B.** Fresh frozen plasma
- C.** Platelets
- D.** Albumin
- 70.** A nurse is to administer 675 mL of IV solution over 9 hours. The IV has already infused 225 mL in the first 3 hours. If the nurse wants the remaining fluid to finish on time, what should the infusion pump be set to for the rest of the infusion?
- A.** 50 mL/hr
- B.** 67.5 mL/hr
- C.** 75 mL/hr
- D.** 112.5 mL/hr

71. A practical nurse enters a client's room to give a prescribed controlled pain medication and finds several visitors at the bedside. The client quietly says, "Please don't talk about my pain medicine in front of them." Which action by the nurse is best?

- A.** Ask the visitors to leave the room, verify the medication privately with the client, and administer it according to policy
- B.** Explain to the visitors that the medication is a controlled substance and must be discussed before it can be given
- C.** Tell the client that medication teaching must be done out loud before any controlled drug can be administered
- D.** Return the medication to storage and document that the client refused the dose

72. A nurse is caring for a postoperative client who is receiving morphine through a patient-controlled analgesia (PCA) pump. The client tells the nurse, "I'm worried I'll get too much medicine if I keep pressing the button." Which explanation by the nurse is best?

- A.** The pump is programmed with a lockout interval, so extra doses will not be delivered before it is safe.
- B.** Pressing the button more often causes the pump to deliver smaller doses each time.
- C.** The pump only works when the client's pain rating is above 5 on a 0-to-10 scale.
- D.** Using the PCA button frequently means the client is becoming dependent on the medication.

73. A nurse is preparing to administer the next dose of cefazolin IV to a client who developed a postoperative infection. Which pre-administration information is most important for the nurse to review?

- A.** The client's most recent white blood cell count
- B.** The client's documented medication allergies
- C.** The client's pain rating from the previous shift
- D.** The client's intake and output for the last 24 hours

74. A client with myasthenia gravis has been taking pyridostigmine for 1 week. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client swallows meals without coughing or choking
- B.** The client reports sleeping 8 hours each night
- C.** The client has a urine output of 45 mL/hr
- D.** The client has a blood pressure of 128/76 mm Hg

75. A client with type 2 diabetes started metformin 2 days ago. During a follow-up call, which client statement should the nurse identify as the highest-priority adverse response requiring immediate evaluation?

- A.** "I have had some loose stools since starting the medicine."
- B.** "Food tastes metallic, and I do not feel very hungry."

C. "I feel very weak, my breathing seems fast, and I have severe muscle aches."

D. "I notice mild nausea for about an hour after I take the pill."

76. A patient who takes ferrous sulfate for iron-deficiency anemia says, "I usually take it with breakfast and my antacid so it won't upset my stomach." Which response by the nurse is best?

- A.** "That is a good plan because antacids improve iron absorption."
- B.** "Take the iron with the antacid to reduce stomach irritation, but avoid orange juice."
- C.** "Antacids can decrease iron absorption, so you should separate the iron and antacid doses."
- D.** "Stop taking the iron until the provider changes the prescription to a different form."

77. A nurse is preparing medications for administration and notices that a unit of packed red blood cells is infusing through the client's only IV line. The provider has also prescribed an IV dose of cefazolin due now. Which action by the nurse is most appropriate?

- A.** Add the cefazolin to the blood tubing so both can infuse together
- B.** Pause the blood transfusion, flush the line, and administer cefazolin through the same tubing
- C.** Delay the cefazolin until the blood transfusion is complete or obtain separate IV access per facility policy
- D.** Dilute the cefazolin in dextrose solution and infuse it slowly with the blood

78. A nurse is preparing to administer ampicillin 1.2 g IV. The medication available after reconstitution is labeled 300 mg/mL. How many milliliters should the nurse prepare for one dose?

- A.** 2 mL
- B.** 3 mL
- C.** 4 mL
- D.** 6 mL

79. A practical nurse is collecting data on four clients in a primary care clinic. Which client should the nurse identify as being at greatest risk for acute kidney injury?

- A.** A client with osteoarthritis who takes ibuprofen daily and reports poor oral intake for 3 days after a viral illness
- B.** A client with seasonal allergies who started a daily antihistamine 1 week ago
- C.** A client with controlled hypothyroidism who takes levothyroxine each morning
- D.** A client with gastroesophageal reflux disease who uses calcium carbonate as needed

80. A nurse is collecting a nasopharyngeal specimen for viral testing from an adult client. Which action is most appropriate to obtain an adequate specimen?

- A.** Insert the swab gently along the floor of the nasal passage until resistance is met, then rotate it before removal
- B.** Swab only the outer edge of each nostril to reduce discomfort
- C.** Ask the client to blow the nose forcefully immediately after the swab is inserted
- D.** Insert the swab upward toward the bridge of the nose and remove it as soon as tearing occurs

81. A nurse is reviewing laboratory results for a client who has chronic kidney disease and reports fatigue and shortness of breath with activity. Which laboratory finding best explains these symptoms?

- A.** Hemoglobin 8.2 g/dL
- B.** Platelet count 280,000/mm³
- C.** Sodium 138 mEq/L
- D.** Glucose 102 mg/dL

82. A client who is 32 weeks pregnant comes to the prenatal clinic and says, "My left calf has become painful and more swollen since this morning." Which additional finding should the nurse identify as most concerning?

- A.** Shortness of breath and sharp chest pain with breathing
- B.** Urinary frequency without dysuria
- C.** Mild ankle swelling at the end of the day
- D.** Intermittent low back discomfort after standing

83. A nurse is performing a bedside fecal occult blood test on a stool specimen from a client with hemorrhoids that are actively bleeding. Which interpretation of the test result is most appropriate?

- A.** A positive result would confirm bleeding from higher in the gastrointestinal tract
- B.** A negative result would rule out gastrointestinal bleeding
- C.** A positive result may be misleading because the specimen is contaminated with blood from the hemorrhoids
- D.** The test result is reliable as long as the stool specimen is fresh

84. A practical nurse is obtaining routine vital signs on an adult client. The nurse notes the respirations are 24/min, shallow, and irregular while the client is resting quietly in bed. Which action is most appropriate next?

- A.** Document the respiratory rate and continue with the rest of the assessment
- B.** Recount the respirations for a full 60 seconds
- C.** Ask the client to take several deep breaths and then count again

D. Obtain the client's blood pressure before reassessing respirations

85. A client is scheduled for a magnetic resonance imaging (MRI) scan. Which information obtained during the nursing assessment is most important to report before the procedure?

- A.** The client states feeling nervous in small enclosed spaces
- B.** The client has a titanium hip replacement
- C.** The client has a pacemaker
- D.** The client removed all metal jewelry before transport

86. A nurse reviews a client's ECG strip and notes a rhythm of 42/min with regular R-R intervals. P waves are not visible, and the QRS complexes are wide and uniform. Which rhythm should the nurse identify?

- A.** Sinus bradycardia
- B.** Junctional rhythm
- C.** Idioventricular rhythm
- D.** First-degree atrioventricular block

87. A nurse is obtaining a venous blood specimen from an adult client for routine laboratory testing. The nurse has assembled the equipment and applied the tourniquet. The selected vein is still difficult to visualize. Which action is most appropriate to promote venous filling while reducing the risk of altering test results?

- A.** Ask the client to repeatedly open and close the fist several times
- B.** Lower the client's arm below heart level for a short time
- C.** Tap the vein firmly until it becomes more visible
- D.** Keep the tourniquet in place while searching for a better site for several minutes

88. A practical nurse is caring for a client 1 hour after a subtotal thyroidectomy. Which finding requires the nurse to take immediate action?

- A.** The client reports incisional pain when swallowing
- B.** The client frequently clears the throat and has a hoarse voice
- C.** The client rates pain as 5 on a 0-to-10 scale
- D.** The client requests water because of a dry mouth

89. A client with a tunneled central venous catheter is found to have a loose connection between the IV tubing and the catheter hub while the infusion is running. Which action should the nurse take first?

- A.** Tighten the connection and continue the infusion while monitoring the client
- B.** Clamp the catheter and secure the connection using sterile technique
- C.** Flush the catheter to check whether the line is still patent

D. Lower the IV bag below the level of the client's chest

90. A nurse is caring for a client with acute pancreatitis who suddenly reports severe abdominal pain that is now radiating to the back. Which focused assessment should the nurse perform first?

- A.** Inspect the stool for occult blood
- B.** Assess the abdomen for distention, guarding, and bowel sounds
- C.** Review the client's food intake from the previous 24 hours
- D.** Ask the client to rate nausea on a 0 to 10 scale

91. A client with bacterial meningitis has been receiving treatment for 24 hours. Which finding should the nurse report immediately as a possible sign of increasing intracranial pressure?

- A.** Nuchal rigidity when the client flexes the neck
- B.** A positive Kernig sign during assessment
- C.** New onset of decreased level of consciousness and unequal pupils
- D.** Temperature of 38.3°C (100.9°F) and headache rated 6/10

92. A client with chronic glaucoma is prescribed timolol eye drops for daily use at home. Which statement by the client indicates a correct understanding of how to use this medication safely?

- A.** I will press gently on the inner corner of my eye after putting the drop in.
- B.** I should stop the drops when my vision seems clearer.
- C.** I will rinse my eye with water right after each dose.
- D.** I should expect the drops to cure the glaucoma completely.

93. A client 18 hours after a subtotal thyroidectomy suddenly develops tingling around the mouth and muscle twitching. When the nurse inflates the blood pressure cuff, the client's hand spasms. Which complication should the nurse suspect?

- A.** Hypocalcemia from accidental parathyroid injury
- B.** Thyroid storm caused by excess thyroid hormone release
- C.** Laryngeal edema from fluid retention
- D.** Wound dehiscence related to coughing

94. A client has a chest tube connected to a drainage system after chest trauma. During assessment, the nurse finds the chest tube has been accidentally pulled completely out of the client's chest. Which action should the nurse take first?

- A.** Apply a sterile occlusive dressing taped on three sides over the insertion site
- B.** Place the disconnected tube end in sterile water
- C.** Clamp the chest tube tubing close to the client

D. Reconnect the tube to wall suction immediately

95. A nurse is assessing a client who became chilled after receiving several units of refrigerated IV fluids rapidly during emergency treatment. Which client response reflects the body's immediate attempt to conserve heat?

- A.** Peripheral vasoconstriction
- B.** Profuse sweating
- C.** Decreased muscle activity
- D.** Peripheral vasodilation

96. A nurse is assessing a client's postoperative incision 48 hours after surgery. Which finding should the nurse identify as most consistent with wound infection?

- A.** Serosanguineous drainage on the dressing
- B.** Mild swelling localized to the incision edges
- C.** Increasing pain with purulent drainage from the wound
- D.** A thin pink scar forming along the incision line

97. A nurse begins rescue efforts for an adult client in cardiac arrest. After the first shock is delivered with an AED, which action should the nurse take next?

- A.** Check for a carotid pulse for at least 15 seconds before restarting CPR
- B.** Resume chest compressions immediately, starting with compressions
- C.** Provide 2 rescue breaths before deciding whether to continue CPR
- D.** Remove the AED pads and reassess the client's level of consciousness

98. A client with chronic bronchitis is prescribed oxygen at 2 L/min by nasal cannula. During the morning assessment, the nurse notes the tubing is in place, but the humidifier bottle is empty and the client reports thick, hard-to-cough secretions and a dry nose. Which nursing action is most appropriate?

- A.** Increase the oxygen flow rate to 6 L/min to loosen the secretions
- B.** Refill the humidifier setup as prescribed and provide nasal/oral care
- C.** Switch the client to a nonrebreather mask for better moisture delivery
- D.** Discontinue the oxygen until new humidifier equipment is available

99. A nurse is caring for a client who has acute pancreatitis and is receiving IV fluids. Which assessment finding most strongly suggests hypocalcemia?

- A.** Numbness and tingling around the mouth
- B.** Bounding peripheral pulses
- C.** Warm, flushed skin

D. Decreased deep tendon reflexes

100. A client with end-stage kidney disease is receiving hemodialysis through a left forearm arteriovenous fistula. Which finding during routine assessment should the practical nurse recognize as the highest priority to report immediately?

- A.** The client reports mild fatigue after today's treatment
- B.** The fistula site has a palpable thrill and an audible bruit
- C.** The left hand is cool and pale with numbness reported by the client
- D.** A small area of bruising is present near the access site from the last treatment

Simulation Test 11 — Answer Key

1. A	26. A	51. C	76. C
2. B	27. B	52. B	77. C
3. A	28. B	53. B	78. C
4. D	29. C	54. C	79. A
5. C	30. B	55. B	80. A
6. D	31. C	56. C	81. A
7. C	32. C	57. A	82. A
8. C	33. B	58. A	83. C
9. C	34. A	59. B	84. B
10. A	35. C	60. A	85. C
11. B	36. B	61. B	86. C
12. C	37. B	62. B	87. B
13. C	38. B	63. A	88. B
14. A	39. A	64. C	89. B
15. C	40. A	65. C	90. B
16. B	41. B	66. C	91. C
17. C	42. B	67. B	92. A
18. B	43. B	68. B	93. A
19. C	44. B	69. B	94. A
20. B	45. A	70. C	95. A
21. B	46. C	71. A	96. C
22. B	47. C	72. A	97. B
23. B	48. B	73. B	98. B
24. C	49. C	74. A	99. A
25. B	50. C	75. C	100. C

Simulation Test 12

100 questions

1. A practical nurse in an outpatient surgery center is preparing four clients for discharge at the same time. Which client should the nurse assess first?

- A.** A client after laparoscopic cholecystectomy who reports nausea and has small amounts of green emesis
- B.** A client after colonoscopy who has abdominal cramping and asks when to resume a regular diet
- C.** A client after knee arthroscopy whose dressing is saturated with bright red drainage
- D.** A client after cataract surgery who states that the operated eye feels scratchy

2. A practical nurse asks an assistive personnel (AP) to help a stable client shower. A few minutes later, the AP reports, "The client became very tired, said the room was spinning, and had to sit down." Which action by the practical nurse is best?

- A.** Tell the AP to finish the shower after the client rests for 10 minutes
- B.** Ask the AP to document the event and continue caring for other clients
- C.** Go assess the client immediately and determine whether the activity should be stopped
- D.** Instruct the AP to obtain the next scheduled set of vital signs later in the shift

3. A practical nurse is making assignments for an assistive personnel (AP) at the start of the shift on a rehabilitation unit. Which task is most appropriate to delegate to the AP?

- A.** Teach a client how to use a newly prescribed walker before physical therapy
- B.** Assess a client who reports numbness under a leg cast
- C.** Assist a stable client with dressing and grooming before breakfast
- D.** Evaluate whether a client can safely transfer from bed to chair without help

4. A client who uses a communication board after jaw surgery becomes visibly upset when a dietary staff member says, "I'll just ask the family what the client wants." The client points to the call light and then to the communication board. What is the nurse's best action?

- A.** Ask the family to choose meals until the client can speak again
- B.** Tell the dietary staff member that meal selection can wait until later
- C.** Use the communication board with the client to determine preferences and explain to staff that the client should be included in decisions

D. Document that the client is frustrated and continue with the current diet order

5. An adult client scheduled for a nonurgent diagnostic procedure appears anxious when the consent form is presented. The client says, "I can read English, but I do much better when someone explains things in simple words and gives me time to ask questions." The provider is in another room and the transport staff has arrived to take the client now. What should the nurse do first?

- A.** Send the client to the procedure because the form can be reviewed again there
- B.** Ask a family member to explain the procedure in simpler language so the schedule is not delayed
- C.** Hold transport and notify the provider that the client needs a clearer explanation before proceeding
- D.** Read the consent form aloud and ask the client to sign if no more questions are raised

6. A practical nurse is updating a client care whiteboard in a semiprivate room. Which information is most appropriate to write on the board to support care while protecting confidentiality?

- A.** The client's full diagnosis and planned procedure time
- B.** The client's medication schedule and most recent laboratory results
- C.** The nurse's name, the date, and the day's mobility goal
- D.** The client's insurance status and expected discharge destination

7. A practical nurse is caring for a client who asks that no medical information be shared with family members. Later, the client's adult daughter calls the unit, correctly states the client's full name and date of birth, and asks whether the client had surgery this morning. Which action by the nurse is most appropriate?

- A.** Tell the daughter only that the client is stable, because she verified identifying information
- B.** Confirm that surgery occurred but decline to discuss the outcome
- C.** Ask the daughter to call back when the provider is available to speak with her
- D.** State that the nurse cannot share information without the client's permission and offer to take a message

8. During admission, an alert adult client says, "I completed an advance directive a few years ago, but I forgot to bring it with me." What is the nurse's most appropriate action?

- A.** Ask the client to summarize the document and enter the summary in the chart as the legal directive
- B.** Document that the client reports having an advance directive and ask the client or family to provide a copy for the record

C. Tell the client the prior directive cannot be considered unless a new one is completed in the hospital

D. Delay admission care until the advance directive is brought to the unit

9. An adult client is alert and has decision-making capacity. The client tells the nurse, "I signed papers saying I would not want mechanical ventilation if I were permanently unconscious. But if I can still speak for myself, I want to decide based on the situation." Which understanding of advance directives is most accurate?

A. The written directive always overrides the client's current verbal decisions

B. The client's named family member must decide whether the directive or the client's statement should be followed

C. The client's current decisions take priority while the client has decision-making capacity

D. The advance directive becomes active as soon as the client is admitted to the hospital

10. A client is scheduled for an elective liver biopsy. The client is alert and says, "I trust my doctor, so I signed the paper without reading it. I don't really know what problems could happen from the biopsy." Which action by the nurse is most appropriate?

A. Proceed with the preparation because the client already signed the consent form

B. Explain the possible complications of the biopsy to complete the informed consent process

C. Notify the provider because the client may not be adequately informed about the procedure

D. Ask a family member to review the consent form with the client

11. A client arrives in the emergency department unconscious with severe internal bleeding after a crash. The surgeon says the client needs immediate emergency surgery to save the client's life. No family or legal representative can be reached. Which action is most appropriate?

A. Delay surgery until a family member can provide consent

B. Proceed with treatment under implied consent for an emergency

C. Ask two nurses to sign the consent form for the client

D. Wait until the client regains consciousness to decide

12. A client is preparing for discharge after hospitalization for a new diagnosis of heart failure. The provider prescribes a low-sodium diet, daily weight monitoring, and several new medications. The client says, "My daughter will help me, but she works during the day. I get confused when there are too many instructions at once." Which action by the nurse is most appropriate before discharge?

A. Provide all teaching in writing so the client can read it later

B. Ask the daughter to call the unit after discharge for instructions

C. Break the teaching into short parts and have the client repeat back key points

D. Focus on medication teaching and leave diet instruction for the follow-up visit

13. A client is being discharged after below-knee amputation related to peripheral arterial disease. The client will need wound checks, prosthetic evaluation later, and help adjusting to daily activities at home. The client says, "I don't want to keep coming back to the hospital, and I feel embarrassed asking my family for help." Which referral should the nurse prioritize to support the client's transition after discharge?

A. Home health services

B. Occupational therapy in 6 months

C. Hospice care

D. A routine dental evaluation

14. A home health agency wants to improve medication reconciliation after clients are discharged from the hospital. Review of recent charts shows that discrepancies are often found at the first home visit, but not all nurses document reconciliation in the same place. What should the quality improvement team do first?

A. Require every nurse to call the prescriber after each first home visit

B. Collect baseline data using a standard definition and a consistent documentation method

C. Teach all clients to bring pill bottles to their next clinic appointment

D. Counsel nurses whose charts are missing medication reconciliation notes

15. A medical unit is trying to reduce omission of prescribed turning and repositioning for clients at risk for pressure injury. For 1 month, the team tests a bedside turning schedule and staff education. At the end of the month, chart audits show documented repositioning increased from 61% to 93%, but the number of new pressure injuries on the unit did not change. Which interpretation is most appropriate?

- A.** The project was successful because improved documentation proves client outcomes improved
- B.** The intervention should be stopped because process improvement never matters unless outcomes change immediately
- C.** The team improved a process measure, but more time or additional analysis is needed to know whether client outcomes improved
- D.** The unchanged outcome means staff education caused more inaccurate charting

16. A practical nurse is caring for a client with Parkinson disease who coughs during meals and has lost 1.8 kg (4 lb) in 2 weeks. The client is scheduled to receive oral medications now and has a speech-language pathology evaluation tomorrow morning. Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the coughing after the meal and wait for the scheduled evaluation the next day
- B.** Hold the oral medications and notify the appropriate licensed nurse and speech-language pathologist about the swallowing concerns and recent weight loss
- C.** Ask the nursing assistant to encourage the client to eat more at the next meal
- D.** Switch the client to a regular diet so the team can better observe the swallowing problem

17. A practical nurse is preparing for interdisciplinary rounds about a client recovering from a pelvic fracture. The client says, "I live on the second floor, and my bathroom is down the hall." Today the client required help from two staff members to transfer to a chair, became dizzy after standing for 1 minute, and reports pain of 7/10 with movement despite medication. Which information should the nurse communicate first during rounds to support coordinated care planning?

- A.** The client likes to sit in the chair near the window each afternoon
- B.** The client had visitors for most of the morning and seemed in better spirits
- C.** The client may need discharge teaching because the fracture is painful
- D.** The client needs a two-person assist for transfers, becomes dizzy with standing, has significant pain with movement, and lives in a second-floor home

18. A practical nurse is caring for a client who asks to see a copy of the nursing notes from today's shift. The nurse is unsure of the facility process and says, "I can't show you the chart myself, but I will help connect you with the right person." Which action by the nurse is most appropriate next?

- A.** Tell the client that only providers are allowed to discuss or release any part of the medical record
- B.** Ask the charge nurse or follow facility procedure for client access to health records
- C.** Photograph the nursing notes on the unit computer and show them to the client at the bedside
- D.** Refuse the request because nursing documentation is for staff use only

19. A practical nurse in a long-term care facility notices that an 82-year-old client with mild confusion has a strong urine odor, is wearing the same soiled clothing from the previous day, and has missed two doses of prescribed antibiotics because the family caregiver did not bring the medication from home. The caregiver says, "I'm doing my best, so don't start making reports." Which action by the nurse is most appropriate?

- A.** Wait to see whether the client's condition improves before taking further action
- B.** Report the suspected neglect according to facility policy and document objective findings
- C.** Confront the caregiver and demand an explanation for the missed medication
- D.** Ask another nurse to decide whether the situation meets reporting requirements

20. A practical nurse (PN) is working with an assistive personnel (AP) who says, "I have not cared for a client with a hearing aid before." Which action by the PN is best?

- A.** Assign the AP to a different client and perform all hearing-aid care personally
- B.** Tell the AP to read the manufacturer instructions and report back when finished
- C.** Demonstrate the needed care, explain what observations to report, and verify the AP's understanding before assigning the task
- D.** Ask another AP to explain the task while the PN continues other client care

21. A practical nurse in a rural clinic is helping with follow-up planning for clients who were recently started on new medications. Which situation most clearly requires advocacy to promote equitable access to care?

- A.** A client asks whether the medication can be taken with breakfast instead of lunch
- B.** A client says the pharmacy app sends refill reminders that are sometimes annoying

C. A client says, "The only pharmacy near me closed last month, and now I have no way to get to the next town to pick up my prescription"

D. A client says a family member usually reminds them to take the medication at night

22. A nurse in a pediatric clinic is preparing to administer an oral antibiotic to a 6-year-old child. The child's parent says, "This is Noah, and we're in a hurry because we have to pick up his sister." Another child with a similar first name is waiting in the next room. Which action should the nurse take to verify the correct client before giving the medication?

A. Ask the parent to state the child's full name and date of birth, and compare the information with the child's identification record

B. Ask the child to confirm his first name and then administer the medication

C. Use the room assignment and the parent's confirmation as sufficient identification

D. Check the medication label against the clinic schedule without asking for identifiers

23. A nurse is preparing to use sequential compression devices (SCDs) for a postoperative client. Before applying the sleeves, the nurse notes that the tubing on one sleeve is split near the connector. The client is at risk for venous thromboembolism and is due to leave for imaging in 10 minutes. Which action should the nurse take?

A. Apply the intact sleeve to one leg and leave the damaged sleeve off until the client returns

B. Reinforce the split area with tape and use the device for transport

C. Obtain functioning SCD equipment or notify the charge nurse if a replacement is not immediately available

D. Delay use of the SCDs because they are not needed during short periods away from the unit

24. A nurse is preparing a client for a blood transfusion. During the pretransfusion interview, the client states, "I had a bad reaction to a transfusion years ago, but I don't remember what caused it." Which action should the nurse take first?

A. Begin the transfusion slowly and observe closely for fever or chills

B. Clarify the reaction details and verify whether the client has any known blood product allergies before proceeding

C. Administer the prescribed pretransfusion medications immediately

D. Ask another nurse to stay in the room during the first 15 minutes of the transfusion

25. A nurse is preparing to administer a newly prescribed oral analgesic. The electronic health record lists hydromorphone as an allergy. When questioned, the client says, "It made me very sleepy, and I needed help walking." Which nursing action is most appropriate?

A. Hold the medication and treat the reaction as a confirmed allergy

B. Remove the allergy entry from the chart because the client denies hives or swelling

C. Recognize this as a possible adverse effect rather than a true allergy and notify the appropriate provider or follow facility process to clarify the record

D. Give the medication as prescribed because drowsiness is expected with pain medicine

26. A practical nurse is preparing to move several residents from a day room to an interior hallway after a severe weather alert in a long-term care facility. Which item should the nurse identify as the greatest environmental safety hazard to correct first?

A. A rolling overbed table parked against the hallway wall

B. A portable oxygen tank secured upright beside a resident's wheelchair

C. A wet floor at the hallway entrance with no warning sign posted

D. A stack of folded blankets placed on chairs along the hallway

27. A client with acute confusion has a new chest tube and repeatedly reaches toward the drainage tubing when left alone. The client becomes calmer when the nurse explains the equipment and places the tubing out of the client's direct line of sight, but the client starts reaching again a few minutes later. Which action should the nurse take next to follow least-restrictive restraint principles?

A. Apply soft wrist restraints to prevent accidental tube removal

B. Ask a family member to hold the client's hands down at the bedside

C. Increase observation and continue distraction and reorientation measures

D. Secure the client in a jacket restraint until the confusion improves

28. While reviewing infusion pumps at the start of the shift, a nurse notices that a pump assigned to a client has been programmed to deliver 125 mL/hr instead of the prescribed 75 mL/hr. The infusion is still paused, so no fluid has been infused at the wrong rate. Which action is most appropriate?

A. Start the infusion at 125 mL/hr because the client has not been harmed yet

B. Reprogram the pump to 75 mL/hr, begin the infusion as prescribed, and report the near miss according to facility policy

C. Ask the next nurse whether the rate should be changed before taking any action

D. Document the incorrect pump setting in the chart but do not report it because no fluid was infused

29. A nurse is preparing to transfer a client from the bed to a chair with a gait belt. After the client sits on the side of the bed, the client reports feeling lightheaded. Which action should the nurse take next?

A. Proceed with the transfer quickly before the client becomes weaker

B. Return the client to a lying position and reassess before attempting the transfer again

C. Tighten the gait belt and ask the client to stand slowly

D. Have the client take deep breaths and continue the transfer with one assistant

30. A practical nurse is preparing to administer an intramuscular vaccine at a community clinic. After performing hand hygiene and applying clean gloves, the nurse cleans the client's deltoid with an alcohol swab. The client suddenly coughs, and the nurse reflexively uses the gloved hand that will hold the syringe to pull the privacy curtain closed before returning to the bedside. Which action should the nurse take next?

A. Proceed with the injection because the glove protected the nurse's hand from contamination

B. Remove the gloves, perform hand hygiene, apply new gloves, and clean the injection site again before giving the vaccine

C. Keep the same gloves and clean only the syringe hub with alcohol before giving the injection

D. Ask another nurse to give the vaccine because the procedure can no longer be completed safely

31. A nurse is preparing to irrigate an indwelling urinary catheter after surgery. The client is not on isolation precautions. The nurse expects possible splashing of urine during the procedure but no contact with clothing beyond routine bedside care. Which personal protective equipment should the nurse use?

A. Gloves only

B. Gloves and gown

C. Gloves and face shield or goggles

D. Gloves, gown, and N95 respirator

32. A practical nurse is opening a sterile dressing kit on a bedside table for a wound procedure. After unfolding the first flap away from the body, which step should the nurse take next to maintain asepsis?

A. Open the side flaps one at a time, touching only the outer edges of the wrapper

B. Reach across the center of the field to fully spread the drape flat

C. Pull the flap closest to the body outward first so the field opens faster

D. Adjust the inside of the wrapper with bare fingers if it begins to curl

33. A practical nurse is caring for a client with suspected pulmonary tuberculosis who is placed on airborne precautions. The client asks why the room door must stay closed. Which response by the nurse is best?

A. Keeping the door closed prevents contaminated equipment from being taken into the hallway.

B. Keeping the door closed helps contain tiny particles that can remain suspended in the air.

C. Keeping the door closed reduces the risk of infection from touching surfaces in the room.

D. Keeping the door closed is mainly to protect the client from other clients' infections.

34. A practical nurse is preparing to draw a venous blood sample from a client with no isolation orders. The client says, "I get light-headed with needles," and the nurse anticipates the client may jerk the arm suddenly during the venipuncture. Which action best reflects standard precautions?

A. Apply sterile gloves before the procedure because all blood draws require sterile technique

B. Wear gloves and have a face shield or mask with eye protection available if blood splashing is possible

C. Place the client in a private room before obtaining the specimen

D. Use hand sanitizer after the procedure instead of before and after client contact

35. A nurse is caring for a stable client 8 hours after an uncomplicated vaginal birth. Which finding should the nurse report to the provider?

A. A temperature of 37.8°C (100.0°F)

B. Lochia rubra with a few small clots

C. A boggy uterus that remains displaced to the right after voiding

D. Mild perineal edema at the site of a first-degree laceration

36. A nurse is reinforcing teaching with the parent of a 12-month-old child. Which behavior should the nurse identify as an expected developmental milestone at this age?

A. Uses a spoon without spilling most of the food

B. Builds a tower of 6 blocks

C. Pulls to a standing position and cruises along furniture

D. Rides a tricycle independently

37. A practical nurse is reinforcing teaching with a 45-year-old client whose youngest child recently moved out of the home. The client says, "I thought I would enjoy the quiet, but instead I feel sad and unsure what to do with myself." Which response by the nurse is most appropriate?

- A.** "These feelings suggest depression and should be reported right away."
- B.** "Many parents need time to adjust when family roles change, and finding new routines can help during this transition."
- C.** "You should avoid thinking about your child leaving so the sadness will pass faster."
- D.** "This reaction usually means there has been a problem with family bonding for years."

38. A practical nurse is reinforcing teaching with a 50-year-old client who says, "I don't want screening tests because finding a disease early would not change anything." Which response by the nurse is best?

- A.** Screening tests are mainly used to confirm a diagnosis after symptoms begin.
- B.** Early detection can identify problems before symptoms appear, when treatment may be more effective.
- C.** Most screening tests prevent disease from developing if they are done on schedule.
- D.** Screening is only useful for clients who already know they are at high risk.

39. A nurse is obtaining a health history from an adult client who says, "I had surgery years ago, but it wasn't a big deal." Which response by the nurse is best?

- A.** If it was minor, we can skip it and move on to your current problem.
- B.** About how long ago was the surgery, and what procedure was done?
- C.** Did the surgery make you nervous before it happened?
- D.** Were any of your family members upset about the surgery?

40. A practical nurse is admitting an adult client with chronic obstructive pulmonary disease. To make later comparisons most meaningful, which documentation best establishes the client's baseline oxygenation status?

- A.** Oxygen saturation 91% on 2 L/min nasal cannula at rest
- B.** Client states, "I hope I won't need oxygen for long."
- C.** Skin warm and dry with no visible rash
- D.** Ate 75% of the evening meal before admission

41. A nurse is reinforcing teaching with a client who will receive the intranasal live attenuated influenza vaccine. Which client statement should cause the nurse to notify the provider before administration?

- A.** I had a sore arm for a day after a flu shot a few years ago.

B. I am taking an antibiotic for a sinus infection.

C. I am 10 weeks pregnant.

D. I have not eaten since early this morning.

42. A nurse is assisting with discharge teaching for the parent of a healthy 2-year-old child. Which statement by the parent indicates understanding of an expected developmental characteristic of this age?

- A.** My child should be able to share toys easily with other children.
- B.** My child will probably use parallel play rather than cooperative play.
- C.** My child should be able to print simple letters of the alphabet.
- D.** My child will likely prefer abstract games with rules over pretend play.

43. A nurse is teaching a client how to care for a colostomy at home. The client speaks the same language as the nurse and says, "I understand what you're saying, but I don't know any of these medical words." Which action by the nurse is best to reduce this communication barrier?

- A.** Provide the client with a detailed handout that uses standard medical terminology
- B.** Repeat the teaching more slowly using the same terms
- C.** Use plain language and ask the client to explain the steps back in their own words
- D.** Complete the teaching quickly so the client does not become embarrassed

44. A client who was told today that a hysterectomy is needed says to the practical nurse, "I know they say it's necessary, but I feel like I'm losing part of who I am." Which response by the nurse best provides emotional support?

- A.** You should focus on the fact that the surgery will treat the problem.
- B.** Many clients do well after this surgery, so try not to think about it that way.
- C.** It sounds like this surgery feels like a very personal loss to you.
- D.** Why do you think this is affecting you so strongly?

45. A client with newly diagnosed chronic obstructive pulmonary disease tells the nurse, "When I start feeling short of breath, I tell myself there's no point trying anything, and I just sit there until it gets worse." Which response by the nurse best promotes effective coping?

- A.** "You should try not to think so negatively when your breathing changes."
- B.** "Let's talk about what you can do as soon as you notice shortness of breath so you feel more in control."
- C.** "Most clients with this diagnosis feel this way at first, so this is normal."

D. "If you avoid activity for a few days, the breathing episodes may stop happening as often."

46. A nurse in a community health clinic is interviewing a client who is upset about losing a job. During the interview, the client suddenly leans forward, speaks in a harsh voice, and says, "My supervisor ruined my life. I bought a gun yesterday, and I know where he parks after work." Which action should the nurse take first?

- A.** Ask the client to describe the argument with the supervisor in more detail
- B.** Notify the provider and follow agency procedure for an immediate threat of violence
- C.** Encourage the client to use deep-breathing techniques to calm down
- D.** Reassure the client that these feelings will likely pass with time

47. A client who stopped heavy alcohol use yesterday is on an inpatient unit. Which finding should the nurse recognize as most consistent with worsening alcohol withdrawal rather than uncomplicated opioid withdrawal?

- A.** Piloerection and yawning
- B.** Marked hypertension and coarse tremors
- C.** Muscle aches and dilated pupils
- D.** Abdominal cramping and rhinorrhea

48. A client with autism spectrum disorder is waiting for an x-ray when the imaging schedule is delayed. The client begins pacing, flapping both hands, and repeating, "Now x-ray, now x-ray." The waiting area is crowded and noisy. Which action by the nurse is most appropriate?

- A.** Tell the client to sit down and wait quietly until called
- B.** Move the client to a quieter area and give short, simple explanations about what will happen next
- C.** Ask several staff members to stand nearby in case the behavior escalates
- D.** Explain in detail why the department is behind schedule and why patience is necessary

49. A client with heart failure tells the nurse, "I know they explained this low-salt diet, but when I get home I'll probably just eat the way I always have." Which response by the nurse is most therapeutic?

- A.** "You need to follow the diet exactly or you could end up back in the hospital."
- B.** "Why would you choose not to follow instructions that are meant to help you?"
- C.** "It sounds like making that change feels difficult. Tell me what concerns you most about it."
- D.** "Most clients get used to the diet after a few weeks, so you should be fine."

50. A nurse is preparing to administer an evening medication when a client with a history of PTSD suddenly becomes tense after hearing a loud noise in the hallway. The client backs into a corner, breathes rapidly, and says, "Don't touch me. Stay away." Which action by the nurse is most appropriate?

- A.** Approach the client slowly and place a hand on the client's shoulder to provide reassurance
- B.** Speak calmly, give the client physical space, and say, "You are safe. I will stay back while we talk."
- C.** Tell the client that the noise was harmless and that there is no reason to be afraid
- D.** Leave the room immediately and return after the client has had time to calm down alone

51. A hospitalized older adult with dementia becomes increasingly agitated during the evening, pulls at the bedsheets, and says, "People keep walking through my room." The practical nurse notes glare from the hallway, a loud television, and several unfamiliar staff entering and leaving the room. Which action is most appropriate first?

- A.** Ask the client to explain exactly what is being seen so the nurse can reorient the client
- B.** Reduce environmental stimulation by dimming glare, lowering noise, and limiting unnecessary traffic in the room
- C.** Tell the client firmly that no one is in the room and the behavior must stop
- D.** Apply a restraint to prevent the client from pulling at the bedsheets

52. A client tells the nurse, "I have been feeling stressed for weeks. I keep saying yes to extra responsibilities, and then I get angry that I have no time for myself." Which client statement shows the need for further teaching about healthy stress management?

- A.** I should set limits and say no when I am already overloaded.
- B.** I can reduce stress by planning short breaks during the day.
- C.** If I ignore the stress and stay busy, the problem will usually go away on its own.
- D.** Talking with someone I trust may help me handle stress better.

53. A client who recently underwent a below-the-knee amputation says to the practical nurse, "I used to believe my life had a purpose, but now I feel abandoned and don't know who I am anymore." Which response by the nurse is most appropriate?

- A.** You should focus on your physical recovery first, and these feelings will likely improve later.
- B.** Many people feel this way after surgery, so try not to think about it too much.

C. It sounds like you are struggling with loss and meaning right now. Would you like to talk more about that or see someone who can support you spiritually?

D. You need to stay positive because negative thoughts can slow healing.

54. A nurse is caring for a client who is scheduled for a blood transfusion. The client says, "I need a few minutes to pray facing a certain direction before any treatment begins." The transfusion has not started, the client is stable, and the blood is available on the unit. Which action by the nurse is most appropriate?

A. Tell the client that the transfusion must begin immediately once the blood reaches the unit

B. Ask the client to complete the prayer after the transfusion has started to avoid delaying care

C. Provide privacy and assist with safe positioning for prayer before starting the transfusion

D. Explain that religious practices cannot be accommodated during hospital treatments

55. A client who identifies as transgender says to the nurse, "I need to tell you something important. I haven't been taking one of my medications because I was afraid the staff here would judge me." Which response by the nurse is best?

A. You should have told the provider that as soon as you arrived.

B. What medication have you been avoiding, and what concerns do you have about discussing it with the team?

C. The staff here are professionals, so there is no reason to worry.

D. Let's wait and talk about that after your other treatments are finished.

56. A nurse is preparing to change the dressing around a newly placed gastrostomy tube. Which action is most appropriate when providing routine stoma care?

A. Clean the area gently with mild soap and water, then pat the skin dry

B. Apply hydrogen peroxide to the site with each dressing change

C. Rotate the tube 360 degrees to prevent tissue adherence

D. Secure the external bolster tightly against the skin to limit movement

57. A nurse is assisting a client from the bed to a stretcher after surgery. Which action is most appropriate to promote safe movement and reduce friction during the transfer?

A. Pull the client across the bed by grasping under the client's arms

B. Ask the client to keep both legs straight and remain still while being dragged onto the stretcher

C. Use a draw sheet with assistance to lift and slide the client across

D. Raise the bed to its highest position and move the client alone as quickly as possible

58. A client who is 1 day postoperative after a chest tube insertion reports pain at the insertion site, especially when taking deep breaths. Which nonpharmacologic intervention should the nurse teach to help reduce pain during deep breathing exercises?

A. Place a pillow or folded blanket against the insertion area for support while breathing deeply

B. Limit deep breathing until the pain completely subsides

C. Massage the skin directly around the chest tube insertion site

D. Keep the client flat in bed during breathing exercises

59. A nurse is caring for a client with osteoarthritis of the hips who wants to put on socks without help. Which nursing intervention best supports safe independence with this activity of daily living?

A. Teach the client to bend forward at the waist to reach both feet quickly

B. Provide a long-handled sock aid for the client to use while seated

C. Ask the client to stand at the bedside while pulling on the socks

D. Complete the task for the client to prevent discomfort

60. A nurse is caring for an older adult client who has overflow urinary incontinence related to chronic urinary retention. Which finding should the nurse expect?

A. Loss of a small amount of urine when coughing or sneezing

B. Sudden, intense urge to void followed by inability to reach the toilet in time

C. Frequent dribbling of urine with a distended bladder

D. Urine leakage caused by inability to remove clothing quickly enough

61. A nurse is caring for a client who reports difficulty sleeping since being admitted to the facility. The client says, "At home I usually have a glass of wine before bed because it helps me relax." Which response by the nurse is best?

A. Continue the wine at bedtime because alcohol improves sleep quality

B. Avoid alcohol at bedtime because it can cause disrupted sleep later in the night

C. Replace the wine with a large snack right before bed to stay asleep longer

D. Stay in bed longer each morning to make up for interrupted sleep

- 62.** A practical nurse is caring for a client with very dry skin and itching on both lower legs. The client says, "I scratch a lot at night." Which intervention is most appropriate to help protect the client's skin?
- A.** Apply moisturizing lotion after bathing and as needed
 - B.** Wash the legs with hot water and strong soap each shift
 - C.** Dust the lower legs with powder to keep them dry
 - D.** Massage reddened areas on the legs to improve circulation
- 63.** A practical nurse is caring for a client with a new left-sided weakness and decreased awareness of the left arm after a stroke. Which action is most appropriate to reduce the risk of injury during morning care?
- A.** Place the client's call light on the left side to encourage use of the weaker arm
 - B.** Keep the bed in the highest position so the client can transfer more easily
 - C.** Approach the client from the right side and remind the client to check the left arm before moving
 - D.** Cover the left arm with extra blankets to keep it from hanging off the bed
- 64.** A client is on strict intake and output after a swallowing evaluation. During the shift, the client drinks 150 mL of water, receives 240 mL of tube feeding, and has 45 mL of water used to flush the feeding tube. The client also eats one 90 mL serving of pudding and voids 500 mL of urine. Which amount should the practical nurse record as the client's total intake for the shift?
- A.** 435 mL
 - B.** 525 mL
 - C.** 570 mL
 - D.** 1,025 mL
- 65.** A practical nurse is caring for a client who has been on strict bed rest for 1 week. Which intervention is most appropriate to reduce the risk of osteoporosis related to immobility?
- A.** Encourage isometric exercises as prescribed
 - B.** Limit fluid intake between meals
 - C.** Massage the client's lower legs every shift
 - D.** Keep the head of the bed elevated at 45 degrees most of the day
- 66.** A nurse is preparing to administer a scheduled oral medication to a client. After the nurse verifies the prescription, checks the MAR, and identifies the client with two identifiers, the client says, "I don't want to take that until I speak with my provider." Which action is most appropriate?
- A.** Explain that the medication is scheduled and must be taken now
 - B.** Document the refusal and notify the provider according to facility policy
 - C.** Leave the medication at the bedside in case the client changes their mind
 - D.** Ask another nurse to encourage the client to take the medication
- 67.** A practical nurse is preparing the 1700 medication pass and reviews the MAR for a client who is to receive acetaminophen 650 mg PO every 6 hours as needed for fever. The MAR shows the dose was documented as given at 1500 by the previous nurse. The client now has a temperature of 38.4°C (101.1°F) and asks for the medication. Which action should the nurse take?
- A.** Administer the acetaminophen now because the client currently has a fever
 - B.** Hold the medication and recheck the MAR timing because the dose is not yet due
 - C.** Ask the client whether the previous dose was helpful, then decide whether to give another dose
 - D.** Document the fever and skip the medication for the rest of the shift
- 68.** A nurse is reinforcing teaching for a patient who has been prescribed a buccal medication for angina episodes. Which patient statement shows correct understanding of this route of administration?
- A.** I should chew the tablet well before swallowing it.
 - B.** I should place the tablet between my cheek and gum and let it dissolve.
 - C.** I should put the tablet under my tongue and avoid talking until it melts.
 - D.** I should swallow the tablet with a small sip of water.
- 69.** A client with a history of severe allergic reactions to plasma proteins requires a red blood cell transfusion for symptomatic anemia. Which blood product should the nurse expect to administer?
- A.** Packed red blood cells
 - B.** Washed red blood cells
 - C.** Fresh frozen plasma
 - D.** Cryoprecipitate
- 70.** A nurse needs to administer 1,500 mL of IV fluid over 24 hours using an infusion pump. The tubing set will be changed after 8 hours, and the nurse wants to verify how much fluid should have infused by that time if the rate is correct. How many milliliters should the nurse expect to have infused after 8 hours?
- A.** 375 mL
 - B.** 500 mL
 - C.** 625 mL
 - D.** 750 mL

71. A practical nurse is preparing to administer a prescribed controlled opioid to a client. While comparing the medication label with the MAR, the nurse notices the client's documented allergy is hydrocodone, but the prescribed medication is hydrocodone/acetaminophen. Which action should the nurse take?

- A.** Administer the medication because acetaminophen is combined with the opioid
- B.** Ask another nurse to verify the allergy and then give the medication if the pain is severe
- C.** Hold the medication and clarify the prescription with the provider before administering it
- D.** Remove the medication from the dispensing system and leave it at the bedside until the provider responds

72. A nurse is reinforcing teaching for a postoperative client who is starting transdermal fentanyl for ongoing cancer pain after stable opioid use. Which statement by the client indicates a need for further teaching?

- A.** I should avoid placing a heating pad over the patch.
- B.** I need to remove the old patch before applying a new one.
- C.** I can cut the patch in half if my dose feels too strong.
- D.** I should place the patch on clean, dry skin.

73. A nurse is preparing to administer the next dose of oral prednisone to a client who has asthma. Which pre-administration data is most important for the nurse to review?

- A.** The client's most recent blood glucose level
- B.** The client's bowel movement pattern for the past 24 hours
- C.** The client's preferred time for taking medications
- D.** The client's daily fluid intake total

74. A client with *Clostridioides difficile* infection has been receiving oral vancomycin for 4 days. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client reports fewer loose stools and less abdominal cramping
- B.** The client's urine output has increased to 45 mL/hr
- C.** The client's heart rate has decreased from 104/min to 88/min
- D.** The client reports a stronger appetite at breakfast

75. A client with a new prescription for gentamicin has received the medication for 5 days. Which finding should the nurse recognize as the most important adverse response to report to the provider?

- A.** Ringing in the ears and decreased hearing
- B.** Mild redness at the IV insertion site
- C.** Increased appetite at mealtimes

D. Occasional heartburn after dinner

76. A patient who takes prednisone daily for rheumatoid arthritis asks the nurse which over-the-counter medication is safest to use for occasional knee pain. Which response by the nurse is best?

- A.** "Ibuprofen is a good choice because it reduces inflammation."
- B.** "Naproxen is preferred because it lasts longer than other pain medicines."
- C.** "Aspirin is safest because it can be bought without a prescription."
- D.** "Acetaminophen is usually the safer option because it does not increase the risk of stomach irritation the way some other pain medicines can."

77. A client with hypothyroidism is prescribed levothyroxine 75 mcg by mouth each morning. Before administering the medication, the nurse reviews the latest laboratory results and notes a thyroid-stimulating hormone (TSH) level of 22 mIU/L. Which action is most appropriate?

- A.** Administer the medication as prescribed
- B.** Hold the medication and request a lower dose
- C.** Withhold the medication until the TSH returns to the expected range
- D.** Administer half the dose and recheck the TSH later in the day

78. A nurse is preparing to start an IV infusion of 500 mL of 0.9% sodium chloride over 4 hours using tubing with a drop factor of 20 gtt/mL. What flow rate should the nurse set in drops per minute?

- A.** 25 gtt/min
- B.** 42 gtt/min
- C.** 40 gtt/min
- D.** 50 gtt/min

79. A nurse is obtaining a capillary blood specimen for bilirubin screening from a newborn. Which action is most appropriate?

- A.** Use the center of the heel to obtain the specimen
- B.** Puncture the lateral aspect of the heel
- C.** Squeeze the heel firmly to increase blood flow
- D.** Clean the puncture site with povidone-iodine and allow it to dry

80. A nurse is caring for a client with severe nausea and vomiting who now has irregular heartbeats and generalized weakness. Which laboratory result should the nurse identify as the priority finding?

- A.** Sodium 133 mEq/L
- B.** Potassium 2.8 mEq/L
- C.** Chloride 95 mEq/L

D. Magnesium 1.9 mg/dL

81. A client who is 18 weeks pregnant calls the prenatal clinic and says, "I suddenly developed chills and a fever of 101.8°F (38.8°C). I also have pain in my side and feel sick to my stomach." Which instruction should the nurse give first?

- A.** Drink extra fluids and keep the next routine prenatal appointment
- B.** Come to the clinic within the next 24 hours for a urine dipstick test
- C.** Go to the emergency department or provider immediately for evaluation
- D.** Take acetaminophen and rest at home unless contractions begin

82. A nurse is preparing to perform a bedside capillary blood glucose test for a client whose hands are cool after being transported from another unit. Which action is most appropriate to improve the accuracy of the point-of-care result?

- A.** Massage the puncture site firmly until a large drop of blood appears
- B.** Warm the client's hand before obtaining the capillary sample
- C.** Use the first drop of blood without wiping it away
- D.** Clean the site with alcohol and puncture it immediately while still wet

83. A practical nurse is obtaining vital signs on an adult client who is receiving a blood transfusion. Fifteen minutes after the transfusion begins, the client reports chills and low back pain. Which action should the nurse take first?

- A.** Stop the transfusion and obtain a full set of vital signs
- B.** Slow the transfusion rate and reassess in 15 minutes
- C.** Administer an antipyretic as prescribed and continue the transfusion
- D.** Document the findings as an expected response to transfusion

84. A client is scheduled for an intravenous pyelogram (IVP). Which information obtained by the nurse is most important to report to the provider before the procedure?

- A.** The client drank 240 mL (8 oz) of water 2 hours ago
- B.** The client has a history of allergy to iodinated contrast dye
- C.** The client reports urinary frequency for the past 2 days
- D.** The client says the last bowel movement was yesterday morning

85. A nurse reviews a client's ECG rhythm strip and notes the following: atrial rate 78/min, ventricular rate 42/min, regular P-P intervals, regular R-R intervals, and no consistent relationship between P waves and QRS complexes. Which interpretation is most appropriate?

- A.** First-degree AV block
- B.** Second-degree AV block type I
- C.** Second-degree AV block type II
- D.** Third-degree AV block

86. A nurse is preparing to obtain a venous blood specimen from an adult client who says, "I fainted once during a blood draw." Which action is most appropriate to reduce the risk of injury during the procedure?

- A.** Proceed with the venipuncture while the client remains standing so the vein is easier to access
- B.** Ask the client to pump the fist repeatedly throughout the blood draw
- C.** Place the client in a reclining or supine position before starting the venipuncture
- D.** Use the hand veins instead of the antecubital veins for all tubes

87. A practical nurse is caring for a client who returned from surgery 2 hours ago with a Jackson-Pratt drain. Which finding should the nurse report to the RN immediately?

- A.** The bulb contains 35 mL of dark red drainage
- B.** The client reports incisional pain rated 5 on a 0-to-10 scale
- C.** The drain tubing has no compressed suction and the bulb is expanded
- D.** The dressing has a small area of dried serosanguineous drainage

88. A client has a Salem sump nasogastric tube connected to low suction after abdominal surgery. During assessment, the nurse notices that the blue air vent is clamped and taped to the bedrail. Which action is most appropriate?

- A.** Leave the vent clamped to prevent gastric contents from leaking out
- B.** Irrigate the blue air vent with sterile saline every 4 hours
- C.** Unclamp the air vent and keep it above the level of the client's stomach
- D.** Advance the tube 2 inches to improve decompression

89. A nurse is caring for a client with a history of heart failure who suddenly says, "I can't catch my breath when I lie back." Which focused assessment should the nurse perform first?

- A.** Auscultate the lungs and check oxygen saturation
- B.** Ask how many pillows the client uses at home
- C.** Review the client's sodium intake from the past 24 hours

D. Inspect the lower extremities for longstanding varicose veins

90. A client with community-acquired pneumonia has been receiving IV antibiotics for 48 hours. Which finding best indicates the client is responding to treatment?

- A.** White blood cell count decreased from 16,000/mm³ to 9,800/mm³
- B.** Productive cough is present with thick yellow sputum
- C.** Crackles are still heard in both lower lobes
- D.** The client reports fatigue with ambulation to the bathroom

91. A client with a casted lower leg fracture presses the call light and reports severe pain that is not relieved by the prescribed analgesic. The nurse notes the toes are pale and cool, capillary refill is 4 seconds, and the client reports tingling in the foot. Which action should the nurse take first?

- A.** Elevate the leg above the level of the heart
- B.** Apply an ice pack over the casted area
- C.** Notify the provider immediately
- D.** Encourage the client to wiggle the toes for several minutes

92. A client with chronic adrenal insufficiency is admitted with vomiting from viral gastroenteritis and says, "I haven't been able to keep my steroid pills down since yesterday." Which action by the practical nurse is most important to take first?

- A.** Encourage the client to try taking the usual oral dose with crackers
- B.** Report the situation promptly because the client may need parenteral corticosteroid replacement
- C.** Withhold corticosteroids until the vomiting stops to prevent further stomach upset
- D.** Teach the client to double the next scheduled oral dose if one dose is missed

93. A client 6 hours after a carotid endarterectomy suddenly becomes difficult to arouse. The nurse notes a blood pressure of 198/104 mm Hg, a severe headache, and weakness of the right arm. Which action should the nurse take first?

- A.** Document the findings and recheck the client in 15 minutes
- B.** Place the client flat in bed to improve cerebral perfusion
- C.** Notify the provider or rapid response team immediately
- D.** Administer the prescribed PRN opioid for headache relief

94. A client has a chest tube connected to a drainage system after a lobectomy. During assessment, the nurse notes 220 mL of bright red drainage has collected in the past hour. The client is restless, and the blood pressure has dropped from 128/76 mm Hg to 98/60 mm Hg. Which action should the nurse take first?

- A.** Reposition the client onto the operative side
- B.** Clamp the chest tube near the insertion site
- C.** Notify the provider immediately
- D.** Mark the drainage level and recheck in 4 hours

95. A nurse is caring for a client who has a temperature of 40°C (104°F). Which nursing action promotes heat loss primarily through conduction?

- A.** Applying an ice pack to the client's axillae
- B.** Turning on a fan to blow air across the client's skin
- C.** Lowering the thermostat in the client's room
- D.** Encouraging the client to drink cool fluids

96. A nurse is preparing to irrigate a granulating wound with prescribed normal saline. Which action is most appropriate during the irrigation?

- A.** Direct the stream of solution from the least contaminated area toward the most contaminated area
- B.** Use a rapid, forceful stream to remove all visible tissue from the wound bed
- C.** Irrigate the wound from the surrounding skin inward toward the center of the wound
- D.** Save any unused saline from the irrigation bottle for the next dressing change

97. A nurse is teaching newly hired staff about using a bag-valve-mask during adult resuscitation. Which technique best supports effective ventilation during CPR?

- A.** Deliver each breath as quickly as possible to make the chest rise higher
- B.** Give forceful breaths between compressions to prevent carbon dioxide buildup
- C.** Provide each breath over about 1 second, watching for visible chest rise
- D.** Pause compressions after every breath to confirm that air entered both lungs

98. A client with chronic obstructive pulmonary disease is receiving oxygen at 2 L/min by nasal cannula. The nurse notes increasing dyspnea, expiratory wheezing, and thick sputum that is difficult to expectorate. The provider prescribes oxygen humidification. Which outcome best indicates that this intervention is effective?

- A.** The client's sputum becomes thinner and easier to cough up

- B.** The client's wheezing disappears immediately after humidification is started
- C.** The client's respiratory rate decreases to 10 breaths/min
- D.** The client's oxygen flow rate can be discontinued within 1 hour

99. A nurse is caring for a client with a tracheostomy who is receiving intermittent enteral feedings. Which nursing action is most appropriate to reduce the risk of aspiration during the feeding?

- A.** Deflate the tracheostomy cuff before starting the feeding
- B.** Place the client in semi-Fowler position during and after the feeding
- C.** Suction the tracheostomy every hour during the feeding
- D.** Remove the inner cannula before administering the feeding

100. A client with end-stage kidney disease has just returned from hemodialysis. Which finding should the practical nurse identify as the highest priority?

- A.** Blood pressure 88/54 mm Hg with cool, clammy skin
- B.** Mild muscle cramps in both calves
- C.** Small amount of dried blood on the access dressing
- D.** Reports feeling tired and wanting to sleep

Simulation Test 12 — Answer Key

1. C	26. C	51. B	76. D
2. C	27. C	52. C	77. A
3. C	28. B	53. C	78. B
4. C	29. B	54. C	79. B
5. C	30. B	55. B	80. B
6. C	31. C	56. A	81. C
7. D	32. A	57. C	82. B
8. B	33. B	58. A	83. A
9. C	34. B	59. B	84. B
10. C	35. C	60. C	85. D
11. B	36. C	61. B	86. C
12. C	37. B	62. A	87. C
13. A	38. B	63. C	88. C
14. B	39. B	64. B	89. A
15. C	40. A	65. A	90. A
16. B	41. C	66. B	91. C
17. D	42. B	67. B	92. B
18. B	43. C	68. B	93. C
19. B	44. C	69. B	94. C
20. C	45. B	70. B	95. A
21. C	46. B	71. C	96. A
22. A	47. B	72. C	97. C
23. C	48. B	73. A	98. A
24. B	49. C	74. A	99. B
25. C	50. B	75. A	100. A

Simulation Test 13

100 questions

1. The practical nurse is conducting a chart check before beginning scheduled treatments. Which finding requires the nurse's immediate follow-up?

- A.** A client with a blood transfusion started 10 minutes ago now reports chills and low back pain
- B.** A client receiving enteral feedings has a gastric residual of 120 mL
- C.** A client with pneumonia has a temperature of 100.8°F (38.2°C) after acetaminophen
- D.** A client 1 day after knee arthroplasty requests help before ambulating to the bathroom

2. A practical nurse is assigning tasks to an assistive personnel (AP) on a busy medical-surgical unit. Which task is most appropriate to delegate to the AP?

- A.** Teach a client how to use a walker before the first ambulation after surgery
- B.** Change the bed linens of a stable client who is out of bed in a chair
- C.** Assess a client's lung sounds after a nebulizer treatment
- D.** Decide whether a client's pain medication was effective 1 hour after administration

3. A nurse is preparing discharge teaching for a client who speaks English but reads at a very limited level. The client says, "Please don't make a big deal about it. Just tell my nephew everything, and he'll explain it later." The nephew is not present. Which action best supports the client's rights while promoting safe care?

- A.** Document that the client declined teaching and proceed with discharge
- B.** Provide verbal teaching in plain language and ask the client to show or explain key steps back
- C.** Delay all teaching until the nephew arrives so instructions can be given to the family instead
- D.** Ask the provider to rewrite the discharge instructions using medical terminology

4. A nurse is caring for an alert adult client whose sister asks to sign the consent form for a central line placement because the client is "too nervous to deal with paperwork." The client is able to answer questions appropriately and says, "I want to hear more before I decide." Which action by the nurse is best?

- A.** Have the sister sign because she is the closest relative present
- B.** Witness the client's signature now so the procedure is not delayed
- C.** Tell the client that consent is required for treatment and encourage signing

D. Hold the consent process and notify the provider that the client wants more explanation before deciding

5. A practical nurse is caring for a client who asks, "Can you email my test results to my personal email address?" Which action by the nurse is most appropriate?

- A.** Email the results if the client verbally confirms the address
- B.** Tell the client that test results can never be shared electronically
- C.** Follow the facility's approved process for releasing health information electronically
- D.** Ask a coworker to send the results from a personal phone for faster delivery

6. A practical nurse accidentally receives printed laboratory results for a client from another unit who is not assigned to the nurse. What should the nurse do?

- A.** Review the results in case they contain urgent findings that should be remembered later
- B.** Place the results in the correct client's room so the assigned staff can find them
- C.** Give the results only to the appropriate staff member and avoid accessing or discussing the information further
- D.** Keep the results at the nurses' station until someone from the other unit asks for them

7. During admission, an alert adult client asks, "If I complete an advance directive today, does that mean I can never change it later?" Which response by the nurse is best?

- A.** Yes, once it is signed, it cannot be changed under any circumstances.
- B.** No, a client can usually revise or revoke an advance directive later while still able to make decisions.
- C.** No, but only family members can request changes after it is signed.
- D.** Yes, unless the client is admitted to a different facility.

8. An alert adult client admitted for observation says, "I have never completed an advance directive because I thought my family could just decide everything if I became unable to speak for myself." Which response by the nurse is best?

- A.** Your family can always make decisions, so an advance directive is usually unnecessary.
- B.** An advance directive lets you state your wishes or choose a person to decide for you if you cannot make decisions later.
- C.** An advance directive is only used when a client has a terminal illness.
- D.** Once you are hospitalized, it is too late to make an advance directive.

9. A client who recently immigrated is scheduled for an elective thyroidectomy. The client speaks very little English and nods politely when the provider speaks. The client's bilingual teenage grandson says, "I can explain it to my grandmother so she can sign." Which action by the nurse is most appropriate?

- A.** Allow the grandson to translate because he knows the client best
- B.** Ask another staff member who speaks some of the client's language to explain the procedure
- C.** Obtain a qualified medical interpreter before the client signs the consent form
- D.** Have the client sign the form if the client nods and smiles during the discussion

10. A client with severe hearing loss is scheduled for an elective femoral-popliteal bypass. The provider discussed the procedure earlier with the help of a certified sign language interpreter, and the client signed the consent form. In the preoperative area, after receiving prescribed IV opioid pain medication, the client communicates in writing, "I remember the surgeon said there was another treatment option, but I can't remember what it was." Which action should the nurse take first?

- A.** Proceed because the consent was already signed after an interpreter-assisted discussion
- B.** Ask the client's spouse to review the alternatives so surgery is not delayed
- C.** Notify the provider that the client has additional questions and may need consent readdressed before surgery
- D.** Explain the treatment alternatives to the client in writing and witness the consent again

11. A client is preparing for discharge after treatment for a diabetic foot infection. The provider prescribes daily dressing changes at home and follow-up with a podiatry clinic in 1 week. The client says, "My daughter usually helps me, but she will be out of town for the next 2 weeks." Which action by the nurse is most appropriate before discharge?

- A.** Ask the client to skip dressing changes until the daughter returns
- B.** Arrange a home health referral for wound care support
- C.** Tell the client to call the podiatry clinic only if the wound looks worse
- D.** Delay all discharge teaching until the daughter is present

12. A client is being discharged after hospitalization for pneumonia. The client speaks limited English, has a new antibiotic prescription, and must return in 3 days for reevaluation. The client's adult son says, "I'll explain everything later," but he is leaving now for work. Which action by the nurse is most appropriate before discharge?

- A.** Provide the written discharge instructions in English and ask the client to sign them
- B.** Ask a bilingual housekeeping staff member to interpret the discharge instructions
- C.** Use a qualified medical interpreter to review medications, follow-up care, and warning signs with the client
- D.** Postpone all discharge teaching until the next clinic visit

13. A practical nurse joins a quality improvement project to reduce medication administration interruptions on a unit. The team wants to know whether a new "no interruption" zone around the medication cart is helping. Which measure would best show the client-care outcome of the change?

- A.** The percentage of medication passes during which staff used the marked zone correctly
- B.** The number of staff who attended the education session about interruptions
- C.** The rate of medication administration errors on the unit
- D.** The number of reminder signs posted near medication carts

14. A dialysis clinic wants to improve on-time treatment starts. Review of the last month shows that most delays begin when clients arrive on time but machine setup is incomplete because needed supplies are stored in different areas. The team is considering several actions. Which action is the best first quality improvement step?

- A.** Counsel clients again about arriving 15 minutes early for treatment
- B.** Reorganize and standardize setup supplies at each station, then test the change on a small number of stations
- C.** Require staff to stay late for 2 weeks until all treatments begin on time
- D.** Wait 3 months to collect more delay data before trying any change

15. A practical nurse is caring for a client with type 2 diabetes who is scheduled for discharge tomorrow. During morning care, the client says, "I can't read those tiny insulin syringe numbers," and later tells the nurse, "My hands shake too much to draw it up anyway." Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the client's comments in the chart and continue discharge teaching as planned

- B.** Notify the provider and ask to delay discharge until the client feels more confident
- C.** Report the client's vision and hand-control difficulties to the diabetes educator and case manager to support a safe medication plan at home
- D.** Tell the client to ask a family member to prepare the insulin doses after discharge

16. A practical nurse is caring for a client admitted with dehydration and confusion. The provider, pharmacist, and charge nurse are reviewing the client's medications after the family brings in pill bottles from home. Which information is most important for the nurse to contribute to the discussion?

- A.** The client prefers taking pills with applesauce instead of water
- B.** The family says the client organizes medications in a weekly pillbox at home
- C.** The medication list in the chart differs from two pill bottles, and the client received both names of the same blood pressure medication during this admission
- D.** The client usually takes morning medications after breakfast at home

17. A practical nurse is caring for a client who asks, "Can my neighbor come in and hear the provider explain my test results?" The client is alert, oriented, and states, "Yes, I want her here." Which action by the nurse is most appropriate?

- A.** Allow the neighbor to remain during the discussion after confirming the client's permission
- B.** Refuse because only family members may hear private health information
- C.** Ask the neighbor to leave and tell the client that written consent is always required first
- D.** Tell the provider to discuss the results only after discharge

18. A practical nurse is asked by a provider to insert a central venous catheter because the unit is short-staffed and the provider is running behind. The nurse has assisted with the procedure before but has never been trained or authorized to perform it. Which response by the nurse is most appropriate?

- A.** Perform the procedure because prior observation is enough preparation in an emergency staffing situation
- B.** Perform the procedure if another nurse agrees to supervise closely
- C.** Decline to perform the procedure and immediately notify the charge nurse that the request is outside the nurse's authorized scope
- D.** Attempt the procedure only if the client signs a consent form acknowledging the risk

19. A practical nurse in an outpatient dialysis center notices that a 68-year-old client has a dark bruise around one eye and several finger-shaped bruises on the upper arm. The client's adult nephew says, "He bumps into things all the time," and the client looks down and says, "It's nothing." The nurse suspects possible abuse. Which action is most appropriate?

- A.** Wait to report until the client confirms that the nephew caused the injuries
- B.** Document the findings objectively and follow agency policy for mandatory reporting
- C.** Call the nephew later to ask for a more detailed explanation of the injuries
- D.** Avoid reporting because the client did not directly ask for help

20. A practical nurse is caring for four clients with one assistive personnel (AP). Which task is most appropriate for the nurse to assign to the AP?

- A.** Reinforce teaching about use of an incentive spirometer for a client after abdominal surgery
- B.** Apply a prescribed topical medication to a client's rash
- C.** Feed a client who has left-sided weakness and was cleared for a regular diet after swallowing evaluation
- D.** Assess pain level 30 minutes after administering an opioid analgesic

21. A practical nurse in a primary care office is reinforcing follow-up instructions with a client who has heart failure. The client says, "I don't have a smartphone or computer, and the office keeps sending my appointment reminders and lab results through the patient portal. I missed my last blood test because I didn't know it had been scheduled." Which nursing action best promotes equitable access to care?

- A.** Tell the client that using the patient portal is the fastest method and encourage the client to ask a family member to check it regularly
- B.** Ask the office team to document the client's preferred communication method and arrange reminders in an accessible format such as phone calls or mailed instructions
- C.** Advise the client to come to the office weekly to ask whether any new appointments or test orders have been added
- D.** Explain that electronic communication is now standard and that the client is responsible for adapting to the system

22. A nurse in a preoperative area is preparing to administer a prescribed sedative to a client who is waiting for surgery. The client is anxious and says, "I'm here for my gallbladder surgery, and my surgeon just saw me." The nurse notices the client is not wearing an identification band because it was removed after becoming wet during a shower. Which action should the nurse take first?

- A.** Ask the surgeon to confirm the client's identity and then give the sedative
- B.** Use the room assignment and scheduled procedure to identify the client
- C.** Obtain a replacement identification band and verify the client with two identifiers before administering the medication
- D.** Have the client state their name and planned surgery, then administer the sedative

23. A nurse is preparing to use a facility infusion pump for a client receiving IV antibiotics. During the pre-use check, the nurse sees a biomedical engineering tag showing the pump's safety inspection expired 2 months ago. The pump appears to work normally, and no other pump is immediately available in the room. Which action should the nurse take?

- A.** Use the pump for this dose, then send it for inspection afterward
- B.** Silence any alerts and continue since the pump is functioning
- C.** Remove the pump from service and obtain equipment that is current for inspection
- D.** Ask the client to report any problems while the pump is running

24. A nurse is preparing to administer the first dose of trimethoprim-sulfamethoxazole to a client. During the preadministration interview, the client says, "I had a severe reaction to a sulfa drug before, but I don't remember the name." The medication label identifies it as a sulfonamide antibiotic. Which action should the nurse take?

- A.** Administer the medication because the client cannot recall the previous drug name
- B.** Hold the medication and clarify the allergy with the provider before administration
- C.** Give the medication with food to reduce the chance of another reaction
- D.** Administer a test dose first and observe the client closely

25. While entering a semiprivate room, the practical nurse sees a client using an electric fan that has been placed on the sink counter. The fan's cord is draped across the wet counter and plugged into an outlet next to the sink. What should the nurse do first?

- A.** Move the fan away from the sink and unplug it
- B.** Dry the counter and leave the fan in place
- C.** Ask maintenance to inspect the outlet later in the shift
- D.** Teach the client to keep electrical items away from water

26. A practical nurse is assigned to a long-term care unit when an overhead announcement states that the facility must shelter in place because of a hazardous chemical release in the surrounding area. Which action is most appropriate for the nurse to take?

- A.** Move residents to the parking lot for fresh air
- B.** Open hallway doors and windows to improve ventilation
- C.** Bring residents indoors and help keep windows and outside doors closed
- D.** Evacuate all ambulatory residents to another building across the street

27. A client with pneumonia and mild hypoxia becomes increasingly confused at night and repeatedly leans over the side rails, saying, "I need to find my shoes." The client is not violent but is at high risk for falling. Which action should the nurse take first to follow least-restrictive restraint principles?

- A.** Apply a vest restraint to keep the client from climbing out of bed
- B.** Request an order for bilateral wrist restraints until morning
- C.** Move the client closer to the nurses' station and use frequent reorientation and observation
- D.** Raise all 4 side rails to keep the client in bed

28. A nurse preparing to administer 2100 medications scans a client's wristband and sees an alert that the medication administration record already shows the same sedative as given 15 minutes earlier by another nurse. The client is awake, reports not remembering whether the pill was taken, and shows no signs of oversedation. Which action is most appropriate?

- A.** Administer the medication now so the client does not miss the scheduled dose
- B.** Hold the medication, assess the client, verify whether the dose was actually given, notify the appropriate nurse or provider as needed, and complete an incident report per facility policy
- C.** Ask the client to decide whether to take the medication based on how sleepy the client feels
- D.** Document the medication as refused and return the sedative to the dispensing system

29. A nurse is planning a transfer for a client who is alert and able to follow commands. The client has severe trunk instability, cannot bear weight, and is at risk for sliding out of a seated position during movement. Which transfer device is the safest choice?

- A.** Transfer board
- B.** Sit-to-stand lift
- C.** Full-body mechanical lift with a supportive sling
- D.** Gait belt with a one-person pivot transfer

30. A practical nurse is caring for a client with a draining wound that requires contact precautions. After completing the dressing change and removing gloves, the nurse's hands are visibly soiled with drainage residue. Which action is most appropriate next?

- A.** Apply an alcohol-based hand rub until the hands are dry
- B.** Wash hands with soap and water
- C.** Put on a new pair of gloves before touching anything else
- D.** Wipe the hands with a disinfectant wipe used for equipment

31. A nurse is preparing to perform endotracheal suctioning for a client with copious respiratory secretions. The client is not on isolation precautions. Which personal protective equipment is most appropriate for the nurse to wear?

- A.** Gloves only
- B.** Gloves and gown
- C.** Gloves, mask, and eye protection
- D.** Sterile gloves, gown, and shoe covers

32. A practical nurse is preparing a sterile field for insertion of an indwelling urinary catheter. The nurse opens the kit correctly and places the sterile drape on a clean bedside table. While waiting for the nurse administering the procedure to arrive, the nurse turns to document at the computer across the room, leaving the sterile field out of sight for about 1 minute. Which action is most appropriate when the nurse returns?

- A.** Use the field because no one touched it
- B.** Use the field if the top of the drape still looks dry and clean
- C.** Discard the field and set up a new sterile field
- D.** Ask another staff member whether the field was contaminated

33. A practical nurse is caring for four clients. Which client requires both contact and droplet precautions?

- A.** A client with respiratory syncytial virus (RSV)
- B.** A client with pulmonary tuberculosis
- C.** A client with varicella (chickenpox)
- D.** A client with methicillin-resistant *Staphylococcus aureus* (MRSA) in a wound

34. A practical nurse is about to remove a peripheral IV catheter from a client who has no isolation orders. The insertion site appears intact, but the nurse knows a small amount of blood may leak when the catheter is withdrawn. Which action best follows standard precautions?

- A.** Wear clean gloves during catheter removal and perform hand hygiene after glove removal
- B.** Use sterile gloves because any contact with blood requires a sterile technique
- C.** Remove the catheter with bare hands if the site shows no active bleeding
- D.** Apply a mask to the client before removing the catheter to prevent contamination

35. A nurse is assessing a 7-month-old infant in the clinic. Which finding should the nurse recognize as the most concerning and requiring further evaluation?

- A.** The infant rolls from abdomen to back
- B.** The infant transfers a toy from one hand to the other
- C.** The infant is unable to sit briefly while leaning on the hands
- D.** The infant laughs aloud when interacting with a caregiver

36. A practical nurse is reinforcing teaching with the parent of a 15-year-old who recently began high school. The parent says, "My teen changes friend groups often, argues about rules more than before, and seems very focused on fitting in." Which response by the nurse is most appropriate?

- A.** "This behavior suggests a serious emotional problem and should stop quickly with stricter discipline."
- B.** "These behaviors are common during adolescence as teens work on identity and peer relationships."
- C.** "Most teens this age should be more interested in family approval than peer acceptance."
- D.** "This usually means your teen is not mature enough for the responsibilities of high school."

37. A practical nurse is reinforcing teaching at a community clinic about prevention strategies. Which action is an example of primary prevention?

- A.** Teaching adults to use sunscreen to reduce the risk of skin cancer
- B.** Checking a client's blood glucose during a screening event
- C.** Referring a client with a breast lump for diagnostic testing
- D.** Providing foot care to a client with diabetic neuropathy

38. A nurse is obtaining a health history from an adult client who says, "I had a reaction after eating shellfish once." Which follow-up question is most important for the nurse to ask?

- A.** How often do you usually eat shellfish?
- B.** What symptoms did you have during that reaction?
- C.** When was the last time you ate shellfish?
- D.** Do other members of your family avoid shellfish?

39. A practical nurse is collecting baseline physical data for a newly admitted adult client with arthritis. Which additional information is most useful to document so future assessments can be compared with the client's usual status?

- A.** The client needed help buttoning a shirt at home because of hand stiffness
- B.** The client states that arthritis can flare during cold weather
- C.** The client reports being worried about the hospital stay
- D.** The client says a sibling also has arthritis

40. A nurse is reinforcing teaching for the parent of a child who received a live vaccine 5 days ago. The parent asks whether the child can receive a tuberculin skin test at tomorrow's visit. Which response by the nurse is best?

- A.** Yes, because live vaccines do not affect tuberculin skin test results
- B.** Yes, but only if the child has no redness or swelling from the vaccine
- C.** No, because a live vaccine can interfere with the tuberculin skin test response
- D.** No, because tuberculin skin testing is permanently contraindicated after a live vaccine

41. A nurse is contributing to a wellness class for adults in middle adulthood. Which focus is most appropriate for this age group?

- A.** Establishing trust with primary caregivers
- B.** Preparing for rapid language development
- C.** Maintaining productivity and contributing to family or community roles
- D.** Developing comfort with separation from parents

42. A nurse is teaching a client how to use a walker after hip surgery. The client speaks clearly, makes eye contact, and says, "I forgot my dentures this morning, so some words are hard for me to say." Which action by the nurse is best to reduce this communication barrier?

- A.** Ask the client to write all questions instead of speaking
- B.** Use yes-or-no questions and allow extra time for the client to respond
- C.** Delay all teaching until the client can obtain the dentures

D. Provide the instructions only to the client's family member

43. A nurse is preparing to teach a client about foot care after a new diagnosis of diabetes mellitus. The client says, "I don't understand those medical words very well. Can you explain it in a simpler way?" Which nursing action is best?

- A.** Provide the client with a detailed pamphlet that uses standard medical terminology
- B.** Ask the client to wait until a family member arrives to hear the instructions
- C.** Use plain language and ask the client to repeat the information in their own words
- D.** Give all of the teaching at once so the client receives complete information

44. A client who was diagnosed with chronic kidney disease 2 weeks ago tells the nurse, "I keep thinking about everything that might happen later, so I haven't even started reading the diet information they gave me." Which response by the nurse best promotes effective coping?

- A.** "Try not to think about it so much. Most clients feel better once they stop dwelling on the future."
- B.** "Let's focus on one manageable step right now. What part of the diet plan feels easiest for you to start with today?"
- C.** "You need to read all of the information before discharge so you can avoid complications."
- D.** "It may be better to wait until you feel less anxious before making any changes."

45. A nurse in a correctional health clinic is assessing a client who is angry after a court hearing was postponed. The client is sitting rigidly, avoids eye contact, and answers questions with one-word responses. Which additional finding should the nurse identify as the strongest indicator of an increased risk for violence?

- A.** The client asks to speak with an attorney before discussing the situation further
- B.** The client begins rapidly pacing the room and repeatedly clenches both fists
- C.** The client states, "I need to be alone right now" and turns away from the nurse
- D.** The client refuses to eat the meal tray that was brought in earlier

46. A client admitted for pancreatitis tells the nurse, "I usually drink every day, but I haven't had a drink since yesterday morning." Four hours later, the client is anxious, has a fine tremor, and reports nausea. Which additional assessment finding would most strongly suggest progression toward severe alcohol withdrawal?

- A.** Pinpoint pupils and slowed respirations
- B.** Visual hallucinations and increasing disorientation

C. Yawning, rhinorrhea, and muscle aches

D. Drowsiness and slurred speech

47. A client with major depressive disorder has been sitting silently in the dayroom for 30 minutes. When another client accidentally bumps the chair, the client suddenly stands up, yells, "Just leave me alone!", and knocks over a nearby magazine rack. Which action should the nurse take first?

A. Ask the client to explain what triggered the outburst

B. State calmly, "Come with me to a quiet area so we can talk"

C. Tell the client that this behavior is unacceptable on the unit

D. Administer a prescribed PRN sedative immediately

48. A client who recently received a diagnosis of multiple sclerosis tells the nurse, "I don't want my friends visiting me. I don't want them to see what I might become." Which response by the nurse is most therapeutic?

A. You should let your friends visit so you do not become isolated.

B. It sounds like you are worried about how this illness could change the way others see you.

C. Your friends probably will not think differently of you.

D. Try to focus on the fact that treatments are available.

49. A nurse is caring for a client who becomes angry after being told that a requested snack is not available. The client begins pacing, speaks in a harsh tone, and says, "You'd better fix this now." Which action by the nurse is most appropriate to help de-escalate the situation?

A. Stand directly in front of the client and explain the unit policy in detail

B. Tell the client that threatening language will result in loss of privileges

C. Speak calmly, maintain personal space, and offer simple choices for another snack

D. Leave the room immediately without responding to avoid reinforcing the behavior

50. A practical nurse in a rehabilitation unit is caring for an older adult who has mild dementia. The client becomes upset when asked open-ended questions and says, "Just tell me what I should do next." Which nursing approach is most appropriate to support the client's functioning?

A. Provide one simple step at a time and allow extra time for the client to respond

B. Ask the client to make as many independent decisions as possible to preserve memory

C. Speak loudly and repeat the full set of instructions until the client follows them

D. Correct the client each time confusion occurs so the client stays oriented

51. A client tells the nurse, "I have been so stressed that I jump from one task to another and end the day feeling like nothing got done." Which nursing suggestion is most appropriate to improve the client's stress management?

A. Complete the most difficult tasks first without taking breaks

B. Choose one or two priority tasks each day and schedule short breaks between them

C. Avoid making plans until the stressful period has passed

D. Stay busy with several tasks at once to keep the mind occupied

52. A client who recently learned that a chronic illness will require lifelong treatment says to the practical nurse, "I used to volunteer every week and felt useful. Now I lie here thinking my life no longer matters." Which response by the nurse is most appropriate?

A. "You should focus on getting stronger before worrying about those thoughts."

B. "Many people feel this way at first, so try not to think about it too much."

C. "It sounds like this illness is affecting your sense of purpose. Would you like to talk more about what gives your life meaning?"

D. "I can ask your family to remind you that you still have plenty to live for."

53. A nurse is caring for a client who recently underwent abdominal surgery. Before the nurse begins postoperative teaching, the client says, "Please do not tell me everything directly. In my family, my son usually receives medical information first and then explains it to me." The client is alert, oriented, and able to make decisions. Which action by the nurse is most appropriate?

A. Explain that all information must be given only to the client and refuse to include the son

B. Ask the client for permission to include the son in discussions while continuing to recognize the client's right to receive information and make decisions

C. Provide the teaching only to the son because the client requested that family communication be used

D. Delay all teaching until the provider can decide whether the son should receive the information

54. A client is admitted for postoperative pain control and says quietly to the nurse, "I identify as nonbinary. My partner is supportive, but my parent keeps saying this is just a phase." The client begins to cry and says, "I don't even want them visiting." Which nursing response is best?

- A.** "You should try to forgive your parent because family stress can slow recovery."
- B.** "Tell me more about what concerns you most about having your parent visit."
- C.** "I can ask your partner to explain your identity to your parent for you."
- D.** "It may be best to allow both visitors so your parent does not feel excluded."

55. A client with a new central vision loss diagnosis tells the nurse, "I can still get around, but I stopped writing cards and reading messages from my grandchildren. It just reminds me of what I can't do anymore." Which nursing response is best?

- A.** You should focus on the abilities you still have instead of thinking about what you lost.
- B.** Let's talk about which activities matter most to you and ways to adapt them so you can stay involved.
- C.** Your family can read and write things for you until you feel less upset.
- D.** Many clients feel this way at first, but most adjust once they accept the diagnosis.

56. A nurse is caring for a client receiving continuous enteral feeding through a gastrostomy tube. During the shift, the tubing becomes accidentally disconnected from the feeding bag, and formula leaks onto the bed linens. Which action is most appropriate?

- A.** Reconnect the same tubing and continue the feeding after wiping the ends with gauze
- B.** Clamp the tube, obtain new feeding supplies, and restart the feeding using clean equipment
- C.** Stop the feeding for the rest of the shift and document the interruption
- D.** Flush the tubing with sterile saline and reconnect it to avoid wasting formula

57. A nurse is preparing to help a client move from a bed into a wheelchair. The client can bear weight but tires easily. Which action should the nurse take first to promote safety during the transfer?

- A.** Lock the wheels on the bed and wheelchair
- B.** Place the wheelchair on the client's weak side
- C.** Ask the client to stand and pivot quickly
- D.** Raise the bed to the highest position

58. A nurse is reinforcing teaching for a client who is prescribed a low-residue diet during recovery from an acute flare of inflammatory bowel disease. Which food choice by the client indicates an understanding of the teaching?

- A.** Raw carrot sticks and hummus
- B.** Oatmeal topped with raisins
- C.** Baked chicken with white rice
- D.** Bean soup with whole-grain crackers

59. A nurse is caring for a client with a fresh cataract diagnosis who is embarrassed about needing help with shaving and hair care. The client says, "I don't want staff doing everything for me." Which response by the nurse is most appropriate?

- A.** "It is safer if the staff completes all of your grooming for now."
- B.** "Let's identify which parts of grooming you can do safely and help only with the parts that are difficult."
- C.** "You should wait until your vision improves before trying to groom yourself."
- D.** "Family members usually handle these tasks better than staff members do."

60. A nurse is caring for a client who has paraplegia and an areflexic bladder. The provider prescribes a bladder-emptying routine. Which intervention should the nurse include in the plan of care?

- A.** Encourage the client to void every 2 to 3 hours using relaxation techniques
- B.** Perform intermittent catheterization at scheduled intervals
- C.** Limit oral fluids to reduce the need for bladder emptying
- D.** Stimulate the suprapubic area to trigger a voiding reflex

61. A nurse is caring for a client who reports, "I have not been sleeping well since my leg cast was applied 2 days ago. I keep waking up because my foot feels numb and tingly." Which action should the nurse take first?

- A.** Reposition the client with extra pillows and reassess in 1 hour
- B.** Offer a warm drink to help the client relax before sleep
- C.** Assess the casted extremity for circulation, movement, and sensation
- D.** Teach the client to avoid napping during the day

62. A practical nurse is caring for a client who has a reddened bony prominence on the elbow. To determine whether the area may indicate an early pressure injury, which assessment should the nurse perform?

- A.** Massage the reddened area for 30 seconds and reassess the color
- B.** Press the area gently to see whether the redness blanches

- C.** Apply a warm compress and check whether the skin becomes pinker
- D.** Measure the circumference of the arm above and below the elbow
- 63.** A practical nurse is caring for a client with left-sided neglect after a stroke. Which action is most appropriate when setting up the client's meal tray?
- A.** Place all food and utensils on the client's right side and leave the left side empty
- B.** Turn the client's chair to face the left side of the room during meals
- C.** Arrange the tray so needed items are easy to reach on the right, while reminding the client to scan toward the left side
- D.** Feed the client quickly to prevent frustration related to missing items on the left side
- 64.** A practical nurse is collecting a 24-hour urine specimen that began at 7:00 a.m. Which action is correct?
- A.** Save the client's first void at 7:00 a.m. to start the collection
- B.** Discard the client's first void at 7:00 a.m., then collect all urine for the next 24 hours including the 7:00 a.m. void the next day
- C.** Collect urine for 24 hours, but stop the collection before the client's first void the next morning
- D.** Save only the largest void from each shift to reduce handling errors
- 65.** A practical nurse is caring for a client who has had very limited movement for 1 week following multiple fractures. Which action is most appropriate to maintain joint mobility while the client is on bed rest?
- A.** Place pillows under the knees at all times
- B.** Perform range-of-motion exercises to each joint as tolerated
- C.** Massage reddened areas over bony prominences
- D.** Keep the client in a high-Fowler position for most of the day
- 66.** A nurse is preparing to administer a scheduled oral medication to a client. After comparing the medication label with the MAR, the nurse notices the medication is due at 1000, but it is now 1130 and the dose was not given earlier. Which action is most appropriate?
- A.** Administer the medication now and document the actual time given
- B.** Skip the medication and wait until the next scheduled dose
- C.** Change the MAR time to 1130 so the medication matches the current time
- D.** Ask another nurse to give the medication because it is already late
- 67.** A practical nurse is preparing to administer a client's 0800 medications. The MAR shows glipizide 5 mg PO due before breakfast. While checking the chart, the nurse sees a provider prescription entered at 0730: "Discontinue glipizide; client will be NPO for testing this morning." The client has not received the medication yet. What should the nurse do?
- A.** Administer the glipizide because it is still listed on the MAR for 0800
- B.** Hold the glipizide and clarify that the MAR is updated before giving medications
- C.** Ask the client whether glipizide is usually taken at home and give it if the client says yes
- D.** Delay all morning medications until the test is completed
- 68.** A nurse is preparing to administer timolol eye drops to a patient with glaucoma. Which route of administration should the nurse use?
- A.** Ophthalmic
- B.** Otic
- C.** Intranasal
- D.** Sublingual
- 69.** A nurse is caring for a client with massive blood loss after trauma. The provider prescribes a blood product to expand intravascular volume and increase oxygen-carrying capacity. Which product should the nurse expect to administer?
- A.** Packed red blood cells
- B.** Albumin
- C.** Whole blood
- D.** Cryoprecipitate
- 70.** A nurse needs to infuse 720 mL of IV fluid evenly over 8 hours. After 3 hours, 510 mL remains in the bag. To have the full amount infuse on time, what rate should the nurse set on the infusion pump for the remaining time?
- A.** 84 mL/hr
- B.** 90 mL/hr
- C.** 102 mL/hr
- D.** 128 mL/hr
- 71.** A practical nurse is preparing to administer a prescribed controlled opioid to a client. At the bedside, the client says, "I don't want that pill right now. Leave it here, and I'll take it after I eat." Which action should the nurse take?
- A.** Leave the medication at the bedside and document that the dose was administered
- B.** Ask a family member to stay and watch the client take the medication later

- C.** Return the medication to the secured system according to facility policy and document the refusal
- D.** Crush the medication and mix it with food so the client can take it more easily later

72. A nurse is caring for a client who is receiving IV morphine through a patient-controlled analgesia (PCA) pump after surgery. The client reports pain at a level of 8 out of 10. The pump history shows many attempts but few delivered doses over the last hour. Which action by the nurse is best?

- A.** Tell the client to wait longer between button presses so the medication can build up
- B.** Assess the client, verify the PCA settings and IV line patency, and notify the provider that pain control is inadequate
- C.** Ask a family member to press the PCA button when the client appears uncomfortable
- D.** Turn off the PCA pump for 30 minutes and then restart it

73. A nurse is preparing to administer the next dose of oral potassium chloride to a client receiving hydrochlorothiazide. Which pre-administration data is most important for the nurse to review?

- A.** The client's most recent serum potassium level
- B.** The client's last bowel movement
- C.** The client's daily fluid intake
- D.** The client's pain rating

74. A client with osteoarthritis has been taking prescribed ibuprofen for 1 week. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client reports less joint pain when walking
- B.** The client has a heart rate of 88/min
- C.** The client voids 1,500 mL in 24 hours
- D.** The client has a fasting blood glucose of 96 mg/dL

75. A client with rheumatoid arthritis has been taking methotrexate once weekly for 6 weeks. Which new finding should the nurse report to the provider immediately as a possible serious adverse response?

- A.** Mild nausea the day after the dose
- B.** Small mouth sores and a sore throat
- C.** Decreased appetite at mealtime
- D.** Occasional headache relieved by rest

76. A patient takes metronidazole for a gastrointestinal infection. During discharge teaching, the patient says, "I plan to have wine with dinner tonight since I started feeling better." Which response by the nurse is best?

- A.** That is acceptable if the wine is limited to one glass.

B. Alcohol should be avoided while taking metronidazole because it can cause a severe reaction.

C. Alcohol is only a problem if it is taken at the same exact time as the medication.

D. You should skip tonight's dose of metronidazole if you plan to drink alcohol.

77. A client with *Pneumocystis jirovecii* pneumonia is prescribed trimethoprim-sulfamethoxazole IV. Before administering the next dose, the nurse reviews the laboratory results and notes a serum potassium level of 6.1 mEq/L. Which action is most appropriate?

- A.** Administer the medication as prescribed and recheck the potassium level after the infusion
- B.** Hold the medication and notify the provider of the critical potassium level
- C.** Give the medication with food to reduce gastrointestinal upset
- D.** Slow the infusion rate and continue to monitor the client

78. A nurse is teaching a client who has a new prescription for a fluticasone inhaler to use twice daily for asthma control. Which instruction should the nurse include?

- A.** Use the inhaler when sudden wheezing begins to get immediate relief.
- B.** Rinse your mouth with water after each use of the inhaler.
- C.** Stop using the inhaler once your breathing improves for 2 days.
- D.** Take the medication only on days when you expect asthma triggers.

79. A practical nurse is collecting data on four clients in a skilled nursing facility. Which client should the nurse identify as being at greatest risk for developing sepsis?

- A.** A client with a temperature of 37.2°C (99°F) who has a healing surgical incision with well-approximated edges
- B.** A client with an indwelling urinary catheter who is newly confused and has cloudy urine
- C.** A client with chronic heart failure who has bilateral ankle edema at the end of the day
- D.** A client with osteoarthritis who reports morning joint stiffness that improves with movement

80. A nurse is reviewing laboratory results for a client who has bacterial endocarditis and is receiving IV gentamicin. The client reports decreased urine output and a feeling of fullness in both ears. Which laboratory finding is most important for the nurse to report to the provider?

- A.** Blood urea nitrogen (BUN) 16 mg/dL
- B.** Serum creatinine 2.1 mg/dL
- C.** WBC count 12,000/mm³
- D.** Hemoglobin 11.2 g/dL

- 81.** A client who is 26 weeks pregnant calls the clinic and says, "I've had itching on my hands and feet for the past several nights, and it's getting worse. I don't have a rash." Which response by the nurse is most appropriate?
- A.** Advise the client to apply a moisturizing lotion and mention it at the next routine prenatal visit
 - B.** Instruct the client to come in promptly for evaluation of possible liver-related pregnancy complications
 - C.** Tell the client that this is a normal skin change of pregnancy caused by stretching
 - D.** Recommend soaking the hands and feet in cool water and avoiding tight shoes for 48 hours
- 82.** A nurse is about to perform a bedside urine dipstick test for a client with suspected dehydration. While gathering supplies, the nurse notices the reagent strip container expired 3 days ago but has remained tightly capped and appears dry. What is the nurse's best action?
- A.** Use the strips because the container was sealed and expired only recently
 - B.** Use the strips for this client, but document that the expiration date has passed
 - C.** Discard the expired strips and obtain a new, in-date reagent strip container
 - D.** Perform the test with the expired strip, then confirm only abnormal results with the laboratory
- 83.** A practical nurse is obtaining routine vital signs on an adult client. The automated blood pressure machine gives a reading of 78/46 mm Hg, but the client is awake, talking, and denies dizziness. Which action should the nurse take next?
- A.** Document the reading because the machine provides an objective result
 - B.** Repeat the blood pressure manually and assess the client for other signs of poor perfusion
 - C.** Place the client flat and call the rapid response team immediately
 - D.** Wait 30 minutes and recheck the blood pressure with the same machine
- 84.** A client is scheduled for a cerebral angiography later today. Which assessment finding is most important for the nurse to report to the provider before the procedure?
- A.** The client ate breakfast 6 hours ago
 - B.** The client has a documented allergy to latex
 - C.** The client reports taking metformin this morning
 - D.** The client says there is a possibility of pregnancy
- 85.** A nurse reviews a client's ECG tracing and notes a P wave before each QRS complex, a regular rhythm, and a PR interval of 0.16 second. The QRS duration is 0.08 second, and the ventricular rate is 76/min. Which interpretation is most appropriate?
- A.** Normal sinus rhythm
 - B.** Sinus bradycardia
 - C.** First-degree AV block
 - D.** Atrial fibrillation
- 86.** A nurse is preparing to obtain a venous blood specimen from an adult client. The client repeatedly pumps and opens the fist while the tourniquet is in place to make the veins easier to see. Which nursing action is most appropriate?
- A.** Continue with the procedure because fist pumping improves vein access without affecting results
 - B.** Ask the client to relax the hand and keep the fist still while the nurse proceeds
 - C.** Remove the tourniquet and apply it more tightly above the selected site
 - D.** Have the client lower the arm below heart level and keep pumping the fist until the vein enlarges
- 87.** A practical nurse is caring for a client who is scheduled for surgery in 2 hours. Which action by the nurse is most appropriate to help reduce the risk of postoperative wound infection?
- A.** Verify that the client signed the operative consent form
 - B.** Ask the client to void immediately before preoperative medication is given
 - C.** Clip hair at the operative site only if prescribed and immediately before surgery
 - D.** Apply lotion to the skin around the planned incision site after the preoperative bath
- 88.** A nurse is caring for a client with a chest tube connected to a water-seal drainage system. During assessment, the nurse notes that the tubing has become disconnected from the drainage system. Which action should the nurse take first?
- A.** Place the disconnected end of the chest tube in sterile water
 - B.** Clamp the chest tube near the insertion site
 - C.** Apply an occlusive dressing over the chest tube insertion site
 - D.** Reconnect the tubing and wait to notify the provider until the next round
- 89.** A nurse is caring for a client 1 hour after receiving an IV opioid for postoperative pain. The client is difficult to arouse and answers questions slowly. Which focused assessment should the nurse perform first?
- A.** Check the client's respiratory rate and oxygen saturation
 - B.** Ask the client to rate pain on a 0 to 10 scale
 - C.** Inspect the incision for drainage
 - D.** Review the client's bowel sounds in all quadrants

90. A client with heart failure is started on digoxin for symptomatic control. Which finding most strongly indicates the medication is having the desired therapeutic effect?

- A.** Apical pulse decreases from 112/min to 88/min and the client reports less shortness of breath with activity
- B.** Urine output increases from 30 mL/hr to 90 mL/hr during the shift
- C.** Blood pressure rises from 104/62 mm Hg to 138/86 mm Hg
- D.** Serum potassium increases from 3.2 mEq/L to 4.8 mEq/L

91. A client 18 hours after a transurethral resection of the prostate has continuous bladder irrigation running. The nurse notes increasing suprapubic distention, reports of bladder spasms, and a sudden decrease in urine drainage into the collection bag. Which action should the nurse take first?

- A.** Increase the irrigation flow rate
- B.** Check the tubing for kinks or obstruction
- C.** Administer the prescribed antispasmodic medication
- D.** Document the findings and reassess in 30 minutes

92. A client with chronic atrial fibrillation takes warfarin daily at home. Which statement by the client indicates a need for further teaching?

- A.** I will use a soft toothbrush when brushing my teeth.
- B.** I should call my provider if I notice black, tarry stools.
- C.** I can start taking ibuprofen for knee pain without checking first because it is sold over the counter.
- D.** I will try to take my medication at the same time each day.

93. A client with a severe bacterial infection is being monitored on a medical unit. Which finding most strongly suggests the client is progressing from sepsis to septic shock?

- A.** Temperature 38.9°C (102°F) and heart rate 118/min
- B.** Blood pressure 82/40 mm Hg that remains low after fluid replacement
- C.** White blood cell count 18,000/mm³
- D.** Respiratory rate 24/min with crackles at the lung bases

94. A client is 24 hours postoperative after a lumbar laminectomy and has a Hemovac drain. The nurse notes 120 mL of drainage collected during the previous 8 hours and 10 mL during the last 8 hours. The drainage has changed from dark red to light pink. The insertion site dressing is dry, and the client reports improved incisional pressure. Which interpretation is most appropriate?

- A.** The drain is functioning as expected, with decreasing output and lighter drainage over time

B. The drain should be removed immediately because any drainage after 24 hours is abnormal

C. The client is developing a wound infection, as shown by the change in drainage color

D. The nurse should suspect hemorrhage because the output was higher during the first 8 hours

95. A nurse is teaching a newly licensed practical nurse about methods of body heat transfer. Which client situation is the best example of heat loss through convection?

- A.** A client cools off as air from a fan moves across the skin
- B.** A client loses heat by lying on a cold mattress
- C.** A client becomes cooler as perspiration dries on the skin
- D.** A client loses heat by giving off infrared heat to a nearby window

96. A nurse is assessing a client's postoperative incision on day 4 after surgery. Which finding should the nurse recognize as most suggestive of wound dehiscence?

- A.** A sudden increase in serosanguineous drainage from the incision
- B.** Mild incisional pain when the client changes position
- C.** A thin pink scar forming along the wound line
- D.** Slight localized warmth at the incision site

97. A nurse sees an adult visitor suddenly grab the throat, become unable to speak, and make no sound while attempting to cough after taking a bite of food in the cafeteria. What is the nurse's priority action?

- A.** Begin abdominal thrusts
- B.** Give 2 rescue breaths
- C.** Start chest compressions
- D.** Encourage the visitor to keep coughing

98. A client with pneumonia is receiving oxygen by nonrebreather mask. During assessment, the nurse notes the reservoir bag is fully collapsed during inspiration. Which action is most appropriate?

- A.** Lower the oxygen flow rate to reduce oxygen toxicity risk
- B.** Encourage the client to take slower, deeper breaths
- C.** Increase the oxygen flow rate so the reservoir bag stays partially inflated
- D.** Switch the client to a nasal cannula at 2 L/min

99. A nurse is caring for a client who had a tracheostomy placed 3 days ago. During routine care, the nurse notes the skin around the stoma is red and moist with a small amount of drainage on the old dressing. Which nursing action is most appropriate?

- A.** Clean the site, apply a dry sterile dressing, and place the tracheostomy ties loosely to reduce pressure
- B.** Clean the site, place a sterile split gauze dressing, and secure the ties so that one finger fits under them

- C.** Remove the dressing and leave the stoma open to air to promote drying
- D.** Apply petroleum gauze around the stoma to protect the skin from drainage

100. A client on mechanical ventilation becomes suddenly agitated. The ventilator's low exhaled-volume alarm sounds, and the nurse hears air leaking around the client's mouth. Which action should the nurse take first?

- A.** Reposition and secure the endotracheal tube, then reassess ventilator function
- B.** Increase the fraction of inspired oxygen on the ventilator
- C.** Suction the airway using sterile technique
- D.** Administer a prescribed sedative PRN

Simulation Test 13 — Answer Key

1. A	26. C	51. B	76. B
2. B	27. C	52. C	77. B
3. B	28. B	53. B	78. B
4. D	29. C	54. B	79. B
5. C	30. B	55. B	80. B
6. C	31. C	56. B	81. B
7. B	32. C	57. A	82. C
8. B	33. A	58. C	83. B
9. C	34. A	59. B	84. D
10. C	35. C	60. B	85. A
11. B	36. B	61. C	86. B
12. C	37. A	62. B	87. C
13. C	38. B	63. C	88. A
14. B	39. A	64. B	89. A
15. C	40. C	65. B	90. A
16. C	41. C	66. A	91. B
17. A	42. B	67. B	92. C
18. C	43. C	68. A	93. B
19. B	44. B	69. C	94. A
20. C	45. B	70. C	95. A
21. B	46. B	71. C	96. A
22. C	47. B	72. B	97. A
23. C	48. B	73. A	98. C
24. B	49. C	74. A	99. B
25. A	50. A	75. B	100. A

Simulation Test 14

100 questions

1. A practical nurse in a long-term care facility is told about four client situations during medication pass. Which client should the nurse see first?

- A. A client with Parkinson disease who is 20 minutes late for a scheduled dose of carbidopa-levodopa
- B. A client with chronic heart failure who has gained 2 lb since yesterday and has mild ankle edema
- C. A client with diabetes who is pale, sweating, and difficult to awaken before lunch
- D. A client with osteoarthritis who reports knee pain of 7/10 while walking to the dining room

2. A practical nurse is supervising an assistive personnel (AP) in an outpatient clinic. Which task is most appropriate for the nurse to delegate to the AP?

- A. Teach a client how to collect a clean-catch urine specimen at home
- B. Call a provider to report a client's new allergic rash after starting an antibiotic
- C. Perform a routine height and weight measurement for a stable adolescent
- D. Decide whether a client with chest discomfort can wait for a later appointment time

3. A nurse is arranging education for an adult client who is alert, oriented, and newly prescribed a complex medication schedule. The client says, "I understand spoken English, but I read very little and I do best when information is shown to me." The client's friend says, "Just give me the papers. I'll explain them later." What is the nurse's best action?

- A. Provide the written instructions to the friend and document that teaching was completed
- B. Ask the provider to simplify the prescription so written teaching is unnecessary
- C. Use plain-language verbal teaching with demonstration and have the client repeat the schedule back
- D. Delay all teaching until a family member can come and review the instructions with the client

4. A practical nurse is caring for a client with a communicable disease. During the shift, the nurse notices another staff member photographing the client's paper flow sheet with a personal smartphone to "review the vital signs later." Which action should the nurse take first?

- A. Ask the staff member to delete the photo and report the incident according to facility policy
- B. Wait until the end of the shift to remind the staff member about confidentiality rules

C. Document the client's vital signs again so the photo is no longer needed

D. Tell the client that staff sometimes use phones for work-related reminders

5. A practical nurse is caring for a well-known local business owner. During a meal break in the hospital cafeteria, the nurse's neighbor says, "I heard he was admitted here. You work on that floor—what happened to him?" Which response by the nurse is most appropriate?

- A. He is stable now, but I can't discuss the details of his condition.
- B. I can't share any information about whether someone is a client here.
- C. He asked for privacy, so I am not allowed to talk about his care.
- D. You should ask his family because they may be willing to update you.

6. During admission, an alert adult client tells the nurse, "I already have an advance directive, but I don't want my daughter to see it right now." Later, the daughter asks the nurse for a copy of the document. What is the nurse's best response?

- A. Give the daughter a copy because immediate family members have a right to review it
- B. Ask the daughter which parts she wants to see before deciding what to share
- C. Explain that the document is part of the client's private health information and cannot be shared without the client's permission
- D. Provide a summary of the document instead of the full copy to protect confidentiality

7. A nurse is admitting an alert adult client who says, "I signed an advance directive in another state several years ago. Since I'm in a different state now, I guess it doesn't matter anymore." Which response by the nurse is most appropriate?

- A. That document is automatically invalid here, so you will need to complete a new one today
- B. Advance directives are only used in the state where they were first signed
- C. Let's place a copy in your record and notify the provider so the document can be reviewed and your current wishes clarified if needed
- D. Because the document is old, it should not be used unless your family approves it first

8. A nurse is preparing an alert adult client for a scheduled knee arthroscopy. The client looks at the consent form and says, "I want the surgery, but I don't want to read all of this. Just tell me where to sign." Which action by the nurse is most appropriate?

- A.** Ask the client to sign because agreeing to the surgery is enough to show informed consent
- B.** Explain the procedure, risks, and alternatives in detail before the client signs
- C.** Witness the signature only after confirming the provider has already obtained informed consent
- D.** Have a family member review the form with the client and decide whether the client should sign

9. A nurse is completing preoperative preparation for a client scheduled for an elective abdominal surgery. The consent form was signed earlier after the surgeon's discussion. In the holding area, the client quietly tells the nurse, "I still want the surgery, but my spouse threatened to leave me if I refused, so I felt like I had no real choice." Which action should the nurse take first?

- A.** Notify the surgeon and withhold further preparation because the consent may not be voluntary
- B.** Continue the preparation because the client states a desire to proceed with surgery
- C.** Ask the spouse to leave and then have the client sign a new consent form
- D.** Request a social worker after the surgery because the immediate priority is staying on schedule

10. A client is being discharged after hospitalization for a new colostomy. The client can empty the pouch with prompting but says, "I still get mixed up about which supplies to use and when to call for help." The client lives with an older sibling who also wants to learn the care. Which nursing action is most appropriate before discharge?

- A.** Ask the client to sign the discharge papers and review the written instructions again at home
- B.** Arrange for home health nursing follow-up to reinforce ostomy care and assess adaptation at home
- C.** Request that the provider delay discharge until the client can perform all ostomy care independently
- D.** Tell the sibling to watch online videos about ostomy care after the client is discharged

11. A client is ready for discharge after treatment for a deep-vein thrombosis. The provider prescribes an anticoagulant and follow-up laboratory testing. During discharge teaching, the client says, "I move between friends' houses, so I never know where my mail goes, and I don't always have a phone." Which action by the nurse is the priority to support safe follow-up?

- A.** Encourage the client to keep the follow-up appointment if transportation becomes available

- B.** Provide the client with extra printed instructions in case the first copy is lost
- C.** Notify case management or social services to help arrange reliable follow-up contact and access to care
- D.** Ask the client to choose one friend who can remind the client to take the medication

12. A long-term care facility wants to improve the accuracy of residents' medication lists after transfers from the hospital. The team plans to test a new admission reconciliation checklist on one unit for 2 weeks before using it facility-wide. What is the best reason for starting with this small test?

- A.** It allows the team to see whether the change is workable and identify problems before expanding it
- B.** It guarantees the checklist will improve resident outcomes on every unit
- C.** It removes the need to collect baseline data before the project begins
- D.** It ensures staff will not need any further training after the test period

13. A clinic is trying to reduce delays in calling clients about critical laboratory results. Baseline review shows that 18 of 20 delayed notifications happened on evenings when only one nurse was covering phones, triage, and incoming results. Which action is the most appropriate next step in the quality improvement process?

- A.** Immediately require all nurses to stay 1 hour late until delays stop
- B.** Begin disciplinary action for any nurse involved in a delayed call
- C.** Test a focused change aimed at evening result handling, then measure whether delays improve
- D.** Wait several months to see whether the problem improves without intervention

14. A practical nurse is caring for a client admitted with community-acquired pneumonia. At interdisciplinary rounds, the respiratory therapist asks whether the client is improving with activity. Which nurse response provides the most useful information for team planning?

- A.** The client says breathing feels a little better today.
- B.** The client ambulated 40 feet with a walker, stopped once to rest, and oxygen saturation decreased from 94% at rest to 88% during activity on 2 L/min by nasal cannula.
- C.** The client has been cooperative with treatment and wants to go home soon.
- D.** The client received all scheduled respiratory medications this morning.

15. A practical nurse is caring for a client who is 1 day postoperative after abdominal surgery. The client has not walked since surgery. During the morning assessment, the nurse notes pain 8/10 with movement, dizziness when sitting at the bedside, and a blood pressure drop from 128/76 mm Hg lying to 102/64 mm Hg standing. Physical therapy is scheduled to arrive in 15 minutes. What is the nurse's best action to support interdisciplinary information sharing?

- A.** Wait for physical therapy to assess the client and then read the therapist's note later.
- B.** Assist the client back to bed and promptly notify the RN and physical therapist of the client's pain, dizziness, and orthostatic blood pressure change before therapy begins.
- C.** Encourage the client to continue with therapy because early ambulation is expected after surgery.
- D.** Document the findings in the chart and discuss them with the provider at the next routine rounds.

16. A practical nurse is charting at the nurses' station when the unit clerk asks, "What surgery is the client in room 208 having? Her employer is on the phone and says he needs to know why she missed work." Which action by the nurse is most appropriate?

- A.** Provide the reason for hospitalization because the caller represents the client's workplace
- B.** Ask the unit clerk to give the caller a brief update without using medical terms
- C.** Refuse to share the information and state that no client information can be released without the client's permission and proper process
- D.** Confirm only that the client is hospitalized, but do not discuss the surgery

17. A practical nurse is caring for a client with a new tracheostomy. The nurse observes an experienced unlicensed assistive personnel suction the client because "I've done this for years and the RN knows I help when things get busy." Facility policy states suctioning a tracheostomy is a licensed nursing task. The client is breathing comfortably after the suctioning. Which action by the practical nurse is most appropriate?

- A.** Say nothing because the client was not harmed and the assistive personnel is experienced
- B.** Document that the assistive personnel performed the suctioning so the record is complete, then continue care
- C.** Immediately report the situation to the charge nurse according to policy and ensure future care is assigned within scope
- D.** Tell the assistive personnel to ask permission next time, but do not involve anyone else

18. A practical nurse in a primary care clinic is caring for a 7-year-old child with abdominal pain. During assessment, the nurse notices round bruises on both upper arms and a healing cut above the lip. The caregiver says the child is "just clumsy." The nurse suspects possible abuse but is not completely certain. What is the best action?

- A.** Wait to report until the nurse can confirm abuse with more evidence
- B.** Discuss the suspicion with the caregiver and decide whether reporting is still necessary
- C.** Follow agency procedure for reporting suspected abuse promptly
- D.** Document the findings and report only if the child later discloses abuse

19. A practical nurse in a long-term care facility notices that a resident with limited mobility has a new bruise on the inner thigh and repeatedly whispers, "Don't let him help me anymore." Later, the resident becomes tearful when a specific assistive personnel member enters the room. What information should the nurse include in documentation to best support mandatory reporting?

- A.** The resident was probably abused by the staff member based on the nurse's judgment
- B.** The resident had a bruise and seemed upset, so abuse is likely occurring
- C.** A detailed, objective description of the bruise, the resident's exact statements, and the time the findings were observed
- D.** A summary stating the nurse believes the staff member should be suspended immediately

20. A practical nurse is supervising an assistive personnel (AP) on an orthopedic unit. Which task is appropriate for the nurse to delegate to the AP?

- A.** Teach a client how to use a newly prescribed walker before the first ambulation
- B.** Reinforce discharge instructions about enoxaparin injections after hip surgery
- C.** Measure and record the amount of drainage in a Hemovac after knee replacement
- D.** Assess a client's report of increasing numbness in the toes after cast application

21. A practical nurse in a primary care clinic is scheduling follow-up for four clients. Which client situation most clearly shows a financial barrier to equitable access to care that requires nursing advocacy?

- A.** A client says the clinic parking lot is crowded and prefers an earlier appointment time
- B.** A client says prescribed tablets are being skipped every other day to make the medication last until the next paycheck

- C.** A client asks to bring a sibling to the visit for emotional support
- D.** A client says the discharge papers were misplaced after returning home

22. A nurse is preparing to transport a client from the medical-surgical unit to radiology for a chest x-ray. The client has just returned from the bathroom and is now in the hallway with several other clients waiting for transport. Which action best verifies the correct client before the nurse sends the client with the transporter?

- A.** Ask the client to state their full name and date of birth, then compare that information with the identification band and the transport request
- B.** Confirm the client is the only person waiting for a chest x-ray and send the client with the transporter
- C.** Ask the transporter whether this is the client listed for pickup
- D.** Verify the client's room number and diagnosis with the unit secretary

23. A nurse is preparing to apply antiembolism stockings for a postoperative client. While selecting the equipment, the nurse finds a package labeled medium, but the client's leg measurements match the large size on the facility sizing guide. The client says, "Those should be fine. I just want them on now." Which action by the nurse is best?

- A.** Apply the medium stockings because some compression is better than none
- B.** Stretch the medium stockings into place and reassess the client's comfort
- C.** Wait to apply stockings until the next shift can decide what size is needed
- D.** Obtain the correctly sized stockings before application

24. A nurse is preparing to administer the first dose of an IV antibiotic to a client. The electronic record lists "allergy: vancomycin," but when the nurse asks about it, the client says, "I remember feeling flushed and itchy during the infusion, but I did not have trouble breathing." Which action by the nurse is most appropriate?

- A.** Administer the medication because flushing and itching are expected effects of antibiotics
- B.** Hold the medication and clarify the reported reaction before administration
- C.** Remove vancomycin from the allergy list because the reaction was not anaphylaxis
- D.** Premedicate the client with an antihistamine and then begin the infusion

25. During a countywide blackout, the emergency generator activates on a long-term care unit. Which action by the practical nurse is the priority for environmental safety?

- A.** Check which clients rely on electrically powered equipment and verify that backup equipment is functioning
- B.** Open hallway doors to increase airflow throughout the unit
- C.** Begin restocking the medication cart before routine pass time
- D.** Call families to inform them that the facility has lost power

26. A client with acute confusion after a hip fracture repair keeps trying to stand up without assistance to "go home." The client is not combative and settles briefly when the nurse reorients the client. Which action is the best initial approach based on least-restrictive restraint principles?

- A.** Apply a vest restraint to prevent the client from rising
- B.** Move the client closer to the nurses' station and use frequent observation
- C.** Request an as-needed sedative to keep the client in bed
- D.** Raise all 4 side rails to keep the client from getting up

27. A client with delirium has soft wrist restraints in place after repeatedly attempting to remove a life-sustaining endotracheal tube despite repeated reorientation and one-to-one observation. During reassessment, the client is awake, follows simple commands, and no longer reaches for the tube while a staff member remains at the bedside. Which action by the nurse is most appropriate?

- A.** Discontinue the restraints and continue close observation
- B.** Leave the restraints on until the endotracheal tube is removed
- C.** Tighten the restraints because the client could become restless again
- D.** Replace the soft restraints with mitten restraints for greater security

28. During rounds, a nurse sees a licensed practical/vocational nurse place a cup of medications on the bedside table of the wrong client, then immediately notice the room number mistake and remove the cup before the client touches it. What is the best action by the observing nurse?

- A.** Tell the nurse to be more careful and take no further action because no harm occurred
- B.** Ensure the medications are kept from the client, then follow facility process for reporting the near miss
- C.** Document the event in the client's chart as a medication error that caused injury

D. Discard the medications and wait to see whether the charge nurse asks about the event

29. A nurse must move a client from the operating room stretcher to the hospital bed using a lateral transfer device. Which action is most important before the move begins?

- A.** Raise the bed slightly higher than the stretcher and lock both surfaces
- B.** Place the bed in the lowest position and leave the stretcher unlocked for easier movement
- C.** Lower the bed farther than the stretcher so the client can slide downhill
- D.** Position the bed several inches away from the stretcher to allow staff to stand between them

30. A practical nurse is preparing to change the dressing on a client's stage 3 pressure injury. After setting up a sterile field, the nurse notices that the outside of the sterile saline bottle touched the sterile gauze while the solution was being poured. Which action should the nurse take?

- A.** Continue the dressing change because the saline inside the bottle is still sterile
- B.** Remove the gauze that was touched and continue using the rest of the supplies
- C.** Consider the sterile field contaminated and obtain new sterile supplies
- D.** Wipe the gauze with sterile saline and use it for the dressing change

31. A nurse wears clean gloves to provide perineal care for a client. During the task, the nurse needs to stop and adjust the client's bed controls, then continue the perineal care. Which action is most appropriate?

- A.** Use the gloved hand to adjust the controls, then continue care with the same gloves
- B.** Remove the gloves, perform hand hygiene, adjust the controls, then apply new gloves before resuming care
- C.** Ask the client to adjust the controls to avoid interrupting the procedure
- D.** Keep the gloves on and cover the controls with a towel before touching them

32. A practical nurse is preparing to assist with insertion of an indwelling urinary catheter using sterile technique. After the sterile kit is opened, the nurse realizes an extra sterile cotton ball is needed. Which action best maintains asepsis when adding the cotton ball to the sterile field?

- A.** Drop the cotton ball from just above the center of the sterile field without reaching over the field
- B.** Place the cotton ball gently onto the nearest edge of the sterile field with a gloved hand

C. Set the cotton ball package on the sterile drape and let the nurse performing the procedure pick it up

D. Hand the unopened cotton ball package to the sterile nurse, who can open it over the field

33. A practical nurse is planning care for a client with suspected influenza who has been placed on droplet precautions. Which action is most appropriate?

- A.** Place the client in a negative-pressure room with the door closed at all times
- B.** Wear a surgical mask when coming within close range of the client
- C.** Use sterile gloves for all contact with the client
- D.** Require the client to use dedicated disposable dishes and utensils

34. A practical nurse is preparing to clean stool from a bedside commode used by a client who has no isolation orders. The nurse notices dried feces on the seat and expects contact with body substances during cleaning, but no splashing is likely. Which action best follows standard precautions?

- A.** Put on clean gloves before cleaning the commode and perform hand hygiene after removing them
- B.** Wear sterile gloves because the commode came in contact with feces
- C.** Skip gloves because the stool is dried and the client has no isolation orders
- D.** Place the client on contact precautions until the commode is disinfected

35. A nurse is reinforcing teaching with a client who is 25 weeks pregnant about when to begin daily fetal movement counting. Which instruction should the nurse include?

- A.** Begin counting fetal movements every day starting now
- B.** Wait until labor begins to monitor fetal movement patterns
- C.** Start counting only if you notice vaginal bleeding or contractions
- D.** Count fetal movements once a week after 36 weeks of pregnancy

36. A practical nurse is reinforcing teaching with the parent of a 16-year-old who recently got a driver's license. The parent says, "My teen wants to drive everywhere with friends and gets irritated when I ask questions about where they are going." Which response by the nurse is most appropriate?

- A.** "Teens who question limits usually need fewer rules so they can learn responsibility on their own."
- B.** "Wanting more independence is common at this age, but clear driving rules and safety expectations should still be maintained."

- C. "This behavior suggests your teen is not emotionally ready for driving privileges."
- D. "It is best to avoid discussing plans so your teen does not feel mistrusted."

37. A practical nurse is reinforcing teaching with a client who asks why a screening test might be recommended even when the test can sometimes miss disease or lead to follow-up testing. Which response by the nurse is best?

- A. Screening is still useful because it can identify people who may need more evaluation before symptoms appear.
- B. A screening test is meant to give a final diagnosis, so follow-up testing is rarely needed.
- C. If a screening test is not perfect, it should only be used for clients who already have symptoms.
- D. Screening is mainly done to replace regular health assessments and provider visits.

38. A nurse is obtaining a health history from an adult client who says, "I fainted at work yesterday." Which follow-up question is most important for the nurse to ask first?

- A. What did you eat for breakfast that morning?
- B. Did you have any warning signs such as dizziness, palpitations, or chest pain before you fainted?
- C. Have you ever missed work because of illness before?
- D. How did your coworkers react when the episode happened?

39. A practical nurse is admitting an adult client who says, "My blood pressure is usually a little high at doctor visits, but at home it runs around 128 over 78." Which documentation best identifies the client's usual baseline blood pressure?

- A. Blood pressure elevated today during admission assessment
- B. Client reports home blood pressure usually about 128/78 mm Hg
- C. Client appears anxious while blood pressure is being measured
- D. Blood pressure should be rechecked later in the shift

40. A nurse is reviewing the chart of a child scheduled to receive routine immunizations at a well-child visit. Which finding is a contraindication that should be reported to the provider before a live vaccine is given?

- A. The child is taking high-dose oral corticosteroids for a chronic condition
- B. The child had mild diarrhea 2 days ago that has resolved
- C. The child is currently taking an oral antibiotic for otitis media
- D. The child developed redness and soreness at the injection site after a previous vaccine

41. A nurse is reinforcing teaching with the parent of a healthy newborn. Which finding should the nurse identify as expected during the first 24 hours after birth?

- A. The newborn has acrocyanosis of the hands and feet
- B. The newborn has a sunken anterior fontanel
- C. The newborn has absent movement of all extremities
- D. The newborn has bilious vomiting after feeding

42. A nurse is interviewing a client during an admission assessment. The client answers questions appropriately but keeps rubbing one eye and says, "The room is so dim that I can barely see your face." Which nursing action is best to reduce this communication barrier?

- A. Move the client to a brighter area or increase the lighting before continuing the interview
- B. Speak more loudly so the client can better understand the questions
- C. Ask the client to wait until the provider arrives to complete the assessment
- D. Continue the interview and provide written materials at the end

43. A nurse is planning discharge teaching for a client who was newly prescribed a bronchodilator inhaler. When the nurse enters the room, the client says in accented but clear English, "I understand you better when I read lips, but this mask makes it hard." Which action by the nurse is best?

- A. Ask a family member to explain the instructions to the client later at home
- B. Provide the client with a written handout only and skip verbal teaching
- C. Move to a quiet area, face the client, and use written and demonstration-based teaching
- D. Speak much louder and repeat each instruction several times from the doorway

44. A nurse in a psychiatric unit is assessing several clients. Which client presents the highest immediate risk for violence?

- A. A client who says, "People on this unit talk about me," and asks to move rooms
- B. A client who is tearful after a family visit and refuses to attend group therapy
- C. A client who is pacing, has clenched fists, and says, "When staff change shifts, I'm going to settle this with my roommate"
- D. A client who reports trouble sleeping and asks for medication to help relax

45. A client who has been using cocaine heavily for several months tells the nurse, "I'm trying to stop, but every time I do, I feel exhausted and down." Which finding is most consistent with cocaine withdrawal?

- A. Increased appetite and hypersomnia

- B.** Pinpoint pupils and slowed respirations
- C.** Visual hallucinations and severe hypertension
- D.** Tremor, seizure risk, and marked autonomic instability

46. A client with methamphetamine use disorder is admitted to the inpatient unit and has been awake most of the night. The client is pacing in the room, speaking loudly, and saying, "Don't come near me. I know what you're planning." The client has not attempted to hit anyone but startles easily when staff approach. Which nursing action is most appropriate?

- A.** Assign several staff members to enter the room together and explain unit rules
- B.** Approach the client calmly with brief statements and reduce environmental stimulation
- C.** Challenge the client's statement directly by insisting that no one is planning harm
- D.** Leave the client alone until the client is ready to talk with staff

47. A client who was assaulted several weeks ago becomes tense during a dressing change and says, "When someone stands over me like that, I feel trapped." Which response by the nurse is most therapeutic?

- A.** You need to remember that you are safe here and try not to think about the assault.
- B.** It sounds like being approached that way is upsetting for you. What can I do right now to help you feel more comfortable?
- C.** Most clients feel nervous during dressing changes, so this reaction is understandable.
- D.** I will ask another nurse to finish the dressing change since this seems too difficult for you.

48. A client with alcohol withdrawal is on a medical unit and becomes increasingly restless during the evening. The client is sweating, pacing near the nurses' station, and says in a loud voice, "You need to stop staring at me. I know you're all talking about me." Which nursing action is most appropriate to help de-escalate the situation?

- A.** Stand directly in front of the client and firmly explain that the behavior must stop
- B.** Guide the client to a quieter area, speak in a calm voice, and use short, simple statements
- C.** Ask several detailed questions to determine exactly what triggered the client's reaction
- D.** Tell the client that no one is talking about them and that they are overreacting

49. A client who lives in an assisted living facility has mild dementia and often becomes upset when the daily schedule changes. This morning, the breakfast tray arrives late, and the client says, "Nobody tells me what's going on here!" Which action by the practical nurse is most appropriate?

- A.** Explain that occasional delays are unavoidable and the client should try to be patient
- B.** Provide a simple explanation for the delay and review the next part of the morning routine
- C.** Ask the client to calm down and wait quietly until breakfast arrives
- D.** Avoid discussing the delay so the client will be less likely to focus on it

50. A client tells the nurse, "I am trying to lower my stress, so I started having a few drinks every evening to help me relax." Which response by the nurse is most appropriate?

- A.** That can work for short-term stress relief if you limit yourself to one or two drinks.
- B.** Alcohol may seem calming at first, but it can worsen coping and create new problems; let's talk about safer ways to manage stress.
- C.** You should stop drinking immediately and replace it with energy drinks so you can stay productive.
- D.** If the drinks help you sleep, that is a healthy stress-management plan.

51. A client who is anxious before a cardiac catheterization asks the practical nurse, "Would you stay with me while I say a short prayer?" Which response by the nurse is most appropriate?

- A.** "I can stay with you quietly while you pray if you would like."
- B.** "You should wait until the chaplain arrives so the prayer is done correctly."
- C.** "Let's focus on your procedure right now and talk about spiritual needs afterward."
- D.** "I'm not allowed to be present during personal religious practices."

52. A nurse is caring for a client who has newly diagnosed hypertension. During teaching, the client says, "I usually use traditional foods and home remedies before I take pills." Which response by the nurse is most appropriate?

- A.** Tell the client that prescribed medication should replace all traditional practices.
- B.** Ask the client to describe the home remedies used and discuss how they may fit safely with the treatment plan.
- C.** Explain that cultural practices are not relevant while the client is in the hospital.
- D.** Advise the client to wait until discharge to mention any traditional remedies to the provider.

53. A client tells the nurse, "I identify as nonbinary, but I have not told my family. Please do not use my chosen name or pronouns when they visit." Which action by the nurse is most appropriate?

- A.** Document the client's preference and use the chosen name and pronouns only when speaking privately with the client
- B.** Explain that staff must use the same name and pronouns in all situations to provide consistent care
- C.** Ask the family how they would like staff to address the client during visits
- D.** Avoid using any name or pronouns when family members are present to prevent conflict

54. A client with newly diagnosed aphakia after cataract surgery says, "Without my glasses, everything looks distorted. I don't want visitors because I'm afraid they'll stare at me struggling." Which response by the nurse is best?

- A.** You should avoid visitors until your vision improves so you can feel more comfortable.
- B.** Many clients feel self-conscious at first. Tell me what part of having visitors worries you the most.
- C.** Your family already knows about the surgery, so they will understand if things look different.
- D.** Try not to focus on it; once you adjust to the glasses, this will not bother you anymore.

55. A client receiving end-of-life care tells the nurse, "My son keeps saying I need to fight, but I am tired and I want to talk about making the most of the time I have left." Which response by the nurse is most therapeutic?

- A.** "You should try not to give up hope because your son is only trying to help."
- B.** "It sounds like you want space to talk honestly about what matters most to you right now."
- C.** "Many clients feel this way near the end of life, so this is a normal reaction."
- D.** "Have you told your son that continuing to fight is no longer realistic for you?"

56. A nurse is caring for a client with a mature gastrostomy tube who receives intermittent enteral feedings. Before administering the next feeding, the nurse measures the external length of the tube and finds it is 4 cm longer than documented on the previous shift. The client denies pain, but the nurse hears air over the epigastric area after instilling air into the tube. Which action should the nurse take?

- A.** Administer the feeding because the air bolus suggests gastric placement
- B.** Advance the tube to the previous marking and secure it in place
- C.** Hold the feeding and notify the provider that tube displacement is suspected

D. Flush the tube with water and remeasure the external length after the flush

57. A nurse is preparing to logroll a client with a suspected spinal injury to change the bed linens. Which action is most appropriate?

- A.** Twist the client's shoulders first, then turn the hips after the upper body is in position
- B.** Place the client in high-Fowler's position before turning to reduce pressure on the spine
- C.** Keep the client's head, neck, and back aligned while turning the client as one unit
- D.** Ask the client to reach across the body and grasp the side rail during the turn

58. A nurse is reinforcing teaching for a client who has hypertension and asks how to season food without adding salt. Which client choice indicates an understanding of the teaching?

- A.** Adding soy sauce to vegetables
- B.** Using lemon juice and herbs on baked fish
- C.** Sprinkling garlic salt on potatoes
- D.** Choosing canned soup as a side dish

59. A client with diabetic peripheral neuropathy reports burning pain in both feet at bedtime. The client asks for a nonpharmacologic way to make the pain more manageable before sleep. Which nursing intervention is most appropriate?

- A.** Apply a heating pad directly to the feet
- B.** Encourage the client to walk barefoot in the room to distract from the pain
- C.** Teach the client to use guided relaxation and provide a calm sleep environment
- D.** Massage the feet vigorously for several minutes

60. A nurse is caring for a client who has an indwelling urinary catheter scheduled for removal this morning. Which action is most appropriate to help reduce the risk of urinary retention after catheter removal?

- A.** Encourage the client to limit oral fluids for 8 hours after removal
- B.** Offer toileting or a bedside commode at regular intervals after removal
- C.** Clamp the catheter for 12 hours before removal to strengthen bladder tone
- D.** Keep the client on bed rest until the first spontaneous void occurs

61. A nurse is caring for a client who says, "I go to bed earlier and earlier because I'm so tired, but then I lie awake for hours." Which teaching should the nurse provide first to help improve the client's sleep pattern?

- A.** Go to bed only when sleepy and keep a consistent wake-up time each morning

- B. Stay in bed until falling asleep becomes easier
 - C. Add a long evening nap to reduce exhaustion before bedtime
 - D. Watch television in bed until sleep occurs
- 62.** A practical nurse is caring for a client who has just returned from surgery with antiembolism stockings in place. Which action is most important for protecting the client's skin and tissue integrity?
- A. Roll the stockings down to the ankles when the client is resting
 - B. Remove the stockings at least once each shift to inspect the skin
 - C. Apply powder under the stockings to reduce moisture
 - D. Massage the calves before reapplying the stockings
- 63.** A practical nurse is caring for a client who has severe photophobia after an eye procedure. The client says, "The sunlight from the window is making my eyes hurt." Which action is most appropriate?
- A. Open the curtains fully so the client can adjust to the light
 - B. Dim the room and close the blinds to reduce bright light exposure
 - C. Encourage the client to keep the eyes open to build tolerance
 - D. Ask the client to avoid reporting the discomfort until the next assessment
- 64.** A practical nurse is measuring a client's urinary output from an indwelling catheter. At 1:00 p.m., the drainage bag contains 300 mL of urine and is emptied. At 5:00 p.m., the bag contains 260 mL. The client then vomits 90 mL and has 40 mL of liquid drainage from a wound drain. What total output should the nurse record from 1:00 p.m. to 5:00 p.m.?
- A. 260 mL
 - B. 350 mL
 - C. 390 mL
 - D. 650 mL
- 65.** A practical nurse is caring for a client who has been on bed rest for several days and now reports pain in the tailbone area. On assessment, the nurse notes a small reddened area over the coccyx that does not blanch when pressed. Which action is most appropriate?
- A. Massage the reddened area to increase circulation
 - B. Place the client in a donut-shaped cushion to relieve pressure
 - C. Relieve pressure from the area and reposition the client regularly
 - D. Clean the area once per shift and leave the client supine for comfort
- 66.** A practical nurse is reviewing the MAR before giving a client's 1200 medications. The MAR shows hydralazine 25 mg PO at 1200 and hydralazine 25 mg PO at 1230. The provider's prescription in the chart states hydralazine 25 mg PO every 12 hours, and the 0000 dose was documented as given. The client has no current symptoms and asks, "Why are you bringing the same pill twice?" What is the nurse's best action?
- A. Administer both doses because both are listed on the MAR
 - B. Hold the 1200 dose and give only the 1230 dose to space the medication more evenly
 - C. Clarify the duplicate MAR entries before administering hydralazine
 - D. Ask the client which time is usually correct and follow the client's preference
- 67.** A nurse is preparing to administer a medication that is designed to produce a local anti-inflammatory effect inside the nose for a patient with seasonal allergic rhinitis. Which route of administration should the nurse use?
- A. Intranasal
 - B. Sublingual
 - C. Transdermal
 - D. Subcutaneous
- 68.** A client with hypovolemic shock from gastrointestinal bleeding has already received crystalloids and packed red blood cells. The provider now prescribes a product to increase plasma oncotic pressure and pull fluid back into the intravascular space. Which product should the nurse expect to administer?
- A. Albumin
 - B. Platelets
 - C. Cryoprecipitate
 - D. Fresh frozen plasma
- 69.** A nurse is administering 1,000 mL of IV fluid at 125 mL/hr. The bag was hung at 0800. If the infusion runs as prescribed, at what time should the nurse expect the bag to be empty?
- A. 1400
 - B. 1500
 - C. 1600
 - D. 1700
- 70.** A practical nurse removes a prescribed controlled tablet from the automated dispensing system for a client. On entering the room, the nurse finds the client asleep and difficult to arouse. Which action should the nurse take first?
- A. Leave the medication at the bedside for the client to take when awake

- B.** Administer the medication to prevent the client's pain from worsening
- C.** Withhold the medication and assess the client's respiratory status and level of sedation
- D.** Return the medication at the end of the shift if the client does not request it

71. A nurse is assessing a client 24 hours after surgery who is receiving morphine by patient-controlled analgesia (PCA). The client is difficult to arouse, has a respiratory rate of 8/min, and an oxygen saturation of 89% on room air. Which action should the nurse take first?

- A.** Encourage the client to use the PCA button more often to improve pain control
- B.** Stop using the PCA and notify the registered nurse or provider immediately
- C.** Place the client in a supine position and reassess in 30 minutes
- D.** Document the findings as expected opioid effects after surgery

72. A nurse is preparing to administer the next dose of oral warfarin to a client who has a history of deep vein thrombosis. Which pre-administration data is most important for the nurse to review?

- A.** The client's most recent INR
- B.** The client's daily fluid intake
- C.** The client's bowel movement pattern
- D.** The client's appetite at breakfast

73. A client with rheumatoid arthritis started taking hydroxychloroquine 3 months ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client reports less morning joint stiffness and improved ability to perform daily activities
- B.** The client has increased urine output and a 2-lb weight loss in 24 hours
- C.** The client's fasting blood glucose is 92 mg/dL
- D.** The client reports sleeping 8 hours each night without interruption

74. A client with newly diagnosed tuberculosis is taking rifampin. At a follow-up visit 3 weeks later, which finding should the nurse identify as the highest-priority adverse response to report to the provider?

- A.** Orange-red discoloration of urine
- B.** Mild decrease in appetite
- C.** Yellowing of the sclera and dark urine
- D.** Soft stools occurring twice daily

75. A patient takes phenytoin for seizure control and has just been prescribed trimethoprim-sulfamethoxazole for a urinary tract infection. Which assessment finding should the nurse recognize as the highest priority possible indication of a drug interaction?

- A.** New unsteady gait and slurred speech
- B.** Increased urinary frequency and burning
- C.** Mild decrease in appetite
- D.** Reports of falling asleep earlier than usual

76. A client with asthma is prescribed theophylline sustained-release by mouth every 12 hours. Before administering the next dose, the nurse reviews the laboratory results and notes a serum theophylline level of 24 mcg/mL. Which action is most appropriate?

- A.** Administer the medication as prescribed because the level is therapeutic
- B.** Hold the dose and notify the provider of the elevated drug level
- C.** Give the dose with food to reduce gastrointestinal upset
- D.** Reduce the oxygen flow rate to decrease the risk of toxicity

77. A nurse is teaching a client who has a new prescription for phenazopyridine to relieve urinary discomfort caused by a urinary tract infection. Which instruction should the nurse include?

- A.** Stop taking the medication if your urine becomes orange-red.
- B.** Take the medication on an empty stomach for faster relief.
- C.** This medication can change the color of your urine and may stain clothing.
- D.** You should expect this medication to cure the infection within 24 hours.

78. A nurse is preparing to administer a premixed IV potassium chloride solution. Before hanging the bag, the nurse notices a thin crack near the upper port and a small amount of dampness on the outside of the bag. The solution inside appears clear, and the expiration date is valid. Which action should the nurse take?

- A.** Wipe the bag dry and infuse it if no further leaking is seen
- B.** Hang the solution because the medication itself is still clear
- C.** Discard the bag and obtain a new solution from the pharmacy
- D.** Reinforce the cracked area with tape and administer the infusion slowly

79. A practical nurse is collecting data on four clients in an adult care unit. Which client should the nurse identify as being at greatest risk for developing a latex allergy reaction during care?

- A.** A client with a history of multiple surgeries for spina bifida
- B.** A client who reports seasonal pollen allergies each spring
- C.** A client who had mild nausea after taking an oral antibiotic last year
- D.** A client with osteoarthritis who uses a cane for ambulation

80. A nurse is preparing a client for a fasting lipid panel scheduled for 0700. At 0600, the client asks for coffee with cream. Which response by the nurse is most appropriate?

- A.** You may have the coffee now because the blood sample is not due for another hour.
- B.** You may have black coffee, but avoid cream and sugar until after the specimen is collected.
- C.** Wait to consume anything by mouth until after the blood specimen is obtained.
- D.** Drink the coffee now, and I will document the intake for the laboratory.

81. A client who is 31 weeks pregnant comes to the clinic reporting swelling in one leg and sudden shortness of breath that began 20 minutes ago. Which action should the nurse take first?

- A.** Place the client in high-Fowler's position and notify the provider immediately
- B.** Instruct the client to elevate the swollen leg and rest at home
- C.** Obtain a urine specimen to check for protein
- D.** Schedule the client for a routine ultrasound later that day

82. A nurse performs a bedside capillary blood glucose test for a client who has severe edema of the hands. The drop of blood is difficult to obtain, and the nurse is concerned the result may not reflect the client's true glucose level. Which action is most appropriate?

- A.** Use the capillary result because edema does not affect bedside glucose testing
- B.** Milk the finger firmly to obtain a larger blood sample for the strip
- C.** Request or obtain a venous blood sample for laboratory glucose testing per agency protocol
- D.** Repeat the finger-stick immediately on the same edematous finger

83. A practical nurse is obtaining routine vital signs on an adult client who had a left mastectomy 2 years ago. The client also has an IV infusing in the right forearm. Which action is most appropriate for measuring blood pressure?

- A.** Measure the blood pressure in the right arm below the IV site
- B.** Measure the blood pressure in the left arm because there is no IV present
- C.** Use a thigh blood pressure measurement and follow facility procedure
- D.** Remove the IV temporarily and measure the blood pressure in the right arm

84. A client is scheduled for an electroencephalogram (EEG) this afternoon. Which statement by the client requires the nurse to follow up before the test?

- A.** I washed my hair this morning and did not use conditioner.
- B.** I had coffee and an energy drink to help me stay awake for the test.
- C.** I removed my metal hair clips before coming to the hospital.
- D.** I understand that the test records electrical activity in my brain.

85. A nurse is reviewing a client's ECG tracing and notes a rhythm that is regular at 74/min. The P wave is upright and present before each QRS complex. The PR interval measures 0.18 second, and the QRS duration is 0.16 second. Which interpretation is most appropriate?

- A.** Normal sinus rhythm with bundle branch block
- B.** First-degree AV block
- C.** Sinus bradycardia
- D.** Atrial flutter

86. A nurse is collecting a venous blood specimen from an adult client for routine laboratory testing. After the needle is inserted and blood is flowing well into the first tube, the nurse notices the tourniquet is still in place. Which action is most appropriate to reduce the risk of specimen error and client discomfort?

- A.** Leave the tourniquet in place until all tubes are filled
- B.** Release the tourniquet as soon as blood flow is established
- C.** Withdraw the needle and restart the venipuncture at a different site
- D.** Ask the client to pump the fist until the collection is complete

87. A practical nurse is caring for a client who is 18 hours postoperative after abdominal surgery. The client suddenly reports sharp chest pain and shortness of breath. Which action should the nurse take first?

- A.** Assist the client to ambulate in the hallway

- B.** Place the client in semi-Fowler position and apply oxygen as prescribed
- C.** Offer the client prescribed pain medication for discomfort
- D.** Encourage the client to use the incentive spirometer
- 88.** A client with a gastrostomy tube is prescribed an extended-release oral pain medication. What is the nurse's best action?
- A.** Crush the medication finely, dilute it with water, and administer it through the tube
- B.** Mix the medication with the enteral formula so it will pass through the tube more easily
- C.** Hold the dose and contact the provider for an alternative form or route
- D.** Administer the medication through the tube, then clamp the tube for 2 hours
- 89.** A nurse is caring for a client who returned from a cardiac catheterization 30 minutes ago through the right femoral artery. The client reports sudden warmth and wetness at the groin site. Which focused assessment should the nurse perform first?
- A.** Inspect the catheter insertion site for bleeding or hematoma
- B.** Ask the client to rate the discomfort on a 0 to 10 scale
- C.** Auscultate bowel sounds in all four quadrants
- D.** Assess pedal pulses bilaterally after the next scheduled vital signs
- 90.** A client with dehydration from gastroenteritis began oral rehydration therapy 8 hours ago. Which assessment finding best indicates the treatment is effective?
- A.** Urine specific gravity decreases from 1.035 to 1.015
- B.** Serum sodium increases from 146 mEq/L to 150 mEq/L
- C.** Blood urea nitrogen rises from 20 mg/dL to 28 mg/dL
- D.** Heart rate increases from 96/min to 112/min when standing
- 91.** A client with chronic hypertension takes enalapril daily at home. Which statement by the client indicates a need for further teaching?
- A.** I will change positions slowly if I feel dizzy.
- B.** I should report swelling of my lips or face right away.
- C.** I can stop taking this medicine once my blood pressure stays normal for a while.
- D.** I will keep my follow-up appointments to have my blood pressure checked.
- 92.** A client 24 hours after a bowel resection suddenly reports shortness of breath and chest pain. The nurse notes a respiratory rate of 32/min, heart rate of 124/min, oxygen saturation of 86%, and new hemoptysis. Which complication should the nurse suspect?
- A.** Pulmonary embolism
- B.** Atelectasis
- C.** Fluid volume overload
- D.** Myocardial infarction
- 93.** A client has a chest tube connected to a drainage system following thoracic surgery. On assessment, the nurse notes intermittent bubbling in the water-seal chamber during exhalation, the client's oxygen saturation is 96% on room air, and breath sounds are improving. Which interpretation is most appropriate?
- A.** The drainage system is functioning normally as air leaves the pleural space
- B.** The suction pressure is set too high and should be decreased immediately
- C.** The chest tube is obstructed and should be milked to restore patency
- D.** The tube has migrated out of the pleural space and must be removed
- 94.** A nurse is caring for a client with a temperature of 38.9°C (102°F). To help lower the fever, the nurse applies a cooling blanket. Which mechanism of heat transfer is the nurse using?
- A.** Radiation
- B.** Conduction
- C.** Evaporation
- D.** Convection
- 95.** A nurse is assessing a chronic leg ulcer and notes tissue that is yellow, stringy, and loosely attached in the wound bed. How should the nurse document this finding?
- A.** Eschar
- B.** Slough
- C.** Granulation tissue
- D.** Epithelial tissue
- 96.** A nurse and a second rescuer are performing 2-rescuer CPR on an adult client. An advanced airway has not been placed. Which action is most appropriate?
- A.** Deliver 1 breath after every 5 compressions
- B.** Pause compressions every 30 seconds to give 4 breaths
- C.** Use a compression-to-ventilation ratio of 30:2
- D.** Provide continuous compressions with 1 breath every 6 seconds

97. A client with pneumonia is receiving oxygen by simple face mask. On assessment, the nurse notes the oxygen flowmeter is set at 3 L/min. The client says, "I feel like I'm not getting enough fresh air." Which action should the nurse take?

- A.** Increase the flow rate to at least 5 L/min as prescribed or notify the provider for a new prescription
- B.** Switch the client to a nasal cannula at 3 L/min without a prescription
- C.** Remove the mask for 15 minutes to let the client rest
- D.** Encourage slow, deep breathing and leave the flow rate unchanged

98. A nurse is teaching an assistive personnel (AP) how to help with care for a client who has a tracheostomy. Which instruction by the nurse is most appropriate?

- A.** Clean around the stoma with cotton-tipped swabs whenever secretions are seen
- B.** Report any difficulty breathing or increased work of breathing to the nurse right away
- C.** Change the tracheostomy ties if they become damp or soiled
- D.** Suction the tracheostomy when the client asks for help clearing secretions

99. A client receiving mechanical ventilation is prescribed oral care every 4 hours. Which finding best indicates that the oral care is helping reduce respiratory complications?

- A.** The client requires less frequent endotracheal suctioning during the shift.
- B.** The client has fewer ventilator alarms after oral care is completed.
- C.** The oral mucosa remains moist and clean, with less visible plaque buildup.
- D.** The client tolerates turning and repositioning without coughing.

100. A client with an acute myocardial infarction is on continuous cardiac monitoring. The monitor alarm shows a heart rate of 36/min. The client is pale, reports dizziness, and has a blood pressure of 78/46 mm Hg. After calling for assistance, which action should the nurse anticipate preparing for immediately?

- A.** Synchronized cardioversion
- B.** Transcutaneous pacing
- C.** Defibrillation
- D.** Vagal stimulation

Simulation Test 14 — Answer Key

1. C	26. B	51. A	76. B
2. C	27. A	52. B	77. C
3. C	28. B	53. A	78. C
4. A	29. A	54. B	79. A
5. B	30. C	55. B	80. C
6. C	31. B	56. C	81. A
7. C	32. A	57. C	82. C
8. C	33. B	58. B	83. C
9. A	34. A	59. C	84. B
10. B	35. A	60. B	85. A
11. C	36. B	61. A	86. B
12. A	37. A	62. B	87. B
13. C	38. B	63. B	88. C
14. B	39. B	64. C	89. A
15. B	40. A	65. C	90. A
16. C	41. A	66. C	91. C
17. C	42. A	67. A	92. A
18. C	43. C	68. A	93. A
19. C	44. C	69. C	94. B
20. C	45. A	70. C	95. B
21. B	46. B	71. B	96. C
22. A	47. B	72. A	97. A
23. D	48. B	73. A	98. B
24. B	49. B	74. C	99. C
25. A	50. B	75. A	100. B

Simulation Test 15

100 questions

1. While giving routine morning medications in a skilled nursing facility, the practical nurse is told about these new situations. Which one requires the nurse's immediate action?

- A.** A resident receiving furosemide reports leg cramps and has a potassium level of 3.0 mEq/L from this morning's lab work
- B.** A resident with Parkinson disease needs extra time and cueing to swallow oral medications
- C.** A resident with chronic venous insufficiency has increased brown discoloration around both ankles
- D.** A resident scheduled for a dressing change asks to wait until after breakfast

2. A practical nurse is directing care with help from an assistive personnel (AP) in a rehabilitation unit. Which task is most appropriate to delegate to the AP?

- A.** Determine whether a client with a new ankle-foot orthosis can transfer safely from bed to wheelchair
- B.** Reinforce discharge teaching for a client who will use a walker at home
- C.** Assist a stable client who has already been assessed with grooming after breakfast
- D.** Evaluate whether a client's hand-strength exercises are improving fine motor function

3. A nurse is admitting an adult client who speaks limited English for a planned procedure later that morning. The client's teenage daughter says, "I always translate for my parent. We don't need anyone else." The client looks at the daughter, nods, and remains quiet. Which action by the nurse best protects the client's rights?

- A.** Proceed with the daughter translating because the client nodded in agreement
- B.** Ask a bilingual staff member from the unit to explain the procedure and obtain consent
- C.** Use a qualified medical interpreter to communicate with the client before further teaching or consent
- D.** Delay all communication until the provider is available to speak with the daughter

4. A practical nurse is giving end-of-shift report to the oncoming nurse using a bedside format. The client has asked that a recent diagnosis not be discussed in front of visiting family members. Several family members are in the room when report is about to begin. Which action by the nurse is most appropriate?

- A.** Proceed with the full report because bedside report is part of routine care

- B.** Ask the family to step out briefly and confirm with the client what information may be discussed at the bedside
- C.** Speak quietly so the family cannot hear the report
- D.** Skip the diagnosis entirely and leave out related care details from report

5. A nurse is caring for a client who has decision-making capacity and is scheduled for discharge. The client asks for information about advance directives. The client's adult child says, "Please don't bring that up. Talking about those forms will upset my parent." Which action by the nurse is most appropriate?

- A.** Stop the discussion because family members may refuse advance directive teaching
- B.** Ask the provider to decide whether the client should receive the information
- C.** Privately ask the client whether the client wants information and support about advance directives
- D.** Give the forms to the adult child and ask the child to explain them later

6. An alert adult client says, "I want to be sure my family is not left guessing about what care I would want if I become unable to speak for myself." Which statement by the nurse is best?

- A.** An advance directive can communicate your wishes for future care if you later cannot make decisions.
- B.** An advance directive is only used when a client is diagnosed with a terminal illness.
- C.** Once you are admitted to the hospital, your family automatically has to follow whatever the provider decides.
- D.** Completing an advance directive means you give up the right to make your own decisions now.

7. A nurse is preparing an alert adult client for an elective percutaneous gastrostomy tube placement. The client says, "I signed the consent form yesterday, but after thinking about it, I do not want this procedure anymore." Which action should the nurse take first?

- A.** Notify the provider and withhold preparation for the procedure
- B.** Explain that the consent form is already signed and the procedure must continue
- C.** Ask the client's spouse to encourage the client to proceed
- D.** Have the client sign a second form stating the refusal

8. A nurse is caring for a client with moderate dementia who is scheduled for a nonemergency hip repair after a fall. The client is awake and can state name but is unable to explain the procedure, risks, or alternatives consistently. The provider asks the nurse to obtain the client's signature on the consent form because the procedure is scheduled soon. Which response by the nurse is most appropriate?

- A.** Obtain the client's signature because the client is awake and can state a name
- B.** Ask the unlicensed assistive personnel who knows the client well to witness the consent
- C.** Inform the provider that the nurse cannot obtain consent from this client and that the legally authorized representative is needed
- D.** Have the client sign now and ask a family member to confirm the decision later

9. A client is preparing for discharge after hospitalization for a venous stasis ulcer. The provider prescribes weekly wound-clinic visits and compression therapy at home. The client says, "I live 40 miles away, my truck is not running, and I already missed appointments last month because I couldn't get a ride." Which nursing action is most appropriate before discharge?

- A.** Ask the client to call the wound clinic if transportation becomes a problem after discharge
- B.** Notify the case manager or social worker to help arrange transportation resources and follow-up access
- C.** Teach the client how to remove the compression wrap if attending follow-up is not possible
- D.** Delay discharge until a family member agrees to provide transportation every week

10. A client is being discharged after a new diagnosis of type 2 diabetes mellitus. The provider prescribes a glucometer, daily blood glucose checks, diet teaching, and follow-up at a community clinic. During discharge teaching, the client correctly repeats the glucose-check steps but says, "I sleep in a shelter, so I don't always have a place to keep supplies, and some of my things get stolen." Which action by the nurse is the priority?

- A.** Reinforce the glucose-monitoring procedure again until the client can repeat it without hesitation
- B.** Request a referral to case management to coordinate access to supplies, storage options, and follow-up resources
- C.** Suggest the client keep the glucometer at the shelter desk and pick it up only when needed
- D.** Instruct the client to focus on diet changes first and begin glucose monitoring after stable housing is found

11. A practical nurse is helping a clinic improve the percentage of clients with asthma who leave visits with an updated written action plan. The team begins using a discharge checklist that includes a reminder to review and print the plan. Which measure is the best process measure for this project?

- A.** The percentage of asthma visits in which staff used the discharge checklist
- B.** The number of clients who return to the emergency department for asthma within 30 days
- C.** The percentage of clients who report fewer nighttime symptoms 3 months later
- D.** The average length of each clinic visit

12. A practical nurse on a postoperative unit joins a quality improvement team after several clients reported severe pain before receiving as-needed analgesics. Chart review shows that the longest delays occur between the time pain is documented and the time the medication is actually given. Which action should the team take first?

- A.** Require all nurses to review pain management policies again at the next staff meeting
- B.** Begin a unit-wide client satisfaction survey about pain control before examining the workflow further
- C.** Map the medication administration steps to identify where the delay occurs between assessment and delivery
- D.** Set a goal that all as-needed pain medications will be given within 10 minutes starting tomorrow

13. A practical nurse is caring for a client with heart failure who is scheduled for discharge tomorrow. During the morning assessment, the nurse notes 2+ ankle edema, a weight gain of 1.4 kg (3 lb) since yesterday, and new crackles at the lung bases. Which information should the nurse share first with the provider to support interdisciplinary care?

- A.** The client says breakfast was not appealing
- B.** The client gained 1.4 kg (3 lb) since yesterday and now has new crackles at the lung bases
- C.** The client asked whether home health visits will be covered
- D.** The client would like to walk in the hall after lunch

14. A practical nurse is updating the interdisciplinary team about a client receiving antibiotics for a urinary tract infection. Which report best demonstrates effective information sharing?

- A.** "The client seems a little worse today and probably needs a different medication."
- B.** "The client is upset and says nothing is helping."
- C.** "Since 0800, the client's temperature increased from 37.6°C (99.7°F) to 38.5°C (101.3°F), heart rate is 108/min, urine is cloudy, and the client reports new right flank pain rated 7/10."

D. "The infection does not appear to be improving, so someone should come assess the client."

15. A practical nurse is preparing a client for discharge when the client asks, "Can you text my wound photo to my son so he can help me decide whether it looks normal later?" The son is listed in the chart as the emergency contact. What is the nurse's best response?

- A.** Text the photo because the son is listed as the emergency contact
- B.** Send the photo if the client gives verbal permission in front of another staff member
- C.** Explain that the nurse should follow facility-approved communication and release-of-information procedures instead of using personal texting
- D.** Refuse because wound photos can never be shared with family under any circumstance

16. At the end of shift, a practical nurse realizes that a blood glucose check performed 2 hours earlier was not documented. The result was reported to the RN at the time, and the client has remained stable. Which charting action is most appropriate?

- A.** Enter the result in the current note and identify it as a late entry with the actual time the glucose was checked
- B.** Record the result under the earlier time slot without noting that the entry was delayed so the chart stays in sequence
- C.** Ask the RN to document the glucose result instead, since the RN was notified
- D.** Do not document it now because delayed charting could create legal risk

17. A practical nurse in a pediatric office is helping with a sports physical for a 14-year-old client. While obtaining the history, the client says, "My coach has been texting me late at night and touching me when no one is around. Please don't tell anyone." What is the nurse's best action?

- A.** Keep the disclosure confidential because the client asked the nurse not to tell anyone
- B.** Ask the client to confront the coach before any report is made
- C.** Report the suspicion according to agency policy and state law, and notify the appropriate supervisor
- D.** Wait to report until the client repeats the statement in front of a parent or guardian

18. A practical nurse in a home health agency visits a 38-year-old client with a spinal cord injury who depends on a partner for daily care. The client is alert, oriented, and has decision-making capacity. During the visit, the nurse notices multiple untreated pressure injuries, spoiled food in the home, and an empty medication organizer that should still contain 3 days of doses. When asked privately, the client says, "My partner leaves me alone most weekends, but I don't want any report made." Which action by the nurse is most appropriate?

- A.** Respect the client's request and only provide teaching about skin care and medication use
- B.** Document the findings objectively and follow agency policy for reporting suspected abuse or neglect of a vulnerable dependent adult
- C.** Call the partner during the visit to obtain an explanation before deciding whether to report
- D.** Arrange a routine follow-up visit in 1 week to see whether the situation improves before taking further action

19. A practical nurse is planning assignments for a busy morning on a postoperative unit. Which task is most appropriate to delegate to an assistive personnel (AP)?

- A.** Reinforce discharge teaching about incision care for a client going home today
- B.** Obtain routine morning vital signs for a client 2 days after surgery whose condition has been stable
- C.** Assess a client who reports new shortness of breath after walking to the bathroom
- D.** Evaluate whether a client's pain medicine was effective 30 minutes after administration

20. A practical nurse delegates hygiene care to an assistive personnel (AP) for a client who had a total hip replacement yesterday. Which instruction is most important for the nurse to include when giving the assignment?

- A.** Report if the client refuses to eat breakfast after morning care
- B.** Keep the client's room door closed during care
- C.** Do not flex the client's hip beyond 90 degrees while turning or bathing
- D.** Offer the client a complete bed bath before ambulation

21. A practical nurse in a community clinic is arranging follow-up care for a client with chronic venous leg ulcers. The provider recommends weekly wound checks, but the client says, "I can only come after 6 p.m. because I clean offices during the day. If I miss work, I don't get paid, and I could lose the job." Which action by the nurse best promotes equitable access to care?

- A.** Advise the client that wound care must take priority over work and schedule the next available daytime appointment

B. Ask the clinic team whether wound visits can be offered during extended evening hours or coordinated with another accessible time option

C. Tell the client to go to the emergency department whenever the dressing needs to be checked

D. Document that the client is nonadherent because the recommended follow-up schedule was declined

22. A nurse in a rehabilitation unit is preparing to assist a client to physical therapy. The client has expressive aphasia and cannot clearly state a name. The room number on the transport slip matches the room, and the nursing assistant says, "That's the right patient." Which action should the nurse take to verify the client's identity before transport?

A. Use the room number and the nursing assistant's statement as the two identifiers

B. Ask the client to nod when the nurse says the name printed on the transport slip

C. Compare the client's identification band with the transport slip and the medical record

D. Confirm the client's identity with the physical therapist when the client arrives in the therapy department

23. A nurse is preparing to use a mechanical lift to transfer a dependent client from the bed to a chair. After positioning the sling under the client, which action is most important before the nurse begins lifting?

A. Raise the bed to its highest position to reduce staff back strain

B. Verify that the sling loops are attached securely and evenly to the lift

C. Ask the client to hold onto the lift bar during the transfer

D. Move the chair slightly farther away to give the lift more room

24. A nurse is preparing to place a topical iodine-based antiseptic on a client's skin before a bedside procedure. The client states, "When iodine was used before, my throat felt tight and I became short of breath." The provider is ready to begin, and the reaction is not listed in the chart. Which action by the nurse is most appropriate?

A. Proceed with the antiseptic because the provider is present if a reaction occurs

B. Hold the antiseptic, treat the reaction history as a possible allergy, and notify the provider

C. Apply a small amount first to see whether the client reacts again

D. Ask the client whether shellfish also causes symptoms before deciding what to do

25. A practical nurse in a community clinic is preparing clients for evacuation after an earthquake. While moving through the hallway, the nurse notes four environmental conditions. Which condition should the nurse correct first?

A. A rolling supply cart is left unlocked near the nurses' station

B. A cracked window in the waiting area is still intact

C. An oxygen cylinder is standing unsecured beside the exit route

D. Several magazines are scattered across chairs in the lobby

26. A client with acute confusion after a head injury keeps trying to stand up from the stretcher during a busy shift change. The client becomes more agitated when several staff members hold the client's arms, but briefly calms when one nurse speaks slowly and stays in view. Which action by the nurse best follows least-restrictive restraint principles?

A. Ask two assistive personnel to hold the client in bed until the provider can be notified

B. Apply a vest restraint to prevent the client from getting off the stretcher

C. Move the client closer to the nurses' station and assign one staff member to provide calm, frequent observation

D. Raise all four side rails and leave the client to decrease stimulation

27. At the end of a shift, a nurse notices that a portable oxygen tank used earlier for a client transfer is now stored with the gauge reading nearly empty. No client experienced distress, but the tank would not have been ready for an emergency transport. Which description best fits this situation?

A. An adverse event because client harm almost occurred

B. A near-miss because a safety risk was identified before causing harm

C. An expected equipment issue that does not need follow-up

D. A sentinel event because emergency equipment was involved

28. A nurse reviewing the wound-care cart before a dressing change finds that several sterile saline bottles are past their expiration date. None have been used on clients. After removing the bottles from the cart, which action is most appropriate next?

A. Continue with care and mention the expired supplies to the next shift if there is time

B. Discard the expired supplies and complete an incident report according to facility policy

C. Relabel the bottles with today's date so they are not mistaken as current stock

D. Place the bottles in a drawer for the charge nurse to inspect later

29. A nurse is selecting equipment to move a client from a wheelchair to a shower chair. The client is awake, able to follow commands, and can bear weight on one leg for a few seconds, but has poor standing balance and is not reliable with pivoting. Which transfer device is most appropriate?

- A.** Transfer board
- B.** Sit-to-stand lift
- C.** Mechanical lateral transfer device
- D.** Full-body mechanical lift

30. A practical nurse is preparing to assist with insertion of an indwelling urinary catheter using sterile technique. After opening the sterile kit and donning sterile gloves, the nurse notices that the client has moved and the fenestrated drape slipped off the bed onto the floor. Which action should the nurse take?

- A.** Place the drape back on the sterile field and continue because only the outside touched the floor
- B.** Ask the assistant to pick up the drape and place it near the client while the nurse continues the procedure
- C.** Stop the setup, discard the contaminated drape, and obtain a new sterile drape before proceeding
- D.** Continue without the drape as long as the catheter itself remains sterile

31. A nurse is preparing to obtain a nasopharyngeal specimen from a client with fever, cough, and sneezing. The nurse expects close contact with the client's face and possible exposure to respiratory secretions during the swab. Which personal protective equipment is most appropriate?

- A.** Gloves only
- B.** Gloves and gown only
- C.** Gloves, mask, and eye protection
- D.** Mask and gown only

32. A practical nurse is assisting with a sterile dressing change and has just donned sterile gloves. While arranging supplies on the sterile field, the nurse feels an itch on the cheek beneath the mask. Which action is best?

- A.** Use the sterile glove to quickly scratch the cheek, then continue the procedure
- B.** Ask another staff member to adjust the mask area, or step away and reglove if the face must be touched
- C.** Touch the cheek with the back of the sterile glove because the outside of the glove remains sterile
- D.** Lower the mask and scratch under it, then pull the mask back into place

33. A practical nurse is assigned to a client with a draining wound infected with methicillin-resistant *Staphylococcus aureus* (MRSA). Which action is most appropriate to prevent transmission to other clients?

- A.** Place the client on contact precautions and use dedicated equipment when possible
- B.** Place the client on droplet precautions and keep the room door closed
- C.** Wear an N95 respirator each time care is provided
- D.** Transport the client freely in the hallway as long as the wound is covered

34. A practical nurse has finished emptying a client's wound drainage reservoir while wearing gloves. Before removing the gloves, the nurse notices that the medication administration record on the bedside computer needs to be reviewed. Which action best follows standard precautions?

- A.** Use the gloved hand to scroll through the record, then remove the gloves afterward
- B.** Remove the gloves, perform hand hygiene, and then touch the computer
- C.** Ask the client to touch the computer screen instead of the nurse
- D.** Wipe the gloves with an alcohol pad and then use the computer

35. A nurse is reinforcing teaching with a client who is 14 weeks pregnant about preventing supine hypotension. Which instruction should the nurse include?

- A.** Lie flat on your back for 30 minutes after meals
- B.** Change positions slowly and rest on your side, especially the left side, if you feel dizzy
- C.** Limit fluid intake before bedtime to reduce pressure on blood vessels
- D.** Avoid bending your knees when getting out of bed

36. A client in labor is being monitored externally. The fetal heart rate baseline is 142/min with moderate variability. With several contractions, the nurse notes gradual decreases in fetal heart rate that begin before the contraction starts, reach their lowest point at the same time as the peak of the contraction, and return to baseline by the end of the contraction. Which interpretation is most appropriate?

- A.** Early decelerations related to fetal head compression
- B.** Late decelerations related to uteroplacental insufficiency
- C.** Variable decelerations related to umbilical cord compression
- D.** Prolonged decelerations related to fetal hypoxia

37. A practical nurse is reinforcing teaching with a 23-year-old client who asks why screening questions about alcohol use are included at routine visits. Which response by the nurse is best?

- A.** These questions are asked only when the provider suspects alcohol dependence.
- B.** Routine screening can help identify risky use early, before serious health or safety problems develop.
- C.** People under age 30 are screened because alcohol misuse is rare after that age.
- D.** If you have no symptoms related to alcohol use, screening is usually unnecessary.

38. A nurse is obtaining a health history from an adult client who says, "I stopped taking my blood pressure medicine because it made me feel bad." Which follow-up question is best for the nurse to ask first?

- A.** Which pharmacy do you use for your prescriptions?
- B.** Can you describe what you mean by 'feel bad'?
- C.** How long have you had high blood pressure?
- D.** Do any of your family members also have hypertension?

39. A practical nurse is collecting baseline physical data from a newly admitted adult client with type 2 diabetes. Which information is most useful to document as baseline sensory status for later comparison?

- A.** The client reports numbness and tingling in both feet that has been present for several months
- B.** The client ate 75% of breakfast this morning
- C.** The client states that diabetes runs in the family
- D.** The client says a neighbor drove them to the hospital

40. A nurse is counseling a 20-year-old client at a college health clinic about the human papillomavirus (HPV) vaccine. The client says, "I already became sexually active, so there is no reason to get that vaccine now." Which response by the nurse is best?

- A.** The vaccine is only effective if it is given before any sexual activity begins.
- B.** You should wait until age 26 because the vaccine works better in older adults.
- C.** The vaccine can still provide protection against HPV strains you may not have been exposed to.
- D.** The vaccine is used to treat existing HPV infection, so it is still recommended for that reason.

41. A nurse is collecting data during a wellness visit for a 5-year-old child. Which statement by the parent describes an expected developmental finding?

- A.** My child still prefers to play beside other children rather than with them.
- B.** My child can usually dress without help except for tying shoes.
- C.** My child cannot speak in complete sentences yet.

D. My child becomes very upset when left with a babysitter for a short time.

42. A nurse is preparing to explain a new medication schedule to a client. The client says, "My hearing aid battery died this morning, and I don't have a spare." Which action by the nurse is best to reduce this communication barrier?

- A.** Speak loudly into the client's affected ear while continuing the teaching
- B.** Use short, simple written instructions and confirm understanding with the client
- C.** Ask a family member to listen to the teaching and explain it later to the client
- D.** Delay all communication with the client until the hearing aid is working again

43. A nurse is preparing to teach a client about care of a new walking boot after an ankle fracture. The client says, "I can read and hear you fine, but I get mixed up when people tell me too many steps at once." Which action by the nurse is best?

- A.** Provide all instructions at one time using detailed medical terms
- B.** Ask a family member to learn the instructions instead of the client
- C.** Break the teaching into short parts and have the client repeat each step back
- D.** Delay all teaching until the follow-up visit next week

44. A client with newly diagnosed chronic obstructive pulmonary disease says to the practical nurse, "I get short of breath just walking to the bathroom. I feel scared all the time now." Which response by the nurse best provides emotional support?

- A.** You need to stay calm because anxiety will only make your breathing worse.
- B.** Many people learn to live with this condition, so try not to think about it too much.
- C.** It sounds like these changes are frightening for you. Tell me more about what worries you most.
- D.** Let me teach you the breathing exercises your provider prescribed.

45. A client with a history of heavy daily alcohol use is hospitalized for pneumonia. On the second hospital day, the client becomes agitated, reports seeing "people standing in the corner," and has a blood pressure of 182/104 mm Hg and pulse of 124/min. Which nursing action is the priority?

- A.** Offer the client coffee and conversation to decrease drowsiness
- B.** Place the client in seclusion to reduce stimulation
- C.** Notify the provider immediately and prepare for urgent treatment of severe alcohol withdrawal

D. Explain that the hallucinations are not real and reassess in 1 hour

46. A client with delirium after a urinary tract infection becomes restless at night, pulls at the bed linens, and says, "I need to get to the train station now." The client is not trying to hit staff. Which nursing intervention is most appropriate?

A. Explain in detail that the client is in the hospital and ask the client to state the date and location

B. Gently guide the client to a quiet area, speak calmly, and use short simple statements of reassurance

C. Ask several questions to determine exactly why the client believes a train is leaving soon

D. Tell the client that this behavior is unacceptable and that privileges will be limited if it continues

47. A client who is hospitalized after a new colostomy says to the nurse, "I can't even look at it. My life is ruined." Which response by the nurse is most therapeutic?

A. You will get used to it over time, so try not to think so negatively.

B. Many people live full lives with a colostomy, and you will too.

C. It sounds like this change feels overwhelming for you right now.

D. Why do you think your life is ruined?

48. A nurse is caring for a client with mild dementia who becomes upset during morning care. The client pulls away, raises a hand as if to strike, and says, "You're trying to hurt me." Which action by the nurse is most appropriate first to de-escalate the situation?

A. Step back, speak calmly, and explain each action in short, simple phrases

B. Firmly remind the client that hitting staff is not allowed

C. Ask another staff member to help complete the care quickly

D. Continue the care task while distracting the client with unrelated conversation

49. A resident with moderate dementia becomes very upset when the practical nurse prepares to trim the resident's fingernails. The resident pulls both hands away and says, "Don't cut me!" Which action by the practical nurse is most appropriate?

A. Explain that nail care is required and continue quickly before the resident becomes more resistant

B. Stop the task for now, speak calmly, and try again later with a simple step-by-step approach

C. Ask another staff member to hold the resident's hands still so the nail care can be completed

D. Tell the resident that there is nothing to be afraid of and that the nurse would never cause pain

50. A client tells the nurse, "I have so many demands at home and work that I feel overwhelmed before the day even starts." Which client action best demonstrates an effective stress-management strategy?

A. Making a list of tasks and completing the highest-priority items first

B. Ignoring the stressful situations to avoid thinking about them

C. Sleeping extra hours on days off instead of addressing responsibilities

D. Waiting until stress feels severe before asking for support

51. An older adult in rehabilitation says to the practical nurse, "Every week, members of my faith community meet online, and being part of it keeps me going. I do not know how to use the tablet here, so I have missed two meetings." Which nursing action is most appropriate?

A. Tell the client that personal spiritual activities should wait until discharge

B. Ask the client if help setting up the tablet for the next meeting would be useful

C. Contact the provider to request an antidepressant for the client

D. Explain that unit staff cannot assist with nonmedical activities

52. A nurse is reinforcing teaching with a client who has newly diagnosed diabetes. The client says, "In my family, we usually eat rice at every meal, and sharing those meals is very important." Which response by the nurse is most appropriate?

A. You will need to stop eating rice completely to control your blood sugar.

B. Tell your family to prepare separate meals for you until your blood sugar improves.

C. Let's talk about how rice fits into your usual meals so we can plan portions and food choices that work for you.

D. The dietitian will handle food preferences later, so for now just follow the standard diabetic diet.

53. A nurse is caring for a hospitalized client who identifies as transgender. Before the client's estranged sibling arrives, the client says, "Please use my legal name and former pronouns only when my sibling is in the room. I have not shared my identity with them, and I do not want that disclosed." Which nursing action is most appropriate?

A. Tell the client that staff must use the same name and pronouns at all times to avoid confusion

B. Document the client's request and communicate the plan to staff so the client's privacy is protected during the visit

C. Encourage the client to disclose their gender identity to the sibling to reduce long-term stress

D. Explain to the sibling that the client has requested different language in order to support authenticity

54. A client with newly diagnosed color vision loss says to the nurse, "I stopped helping with my grandchildren's art projects because I keep picking the wrong colors. It's embarrassing, and now I just avoid it." Which response by the nurse is best?

A. You should let your family handle those activities until your vision improves.

B. It sounds frustrating to lose confidence in something you enjoy. Let's talk about ways to stay involved that feel more comfortable for you.

C. Color vision loss is usually manageable, so try not to worry so much about it.

D. Ask your grandchildren to stop bringing art supplies so you are not reminded of the problem.

55. A client with advanced cancer who is receiving end-of-life care becomes tearful and tells the nurse, "I am worried my partner will not know how to manage the bills and paperwork after I'm gone." Which nursing action is most appropriate?

A. Tell the client to focus on staying calm and avoid discussing stressful topics

B. Reassure the client that the partner will figure things out after the death occurs

C. Explore the client's concerns and offer to contact social services for practical planning support

D. Encourage the client to wait and discuss financial matters only with the provider

56. A nurse is reviewing care for a client who receives bolus enteral feedings through a mature gastrostomy tube at home. Which instruction should the nurse include to help reduce the risk of aspiration after each feeding?

A. Lie on the left side for 15 minutes after the feeding

B. Remain sitting upright for at least 30 to 60 minutes after the feeding

C. Drink a full glass of water immediately after the feeding

D. Clamp the tube tightly and place a dry dressing over the stoma

57. A nurse is ambulating a client in the hallway when the client suddenly says, "I feel faint." Which action should the nurse take first?

A. Support the client and ease the client into a chair or onto the floor

B. Check the client's blood pressure immediately

C. Encourage the client to take slow deep breaths and continue walking

D. Return the client to bed as quickly as possible

58. A nurse is reinforcing discharge teaching for a client who takes monoamine oxidase inhibitors (MAOIs) for depression. Which food should the nurse instruct the client to avoid?

A. Fresh apple slices

B. Grilled chicken breast

C. Aged cheddar cheese

D. Steamed green beans

59. A client with herpes zoster (shingles) has burning pain and says that clothing rubbing against the rash makes it worse. Which nonpharmacologic intervention should the nurse implement?

A. Apply a loose, dry dressing over the affected area

B. Massage the skin around the rash for 10 minutes every 2 hours

C. Use a heating pad on the affected area to decrease discomfort

D. Rub the lesions gently with scented lotion after bathing

60. A nurse is caring for a client who has a left above-the-elbow amputation and is learning to perform personal hygiene independently. Which nursing action best promotes the client's ability to complete this activity of daily living?

A. Encourage the client to practice grooming with adaptive devices and allow extra time to complete the task

B. Complete the hygiene routine for the client to prevent frustration

C. Ask a family member to perform hygiene care until the client receives a prosthesis

D. Limit hygiene to once daily to reduce the client's energy use

61. A nurse is reinforcing sleep-hygiene teaching with a client who reports trouble falling asleep most nights. Which client statement indicates correct understanding of the teaching?

A. I will go to bed at a regular time each night and use my bed only for sleep.

B. I will stay in bed longer on weekends so I can catch up on missed sleep.

C. I will have a large snack right before bed so I do not wake up hungry.

D. I will watch television in bed until I feel drowsy enough to sleep.

62. A practical nurse is caring for a client on prolonged bed rest. During the morning skin assessment, the nurse finds a red area over the sacrum that remains red when gently pressed. The skin is intact, and the client reports mild tenderness at the site. Which action is most appropriate?

A. Massage the reddened area to improve circulation

- B. Reposition the client and relieve pressure from the area
 - C. Apply a heating pad to increase blood flow to the tissue
 - D. Leave the area open to air and reassess at the end of the shift
- 63.** A practical nurse is caring for a client who has reduced peripheral sensation in the hands after chemotherapy. During hygiene care, which action is most appropriate to help prevent injury?
- A. Test the water temperature with the client's hand before washing
 - B. Use warm water and verify the temperature with a bath thermometer or the nurse's inner wrist
 - C. Soak the client's hands in hot water to improve circulation
 - D. Massage the hands vigorously if the client reports numbness
- 64.** A client is on strict intake and output. During the shift, the client receives 750 mL of IV fluid, drinks 180 mL of tea, and eats 1 cup of gelatin. The client voids 620 mL of urine and has 110 mL of liquid stool. Which amount should the practical nurse document as the client's fluid balance for the shift?
- A. +440 mL
 - B. +320 mL
 - C. +200 mL
 - D. -320 mL
- 65.** A practical nurse is caring for a client who has been immobilized for several days following a femur fracture. Which action is most appropriate to maintain proper body alignment while the client is in bed?
- A. Place the client in the same semi-Fowler position throughout the shift for comfort
 - B. Support the client with pillows and trochanter rolls as prescribed
 - C. Massage reddened bony areas during routine skin care
 - D. Keep the knees slightly flexed with a pillow under them at all times
- 66.** A nurse is reinforcing teaching for a patient prescribed a vaginal antifungal suppository for a localized yeast infection. Which instruction should the nurse include about this route of administration?
- A. Swallow the suppository with water at bedtime
 - B. Insert the medication into the vagina as directed
 - C. Place the suppository under the tongue until it dissolves
 - D. Insert the medication into the rectum after a bowel movement
- 67.** A nurse is caring for a client who is receiving a unit of packed red blood cells. Ten minutes after the transfusion begins, the client reports chills and flank pain and becomes short of breath. What is the nurse's priority action?
- A. Slow the transfusion rate and continue to monitor the client closely
 - B. Stop the transfusion immediately and maintain IV access with normal saline using new tubing
 - C. Administer acetaminophen as prescribed and reassess in 15 minutes
 - D. Document the findings as an expected early transfusion response
- 68.** An IV bag contains 500 mL of 0.9% sodium chloride and is ordered to infuse over 4 hours using tubing with a drop factor of 15 gtt/mL. What flow rate should the nurse set in drops per minute?
- A. 25 gtt/min
 - B. 31 gtt/min
 - C. 42 gtt/min
 - D. 50 gtt/min
- 69.** A practical nurse is preparing the controlled-substance count with the oncoming nurse at shift change. The count for one opioid drawer does not match the inventory record, but all clients are stable and no medication is due immediately. Which action should the practical nurse take next?
- A. Complete the rest of the count first and investigate the discrepancy after report
 - B. Document the discrepancy at the end of the shift if the medication is not needed
 - C. Recount the medication with the oncoming nurse and follow facility procedure for reporting the discrepancy immediately if it remains unresolved
 - D. Ask the charge nurse to adjust the count so the drawer can be closed on time
- 70.** A nurse is caring for a postoperative client who is receiving analgesia through an epidural infusion. Which finding is most important for the nurse to identify as a possible complication of the epidural analgesia?
- A. New numbness and weakness in both legs
 - B. Pain rating of 4 on a 0-to-10 scale
 - C. Urine output of 35 mL/hr
 - D. Mild itching around the nose
- 71.** A nurse is preparing to administer the next dose of gentamicin IV to a client with pyelonephritis. Which pre-administration data is most important for the nurse to review?
- A. The client's most recent creatinine and BUN levels
 - B. The client's dietary sodium intake for the day

- C.** The number of visitors the client had this morning
D. The time of the client's last bowel movement
- 72.** A client with gout began taking allopurinol 2 months ago. Which finding best indicates the medication is producing the intended therapeutic response?
A. Serum uric acid has decreased since therapy began
B. The client reports mild drowsiness after each dose
C. Urine output has increased to 3000 mL/day
D. The client has gained 1.8 kg (4 lb) in 1 week
- 73.** A client with stable angina applies a new nitroglycerin transdermal patch in the morning. About 45 minutes later, the client reports feeling dizzy when standing and says, "I think my heart is racing." Which finding should the nurse recognize as the most likely adverse response to the medication?
A. Orthostatic hypotension with reflex tachycardia
B. Bronchospasm caused by airway constriction
C. Hyperglycemia caused by impaired insulin release
D. Urinary retention caused by smooth muscle spasm
- 74.** A patient who takes warfarin after a heart valve replacement calls the clinic and says, "My provider just started me on trimethoprim-sulfamethoxazole for a urinary tract infection." Which instruction by the nurse is most appropriate?
A. "Start the antibiotic as prescribed, but watch for signs of bleeding and notify the provider because this combination can increase warfarin's effect."
B. "Skip your warfarin doses while you are taking the antibiotic, then restart it when the antibiotic is finished."
C. "Take both medications together with food to prevent the interaction from occurring."
D. "There is no important interaction between these medications if your warfarin dose has been stable."
- 75.** A client with a seizure disorder is prescribed carbamazepine 200 mg by mouth twice daily. Before administering the morning dose, the nurse reviews the laboratory results and notes a sodium level of 122 mEq/L. Which action is most appropriate?
A. Administer the medication as prescribed and document the sodium level
B. Hold the dose and notify the provider of the critical sodium level
C. Give the medication with extra oral fluids to correct the sodium level
D. Reduce the dose by half and recheck the sodium level in 24 hours
- 76.** A nurse is teaching a client who has a new prescription for oral vancomycin to treat *Clostridioides difficile* infection. Which instruction should the nurse include?
A. Stop taking the medication as soon as the diarrhea decreases
B. Take the medication with antacids to reduce stomach upset
C. Complete the full course of the medication as prescribed
D. Expect the medication to treat any future viral stomach illness
- 77.** A nurse is preparing to administer an oral medication from a unit-dose package. While checking the package, the nurse notices the seal is partially open, although the medication tablet inside appears intact and the expiration date is valid. Which action should the nurse take?
A. Administer the medication because the tablet itself does not appear damaged
B. Repackage the medication in a clean medication cup and give it as scheduled
C. Discard the medication and obtain a new dose from a properly sealed package
D. Ask another nurse to verify the tablet before administering it
- 78.** A nurse is preparing to administer cefuroxime 375 mg by mouth. The suspension available is labeled 250 mg per 5 mL. How many milliliters should the nurse administer for one dose?
A. 5 mL
B. 7.5 mL
C. 10 mL
D. 12.5 mL
- 79.** A practical nurse is reinforcing discharge teaching for a client with a new prescription for warfarin. Which finding should the nurse identify as the greatest risk factor for serious injury after discharge?
A. The client uses a soft-bristle toothbrush
B. The client reports drinking 2 to 3 glasses of wine each evening
C. The client prefers to take the medication with dinner
D. The client checks blood glucose twice daily
- 80.** A nurse is teaching a client how to obtain a midstream clean-catch urine specimen for culture. Which client statement indicates correct understanding of the teaching?
A. I will collect the first part of the urine stream in the cup.
B. I will cleanse the area, begin urinating, and then collect urine in the container.

C. I should touch the inside of the cup to my skin so the specimen is easier to catch.

D. I can collect the specimen from urine left in the toilet hat after I finish voiding.

81. A nurse is reviewing laboratory results for a client receiving IV unfractionated heparin for treatment of a deep-vein thrombosis. Which result indicates the therapy is currently within the expected therapeutic range?

A. aPTT 18 seconds

B. aPTT 32 seconds

C. aPTT 68 seconds

D. Platelet count 250,000/mm³

82. A nurse performs a bedside urine pregnancy test for a client who reports a missed period and lower abdominal cramping. The test result is negative, but the client states she drank several large bottles of water before arriving because she expected to give a urine sample. Which action is most appropriate?

A. Document the negative result as conclusive and prepare the client for discharge

B. Repeat the urine test immediately using the same specimen

C. Recognize the result may be falsely negative from dilute urine and notify the provider

D. Interpret the result as showing the abdominal cramping is unrelated to pregnancy

83. A practical nurse is checking vital signs on an adult client who is receiving an opioid for pain. Which assessment finding requires the most immediate action?

A. Temperature 37.8°C (100.0°F)

B. Blood pressure 104/68 mm Hg

C. Respiratory rate 8/min and difficult to arouse

D. Heart rate 102/min

84. A client is scheduled for a lumbar puncture in 30 minutes. Which finding should the nurse report to the provider before the procedure?

A. The client reports a mild headache this morning

B. The client received subcutaneous heparin 2 hours ago

C. The client asks whether sedation will be used

D. The client voided immediately before transport

85. A nurse is reviewing an ECG rhythm strip and notes a ventricular rate of 96/min with QRS complexes that are wide and bizarre in appearance. P waves are not clearly associated with the QRS complexes. Which rhythm should the nurse identify?

A. Sinus tachycardia

B. Ventricular tachycardia

C. Junctional rhythm

D. First-degree AV block

86. A nurse is preparing to obtain a venous blood specimen from an adult client for coagulation studies. The nurse must collect more than one tube of blood. Which action is most important to reduce the risk of an inaccurate laboratory result related to additive carryover?

A. Use the smallest needle available for the venipuncture

B. Fill the collection tubes in the correct order of draw

C. Ask the client to repeatedly open and close the fist during collection

D. Keep the tourniquet in place until all tubes have been filled

87. A practical nurse is preparing to transfer a client from the stretcher to the operating room table. The client says, "Wait, I need to take out my contact lenses." Which response by the nurse is most appropriate?

A. Allow the client to keep the lenses in place because they help the client see clearly after surgery

B. Ask the client to remove the contact lenses now and store them safely according to facility policy

C. Tell the client the anesthesia provider will remove the lenses after the client is asleep

D. Cover the client's eyes with gauze and leave the lenses in place during the procedure

88. A client has a newly inserted central venous catheter. During the initial assessment, the nurse notes swelling of the neck and face, distended neck veins, and the client reports a feeling of fullness in the head. Which complication should the nurse suspect?

A. Superior vena cava syndrome

B. Air embolism

C. Catheter-related bloodstream infection

D. Infiltration of the IV solution

89. A nurse is caring for a client with type 1 diabetes who received the usual dose of rapid-acting insulin before breakfast. Thirty minutes later, the client becomes pale, diaphoretic, and says, "My heart is pounding." Which focused assessment should the nurse perform first?

A. Check the client's capillary blood glucose level

B. Auscultate the apical pulse for 1 full minute

C. Ask the client how much breakfast was eaten

D. Review the client's most recent serum potassium level

90. A client with a lower-extremity deep-vein thrombosis has been receiving a continuous heparin infusion for 24 hours. Which laboratory result best indicates the treatment is having the desired effect?

A. Activated partial thromboplastin time (aPTT) within the prescribed therapeutic range

B. Platelet count decreased from 220,000/mm³ to 150,000/mm³

C. Hemoglobin decreased from 13.2 g/dL to 11.0 g/dL

D. International normalized ratio (INR) of 2.5

91. A client with Addison disease has had vomiting and diarrhea for 24 hours and now reports extreme weakness and dizziness. Assessment findings include blood pressure 76/40 mm Hg, heart rate 128/min, cool skin, and confusion. Which action should the nurse take first?

A. Place the client on a low-sodium diet

B. Administer prescribed IV hydrocortisone and isotonic fluids

C. Encourage oral fluids with potassium supplements

D. Check the client's daily weight and intake record

92. A client is 2 days postoperative after abdominal surgery and has a Penrose drain in the incision. Which nursing action is most appropriate?

A. Pin the drain to the bed linens to prevent tension on the incision

B. Cover the drain with a sterile absorbent dressing and monitor the amount and character of drainage

C. Compress the drain after emptying it to reestablish suction

D. Irrigate the drain every 4 hours with normal saline to maintain patency

93. A nurse is planning care for four clients on a medical unit. Which client is at greatest risk for impaired thermoregulation?

A. A 22-year-old client who has a sprained ankle and is using an ice pack

B. A 34-year-old client who has seasonal allergies and a temperature of 37.2°C (99.0°F)

C. A 78-year-old client who has had a cerebrovascular accident and is immobile in bed

D. A 45-year-old client who had an uncomplicated cataract procedure earlier that day

94. A nurse is caring for a client with a newly placed wound drain after abdominal surgery. Which observation should the nurse report to the provider immediately?

A. The drain output is 35 mL of serosanguineous fluid in 8 hours

B. The suction device is compressed and the dressing is dry

C. The drain suddenly stops collecting fluid, and the client reports increasing incisional pressure

D. A small amount of bloody drainage is present in the collection bulb

95. A nurse is participating in a mock code for an adult client in cardiac arrest. The compressor has been doing chest compressions for about 2 minutes and says, "I'm getting tired." Which action by the team best supports high-quality CPR?

A. Pause compressions for 20 seconds while the next rescuer gets into position

B. Switch compressors with the change completed in less than 10 seconds

C. Continue with the same compressor until an AED advises a shock

D. Stop compressions and give 2 rescue breaths before changing rescuers

96. A client with chronic obstructive pulmonary disease is prescribed oxygen by Venturi mask. During the assessment, the nurse notes the client has removed the mask repeatedly, saying, "It keeps blowing in my eyes." Which nursing action is most appropriate?

A. Replace the Venturi mask with a nasal cannula at the same oxygen flow rate

B. Apply a face tent instead of the Venturi mask to improve comfort

C. Reposition and secure the mask for a better fit while maintaining the prescribed device

D. Increase the oxygen concentration so the client can wear the mask for shorter periods

97. A nurse is preparing to clean the inner cannula of a client's tracheostomy during routine care. The client suddenly begins coughing repeatedly and says, "I can't breathe well." Which action should the nurse take first?

A. Remove the tracheostomy ties and replace them with clean ties

B. Stop the procedure and assess the patency of the airway

C. Continue the procedure quickly to reduce the client's anxiety

D. Place sterile dressing under the tracheostomy flange

98. A client receiving mechanical ventilation is being turned when the high-pressure alarm sounds and the client becomes visibly distressed. The nurse notes that the ventilator tubing is sharply bent under the side rail. Which action should the nurse take first?

A. Administer a PRN sedative to reduce anxiety

B. Straighten the ventilator tubing and reassess the client

C. Increase the oxygen concentration on the ventilator

D. Call the respiratory therapist immediately

99. A client with a newly implanted permanent pacemaker is on telemetry. During morning care, the nurse sees small pacing spikes occurring regularly, but several of the spikes appear after the QRS complex instead of before it. The client is awake and says, "My heart feels like it's skipping," but vital signs are stable. Which pacemaker problem should the nurse identify?

- A. Failure to capture
- B. Failure to sense
- C. Failure to pace
- D. Normal demand pacing

100. A nurse is caring for a client 24 hours after creation of a new stoma. Which stoma assessment finding should the nurse identify as expected at this time?

- A. Dusky blue color with cool surface temperature
- B. Beefy red appearance with mild edema
- C. Blackened tissue around the stoma opening
- D. Pale gray color with no moisture

Simulation Test 15 — Answer Key

1. A	26. C	51. B	76. C
2. C	27. B	52. C	77. C
3. C	28. B	53. B	78. B
4. B	29. B	54. B	79. B
5. C	30. C	55. C	80. B
6. A	31. C	56. B	81. C
7. A	32. B	57. A	82. C
8. C	33. A	58. C	83. C
9. B	34. B	59. A	84. B
10. B	35. B	60. A	85. B
11. A	36. A	61. A	86. B
12. C	37. B	62. B	87. B
13. B	38. B	63. B	88. A
14. C	39. A	64. A	89. A
15. C	40. C	65. B	90. A
16. A	41. B	66. B	91. B
17. C	42. B	67. B	92. B
18. B	43. C	68. B	93. C
19. B	44. C	69. C	94. C
20. C	45. C	70. A	95. B
21. B	46. B	71. A	96. C
22. C	47. C	72. A	97. B
23. B	48. A	73. A	98. B
24. B	49. B	74. A	99. B
25. C	50. A	75. B	100. B

Simulation Test 16

100 questions

1. A practical nurse in a clinic is preparing to room the next client when an assistive personnel reports four new situations. Which situation should the nurse address first?

- A.** A client with hypertension says, "I forgot whether I took my blood pressure pill this morning."
- B.** A client with diabetes says, "I feel shaky and sweaty," after waiting longer than expected for the appointment.
- C.** A client with chronic knee pain asks for help completing an insurance form.
- D.** A client with dermatitis reports that the prescribed cream has not reduced itching after 2 days.

2. A practical nurse is caring for clients on a long-term care unit with help from an assistive personnel (AP). Which task is most appropriate for the nurse to delegate to the AP?

- A.** Assess a resident's new complaint of burning with urination
- B.** Apply a prescribed barrier cream to a reddened area on the sacrum
- C.** Escort a stable resident to the dining room for lunch
- D.** Teach a resident how to use a newly ordered walker

3. A nurse is caring for an adult client who is alert and able to make decisions. Before a provider enters the room, the client's adult child tells the nurse, "Do not tell my parent that the scan might show cancer. That news would be too upsetting, so just explain everything to me instead." When the provider arrives, the client asks, "What are they looking for on the scan?" What should the nurse do?

- A.** Ask the provider to discuss the scan results only with the adult child until the family agrees on what to tell the client
- B.** Tell the client that the provider will decide later what information is appropriate to share
- C.** Support the client's right to receive information and ensure the provider answers the client's question directly
- D.** Suggest postponing the discussion until the adult child can speak privately with the provider

4. A practical nurse is preparing a discharge packet for a client going to a rehabilitation facility. The packet includes the client's medication list, recent progress notes, and insurance information. The unit clerk offers to give the sealed packet to the client's adult son, who is waiting at the desk and says, "I'm taking my mother to rehab now." No documentation identifies the son as an approved recipient. Which action by the nurse is most appropriate?

- A.** Give the packet to the son because he is involved in the client's transportation
- B.** Ask the son to sign for the packet, then release it to him
- C.** Verify with the client that the packet may be released to the son before giving it to him
- D.** Remove the insurance information and give the rest of the packet to the son

5. A nurse is admitting an alert adult client for treatment of COPD exacerbation. The client says, "I filled out a paper years ago saying what care I would want if I could not speak for myself, but I have not talked with anyone about it since then." Which nursing response is most appropriate?

- A.** "Once an advance directive is signed, the health care team will automatically follow it in every situation."
- B.** "You should review the document periodically and discuss your wishes with the person who may need to speak for you."
- C.** "Advance directives are only useful after a provider determines that treatment is no longer beneficial."
- D.** "If your family disagrees with the document, their decision will usually replace what you wrote."

6. An alert adult client is scheduled for an elective cardiac catheterization in 1 hour. The client tells the nurse, "I signed the consent this morning, but now I have a lot of questions about the risks. I do not want to go until those questions are answered." Which action by the nurse is most appropriate?

- A.** Explain the common risks of the procedure so the client will feel comfortable proceeding
- B.** Notify the provider that the client wants more information before the procedure
- C.** Remind the client that the form is already signed and preparation should continue
- D.** Ask a family member to encourage the client to proceed as planned

7. A client with schizophrenia is admitted for treatment of gallstones and is scheduled for an elective cholecystectomy tomorrow. The client is calm, oriented, explains the procedure in simple terms, states the possible complications discussed by the surgeon, and says, "I want the surgery because the pain keeps coming back." The client's sibling tells the nurse, "Because of the schizophrenia, my brother is not allowed to sign anything. I need to do it." Which action should the nurse take?

- A.** Have the sibling sign because a psychiatric diagnosis automatically prevents legal consent
- B.** Delay the surgery until a court determines whether the client is competent to consent
- C.** Accept the client's consent if the client demonstrates understanding and decision-making ability
- D.** Ask both the client and the sibling to sign so the consent will be legally valid

8. A client is preparing for discharge after hospitalization for a newly diagnosed pulmonary embolism. The provider prescribes an oral anticoagulant and a follow-up appointment in 5 days. During teaching, the client says, "I travel for work and might be in another city next week, so I'm not sure I can make that visit." Which nursing action is most appropriate before discharge?

- A.** Tell the client to skip the follow-up visit if there are no symptoms
- B.** Ask the provider to stop the anticoagulant until the client can return
- C.** Collaborate with the case manager to help arrange a realistic follow-up plan before discharge
- D.** Instruct the client to call the clinic only if chest pain returns

9. A client is being discharged after total hip arthroplasty. The client will go home with a raised toilet seat and a walker. The client can describe the medication schedule correctly but becomes short of breath and needs help putting on socks and getting from bed to chair. The spouse says, "I can help in the evenings, but I work during the day." Which referral should the nurse prioritize?

- A.** Home health services
- B.** Speech therapy
- C.** Hospice care
- D.** Cardiac rehabilitation

10. A rehabilitation unit wants to improve handoff communication when clients transfer to physical therapy. The team plans to start using a brief transfer checklist. Which measure would best show whether the new checklist is being carried out as intended?

- A.** The percentage of transfers in which the checklist is completed
- B.** The number of clients who are discharged home from rehabilitation
- C.** The average length of stay on the unit
- D.** The percentage of clients who report satisfaction with meals

11. A clinic starts a quality improvement project to reduce missed wound-care follow-up visits. During the first month, reminder calls are documented for nearly all scheduled clients, but the missed-visit rate does not improve. Which action is best for the team to take next?

- A.** Stop the project because the intervention clearly failed
- B.** Adopt reminder calls permanently because staff compliance was high
- C.** Study why clients are still missing visits and test a different change based on that finding
- D.** Wait several more months without reviewing any additional data

12. A practical nurse is giving report to the wound care nurse about a client with a sacral pressure injury. Which information is most important to include to support the next step in the client's care?

- A.** The client prefers dressing changes after breakfast
- B.** The wound drainage increased, now has a foul odor, and the surrounding skin is warmer than earlier
- C.** The client's family visited during the last dressing change
- D.** The client asked whether the wound will leave a scar

13. A practical nurse is preparing to leave a voicemail for the provider about a client with heart failure who is more short of breath today. Which message best demonstrates effective interdisciplinary information sharing?

- A.** This is the nurse calling about a client who seems worse and may need a medication change. Please call back.
- B.** I'm calling about Ms. Lewis in room 412. She reports increased shortness of breath since this morning, has new crackles at both lung bases, oxygen saturation is 91% on room air, respirations are 26/min, and weight is up 1.1 kg since yesterday. Please return the call for further orders.
- C.** Ms. Lewis is probably retaining fluid again. I think she needs more diuretic because she has heart failure.
- D.** Your patient in 412 is uncomfortable and breathing fast. Can you come see her when available?

14. A practical nurse receives a telephone prescription from a provider to change a client's pain medication. To support safe and legally sound practice, which action should the nurse take?

- A.** Write the order in the chart and have the provider sign it sometime during the next week
- B.** Repeat the prescription back to the provider for verification and follow agency policy for telephone orders
- C.** Ask the unlicensed assistive personnel to listen on speakerphone and confirm the prescription
- D.** Wait to carry out the prescription until the provider comes to the unit and rewrites it in person

15. A practical nurse is documenting a dressing change for a postoperative client. After entering the note, the nurse notices that the drainage amount was typed as 150 mL instead of 15 mL. Which action is most appropriate?

- A.** Delete the original entry so the incorrect information cannot be seen
- B.** Leave the error unchanged because altering the record could appear dishonest
- C.** Correct the entry according to facility policy so the original documentation remains traceable
- D.** Ask another nurse to document the correct amount in a separate note without mentioning the error

16. A practical nurse in a rural health clinic is caring for a 9-year-old child with frequent headaches. During the visit, the child quietly says, "Sometimes my uncle comes into my room at night," then becomes silent and avoids eye contact. What is the nurse's best action?

- A.** Ask the child detailed follow-up questions to confirm exactly what happened before telling anyone
- B.** Report the suspicion according to facility policy and document the child's statement objectively
- C.** Wait to see whether the child brings it up again at a future visit
- D.** Discuss the statement with the uncle first to hear his explanation

17. A practical nurse in an assisted living facility notices that a resident with limited mobility has repeated unexplained missed meals, a strong body odor, and a stage 2 pressure injury not present last week. The resident's paid caregiver says, "She's difficult, and I can't do everything," but no one has directly witnessed mistreatment. Which action by the nurse is most appropriate?

- A.** File a report only after another staff member confirms the neglect
- B.** Document the findings and monitor the resident for 24 hours before deciding whether to report
- C.** Report suspected neglect based on the observed findings and follow facility policy

D. Wait until the resident states that the caregiver is neglecting her

18. A practical nurse asks an assistive personnel (AP) to help care for several clients on a rehabilitation unit. Which statement by the nurse shows the best delegation practice?

- A.** Please help Room 204 get cleaned up sometime this morning.
- B.** Take vital signs for all clients and let me know if any are abnormal.
- C.** Ambulate Mr. Diaz 50 feet with a gait belt after I give his pain medication, and tell me immediately if he reports dizziness or needs more than one-person assistance.
- D.** You know the routine for postop clients, so just do what you usually do.

19. A practical nurse is caring for clients on a medical-surgical unit and is working with an assistive personnel (AP) who floated from another unit. Which assignment is most appropriate for the nurse to delegate to the AP?

- A.** Teach a client with newly prescribed inhalers how to use the devices correctly
- B.** Reinforce discharge instructions for a client newly diagnosed with heart failure
- C.** Assist a stable client who uses a walker with routine ambulation in the hallway after the nurse has assessed the client
- D.** Evaluate whether a postoperative client's pain medication was effective 30 minutes after administration

20. A practical nurse in a pediatric clinic is reinforcing an asthma action plan with a parent who has a hearing impairment and communicates primarily through American Sign Language. The child has had two recent emergency visits for asthma flare-ups. Which action by the nurse best promotes equitable access to care?

- A.** Ask the child's older sibling to explain the asthma plan at home
- B.** Provide the parent with written instructions only and speak slowly while facing the parent
- C.** Arrange for a qualified sign language interpreter during teaching and follow-up communication
- D.** Delay teaching until the parent brings a friend who can help interpret

21. A practical nurse in a large primary care practice reviews follow-up data for clients with newly diagnosed hypertension. Which finding most strongly suggests a structural inequity that the nurse should escalate for system-level change rather than address only one client at a time?

- A.** One client missed an appointment after forgetting the visit date
- B.** Clients with private insurance and Medicaid have similar blood pressure control rates

C. Clients living in one low-income zip code have the highest no-show rates, and the clinic stopped offering evening visits last quarter

D. Several clients ask questions about medication side effects during routine teaching

22. A nurse in an outpatient infusion clinic is ready to connect a prescribed IV medication for a client who has just been seated in a treatment chair. The client is awake and oriented but says, "My hands are full, so I can't reach for my card. I'm Linda, and I'm here for the same infusion I get every month." Which action by the nurse is best to verify the client's identity before starting the infusion?

A. Ask the client to state her full name and date of birth, then compare that information with the infusion order and the client's identification band

B. Confirm the client's first name and treatment with the receptionist who checked her in

C. Use the treatment chair assignment and the client's statement about monthly infusions as identification

D. Ask the client to describe her diagnosis and compare it with the medical record

23. A nurse is preparing to use a portable oxygen cylinder to transport a client to x-ray. The flowmeter is attached, but when the nurse opens the cylinder, the pressure gauge reads near empty. Wall oxygen is not available in the hallway, and transport is scheduled now. Which action should the nurse take?

A. Proceed with transport and monitor the client closely for signs of distress

B. Tap the gauge and reopen the cylinder to see if the reading changes

C. Replace the cylinder with a full one before transporting the client

D. Increase the flow rate briefly to check whether enough oxygen remains

24. A nurse is about to give the first dose of amoxicillin to a client. During the medication check, the client says, "Years ago, after penicillin, my lips swelled and I had trouble breathing." The medication administration record does not list this allergy. Which action should the nurse take first?

A. Administer the medication and observe the client closely

B. Hold the medication and notify the provider of the reported reaction

C. Ask the client whether the reaction might have been caused by food instead

D. Document the allergy after giving the first dose

25. A practical nurse in a long-term care facility is conducting morning rounds when the nurse notices a strong smell of natural gas coming from the kitchenette. Several residents are seated in the adjacent dining area, and one staff member is about to switch on the kitchen light. Which action should the nurse take first?

A. Turn on the exhaust fan to clear the smell from the room

B. Instruct the staff member not to use the light switch and move residents away from the area

C. Search the kitchenette to locate the exact source of the leak

D. Call maintenance to report the odor before taking further action

26. In a long-term care unit, a client with moderate dementia becomes restless during mealtimes, stands up repeatedly, and starts walking away before finishing meals. The client is not combative and usually sits back down when a staff member stays nearby and offers simple cues. The nurse is concerned about falls. Which action best follows the principle of least-restrictive restraint use?

A. Place a lap belt on the client in the wheelchair during meals

B. Move the client to a chair near the nurses' station and provide close supervision during meals

C. Raise all four side rails on the client's bed after meals

D. Request a PRN sedative to keep the client seated through meals

27. A nurse brings a client's prescribed blood product to the bedside and begins the verification process with another licensed nurse. Before the transfusion is started, they realize the compatibility label lists a different medical record number than the client wearing the identification band. The client has not received any blood and has no symptoms. Which action is most appropriate?

A. Start the transfusion because the blood type matches and the error was caught before any reaction occurred

B. Return the blood product according to facility policy, notify the appropriate personnel, and complete an incident report for the near miss

C. Ask the client to state their name again and continue if the client confirms the correct identity

D. Store the blood product in the unit refrigerator until the provider decides whether it can still be used

28. A nurse is choosing a transfer device for a client who needs to move from the bed to a chair. Which client is the best candidate for transfer with a gait belt and one assisting nurse?

A. A client who is alert, follows directions, bears weight well, and has only mild unsteadiness

- B.** A client who is confused, cannot follow commands, and tries to sit down during standing
- C.** A client who is unable to bear weight on either leg after surgery
- D.** A client who becomes combative when touched and has severe trunk instability

29. A nurse plans to transfer a client with a mechanical lift from the bed to a chair. The nurse notes that the lift's sling has a torn seam near one of the attachment points. The client is waiting in bed and appears stable. What should the nurse do?

- A.** Use the sling for this one transfer because the client is stable
- B.** Place an extra draw sheet under the sling to reinforce the torn area
- C.** Delay the transfer until an intact sling is available and remove the damaged sling from use
- D.** Ask a second staff member to hold the torn side of the sling during the transfer

30. A practical nurse is preparing to administer a subcutaneous anticoagulant injection to a client. After performing hand hygiene, the nurse cleans the client's skin and removes the needle cap. Just before the injection, the client drops a tissue on the floor and asks the nurse to pick it up. The nurse uses the nondominant gloved hand to retrieve the tissue, places it in the trash, and returns to the bedside. Which action should the nurse take next?

- A.** Administer the injection using the same gloves because the needle and medication remain sterile
- B.** Remove the gloves, perform hand hygiene, apply new gloves, and prepare a new injection if contamination is suspected
- C.** Wipe the gloved hand with an alcohol swab and then give the injection
- D.** Ask another nurse to give the injection so the original nurse does not contaminate the medication

31. A nurse is preparing to collect a venous blood sample from a client using a vacuum tube system. The client is not on isolation precautions, and the nurse does not anticipate splashing or spraying. Which personal protective equipment is most appropriate for the nurse to wear?

- A.** Gloves only
- B.** Gloves and gown
- C.** Gloves, mask, and goggles
- D.** Sterile gloves only

32. A practical nurse is assisting with a sterile dressing change. While wearing sterile gloves, the nurse accidentally brushes the gloved nondominant hand against the metal bedrail. The dominant hand has not touched anything nonsterile. Which action is best?

- A.** Continue the procedure using only the dominant hand to handle sterile supplies
- B.** Ask a coworker to pour sterile solution over the contaminated glove and continue
- C.** Remove both gloves and obtain a new pair of sterile gloves before proceeding
- D.** Wipe the contaminated glove with sterile gauze and continue the dressing change

33. A practical nurse is preparing to move a client with varicella from the room to the treatment area for an urgent procedure. Which action is most appropriate during transport?

- A.** Place a surgical mask on the client and keep any skin lesions covered if possible
- B.** Have the client wear sterile gloves during transport
- C.** Ask transport staff to open the room door early to improve air circulation
- D.** Remove the client's isolation restrictions once the client leaves the room

34. A practical nurse is helping a client with oral care. The client has no isolation orders, but while the nurse is brushing the client's teeth, the client suddenly coughs and sprays saliva onto the nurse's forearm. Which action best follows standard precautions?

- A.** Continue the oral care, then wash the area with soap and water after leaving the room
- B.** Stop the procedure, remove the contaminated gloves, and document the exposure before completing care
- C.** Clean the forearm promptly, perform hand hygiene as needed, and replace any contaminated PPE before resuming care
- D.** Ask another staff member to finish the oral care because exposure to saliva requires reassignment

35. A nurse is reinforcing teaching with a client who is 22 weeks pregnant about when to contact the provider between prenatal visits. Which statement by the client indicates correct understanding?

- A.** I should call if I notice swelling in my ankles at the end of the day.
- B.** I should call if I have burning when I urinate.
- C.** I should call if I have mild heartburn after meals.
- D.** I should call if I have a small amount of white vaginal discharge.

36. A client at 38 weeks' gestation in active labor suddenly reports constant, severe abdominal pain that does not lessen between contractions. The nurse notes a rigid abdomen, scant dark vaginal bleeding, and a fetal heart rate of 90/min. Which complication should the nurse suspect first?

- A.** Placental abruption
- B.** Uterine atony
- C.** Precipitous labor
- D.** Umbilical cord prolapse

37. A nurse is reinforcing discharge teaching for a client who is 10 days postpartum after an uncomplicated vaginal birth. Which statement by the client indicates correct understanding of postpartum sexual activity and contraception?

- A.** I can resume intercourse as soon as my lochia becomes light because pregnancy is not possible this soon after birth.
- B.** I should wait until my provider says healing is adequate before resuming intercourse, and I still need birth control if I do not want to become pregnant.
- C.** Breastfeeding will prevent ovulation, so contraception is unnecessary until my first menstrual period returns.
- D.** Using tampons during intercourse is a good way to keep the area clean while I am still having light lochia.

38. A nurse is obtaining a health history from an adult client who says, "My mother and older brother both have diabetes." Which follow-up question is most appropriate to further assess the client's family health history?

- A.** What time of day do you usually check your blood sugar?
- B.** How old were your mother and brother when they were diagnosed?
- C.** Have you ever had trouble paying for your prescriptions?
- D.** What did you eat in the 24 hours before today's visit?

39. A practical nurse is completing an admission assessment for a client with chronic kidney disease. Which finding is most important to record as baseline physical data because it will be especially useful for comparing the client's fluid status over time?

- A.** The client's favorite beverages
- B.** The client's current body weight
- C.** The name of the client's pharmacy
- D.** The client's preferred sleep position

40. A nurse is reinforcing teaching for the parent of a child who received an inactivated influenza vaccine today. Which statement by the parent indicates a need for further teaching?

- A.** My child might have some soreness where the shot was given.
- B.** A mild fever later today can happen after this vaccine.
- C.** If my child develops nasal congestion, that means the vaccine caused influenza.
- D.** I should watch for unusual reactions and seek care if severe symptoms occur.

41. A nurse is reinforcing teaching with the parent of an 8-month-old infant. Which activity is most appropriate to support the infant's developmental needs?

- A.** Provide large blocks for building towers independently
- B.** Play peekaboo and offer brightly colored toys the infant can transfer from one hand to the other
- C.** Encourage the infant to complete simple puzzles during floor time
- D.** Schedule daily group play sessions with other infants to promote cooperative play

42. A nurse is obtaining a health history from an older adult client. The client answers some questions inappropriately and then says, "I hear you talking, but I can't tell which words you're saying." Which action by the nurse is best to reduce this communication barrier?

- A.** Ask the client to read and sign a written questionnaire instead
- B.** Speak directly into the client's ear using a louder voice
- C.** Lower the pitch of the voice and speak slowly while facing the client
- D.** Repeat each question several times using the same wording

43. A nurse is preparing to teach a client about a newly prescribed low-residue diet for ulcerative colitis. Before the teaching begins, the client says, "My daughter usually explains medical things to me. I don't really speak much English." Which action by the nurse is best?

- A.** Ask the client's daughter to translate the teaching instructions
- B.** Provide the client with a written diet sheet in English to review later
- C.** Arrange for a qualified medical interpreter before providing the teaching
- D.** Speak more loudly and use short sentences during the teaching session

44. A client who was diagnosed with infertility last week tells the practical nurse, "Everyone keeps telling me to relax, but that just makes me feel worse." Which response by the nurse best provides emotional support?

- A.** You should try not to focus on it so much.

- B. At least you still have time to explore other options.
- C. It sounds like those comments feel hurtful instead of comforting.
- D. Many couples go through this, so you are not alone.

45. A client who was recently diagnosed with Parkinson disease tells the nurse, "I keep thinking about how much worse this will get, so I haven't read any of the teaching materials or told my family yet." Which response by the nurse is most appropriate to promote effective coping?

- A. "Try not to think about the future right now, because stress can make symptoms worse."
- B. "Let's focus on one manageable step today, such as reviewing the information together and deciding whom you would like to tell first."
- C. "You should tell your family everything tonight so they can start making plans for you."
- D. "It is normal to feel overwhelmed, so you can wait until you feel ready to deal with this."

46. A client with moderate dementia becomes upset during morning care and swats at the nurse's hand while saying, "No bath! Go away!" The client is not injured, and vital signs are stable. Which action by the nurse is most appropriate?

- A. Stop the bath, step back briefly, and return later using a calm, simple explanation
- B. Firmly tell the client that hygiene is required and complete the bath quickly
- C. Ask another staff member to help hold the client's arms so the bath can be finished
- D. Explain in detail why refusing care is unsafe and may lead to skin problems

47. A client who recently learned that treatment for breast cancer will begin next week tells the nurse, "I haven't told my teenage daughter yet. Every time I try, I freeze." Which response by the nurse is most therapeutic?

- A. "You need to tell her as soon as possible so she can prepare herself."
- B. "What do you think makes starting that conversation feel so hard right now?"
- C. "Teenagers usually handle this better than parents expect."
- D. "Maybe another family member should explain it to her instead."

48. A nurse is caring for a client on a medical-surgical unit who becomes upset after learning a procedure has been delayed. The client is standing in the room, speaking loudly, and repeatedly saying, "This always happens to me." The client is not making threats or moving toward staff. Which response by the nurse is most appropriate?

- A. "You need to lower your voice so we can talk about this."
- B. "I can see this delay is upsetting. Tell me what is bothering you most right now."
- C. "There is nothing I can do until the provider calls back."
- D. "If you keep acting like this, I will need to call security."

49. A practical nurse is caring for an older adult with moderate dementia who becomes confused when walking back from the bathroom at night. The client often stops in the hallway, looks around, and says, "Where is my room?" Which intervention is most appropriate to promote the client's safety and independence?

- A. Place a large, easy-to-recognize personal photo on the client's room door
- B. Move the client to a room farther from the nurses' station to reduce noise
- C. Instruct the client to use the call light and wait for staff before every bathroom trip
- D. Tell the client the room number each time and ask the client to repeat it back

50. A client tells the nurse, "I have been under constant stress for months. On my days off, I stay in bed most of the day because I feel drained, but afterward I feel even worse." Which nursing response is most appropriate?

- A. "Try to ignore the stress so you can catch up on your rest."
- B. "Plan one small enjoyable activity for each day, even if you do not feel like doing it at first."
- C. "You should avoid activity until your stress is completely gone."
- D. "It is best to wait until you have more energy before changing your routine."

51. A client hospitalized for worsening chronic obstructive pulmonary disease says to the practical nurse, "I keep a small sacred bracelet on for protection, and I get anxious when people tell me to remove it." The client is scheduled for an MRI in 1 hour. Which nursing action is most appropriate?

- A. Tell the client the bracelet must be removed and place it in the bedside drawer without further discussion
- B. Ask the client to explain the bracelet's importance, then inform the RN and MRI staff so safe alternatives can be explored before the test
- C. Allow the client to keep the bracelet on because spiritual items should never be removed in the hospital

D. Suggest the client focus on breathing exercises instead of spiritual concerns until after the MRI

52. A nurse is admitting a client for treatment of pneumonia. During the interview, the client answers briefly and looks to the spouse before responding to questions about treatment preferences. Which action by the nurse is most appropriate?

- A.** Direct all questions to the spouse because the spouse appears to be the family decision-maker
- B.** Ask the client how they prefer health information and decisions to be discussed during care
- C.** Explain that hospital policy requires the client to answer independently without family input
- D.** Assume the client does not understand the treatment plan and repeat all questions more loudly

53. A client who identifies as transgender tells the nurse before discharge, "Please don't discuss my gender identity when my employer calls. They only know that I was hospitalized." Later, the unit clerk says, "The client's supervisor is on the phone asking why so many follow-up visits are needed." Which action by the nurse is most appropriate?

- A.** Explain that the visits are related to the client's gender transition so the employer understands the need for time off
- B.** Tell the employer that the client is stable and will return the call personally after discharge
- C.** Ask the employer what details are needed, then answer only the questions related to scheduling
- D.** Transfer the call to the client room so the client can be asked in real time whether disclosure is acceptable

54. A client with newly diagnosed legal blindness tells the nurse, "My daughter keeps trying to do everything for me now. I know she means well, but it makes me feel helpless and useless." Which nursing response is best?

- A.** "You should let your daughter help until you get used to the diagnosis."
- B.** "Try not to focus on those feelings. Your daughter is only trying to be supportive."
- C.** "It sounds frustrating to feel less independent. Let's talk about tasks you want to keep doing yourself and how we can support that safely."
- D.** "Most people adjust to vision loss over time, so this feeling will probably pass soon."

55. A client who is receiving end-of-life care says to the nurse, "I need to get some things in order, but every time my sister comes in, she tells me not to talk like that and to focus on getting better." Which nursing action is most appropriate?

- A.** Tell the client that the sister is probably trying to be supportive and encourage the client to avoid upsetting conversations

B. Ask the client whether the nurse may help arrange a private conversation so the client can discuss priorities and wishes without interruption

C. Explain to the sister that she must accept the client's condition and stop discussing recovery

D. Redirect the client to a pleasant topic to reduce emotional distress during the visit

56. A nurse is caring for a client with a mature gastrostomy tube who receives intermittent feedings. When the nurse arrives to give the next feeding, the external bumper is pressing tightly against the skin, and the tissue around the stoma is swollen and painful. Which action is most appropriate?

- A.** Loosen the external bumper slightly and reassess the stoma site
- B.** Clean the area with hydrogen peroxide and apply a tighter dressing
- C.** Advance the tube 2 cm farther into the stomach and secure it
- D.** Begin the feeding as prescribed because mild swelling is expected

57. A nurse is assisting a client who has been on bed rest for several days to dangle at the bedside before ambulation. The client reports dizziness and appears pale after sitting up. Which action should the nurse take first?

- A.** Help the client stand and begin walking slowly
- B.** Return the client to a lying position and assess vital signs
- C.** Ask the client to take deep breaths until the dizziness passes
- D.** Offer the client water and continue sitting at the bedside

58. A nurse is reinforcing dietary teaching for a client who has heart failure and is prescribed a sodium-restricted diet. Which client statement indicates an understanding of the teaching?

- A.** I will choose canned soup for lunch because it is easy to prepare.
- B.** I will season my food with lemon juice and herbs instead of table salt.
- C.** I will eat smoked ham instead of fresh chicken because it has more flavor.
- D.** I will use soy sauce on vegetables instead of adding salt while cooking.

59. A client with a fresh right below-knee amputation reports incisional pain rated 5 out of 10 and says, "I feel like my leg is still there and it hurts." Which nonpharmacologic intervention should the nurse implement first?

- A.** Apply a heating pad directly to the residual limb

- B.** Reposition the client and use a residual limb shrinker or gentle massage if prescribed
- C.** Keep the client on complete bed rest to avoid worsening the pain
- D.** Place the residual limb in a dependent position over the side of the bed
- 60.** A nurse is caring for a client with Parkinson disease who is slow to move and has difficulty starting tasks in the morning. The client says, "By the time I get my shirt on, I'm worn out." Which nursing action is most appropriate to support the client's performance of activities of daily living?
- A.** Complete the dressing task for the client to prevent fatigue
- B.** Schedule dressing at a time when the client's antiparkinson medication is having its best effect
- C.** Encourage the client to remain in bed until all morning care supplies are gathered
- D.** Ask the client to hurry through dressing before muscle stiffness worsens
- 61.** A nurse is caring for a client who has urge urinary incontinence and is beginning bladder retraining. Which instruction should the nurse reinforce?
- A.** Void as soon as the urge is felt to prevent leakage
- B.** Limit all fluid intake after breakfast
- C.** Use relaxation and distraction techniques to delay voiding gradually
- D.** Drink caffeinated beverages to stimulate a more complete bladder emptying
- 62.** A practical nurse is caring for an older client who is frequently incontinent of stool. During morning care, the nurse notes redness and mild skin breakdown around the perianal area. Which intervention is most appropriate to help protect the client's skin?
- A.** Cleanse the area gently after each episode and apply a moisture barrier ointment
- B.** Massage the reddened area to increase local circulation
- C.** Dust the area with powder and leave the skin exposed to air most of the day
- D.** Scrub the area with soap and a washcloth until all stool residue is removed
- 63.** A practical nurse is caring for a client who has aphasia after a stroke and also wears glasses because of limited vision. The client becomes frustrated when asked questions during care and points instead of speaking. Which nursing action is most appropriate to improve communication?
- A.** Ask only yes-or-no questions and speak for the client when needs are not immediately clear
- B.** Use a communication board with large print and allow the client extra time to respond
- C.** Raise the voice so the client can better understand each instruction
- D.** Limit conversation during care to reduce the client's frustration
- 64.** A practical nurse is monitoring a client on strict intake and output. The client had 360 mL of urine, 120 mL of liquid stool, and 80 mL from a wound drainage device during the shift. The client also had one small formed stool. What total output should the nurse record?
- A.** 480 mL
- B.** 560 mL
- C.** 600 mL
- D.** 640 mL
- 65.** A practical nurse is assisting a client who has been on bed rest for 4 days to sit on the side of the bed for the first time. The client reports dizziness and appears pale. Which action should the nurse take first?
- A.** Help the client lie back down and assess blood pressure
- B.** Encourage the client to remain sitting until the feeling passes
- C.** Assist the client to stand and pivot to a chair
- D.** Offer the client a glass of water and continue to observe
- 66.** A nurse is preparing to administer a scheduled oral medication to a client. After checking the medication label against the MAR, the nurse enters the room and finds another client's spouse sitting at the bedside. The client asks, "Can you tell me what this medicine is for before I take it?" Which action best supports safe medication administration and the client's medication rights?
- A.** Explain the medication's name, purpose, and common effects to the client before administration
- B.** Ask the spouse to explain the medication because family members often know the client's routine
- C.** Tell the client to take the medication first and read the drug information sheet afterward
- D.** State only that the medication was prescribed by the provider and should be taken now
- 67.** A client with thrombocytopenia is scheduled for an invasive bedside procedure later today. Which blood product would the nurse expect to administer to reduce the risk of bleeding related to the low platelet count?
- A.** Platelets
- B.** Packed red blood cells
- C.** Fresh frozen plasma
- D.** Albumin

68. A nurse needs to administer 150 mL of IV medication over 45 minutes using tubing with a drop factor of 20 gtt/mL. What flow rate should the nurse regulate in drops per minute?

- A. 50 gtt/min
- B. 67 gtt/min
- C. 75 gtt/min
- D. 90 gtt/min

69. A practical nurse gives a prescribed controlled opioid to a client at 1400. At 1600, the client tells the nurse, "I think you forgot my pain medicine earlier." The medication administration record shows the dose was documented as given at 1400. Which action by the nurse is best?

- A. Tell the client the medication was already documented and cannot be discussed further
- B. Document the client statement and give another dose to maintain comfort
- C. Assess the client, verify the medication record, and investigate the concern according to facility policy
- D. Ask another nurse to decide whether the medication was actually given

70. A nurse is reinforcing teaching for a postoperative client who will use a patient-controlled analgesia (PCA) pump for the first time. Which client statement indicates correct understanding of how to use the device?

- A. I should wait until my pain is severe before pressing the button.
- B. My family can press the button for me if I am resting.
- C. I am the only person who should press the button when I begin to feel pain.
- D. If one dose does not help, I should keep pressing the button until I get sleepy.

71. A nurse is preparing to administer the next dose of enoxaparin subcutaneously to a client who is on bed rest after hip surgery. Which pre-administration data is most important for the nurse to review?

- A. The client's platelet count
- B. The client's oral fluid intake for the past 24 hours
- C. The client's ability to reposition independently in bed
- D. The client's calcium level

72. A client with community-acquired pneumonia has been receiving IV ceftriaxone for 48 hours. Which finding best indicates the medication is producing the intended therapeutic response?

- A. White blood cell count increases from 11,000/mm³ to 14,000/mm³
- B. Oral temperature decreases from 39.1°C (102.4°F) to 37.6°C (99.7°F)
- C. The client reports mild pain at the IV insertion site

D. Urine output measures 35 mL/hr

73. A client with seizure disorder has been taking phenytoin for 2 weeks. Which new finding should the nurse report to the provider immediately as a possible serious adverse response?

- A. Mild gingival swelling while brushing teeth
- B. Unsteady gait and slurred speech
- C. Decreased appetite at lunchtime
- D. Occasional difficulty falling asleep

74. A patient with type 2 diabetes takes glyburide each morning. The patient says, "I started taking propranolol for my tremors, and now I don't always notice when my blood sugar is dropping." Which explanation by the nurse is best?

- A. Propranolol can increase glyburide absorption from the intestine.
- B. Propranolol can mask adrenergic symptoms of hypoglycemia such as tremor and palpitations.
- C. Propranolol causes glyburide to stop working, so blood glucose changes become unpredictable.
- D. Propranolol delays stomach emptying, which prevents the body from responding to low glucose levels.

75. A client with active tuberculosis is prescribed isoniazid 300 mg by mouth daily. Before administering the next dose, the nurse reviews the laboratory results and notes AST 168 U/L and ALT 194 U/L. Which action is most appropriate?

- A. Administer the dose as prescribed and document the findings
- B. Hold the medication and notify the provider of the elevated liver enzymes
- C. Give the medication with food to reduce gastrointestinal upset
- D. Request a serum potassium level before deciding whether to give the dose

76. A nurse is reinforcing teaching for a client who is starting alendronate for osteoporosis. Which instruction should the nurse include?

- A. Take the medication at bedtime with a full glass of milk.
- B. Remain upright for at least 30 minutes after taking the medication.
- C. Crush the tablet and mix it with applesauce if swallowing is difficult.
- D. Take the medication with breakfast to reduce stomach upset.

77. A nurse is preparing to administer an ophthalmic solution to a client. The bottle was stored in the refrigerator, and the label indicates the medication should be kept refrigerated. Which action is most appropriate before administration?

- A.** Instill the drops immediately to maintain the medication at the coldest temperature possible
- B.** Warm the bottle in the microwave for a few seconds before use
- C.** Hold the bottle in the hands briefly to bring the solution closer to room temperature before instilling
- D.** Place the bottle in hot water until the solution feels warm

78. A nurse is preparing to administer acetaminophen 650 mg by mouth. The tablets available are labeled 325 mg each. How many tablets should the nurse administer?

- A.** 1 tablet
- B.** 1.5 tablets
- C.** 2 tablets
- D.** 2.5 tablets

79. A practical nurse is collecting data on four clients in an adult day program. Which client should the nurse identify as being at greatest risk for burns in the home?

- A.** A client with diabetic neuropathy who has decreased sensation in both feet
- B.** A client with seasonal allergies who uses tissues frequently
- C.** A client with presbyopia who wears reading glasses
- D.** A client with osteoarthritis who reports morning joint stiffness

80. A nurse is collecting a throat culture from a school-age child with a sore throat. Which action should the nurse take to obtain the specimen correctly?

- A.** Swab the inside of the cheeks and tongue to avoid triggering gagging
- B.** Ask the child to drink water just before the specimen is collected
- C.** Use a sterile swab to sample the tonsillar area and posterior pharynx without touching the lips or cheeks
- D.** Apply a topical antiseptic spray to the throat before swabbing

81. A nurse is reviewing laboratory results for a client who has been taking metformin and now reports increasing fatigue, abdominal discomfort, and rapid breathing. Which laboratory finding should the nurse identify as most concerning?

- A.** Serum lactate 5.2 mmol/L
- B.** Hemoglobin A1c 8.4%
- C.** Fasting blood glucose 168 mg/dL

D. Blood urea nitrogen 24 mg/dL

82. A client who is 34 weeks pregnant comes to the prenatal clinic for a routine visit. The client says, "My face and hands seem puffer than usual, and this morning I had a bad headache that didn't go away after I rested." Which assessment finding should the nurse report to the provider immediately?

- A.** Blood pressure of 164/106 mm Hg
- B.** Weight gain of 2 lb (0.9 kg) in 1 week
- C.** Trace protein on urine dipstick
- D.** Bilateral ankle edema at the end of the day

83. A practical nurse is obtaining orthostatic vital signs for an adult client who reports dizziness when getting out of bed. After the client rests supine for 5 minutes, the blood pressure is 128/76 mm Hg and pulse is 82/min. After standing for 3 minutes, the blood pressure is 104/68 mm Hg and pulse is 102/min, and the client says, "I feel lightheaded." Which interpretation is most appropriate?

- A.** These findings are expected because standing normally lowers systolic blood pressure by more than 20 mm Hg
- B.** These findings suggest orthostatic hypotension and should be reported
- C.** The standing blood pressure is inaccurate because the diastolic pressure also changed
- D.** The pulse increase shows adequate compensation, so no further action is needed

84. A client is scheduled for a computed tomography (CT) scan with IV contrast. Which assessment finding is most important for the nurse to report to the provider before the procedure?

- A.** The client reports claustrophobia during past imaging tests
- B.** The client takes metformin for type 2 diabetes mellitus
- C.** The client drank coffee with breakfast 6 hours ago
- D.** The client had a CT scan without contrast 2 years ago

85. A nurse is reviewing a client's ECG rhythm strip. The rhythm is irregularly irregular, no distinct P waves are visible, and the ventricular rate is 118/min. Which rhythm should the nurse identify?

- A.** Atrial fibrillation
- B.** Sinus tachycardia
- C.** First-degree AV block
- D.** Ventricular tachycardia

86. A nurse is preparing to obtain a venous blood specimen from an adult client. The nurse notes a peripheral IV infusing in the left forearm. Both antecubital veins are visible, and the client has no history of mastectomy or dialysis access. Which venipuncture site is most appropriate to reduce the risk of a diluted or inaccurate specimen?

- A. A vein in the left forearm distal to the IV site
- B. A vein in the left hand proximal to the IV site
- C. A vein in the right antecubital area
- D. Any visible vein after pausing the IV for 1 minute

87. A practical nurse is caring for a client who is being transported to surgery for a left knee arthroscopy. In the preoperative holding area, the client says, "I'm here for my right knee procedure." Which action should the nurse take first?

- A. Proceed with transport because the consent form has already been signed
- B. Ask the client which knee has been painful recently and document the response
- C. Stop the preoperative process and notify the RN and surgical team immediately
- D. Reassure the client that the surgeon will verify the correct site in the operating room

88. A client has a central venous access device. During the dressing change, the nurse notes that the insertion site is red and warm with a small amount of purulent drainage. The client also reports tenderness at the site. Which action is most appropriate?

- A. Document the findings and continue to use the catheter as prescribed
- B. Obtain a blood pressure in the affected arm to compare circulation
- C. Stop using the catheter and notify the provider of possible infection
- D. Flush the catheter with heparinized solution to maintain patency

89. A nurse is caring for a client who is receiving IV vancomycin. Fifteen minutes after the infusion begins, the client reports itching and feels "hot all of a sudden." The nurse notes flushing of the face and neck. Which focused assessment should the nurse perform first?

- A. Measure the client's blood pressure and assess airway status
- B. Inspect the IV site for infiltration
- C. Ask the client about a history of seasonal allergies
- D. Auscultate bowel sounds in all four quadrants

90. A client with syndrome of inappropriate antidiuretic hormone secretion (SIADH) has been on fluid restriction and prescribed therapy for 24 hours. Which finding best indicates the treatment is effective?

- A. Urine specific gravity increases from 1.020 to 1.035
- B. Serum sodium increases from 122 mEq/L to 130 mEq/L
- C. The client reports increased thirst
- D. Urine output decreases from 45 mL/hr to 20 mL/hr

91. A client has a nasogastric tube connected to low suction for gastric decompression after abdominal surgery. During rounds, the nurse finds the suction canister is nearly full of green drainage, but the client denies nausea and the abdomen is soft. Which action is most appropriate?

- A. Irrigate the nasogastric tube with sterile water immediately
- B. Replace the suction canister and maintain the prescribed suction
- C. Clamp the tube for 30 minutes to prevent fluid loss
- D. Advance the tube 2 inches to improve drainage

92. A nurse is caring for a client whose temperature is 35.8°C (96.4°F) after several hours in a cool environment. Which assessment finding is the clearest indication that the body is attempting to increase heat production?

- A. Peripheral vasodilation
- B. Shivering
- C. Diaphoresis
- D. Warm, flushed skin

93. A nurse is assessing a client's surgical incision 24 hours after surgery. Which finding is most consistent with normal wound healing at this time?

- A. A small amount of serosanguineous drainage on the dressing
- B. Thick yellow drainage with a strong odor
- C. Separation of the wound edges with visible tissue layers
- D. Increasing warmth, swelling, and severe pain at the incision

94. A nurse enters a room and finds an adult client collapsed on the floor beside the bed. The nurse confirms the scene is safe, taps the client, and shouts, but the client does not respond. What should the nurse do next to assess for cardiac arrest?

- A. Check for a carotid pulse and normal breathing at the same time for no more than 10 seconds
- B. Open the airway and give 2 rescue breaths before checking for a pulse
- C. Auscultate the apical pulse for 1 full minute
- D. Place the client in a side-lying position and observe for spontaneous movement

95. A client with chronic obstructive pulmonary disease is prescribed oxygen at 2 L/min by nasal cannula. While ambulating to the bathroom, the client becomes short of breath and says, "I can't catch my breath." The pulse oximeter reads 82%. Which action should the nurse take first?

- A.** Increase the oxygen flow rate to 6 L/min
- B.** Assist the client to sit upright and encourage pursed-lip breathing
- C.** Notify the provider that the client is hypoxic
- D.** Apply a nonrebreather mask at 15 L/min

96. A nurse is caring for a client with a chronic tracheostomy who is alert and receiving oxygen by tracheostomy collar. During assessment, the nurse notes a respiratory rate of 24/min, audible gurgling from the tracheostomy, and a strong cough. The client nods yes when asked whether breathing feels harder than usual. Which nursing action is most appropriate?

- A.** Suction the tracheostomy using sterile technique
- B.** Deflate the tracheostomy cuff to improve airflow
- C.** Remove the tracheostomy ties to inspect the skin underneath
- D.** Encourage oral fluids to thin secretions before intervening

97. A client receiving mechanical ventilation has an endotracheal tube cuff pressure measured at 38 mm Hg. Which nursing action is most appropriate?

- A.** Increase the pressure slightly to prevent air leakage
- B.** Deflate the cuff completely for 5 minutes, then reinflate
- C.** Report the finding as expected for a ventilated client
- D.** Reduce the cuff pressure according to protocol and reassess

98. A client with chest pain is on continuous cardiac monitoring. The monitor displays a chaotic, irregular waveform with no identifiable P waves, QRS complexes, or T waves, and the client is unresponsive and has no palpable pulse. Which action should the nurse take immediately?

- A.** Prepare for synchronized cardioversion
- B.** Begin CPR and defibrillation per protocol
- C.** Administer atropine IV push
- D.** Obtain a 12-lead ECG before intervening

99. A nurse is reinforcing teaching for a client with a new colostomy who wants to resume wearing regular clothing. Which instruction is most appropriate for the nurse to include?

- A.** Avoid wearing any clothing over the stoma to prevent tissue damage.
- B.** Choose clothing that does not bind tightly across the stoma or pouch.

C. Wear a belt directly over the stoma to keep the pouch flat.

D. Limit activity outside the home until loose stools are no longer present.

100. A nurse is caring for a client who is 6 hours postoperative after carotid endarterectomy. Which change in the client's condition should the nurse recognize as the priority to report immediately?

- A.** Blood pressure increases from 138/82 mm Hg to 156/88 mm Hg
- B.** The client reports a sore throat when swallowing
- C.** The client develops new swelling at the incision with increasing difficulty breathing
- D.** The client rates incisional pain as 6 on a scale of 0 to 10

Simulation Test 16 — Answer Key

1. B	26. B	51. B	76. B
2. C	27. B	52. B	77. C
3. C	28. A	53. B	78. C
4. C	29. C	54. C	79. A
5. B	30. B	55. B	80. C
6. B	31. A	56. A	81. A
7. C	32. C	57. B	82. A
8. C	33. A	58. B	83. B
9. A	34. C	59. B	84. B
10. A	35. B	60. B	85. A
11. C	36. A	61. C	86. C
12. B	37. B	62. A	87. C
13. B	38. B	63. B	88. C
14. B	39. B	64. B	89. A
15. C	40. C	65. A	90. B
16. B	41. B	66. A	91. B
17. C	42. C	67. A	92. B
18. C	43. C	68. B	93. A
19. C	44. C	69. C	94. A
20. C	45. B	70. C	95. B
21. C	46. A	71. A	96. A
22. A	47. B	72. B	97. D
23. C	48. B	73. B	98. B
24. B	49. A	74. B	99. B
25. B	50. B	75. B	100. C

Simulation Test 17

100 questions

1. The practical nurse is passing afternoon medications in an assisted-living facility when several needs arise at once. Which action should the nurse take first?
 - A. Escort a resident with mild confusion back to the dining room after she enters the hallway
 - B. Recheck a resident's blood pressure 20 minutes after giving a newly prescribed antihypertensive
 - C. Go to a resident who is coughing during lunch and is unable to speak
 - D. Bring scheduled eye drops to a resident who is waiting in his room
2. The practical nurse is planning the order of care for four assigned clients. Which client should the nurse see first?
 - A. A client with type 1 diabetes who is pale, diaphoretic, and confused before dinner
 - B. A client 1 day after a laparoscopic cholecystectomy who reports incisional pain rated 7/10
 - C. A client with chronic kidney disease whose potassium level from this morning is 5.3 mEq/L
 - D. A client with pneumonia who needs teaching about using an incentive spirometer
3. A practical nurse is planning assignments for an assistive personnel (AP) on a medical unit. Which task is most appropriate to delegate to the AP?
 - A. Apply a sterile dressing change to a chronic leg ulcer
 - B. Teach a client how to use a newly prescribed walker
 - C. Reposition a client every 2 hours after the nurse has assessed the client's mobility and skin condition
 - D. Determine whether a client's as-needed pain medication was effective
4. A nurse is caring for an alert adult client who is scheduled to begin a blood transfusion. After the provider explains the treatment and leaves, the client tells the nurse, "I signed the form because everyone seemed rushed, but I do not want the transfusion now." The client's hemoglobin is low, but the client is speaking clearly and answers questions appropriately. What is the nurse's best action?
 - A. Tell the client the transfusion must proceed because the consent form has already been signed
 - B. Notify the provider and support the client's decision to refuse the transfusion at this time
 - C. Ask the client's closest family member to talk with the client and sign again if they agree
 - D. Delay the transfusion briefly and prepare the blood anyway in case the client changes their mind
5. A practical nurse is caring for a client whose coworker arrives on the unit and says, "We work together, and the client asked me to bring a phone charger. Is the client still here, and what room number are they in?" The visitor is not listed in the chart as an approved contact. Which response by the nurse is most appropriate?
 - A. Tell the coworker the room number because bringing personal items supports the client's comfort
 - B. Ask the coworker to show identification, then provide the room number
 - C. State that the nurse cannot confirm whether the person is a client or share location information without authorization
 - D. Call the client's room and ask the client whether the coworker may visit, then give the coworker the answer
6. An alert adult client with decision-making capacity tells the nurse, "I want to make my own choices while I can, but if I become unable to speak for myself later, I want my partner to decide for me even if other relatives disagree." Which nurse response is most appropriate?
 - A. A living will is best because it lets your family vote on decisions if you cannot speak.
 - B. You can discuss naming your partner in a durable power of attorney for health care.
 - C. A do-not-resuscitate order will allow your partner to make all future treatment decisions.
 - D. Once you are hospitalized, your next of kin automatically replaces any need for written planning.
7. A nurse is caring for a client who is scheduled for an elective above-knee amputation. The client is awake, alert, and has signed the consent form after speaking with the surgeon. While the nurse is completing preoperative care, the client says, "I know this leg is in bad shape, but I thought they were only taking my foot, not my whole leg." Which action should the nurse take first?
 - A. Notify the surgeon and hold preoperative preparation until the client receives clarification
 - B. Explain that the signed consent form allows the planned surgery to proceed
 - C. Ask a family member to help reassure the client about the need for the surgery
 - D. Document the client's statement and continue preparing the client for transport

8. A client is being discharged after hospitalization for cellulitis of the lower leg. The provider prescribes an oral antibiotic, daily wound care, and a follow-up visit in 4 days. The client says, "I can get my medicine, but I don't know how I'll change this dressing on the back of my calf by myself." Which action by the nurse is most appropriate before discharge?

- A.** Ask the client to skip dressing changes until the follow-up visit
- B.** Arrange a home health referral for wound care assistance
- C.** Teach the client to clean the wound only when drainage is visible
- D.** Tell the client to have a neighbor decide whether the wound looks infected

9. A client is preparing for discharge after hospitalization for a new spinal cord injury. The client uses a wheelchair, needs assistance transferring, and will return to a second-floor apartment with no elevator. The spouse says, "We'll figure it out somehow once we get home." Which nursing action is the priority before discharge?

- A.** Provide written instructions about wheelchair safety and continue with discharge
- B.** Encourage the spouse to ask friends to help carry the client upstairs
- C.** Request case management involvement to address the unsafe home environment and discharge needs
- D.** Schedule outpatient physical therapy to begin the following week

10. A practical nurse is helping with a quality improvement project on a unit where client falls have increased during evening shifts. Before making changes, the team wants to understand when and where the falls are happening most often. Which action is most appropriate first?

- A.** Begin a new hourly rounding schedule for all evening staff immediately
- B.** Collect and review fall data by time, location, and circumstances
- C.** Post fall-prevention reminders in every client room
- D.** Require all clients to use bed alarms during the evening shift

11. A community clinic is working to improve blood pressure follow-up after elevated readings are found during walk-in visits. The team tests a new workflow in which staff give clients a printed follow-up appointment before they leave. After 6 weeks, 92% of eligible clients received printed appointments, but the percentage who returned for follow-up within 30 days changed very little. Which action is best for the team to take next?

- A.** Conclude the project was successful because staff followed the new workflow
- B.** Stop the project and return to the old process because the outcome did not improve quickly
- C.** Investigate additional reasons clients are not returning and refine the intervention
- D.** Expand the same workflow to every clinic site because compliance with the process is high

12. A practical nurse is caring for a client newly diagnosed with tuberculosis who will need several months of medication therapy after discharge. During teaching, the client says, "I work nights, move between relatives' homes, and I probably won't remember all these pills." Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the statement in the chart and continue routine discharge teaching
- B.** Notify the case manager or public health/discharge planning team that the client reports unstable housing and difficulty managing the medication schedule
- C.** Tell the client to set a phone alarm and bring any questions to the next clinic visit
- D.** Wait to see whether the client misses any doses before involving other team members

13. A practical nurse is caring for a client receiving warfarin. Morning laboratory results show an INR of 4.8. The client also tells the nurse, "My gums bled when I brushed my teeth," and physical therapy is scheduled to begin gait training in 20 minutes. Which action best supports interdisciplinary information sharing?

- A.** Send the client to physical therapy as scheduled because activity promotes recovery
- B.** Report only the elevated INR to the provider and wait for further instructions before informing anyone else
- C.** Notify the provider of the elevated INR and bleeding, and inform physical therapy that the client may be at increased bleeding risk before the session begins
- D.** Ask the client to use a softer toothbrush and recheck for bleeding later in the day

14. A practical nurse is caring for a client who is alert and oriented and scheduled for discharge later that day. The client's adult son arrives and says, "I handle all of my mother's affairs, so give me her discharge instructions and medication list now." The client is resting in bed and has not stated that information may be shared. Which action by the nurse is most appropriate?

- A.** Provide the information because close family members may receive discharge instructions
- B.** Ask the client whether she wants the information shared with her son before discussing her care
- C.** Give the son the medication list but withhold the diagnosis
- D.** Refuse to speak with the son under any circumstances

15. Near the end of a shift, a practical nurse realizes that a medication was administered 45 minutes late because the dose had been stored in the wrong drawer. The client remains stable and reports no symptoms. The nurse is worried about disciplinary action. Which action is most appropriate?

- A.** Document the medication as having been given at the scheduled time to keep the record consistent
- B.** Tell the oncoming nurse privately so the delay does not need to be entered in the chart
- C.** Record the actual administration time, assess the client, and follow facility procedure for reporting the medication variance
- D.** Wait to see whether the client develops a problem before documenting anything further

16. A practical nurse in a medical office is caring for a 6-year-old child with a scalp laceration. The caregiver says the child fell off a porch step. During the assessment, the nurse notices bruises in different stages of healing on the child's back and thighs. What is the nurse's best action?

- A.** Wait to report until the health care provider confirms that abuse occurred
- B.** Question the caregiver repeatedly until the explanation matches the injuries
- C.** Report the suspected abuse according to facility policy and legal requirements
- D.** Discharge the child after treatment because the cause of injury is unclear

17. A practical nurse in a long-term care facility suspects financial exploitation of an older adult who is alert and oriented. The client states, "My grandson keeps taking my debit card and withdrawing money, but I don't want any trouble." Which action by the nurse is most appropriate?

- A.** Respect the client's wishes and take no further action because the client is competent

- B.** Document the statement and follow facility procedure for reporting suspected abuse or exploitation
- C.** Call the grandson and instruct him to return the money immediately
- D.** Advise the client to hire an attorney before telling anyone else

18. A practical nurse asks an assistive personnel (AP) to help with morning care for several clients. Which situation requires the nurse to intervene immediately?

- A.** The AP gives a client with mild hand weakness a plate guard and allows extra time for breakfast
- B.** The AP reports that a client's antiembolism stockings feel tight and asks the nurse to check the legs
- C.** The AP applies a heating pad to the lower back of a client who reports soreness after walking
- D.** The AP reminds a client with prescribed fluid restriction to use the marked water pitcher at the bedside

19. A practical nurse is working with an assistive personnel (AP) in a long-term care facility. Which task is most appropriate for the nurse to assign to the AP?

- A.** Reinforce teaching to a newly admitted client about how to use an incentive spirometer
- B.** Apply a prescribed medicated cream to a resident's pressure injury
- C.** Escort a stable resident to the dining room and assist with opening food containers
- D.** Assess a resident's pain 30 minutes after receiving an opioid analgesic

20. A practical nurse in a community health center is reinforcing instructions for a client with newly prescribed antibiotics. The client says, "I can read English a little, but I understand medicine better in Somali. I usually just guess from the bottle." Which action by the nurse best promotes equitable access to care?

- A.** Ask the client's teenage son to explain the medication directions at home
- B.** Provide medication teaching using a qualified medical interpreter and request written instructions in the client's preferred language if available
- C.** Speak more loudly and repeat the instructions several times in English
- D.** Tell the client to call the pharmacy later if questions come up

21. A practical nurse in a free clinic notices that many clients with newly diagnosed depression do not return for counseling follow-up. Review of clinic notes shows a pattern: clients without private transportation miss visits more often, and the counseling office is located in another building that is open only from 9 a.m. to 3 p.m. Monday through Friday. Which nursing recommendation best addresses equitable access to care at the system level?

- A.** Advise each affected client to arrange rides with friends or relatives
- B.** Have staff remind clients that follow-up is important for recovery
- C.** Recommend expanded counseling hours and coordination of same-day behavioral health visits in the main clinic location
- D.** Schedule only the most symptomatic clients for counseling appointments

22. A nurse is preparing to collect a sputum specimen from a client who is asleep. The client's roommate says, "That's Mr. Greene by the window." The name on the specimen label is Thomas Greene. Which action should the nurse take to identify the client correctly before obtaining the specimen?

- A.** Ask the roommate to confirm Thomas Greene's date of birth
- B.** Wake the client and compare two client identifiers with the identification band and the specimen label
- C.** Use the bed location and the name on the label to identify the client
- D.** Collect the specimen now and verify the client's identity after the client wakes up

23. A nurse is preparing to use a portable pulse oximeter to assess a client reporting shortness of breath. During the equipment check, the nurse notices the probe cable is cracked with exposed inner wiring near the connector, but the device still powers on and displays a reading. Which action should the nurse take?

- A.** Use the device briefly because it is giving a reading, then report the damage after assessing the client
- B.** Cover the cracked area with tape and continue using the device until a replacement is available
- C.** Remove the equipment from use and obtain a functioning pulse oximeter before assessing the client
- D.** Ask the client to hold the connector still so the nurse can get the most accurate reading possible

24. A nurse is preparing to insert an indwelling urinary catheter. The client says, "I had a severe reaction during a dental procedure after the staff used latex gloves. My throat started to swell." The catheterization kit is already opened on the bedside table. Which action should the nurse take first?

- A.** Proceed with the procedure using the opened kit because the catheter itself has not touched the client yet
- B.** Ask the client whether the reaction required hospitalization before deciding what to do
- C.** Stop the procedure and obtain latex-free supplies before continuing
- D.** Apply the sterile gloves from the kit carefully and monitor the client for any itching

25. A practical nurse in a rehabilitation unit hears the facility's tornado warning siren. Several clients are in the hallway near large exterior windows, and one nursing assistant is pushing a metal supply cart toward the day room. Which action should the nurse take first?

- A.** Move clients away from the windows to an interior hallway
- B.** Close the doors to all client rooms before relocating anyone
- C.** Secure the metal supply cart in the day room and then assist clients
- D.** Call each client's family to explain the shelter procedure

26. An older adult with dementia is admitted from a nursing home with dehydration. During the night, the client repeatedly stands up from the bedside commode before staff can assist, but sits back down when a nursing assistant remains in the room and gives step-by-step directions. The charge nurse suggests using a vest restraint to prevent falls. Which response by the practical nurse best supports least-restrictive restraint use?

- A.** Apply the vest restraint because preventing a fall is the highest priority
- B.** Request a sedative PRN so the client will sleep through the night
- C.** Continue one-to-one supervision and simple cueing because these measures are effective
- D.** Raise all four side rails to keep the client from getting up again

27. A nurse preparing to administer an oral antibiotic notices that the unit-dose package has the correct client name and medication name, but the printed expiration date shows the medication expired last month. The package is unopened, the medication has not been given, and the client has no adverse effects. How should the nurse interpret this situation?

- A.** It is an adverse event because expired medication was available on the unit

- B.** It is a near miss because the problem was identified before the medication reached the client
- C.** It is not reportable because no client harm occurred
- D.** It is a documentation error because the package label was outdated

28. A nurse is selecting a transfer device for a client who needs to move from a recliner to a wheelchair. The client is awake, follows commands, can grasp handles, and can bear weight on both legs for a few seconds, but is too weak to rise to standing without help. Which transfer device is most appropriate?

- A.** Transfer board
- B.** Sit-to-stand lift
- C.** Full-body mechanical lift
- D.** Lateral transfer device

29. A practical nurse is preparing to administer an eye ointment to a client. After performing hand hygiene, the nurse applies clean gloves and lowers the client's lower eyelid. The tip of the ointment tube accidentally touches the client's eyelashes before any medication is instilled. Which action should the nurse take?

- A.** Wipe the tube tip with sterile gauze and continue the application
- B.** Administer the ointment to the other eye first, then return to the affected eye
- C.** Discard the tube and obtain a new one before giving the medication
- D.** Ask the client to blink several times, then continue with the same tube

30. A practical nurse is caring for a client with vomiting and profuse diarrhea caused by suspected *Clostridioides difficile* infection. After helping the client to the bathroom, the nurse removes the gown and gloves correctly. Which action is most appropriate before leaving the room?

- A.** Use an alcohol-based hand rub until the hands are dry
- B.** Wash hands thoroughly with soap and water
- C.** Put on a clean pair of gloves to open the door
- D.** Apply hand lotion to prevent skin irritation, then exit the room

31. A nurse is preparing to change the bed linens of a client who is not on isolation precautions. The linens are dry, and the nurse does not expect contact with blood, body fluids, secretions, or excretions. Which personal protective equipment is most appropriate for this task?

- A.** Sterile gloves
- B.** Clean gloves
- C.** Gown and gloves
- D.** No PPE is required

32. A practical nurse is setting up a sterile field for a dressing change. After opening the sterile package correctly, the nurse notices the outer 1-inch edge of the drape is closest to the supplies that need to be picked up first. Which action by the nurse is appropriate?

- A.** Place sterile items on the outer 1-inch edge because the drape was just opened
- B.** Touch the outer 1-inch edge with sterile gloves to move supplies inward
- C.** Keep supplies in the center of the field and avoid using the outer 1-inch edge as sterile
- D.** Fold the outer 1-inch edge under the drape so the entire top surface remains sterile

33. A practical nurse is caring for several clients on a medical unit. Which client should the nurse see first to initiate isolation precautions?

- A.** A client with localized cellulitis and no drainage who is receiving oral antibiotics
- B.** A client with a positive sputum culture for methicillin-sensitive *Staphylococcus aureus* and no cough
- C.** A client with fever, productive cough, weight loss, and a chest x-ray suspicious for pulmonary tuberculosis
- D.** A client with a healed surgical incision who reports mild itching around the scar

34. A practical nurse is preparing to change the dressing on a client's draining leg ulcer. The client has no isolation orders. The old dressing is saturated, and the nurse expects direct contact with wound drainage but does not expect splashing. Which action best follows standard precautions?

- A.** Put on clean gloves before removing the old dressing
- B.** Wear a sterile gown because any draining wound requires full barrier protection
- C.** Skip gloves because the client is not on contact precautions
- D.** Apply a surgical mask because wound drainage is always spread through the air

35. A nurse is reinforcing teaching with a client who is 20 weeks pregnant about minor discomforts of pregnancy. Which instruction should the nurse include to help decrease heartburn?

- A.** Lie down for 30 minutes after meals
- B.** Drink most fluids with meals instead of between meals
- C.** Eat small, frequent meals and avoid spicy or fatty foods
- D.** Wear a tight abdominal binder after eating

36. A client in labor at 39 weeks' gestation has been pushing for 20 minutes. The fetal heart rate tracing shows a baseline of 140/min with moderate variability. With each contraction, the fetal heart rate briefly increases to 155/min and then returns to baseline. How should the nurse interpret this finding?

- A.** It suggests fetal well-being
- B.** It indicates uteroplacental insufficiency
- C.** It reflects umbilical cord compression
- D.** It is a sign of fetal hypoxia

37. A nurse is reinforcing discharge teaching for a client who is 2 days postpartum after an uncomplicated vaginal birth and plans to breastfeed. Which instruction is most appropriate?

- A.** Wear a supportive bra continuously and expose nipples to air after feedings if they are sore
- B.** Wash the nipples with soap before and after each feeding to prevent infection
- C.** Limit fluid intake between feedings to reduce breast fullness
- D.** Apply a heating pad to the breasts for 30 minutes every 2 hours to prevent engorgement

38. A nurse is talking with the parent of an 18-month-old toddler. Which statement by the parent suggests the child's development is on target?

- A.** My child can hop on one foot for a few seconds.
- B.** My child uses a spoon but spills some food.
- C.** My child draws a person with three body parts.
- D.** My child ties shoes without help.

39. A practical nurse is gathering baseline physical data from a client who is being admitted for cellulitis of the left leg. Which assessment finding would be most useful to document as baseline peripheral vascular status for later comparison?

- A.** Pedal pulses are 2+ bilaterally, skin is warm, and capillary refill in both feet is less than 3 seconds
- B.** Client states lunch was eaten 2 hours before arrival
- C.** Bowel sounds are present in all four quadrants
- D.** Pupils are equal and reactive to light

40. A nurse is reviewing the immunization record of a 4-year-old child who is behind on vaccines. The child's parent asks whether the child must restart the vaccine series because several doses were missed over the past 2 years. Which response by the nurse is best?

- A.** The entire series must be restarted if more than 12 months have passed since the last dose.
- B.** The child will need antibody testing before any additional vaccines can be given.
- C.** The child can continue the immunization schedule without restarting the series.

D. Only live vaccines can be given now, and inactivated vaccines should be delayed.

41. A nurse is collecting data from the parent of a 10-month-old infant during a health maintenance visit. Which parent statement describes an expected developmental finding?

- A.** My baby says 3-word sentences when asking for toys.
- B.** My baby pulls to a standing position while holding onto the couch.
- C.** My baby uses a spoon independently at each meal.
- D.** My baby rides a tricycle in the driveway with supervision.

42. A nurse is reviewing home oxygen safety with a client who speaks English clearly and reads without difficulty. The client avoids looking at the nurse, grips the side rails, and says, "I can't think about this right now. My husband was just taken back to surgery." Which action by the nurse is best to reduce the communication barrier?

- A.** Continue the teaching now so the client receives all required information before discharge
- B.** Speak more slowly and repeat each safety rule twice
- C.** Acknowledge the client's stress and postpone teaching until the client is better able to focus
- D.** Hand the client a printed oxygen safety sheet to read independently

43. A nurse is teaching a client how to change the dressing on a surgical incision at home. After the nurse explains the steps, the client smiles, nods, and says, "Yes, I understand." Which action by the nurse is best to evaluate whether a learning barrier is still present?

- A.** Ask the client to repeat the instructions using the nurse's exact words
- B.** Provide a detailed written handout for the client to review later
- C.** Have the client demonstrate the dressing change while the nurse observes
- D.** Ask the client whether any family member can do the dressing change instead

44. A client who recently learned that chronic back pain will likely limit a return to construction work says to the practical nurse, "If I can't do my job, I don't know who I am anymore." Which response by the nurse best provides emotional support?

- A.** You should focus on the treatments that are available instead of thinking about work right now.
- B.** At least you can still look for a less physical job once your pain is better controlled.
- C.** It sounds as if this loss is affecting both your income and your sense of identity.

D. Many people adjust to career changes over time, so try not to worry too much yet.

45. A client with newly diagnosed ulcerative colitis tells the nurse, "Every time my symptoms flare up, I cancel plans, stop eating with my family, and stay in my room so no one sees me." Which response by the nurse is most appropriate to promote effective coping?

A. "You should try not to think about your illness so much when symptoms get worse."

B. "What situations during a flare feel hardest for you, and what has helped you manage stress in the past?"

C. "It is best to avoid other people until your symptoms are completely under control."

D. "If you keep a positive attitude, these feelings will probably go away on their own."

46. A nurse in a home health setting is visiting a client whose adult son is present in the apartment. During the visit, the son begins speaking loudly, blocks the doorway, and says, "You're not taking her anywhere until I get some answers." The client's hands are shaking, and the son is pacing and glaring at the nurse. Which action should the nurse take first?

A. Calmly maintain personal space and use a clear, nonthreatening voice to de-escalate the situation

B. Tell the son that his behavior is inappropriate and must stop immediately

C. Move closer to the client to complete a rapid physical assessment

D. Attempt to leave by stepping past the son and walking to the doorway

47. A client newly admitted for alcohol withdrawal says to the nurse, "Everyone keeps asking me questions, but none of this matters. I already messed everything up with my family." Which response by the nurse is most therapeutic?

A. "You need to focus on getting better right now instead of dwelling on the past."

B. "Why do you think your family reacted so strongly?"

C. "It sounds like you're feeling overwhelmed and guilty about what has happened."

D. "If you follow treatment, your family will probably forgive you."

48. A nurse is speaking with a client whose blood glucose check is delayed because the unit is short-staffed. The client suddenly stands up, knocks a water cup onto the floor, and says in a loud voice, "This is ridiculous. Nobody here cares if something happens to me." The client is breathing quickly and glaring at the nurse but is not advancing toward anyone. Which response by the nurse is most appropriate?

A. "You need to lower your voice right now or I will have to call security."

B. "I can see that you are very frustrated. Let's move to a quieter area so we can talk and I can stay with you."

C. "There is no reason to act like this. Your blood glucose will be checked when staff are available."

D. "If you sit back down and calm yourself, I will come back when you are ready to speak respectfully."

49. A client with moderate dementia becomes upset when asked to brush their teeth and says, "I already did that! Leave me alone." The practical nurse sees that the toothbrush is dry and the sink has not been used. Which action is most appropriate?

A. Tell the client firmly that the teeth must be brushed now to prevent infection

B. Argue briefly with the client to remind them that they have not brushed their teeth yet

C. Offer the toothbrush, give one simple step at a time, and provide calm praise during the task

D. Skip oral care for the day to avoid increasing the client's frustration

50. A client who is caring for two young children and working full time tells the nurse, "By the end of the week, I feel overwhelmed because every problem seems equally urgent." Which strategy should the nurse recommend first to improve the client's stress management?

A. List all current stressors and rank them in order of priority

B. Avoid thinking about stressful situations until there is more free time

C. Take on extra tasks now to prevent disappointing others later

D. Handle the most unpleasant task first, even if it is the least important

51. A client with chronic kidney disease tells the practical nurse, "I use these beads to meditate several times a day. Since I was admitted, they were put in the drawer, and I feel more anxious without them." Which action by the nurse is most appropriate?

A. Tell the client that meditation items can be used only after discharge

B. Ask the client how the beads are used and help keep them within reach if they do not interfere with care

C. Remove the beads from the room because personal items increase infection risk

D. Explain that the health care team will decide whether the beads are spiritually appropriate

52. A nurse is caring for a client who avoids direct eye contact, gives very short answers, and becomes quiet when asked personal questions during the admission interview. Which action by the nurse is most appropriate?

- A.** Document that the client is uncooperative and end the interview
- B.** Explain that all clients are expected to answer questions directly
- C.** Recognize that communication style may be culturally influenced and continue the assessment respectfully
- D.** Ask the client's roommate to help interpret the client's behavior

53. A nurse is obtaining a history from a client who says, "I am a transgender man, and I have been skipping appointments because I worry staff will focus on my identity instead of why I came in." Which nursing response is best?

- A.** Your gender identity is not relevant here, so let's just talk about your current symptoms.
- B.** Can you tell me what has happened in past visits that made you feel that way, so we can plan care in a way that feels respectful and safe for you?
- C.** Most staff members are professional, so you should try not to assume this visit will go badly.
- D.** If you bring a family member with you next time, the staff may treat you differently.

54. A client with new low vision tells the nurse, "At supper, I can't tell who's waving at me from across the room, so I stopped going. I'd rather stay in my room than ignore people by accident." Which nursing response is best?

- A.** You should stay in your room until you feel more confident.
- B.** Ask your provider whether sedative medication would help before meals.
- C.** It sounds upsetting to feel cut off from others. Let's talk about ways to help you join meals more comfortably.
- D.** If people know about your vision problem, they will understand, so this should not bother you.

55. A client with terminal liver disease who is receiving end-of-life care tells the nurse, "I want my roommate from the shelter to visit, but my family says only relatives should be here now." Which nursing action is most appropriate?

- A.** Tell the family that visitation decisions must be made only by the healthcare team
- B.** Ask the client whom they want included in visits and support that preference as able
- C.** Explain to the client that family members have the right to limit visitors at the end of life

D. Delay the visit until the family agrees so conflict can be avoided

56. A nurse is caring for a client who receives intermittent enteral feedings through a mature gastrostomy tube. Before starting the feeding, the nurse checks placement and finds the external tube marking matches the documented length from the previous shift. Which additional finding best supports that the tube remains in the stomach?

- A.** The aspirated contents have a pH of 3
- B.** The client reports mild hunger before the feeding
- C.** The stoma dressing is dry and intact
- D.** The tube flushes easily with 30 mL of water

57. A nurse is assisting a client who has right-sided weakness to transfer from the bed to a chair using a transfer belt. Which action is most appropriate during the transfer?

- A.** Place the chair on the client's strong side and have the client push off the bed with the left hand
- B.** Stand behind the client and lift under the client's arms while pivoting to the chair
- C.** Place the chair at the foot of the bed and ask the client to walk forward to it without turning
- D.** Have the client hold the nurse's neck for support while standing and pivoting

58. A nurse is reinforcing dietary teaching for a client who is recovering from extensive burns. Which meal selection should the nurse recommend to support healing?

- A.** Apple juice, gelatin, and dry toast
- B.** Grilled chicken, scrambled eggs, and milk
- C.** Green salad, raw carrots, and diet soda
- D.** Plain rice, steamed apples, and tea

59. A client with acute pancreatitis reports constant abdominal pain and says, "Please don't touch my stomach any more than necessary." Which nonpharmacologic nursing intervention is most appropriate to help reduce the client's pain?

- A.** Assist the client into a side-lying position with knees flexed
- B.** Encourage the client to walk in the hallway every 2 hours
- C.** Apply a heating pad directly over the abdomen
- D.** Offer a large meal to distract the client from the pain

60. A nurse is caring for a client with severe kyphosis who wants to continue eating meals independently. The client has difficulty bending forward enough to reach food on a standard overbed table. Which intervention is most appropriate to support the client's independence with this activity of daily living?

- A.** Feed the client each meal to prevent frustration
- B.** Place the meal tray on a pillow in the client's lap while the client is seated upright
- C.** Encourage the client to lean forward as far as possible during meals
- D.** Have the client remain in a side-lying position while eating

61. A nurse is reinforcing teaching for a client who has a flaccid neurogenic bladder after a sacral spinal cord injury. Which instruction should the nurse include to help reduce urinary retention?

- A.** Void only when the urge to urinate is strong
- B.** Drink most fluids at bedtime to promote nighttime emptying
- C.** Perform clean intermittent catheterization at scheduled intervals
- D.** Limit toileting attempts to once every 8 hours

62. A nurse is caring for a client who reports difficulty falling asleep each night. Which snack should the nurse suggest at bedtime to help promote sleep?

- A.** Cheese and whole-grain crackers
- B.** Chocolate pudding
- C.** Potato chips and salsa
- D.** A large glass of cola

63. A practical nurse is caring for a client with reduced sensation in the lower legs from spinal cord injury. During evening care, the client asks for the bedcovers to be tucked in tightly because the room feels cool. Which action by the nurse is most appropriate?

- A.** Tuck the blankets in snugly to help the client stay warm and comfortable
- B.** Place a hot water bottle under the blankets near the client's feet
- C.** Loosen the bedding over the legs and feet to reduce pressure on areas with decreased sensation
- D.** Massage the client's lower legs briskly before tucking in the blankets

64. A client is on strict intake and output. During the evening shift, the client drinks 180 mL of broth, 4 oz of cranberry juice, and 120 mL of oral nutritional supplement. The client also receives 500 mL of IV fluids. Which total intake should the practical nurse document?

- A.** 740 mL
- B.** 800 mL

C. 920 mL

D. 980 mL

65. A practical nurse is caring for a client who has been on prolonged bed rest after abdominal surgery. Which action is most appropriate to help prevent hypostatic pneumonia related to immobility?

- A.** Encourage the client to use an incentive spirometer and cough regularly
- B.** Limit fluid intake during the day to reduce respiratory secretions
- C.** Keep the client in a flat supine position as much as possible
- D.** Massage the client's lower legs every shift

66. A nurse is preparing to administer a scheduled oral medication to a client. After verifying the prescription and checking the MAR, the nurse scans the client's wristband and receives a barcode alert that the medication does not match the client. The package label and MAR appear to show the same drug and dose. Which action is most appropriate?

- A.** Override the alert and give the medication because the label matches the MAR
- B.** Ask another nurse to administer the medication instead
- C.** Stop and resolve the discrepancy before administering the medication
- D.** Give the medication now and complete an incident report afterward if needed

67. A practical nurse is completing medication reconciliation for a client being discharged after treatment for heart failure. The discharge paperwork lists the client's home medications and new prescriptions. The home list still includes ibuprofen 400 mg PO every 6 hours as needed for pain, but the provider's discharge instructions say to avoid NSAIDs and use acetaminophen instead. Which action by the nurse is best?

- A.** Tell the client to follow the new prescription list and ignore the home medication list
- B.** Remove ibuprofen from the reconciled discharge medication list and clarify the final instructions with the provider if needed before teaching the client
- C.** Leave ibuprofen on the list because it is an over-the-counter medication
- D.** Ask the client which pain medicine works better and document the client's preference

68. A nurse is infusing 960 mL of IV fluid over 12 hours using an infusion pump. Six hours after the infusion begins, the nurse notes that 520 mL remains in the bag. To deliver the full prescribed amount on time, what rate should the nurse set for the remaining infusion?

- A.** 70 mL/hr

- B. 73 mL/hr
- C. 80 mL/hr
- D. 87 mL/hr

69. A practical nurse removes a prescribed controlled opioid from the automated dispensing system for Client A. Before the nurse reaches Client A's room, Client B urgently needs assistance after a fall in the hallway. What should the nurse do with the controlled medication?

- A. Place the medication in the uniform pocket and assist Client B immediately
- B. Set the medication on the medication cart and return for it after helping Client B
- C. Return or secure the medication according to facility procedure before assisting Client B
- D. Ask Client A's family member to hold the medication until the nurse returns

70. A nurse is preparing to apply a transdermal fentanyl patch for a client with chronic cancer pain who has been taking scheduled oral opioids. Which action should the nurse take?

- A. Apply the patch to an area covered with lotion to improve absorption
- B. Place the patch on intact skin and rotate application sites with each new patch
- C. Cut the patch in half if the prescribed dose needs to be reduced slightly
- D. Cover the patch with a heating pad for the first several hours to improve pain relief

71. A nurse is preparing to administer the next dose of vancomycin IV to a client with a serious wound infection. Which pre-administration data is most important for the nurse to review?

- A. The client's most recent serum creatinine level
- B. The client's breakfast intake from this morning
- C. The client's last bowel movement
- D. The client's daily weight from admission

72. A client with overactive bladder began taking oxybutynin 3 weeks ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A. The client reports fewer episodes of urinary urgency and incontinence
- B. The client voids large amounts every hour
- C. The client reports a stronger urinary stream with less hesitancy
- D. The client has cloudy urine with a foul odor

73. A client is receiving the first unit of packed red blood cells. Fifteen minutes after the infusion begins, the client reports sudden shortness of breath and chest tightness. Which action should the nurse take first?

- A. Slow the infusion rate and continue to monitor the client
- B. Stop the transfusion and maintain IV access with normal saline
- C. Administer prescribed PRN acetaminophen for discomfort
- D. Document the findings and recheck vital signs in 30 minutes

74. A patient takes lithium for bipolar disorder and asks whether it is okay to start taking ibuprofen every day for chronic knee pain. Which response by the nurse is best?

- A. That is fine as long as the ibuprofen is taken with food.
- B. Ibuprofen can increase lithium levels, so the patient should check with the provider before taking it regularly.
- C. Ibuprofen will make lithium less effective, so the lithium dose should be doubled.
- D. There is no important interaction because one medication is for pain and the other is for mood.

75. A client with heart failure is prescribed spironolactone 25 mg by mouth daily. Before administering the scheduled dose, the nurse reviews the morning laboratory results and notes a serum potassium level of 5.9 mEq/L. Which action should the nurse take?

- A. Administer the medication and encourage the client to increase fluid intake
- B. Hold the medication and notify the provider of the potassium level
- C. Give the medication with food to reduce gastrointestinal upset
- D. Administer the medication and recheck the potassium level in 24 hours

76. A nurse is preparing discharge teaching for a client who has a new prescription for digoxin to treat heart failure. Which instruction should the nurse include?

- A. Take the medication with an antacid to prevent stomach upset.
- B. Check your pulse before taking the medication and call the provider if it is below the prescribed limit.
- C. Stop taking the medication as soon as your breathing improves.
- D. Increase your dose if you notice swelling in your feet at the end of the day.

77. A nurse is preparing to administer an IV medication from a multidose vial. While checking the vial, the nurse notes that it was first opened 35 days ago. The label states, "Discard 28 days after first puncture." The solution appears clear, and the printed manufacturer expiration date is 8 months away. Which action should the nurse take?

- A.** Administer the medication because the solution is clear and the manufacturer expiration date has not passed
- B.** Administer the medication if a second nurse verifies that the vial was stored correctly
- C.** Discard the vial and obtain a new one for administration
- D.** Use the vial for this dose only, then discard the remaining medication

78. A nurse is preparing to administer metoprolol 37.5 mg by mouth. The available tablets are labeled 25 mg each. How many tablets should the nurse administer?

- A.** 1 tablet
- B.** 1.5 tablets
- C.** 2 tablets
- D.** 2.5 tablets

79. A practical nurse is reinforcing teaching for a client who will use oxygen at home. Which client statement should the nurse identify as the greatest safety risk requiring immediate correction?

- A.** I will keep the oxygen tubing coiled neatly along the wall when I am not walking.
- B.** I will apply a water-based lubricant to my lips if my nose feels dry.
- C.** I will turn off the oxygen when I cook on my gas stove.
- D.** I will smoke on the porch instead of inside the house.

80. A nurse is collecting a urine specimen from a client for a urine pregnancy test. The client says she has already voided several times this morning. Which instruction should the nurse give to help obtain the most accurate specimen?

- A.** Collect the first small amount of urine that begins the stream
- B.** Drink 1 L of water, then provide a specimen as soon as possible
- C.** Provide a first-morning urine specimen if possible
- D.** Save all urine passed during the next 8 hours

81. A nurse is reviewing laboratory results for a client who is receiving vancomycin for a methicillin-resistant infection. The client reports decreased urine output since yesterday. Which laboratory finding should the nurse report to the provider first?

- A.** Platelet count 180,000/mm³
- B.** Hemoglobin 13.2 g/dL
- C.** Serum creatinine 2.1 mg/dL

D. Sodium 136 mEq/L

82. A client who is 30 weeks pregnant comes to the clinic and says, "My hands and face look puffy, and for the last 2 days I've had a bad headache." Which additional finding should the nurse identify as the highest priority to report immediately?

- A.** A blood pressure of 164/106 mm Hg
- B.** Urinary frequency without dysuria
- C.** Dependent ankle edema at the end of the day
- D.** Intermittent low back discomfort after standing

83. A nurse is preparing to perform a bedside capillary blood glucose test for a client who is receiving oxygen and has a pulse oximetry reading of 99%. The nurse notices the glucometer quality-control check was not documented for the current shift. Which action is most appropriate before testing the client's blood?

- A.** Perform the finger-stick test now because the client's oxygenation is stable
- B.** Use the glucometer only if the previous shift documented a normal client result
- C.** Complete the required quality-control check according to facility policy before using the meter
- D.** Calibrate the meter after obtaining the client's blood sample to avoid delaying care

84. A client is scheduled for a transesophageal echocardiogram (TEE) in 1 hour. Which finding should the nurse report to the provider before the procedure?

- A.** The client has been NPO since midnight
- B.** The client has a history of esophageal varices
- C.** The client signed the informed consent form
- D.** The client reports mild anxiety about gagging

85. A nurse is preparing to place ECG electrodes for continuous monitoring on a client who is diaphoretic. Which action is most appropriate to promote electrode adherence and obtain a clearer tracing?

- A.** Apply the electrodes over moist skin to improve conduction
- B.** Dry the skin thoroughly before applying the electrodes
- C.** Place the electrodes over bony prominences to reduce artifact
- D.** Use lotion on the skin before electrode placement

86. A nurse is preparing to obtain a venous blood specimen from an adult client. The nurse applies a tourniquet and begins assessing veins. After about 1 minute, the nurse still has not identified an acceptable site. Which action is most appropriate?

- A.** Leave the tourniquet in place while continuing to search for a vein

- B.** Ask the client to repeatedly clench and unclench the fist until a vein appears
- C.** Release the tourniquet, wait briefly, and then reapply it before reassessing
- D.** Proceed with venipuncture at the best visible site to avoid further delay

87. A practical nurse is caring for a client who is 12 hours postoperative after an open cholecystectomy. Which finding is most important to report to the RN immediately?

- A.** The client rates incisional pain as 7 out of 10 when coughing
- B.** The client uses the incentive spirometer only a few times each hour
- C.** The client has a temperature of 99.1°F (37.3°C)
- D.** The client has a rigid abdomen with increasing abdominal distention

88. A client has a chest tube to water-seal drainage after a lobectomy. While assisting the client to turn, the nurse sees that the drainage system has tipped over and the water-seal chamber is now empty. Which action should the nurse take first?

- A.** Clamp the chest tube close to the client's chest
- B.** Place the drainage system back upright and refill the water-seal chamber to the prescribed level
- C.** Raise the drainage system above the level of the client's chest
- D.** Strip the chest tube to remove any air in the tubing

89. A nurse is caring for a client 8 hours after a thyroidectomy. The client says, "My neck feels tight," and has a hoarse voice. Which focused assessment should the nurse perform first?

- A.** Inspect the neck dressing and area behind the neck for swelling or bleeding
- B.** Ask the client to rate the incisional pain on a 0 to 10 scale
- C.** Auscultate bowel sounds in all four quadrants
- D.** Assess hand grasps bilaterally

90. A client with myasthenia gravis received pyridostigmine 45 minutes ago. Which finding best indicates the medication is having the desired therapeutic effect?

- A.** The client swallows water without coughing
- B.** The client reports less urinary frequency
- C.** The client has a heart rate of 54/min
- D.** The client has pupils that are more constricted than before

91. A nurse is caring for a postoperative client whose core temperature has dropped to 35.9°C (96.6°F) in the recovery area. Which additional finding is most consistent with the body's attempt to conserve heat?

- A.** Peripheral vasoconstriction with cool, pale skin
- B.** Profuse diaphoresis
- C.** Decreased muscle activity
- D.** Peripheral vasodilation with warm extremities

92. A nurse is caring for a client with a chronic wound that is healing by secondary intention. Which finding best indicates that the wound is healing?

- A.** The wound edges are becoming widely separated
- B.** The wound bed is filling with pink-red tissue
- C.** The drainage has changed to thick green exudate
- D.** The skin around the wound is cool, pale, and taut

93. A nurse arrives at the scene of an adult visitor who has collapsed in the hospital lobby. The area is safe. The visitor is unresponsive, has no normal breathing, and has no pulse. An AED is available immediately. After the pads are applied, the AED announces, "No shock advised." Which action should the nurse take next?

- A.** Reassess the pulse for 30 seconds before deciding whether to continue
- B.** Begin chest compressions immediately and continue CPR
- C.** Remove the AED pads and provide rescue breaths only
- D.** Place the client in the recovery position and monitor closely

94. A client with left lower lobe pneumonia is receiving chest physiotherapy and postural drainage. To promote drainage from the affected lung segment, which position should the nurse place the client in?

- A.** Supine with the head of the bed elevated 45 degrees
- B.** Right side-lying with the foot of the bed elevated
- C.** Left side-lying with the head of the bed elevated
- D.** High Fowler's position leaning forward over a bedside table

95. A nurse is caring for a client with a tracheostomy who needs endotracheal suctioning. Which technique is most appropriate during the procedure?

- A.** Apply suction continuously while inserting the catheter into the tracheostomy tube
- B.** Insert the catheter without suction, then apply suction while withdrawing it with a rotating motion
- C.** Suction for 20 to 30 seconds at a time to ensure all secretions are removed
- D.** Instill sterile saline routinely before each suction pass to loosen secretions

96. A client receiving mechanical ventilation has a sudden increase in peak inspiratory pressure alarms. The nurse finds the client anxious, with inspiratory wheezes and diminished air movement bilaterally. The ventilator tubing is patent, and there are no visible secretions in the endotracheal tube. Which intervention should the nurse anticipate first?

- A.** Administer a prescribed bronchodilator
- B.** Increase the tidal volume setting
- C.** Deflate the endotracheal tube cuff
- D.** Lower the ventilator alarm limits

97. A client with heart failure is on continuous telemetry. The monitor alarm sounds, and the rhythm appears as a flat line in lead II. The client is awake in bed and talking with the nurse. Which action should the nurse take first?

- A.** Begin chest compressions immediately
- B.** Check the client for a pulse for 5 to 10 seconds
- C.** Assess the monitor leads and cable connections
- D.** Prepare for transcutaneous pacing

98. A nurse is reinforcing teaching for a client who has had a sigmoid colostomy for several months and wants to begin colostomy irrigation. Which instruction is most appropriate for the nurse to include?

- A.** Perform the irrigation at the same time each day to help establish a regular pattern
- B.** Use sterile technique for the entire procedure to prevent bowel infection
- C.** Irrigate only when the pouch is completely full of stool
- D.** Stop drinking fluids for 4 hours before irrigation to reduce cramping

99. A nurse is caring for a client who is 1 hour post-op after a tonsillectomy. Which finding should the nurse recognize as the most urgent change in condition?

- A.** Frequent swallowing and throat clearing
- B.** Reports throat pain rated 6 on a 0 to 10 scale
- C.** Refuses solid food at lunch
- D.** Has a temperature of 37.7°C (99.9°F)

100. A nurse is caring for a client who has a new ileostomy and has had high-output liquid drainage since surgery. Which assessment finding is most important for the nurse to report to the provider immediately?

- A.** Urine output of 20 mL/hr for the last 3 hours
- B.** Mild thirst and dry lips
- C.** Heart rate of 102/min after ambulating
- D.** Serum magnesium level of 1.9 mg/dL

Simulation Test 17 — Answer Key

1. C	26. C	51. B	76. B
2. A	27. B	52. C	77. C
3. C	28. B	53. B	78. B
4. B	29. C	54. C	79. D
5. C	30. B	55. B	80. C
6. B	31. D	56. A	81. C
7. A	32. C	57. A	82. A
8. B	33. C	58. B	83. C
9. C	34. A	59. A	84. B
10. B	35. C	60. B	85. B
11. C	36. A	61. C	86. C
12. B	37. A	62. A	87. D
13. C	38. B	63. C	88. B
14. B	39. A	64. C	89. A
15. C	40. C	65. A	90. A
16. C	41. B	66. C	91. A
17. B	42. C	67. B	92. B
18. C	43. C	68. D	93. B
19. C	44. C	69. C	94. B
20. B	45. B	70. B	95. B
21. C	46. A	71. A	96. A
22. B	47. C	72. A	97. C
23. C	48. B	73. B	98. A
24. C	49. C	74. B	99. A
25. A	50. A	75. B	100. A

Simulation Test 18

100 questions

1. Near the end of a shift, a practical nurse has several tasks pending. Which action is the highest priority?
 - A. Document intake and output totals for a client who is being discharged in 2 hours
 - B. Check on a client who received insulin 20 minutes ago and is now reported to be sweaty and shaky
 - C. Call central supply to request additional IV tubing for the next shift
 - D. Teach a client how to use an incentive spirometer before dinner
2. A practical nurse in a skilled nursing facility is completing a noon medication pass when four interruptions occur. Which situation should the nurse address first?
 - A. A client newly started on an antibiotic reports an itchy rash on the chest and neck
 - B. A client with chronic arthritis asks whether the scheduled acetaminophen will be given soon
 - C. A family member wants to discuss tomorrow's care conference time
 - D. An assistive person reports that a client who uses a walker needs help to the bathroom
3. A practical nurse delegates routine blood pressure measurements to an assistive personnel (AP) in a long-term care facility. The AP reports, "Ms. Lee's blood pressure is 178/96 mm Hg, and she says she has a bad headache." Which action by the practical nurse is best?
 - A. Tell the AP to recheck the blood pressure in 4 hours during the next routine vital-sign round
 - B. Ask the AP whether the client took her antihypertensive medication this morning
 - C. Immediately assess the client and verify the finding before taking further action
 - D. Document the report and notify the provider at the end of the shift if the pressure stays elevated
4. A practical nurse is making assignments on a rehabilitation unit. Which task is most appropriate to delegate to an assistive personnel (AP)?
 - A. Observe a client who started a new opioid this morning for increasing sedation
 - B. Help a client with chronic right-sided weakness get dressed using the client's usual adaptive equipment
 - C. Teach a client how to use a walker before discharge later that day
 - D. Decide whether a client's reddened heel needs protective dressings
5. A nurse is caring for an adult client who is alert, oriented, and receiving palliative care for advanced heart failure. The client tells the nurse, "I do not want my prognosis discussed when my cousin visits. I only want my care decisions talked about when my sister is here with me." Later that day, the cousin arrives alone and says, "I am family, so tell me what the provider said about how much time is left." What is the nurse's best response?
 - A. Discuss the prognosis because the cousin is a family member involved in support
 - B. Ask the cousin to wait while the nurse checks the chart for the latest provider note
 - C. Tell the cousin that the nurse cannot share that information without the client's permission
 - D. Give a brief update without details because the cousin requested only general information
6. A practical nurse notices that another staff member has left a workstation open to a client's electronic health record in an empty room. The staff member is not nearby. Which action by the nurse is most appropriate?
 - A. Review the chart for important updates before logging off
 - B. Log off the workstation and report the privacy risk according to facility policy
 - C. Leave the workstation alone because it belongs to another staff member
 - D. Print the screen information so the staff member does not lose documentation
7. A nurse is caring for an adult client who is alert and has decision-making capacity. The client says, "I want to complete an advance directive, but I do not want anyone to think I am refusing treatment today." Which response by the nurse is best?
 - A. Completing an advance directive means the health care team should limit invasive treatment from this point forward.
 - B. An advance directive lets you state your wishes for future care if you cannot speak for yourself later.
 - C. Once you sign an advance directive, your provider must discuss all treatment decisions only with your chosen surrogate.
 - D. An advance directive is used only for clients who have a terminal illness or are receiving hospice care.

8. A nurse is preparing an alert adult client for an elective laparoscopic cholecystectomy. The provider explained the procedure earlier, and the client signed the consent form. Just before transport to surgery, the client says, "I know I agreed to have my gallbladder removed, but I do not remember anyone telling me what could happen if I choose not to have surgery yet." Which action by the nurse is most appropriate?

- A.** Explain the likely consequences of delaying treatment so the client can make an informed decision
- B.** Have the client sign a new consent form after reviewing the preoperative instructions
- C.** Notify the provider that the client needs more information about alternatives, including no treatment
- D.** Reassure the client that the surgeon would not recommend surgery unless it was the best option

9. A client is being discharged after an exacerbation of chronic obstructive pulmonary disease and will use oxygen at home for the first time. The client demonstrates correct inhaler use but says, "I smoke only on the back porch, so the oxygen tank in the living room should be fine." Which nursing action is most appropriate before discharge?

- A.** Reinforce that oxygen can be used safely as long as the client smokes outside
- B.** Ask the provider to delay discharge until the client agrees to stop smoking permanently
- C.** Provide and verify understanding of home oxygen safety teaching, including no smoking near oxygen equipment
- D.** Arrange a referral to pulmonary rehabilitation instead of reviewing oxygen safety

10. A practical nurse is helping with a quality improvement project on a unit that wants to reduce blood glucose treatment delays for clients with hypoglycemia. Before testing any changes, the team wants to understand the current process from the time a low glucose value is identified until treatment is given. Which action is most appropriate first?

- A.** Create a flowchart of the current steps and where delays occur
- B.** Require all staff to repeat training on hypoglycemia treatment
- C.** Compare the unit's hypoglycemia rate with that of another facility
- D.** Begin stocking extra juice in each client room immediately

11. An outpatient surgery center is trying to improve the completeness of discharge teaching after sedation. The team introduces a standardized discharge checklist. After 4 weeks, chart review shows the checklist was completed in 98% of eligible discharges, but follow-up calls the next day show many clients still cannot correctly describe warning signs that require medical attention. Which action is best for the quality improvement team to take next?

- A.** Conclude that the project was successful because staff used the checklist consistently
- B.** Stop measuring client understanding and track only checklist completion
- C.** Study how the teaching is being delivered and revise the intervention to better assess teach-back
- D.** Expand the checklist to all departments before making any further changes

12. A practical nurse is caring for a client with chronic obstructive pulmonary disease who is expected to be discharged in 2 days with home oxygen. During teaching, the client says, "I still smoke a few cigarettes a day, but I won't do it near the tank." Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the statement in the chart and continue routine care
- B.** Tell the client to stop smoking and recheck understanding at the next medication pass
- C.** Notify the registered nurse or provider and coordinate with case management or respiratory therapy because the statement affects discharge safety planning
- D.** Wait until discharge instructions are printed so all teaching can be reviewed at one time

13. A practical nurse is caring for a client with a new below-knee amputation. The client is scheduled to meet with physical therapy this afternoon. During morning care, the client says, "There is no ramp at home, and my bedroom is upstairs." The nurse also notes the client becomes tearful, refuses to practice transferring, and says, "I don't think I can manage at home." Which information is most important for the nurse to communicate to the interdisciplinary team first?

- A.** The client has a physical therapy session scheduled this afternoon
- B.** The client has environmental barriers at home and is expressing inability to manage after discharge
- C.** The client became tearful during morning care
- D.** The client refused transfer practice one time this morning

14. A practical nurse is caring for a client who has a signed durable power of attorney for health care naming the client's sister as decision-maker if the client becomes unable to make decisions. Today the client is awake, alert, and clearly states, "I want to speak for myself, and I do not want my sister making decisions right now." Which action by the nurse is most appropriate?

- A.** Follow the sister's decisions because the document is already signed
- B.** Ask the provider to decide whether the client may speak for himself
- C.** Recognize the client's right to make current decisions while the client has decision-making capacity
- D.** Delay all care until the ethics committee reviews the situation

15. A practical nurse is caring for a client who had a witnessed fall while trying to get to the bathroom alone. The client reports mild hip pain but no dizziness. After helping the client back to bed and obtaining vital signs, the nurse notifies the RN. The RN is temporarily off the unit with another unstable client. The charge nurse tells the practical nurse, "The provider is tied up. Just document that the client was found on the floor so it doesn't look as serious." Which action by the practical nurse is most appropriate?

- A.** Document exactly what was observed, complete required reporting per policy, and avoid altering the facts
- B.** Wait to chart anything until the RN returns and decides what should be written
- C.** Document the client's pain only and leave out the fall until the provider has assessed the client
- D.** Change the note to say the client slid to the floor because that wording is less likely to create legal problems

16. A practical nurse in a clinic notices that a 10-year-old child has a cigarette-shaped burn on the forearm and several fading bruises on the back. The caregiver states the child is "always getting hurt," but the injury pattern seems inconsistent. Which action by the nurse is most appropriate?

- A.** Wait to report until the nurse can confirm abuse with photographs and witness statements
- B.** Ask the caregiver to explain the injuries again and decide about reporting after the visit
- C.** Report the suspicion according to facility policy and document objective findings
- D.** Tell the child that the nurse will keep the information secret unless the child agrees to a report

17. A practical nurse in a prenatal clinic is caring for a 15-year-old client who is 10 weeks pregnant. The client says she is living with her 24-year-old boyfriend and adds, "Please don't tell anyone. I don't want this reported." What is the nurse's best action?

- A.** Keep the information confidential because the client requested privacy
- B.** Encourage the client to tell a parent first, and wait to act unless the client agrees
- C.** Follow facility policy for mandatory reporting of suspected abuse involving a minor and document the client's statement objectively
- D.** Question the boyfriend directly before deciding whether a report is needed

18. A practical nurse is coordinating care with an assistive personnel (AP) in an outpatient surgery center. Which task is most appropriate for the nurse to assign to the AP?

- A.** Reinforce discharge teaching about signs of wound infection
- B.** Obtain and document a routine set of vital signs for a client preparing for discharge
- C.** Assess a client's readiness to go home after sedation
- D.** Decide whether a client's postoperative nausea requires the provider to be notified

19. On a busy pediatric unit, a practical nurse asks an assistive personnel (AP) to collect a clean-catch urine specimen from a school-age child with a suspected urinary tract infection. Which instruction by the nurse is most important to include?

- A.** "If the specimen looks cloudy, send it right away without labeling it so the lab can test it faster."
- B.** "Use the first urine that comes out, because it is less likely to be contaminated."
- C.** "Clean the area as directed, collect the midstream sample in the sterile cup, and tell me if you have any difficulty getting the specimen."
- D.** "Ask the child's parent to collect the specimen alone so the child feels less embarrassed."

20. A practical nurse in a community health clinic is reinforcing discharge instructions for a client with newly prescribed heart medication. The client says, "I can pick up the medicine, but I will probably have to skip my follow-up visit because I take care of my mother during the day and cannot leave her alone." Which nursing action best promotes equitable access to care?

- A.** Tell the client that the follow-up visit is important and should be kept as scheduled
- B.** Ask the client what caregiving barriers exist and help explore options such as alternate appointment times or community respite resources

C. Document that the client is noncompliant with the recommended plan of care

D. Advise the client to ask a neighbor to stay with the mother during appointments

21. A practical nurse in an outpatient nephrology office helps review quality-improvement data. Which finding most strongly suggests an inequitable system barrier to care that should be addressed at the clinic level?

A. Clients ages 18 to 30 ask more medication questions than older adults during teaching sessions

B. One provider has a higher rate of canceled appointments during a month when the provider took vacation days

C. Clients who live in public housing complete far fewer post-hospital follow-up visits after the clinic switched all scheduling and reminders to an online portal

D. Several clients with new dialysis referrals say they feel anxious at their first specialist visit

22. A nurse in an outpatient laboratory is ready to label a blood specimen after venipuncture. The client is alert and states their full name correctly, but the date of birth on the printed specimen label does not match the date of birth on the client's identification band. Which action should the nurse take?

A. Label the specimen using the client's stated name because the client is alert and oriented

B. Ask another nurse to confirm the client's identity and then send the specimen

C. Stop the process and resolve the identifier discrepancy before labeling or sending the specimen

D. Use the identification band as the only source and ignore the printed label error

23. A nurse is preparing to use a bedside commode for a client who is weak and unsteady. Before assisting the client onto the commode, the nurse notices the rubber tip is missing from one commode leg. Which action should the nurse take?

A. Place a folded towel under the uneven leg and assist the client carefully

B. Use the commode as long as another staff member helps steady it

C. Remove the commode from use and obtain equipment in safe working condition

D. Ask the client to bear more weight on the stronger leg during the transfer

24. A nurse is preparing to administer the first dose of a prescribed stool softener in liquid form. During the medication check, the client says, "I get swelling and trouble breathing if I eat strawberries." The nurse notices the medication contains strawberry flavoring, and no food allergies are documented in the record. Which action should the nurse take first?

A. Hold the medication and clarify the allergy with the provider and pharmacist before administration

B. Administer the medication because strawberry flavoring is not the same as fresh fruit

C. Give the medication with food to reduce the chance of a reaction

D. Document the allergy after administering the first dose so treatment is not delayed

25. A practical nurse in an outpatient dialysis center hears the building fire alarm. In the treatment area, one client is connected to dialysis, another client is standing near the exit with a walker, a wastebasket in a corner is smoldering, and visitors are beginning to crowd the hallway. Which action should the nurse take first?

A. Escort the visitors out of the hallway to reduce crowding

B. Move the client standing near the exit to the assembly area outside

C. Rescue clients from immediate danger, beginning with those closest to the smoldering wastebasket

D. Use a fire extinguisher on the smoldering wastebasket before moving any clients

26. A hospitalized client with moderate dementia becomes restless during the evening and repeatedly tries to walk into other clients' rooms. The client is not aggressive and usually calms when a staff member walks with the client and redirects attention to folding washcloths. The unit is short-staffed, and another nurse suggests a geri chair with a locked tray so the client will stay seated at the desk. Which response by the practical nurse best reflects least-restrictive restraint principles?

A. Use the chair with the locked tray because wandering can quickly become unsafe

B. Ask the provider for a PRN sedative so the client will rest through the evening

C. Continue redirection and supervised activity because an effective less-restrictive measure is working

D. Apply bilateral wrist restraints during peak restlessness to prevent the client from leaving the room

27. A nurse preparing a client's noon enteral feeding compares the formula bag with the prescription and notices the bag was mixed for a different client with a renal formula instead of the standard formula ordered. The feeding has not been started, and the client has no adverse effects. Which action is most appropriate?

- A.** Hang the prescribed formula instead and continue care without further follow-up because no harm occurred
- B.** Discard the incorrect formula bag, obtain the correct feeding, and report the near miss according to facility policy
- C.** Start the incorrect formula temporarily so the client does not miss the scheduled feeding, then notify the provider
- D.** Ask another nurse whether the renal formula would still be acceptable for this client before deciding what to do

28. A nurse is preparing to move a client from bed to chair with a sit-to-stand lift. Which finding indicates that the nurse should use a different transfer method?

- A.** The client can follow simple commands
- B.** The client can bear weight briefly on both legs
- C.** The client has severe pain with knee flexion
- D.** The client is unable to maintain head and trunk control

29. A practical nurse is preparing to administer insulin with a pen device to a client in a long-term care setting. Another nurse says, "The needle is changed between clients, so the same insulin pen can be used for more than one person." Which response by the practical nurse is most appropriate?

- A.** Reuse of the pen is acceptable if the cartridge still contains enough insulin and the rubber stopper is disinfected
- B.** Reuse of the pen is acceptable only for clients who do not have bloodborne infections
- C.** Each insulin pen must be used for only one client, even if a new needle is attached
- D.** The pen may be shared if it is stored in a medication cart drawer between uses

30. A nurse is preparing to assist with a sterile dressing change for a client with a draining abdominal wound. The client is not on isolation precautions, but the nurse expects wound drainage could splash during irrigation. Which personal protective equipment should the nurse use?

- A.** Sterile gloves only
- B.** Gloves and gown only
- C.** Gloves, gown, mask, and eye protection
- D.** Mask only

31. A nurse wearing a gown, gloves, surgical mask, and goggles cares for a client on droplet precautions. After leaving the room, the nurse realizes the goggles are still on. Which rationale best explains why this is a problem?

- A.** Eye protection should be removed with the other contaminated PPE to avoid carrying contaminated equipment into a clean area
- B.** Goggles are unnecessary for droplet precautions and should never have been worn
- C.** Goggles must always be removed before gloves so the nurse can see clearly during glove removal
- D.** Once outside the room, the goggles are considered clean because they were not touched during care

32. A practical nurse is preparing a sterile field for a central line dressing change. After opening the sterile drape correctly, the nurse notices a small tear near the center of the drape that was not visible on the outside of the package. Which action should the nurse take?

- A.** Cover the tear with a sterile towel and continue setting up the field
- B.** Use the drape if the tear does not touch the supplies
- C.** Discard the drape and obtain a new sterile drape
- D.** Avoid using the torn area and place supplies only around the edges

33. A practical nurse is preparing a semi-private room assignment for four newly admitted clients. Which client is the best candidate to room with another client rather than be placed in a private room for isolation purposes?

- A.** A client with respiratory syncytial virus requiring contact precautions
- B.** A client with neutropenia receiving protective precautions after chemotherapy
- C.** A client with a postoperative wound draining large amounts of purulent fluid
- D.** A client with acute viral hepatitis A who is incontinent of stool

34. A practical nurse is preparing to irrigate a client's chronic wound. The client has no isolation orders, but the nurse expects the irrigation fluid to splash while cleansing the wound. Which action best reflects standard precautions?

- A.** Wear gloves, a gown, and a mask with eye protection during the procedure
- B.** Wear sterile gloves only because the wound itself requires the highest level of protection
- C.** Wear gloves for the procedure and add a gown only if the client is known to have an infection
- D.** Use no personal protective equipment unless visible blood is present before irrigation begins

35. A nurse is reinforcing teaching with a client who is 27 weeks pregnant about self-care for ankle edema at the end of the day. Which instruction should the nurse include?

- A. Limit fluid intake after noon each day
- B. Rest on the left side with the legs elevated
- C. Apply a heating pad to both ankles for 30 minutes
- D. Remain standing for longer periods to improve circulation

36. A client in active labor at 39 weeks' gestation has been pushing for 1 hour with strong contractions every 2 to 3 minutes. The fetal head remains visible at the vaginal opening between contractions. Which finding should the nurse document?

- A. Station +1
- B. Crowning
- C. Engagement
- D. Breech presentation

37. A nurse is reinforcing discharge teaching for a client who is 2 days postpartum after an uncomplicated vaginal birth. Which statement by the client indicates correct understanding of expected postpartum changes?

- A. "My vaginal discharge should become bright red again after it starts getting lighter."
- B. "I may continue to have night sweats for a while as my body gets rid of extra fluid."
- C. "My temperature should stay a little elevated for the first 2 weeks after birth."
- D. "My uterus should no longer be felt in my abdomen by tomorrow."

38. A nurse is assessing a 2 1/2-year-old child during a wellness visit. Which behavior is most consistent with expected development for this age?

- A. Uses 2- to 3-word phrases and names familiar objects
- B. Hops on one foot and skips across the room
- C. Reads several simple words from a picture book
- D. Draws a person with six body parts

39. A practical nurse is reinforcing teaching with the parent of a 5-year-old child whose family moved to a new neighborhood 3 weeks ago. The parent says, "Since the move, my child wants the same bedtime routine every night and gets upset when anything changes." Which response by the nurse is most appropriate?

- A. This behavior suggests the child is not adjusting well and should be evaluated immediately.
- B. Wanting predictable routines is a common way for a young child to feel secure during a major change.
- C. Children this age usually adapt quickly, so the behavior is most likely attention seeking.

D. You should avoid routines for now so your child learns to adjust more quickly.

40. A nurse in an employee health clinic is planning seasonal influenza immunization for several staff members. Which staff member should the nurse identify as an appropriate candidate for the live attenuated intranasal influenza vaccine?

- A. A 27-year-old nurse who is 24 weeks pregnant
- B. A 33-year-old assistive personnel worker who has asthma and uses an albuterol inhaler
- C. A 22-year-old unit clerk who is healthy, not pregnant, and has no chronic illness
- D. A 41-year-old nurse who is receiving chemotherapy for lymphoma

41. A nurse is reinforcing teaching with the parent of a 15-month-old toddler. Which play activity is most appropriate for this child?

- A. Putting large blocks into a container and dumping them out
- B. Playing a board game that requires taking turns by rules
- C. Completing a 100-piece jigsaw puzzle independently
- D. Joining a competitive team sport with same-age peers

42. A nurse is reviewing discharge instructions with a client who says, "I forgot my glasses, and these papers are just a blur to me." The client speaks clearly, follows the conversation, and asks appropriate questions. Which nursing action is best to reduce this communication barrier?

- A. Read the instructions aloud and ask the client to sign the paperwork when finished
- B. Provide a magnifying device or large-print written instructions if available
- C. Ask a family member to listen carefully and explain the instructions later at home
- D. Delay all discharge teaching until the client can return for a follow-up visit

43. A nurse is preparing to teach a client about home exercises after a wrist fracture. The client says, "I can understand what you're saying, but these pain pills make me feel sleepy and foggy." Which action by the nurse is best?

- A. Continue the teaching now and leave written instructions at the bedside
- B. Ask a family member to learn the exercises instead of the client
- C. Delay the teaching until the client is more alert and able to concentrate
- D. Speak louder and repeat each instruction several times

44. A client with a recent spinal cord injury tells the practical nurse, "My friends keep visiting, but I don't want to talk to anyone. They all act awkward, and it just makes me feel worse." Which response by the nurse best provides emotional support?

- A.** You should try to be grateful that your friends still want to see you.
- B.** It sounds like these visits have been uncomfortable for you. Do you want to talk about what has been hardest?
- C.** Maybe you need more time alone until you feel ready to socialize again.
- D.** I can ask your friends to stop visiting so you will not have to deal with this.

45. A client with newly diagnosed type 2 diabetes tells the nurse, "I keep thinking about everything I'll have to change, so I just avoid checking my blood sugar because it makes me more anxious." Which nursing response is most appropriate to promote effective coping?

- A.** You need to check it as prescribed, even if it makes you uncomfortable.
- B.** Try not to think about diabetes so much, and the anxiety should lessen.
- C.** Let's focus on one step you feel ready to start with today, such as learning how to do one blood sugar check.
- D.** If the anxiety continues, you should let a family member manage your diabetes care for you.

46. A nurse on an inpatient unit is talking with a client who has been increasingly irritable throughout the day. The client is now pacing, staring at another client, and says, "He keeps laughing at me. I'm going to take care of him right now." Which action should the nurse take first?

- A.** Ask the client to explain why the other client is upsetting him
- B.** Move the client to a quieter area and speak with the client in a calm, clear manner
- C.** Tell the client that threatening another client is unacceptable behavior
- D.** Administer a PRN sedative medication immediately

47. A client who has been using methamphetamine heavily for several weeks is brought to the clinic by a family member 2 days after stopping use. The client is sleeping much more than usual, reports feeling "empty," and says, "Nothing seems worth it anymore." Which nursing action is the priority?

- A.** Encourage the client to increase activity to reduce fatigue
- B.** Assess the client for suicidal thoughts or plans
- C.** Teach the client that these symptoms usually resolve without treatment
- D.** Limit fluids until the client's sleep pattern returns to normal

48. A nurse is caring for a client in a clinic waiting area who becomes increasingly frustrated after learning that a prescription refill will take longer than expected. The client stands up, speaks loudly, and says, "This is unbelievable. Nobody knows what they're doing here." The client is not threatening anyone and is several feet away from the nurse. Which action by the nurse is most appropriate?

- A.** Tell the client that yelling is unacceptable and that security will be called if the behavior continues
- B.** Ask the client to sit down immediately so the nurse can explain the clinic policy
- C.** Speak calmly, acknowledge the client's frustration, and offer to move to a quieter area to talk
- D.** Ignore the behavior until the client stops raising the voice and then continue the refill discussion

49. A long-term care resident with moderate dementia begins wandering into other residents' rooms and opening dresser drawers. When staff redirect the resident, the resident says, "I'm looking for the bathroom," even though the bathroom is nearby. Which intervention is most appropriate?

- A.** Place a large, clearly visible sign with a picture on the bathroom door
- B.** Tell the resident that wandering into rooms is unacceptable behavior
- C.** Move the resident to a room at the end of the hallway for closer observation
- D.** Ask other residents to keep their doors closed at all times

50. A client tells the nurse, "I have been under a lot of stress at work, so I decided the best thing to do is stop volunteering for extra projects and set aside 20 minutes each evening to relax without interruptions." Which response by the nurse is most appropriate?

- A.** "That is a good plan because setting limits and scheduling regular relaxation can help reduce stress."
- B.** "You should avoid relaxation time until all of your work responsibilities are caught up."
- C.** "It would be better to stay as busy as possible so you do not think about stress."
- D.** "You should wait to see whether your stress gets worse before making any lifestyle changes."

51. During admission, a client tells the practical nurse, "Being able to read from my holy book each morning helps me stay calm. My vision is poor, and I forgot my large-print copy at home." Which nursing action is most appropriate?

- A.** Tell the client that personal religious materials are not part of the care plan
- B.** Ask the client whether an audio version, family-provided copy, or other preferred alternative would be helpful

- C.** Offer to choose a different spiritual reading that is easier for the nurse to obtain
- D.** Explain that the client should focus on medical treatment rather than spiritual practices during hospitalization

52. A nurse is preparing discharge instructions for a client who had a myocardial infarction. The client says, "Please explain the plan to my wife first. In our family, she manages health decisions and medicines." The client is alert, oriented, and states, "I want to be part of the discussion too." Which action by the nurse is most appropriate?

- A.** Provide all instructions only to the wife because the client identified her as the decision-maker
- B.** Ask the client for permission to include the wife, then review the instructions with both of them together
- C.** Tell the client that discharge teaching must be given only to the client, not to family members
- D.** Delay all teaching until the provider can determine who should receive the information

53. A client who identifies as a transgender man is being admitted for cellulitis. During the interview, the client says, "I bind my chest every day, even when I'm sick, because I panic if I don't." Which response by the nurse is best?

- A.** "You should stop doing that because it is unhealthy."
- B.** "Tell me more about how binding affects your comfort and whether you are having any trouble breathing or skin irritation."
- C.** "We can discuss that later after the admission questions are finished."
- D.** "Most clients find it easier to avoid binding while hospitalized."

54. A client with newly diagnosed auditory hallucinations says to the nurse, "The voices keep telling me the staff are trying to poison me. I know you probably think I'm crazy, so I'd rather stay away from everyone." Which response by the nurse is best?

- A.** "Those voices are not real, so try to ignore them and come sit with the group."
- B.** "I understand this feels frightening. I do not hear the voices, but I want to help you stay safe and talk about what you are experiencing."
- C.** "Why do you think the voices are saying that about the staff?"
- D.** "If you know the voices are not true, there is no reason to avoid other people."

55. A client who is receiving end-of-life care says quietly to the nurse, "I don't need advice right now. I just keep wondering whether anyone will remember me after I'm gone." Which response by the nurse is most therapeutic?

- A.** "Try not to think that way. Your family will always remember you."
- B.** "Many people feel this way at the end of life. Tell me more about what matters most to you."
- C.** "You should make a list of the things you want your family to know before it is too late."
- D.** "Why would you think no one will remember you?"

56. A nurse is caring for a client with a mature gastrostomy tube who receives routine enteral feedings. While providing site care, the nurse notes a small amount of dry crusting around the stoma. The client denies pain, and the surrounding skin is intact without swelling or purulent drainage. Which action should the nurse take?

- A.** Clean the site gently with mild soap and water, dry thoroughly, and apply a clean dressing only if needed
- B.** Scrub the stoma with full-strength hydrogen peroxide until all crusting is removed
- C.** Hold the next feeding and notify the provider that the tube may be infected
- D.** Apply an occlusive dressing tightly around the tube to prevent air from reaching the site

57. A nurse is reinforcing teaching for a client who is learning to walk with a walker after hip surgery. Which client action indicates correct walker use?

- A.** Lifts the walker, moves it forward, then steps into the walker with the affected leg first
- B.** Advances the unaffected leg first, then slides the walker forward
- C.** Keeps the walker far ahead of the body to allow longer steps
- D.** Pulls up on the walker handles while rising from a chair

58. A nurse is reinforcing teaching for a client who takes ferrous sulfate for iron-deficiency anemia. Which client statement indicates an understanding of how to improve iron absorption?

- A.** I will take the pill with milk at breakfast.
- B.** I will take the pill with orange juice.
- C.** I will take the pill with an antacid after meals.
- D.** I will take the pill with tea before bedtime.

59. A client with rheumatoid arthritis reports hand pain and morning stiffness and says, "I want to be able to button my shirt with less pain." Which nonpharmacologic intervention should the nurse recommend first?

- A.** Apply warm compresses to the hands before activity

- B.** Massage the swollen finger joints vigorously
 - C.** Keep the hands immobile in one position for several hours
 - D.** Use a cold pack on the hands immediately before dressing
- 60.** A nurse is caring for a client with rheumatoid arthritis who wants to dress independently but has painful stiffness in the fingers each morning. Which nursing intervention is most appropriate to support the client's ability to perform this activity of daily living?
- A.** Encourage the client to dress immediately after waking to finish before fatigue develops
 - B.** Provide clothing with Velcro fasteners and allow extra time for dressing
 - C.** Dress the client each morning to prevent pain and conserve joint function
 - D.** Ask the client to practice buttoning clothing repeatedly until finger movement improves
- 61.** A nurse is caring for a client who has fecal incontinence after a spinal cord injury. The client is starting a bowel program. Which intervention should the nurse implement to help establish predictable bowel elimination?
- A.** Schedule digital stimulation at the same time each day, preferably after a meal
 - B.** Restrict fluid intake in the evening to reduce stool frequency
 - C.** Perform the bowel program at different times based on the client's daily schedule
 - D.** Keep the client on bed rest during bowel evacuation to prevent fatigue
- 62.** A nurse is planning evening care for a client who reports waking frequently during the night because of dry mouth and thirst. Which action is best to help promote uninterrupted sleep?
- A.** Offer most of the client's oral fluids with the evening meal instead of at bedtime
 - B.** Encourage the client to drink a full glass of water immediately before sleep
 - C.** Provide a caffeinated beverage at bedtime to increase comfort
 - D.** Suggest the client avoid all fluids after lunch
- 63.** A practical nurse is caring for a client who is diaphoretic and requires a full linen change after a fever. To best protect the client's skin while turning the client in bed, which action should the nurse take?
- A.** Pull the client across the sheet quickly to reduce time exposed to air
 - B.** Lift the client with a drawsheet and keep the skin as clean and dry as possible
 - C.** Massage reddened areas after the linen change to improve circulation
 - D.** Place the client directly on the side rail padding to keep the back off the mattress
- 64.** A practical nurse begins a 24-hour urine collection for a client at 9:00 a.m. Which action is appropriate?
- A.** Save the client's first void at 9:00 a.m. as the start of the specimen
 - B.** Discard the client's first void at 9:00 a.m., then collect all urine for the next 24 hours
 - C.** Collect urine only during daytime hours and refrigerate the specimen overnight
 - D.** Stop the collection after the client has voided eight times
- 65.** A practical nurse is caring for a client who has been on prolonged bed rest after a stroke. Which intervention is most appropriate to help reduce venous stasis in the legs related to immobility?
- A.** Massage the client's calves every 4 hours
 - B.** Place a pillow directly under the knees while the client is supine
 - C.** Encourage active or passive leg exercises at regular intervals
 - D.** Limit fluid intake to decrease dependent edema
- 66.** A nurse is preparing to administer a scheduled oral antihypertensive medication. After verifying the prescription, checking the MAR, and identifying the client with two identifiers, the nurse finds that the client is leaving the unit for physical therapy and says, "I'll just take it when I get back." The medication is due now and has no special time-critical instructions. Which action is most appropriate?
- A.** Ask the physical therapist to give the medication when the session is finished
 - B.** Document the dose as refused and discard the medication immediately
 - C.** Hold the medication temporarily, follow facility policy for timing, and reschedule or administer it within the allowed window
 - D.** Leave the labeled medication cup at the bedside so the client can take it after returning
- 67.** A practical nurse is admitting a client from an assisted-living facility and begins medication reconciliation. The transfer list includes prescribed medications, but the client also says, "I use eye drops every night, a patch for motion sickness when I travel, and an herbal tea to help me sleep." None of these appear on the current medication list. Which action by the nurse is best?
- A.** Document only the prescribed medications because nonprescription products are not included in reconciliation

- B.** Add all reported medications and products, including over-the-counter, topical, and herbal items, for provider review
- C.** Wait until discharge planning to update the medication list because these items are taken at home
- D.** Ask the client to stop using all nonprescription products until the next clinic visit

68. A nurse is preparing to administer diazepam to a patient who is actively seizing. The provider needs the medication delivered quickly, but IV access is not available. Which route of administration is most appropriate?

- A.** Rectal
- B.** Oral
- C.** Transdermal
- D.** Topical

69. A practical nurse accidentally drops a prepared controlled-substance tablet onto the client's bedsheet before the client takes it. The tablet cannot be safely administered. Which action should the nurse take?

- A.** Discard the tablet in the trash, obtain another dose, and document only the dose that was eventually given
- B.** Return the contaminated tablet to the medication drawer and remove a replacement dose
- C.** Follow facility procedure for witnessed disposal of the contaminated tablet and document the waste and replacement appropriately
- D.** Give the tablet to the client because it was dropped only on the clean bedsheet

70. A nurse is caring for a client who has a continuous epidural analgesia infusion after abdominal surgery. Which finding should the nurse identify as the priority complication of this pain-control device?

- A.** Blood pressure decreases from 128/78 mm Hg to 88/54 mm Hg
- B.** Reports itching around the nose and chest
- C.** States pain is 4 on a 0-to-10 scale with movement
- D.** Has not had a bowel movement since surgery this morning

71. A nurse is preparing to administer the next dose of levothyroxine to a client who has hypothyroidism. Which pre-administration information is most important for the nurse to review?

- A.** The client's most recent apical pulse
- B.** The client's last bowel movement
- C.** The client's serum sodium level
- D.** The client's daily fluid intake

72. A client with atrial fibrillation started metoprolol 1 week ago for rate control. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** Apical pulse decreased from 128/min to 82/min
- B.** Blood pressure increased from 104/62 mm Hg to 138/84 mm Hg
- C.** Urine output increased from 30 mL/hr to 60 mL/hr
- D.** Oxygen saturation increased from 93% to 99%

73. A client with asthma is receiving the first dose of IV vancomycin. Ten minutes after the infusion begins, the client reports flushing and itching of the face and neck. Which assessment finding should the nurse identify as the highest-priority adverse response requiring immediate action?

- A.** Blood pressure 82/48 mm Hg
- B.** Temperature 37.4° C (99.3° F)
- C.** Pain rated 3 on a 0-to-10 scale at the IV site
- D.** Urine output 40 mL/hr

74. A patient takes oral tetracycline for acne and tells the nurse, "I usually swallow it with a glass of milk before bed so it doesn't upset my stomach." Which response by the nurse is best?

- A.** Milk can decrease how well tetracycline is absorbed, so it should not be taken with dairy products.
- B.** Taking tetracycline with milk increases the medication's effect and may lead to toxicity.
- C.** The main concern is that milk delays the medication's onset by about 30 minutes.
- D.** It is best to take tetracycline with milk because dairy prevents most drug interactions.

75. A client with a pulmonary embolism is receiving a continuous infusion of alteplase. Before continuing the infusion, the nurse reviews the latest laboratory results and notes a fibrinogen level of 85 mg/dL. Which action is most appropriate?

- A.** Continue the infusion and document the result as expected
- B.** Slow the infusion rate and recheck the fibrinogen level in 8 hours
- C.** Hold the infusion and notify the provider immediately
- D.** Administer the prescribed dose of vitamin K

76. A nurse is teaching a client who has a new prescription for prednisone to take at home for an inflammatory condition. Which instruction should the nurse include?

- A.** Stop taking the medication as soon as your symptoms improve.
- B.** Take the medication with food to help reduce stomach upset.

- C. Limit fluid intake while taking this medication.
- D. Take the medication at bedtime to reduce daytime effects.

77. A nurse is preparing to administer vancomycin IV through a client's existing peripheral line. The client is receiving lactated Ringer's solution at 100 mL/hr. After checking a compatibility reference, the nurse finds that vancomycin is incompatible with lactated Ringer's solution. Which action should the nurse take?

- A. Add the vancomycin directly to the lactated Ringer's bag and infuse it slowly
- B. Stop the primary infusion, flush the line per facility protocol, administer the vancomycin using compatible tubing/solution, and flush again before restarting lactated Ringer's
- C. Administer the vancomycin through the line anyway because incompatibility only matters for central lines
- D. Mix the vancomycin with heparin in a syringe to improve compatibility before administration

78. A nurse is preparing to administer gentamicin 84 mg IM. The medication available is labeled 40 mg/mL. How many milliliters should the nurse draw up for one dose?

- A. 1.1 mL
- B. 2.1 mL
- C. 2.4 mL
- D. 3.0 mL

79. A practical nurse is collecting data on four clients in a medical-surgical unit. Which client should the nurse identify as being at greatest risk for pressure injury development?

- A. A client with a below-the-knee cast who reports itching under the cast
- B. A client who is incontinent, underweight, and requires assistance to turn in bed
- C. A client who has a healing surgical incision and walks in the hall twice daily
- D. A client with a new colostomy who asks for help emptying the pouch

80. A nurse is teaching a client how to complete a 24-hour urine collection. Which client statement indicates correct understanding of the instructions?

- A. I will save my first urine when I start the test and then collect all urine after that.
- B. If I miss one void during the test, I can keep collecting and still send the specimen.
- C. I should urinate at the start time and discard that urine, then collect all urine for the next 24 hours.
- D. I should keep the container at room temperature so the urine does not get too cold.

81. A nurse is reviewing laboratory results for a client who takes carbamazepine for seizure control and now reports fever, sore throat, and mouth ulcers. Which laboratory finding requires the nurse's immediate follow-up?

- A. Platelet count 180,000/mm³
- B. WBC count 2,100/mm³
- C. Sodium 134 mEq/L
- D. AST 42 units/L

82. A client who is 35 weeks pregnant comes to the emergency department reporting painless, bright red vaginal bleeding that began 30 minutes ago. The client denies abdominal pain. Which nursing action is most appropriate first?

- A. Perform a sterile vaginal examination to assess cervical dilation
- B. Apply an external fetal monitor and assess maternal vital signs
- C. Instruct the client to ambulate to see whether the bleeding decreases
- D. Prepare the client for immediate fundal massage

83. A nurse performs a bedside capillary blood glucose test and obtains a reading of 28 mg/dL. The client is awake, talking with family, has warm dry skin, and denies dizziness, shakiness, or weakness. Which action is most appropriate first?

- A. Administer oral glucose immediately based on the meter reading
- B. Repeat the test with a new strip after checking for possible sampling or equipment error
- C. Document the finding as asymptomatic hypoglycemia and recheck in 1 hour
- D. Notify the provider that the client is experiencing severe hypoglycemia

84. A practical nurse is obtaining vital signs for an adult client who reports feeling weak and "feverish." The oral temperature is 101.8°F (38.8°C), pulse is 112/min, and respirations are 22/min. The client's fingers are cold, and the pulse oximeter reading varies between 84% and 89%. Which action should the nurse take first to obtain the most accurate oxygen saturation reading?

- A. Apply oxygen at 2 L/min by nasal cannula before repeating the reading
- B. Warm the client's hand and reposition the sensor, then recheck the reading
- C. Move the sensor to the same finger and hold it in place for 30 seconds
- D. Document the reading as abnormal because fever can lower oxygen saturation

85. A nurse is preparing to obtain an ECG on a client who is wearing a nitroglycerin transdermal patch on the left upper chest. Which action should the nurse take?

- A.** Place the electrode directly over the patch to avoid interrupting therapy
- B.** Remove the patch and leave it off until the provider rewrites the prescription
- C.** Apply the electrode on clean skin near the site but not over the patch
- D.** Cover the patch with gauze, then place the electrode over the gauze

86. A nurse is preparing to obtain a venous blood specimen from an adult client for a potassium level. After cleansing the site and allowing it to dry, the nurse is ready to insert the needle. Which client instruction is most appropriate to help obtain the specimen while reducing the risk of altering the potassium result?

- A.** "Open and close your fist several times while I insert the needle."
- B.** "Keep your arm bent so the vein stays in place."
- C.** "Make a gentle fist, but do not pump your hand."
- D.** "Hold your breath until the tube is filled."

87. A practical nurse is caring for a client who is 6 hours postoperative after a transurethral resection of the prostate (TURP) with continuous bladder irrigation in place. Which finding should the nurse report to the RN immediately?

- A.** Light pink urine in the drainage tubing
- B.** Output that is greater than the irrigation fluid infused
- C.** Bright red urine with several clots and increasing suprapubic discomfort
- D.** A complaint of burning at the urinary meatus

88. A client with a peripheral IV receiving maintenance fluids reports sudden pain at the insertion site. The nurse notes swelling, cool skin, and blanching around the catheter. Which action should the nurse take first?

- A.** Slow the infusion rate and reassess the site in 15 minutes
- B.** Remove the IV catheter and elevate the affected extremity
- C.** Apply a warm compress and continue the prescribed infusion
- D.** Flush the catheter with normal saline to check patency

89. A nurse is caring for a client 24 hours after a cesarean birth. The client reports pain and tenderness in the left calf when walking. Which focused assessment should the nurse perform first?

- A.** Inspect and compare both lower legs for swelling, warmth, and redness
- B.** Ask the client to rate the incisional pain on a 0 to 10 scale

- C.** Auscultate bowel sounds in all four quadrants
- D.** Check the amount of lochia on the perineal pad

90. A client with iron-deficiency anemia began oral ferrous sulfate therapy 1 month ago. Which assessment finding best indicates the treatment is effective?

- A.** The client reports stools are dark green to black
- B.** The client reports increased energy and less shortness of breath with activity
- C.** The client reports mild nausea after taking the medication
- D.** The client requests to take the medication with milk to reduce stomach upset

91. A nurse is caring for a client with a large open wound that requires packing. Which action is most important when applying the packing material?

- A.** Pack the wound tightly to eliminate all dead space
- B.** Moisten the packing and place it loosely into the wound
- C.** Cut one long strip into several small pieces before insertion
- D.** Cover only the wound bed and leave the surrounding skin exposed

92. A nurse responds to an adult client who suddenly collapses during physical therapy. The client is unresponsive, has no pulse, and CPR is in progress. The nurse notices that the compressions are being delivered at the correct rate and depth, but the rescuer pauses for about 15 seconds each time breaths are given. Which effect of these repeated pauses is the nurse most concerned about?

- A.** Reduced blood flow to the heart and brain
- B.** Excessive stimulation of the vagus nerve
- C.** Increased risk of rib fracture from compressions
- D.** Premature triggering of the AED to analyze rhythm

93. A client with chronic obstructive pulmonary disease is prescribed oxygen via Venturi mask at 24%. During assessment, the nurse finds the client eating breakfast with the mask removed and oxygen saturation at 89%. The client says, "I took it off so I could eat." Which nursing action is most appropriate?

- A.** Increase oxygen delivery to 6 L/min by nasal cannula during meals
- B.** Reapply the Venturi mask immediately and instruct the client to keep it on except as specifically directed
- C.** Switch the client to a nonrebreather mask between meals to raise oxygen saturation more quickly
- D.** Document the finding because the saturation is acceptable for a client with COPD

94. A nurse is caring for a client with a cuffless tracheostomy who is awake, alert, and breathing comfortably. During morning care, the nurse notes that the client mouths words but seems frustrated because staff cannot understand requests. Which intervention is most appropriate to improve communication?

- A.** Place a sterile gauze pad over the stoma when the client wants to speak
- B.** Encourage the client to cover the tracheostomy opening with a finger during exhalation
- C.** Provide the client with a call light and a method such as a communication board or writing materials
- D.** Deflate the tracheostomy tube cuff before asking the client to speak

95. A client receiving mechanical ventilation through an endotracheal tube is scheduled for endotracheal suctioning. Which technique should the nurse use to help minimize hypoxemia and mucosal trauma during the procedure?

- A.** Apply suction continuously while inserting the catheter, and suction for 20 to 30 seconds during each pass
- B.** Insert the catheter without suction, apply suction only while withdrawing, and limit each pass to about 10 to 15 seconds
- C.** Instill 10 mL of normal saline into the endotracheal tube before every suction pass
- D.** Suction the airway on a fixed schedule every hour whether secretions are present or not

96. A client with hyperkalemia is on continuous cardiac monitoring. The nurse notes tall, peaked T waves and then sees the QRS complexes begin to widen. Which interpretation is most appropriate?

- A.** The client is showing worsening conduction disturbance that can progress to a life-threatening dysrhythmia
- B.** The client is developing a normal compensatory response to electrolyte replacement
- C.** The monitor leads are loose and the tracing is likely inaccurate
- D.** The changes suggest improved ventricular depolarization

97. A nurse is caring for a client with a newly created ileostomy. During a pouch change, the nurse notes that the stoma is moist and red, and the skin around the stoma has several small raw areas where stool has been leaking under the skin barrier. Which action is most appropriate?

- A.** Apply the next pouching system with a larger opening to avoid rubbing the stoma
- B.** Clean the skin thoroughly with alcohol before applying a new pouch
- C.** Measure the stoma and cut the skin barrier to fit closely around it

D. Use a gauze dressing over the raw areas and delay reapplying the pouch for 8 hours

98. A nurse is caring for a client with a tracheostomy who has had thick secretions throughout the day. Which new finding should the nurse recognize as the most urgent change in condition?

- A.** The client requests suctioning more often than earlier in the shift
- B.** The client has a respiratory rate of 28/min and new inspiratory stridor
- C.** The client reports soreness at the tracheostomy site when coughing
- D.** The client has a small amount of blood-tinged mucus in the tracheostomy dressing

99. A nurse is caring for a client who has been taking calcium-based antacids several times a day for persistent indigestion. The client now reports constipation, abdominal discomfort, and increased urination. Which additional finding most strongly supports the nurse's suspicion of hypercalcemia?

- A.** Positive Chvostek sign
- B.** Hyperactive deep tendon reflexes
- C.** Shortened QT interval on ECG
- D.** Abdominal cramping with diarrhea

100. A client with end-stage kidney disease is receiving hemodialysis through an arteriovenous fistula. Which finding best indicates disequilibrium syndrome during the treatment?

- A.** Progressive confusion with headache and vomiting
- B.** A palpable thrill and audible bruit at the access site
- C.** Mild muscle cramping in the calves near the end of dialysis
- D.** A small amount of blood on the dressing over the fistula needle site

Simulation Test 18 — Answer Key

1. B	26. C	51. B	76. B
2. A	27. B	52. B	77. B
3. C	28. D	53. B	78. B
4. B	29. C	54. B	79. B
5. C	30. C	55. B	80. C
6. B	31. A	56. A	81. B
7. B	32. C	57. A	82. B
8. C	33. B	58. B	83. B
9. C	34. A	59. A	84. B
10. A	35. B	60. B	85. C
11. C	36. B	61. A	86. C
12. C	37. B	62. A	87. C
13. B	38. A	63. B	88. B
14. C	39. B	64. B	89. A
15. A	40. C	65. C	90. B
16. C	41. A	66. C	91. B
17. C	42. B	67. B	92. A
18. B	43. C	68. A	93. B
19. C	44. B	69. C	94. C
20. B	45. C	70. A	95. B
21. C	46. B	71. A	96. A
22. C	47. B	72. A	97. C
23. C	48. C	73. A	98. B
24. A	49. A	74. A	99. C
25. C	50. A	75. C	100. A

Simulation Test 19

100 questions

1. A practical nurse in a rehabilitation unit is deciding what to do next after receiving several updates. Which situation should the nurse address first?
 - A. A client with Parkinson disease needs help getting to the bathroom
 - B. A client 1 day after a stroke has a blood pressure of 150/88 mm Hg and asks when therapy will begin
 - C. A client with diabetes who received regular insulin 30 minutes ago is now shaky and diaphoretic
 - D. A client with a healing hip fracture reports pain of 5 on a 0-to-10 scale during exercises
2. The practical nurse is caring for four clients in a long-term care facility during breakfast. Which action is the highest priority?
 - A. Check a resident with dysphagia who has a wet voice and is coughing during the first few bites of food
 - B. Administer a scheduled stool softener to a resident who has not had a bowel movement since yesterday
 - C. Assist a resident with chronic arthritis to the dining room because the resident is upset about being late
 - D. Return a call to the daughter of a resident who wants an update about the plan for a podiatry visit
3. A practical nurse asks an assistive personnel (AP) to obtain a clean-catch midstream urine specimen from a stable adult client. Later, the AP says, "I collected the urine from the drainage bag after the client came back from the bathroom because it seemed cleaner that way." Which action by the practical nurse is best?
 - A. Send the specimen to the laboratory because the client is stable
 - B. Document that the specimen source was changed and continue care
 - C. Discard the specimen and obtain a new specimen using correct clean-catch instructions
 - D. Ask another AP to label the specimen as collected from the client
4. A practical nurse is assigning care for four clients to an assistive personnel (AP) on a medical-surgical unit. Which task is most appropriate to delegate to the AP?
 - A. Apply a sterile dressing change to a postoperative abdominal incision
 - B. Assist a client with established dysphagia to eat breakfast using prescribed aspiration precautions
 - C. Teach a client how to use a newly prescribed walker before discharge
 - D. Assess a client who reports new numbness in the foot after a cast was applied
5. An adult client who is alert and oriented tells the nurse, "I do not want any more pain medication because it makes me feel foggy." One hour later, the client's spouse says, "The client is hurting and not thinking clearly. Give the medicine anyway." What should the nurse do?
 - A. Administer the medication because the spouse is acting in the client's best interest
 - B. Hold the medication and continue to respect the client's refusal while assessing pain and offering other measures
 - C. Ask another nurse to witness the spouse's request before giving the medication
 - D. Contact the pharmacy to request a lower dose and then administer it without asking the client again
6. A nurse is caring for an adult client who is alert, oriented, and has decision-making capacity. The provider recommends a colonoscopy for ongoing rectal bleeding. Before scheduling, the client says, "My employer already warned me about missing more work, and I may not have anyone to drive me home after sedation. I want to do the test, but I do not see how I can make it happen." Which nursing action best advocates for this client?
 - A. Tell the client that delaying the test could be dangerous and encourage signing the consent today
 - B. Document the client's concerns and ask the provider to explain again why the test is needed
 - C. Refer the client to case management or social services to explore transportation and practical support options
 - D. Advise the client to ask a coworker for a ride so the procedure can be scheduled sooner
7. A practical nurse is caring for a client who asks, "Can my sister pick up my prescription and written discharge instructions for me on her way here?" The sister is not listed in the chart as an approved contact. Which action by the nurse is most appropriate?
 - A. Give the materials to the sister because family members may receive discharge information for the client
 - B. Ask the sister to show identification, then release the materials if the last names match
 - C. Obtain the client's permission before releasing the prescription and discharge instructions to the sister
 - D. Refuse to release the materials and tell the client that discharge documents can never be given to another person

8. A nurse is admitting an adult client who is alert and has decision-making capacity. The client says, "I wrote in my advance directive that I would not want tube feeding if I were permanently unable to recognize my family. Right now I am able to understand everything and I do want the treatment my provider is recommending." Which action should the nurse take?

- A.** Follow the advance directive now because written instructions take priority over the client's current statement
- B.** Ask the health care proxy to decide whether the treatment should be given
- C.** Honor the client's current treatment decision and document the discussion
- D.** Delay treatment until the ethics committee reviews the conflict

9. A client is scheduled for an elective colonoscopy with procedural sedation. When the nurse brings the consent form, the client says, "I had two glasses of wine about an hour ago because I was nervous." The client is awake, cooperative, and says, "That's fine, I can still sign it." Which action should the nurse take?

- A.** Have the client sign the consent because the client is alert and the procedure is routine
- B.** Ask the client's spouse to sign the consent form instead
- C.** Delay having the client sign and notify the provider because the client's decision-making may be impaired
- D.** Witness the client's signature now and ask the provider to review the risks again later

10. A client is being discharged after hospitalization for a pelvic fracture managed without surgery. The client will use crutches and is prescribed pain medication and a follow-up orthopedic visit in 10 days. During discharge teaching, the client says, "My apartment building has a lot of stairs, and I haven't tried using crutches on stairs yet." Which action by the nurse is most appropriate before discharge?

- A.** Ask the client to sign the discharge papers and review stair safety again at the follow-up visit
- B.** Request a physical therapy evaluation to assess crutch use and stair navigation before the client leaves
- C.** Tell the client to avoid using the stairs as much as possible until the fracture heals
- D.** Arrange for the pharmacy to provide written instructions about the pain medication

11. A pediatric clinic wants to improve completion of vaccine screening forms before well-child visits. Staff members report that some parents receive the form at check-in, while others receive it only after being roomed. Before deciding on a change, the quality improvement team wants to understand how the current workflow actually happens across different visits. Which action is most appropriate first?

- A.** Create a new policy requiring all parents to complete the form at home before arrival
- B.** Map the current process from client arrival to form review and identify where variation occurs
- C.** Begin auditing charts weekly to see whether vaccines were given during the visit
- D.** Provide staff education on the importance of accurate vaccine screening

12. A practical nurse is caring for a client after a stroke. During breakfast, the nurse observes pocketing of food in the left cheek, a wet-sounding voice after swallowing, and repeated throat clearing. The client is scheduled to receive oral medications in 30 minutes. Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the findings and wait to mention them during the next team meeting
- B.** Notify the appropriate licensed nurse and speech-language pathologist promptly, and hold oral intake until the client is further evaluated according to agency policy
- C.** Give the medications with applesauce because soft foods are easier to swallow
- D.** Ask the family which foods the client usually tolerates best at home

13. A practical nurse is caring for a client hospitalized for infected diabetic foot ulcers. The client is expected to be discharged in 2 days with daily dressing changes and a new antibiotic regimen. Today the nurse learns that the client has no refrigerator, shares a motel room with a cousin, and says, "I don't have a ride to follow-up visits, and I can't miss work again." Which communication by the nurse would best support interdisciplinary care planning?

- A.** Tell the client to discuss those concerns with the provider during discharge
- B.** Document that the client is anxious about discharge and continue routine teaching
- C.** Inform case management and the provider about the client's transportation, housing, storage, and work-barrier concerns, including how these may affect wound care and medication adherence
- D.** Ask the cousin whether dressing changes can be done at the motel

14. A practical nurse is caring for an adult client who is alert, oriented, and scheduled for a bedside thoracentesis. The client asks, "Do I have to sign this consent form now? I still have questions about the risks." Which action by the nurse is most appropriate?

- A.** Explain the risks and benefits in detail and ask the client to sign if the explanation seems understood
- B.** Tell the client that signing is required because the procedure has already been scheduled
- C.** Hold the consent process and notify the provider that the client still has questions before signing
- D.** Ask a family member to sign the consent form so the procedure is not delayed

15. A practical nurse administered the wrong dose of a prescribed medication after misreading the label on a unit-dose package. The client is awake, reports no symptoms, and has stable vital signs. After the nurse reports the error to the RN, the charge nurse says, "Since nothing happened, just watch the client and don't mention it to anyone else." Which action by the practical nurse is most appropriate?

- A.** Follow the charge nurse's direction because no harm is currently evident
- B.** Document the medication as originally prescribed and continue monitoring the client
- C.** Monitor the client, complete required factual documentation, and follow facility policy for reporting the medication error
- D.** Wait until the end of the shift to see whether symptoms develop before deciding whether to report the error

16. A practical nurse in a clinic is caring for a 16-year-old client who came in for treatment of a sprained ankle. When the parent steps out, the client says, "I've been sleeping in my friend's garage because my mom threw me out 3 days ago. Please don't tell anyone." Which action by the nurse is most appropriate?

- A.** Promise confidentiality because the client requested privacy
- B.** Report the suspected neglect according to facility policy and legal requirements
- C.** Wait to see whether the client returns with more evidence of harm
- D.** Tell the parent what the client said and ask for an explanation first

17. A practical nurse in an emergency department suspects human trafficking when a young adult client has untreated injuries, no identification, avoids eye contact, and is accompanied by a person who insists on answering every question. After the accompanying person leaves briefly, the client quietly says, "I can't leave with him." Which action should the nurse take first?

- A.** Document the statement and findings, then follow facility protocol for immediate safety and reporting resources
- B.** Ask the accompanying person to explain the client's injuries and living situation
- C.** Tell the client to call law enforcement after discharge if the situation continues
- D.** Avoid documenting the suspicion until the client gives a full written statement

18. A practical nurse delegates routine morning vital signs to an assistive personnel (AP) who is new to the unit. Which instruction by the nurse is best to promote safe task completion?

- A.** Take everyone's vital signs now, and I will review the chart later for anything unusual.
- B.** Report any temperature above 100.4°F (38°C), systolic blood pressure below 90 mm Hg, pulse below 60 or above 100/min, respirations below 12 or above 24/min, or any client who looks different from usual.
- C.** Only interrupt me if a client refuses the vital signs or asks to speak with a nurse.
- D.** Use your judgment about which abnormal values are important enough to mention.

19. A practical nurse is supervising an assistive personnel (AP) in a skilled nursing facility. Which newly assigned task should the nurse complete personally instead of assigning to the AP?

- A.** Reposition a resident with chronic left-sided weakness every 2 hours
- B.** Measure and record the weight of a resident receiving a low-sodium diet
- C.** Apply a prescribed elastic antiembolism stocking to a resident each morning
- D.** Evaluate a resident's ability to use a walker safely for the first time after sedation was stopped

20. A practical nurse in a women's health clinic is reinforcing preparation instructions for a follow-up procedure. The client says, "I want to come, but I ride the city bus and it stops running before your last afternoon appointments are finished. I already missed one visit because I was stranded." Which action by the nurse best promotes equitable access to care?

- A.** Tell the client to arrange a ride with friends or family before another appointment is made

B. Ask the client whether an earlier appointment time or available transportation resource would make attending possible

C. Document the missed visit and remind the client that timely follow-up is the client's responsibility

D. Advise the client to go to the emergency department if symptoms get worse instead of returning to the clinic

21. A practical nurse in a community clinic reviews appointment patterns for clients referred after an emergency department visit. Which finding most strongly suggests an inequitable access problem that should be escalated for system-level action?

A. Clients from one apartment complex have a higher no-show rate during the month when a nearby road is under repair

B. Clients with newly prescribed antibiotics call more often than expected with questions about adverse effects

C. Clients needing specialty follow-up are much less likely to complete visits when referrals require a separate office across town that does not accept evening appointments

D. Clients seen on Mondays are slightly more likely to arrive late than clients seen on other weekdays

22. A nurse in a busy outpatient imaging center is preparing a client for a CT scan with contrast. The receptionist hands the nurse a chart for James Turner, and a client in the waiting area stands up when the nurse says, "Mr. Turner?" What should the nurse do next to identify the client correctly before the procedure?

A. Escort the client to the procedure room because he responded to the name

B. Ask the client to state his full name and date of birth, and compare them with the chart and identification band

C. Confirm the client's identity by checking the appointment time listed on the chart

D. Ask the receptionist whether this is the client who checked in as James Turner

23. A nurse is preparing to use a heating pad for a client with chronic back pain. During the equipment check, the nurse notices the pad's fabric cover is torn and the internal wires can be felt near the damaged area. The client says the pad still works and asks the nurse to apply it. Which action should the nurse take?

A. Cover the torn area with a towel and apply the pad on the lowest setting

B. Use the heating pad for 10 minutes only, then reassess the skin

C. Do not use the heating pad, remove it from service, and obtain safe replacement equipment

D. Apply the heating pad over the client's clothing to prevent direct skin contact

24. A nurse is preparing to administer the first dose of cefazolin to a client before surgery. During the preadministration check, the client says, "After I took cephalixin years ago, my tongue swelled and I had trouble breathing." No allergy is documented in the record. Which action should the nurse take first?

A. Administer the cefazolin because the cephalixin reaction occurred years ago

B. Hold the medication and notify the provider of the reported severe reaction

C. Give the cefazolin slowly while observing for rash or itching

D. Document the client's statement after administering the medication

25. A practical nurse in an assisted-living facility hears a loud crash in a resident hallway during a windstorm. The nurse finds a large window broken inward, with glass scattered across the floor and rain blowing into the corridor. Two residents are approaching the area from opposite directions. Which action should the nurse take first?

A. Sweep up the glass to prevent staff injuries

B. Move the residents away from the area and block access to the hallway

C. Call maintenance to board up the window

D. Place towels on the floor to absorb the rainwater

26. During a blood transfusion, a hospitalized client with intermittent confusion begins reaching toward the tubing and says, "Get this off me." The client is not violent, makes eye contact, and stops reaching for the tubing when the nurse holds the client's hand and explains what is happening, but starts again when the nurse steps away. Which action is most appropriate next?

A. Apply soft wrist restraints to both arms to prevent disruption of the transfusion

B. Ask another staff member or family member to remain with the client for close observation and redirection

C. Place the client in a geri chair with a locked tray until the transfusion is finished

D. Discontinue the transfusion because the client is confused

27. A nurse receives a verbal report from an assistive personnel member who says, "I almost emptied the drainage from one client's urinary catheter bag into another client's measuring container, but I noticed the room label before I did it." No client contact occurred and no fluid was mixed. How should the nurse classify this event?

A. An expected variation in workflow that does not require follow-up

- B.** A near miss because an error was caught before reaching the client
- C.** An adverse event because contamination was likely to occur
- D.** A sentinel event because the mistake involved two clients
- 28.** A nurse needs to move a client from a bed to a stretcher for diagnostic testing. The client is drowsy after receiving pain medication, cannot assist with movement, and must remain in a supine position. Which transfer device is most appropriate?
- A.** Gait belt
- B.** Standing transfer aid
- C.** Lateral transfer device
- D.** Transfer board used with the client in a sitting position
- 29.** A practical nurse is preparing to administer eye drops to a client with bacterial conjunctivitis. After performing hand hygiene and applying clean gloves, the nurse cleans the eyelids from the inner canthus to the outer canthus using sterile cotton balls. Which action is most appropriate to prevent transmission of infection during the procedure?
- A.** Use the same cotton ball to cleanse both eyes so the technique remains consistent
- B.** Allow the dropper tip to rest lightly on the lower conjunctival sac for better accuracy
- C.** Cleanse each eye with a separate cotton ball and avoid touching the dropper tip to the eye
- D.** Remove the gloves before instilling the drops to reduce contamination from the gloves
- 30.** A nurse wears clean gloves to perform oral care for a client. During the procedure, the client begins to cough, and saliva splashes onto the nurse's forearm. What is the nurse's priority action after finishing the immediate client task safely?
- A.** Wipe the forearm with a dry paper towel and continue with other care
- B.** Remove the gloves, wash the exposed skin, and perform hand hygiene
- C.** Keep the gloves on and apply alcohol-based hand rub over the gloves
- D.** Change to a new pair of gloves and document the exposure at the end of the shift
- 31.** A practical nurse is preparing a sterile field for a dressing change. After opening the kit, the nurse needs to reposition the sterile drape slightly on the table because it is too close to the edge. Which action is best?
- A.** Slide the drape carefully to the center by grasping only the outer edge
- B.** Lift the drape from the corners and place it in a better position
- C.** Leave the drape in place and obtain a new sterile drape if repositioning is needed
- D.** Ask another nurse wearing clean gloves to move the drape
- 32.** A practical nurse is assisting with a sterile dressing change in a semiprivate room. The nurse has opened the sterile field correctly on a bedside table. The client's roommate begins walking around the room, and the privacy curtain is open. Which action best protects the sterile field?
- A.** Continue the setup because the field remains sterile unless someone touches it
- B.** Move the sterile field to waist level on the client's bed for closer observation
- C.** Ask the roommate to stay away, close the privacy curtain, and keep the sterile field in constant view
- D.** Cover the sterile field with a clean towel until the procedure starts
- 33.** A practical nurse is preparing to assign a roommate for a newly admitted client who has respiratory syncytial virus (RSV). Which roommate is most appropriate for cohorting with this client if a private room is unavailable?
- A.** A client with pulmonary tuberculosis
- B.** A client with RSV
- C.** A client with neutropenia after chemotherapy
- D.** A client with a postoperative wound infection caused by MRSA
- 34.** A practical nurse is assisting a client with changing a colostomy pouch. The client has no isolation orders. As the old pouch is removed, liquid stool begins to leak onto the client's abdomen and the bedside pad. Which action best follows standard precautions?
- A.** Apply clean gloves before removing the pouch and perform hand hygiene after glove removal
- B.** Wear a sterile gown and sterile gloves because stool is present
- C.** Put on a respirator before starting because stool may contain microorganisms
- D.** Complete the pouch change with bare hands if the client's skin is intact
- 35.** A nurse is reinforcing teaching with a client who is 9 weeks pregnant about measures to reduce nausea and vomiting. Which instruction should the nurse include?
- A.** Drink most fluids between meals instead of with meals
- B.** Eat three large meals each day to maintain nutrition
- C.** Choose spicy foods to stimulate appetite
- D.** Lie flat for 30 minutes after eating

36. A client in labor is 9 cm dilated and reports sudden intense rectal pressure during contractions. On assessment, the fetal heart rate is 136/min, and a bulging perineum is visible. Which action should the nurse take next?

- A.** Encourage the client to begin pushing with each contraction
- B.** Perform a vaginal assessment to determine whether the cervix is fully dilated
- C.** Administer the prescribed opioid analgesic for increasing discomfort
- D.** Assist the client to ambulate to reduce pressure sensations

37. A nurse is reinforcing discharge teaching for a client who is 3 weeks postpartum after an uncomplicated vaginal birth. The client says, "My vaginal bleeding had almost stopped, but today it became bright red again after I spent the afternoon doing housework." Which response by the nurse is best?

- A.** This is expected at 3 weeks postpartum and does not require any change in activity.
- B.** You should increase your fluid intake and continue your usual activity level.
- C.** You may have resumed activity too quickly; rest more and decrease strenuous activity.
- D.** This means you have a postpartum infection and need antibiotics right away.

38. A nurse is assessing a 3-month-old infant at a well-child visit. Which finding is most consistent with expected development?

- A.** The infant briefly holds the head steady when supported in a sitting position
- B.** The infant uses a pincer grasp to pick up a small object
- C.** The infant sits without support for several minutes
- D.** The infant says "mama" specifically to the parent

39. A practical nurse is reinforcing teaching with the son of a 78-year-old client who stopped driving after decreased vision and slower reaction time made driving unsafe. The son says, "My father is angry all the time now and says everyone is treating him like a child." Which response by the nurse is most appropriate?

- A.** "This reaction is uncommon, so he should be evaluated immediately for severe cognitive decline."
- B.** "Losing the ability to drive can feel like a loss of independence, so involving him in other transportation plans may help him adjust."
- C.** "It is best for the family to make transportation decisions without asking him, so he does not become more upset."
- D.** "He should stay home more until he accepts the change and stops arguing about it."

40. A practical nurse is reinforcing teaching with a client about prevention strategies. Which intervention is an example of tertiary prevention?

- A.** Providing a smoking-cessation class for adolescents
- B.** Checking blood pressure at a community screening event
- C.** Teaching foot care to a client who has diabetes and peripheral neuropathy
- D.** Administering an influenza vaccine at a clinic

41. A nurse is reinforcing teaching with the parent of a 12-year-old child about expected physical changes of puberty. Which finding should the nurse identify as an expected early change in girls?

- A.** Breast budding
- B.** Voice deepening
- C.** Facial hair growth
- D.** Broadening of the shoulders

42. A nurse is teaching a client about foot care after a new diagnosis of peripheral neuropathy. The client repeatedly interrupts to answer text messages, looks down at the phone while the nurse is speaking, and says, "Sorry, I just have a lot going on." Which action by the nurse is best to reduce this communication barrier?

- A.** Continue the teaching quickly so all instructions are covered before the client loses interest
- B.** Ask the client to put the phone away and explain that interruptions are not allowed during teaching
- C.** Pause and ask the client whether this is a better time for teaching or whether another time would work better
- D.** Give the client a printed handout and document that the teaching was completed

43. A nurse is planning discharge teaching for a client who was newly prescribed a home blood glucose monitor. When the nurse begins explaining the steps, the client says, "I haven't slept all night because of the noise on this unit. I can't focus on anything right now." Which action by the nurse is best?

- A.** Continue the teaching now, but speak more slowly and repeat each step twice
- B.** Ask a family member to learn the procedure instead of the client
- C.** Postpone teaching until the client is better rested and able to concentrate
- D.** Give the client a detailed written handout to review independently

- 44.** A client newly admitted to a long-term care facility says to the practical nurse, "At home I knew where everything was. Here I feel lost, and I don't think anyone really understands how hard this is." Which response by the nurse best provides emotional support?
- A.** You will get used to it after a few days, so try not to focus on it.
 - B.** Many people feel this way at first, but the important thing is that you are safe here.
 - C.** It sounds like this move feels overwhelming and isolating for you right now.
 - D.** Let me show you the schedule so you can stay busy and not think about it.
- 45.** A client who had a stroke 1 week ago says, "I get so frustrated during therapy that I want to quit when I can't do something right away." Which response by the nurse best promotes adaptive coping?
- A.** "You need to push yourself harder so you recover as fast as possible."
 - B.** "Try to focus on one small goal for today, and we can build from each success."
 - C.** "If therapy upsets you this much, it may be better to skip a few sessions."
 - D.** "You should avoid thinking about your limitations until you feel stronger."
- 46.** A nurse in a long-term care facility is assessing a client who has become increasingly irritable during the evening. Which finding should the nurse identify as the strongest indicator that the client's risk for violence is escalating?
- A.** The client refuses to join the scheduled group activity
 - B.** The client begins muttering while walking alone in the hallway
 - C.** The client is suddenly quiet, clenches fists, and fixes a prolonged stare on a nursing assistant
 - D.** The client asks to return to the room earlier than usual
- 47.** A client who has a history of heavy daily alcohol use is admitted with cellulitis. During the evening assessment, the client says, "I don't need help. I just need a drink to calm my nerves." The client is diaphoretic, anxious, and has a fine tremor. Which statement by the nurse is most appropriate?
- A.** "You should try to relax because these symptoms will probably pass on their own."
 - B.** "Stopping alcohol suddenly can cause serious withdrawal, so I need to assess you and notify the provider right away."
 - C.** "If you are worried about withdrawal, we can ask your family to bring in a small amount of alcohol."
 - D.** "Since you are still oriented, the symptoms are not severe enough to require treatment."
- 48.** A client with traumatic brain injury becomes increasingly agitated during a dressing change. The client grabs at the bandages, shouts, "Stop touching me!", and tries to push the nurse's hands away. Which intervention should the nurse implement first?
- A.** Continue the dressing change as quickly as possible to finish the procedure
 - B.** Step back briefly, speak calmly in short simple statements, and reduce stimulation
 - C.** Tell the client that the behavior is inappropriate and must stop immediately
 - D.** Apply wrist restraints so the dressing can be completed safely
- 49.** A hospitalized older adult who was alert and oriented yesterday is now restless, misidentifies staff, and cannot follow a simple conversation. The family reports the client has had mild forgetfulness at home for several months, but this change started this morning. Which action by the practical nurse is most appropriate?
- A.** Explain that this is an expected progression of dementia and reorient the client once per shift
 - B.** Encourage the family to bring familiar objects from home and reassess the client tomorrow
 - C.** Report the sudden change in mental status promptly for further assessment
 - D.** Limit stimulation by keeping the client alone in a dark, quiet room for the rest of the day
- 50.** A client tells the nurse, "My stress gets worse when my day feels out of control. I made a plan to choose the 2 most important tasks each morning, take a short walk during lunch, and ask my sister to help with errands twice a week." Which response by the nurse is most appropriate?
- A.** "That plan uses prioritizing, physical activity, and social support, which are healthy ways to manage stress."
 - B.** "You should avoid relying on other people, because that can increase stress over time."
 - C.** "It would be better to stay busy all day so you do not have time to think about stress."
 - D.** "You should wait to see whether the stress goes away on its own before making these changes."
- 51.** During admission to a long-term care facility, a client tells the practical nurse, "Friday sunset is the start of an important weekly observance for me. I avoid certain activities during that time, and I am worried people here will not understand." Which response by the nurse is most appropriate?
- A.** "Tell me more about what practices are important to you so we can note your preferences and support them as much as possible."
 - B.** "Most routines in the facility cannot be changed, but you can discuss it with your family."

C. "It is best to focus on your health right now rather than on weekly religious customs."

D. "I will ask the chaplain to explain your observance to you again after dinner."

52. A nurse is admitting a client who recently immigrated to the United States. When the nurse asks whether the client has any allergies, the client replies, "No allergies, but I drink a special root tea every day to keep my blood pressure down." Which response by the nurse is most appropriate?

- A.** Tell the client to stop using the tea because prescribed medicine is safer
- B.** Ask the client what is in the tea, how often it is used, and document the information for the provider
- C.** Explain that herbal remedies are not relevant unless the client is having side effects
- D.** Advise the client to continue the tea and prescribed medications exactly as usual

53. A nurse is caring for a client who identifies as nonbinary. Before morning rounds, the client says, "I'm fine with you using my chosen name, Avery, but please don't put a sign on my door about pronouns. I don't want everyone walking by to know my personal business." Which nursing action is most appropriate?

- A.** Place a pronoun reminder sign on the door so all staff will address the client correctly
- B.** Tell the client that pronoun signage is required to prevent staff errors
- C.** Respect the client's request, communicate the chosen name privately to the care team, and avoid public disclosure
- D.** Use the legal name until the client agrees to visible pronoun identification

54. A client with chronic tinnitus tells the nurse, "The ringing never really stops. I avoid sitting with other people because I can't follow conversations, and then I get irritated and snap at everyone." Which nursing response is best?

- A.** "You should try to ignore the sound so it does not bother you as much."
- B.** "It sounds frustrating. Let's talk about situations that make communication harder and ways to reduce your stress during them."
- C.** "Most people adjust to tinnitus over time, so you will probably feel better soon."
- D.** "If conversations are upsetting, it may be best to stay by yourself until the ringing improves."

55. A client with advanced heart failure who is receiving end-of-life care tells the nurse, "I need to tell my daughter where the insurance papers are and who should get my watch, but every time she visits she says we should only talk about happy things." Which nursing action is most appropriate?

- A.** Tell the client to wait until the daughter is more emotionally prepared for the discussion
- B.** Reassure the client that the daughter is probably trying to help and change the subject to symptom control
- C.** Ask the client whether they would like the nurse to help arrange a time to support a conversation about these wishes
- D.** Explain to the daughter that avoiding the topic is inappropriate and that she needs to listen to the client

56. A nurse is administering prescribed medications through a client's mature gastrostomy tube before an intermittent feeding. The medications available are a liquid multivitamin, a crushed immediate-release tablet mixed with water, and an enteric-coated tablet. Which action by the nurse is most appropriate?

- A.** Mix all three medications together with the formula and give them as one bolus
- B.** Crush the enteric-coated tablet and give all medications back-to-back without flushing
- C.** Administer the medications separately, flushing the tube with water between each medication, and clarify the enteric-coated tablet prescription with the provider
- D.** Hold all medications until the next day so the tube does not become clogged

57. A nurse is caring for a client on prolonged bed rest who is unable to move the ankles independently. Which intervention is most appropriate to help prevent footdrop?

- A.** Place trochanter rolls along the client's hips
- B.** Support the feet in dorsiflexion with a footboard or high-top sneakers
- C.** Keep pillows under both knees while the client is supine
- D.** Position the client in semi-Fowler's position for most of the day

58. A nurse is reinforcing teaching for a client who has a stage 3 pressure injury and asks which foods can help support wound healing. Which meal choice should the nurse recommend?

- A.** Turkey sandwich, yogurt, and orange slices
- B.** Plain pasta, applesauce, and tea
- C.** Green salad with fat-free dressing and diet soda
- D.** White toast, gelatin, and clear broth

59. A client with a fresh episiotomy reports perineal pain and swelling 4 hours after a vaginal birth. Which nonpharmacologic intervention should the nurse implement?

- A.** Apply an ice pack to the perineal area
- B.** Encourage use of a heating pad to the perineum
- C.** Massage the swollen area for 10 minutes
- D.** Place the client in high-Fowler position

60. A nurse is caring for a client with chronic obstructive pulmonary disease who becomes fatigued and short of breath while bathing. Which nursing intervention best supports the client's ability to complete this activity of daily living as independently as possible?

- A.** Encourage the client to finish the bath quickly before resting
- B.** Provide a shower chair and plan rest periods during the bath
- C.** Complete the bath for the client to prevent oxygen desaturation
- D.** Ask the client to remain on bed rest until respiratory status improves

61. A nurse is caring for a client who has chronic constipation and is starting a bowel-training program. Which nursing action is best to help establish a regular pattern of elimination?

- A.** Offer a bedpan at the same time each day, preferably after breakfast
- B.** Limit fluid intake in the evening to reduce abdominal discomfort
- C.** Encourage the client to ignore the urge to defecate until a scheduled time
- D.** Keep the client on strict bed rest until bowel movements become regular

62. A nurse is caring for a client who says, "I drink several cups of coffee during my night shift, and then I still can't fall asleep when I get home in the morning." Which teaching should the nurse provide?

- A.** Avoid all fluids for 6 hours before going to bed
- B.** Exercise vigorously right before trying to sleep
- C.** Limit caffeine several hours before the planned sleep time
- D.** Stay in bed until sleep occurs, even if it takes several hours

63. A practical nurse is caring for a client with diarrhea who has frequent loose stools. During perineal care, the nurse notes diffuse redness over the buttocks with irregular edges. The skin is moist and tender, but there is no wound over a bony prominence. Which interpretation is most appropriate?

- A.** This finding is most consistent with moisture-associated skin damage from stool exposure

- B.** This finding should be staged as a stage 1 pressure injury
- C.** This finding indicates a deep tissue pressure injury
- D.** This finding is expected after diarrhea and does not require intervention

64. A practical nurse is caring for a client with diabetic peripheral neuropathy who cannot reliably feel pressure on the buttocks and heels. Which intervention is most appropriate to help prevent skin injury while the client is on bed rest?

- A.** Massage the reddened areas over bony prominences every 2 hours
- B.** Place a heating pad under the heels to improve circulation
- C.** Reposition the client at regular intervals and inspect pressure areas often
- D.** Keep the head of the bed elevated as much as possible throughout the shift

65. A practical nurse is caring for a client who has been on bed rest for several days. Which intervention is most appropriate to help prevent orthostatic hypotension when the client begins to get out of bed again?

- A.** Have the client stand up quickly to strengthen the leg muscles
- B.** Keep the client lying flat for 30 minutes before ambulation
- C.** Raise the head of the bed gradually and dangle the client's legs before standing
- D.** Limit fluid intake before getting the client out of bed

66. A nurse is preparing to administer insulin lispro 6 units subcutaneously before lunch. After confirming the prescription and checking the MAR, the nurse draws up the dose and asks another licensed nurse to verify the insulin type and amount. Which medication right is most directly supported by this double-check?

- A.** Right route
- B.** Right time
- C.** Right dose
- D.** Right documentation

67. A practical nurse is preparing a client's 0900 medications. The MAR lists metoprolol tartrate 25 mg PO daily. While reviewing the chart, the nurse sees a provider prescription entered at 0700 that changes the dose to metoprolol tartrate 12.5 mg PO daily because of low blood pressure overnight. The MAR has not been updated yet, and the 0900 dose has not been given. What is the nurse's best action?

- A.** Administer 25 mg as listed on the MAR because it is the current administration record
- B.** Administer 12.5 mg based on the most recent prescription after following facility procedure to resolve the MAR discrepancy

- C. Hold the medication and wait until the next day for the MAR to update automatically
- D. Ask the client whether 25 mg or 12.5 mg is the usual home dose before giving the medication

68. A nurse is preparing to give nitroglycerin to a patient who reports sudden chest pressure. The prescription is for a form of the medication that should act rapidly without first passing through the stomach. Which route of administration should the nurse use?

- A. Sublingual
- B. Transdermal
- C. Oral enteric-coated
- D. Intradermal

69. A nurse is caring for a client who has received multiple transfusions over several days. The provider prescribes another unit of packed red blood cells. Which action is most important to reduce the risk of transfusion-associated circulatory overload (TACO)?

- A. Warm the blood before administration
- B. Infuse the blood slowly and monitor for dyspnea and crackles
- C. Use dextrose 5% in water as the primary IV solution
- D. Keep the client flat in bed during the transfusion

70. A nurse is caring for a client with chronic pain who uses a transdermal fentanyl patch at home. Which instruction should the nurse include in the client's teaching to help prevent accidental overdose?

- A. Apply a heating pad over the patch site for better pain relief
- B. Replace the patch whenever breakthrough pain occurs
- C. Avoid exposing the patch to external heat sources such as heating pads or hot tubs
- D. Cut the patch in half if the prescribed dose feels too strong

71. A nurse is preparing to administer the next dose of oral metoprolol to a client with hypertension. Which pre-administration assessment finding is most important for the nurse to review?

- A. Apical pulse of 52/min
- B. Urine output of 500 mL during the last 8 hours
- C. Oral temperature of 37.2°C (99°F)
- D. Blood glucose level of 118 mg/dL

72. A client with iron-deficiency anemia has been taking prescribed oral ferrous sulfate for 6 weeks. Which finding best indicates the medication is producing the intended therapeutic response?

- A. The client reports darker-colored stools
- B. The hemoglobin level has increased from 8.9 g/dL to 11.2 g/dL

- C. The client reports mild nausea after taking the medication
- D. The blood urea nitrogen level is within the expected range

73. A client with a history of deep vein thrombosis began taking warfarin 4 days ago. Which finding should the nurse report to the provider immediately as a possible adverse response to the medication?

- A. Bleeding gums while brushing teeth
- B. Black, tarry stools
- C. Mild nausea after the evening dose
- D. Increased thirst at bedtime

74. A patient taking sertraline for depression comes to the urgent care clinic with a persistent cough and says, "I bought a cough medicine yesterday, and now I feel sweaty, restless, and shaky." Which over-the-counter ingredient should the nurse identify as the most likely cause of a serious drug interaction?

- A. Guaifenesin
- B. Dextromethorphan
- C. Calcium carbonate
- D. Loratadine

75. A client with osteomyelitis is prescribed gentamicin IV. Before administering the next dose, the nurse reviews the laboratory results and notes a serum creatinine increase from 0.9 mg/dL yesterday to 2.1 mg/dL today. Which action is most appropriate?

- A. Administer the medication as prescribed because the creatinine is still within expected range for antibiotic therapy
- B. Hold the dose and notify the provider of the rising creatinine level
- C. Give the medication and encourage the client to increase oral fluid intake
- D. Administer half the prescribed dose and recheck the creatinine level in 24 hours

76. A nurse is providing discharge teaching to a client who has a new prescription for insulin glargine. Which instruction should the nurse include?

- A. Take this insulin only when your blood glucose is above 200 mg/dL
- B. Mix this insulin in the same syringe with regular insulin before giving it
- C. Administer this insulin at the same time each day
- D. Expect this insulin to peak in 1 to 2 hours after injection

- 77.** A nurse is preparing to administer a medication from a multidose ophthalmic drop bottle. The bottle was opened 3 weeks ago, is labeled with the date it was first used, and has been stored as directed. Before giving the dose, the nurse notices the dropper tip touched the client's eyelashes during the previous administration. The solution appears clear, and the manufacturer's expiration date has not passed. Which action should the nurse take?
- A. Wipe the dropper tip with sterile gauze and administer the medication
 - B. Administer the medication because it was stored correctly and is not expired
 - C. Discard the bottle and obtain a new one
 - D. Skip the dose and document that the medication is unavailable
- 78.** A nurse needs to administer potassium chloride 30 mEq diluted in IV fluid over 3 hours using an infusion pump. The pharmacy supplies a premixed bag labeled 40 mEq in 250 mL. How many mL/hr should the nurse program the pump to deliver the prescribed dose within the ordered time?
- A. 62.5 mL/hr
 - B. 75 mL/hr
 - C. 83.3 mL/hr
 - D. 125 mL/hr
- 79.** A practical nurse is collecting data on four clients in a home health program. Which client should the nurse identify as being at greatest risk for carbon monoxide poisoning?
- A. A client using an electric space heater in a bedroom with the door closed
 - B. A client cooking on a gas stove in a kitchen with the exhaust fan turned on
 - C. A client running a portable generator in an attached garage during a power outage
 - D. A client using a microwave oven to heat meals in a small apartment
- 80.** A nurse is collecting a timed 2-hour postprandial blood glucose specimen for a client with diabetes mellitus. The client finished breakfast at 0800. At what time should the nurse obtain the specimen?
- A. 0900
 - B. 1000
 - C. 1030
 - D. 1200
- 81.** A nurse is reviewing morning laboratory results for a client who has syndrome of inappropriate antidiuretic hormone secretion (SIADH) and now reports headache, nausea, and increasing lethargy. Which laboratory finding should the nurse report to the provider first?
- A. Serum sodium 118 mEq/L
 - B. Urine specific gravity 1.032
 - C. Serum osmolality 268 mOsm/kg
 - D. Urine sodium 42 mEq/L
- 82.** A client who is 33 weeks pregnant comes to the labor and delivery unit reporting sudden, severe abdominal pain and dark red vaginal bleeding. On assessment, the nurse notes a rigid, tender abdomen and frequent contractions. Which complication should the nurse suspect first?
- A. Placenta previa
 - B. Abruptio placentae
 - C. Cervical insufficiency
 - D. Hyperemesis gravidarum
- 83.** A nurse obtains a bedside capillary blood glucose reading of 318 mg/dL for a client who has no history of diabetes. The client is alert and asymptomatic, and the nurse notes the client had been eating orange slices just before the test. Which action is most appropriate next?
- A. Administer insulin immediately based on the point-of-care result
 - B. Repeat the test after cleaning and drying the client's fingertip thoroughly
 - C. Document the result as accurate because the client has no symptoms
 - D. Discard the glucometer and replace it with a new device
- 84.** A practical nurse is obtaining vital signs for an adult client. The client drank a cup of hot tea 5 minutes ago and says, "I don't want a rectal temperature." Which action is most appropriate?
- A. Take the oral temperature now and document that the client recently drank a hot beverage
 - B. Wait about 15 to 30 minutes before taking the oral temperature
 - C. Ask the client to rinse the mouth with cool water, then take the oral temperature immediately
 - D. Record the temperature as unavailable and move on to the other vital signs
- 85.** A client is scheduled for bronchoscopy with moderate sedation in 2 hours. Which preprocedure finding is most important for the nurse to report to the provider?
- A. The client drank 240 mL of water 30 minutes ago
 - B. The client has a productive cough with thick yellow sputum

C. The client signed the consent form but has additional questions

D. The client reports a shellfish allergy

86. A nurse is preparing to obtain a venous blood specimen from an adult client. Which vein is generally the best first choice for routine venipuncture because it is well anchored and has a lower risk of injury to major nerves and arteries?

A. Cephalic vein

B. Basilic vein

C. Median cubital vein

D. Dorsal metacarpal vein

87. A practical nurse is caring for a client who is 6 hours postoperative after abdominal surgery. Which finding is most consistent with developing atelectasis?

A. Crackles at the lung bases with pink frothy sputum

B. Diminished breath sounds and a temperature of 100.2°F (37.9°C)

C. Sudden pleuritic chest pain and hemoptysis

D. Productive cough with thick green sputum

88. A client with a peripherally inserted central catheter (PICC) suddenly becomes short of breath while the nurse is changing the IV tubing. The nurse notices that the catheter hub was briefly left open to air. Which action should the nurse take first?

A. Place the client on the left side in Trendelenburg position

B. Flush the catheter with normal saline to clear any air

C. Remove the PICC line immediately

D. Have the client take deep breaths and cough

89. A nurse is caring for a client who is 12 hours post-op after a total hip arthroplasty. The client suddenly reports sharp chest pain and says, "I feel like I can't get enough air." Which focused assessment should the nurse perform first?

A. Assess oxygen saturation and respiratory rate

B. Inspect the hip incision for drainage

C. Ask the client to rate the pain on a 0 to 10 scale

D. Check the client's bowel sounds in all four quadrants

90. A client with hypothyroidism started levothyroxine therapy 6 weeks ago. Which finding best indicates the treatment is having the desired effect?

A. The client reports improved energy and decreased cold intolerance

B. The client has a pulse of 110/min after taking the morning dose

C. The client reports increased appetite with a 4-lb weight gain

D. The client states mild tremors occur in the afternoon

91. A client 6 hours after a laparoscopic cholecystectomy suddenly reports severe right shoulder pain, light-headedness, and increasing abdominal fullness. The nurse finds the skin cool and pale, blood pressure 86/52 mm Hg, heart rate 124/min, and respirations 26/min. Which complication should the nurse suspect?

A. Internal hemorrhage

B. Urinary retention

C. Postoperative ileus

D. Incisional infection

92. A nurse responds to an adult client who collapses shortly after receiving an opioid analgesic. The client is unresponsive and has a palpable carotid pulse of 72/min, but the client is taking only occasional gasping breaths. After calling for help, which action should the nurse take next?

A. Start chest compressions at a rate of 100 to 120/min

B. Provide rescue breaths and support ventilation

C. Apply an AED and follow the prompts

D. Place the client in the recovery position and continue to observe

93. A client with acute pulmonary edema is coughing up pink, frothy sputum and has severe dyspnea. Which oxygen delivery device should the nurse anticipate using to provide the highest concentration of oxygen available without intubation?

A. Nasal cannula

B. Simple face mask

C. Venturi mask

D. Nonrebreather mask

94. A nurse is caring for a client 24 hours after a tracheostomy was placed. During tracheostomy care, the nurse notes a crackling sensation under the skin around the stoma and upper chest. The client has mild swelling at the site. Which action is most appropriate?

A. Document the finding as expected postoperative edema and continue routine care

B. Reposition the client flat in bed and reassess the area in 30 minutes

C. Report the finding to the provider immediately because it may indicate subcutaneous emphysema

D. Remove the tracheostomy dressing and apply a tight occlusive dressing around the stoma

95. A client has been mechanically ventilated through an endotracheal tube for 3 days. Which nursing action is most effective for reducing the risk of ventilator-associated pneumonia?

A. Keep the client in a supine position to limit movement of the endotracheal tube

- B.** Provide scheduled oral care with an antiseptic solution and suction oral secretions as needed
 - C.** Increase the inspired oxygen concentration during each shift change
 - D.** Deflate the endotracheal tube cuff every 2 hours to relieve tracheal pressure
- 96.** A client with a newly implanted permanent pacemaker is being taught how to check the pulse at home. Which instruction should the nurse include?
- A.** Take the pulse once a week and record it only if it feels irregular
 - B.** Check the pulse daily at the same time and report a rate lower than the prescribed limit
 - C.** Count the pulse for 10 seconds and multiply by 6 unless dizziness occurs
 - D.** Assess the apical pulse only after exercise to confirm that the pacemaker is working
- 97.** A nurse is caring for a client 3 days after creation of a new ileostomy. Which finding should the nurse identify as the highest priority?
- A.** The stoma is pink and slightly edematous
 - B.** The client reports cramping before stool is released into the pouch
 - C.** The stoma appears dusky purple
 - D.** The pouch contains liquid green-brown drainage
- 98.** A nurse is caring for a client with acute pancreatitis who has been receiving IV fluids and pain medication. Which new finding should the nurse recognize as the most urgent change in condition?
- A.** Bluish discoloration around the umbilicus
 - B.** Reports pain rated 8 on a 0 to 10 scale
 - C.** Serum potassium level of 3.4 mEq/L
 - D.** Absent bowel sounds in all quadrants
- 99.** A nurse is caring for a client who has had repeated gastric suctioning for 24 hours after abdominal surgery. Which assessment finding is most consistent with developing metabolic alkalosis related to this fluid and electrolyte imbalance?
- A.** Slow, shallow respirations
 - B.** Warm, flushed skin
 - C.** Kussmaul respirations
 - D.** Profuse watery diarrhea
- 100.** A client with chronic kidney disease is receiving hemodialysis through a central venous catheter. Which nursing action is most appropriate to help prevent infection of the dialysis access?
- A.** Use sterile technique when changing the catheter dressing
 - B.** Flush the catheter with normal saline before each meal
 - C.** Obtain routine blood samples from the catheter to reduce venipunctures
 - D.** Keep the catheter uncovered while the client is in bed

Simulation Test 19 — Answer Key

1. C	26. B	51. A	76. C
2. A	27. B	52. B	77. C
3. C	28. C	53. C	78. A
4. B	29. C	54. B	79. C
5. B	30. B	55. C	80. B
6. C	31. C	56. C	81. A
7. C	32. C	57. B	82. B
8. C	33. B	58. A	83. B
9. C	34. A	59. A	84. B
10. B	35. A	60. B	85. A
11. B	36. B	61. A	86. C
12. B	37. C	62. C	87. B
13. C	38. A	63. A	88. A
14. C	39. B	64. C	89. A
15. C	40. C	65. C	90. A
16. B	41. A	66. C	91. A
17. A	42. C	67. B	92. B
18. B	43. C	68. A	93. D
19. D	44. C	69. B	94. C
20. B	45. B	70. C	95. B
21. C	46. C	71. A	96. B
22. B	47. B	72. B	97. C
23. C	48. B	73. B	98. A
24. B	49. C	74. B	99. A
25. B	50. A	75. B	100. A

Simulation Test 20

100 questions

1. A practical nurse in a home health setting receives these phone messages from clients. Which client should the nurse contact first?

- A. A client taking furosemide who reports needing to urinate more often than usual
- B. A client with pneumonia who says, "I am more short of breath walking to the bathroom than I was this morning"
- C. A client with osteoarthritis who asks whether a heating pad can be used on the knee before bed
- D. A client with a new colostomy who wants the nurse to review pouch-changing steps again

2. A practical nurse in a postoperative unit has just finished receiving shift report and can see only one client immediately. Which client is the highest priority to assess first?

- A. A client 4 hours after abdominal surgery who has not voided since returning from the recovery area
- B. A client 8 hours after hip surgery who reports pain of 7/10 and asks for the next PRN analgesic
- C. A client 2 hours after carotid endarterectomy who becomes restless and has a blood pressure of 186/94 mm Hg
- D. A client 1 day after laparoscopic cholecystectomy who has nausea after eating half of breakfast

3. A practical nurse delegated collection of a stool specimen for occult blood testing to an assistive personnel (AP) for a stable client. The AP returns with a specimen cup filled with stool taken from the toilet bowl after the client had a bowel movement. Which action by the practical nurse is best?

- A. Send the specimen to the laboratory because the client is stable and the sample was obtained promptly
- B. Ask the AP to label the specimen and document that it was collected from the toilet
- C. Explain that the specimen may be contaminated, discard it, and provide new instructions for correct collection
- D. Have the AP add water to the specimen cup so the sample can be tested more easily

4. A practical nurse is leading a team on a medical-surgical unit. Which task is most appropriate to assign to an assistive personnel (AP)?

- A. Teach a client with newly prescribed oxygen how to use the equipment at home
- B. Perform oral hygiene for a stable client who had a stroke and has already been assessed as able to swallow safely
- C. Decide whether a client's PRN pain medication relieved incisional pain

D. Assess a newly admitted client with redness over the coccyx

5. A nurse is caring for an adult client who is alert, oriented, and has decision-making capacity. The client says, "I do not want my estranged parent to visit or receive updates." Two hours later, the parent arrives at the unit, states, "I am immediate family, so I have a right to know what is going on," and demands the latest test results at the nurses' station. Which action by the nurse is best?

- A. Provide a brief update without details because the visitor is an immediate family member
- B. Ask the client later whether the parent can receive limited information
- C. Tell the parent that no information can be shared and follow the client's stated wishes about visitation and updates
- D. Refer the parent directly to the provider because only the provider can decide what may be disclosed

6. A nurse is caring for an alert adult client who is scheduled for surgery later today. While reviewing the preoperative checklist, the client says, "I am a Jehovah's Witness, and I do not want blood products even if there is heavy bleeding." The client's spouse says, "Do whatever it takes to save my partner's life." Which action by the nurse best advocates for the client's rights?

- A. Document the spouse's request and prepare to follow it if an emergency occurs
- B. Notify the provider and surgical team immediately of the client's stated refusal of blood products
- C. Ask the spouse to sign a consent form authorizing blood transfusion if needed
- D. Explain to the client that life-saving treatment can be given regardless of the client's wishes

7. A practical nurse is caring for a client who is a minor. The client's divorced parents disagree about who may receive updates. The nurse sees no custody papers or communication restrictions in the chart. Which action is most appropriate?

- A. Give updates only to the parent who asks first
- B. Ask the adolescent client which parent the nurse likes better and follow that choice
- C. Withhold information from both parents until the provider arrives, regardless of the situation
- D. Follow facility policy and consult the charge nurse or supervisor to verify legal authority before sharing information

8. A practical nurse is helping transport a stable client to radiology. In the hallway, the transporter asks loudly, "Is this the client with C. diff from room 312?" Other clients and visitors are nearby. Which response by the nurse is best?

- A.** Yes, and the client has been having diarrhea all morning
- B.** Let's discuss the client's care needs in a private area
- C.** No, but this client also has an infection
- D.** It is fine to talk here because the transporter is part of the care team

9. A nurse is reinforcing teaching with an alert adult client who asks about advance directives. The client says, "I understand a living will gives written instructions, but what is the main purpose of naming a health care proxy?" Which response by the nurse is most appropriate?

- A.** It allows another person to consent to treatment only after the client signs out of the facility.
- B.** It names someone to make health care decisions if the client later loses decision-making capacity.
- C.** It prevents providers from discussing the client's condition with family members.
- D.** It requires the proxy to follow family wishes if relatives disagree about the client's care.

10. A nurse is preparing a client for an elective endoscopy. The consent form has been signed. During the preprocedure assessment, the client says, "The doctor explained the test well, but I only agreed because my son kept saying I would be a burden if I did not go through with it." Which action should the nurse take first?

- A.** Notify the provider that the client's consent may not be voluntary
- B.** Ask the son to step out, then have the client sign a new consent form
- C.** Continue preparing the client because the provider already explained the procedure
- D.** Document the client's statement and send the client to the procedure area as scheduled

11. A client is being discharged after hospitalization for a new seizure disorder. The provider prescribes an anticonvulsant and no driving until cleared at follow-up. During discharge teaching, the client says, "I understand the medicine part, but I work as a delivery driver, and if I stop driving, I may lose my job." Which nursing action is most appropriate before discharge?

- A.** Tell the client to discuss work concerns at the next follow-up visit after discharge
- B.** Reinforce the driving restriction and request a case management or social work referral to address employment and community resource needs

C. Advise the client to continue driving short distances only if no seizure occurs for 24 hours

D. Focus only on medication teaching because work issues are outside the nurse's role in discharge planning

12. A skilled nursing facility wants to reduce delays in responding to residents' call lights during the night shift. Before changing staffing or workflow, the quality improvement team gathers 2 weeks of data and creates a chart showing the number of delayed responses by hour of the night. What is the main purpose of this step?

- A.** To identify patterns in when delays occur so changes can target the most likely causes
- B.** To prove that the current staff members are not performing their duties
- C.** To compare the facility's delays with national mandatory benchmarks
- D.** To determine which residents should stop using the call-light system

13. A practical nurse is caring for a client who had a right total knee replacement yesterday. The client is scheduled for occupational therapy this morning. During assessment, the nurse learns that the client lives alone, the only bathroom is on the second floor, and the client cannot yet lift the operated leg into bed without help. Which communication by the nurse best supports interdisciplinary information sharing?

- A.** "The client had knee surgery yesterday and may need extra therapy before discharge."
- B.** "The client seems worried about going home, so occupational therapy should talk with the client."
- C.** "The client lives alone, has a bathroom only on the second floor, and currently needs help lifting the operated leg into bed; this may affect safe discharge planning and equipment needs."
- D.** "The client asked when occupational therapy will arrive and wants to recover quickly."

14. A practical nurse is caring for an adult client who is alert and oriented. Before a scheduled dressing change, the client says, "I don't want that done right now." The provider's prescription says the dressing is to be changed this shift. Which action by the nurse is most appropriate?

- A.** Explain the purpose of the dressing change, assess the client's concerns, and notify the RN if the client continues to refuse
- B.** Complete the dressing change because there is a prescription for the treatment
- C.** Ask a family member to persuade the client to accept the dressing change
- D.** Document the refusal and take no further action because the client has the right to refuse care

15. A practical nurse is floated to an outpatient clinic and is told to witness a client's signature on a consent form for a procedure. When the nurse enters the room, the client says, "They already told me to sign, but I still don't really understand why this procedure is needed." Which action should the practical nurse take first?

- A.** Explain the procedure in detail and then witness the signature
- B.** Tell the client that signing only means the form was offered, not that the procedure must be done
- C.** Delay witnessing the signature and notify the RN or provider that the client still has questions
- D.** Ask the unlicensed assistive personnel to stay with the client until the provider is available

16. A practical nurse in a rehabilitation unit notices that an assistive personnel staff member slaps a confused older adult client's hand and says, "If you touch that call light again, I'll tie you down." What should the nurse do first?

- A.** Confront the staff member at the end of the shift and suggest anger-management training
- B.** Immediately protect the client and report the incident according to facility policy
- C.** Ask other staff whether they have seen similar behavior before taking action
- D.** Document the event in the staff member's employee record

17. A practical nurse in a pediatric urgent care center suspects physical abuse after assessing a 4-year-old child with patterned bruising. The parent becomes angry and says, "You better not write anything down unless you can prove it." Which response by the nurse is most appropriate?

- A.** I only need to report if the provider confirms abuse after the examination
- B.** I will wait to document until the social worker interviews the family
- C.** I must document objective findings and follow mandatory reporting requirements when abuse is suspected
- D.** I can keep this confidential as long as you agree to bring the child back tomorrow

18. An assistive personnel (AP) tells the practical nurse, "I emptied 900 mL from the client's Foley bag just now, but there was almost nothing in it 2 hours ago." The client is 8 hours postoperative after abdominal surgery. What is the nurse's best action?

- A.** Ask the AP to continue monitoring and report the next urine amount at the end of the shift
- B.** Document the urine output as expected after surgery and make no further changes

- C.** Immediately assess the client and verify the urine output and catheter function
- D.** Tell the AP to clamp the catheter for 30 minutes and then remeasure the output

19. A practical nurse is supervising an assistive personnel (AP) on a medical unit. Which task should the nurse assign to the AP?

- A.** Reinforce discharge teaching for a client newly prescribed warfarin
- B.** Apply a sterile dressing to a stage 3 pressure injury
- C.** Assist a stable client with eating breakfast after the nurse verifies swallowing is safe
- D.** Assess a client's pain 30 minutes after receiving an opioid medication

20. A practical nurse in a community clinic is reinforcing discharge instructions for a client with newly diagnosed hypertension. The client says, "I can read, but these papers are hard for me to understand, and I get confused when people use medical words." Which action by the nurse best promotes equitable access to care?

- A.** Ask the client to sign the form and review the instructions again at the next visit
- B.** Provide plain-language teaching and ask the client to explain the instructions back in the client's own words
- C.** Give the client the full written packet and suggest looking up unfamiliar terms online at home
- D.** Shorten the teaching session so the client does not feel embarrassed

21. A practical nurse in a community vaccination clinic notices that missed second-dose appointments are much higher among clients experiencing homelessness. Many registration records list no phone number or mailing address, and reminder messages are sent only through automated texts. Which nursing recommendation best promotes equitable access to care at the system level?

- A.** Instruct staff to document missed appointments more consistently in the electronic record
- B.** Advise clients to buy low-cost phones so they can receive text reminders
- C.** Develop additional reminder and follow-up options, such as return cards, outreach through shelters, and walk-in follow-up availability
- D.** Schedule clients experiencing homelessness only during the clinic's least busy hours

22. A nurse in a dialysis center is ready to begin treatment for a client who arrives from the waiting area. Two clients have the same last name and are scheduled at similar times. One client says, "I'm Mr. Jackson here for dialysis," and points to the treatment chair he usually uses. Which action should the nurse take to identify the client correctly before starting treatment?

- A.** Ask the client to state his full name and date of birth, then compare both with the identification band and treatment record
- B.** Confirm the client's last name and usual treatment chair with the unit clerk
- C.** Use the time of the appointment and the client's statement that he is there for dialysis
- D.** Ask another staff member whether this is the client who usually receives treatment in that area

23. A nurse is preparing to help a client with left-sided weakness use a rolling walker for the first time after hospitalization. During the equipment check, the nurse notes that one walker leg is adjusted one notch higher than the other three, causing the walker to rock slightly on the floor. Which action should the nurse take?

- A.** Place a folded towel under the shorter side of the walker during ambulation
- B.** Proceed carefully because the client will be supervised during use
- C.** Adjust the walker so all four legs are at the same height before use
- D.** Ask the client to bear more weight on the stronger side while using the walker

24. A nurse is preparing to insert a peripheral IV catheter. The client says, "I had a rash and wheezing after adhesive tape was used during a previous hospital stay." The nurse notes that the standard IV start kit contains an adhesive securement dressing. Which action should the nurse take first?

- A.** Proceed with IV insertion and monitor the client closely for a reaction
- B.** Use the standard dressing but place gauze underneath it
- C.** Stop and obtain allergy-safe alternative supplies before starting the procedure
- D.** Ask the client whether over-the-counter antihistamines helped in the past

25. A practical nurse on a medical-surgical unit hears the facility announce a bomb threat. In a client room, the nurse sees an unfamiliar backpack under a chair near the doorway. The client is stable and asks whether the nurse should move the bag into the hall so no one trips over it. Which action should the nurse take first?

- A.** Move the backpack to the nurses' station for inspection
- B.** Open the backpack carefully to identify its owner

- C.** Keep people away from the area and report the suspicious item per facility protocol
- D.** Place the backpack in a trash receptacle outside the room

26. A client with dementia becomes restless during the evening, repeatedly removes the nonskid socks, and starts sliding toward the edge of a chair while waiting for family to arrive. The client is not combative and follows simple one-step directions. Which action by the nurse best follows least-restrictive restraint principles?

- A.** Apply a pelvic restraint to keep the client safely positioned in the chair
- B.** Ask the provider for an as-needed sedative so the client will remain calm until family arrives
- C.** Move the client closer to the nurses' station and remain with the client while offering redirection and supervised repositioning
- D.** Raise all four side rails when the client is transferred back to bed

27. A nurse is preparing to administer a prescribed dose of insulin and notices that the syringe drawn up by another nurse contains 10 units instead of the prescribed 4 units. The insulin has not been given, and the client has no adverse effects. Which action is most appropriate?

- A.** Discard the incorrect syringe, obtain the correct dose per policy, and complete required reporting of the near miss
- B.** Administer 4 units from the prepared syringe and document that the excess insulin was wasted
- C.** Ask the provider whether the 10-unit dose should be given since the insulin is already prepared
- D.** Return the syringe to the medication refrigerator and monitor the client for signs of hypoglycemia

28. A nurse is deciding whether to use a powered full-body mechanical lift for a transfer from bed to chair. Which client is the best candidate for this device?

- A.** A client who follows commands, bears full weight, and walks with minimal assistance
- B.** A client who can stand briefly, grasp handles, and participate in a sit-to-stand transfer
- C.** A client who is unable to bear weight, cannot assist with the move, and has unpredictable body movements
- D.** A client who reports mild weakness but can pivot safely with a gait belt

29. A practical nurse is preparing to collect a sterile urine specimen from the sampling port of an indwelling urinary catheter for culture. Which action is most appropriate to reduce the risk of contaminating the specimen?

- A.** Disconnect the catheter from the drainage tubing and collect urine directly from the catheter

- B.** Withdraw urine from the sampling port with a sterile syringe after cleansing the port per facility protocol
- C.** Pour fresh urine from the drainage bag into a sterile specimen cup
- D.** Clamp the catheter, then collect the first urine that drains from the bag 30 minutes later
- 30.** A nurse puts on clean gloves to provide incontinence care for a client. While cleaning the client, the nurse notices that the protective gown sleeve has become wet with urine. Which action is most appropriate?
- A.** Continue care because urine exposure is expected during incontinence care
- B.** Remove the wet gown, perform hand hygiene, and apply a clean gown before continuing care
- C.** Place a dry towel over the wet sleeve and finish the task
- D.** Remove the gloves only and continue care with the same gown
- 31.** A practical nurse is preparing a sterile field for a wound irrigation procedure. After opening the sterile tray, the nurse notices the field is set up below waist level on a low bedside stool because the overbed table is being used for a meal tray. Which action should the nurse take?
- A.** Proceed carefully because the sterile supplies have not been touched
- B.** Move only the sterile instruments to a higher surface and continue the procedure
- C.** Discard the setup and prepare a new sterile field on a surface at or above waist level
- D.** Ask a second nurse to watch the field closely while the procedure continues
- 32.** A practical nurse is preparing to enter the room of a client diagnosed with rubella. Which personal protective equipment is most appropriate for the nurse to wear when providing routine care?
- A.** Sterile gloves only
- B.** A surgical mask
- C.** An N95 respirator
- D.** A face shield only
- 33.** A practical nurse is caring for a client who has a draining wound infected with a multidrug-resistant organism and is on contact precautions. After providing care, which action should the nurse take first when removing personal protective equipment before leaving the room?
- A.** Remove the gloves
- B.** Untie the gown at the neck
- C.** Open the room door
- D.** Perform hand hygiene over the gloves
- 34.** A practical nurse is preparing to empty a client's Jackson-Pratt drain. The client has no isolation orders, and the nurse does not expect splashing. Which action best reflects standard precautions?
- A.** Wear clean gloves while handling and emptying the drain, then remove the gloves and perform hand hygiene
- B.** Skip gloves because the drainage is contained inside the bulb reservoir
- C.** Wear sterile gloves because any contact with a surgical drain requires sterile technique
- D.** Put on a gown and N95 respirator before opening the drain
- 35.** A nurse is reinforcing teaching with a client who is 18 weeks pregnant about nutrition during pregnancy. Which food choice should the nurse identify as the best source of iron?
- A.** Baked chicken breast with white rice
- B.** Low-fat yogurt with sliced peaches
- C.** Fortified breakfast cereal with orange juice
- D.** Whole-grain toast with peanut butter
- 36.** A client in active labor has spontaneous rupture of membranes. The nurse notes that the umbilical cord is visible at the vaginal opening, and the fetal heart rate drops to 80/min. Which action should the nurse take first?
- A.** Place the client in a knee-chest or Trendelenburg position
- B.** Apply oxygen at 10 L/min by face mask
- C.** Prepare the client for an immediate vaginal delivery
- D.** Check the maternal blood pressure and pulse
- 37.** A nurse is reinforcing discharge teaching for a client who is 24 hours postpartum after an uncomplicated vaginal birth. Which client statement indicates correct understanding of perineal care?
- A.** "I should wipe from front to back after using the bathroom."
- B.** "I should avoid changing my perineal pad until it feels saturated so I do not irritate the area."
- C.** "I should cleanse the perineal area once each morning, even if I use the toilet several times during the day."
- D.** "I should apply perfumed powder to the perineal area to keep it dry."
- 38.** A nurse is collecting health history from the parent of a 3-year-old child. Which parent statement describes an expected developmental milestone for this age?
- A.** My child can ride a two-wheeled bicycle without training wheels.
- B.** My child speaks in short sentences and asks many questions.
- C.** My child can tie shoelaces independently.

D. My child understands conservation of volume and number.

39. A practical nurse is reinforcing teaching with a 52-year-old client who recently entered menopause. The client says, "I feel unsettled by these changes, and I worry that this means something is wrong with me." Which response by the nurse is most appropriate?

A. Menopause usually means a woman will quickly develop serious health problems.

B. These changes can be a normal life transition, but you should report any unusual or severe symptoms to your provider.

C. Most women do not notice emotional or physical changes during menopause.

D. You should avoid discussing these feelings because they usually go away on their own.

40. A practical nurse is reinforcing teaching with a parent about adolescent preventive care. The parent says, "My teenager is healthy and plays sports, so those routine depression screening questions at checkups are not really necessary." Which response by the nurse is best?

A. Depression screening is mainly used only when a teenager has already been diagnosed with a mental health disorder.

B. Routine screening can help identify concerns early, even when a teenager seems healthy and has no obvious symptoms.

C. If a teenager is active in sports and school, depression screening is usually not needed.

D. Depression screening is done to determine whether a teenager needs emergency hospitalization.

41. A nurse is obtaining a health history from an adult client who says, "I get short of breath sometimes." Which follow-up question best helps the nurse determine whether this symptom is associated with activity intolerance?

A. "When did you first notice the shortness of breath?"

B. "What were you doing when the shortness of breath started?"

C. "Does anyone in your family have asthma?"

D. "Have you traveled anywhere recently?"

42. A nurse is teaching a client about self-administration of insulin. The client says, "I didn't sleep at all last night because my baby cried the whole time." During the teaching session, the client yawns, loses track of the steps, and asks the nurse to start over twice. Which action by the nurse is best?

A. Continue the teaching session and provide a printed handout for the client to read later

B. Ask a family member to listen to the teaching and explain it to the client afterward

C. Postpone the teaching until the client is more rested and then re-teach the skill in short steps

D. Speak more loudly and repeat each instruction several times without changing the timing

43. A nurse is preparing discharge teaching for a client who must learn how to use a walker at home after hip surgery. As the nurse begins, the client says, "I usually understand better when I watch someone do it instead of just listening." Which action by the nurse is best?

A. Provide the client with a detailed written instruction sheet to review later

B. Demonstrate correct walker use, then have the client return-demonstrate the skill

C. Explain the steps more slowly and ask the client whether the instructions are clear

D. Postpone teaching until a family member is present to hear the instructions

44. A client who was told today that a colostomy will be permanent says to the practical nurse, "I can't imagine living like this. I don't want my spouse to see me anymore." Which response by the nurse best provides emotional support?

A. You should focus on the fact that the surgery saved your life.

B. Many people adjust to a colostomy over time, so try not to worry so much.

C. It sounds like you are worried about how this will affect your relationship with your spouse.

D. Let me explain how to care for the colostomy so you will feel more confident.

45. A client who is caring for an older parent at home tells the nurse, "By the end of the day I feel completely drained, but I never ask my brothers for help because I should be able to handle this myself." Which response by the nurse best promotes effective coping?

A. "You have taken on a lot. Let's talk about which caregiving tasks you might be willing to share with family members."

B. "If your brothers wanted to help, they would have offered by now."

C. "Try not to think about it so much, and your stress may improve."

D. "You need to continue managing this alone so you do not lose your independence."

46. A nurse in an outpatient mental health center is meeting with a client who recently separated from a partner. During the interview, the client says, "If she keeps ignoring my calls, I'll make sure she never does it again." The client then pulls a folded paper from a pocket that lists the partner's work schedule and usual gym times. Which action should the nurse take first?

- A.** Encourage the client to write down feelings in a journal and discuss them at the next visit
- B.** Ask the client to sign a no-violence contract before leaving the clinic
- C.** Initiate immediate safety measures and notify the appropriate team members because the threat is specific and targeted
- D.** Redirect the conversation to the client's childhood coping patterns to reduce current anxiety

47. A client who stopped drinking alcohol about 36 hours ago becomes increasingly restless and confused. The client is sweating, trying to get out of bed, and says, "There are animals running across the wall." Which nursing action is the priority?

- A.** Provide black coffee to improve alertness and orientation
- B.** Place the client in seclusion to decrease environmental stimulation
- C.** Stay with the client and obtain immediate assistance for urgent treatment of severe alcohol withdrawal
- D.** Explain that the visual hallucinations are expected and will pass without treatment

48. A client with bipolar disorder who is experiencing mania is in the dayroom speaking loudly, moving from table to table, and interrupting conversations between other clients. The client is smiling, not threatening anyone, and does not respond when peers ask for quiet. Which nursing intervention is most appropriate?

- A.** Tell the client that the behavior is rude and that privileges will be limited if it continues
- B.** Escort the client to a quieter area and offer a brief, simple activity with minimal stimulation
- C.** Ask several clients to help redirect the client back to a seat in the dayroom
- D.** Explain in detail why the client should respect the boundaries of other clients

49. A client scheduled for cardiac catheterization later today says to the nurse, "I know they explained it, but all I heard was that something could go wrong." Which response by the nurse is most therapeutic?

- A.** "There is no reason to worry because complications are rare."
- B.** "Tell me more about what concerns you most about the procedure."

C. "You need to stay calm so your blood pressure does not go up."

D. "The provider already explained everything, so try to focus on the benefits."

50. A client preparing for discharge says, "When I get stressed, I tell myself I have to finish every problem right away or I am failing." Which nursing response is most appropriate to promote healthy stress management?

- A.** Try to avoid thinking about stressful tasks until you feel calmer.
- B.** Pick one realistic task to complete first and remind yourself that not everything has to be solved at once.
- C.** Use self-criticism to push yourself to stay productive during stressful times.
- D.** Wait until all of your responsibilities are listed before starting any of them.

51. A client recovering from abdominal surgery tells the practical nurse, "My faith teaches that serious illness should be faced with acceptance, so I am thinking about refusing any more pain medicine." Which response by the nurse is most appropriate?

- A.** "If you refuse pain medicine, the provider will need to be told right away."
- B.** "Can you tell me more about what accepting illness means to you and how you feel pain medicine fits with your beliefs?"
- C.** "Pain medicine is necessary after surgery, so you should not connect it with spiritual beliefs."
- D.** "Many people change their beliefs during illness, so this may not matter later."

52. A nurse is collecting a medication history from a client who recently moved from another country. The client presents a small bag of unlabeled tablets and says, "I take these when my stomach feels out of balance. They are common in my community." Which nursing action is most appropriate?

- A.** Tell the client to stop taking the tablets immediately because they are not prescribed
- B.** Ask the client to describe how often the tablets are used and notify the provider and pharmacist about them
- C.** Set the tablets aside and continue the admission because nonprescription products are not part of the medication record
- D.** Explain that hospital treatment must replace all cultural health practices during the admission

53. A client who identifies as nonbinary is admitted for treatment of severe migraine. During the admission interview, the client says, "I use they/them pronouns, but please don't include that in bedside shift report if my roommate can hear. I don't want to discuss my gender identity publicly." Which action by the nurse is most appropriate?

- A.** Document the client's request and communicate it privately to the care team while using discretion during bedside report
- B.** Explain that all personal information must be discussed openly during bedside report for safety
- C.** Avoid documenting the client's pronouns so the information cannot be overheard by others
- D.** Tell the client that staff will decide when pronouns should be discussed based on unit routine

54. A client with Ménière disease tells the nurse, "When the room starts spinning, I'm scared to leave my room because I might fall in front of everyone. I don't want people watching me like I'm helpless." Which nursing response is best?

- A.** You should stay in your room until the dizziness is completely gone.
- B.** Let's talk about what situations make you feel most anxious and plan ways to ask for help while keeping as much independence as possible.
- C.** Most people with dizziness get used to it, so try not to focus on it.
- D.** If you use the call light every time you feel dizzy, the embarrassment should go away.

55. A client receiving end-of-life care says to the nurse, "My family keeps asking me to decide who gets what, and I just feel overwhelmed. I want to talk with them, but not all at once." Which nursing action is most appropriate?

- A.** Tell the family they should stop discussing personal belongings until after the client's death
- B.** Ask the client whom they want present and help arrange a calm conversation with only those individuals
- C.** Encourage the client to make all decisions immediately so the family will have clear answers
- D.** Advise the client to let the health care provider handle the discussion with the family

56. A nurse is caring for a client with a mature gastrostomy tube who receives scheduled enteral feedings. Before the feeding, the nurse attempts to flush the tube with water and meets resistance. Which action should the nurse take first?

- A.** Force the water through the tube using a smaller syringe
- B.** Attempt to clear the tube by gently flushing with warm water
- C.** Insert a sylet into the tube to remove the blockage

D. Begin the feeding slowly to see whether the tube will open

57. A nurse is assisting a client who has left-sided weakness to climb stairs using a cane. Which instruction should the nurse give the client?

- A.** Advance the cane and weaker left leg first when going up the stairs
- B.** Lead with the stronger right leg when going up the stairs
- C.** Place the cane on the upper step before moving either leg down the stairs
- D.** Lead with the weaker left leg when going down the stairs

58. A nurse is reinforcing teaching for a client who is prescribed warfarin. Which instruction about nutrition is most appropriate?

- A.** Avoid all foods that contain vitamin K
- B.** Keep intake of vitamin K-rich foods consistent from week to week
- C.** Increase grapefruit juice intake to help the medication work better
- D.** Take the medication only on days when green vegetables are not eaten

59. A client with extensive sunburn to the upper back and shoulders reports burning pain. The skin is red and intact, with no blisters or open areas. Which nonpharmacologic intervention should the nurse implement?

- A.** Apply a warm moist compress to the affected areas
- B.** Place a cool, damp cloth over the sunburned skin
- C.** Massage the reddened skin with lotion using firm pressure
- D.** Keep the affected skin covered with a heating pad

60. A nurse is caring for a client who has heart failure and reports becoming exhausted while getting dressed each morning. The client says, "I can do it myself, but I have to stop several times to catch my breath." Which nursing intervention best supports the client's independence with this activity of daily living?

- A.** Encourage the client to finish dressing as quickly as possible before fatigue worsens
- B.** Lay out clothing and encourage the client to sit and rest between steps while dressing
- C.** Dress the client each morning to prevent overexertion
- D.** Ask the client to remain on bed rest until the provider reevaluates activity tolerance

61. A nurse is caring for a client who has a bowel-training program after a recent hip fracture. To take advantage of the body's normal pattern for defecation, when should the nurse assist the client to the bedside commode each day?

- A.** Immediately before bedtime

- B. At a consistent time after breakfast
 - C. Only when the client reports an urgent need to defecate
 - D. Every 15 minutes until a bowel movement occurs
- 62.** A nurse is planning nighttime care for a client who has been prescribed 2 hours of uninterrupted sleep as part of recovery after a painful procedure. Which action is best to help meet this goal?
- A. Offer fluids and assist the client to the bathroom every 30 minutes during the night
 - B. Cluster vital signs, hygiene, and medication administration before the client goes to sleep
 - C. Wake the client at regular intervals to ask about pain level changes
 - D. Keep the television on at low volume to distract the client from discomfort
- 63.** A practical nurse is caring for a client who has been sitting in a wheelchair for several hours. During skin assessment, the nurse notes a blister-like area with partial loss of skin on the client's coccyx. The wound bed is pink and moist, with no slough visible. Which description best matches this finding?
- A. Stage 1 pressure injury
 - B. Stage 2 pressure injury
 - C. Stage 3 pressure injury
 - D. Deep tissue pressure injury
- 64.** A practical nurse is caring for a client who has glaucoma with markedly reduced peripheral vision. Which action is most appropriate to improve safety in the client's room?
- A. Place frequently used items within the client's direct line of sight and keep pathways uncluttered
 - B. Approach the client quickly from the side to avoid startling the client from the front
 - C. Rearrange the furniture each shift so the client learns the room layout more carefully
 - D. Keep the room dim at all times to reduce visual strain
- 65.** A practical nurse is assisting with a 24-hour urine collection that starts at 6:00 a.m. The client voids at 5:50 a.m., again at 6:15 a.m., and again at 5:45 a.m. the next day. Which action should the nurse take to collect the specimen correctly?
- A. Save the 5:50 a.m. urine, then collect all urine through 5:45 a.m. the next day
 - B. Discard the 5:50 a.m. urine, then collect all urine through 5:45 a.m. the next day
 - C. Discard the 6:15 a.m. urine, then begin collecting with the next void
 - D. Collect urine for exactly 24 hours, but discard the final void at 5:45 a.m. the next day
- 66.** A nurse is preparing to administer furosemide 40 mg PO to a client at 0900. Before giving the medication, the nurse notes the client's blood pressure is 88/56 mm Hg and the client reports dizziness when standing. Which action is most appropriate?
- A. Administer the medication and document the blood pressure afterward
 - B. Withhold the medication temporarily and notify the provider of the findings
 - C. Give the medication with extra water to reduce dizziness
 - D. Ask the client to remain in bed and administer the medication as prescribed
- 67.** A practical nurse is admitting a client with pneumonia and completing medication reconciliation. The client states, "I take my prescription pills every day, plus a calcium supplement, fish oil, and an ointment I put on my knee when it hurts." None of the nonprescription items appear on the admission medication list. Which action should the nurse take?
- A. Document only the prescription medications because supplements and topical products are not part of medication reconciliation
 - B. Add the calcium supplement, fish oil, and topical ointment to the medication history and report them according to facility process
 - C. Tell the client to stop all nonprescription products while hospitalized unless the provider asks about them
 - D. Wait until discharge planning to update the list because home products are not needed during admission
- 68.** A nurse is caring for a patient with bacterial conjunctivitis who has a prescription for an antibiotic solution intended to treat the infection directly at the site. Which route of administration should the nurse use?
- A. Otic
 - B. Ophthalmic
 - C. Intranasal
 - D. Topical transdermal
- 69.** A client with leukemia is scheduled to receive platelet transfusion therapy. Which assessment finding would best indicate that the transfusion achieved its intended effect?
- A. The client's hemoglobin rises from 7.8 g/dL to 9.2 g/dL
 - B. The client's petechiae decrease and bleeding from the gums stops
 - C. The client's serum albumin increases from 2.8 g/dL to 3.5 g/dL
 - D. The client's INR decreases from 2.1 to 1.1

70. A nurse needs to infuse 90 mL of IV medication over 45 minutes using microdrip tubing calibrated at 60 gtt/mL. How many drops per minute should the nurse regulate?

- A. 60 gtt/min
- B. 90 gtt/min
- C. 120 gtt/min
- D. 180 gtt/min

71. A nurse is preparing to administer the next dose of oral oxycodone to a client 2 hours after abdominal surgery. Which pre-administration finding is most important for the nurse to review?

- A. Respiratory rate of 10/min
- B. Pain rating of 7 on a 0-to-10 scale
- C. Blood pressure of 138/84 mm Hg
- D. Temperature of 37.3°C (99.1°F)

72. A client with chronic stable angina began taking a prescribed transdermal nitroglycerin patch 1 week ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A. The client reports fewer episodes of chest pain during usual activity
- B. The client has a mild headache after applying the patch
- C. The client's heart rate increases from 72/min to 96/min after ambulation
- D. The client reports a tingling sensation under the patch

73. A client with glaucoma has been using timolol eye drops for 1 week. Which new finding should the nurse report to the provider immediately as a possible systemic adverse response to this medication?

- A. Burning in the eyes for a few seconds after instillation
- B. Heart rate of 52/min with new dizziness
- C. Mild tearing after applying the drops
- D. Blurred vision lasting 1 to 2 minutes after administration

74. A patient taking warfarin for a history of deep vein thrombosis tells the nurse, "I started an antibiotic 4 days ago, and now my gums bleed when I brush my teeth." Which antibiotic should the nurse recognize as the most likely cause of a clinically significant drug interaction?

- A. Amoxicillin
- B. Nitrofurantoin
- C. Ciprofloxacin
- D. Cephalexin

75. A client with bipolar disorder is prescribed lithium carbonate 300 mg by mouth three times daily. Before giving the scheduled dose, the nurse reviews the latest laboratory results and notes a serum lithium level of 1.8 mEq/L. Which action should the nurse take?

- A. Administer the dose with food and recheck the level at the next scheduled draw
- B. Hold the medication and notify the provider of the critical result
- C. Give the dose and encourage increased sodium restriction
- D. Administer the medication and document that the level is within the expected therapeutic range

76. A nurse is teaching a client who is starting metformin for newly diagnosed type 2 diabetes mellitus. Which instruction should the nurse include?

- A. Take the medication on an empty stomach first thing in the morning.
- B. Stop taking the medication if your blood glucose becomes normal for 2 days.
- C. Take the medication with meals to reduce stomach upset.
- D. Expect the medication to cause weight gain within the first week.

77. A nurse is preparing to administer an IV medication that was just reconstituted from a powdered vial. The medication guide states, "Use within 1 hour of reconstitution." The nurse discovers the vial was mixed 2 hours ago and left at room temperature. The solution looks clear. Which action should the nurse take?

- A. Administer the medication because the solution has no visible change
- B. Refrigerate the vial for 15 minutes, then administer it
- C. Discard the medication and prepare a new dose
- D. Shake the vial and administer only if no particles appear

78. A nurse is preparing to administer trimethoprim-sulfamethoxazole 160 mg/800 mg by mouth. The available oral suspension is labeled 40 mg/200 mg per 5 mL. How many milliliters should the nurse administer for one dose?

- A. 10 mL
- B. 15 mL
- C. 20 mL
- D. 25 mL

79. A practical nurse is collecting data on four clients in a long-term care facility. Which client should the nurse identify as being at greatest risk for falls?

- A. A client with a history of cataracts who uses the call light for assistance before walking

- B.** A client newly started on a diuretic who reports urgency and needs help getting to the bathroom at night
- C.** A client with hearing loss who wears bilateral hearing aids and ambulates with a steady gait
- D.** A client with chronic arthritis pain who asks for prescribed analgesia before participating in physical therapy
- 80.** A nurse is collecting a capillary blood glucose specimen from an adult client. Which action is most appropriate to help obtain an accurate result?
- A.** Use the first drop of blood for the test strip
- B.** Puncture the center of the fingertip pad
- C.** Wipe away the first drop of blood before applying the sample
- D.** Squeeze the finger firmly until a large drop forms
- 81.** A nurse is reviewing preoperative laboratory results for a client scheduled for abdominal surgery later today. Which finding should the nurse report to the provider first?
- A.** Platelet count 48,000/mm³
- B.** Hemoglobin 10.8 g/dL
- C.** WBC count 11,200/mm³
- D.** Sodium 134 mEq/L
- 82.** A client who is 28 weeks pregnant comes to the prenatal clinic and says, "Since this morning, my baby has barely been moving, even after I drank juice and lay down." Which action should the nurse take first?
- A.** Instruct the client to begin daily fetal kick counts at home and return in 1 week
- B.** Notify the provider and prepare for prompt fetal assessment
- C.** Reassure the client that fetal movement often varies from day to day in the third trimester
- D.** Ask the client to increase fluid intake and call back tomorrow if movement does not improve
- 83.** A nurse performs a bedside urine ketone test for a client with nausea, vomiting, and a history of diabetes. The reagent pad turns a darker color than expected, but the client reports no symptoms suggesting ketoacidosis. Which action is best for the nurse to take next?
- A.** Document the result as negative because the client has no symptoms
- B.** Repeat the test using a fresh strip from a properly stored container and compare the result with the color chart at the correct time
- C.** Add water to the urine specimen and repeat the test to reduce concentration error
- D.** Send the same dipstick to the laboratory for confirmation
- 84.** A practical nurse is obtaining routine vital signs on an adult client. The client says, "I just walked back from the bathroom, and my heart feels like it's racing." Which action is most appropriate to obtain the most accurate pulse and blood pressure readings?
- A.** Measure the pulse and blood pressure immediately while the client is standing
- B.** Have the client rest quietly for several minutes, then recheck the pulse and blood pressure
- C.** Ask the client to take several deep breaths, then measure the pulse and blood pressure right away
- D.** Document the elevated readings as the client's baseline because activity commonly raises vital signs
- 85.** A client is scheduled for a bedside thoracentesis. Which action is most important for the nurse to take before the procedure?
- A.** Ask the client to drink extra fluids to thin respiratory secretions
- B.** Verify that informed consent has been obtained
- C.** Place the client flat in bed for 30 minutes
- D.** Administer an opioid analgesic routinely before the provider arrives
- 86.** A nurse is obtaining a bedside ECG rhythm strip for a client and notices a pattern of small, uniform, rapid spikes across the tracing. The client is brushing their teeth with an electric toothbrush during the recording. Which action should the nurse take first?
- A.** Ask the client to stop using the toothbrush and repeat the tracing
- B.** Reposition the precordial leads to different intercostal spaces
- C.** Document ventricular fibrillation and notify the provider immediately
- D.** Increase the paper speed to better distinguish the rhythm
- 87.** A practical nurse is caring for a client who is 10 hours postoperative after a colon resection. The client suddenly reports a popping sensation at the incision site after coughing. The nurse notes a large amount of pink drainage on the dressing and sees loops of bowel protruding from the wound. Which action should the nurse take first?
- A.** Apply sterile saline-moistened dressings over the wound and notify the RN immediately
- B.** Reinforce the dressing with abdominal pads and reassess in 15 minutes
- C.** Gently attempt to place the bowel back into the incision
- D.** Position the client in high Fowler's position to improve breathing

88. A client with a nasogastric tube for gastric decompression has a prescription for oral medications. Which action by the nurse is most appropriate?

- A.** Crush the medications and instill them through the nasogastric tube, then reconnect the tube to suction immediately
- B.** Clamp the suction, administer the medications through the tube as appropriate, flush the tube, and leave suction off for a period as prescribed or per agency policy
- C.** Mix all medications together with the tube-feeding formula to reduce the number of tube flushes needed
- D.** Remove the nasogastric tube before giving the medications and reinsert it after administration

89. A nurse is caring for a client with a history of peptic ulcer disease who suddenly reports black, tarry stools and dizziness when standing. Which focused assessment should the nurse perform first?

- A.** Check the client's blood pressure and pulse, including orthostatic changes if possible
- B.** Ask the client to describe usual bowel habits over the past month
- C.** Inspect the abdomen for striae and old surgical scars
- D.** Review the client's dietary intake for the previous 24 hours

90. A client with asthma received a nebulized albuterol treatment 20 minutes ago for wheezing and shortness of breath. Which finding best indicates the treatment was effective?

- A.** The client reports less chest tightness and bilateral breath sounds are clear to auscultation
- B.** The client has a heart rate increase from 88/min to 112/min
- C.** The client becomes drowsy and wants to rest quietly in bed
- D.** The client coughs up a small amount of thick yellow sputum

91. A client with a history of cirrhosis becomes increasingly confused during the evening shift. The nurse notes slurred speech, drowsiness, and a flapping hand tremor when the client extends the arms. Which action should the nurse take first?

- A.** Administer the prescribed lactulose
- B.** Place the client in high-Fowler position
- C.** Ask the client to rate the abdominal pain
- D.** Encourage the client to drink more water

92. A client with ulcerative colitis has had frequent loose stools for several weeks and now reports increasing fatigue. Which finding is most important for the practical nurse to report to the nurse immediately?

- A.** Dry, cracked lips

B. Potassium level of 2.9 mEq/L

C. Mild abdominal cramping before bowel movements

D. Weight loss of 2 lb over the past month

93. A client with bacterial pneumonia has thick secretions and a weak cough. The provider prescribes oropharyngeal suctioning as needed. Which nursing action is most appropriate when performing this procedure?

- A.** Apply suction continuously while inserting the catheter to remove secretions quickly
- B.** Limit each suction pass to about 10 to 15 seconds and oxygenate the client before and after suctioning
- C.** Instill 10 mL of normal saline into the mouth before each suction pass to loosen secretions
- D.** Suction on a fixed schedule every hour whether secretions are present or not

94. A nurse is caring for a client 48 hours after tracheostomy placement. The provider's prescription states to suction the tracheostomy as needed. Which assessment finding best indicates that suctioning is needed now?

- A.** The client reports incisional discomfort when swallowing
- B.** The client has coarse breath sounds and visible secretions at the tracheostomy opening
- C.** The client has a blood pressure of 138/84 mm Hg
- D.** The client asks for assistance writing on a communication board

95. A client has been receiving mechanical ventilation for 24 hours through an endotracheal tube. Which nursing action is most appropriate to help detect a complication of prolonged immobility related to ventilator support?

- A.** Assess the calves for warmth, swelling, and tenderness
- B.** Measure intake and output every 8 hours
- C.** Auscultate bowel sounds once per shift
- D.** Check the client's pupils every 4 hours

96. A client with a temporary transvenous pacemaker is being monitored after insertion for complete heart block. The nurse notes that each pacing spike is followed by a QRS complex on the monitor. The client's heart rate is 80/min, blood pressure is 118/72 mm Hg, skin is warm, and the client denies dizziness. How should the nurse interpret this finding?

- A.** The pacemaker is failing to capture
- B.** The pacemaker is functioning appropriately
- C.** The pacemaker is failing to sense intrinsic activity
- D.** The client is developing pacemaker-induced dysrhythmia

97. A nurse is caring for a client who has had a colostomy for 6 weeks. The client says, "I feel embarrassed that gas from the ostomy may happen in public." Which nursing response is most appropriate?

- A.** "You should avoid all high-fiber foods so the ostomy will stop producing gas."
- B.** "A pouch with a deodorizing filter can help manage gas and make social situations easier."
- C.** "You need to keep the pouch completely full before emptying it so gas will not escape."
- D.** "This usually means the ostomy is not working correctly and should be reported immediately."

98. A nurse is caring for a client 18 hours after a transurethral resection of the prostate who has been receiving continuous bladder irrigation. Which change in condition should the nurse recognize as the priority?

- A.** The urine in the drainage tubing becomes light pink after previously being clear yellow
- B.** The client reports bladder spasms and rates the discomfort as 6 out of 10
- C.** No urine is draining into the collection bag for 30 minutes, and the client has suprapubic distention
- D.** The client asks why small tissue fragments are visible in the drainage bag

99. A nurse is caring for a client who has syndrome of inappropriate antidiuretic hormone secretion (SIADH). Which finding best indicates that treatment has been effective?

- A.** Urine specific gravity increases from 1.020 to 1.035
- B.** Serum sodium level rises from 124 mEq/L to 136 mEq/L
- C.** Urine output decreases from 45 mL/hr to 20 mL/hr
- D.** Body weight increases by 0.9 kg (2 lb) in 24 hours

100. A client with end-stage kidney disease is receiving hemodialysis and suddenly reports muscle cramps in both legs. Which intervention is most appropriate for the practical nurse to take first?

- A.** Slow the ultrafiltration rate as prescribed or notify the RN/provider to reduce fluid removal
- B.** Encourage the client to drink a large glass of water
- C.** Place a heating pad directly over the cramping muscles
- D.** Have the client stand and walk to improve circulation

Simulation Test 20 — Answer Key

1. B	26. C	51. B	76. C
2. C	27. A	52. B	77. C
3. C	28. C	53. A	78. C
4. B	29. B	54. B	79. B
5. C	30. B	55. B	80. C
6. B	31. C	56. B	81. A
7. D	32. B	57. B	82. B
8. B	33. A	58. B	83. B
9. B	34. A	59. B	84. B
10. A	35. C	60. B	85. B
11. B	36. A	61. B	86. A
12. A	37. A	62. B	87. A
13. C	38. B	63. B	88. B
14. A	39. B	64. A	89. A
15. C	40. B	65. B	90. A
16. B	41. B	66. B	91. A
17. C	42. C	67. B	92. B
18. C	43. B	68. B	93. B
19. C	44. C	69. B	94. B
20. B	45. A	70. C	95. A
21. C	46. C	71. A	96. B
22. A	47. C	72. A	97. B
23. C	48. B	73. B	98. C
24. C	49. B	74. C	99. B
25. C	50. B	75. B	100. A

Simulation Test 21

100 questions

1. A practical nurse in an urgent care clinic is helping manage several clients who arrive within minutes of one another. Which client should the nurse direct for immediate evaluation?

- A.** An adult with seasonal allergies requesting a refill of an antihistamine after running out yesterday
- B.** A teenager with a sore throat, fever of 100.2°F (37.9°C), and difficulty swallowing liquids since this morning
- C.** An older adult with chest pressure, cool clammy skin, and shortness of breath that began 20 minutes ago
- D.** A school-age child with an itchy rash on both arms after playing outside

2. The practical nurse is caring for clients on a medical unit. After returning from lunch, the nurse receives these updates. Which situation should the nurse address first?

- A.** A client receiving packed red blood cells 10 minutes ago reports chills and low back pain
- B.** A client with pneumonia has a temperature of 101.8°F (38.8°C) and is due for the next dose of antibiotic in 1 hour
- C.** A client with heart failure asks for help to the bathroom because the nursing assistant is busy
- D.** A client who started a clear liquid diet after surgery reports nausea after drinking apple juice

3. A practical nurse delegated ambulation of a stable postoperative client to an assistive personnel (AP) after confirming the client had already walked safely once with nursing staff. Ten minutes later, the AP reports, "The client became short of breath halfway down the hall and wants to go back to bed." Which action by the practical nurse is best?

- A.** Instruct the AP to return the client to bed and try again in 1 hour
- B.** Ask the AP how far the client walked, then document the activity
- C.** Assess the client immediately for respiratory status and tolerance of activity
- D.** Notify the provider that the delegated task could not be completed

4. A practical nurse is supervising an assistive personnel (AP) on a postoperative unit. Which assignment is most appropriate for the practical nurse to delegate to the AP?

- A.** Teach a client how to use a walker before the first assisted walk
- B.** Change the dressing on a client's abdominal incision
- C.** Help a stable client who has already been assessed transfer from bed to chair

D. Decide whether a client's report of incisional pain is an expected finding

5. A nurse is caring for an alert adult client who is scheduled for a lumbar puncture in 30 minutes. The client says, "I know I signed the consent earlier, but I still do not understand why this test is being done." Which action by the nurse is best?

- A.** Explain the purpose, risks, and alternatives of the procedure in detail so the test can stay on schedule
- B.** Notify the provider that the client has questions and delay the procedure until the client receives clarification
- C.** Ask a family member to help explain the procedure in simpler language
- D.** Reassure the client that the procedure is routine and encourage the client to proceed

6. A nurse is caring for an adult client who is alert, oriented, and has decision-making capacity. The client quietly tells the nurse, "I want to complete an advance directive, but I do not want my brother involved in that discussion." Later, the client's brother approaches the nurse and says, "I'm family, so I need to be in the room when you go over those papers." What is the nurse's best action?

- A.** Invite the brother into the discussion because family members commonly help with advance directives
- B.** Tell the brother he may participate only if the client gives permission
- C.** Provide the advance directive forms to the brother and ask him to review them with the client
- D.** Delay the discussion until the provider can decide whether the brother should be included

7. A practical nurse is preparing to leave a voicemail for a client about a follow-up appointment. Which message best protects the client's privacy?

- A.** This is the clinic calling about your abnormal test result. Please call us back today.
- B.** This is Jordan from the medical office. Please return our call at 555-0184.
- C.** You missed your diabetes follow-up visit, and the provider wants to adjust your medication.
- D.** This is the pulmonary clinic calling to discuss your chest x-ray and treatment plan.

8. A practical nurse is charting on a client assigned to the nurse and notices a new consult note from behavioral health. The nurse is curious whether the client has a history of substance use but does not need that information to complete the current wound care task. Which action is most appropriate?

- A.** Open and read the consult note because all information in the chart is available to any nurse caring for the client
- B.** Ask another nurse to summarize the consult note so the practical nurse does not access it directly

C. Review only the parts of the record needed for the current assignment and avoid opening information not needed for care

D. Print the consult note and place it in the paper chart in case the provider asks about it later

9. A nurse is caring for a client who has decision-making capacity and says, "I want to complete an advance directive, but I'm not sure what kinds of things should be discussed with the person I choose to speak for me." Which guidance by the nurse is most appropriate?

A. Tell the chosen person only where the document will be stored

B. Discuss values, treatment wishes, and situations in which the person may need to make decisions

C. Ask the chosen person to decide later without any prior conversation to avoid pressure

D. Wait until a serious decline occurs before talking about care preferences

10. A client is alert, oriented, and able to make decisions. The client tells the nurse, "I want to update my advance directive because my preferences have changed since I wrote it." The client's spouse says, "No, keep the old one. You already signed it." Which action by the nurse is most appropriate?

A. Tell the client the previous directive must stay in effect until reviewed by the family

B. Ask the provider to decide whether the client is allowed to make changes

C. Support the client's request to revise the directive and follow facility policy for updating the document

D. Postpone any changes until the client is discharged to avoid conflict

11. A nurse is caring for a client who is scheduled for an elective cataract extraction later that morning. The consent form was signed yesterday. In the preoperative area, the client says, "My vision is so blurry now that I can't read this form, but that's okay. My neighbor already told me what usually happens with this surgery." Which action should the nurse take?

A. Ask the neighbor to review the procedure with the client and confirm the consent

B. Witness the client's signature again because the client still wants the surgery

C. Notify the provider that the client needs the procedure, risks, benefits, and alternatives explained again before proceeding

D. Tell the client that signing the form yesterday is enough, so no further action is needed

12. A client is being discharged after hospitalization for a new ileostomy. The client can empty the pouch independently but says, "I'm worried about paying for supplies after the first box runs out." The prescribed medications have been filled, and the client has transportation to the follow-up visit. Which action by the nurse is most appropriate before discharge?

A. Arrange a referral to a social worker or case manager to help the client obtain ostomy supply resources

B. Instruct the client to reuse ostomy supplies as long as they appear clean

C. Tell the client to discuss supply costs at the follow-up visit after going home

D. Delay discharge until the client can pay for several months of supplies

13. A home health agency is trying to reduce incomplete wound-care supply deliveries to clients' homes. Review of recent incidents shows that missing supplies most often occur when the supply request form is filled out by hand and then re-entered later into the ordering system. The team is considering several next steps. Which action is most appropriate from a quality improvement perspective?

A. Require nurses to be more careful when handwriting supply requests

B. Implement a small test of a direct electronic ordering form for one nurse team and track missing-supply rates

C. Counsel the staff member involved in the most recent incomplete delivery

D. Wait for several more incidents before making any changes

14. A practical nurse is caring for a client with Parkinson disease who is being considered for discharge to home. During the shift, the nurse notes the client needs extra time to swallow pills, shuffles when turning, and requires hands-on help to get on and off the toilet. Which update is most important for the nurse to share with the interdisciplinary team?

A. The client prefers taking medications with applesauce instead of water

B. The client enjoyed the lunch tray and ate most of the meal

C. The client now needs hands-on help with toileting and shows slowed swallowing and unsteady turning

D. The client's spouse visited for 20 minutes in the afternoon

15. A practical nurse is caring for a client with newly diagnosed tuberculosis. During a busy shift, the client's neighbor stops the nurse in the hallway and says, "I saw my neighbor was admitted here. Is it true she has TB? I visit her elderly mother and need to know." Which response by the nurse is most appropriate?

- A.** Tell the neighbor that the client does have tuberculosis because it could affect the mother's health
- B.** Explain that client information is confidential and cannot be discussed without the client's permission
- C.** Ask the neighbor to wait while the nurse checks whether the client wants the information shared
- D.** Confirm only that the client has an infectious condition but avoid naming tuberculosis

16. A practical nurse in a medical-surgical unit is caring for an 82-year-old client who is alert and oriented. During a private conversation, the client says, "My daughter takes my Social Security check and won't let me see my bank statements. I need her help at home, so please don't tell anyone." Which action by the nurse is most appropriate?

- A.** Honor the client's request because the client is competent and does not consent to disclosure
- B.** Document the statement and report the suspected financial exploitation according to facility policy
- C.** Wait to report until the nurse can confirm unauthorized withdrawals with the bank
- D.** Ask the daughter for an explanation before deciding whether a report is needed

17. A practical nurse in a pediatric clinic suspects neglect after repeated missed follow-up visits for a child with poorly controlled asthma. Today, the child arrives wearing dirty clothing, has not had prescribed inhalers refilled for weeks, and the caregiver states, "We've just been too busy." Which action by the nurse best meets mandatory reporting responsibilities?

- A.** First gather additional evidence from the pharmacy to verify that no refill was obtained before reporting
- B.** Teach the caregiver about asthma management and delay reporting unless the pattern continues
- C.** Report the suspected neglect based on the observed pattern and document objective findings
- D.** Ask another nurse to decide whether the situation is serious enough to be reported

18. An assistive personnel (AP) is helping on a subacute unit and says to the practical nurse, "The client in room 214 was sweating and shaky before lunch, so I gave the client some orange juice." Which response by the nurse is best?

- A.** "Thank you for telling me. I will assess the client now and then review what actions you should report right away instead of handling independently."

- B.** "You did the right thing. Just make sure you document what you gave the client in the chart."
- C.** "Next time, check the client's blood glucose before giving anything to drink."
- D.** "Do not worry about it this time since the juice probably solved the problem."

19. A practical nurse (PN) is directing an assistive personnel (AP) during a busy shift on a long-term care unit. Which statement by the PN is the best example of clear delegation?

- A.** "Help the clients get cleaned up this morning."
- B.** "Take routine temperatures after breakfast and let me know right away if any reading is 100.4°F (38°C) or higher."
- C.** "Keep an eye on Mr. Diaz because he did not look well earlier."
- D.** "Do as much as you can for the new admission while I pass medications."

20. A practical nurse is discharging a client who was prescribed a low-sodium diet and daily weight monitoring for heart failure. The client says, "I stay at a shelter, so I eat whatever meals are served, and I do not have a scale." Which nursing action best promotes equitable access to care?

- A.** Tell the client to follow the instructions as closely as possible until the next appointment
- B.** Ask the client to repeat the discharge teaching to confirm understanding
- C.** Notify the supervising nurse or provider and request referral to available community resources for food and weight-monitoring support
- D.** Advise the client to buy a low-cost scale and avoid shelter meals that contain salt

21. At an outpatient clinic, a practical nurse reviews missed-appointment data for clients with newly diagnosed diabetes. Clients who need interpreter services have a much higher no-show rate than English-speaking clients. Staff notes show that reminder calls are made only in English, and interpreter requests are usually arranged only after the client arrives. Which action should the nurse recommend first to promote equitable access to care?

- A.** Have nurses spend extra time teaching clients who miss appointments when they eventually return
- B.** Implement a process to provide appointment reminders in the client's preferred language and arrange interpreter services before the visit
- C.** Schedule clients who need interpreters at the end of the day so delays affect fewer appointments
- D.** Ask family members to translate reminder calls when clinic staff cannot speak the client's language

22. A nurse is preparing to administer an insulin injection to a client in a long-term care facility. The client recognizes the nurse and says, "You know me—I'm in this room every day." The nurse notices the identification band is present and readable. Which action is best to verify the client's identity before giving the insulin?

- A.** Ask the client to state their full name and date of birth, and compare both with the identification band and medication record
- B.** Confirm the room number and ask a nursing assistant whether this is the correct client
- C.** Use the client's recognition of the nurse as adequate identification because the nurse cares for the client regularly
- D.** Check only the name on the identification band because the client is in a familiar setting

23. A nurse is preparing to use an electric hospital bed to reposition a client. When the nurse presses the control to raise the head of the bed, the bed makes a grinding sound and moves unevenly. The client is stable and not in immediate distress. Which action should the nurse take first?

- A.** Continue using the bed carefully because it is still functioning
- B.** Lower the bed, keep the client safe, remove the bed from use, and report the malfunction
- C.** Ask another staff member to hold the bed steady while the nurse repositions the client
- D.** Reposition the client quickly before the bed stops working completely

24. A nurse is preparing to administer the first dose of an influenza vaccine. During screening, the client says, "The last time I got a vaccine, my face swelled and I started wheezing within minutes." No vaccine allergy is documented in the record. Which action should the nurse take first?

- A.** Administer the vaccine and observe the client closely for 30 minutes
- B.** Hold the vaccine and notify the provider after documenting the reported reaction
- C.** Ask the client whether acetaminophen was taken before the previous vaccine
- D.** Give the vaccine in a different injection site than before

25. A practical nurse on a medical-surgical unit is checking client rooms after housekeeping waxed the hallway floor. The nurse notes these conditions: a call light cord hanging off one bed, an overbed table locked in place, a yellow "wet floor" sign posted at one end of the hall, and a client returning from the bathroom wearing nonskid socks and stepping onto a still-glossy section of floor. Which action should the nurse take first?

- A.** Help the client back to bed or to a dry area away from the slippery floor
- B.** Move the call light cord onto the bed within the client's reach
- C.** Unlock the overbed table so it can be repositioned later
- D.** Place an additional wet floor sign at the opposite end of the hallway

26. A client with advanced dementia is seated in a wheelchair after lunch and repeatedly tries to stand without locking the chair, creating a high risk for falling. The client is not aggressive and settles when a staff member sits nearby, offers a hand activity, and gives simple reminders to stay seated until assisted. Which nursing action best follows the principle of least-restrictive restraint use?

- A.** Apply a lap belt to keep the client in the wheelchair during the busy part of the shift
- B.** Move the client near the nurses' station and continue frequent observation and redirection
- C.** Raise all four side rails when the client is transferred back to bed
- D.** Request a sedative so the client will rest and stop trying to stand

27. A nurse is preparing to assist a client from bed to stretcher for transport when the nurse notices that the stretcher wheels are unlocked and the stretcher shifts slightly away from the bed. The client has not moved yet and is not harmed. Which action is most appropriate?

- A.** Proceed with the transfer quickly while holding the stretcher steady
- B.** Lock the stretcher wheels, reassess safety, and report the near-miss according to facility policy
- C.** Ask the client to wait while the transport team completes the transfer instead
- D.** Document that no incident occurred because the client was not injured

28. A nurse is selecting a transfer device for a client who needs to move from the bed to a bedside commode. The client is awake, follows directions, has good upper-body strength, and can bear weight briefly on both legs, but becomes fatigued quickly and is unsafe during a full pivot transfer. Which device is the safest choice?

- A.** Transfer board

- B. Sit-to-stand lift
- C. Lateral transfer device
- D. Full-body mechanical lift

29. A practical nurse is preparing to irrigate a client's chronic wound using clean technique. After setting up the supplies, the nurse puts on clean gloves and removes the old dressing. The nurse then removes the gloves and performs hand hygiene. Which action should the nurse take next to reduce the risk of wound contamination?

- A. Put on a new pair of clean gloves before cleansing and irrigating the wound
- B. Use the same gloves if no drainage is visible on them
- C. Ask an assistive personnel to handle the irrigation supplies
- D. Apply hand sanitizer over the bare hands and proceed without gloves

30. A nurse is preparing to discontinue a peripheral IV catheter for a client who is not on isolation precautions. The nurse expects brief contact with blood at the insertion site but does not anticipate splashing or soiling of clothing. Which personal protective equipment is most appropriate?

- A. Clean gloves only
- B. Clean gloves and a gown
- C. Clean gloves, mask, and goggles
- D. Sterile gloves only

31. A practical nurse is preparing a sterile field for a bedside procedure. Before opening the sterile package, the nurse notices the package has a small puncture hole near one corner, but the contents inside still appear dry and untouched. What is the best action?

- A. Use the package because the sterile contents were not directly touched
- B. Cover the punctured area with sterile tape and proceed
- C. Discard the package and obtain a new sterile item
- D. Open the package and use only the supplies farthest from the puncture

32. A practical nurse is caring for a client with suspected bacterial meningitis who has not yet started antimicrobial therapy. Which room assignment is most appropriate?

- A. Place the client in a private room and implement droplet precautions
- B. Place the client in a private room and implement airborne precautions
- C. Place the client with another client who has influenza and use contact precautions
- D. Place the client in a semi-private room if the curtain is kept closed

33. A practical nurse is preparing to give an intramuscular injection to a client who has no isolation orders. The client's skin is intact, and the nurse does not expect any blood splatter. Which action best reflects standard precautions?

- A. Wear clean gloves for the injection because all medication administrations require gloves
- B. Perform hand hygiene and give the injection without gloves unless contact with blood or body fluids is anticipated
- C. Put on a gown and mask before entering the room because injections break the skin
- D. Use sterile gloves to prevent introducing microorganisms during the injection

34. While starting a peripheral IV infusion for a client with no isolation orders, a practical nurse gets a small amount of the client's blood on an ungloved finger that has a fresh paper cut. Which action should the nurse take first?

- A. Wash the area thoroughly with soap and water
- B. Complete the IV dressing so the site is secured before addressing the exposure
- C. Apply an alcohol-based hand rub to the finger and continue the procedure
- D. Put on gloves to avoid further contamination before cleaning the wound

35. A nurse is reinforcing teaching with a client who is 26 weeks pregnant about warning signs of preterm labor. Which client statement indicates correct understanding?

- A. I should expect occasional low back pain as the uterus grows, so I do not need to report it.
- B. If I notice regular uterine tightening before 37 weeks, I should contact my provider.
- C. It is normal to have a small amount of bright red vaginal bleeding in the second trimester.
- D. I should wait until my next prenatal visit to report pelvic pressure if I do not have a fever.

36. A client in labor at 40 weeks' gestation has just had spontaneous rupture of membranes. The nurse auscultates the fetal heart rate and notes a sudden decrease from 140/min to 82/min that does not quickly return to baseline. Which action should the nurse take first?

- A. Turn the client to a side-lying position
- B. Prepare the client for immediate vaginal birth
- C. Perform a vaginal examination to assess for prolapsed cord
- D. Increase the rate of the primary IV infusion

37. A nurse is reinforcing discharge teaching for a client who is 2 days postpartum after an uncomplicated vaginal birth. The client asks what physical changes are expected over the next several weeks. Which statement by the client indicates correct understanding?

- A.** My uterus should return to its pre-pregnancy size by the end of the first week.
- B.** I should expect my vaginal discharge to gradually change from red to pink or brown, then to whitish.
- C.** My temperature will likely stay mildly elevated for the first 2 weeks after birth.
- D.** My abdominal muscles should feel back to normal within 2 or 3 days.

38. A nurse is reviewing developmental screening findings for several children at a community health fair. Which finding is most appropriate for a 15-month-old child?

- A.** Builds a tower of eight blocks
- B.** Walks without help and uses a cup well
- C.** Rides a tricycle and copies a circle
- D.** Prints first name and skips on one foot

39. A practical nurse is reinforcing teaching with a 66-year-old client who retired 3 months ago. The client says, "I thought retirement would feel relaxing, but instead I feel restless and useless now that I don't have my old routine." Which response by the nurse is most appropriate?

- A.** These feelings mean you are not adjusting well and should try to stay busy all day.
- B.** Many people need time to adjust to retirement, and finding new routines and meaningful activities can help.
- C.** You should avoid making any new commitments until these feelings go away on their own.
- D.** It is best to spend more time alone for a while so you can get used to the change.

40. A practical nurse is reinforcing teaching with a 47-year-old client who asks, "If screening tests are for people without symptoms, why would I need follow-up testing after an abnormal result if I still feel fine?" Which response by the nurse is best?

- A.** An abnormal screening result means the disease is already advanced.
- B.** Screening tests identify people who may need more testing to find out whether a condition is actually present.
- C.** If you do not have symptoms, repeating the screening later is better than getting more tests now.
- D.** Screening tests are mainly used to decide which treatments should be started right away.

41. A nurse is obtaining a health history from an adult client who says, "I get dizzy once in a while." Which follow-up question best helps the nurse assess the timing pattern of this symptom?

- A.** Can you describe what the dizziness feels like?
- B.** What were you doing when the dizziness started?
- C.** How often does the dizziness occur, and how long does it usually last?
- D.** What makes the dizziness better or worse?

42. A practical nurse is completing an admission assessment for an adult client with a right below-knee amputation. Which documentation best establishes baseline physical data for future comparison?

- A.** Client appears cooperative and answers questions appropriately
- B.** Skin warm and dry; respirations even and unlabored
- C.** Ambulates 20 feet with prosthesis and standby assist; gait slightly unsteady when turning
- D.** Reports family will bring personal belongings later today

43. A nurse is teaching a client who has heart failure about weighing themselves daily at home. The client says, "I forgot my glasses, and I can't make out the numbers on this handout or on that scale across the room." Which nursing action is best?

- A.** Speak louder while reviewing the written instructions word for word
- B.** Postpone all teaching until a family member can come to the hospital
- C.** Provide teaching using large-print materials and have the client practice reading a scale up close
- D.** Ask the client to sign the discharge instructions after the explanation is finished

44. A client who had a below-the-knee amputation 4 days ago tells the practical nurse, "I don't want my partner to see me like this. I'm afraid nothing will ever feel normal again." Which response by the nurse best provides emotional support?

- A.** You need to stay positive and focus on walking with a prosthesis later.
- B.** Many people adjust over time, so try not to worry too much right now.
- C.** It sounds like you're worried about how this change may affect your relationship and your future.
- D.** Your partner probably won't feel any differently once the incision heals.

45. A client who is 2 days postpartum tells the nurse, "I know I should be happy, but I feel overwhelmed. When the baby cries, I start thinking I'm already a terrible mother." Which nursing response is most appropriate to promote effective coping?

- A.** "You need to focus on the positive and stop thinking that way."
- B.** "Many new parents feel overwhelmed at first. Let's talk about what moments feel hardest and identify one support you can use today."
- C.** "If you keep feeling this way, the baby will pick up on your stress."
- D.** "Try to ignore those thoughts, and they should go away on their own."

46. A nurse in an outpatient mental health program is leading a small group when one client suddenly stands up, kicks a chair, and begins shouting, "Nobody here listens to me!" The client is breathing rapidly and moving toward another client. Which action should the nurse take first?

- A.** Tell the client that this behavior is unacceptable and that privileges will be restricted
- B.** Position the nurse near the exit and speak to the client in a calm, clear voice
- C.** Ask the other group members to explain why they disagree with the client
- D.** Move close to the client and place a hand on the client's shoulder

47. A client admitted for a gastrointestinal bleed tells the nurse, "I usually drink heavily every night, but I haven't had anything for about 18 hours." The client is anxious, diaphoretic, and has a noticeable hand tremor. Which prescription should the nurse anticipate as most appropriate to help prevent progression of withdrawal symptoms?

- A.** Lorazepam as needed based on withdrawal symptoms
- B.** Naloxone IV for respiratory support
- C.** Flumazenil IV to reverse sedation
- D.** Acetaminophen as needed for discomfort

48. A client with generalized anxiety disorder is waiting for a wound dressing change. The client begins breathing rapidly, wringing both hands, and repeatedly asking, "When are you going to start? I can't stand waiting." Which nursing action is most appropriate?

- A.** Explain that the procedure will begin soon and ask the client to stop interrupting
- B.** Guide the client to take slow deep breaths and give short, clear information about what will happen next
- C.** Leave the room for a few minutes so the client can calm down alone
- D.** Tell the client that the behavior is inappropriate and may delay care

49. A client newly diagnosed with Parkinson disease looks at the floor and says, "I'm afraid people will notice my shaking and stop treating me the same." Which response by the nurse is most therapeutic?

- A.** "You should try not to worry about what other people think."
- B.** "Many people adjust well after diagnosis, so this may not be as bad as it seems."
- C.** "It sounds like you're worried about how this diagnosis could affect the way others see you."
- D.** "Have you told your family exactly when the shaking started?"

50. A nurse is caring for a client who becomes upset when told a scheduled diagnostic test has been postponed. The client begins pacing near the doorway, speaks in a loud voice, and says, "If one more person lies to me, I'm going to explode." Which nursing action is most appropriate?

- A.** Stand directly in front of the doorway and firmly tell the client to lower their voice
- B.** Ask several questions about why the client is so angry in order to gather more information
- C.** Maintain a calm tone, give the client personal space, and say, "I can see you're upset. Let's move to a quieter area and talk."
- D.** Explain that the delay is unavoidable and remind the client that other clients are waiting too

51. A client newly diagnosed with cancer tells the practical nurse, "My tradition teaches that bad news should be discussed with my family first, not directly with me. I do not want to hear details until they are with me." Which action by the nurse is most appropriate?

- A.** Document the client's preference and inform the nurse in charge and health care team so communication can be planned accordingly
- B.** Explain that the client has a right to know everything immediately and begin reviewing the diagnosis
- C.** Ask the family to decide how much information the client should receive without involving the client further
- D.** Avoid discussing the diagnosis with anyone until the family arrives later in the day

52. A nurse is preparing to insert an indwelling urinary catheter for a client who says, "In my faith, I do not want intimate care from someone of the opposite sex." No same-gender nurse is immediately available, and the catheterization is needed soon but is not an emergency. Which action by the nurse is most appropriate?

- A.** Tell the client that hospital policy requires the available nurse to perform the procedure
- B.** Ask the client to explain the concern, attempt to arrange a same-gender caregiver, and discuss any delay with the provider if needed

- C.** Proceed with the catheterization because delaying treatment could increase the risk of urinary retention
- D.** Request that the client's family member assist with the catheterization to respect the client's preference

53. A client who identifies as transgender is admitted for treatment of asthma exacerbation. While reviewing the medication history, the nurse notes a hormone medication that is not listed on the admitting provider's initial orders. The client says, "I was afraid to mention it because people get weird about it." Which nursing response is best?

- A.** "You should wait and discuss that medication only with the provider during rounds."
- B.** "Tell me the name, dose, and schedule of the medication so I can document it and help make sure the provider is aware."
- C.** "If it is not related to your breathing problem, it is probably not important right now."
- D.** "Bring in the bottle later so we can decide whether you still need to take it."

54. A client with recent hearing loss tells the nurse, "When people repeat things louder and louder, I feel embarrassed and angry. Now I avoid talking to anyone because it's easier." Which response by the nurse is best?

- A.** You should let people know that raising their voice is helpful for you right now.
- B.** It sounds frustrating to feel singled out. Let's talk about communication approaches that may make conversations less stressful for you.
- C.** Try not to take it personally because most people are only trying to help.
- D.** If you stay around other people more often, you will probably get used to the problem.

55. A client with advanced cancer who is receiving end-of-life care tells the nurse, "My daughter flew in from another state, and everyone expects me to spend all day visiting. I love them, but I get exhausted and feel guilty when I ask for quiet." Which nursing response is most appropriate?

- A.** "You should try to stay awake as much as possible since your family came so far to see you."
- B.** "It sounds like you want time with your family and also need rest. Let's talk about ways to set short visits and quiet periods."
- C.** "Your family probably just does not understand how tired you are, so try not to take it personally."
- D.** "You need to tell them no visitors at all until you feel stronger."

56. A nurse is assisting a client who had a right-sided stroke to move up in bed. The client can follow directions but has limited strength on the right side. Which action by the nurse is best to move the client safely while reducing the nurse's risk of injury?

- A.** Place one arm under the client's shoulders and one under the hips, then lift the client toward the head of the bed
- B.** Lower the head of the bed, place the client in a supine position, and use a draw sheet with help from another staff member to slide the client up
- C.** Raise the head of the bed and ask the client to pull on the side rail while the nurse pushes from the foot of the bed
- D.** Turn the client onto the weak side first, then pull the client up by grasping under the axillae

57. A nurse is reinforcing teaching for a client who has celiac disease. Which food choice should the nurse recommend?

- A.** Scrambled eggs with fresh fruit
- B.** Whole-wheat toast with peanut butter
- C.** Barley soup with crackers
- D.** Pasta salad made with regular noodles

58. A client with sickle cell crisis reports severe generalized pain and appears tense and anxious. Analgesic medication has been administered as prescribed. Which nonpharmacologic nursing intervention is most appropriate to help reduce the client's pain?

- A.** Apply cold packs to the painful areas
- B.** Encourage guided relaxation and a calm, quiet environment
- C.** Massage the client's lower legs firmly
- D.** Assist the client to ambulate in the hallway every hour

59. A nurse is caring for a client with chronic low back pain who wants to put on shoes independently. The client can sit at the side of the bed but says, "I can't bend down that far without pain." Which nursing intervention best supports the client's independence with this activity of daily living?

- A.** Tell the client to remain on bed rest until the back pain is fully relieved
- B.** Put the client's shoes on for them each morning to prevent discomfort
- C.** Provide a long-handled shoehorn and encourage the client to use it while seated
- D.** Instruct the client to stand and lean forward quickly to reduce the time spent bending

60. A nurse is caring for a client who is 1 day postoperative and reports feeling the urge to have a bowel movement but is unable to pass stool. Which action is most appropriate to promote defecation without straining?

- A. Encourage the client to use a bedside commode or toilet for elimination
- B. Ask the client to remain in bed and use a bedpan after the next meal
- C. Limit oral fluids until bowel function returns to normal
- D. Instruct the client to bear down forcefully for 10 seconds at a time

61. A nurse is caring for four clients at bedtime. Which client should the nurse identify as having the greatest risk for a sleep disturbance related to medication effects?

- A. A client taking a bronchodilator in the evening
- B. A client applying a warm compress to the shoulder before sleep
- C. A client drinking warm milk with the evening snack
- D. A client using an extra pillow to elevate the legs

62. A practical nurse is caring for a bedbound client whose Braden Scale score indicates high risk for pressure injury. Which finding requires the nurse to revise the plan of care because it increases pressure on the sacral area?

- A. The head of the bed is elevated to 45 degrees for several hours
- B. A pillow is placed under the calves to float the heels
- C. The client is repositioned at regular intervals
- D. A draw sheet is used during repositioning

63. A practical nurse is caring for a client who has hearing loss and communicates best by speechreading. Which action by the nurse is most appropriate when giving instructions?

- A. Stand facing the client and speak clearly at a normal pace
- B. Speak loudly from the doorway so the client can hear better
- C. Cover the mouth while talking to reduce distractions
- D. Repeat the same words more quickly if the client seems confused

64. A nurse is calculating a client's intake and output for an 8-hour shift. The client drank 8 oz of coffee, 240 mL of juice, and 6 oz of water. The client also received 500 mL of IV fluid. Urine output was 900 mL. How many milliliters should the nurse document as the client's total intake for the shift?

- A. 920 mL
- B. 1,160 mL

C. 1,220 mL

D. 1,400 mL

65. A practical nurse is caring for a client who has been immobilized in bed for several days after a spinal injury. Which intervention is most appropriate to help prevent urinary stasis related to immobility?

- A. Encourage the client to limit fluid intake during the day
- B. Offer fluids regularly unless contraindicated
- C. Keep the client in a flat supine position as much as possible
- D. Insert an indwelling urinary catheter for convenience

66. A nurse is preparing to administer 0900 medications to a client. The nurse verifies the prescription, checks the MAR, and identifies the client using two identifiers. The client then says, "I have trouble swallowing big pills. Can this one be crushed in applesauce?" The medication is an extended-release tablet. Which action is most appropriate?

- A. Crush the tablet and mix it with applesauce so the client can take the full dose
- B. Split the tablet in half and give each half separately
- C. Hold the medication and contact the provider or pharmacist for a safe alternative
- D. Ask the client to swallow the tablet with extra water because it is scheduled for 0900

67. A practical nurse is reviewing the MAR before administering a client's 0800 medications. The MAR shows levothyroxine 75 mcg PO due at 0800. In the chart, the nurse finds a provider prescription entered at 0600 that states, "Increase levothyroxine to 100 mcg PO daily starting today." The pharmacy has not updated the MAR yet, and the client has not received the dose. Which action should the nurse take?

- A. Administer the 75 mcg dose listed on the MAR and report the discrepancy after the medication pass
- B. Hold the medication until the next day so the MAR and chart will match
- C. Clarify the discrepancy and follow agency procedure to have the MAR corrected before administering the medication
- D. Administer both the 75 mcg dose on the MAR and the additional 25 mcg to match the new prescription

68. A nurse is caring for a patient with severe nausea who cannot swallow. The provider prescribes a medication that should be placed in the cheek pouch so it can dissolve and be absorbed through the oral mucosa. Which route of administration should the nurse use?

- A. Buccal
- B. Sublingual
- C. Enteral

D. Topical

69. A client with liver failure is actively bleeding from the gums and has a platelet count of $220,000/\text{mm}^3$, hemoglobin of 11.8 g/dL, and prolonged PT/INR and aPTT. Which blood product would the nurse anticipate administering to replace multiple clotting factors?

- A.** Packed red blood cells
- B.** Fresh frozen plasma
- C.** Platelets
- D.** Cryoprecipitate

70. A nurse needs to administer 1,050 mL of IV fluid over 14 hours using tubing with a drop factor of 15 gtt/mL. How many drops per minute should the nurse regulate?

- A.** 16 gtt/min
- B.** 19 gtt/min
- C.** 22 gtt/min
- D.** 27 gtt/min

71. A practical nurse is preparing to administer a prescribed controlled opioid liquid to a client. The order is for 7.5 mg by mouth, and the available medication is 5 mg per 5 mL. After drawing up 7.5 mL, the nurse is interrupted and realizes there is no witness available for the remaining unused portion that must be wasted according to facility policy. Which action is most appropriate?

- A.** Administer the 7.5 mL dose and place the remaining medication aside until another nurse is available to witness the waste
- B.** Ask the client to verify that the remaining medication was discarded after the dose is given
- C.** Wait to administer the medication until the nurse can complete the required waste process according to policy
- D.** Return the unused portion to the original container after administering the ordered dose

72. A client with bipolar disorder began taking lithium 10 days ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client sleeps 6 to 7 hours at night and speech is less pressured
- B.** The client reports mild hand tremors after each dose
- C.** The client's serum sodium level is 130 mEq/L
- D.** The client has increased thirst and urinates more often

73. A client with a new prescription for enalapril receives the first dose on a medical unit. About 30 minutes later, the client says, "My lips feel strange." Which finding should the nurse identify as the highest-priority adverse response to report immediately?

- A.** Swelling of the lips and tongue

B. A dry, hacking cough

C. A blood pressure of 104/68 mm Hg

D. A mild headache rated 2 out of 10

74. A patient takes oral contraceptive pills daily for pregnancy prevention and has just been prescribed rifampin for tuberculosis. Which instruction should the nurse provide?

- A.** Continue both medications as prescribed and use a nonhormonal backup method during rifampin therapy
- B.** Stop the oral contraceptive pills while taking rifampin to avoid toxicity
- C.** Take the oral contraceptive pill 2 hours before rifampin to prevent the interaction
- D.** Increase the oral contraceptive dose on days rifampin is taken

75. A client with atrial fibrillation is prescribed warfarin 5 mg by mouth each evening. When reviewing the day's laboratory results before administration, the nurse notes an INR of 4.8. Which action is most appropriate?

- A.** Administer the dose as prescribed and recheck the INR in the morning
- B.** Hold the warfarin dose and notify the provider of the elevated INR
- C.** Give the warfarin with food to reduce the anticoagulant effect
- D.** Administer half the prescribed dose and document the change

76. A nurse is teaching a client who has a new prescription for tetracycline to treat acne. Which instruction should the nurse include?

- A.** Take the medication with milk to reduce stomach upset.
- B.** Stop taking the medication as soon as the skin begins to clear.
- C.** Avoid excessive sun exposure while taking this medication.
- D.** Crush the tablets and mix them with food if they are hard to swallow.

77. A nurse is preparing to administer an IV dose of furosemide using a client's existing peripheral line. The client is receiving total parenteral nutrition (TPN) through that line. After checking the compatibility reference, the nurse finds that furosemide is incompatible with the TPN solution. Which action should the nurse take?

- A.** Stop the TPN briefly, flush the line, administer the furosemide, and restart the TPN
- B.** Add the furosemide directly to the TPN bag to avoid wasting supplies
- C.** Administer the furosemide through a separate IV line or obtain another compatible access site

D. Slow the TPN rate and administer the furosemide through the same line at the same time

78. A nurse is preparing to administer ceftriaxone 1.5 g IV. The medication available is labeled 500 mg per 4 mL after reconstitution. How many milliliters should the nurse prepare for one dose?

- A.** 8 mL
- B.** 10 mL
- C.** 12 mL
- D.** 16 mL

79. A practical nurse is collecting data on four clients in an assisted-living clinic. Which client should the nurse identify as being at greatest risk for developing hypoglycemia?

- A.** A client with type 2 diabetes who took prescribed insulin and skipped breakfast because of nausea
- B.** A client with hypertension who reports eating a salty lunch
- C.** A client with osteoarthritis who walked for 10 minutes this morning
- D.** A client with chronic bronchitis who has a productive cough

80. A nurse is instructing a client about a 24-hour urine collection that will begin at 0700. Which statement by the client indicates a need for further teaching?

- A.** I will urinate at 0700 and discard that specimen before the collection starts.
- B.** I will save all urine I void for the next 24 hours in the collection container.
- C.** If I forget and miss one specimen, I should tell the nurse because the test may need to be restarted.
- D.** I should save the first urine I void at 0700 the next day to finish the collection.

81. A nurse is reviewing laboratory results for a client who has been receiving phenytoin for seizure control. The client is difficult to arouse and has new unsteady gait and slurred speech. Which laboratory result should the nurse report to the provider first?

- A.** Serum phenytoin level 24 mcg/mL
- B.** WBC count 10,200/mm³
- C.** Platelet count 220,000/mm³
- D.** Fasting blood glucose 108 mg/dL

82. A client who is 27 weeks pregnant comes to the prenatal clinic and says, "Since early this morning, my contractions have been coming every 8 to 10 minutes, and I feel a dull ache in my lower back." Which additional finding should the nurse identify as most concerning for preterm labor?

- A.** A small amount of thick, white vaginal discharge without odor

- B.** Pelvic pressure with pink-tinged mucus from the vagina
- C.** Mild ankle swelling at the end of the day
- D.** Urinary frequency without dysuria

83. A nurse is performing a bedside urine dipstick test on a freshly collected specimen. The nurse compares the reagent pads with the color chart 5 minutes after dipping because another client needed assistance. Which concern about the result is most important?

- A.** The urine is too old to test accurately
- B.** The strip may show invalid results because it was read outside the recommended time
- C.** The specimen is contaminated unless it was refrigerated first
- D.** The color chart is unreliable unless checked under fluorescent light only

84. A practical nurse is obtaining vital signs on an adult client with a history of chronic obstructive pulmonary disease (COPD). The client is resting in bed and appears comfortable. Which finding should the nurse report promptly to the provider?

- A.** Blood pressure 138/84 mm Hg
- B.** Temperature 99.1°F (37.3°C)
- C.** Oxygen saturation 88% on room air
- D.** Respiratory rate 10/min

85. A client is scheduled for a liver biopsy this morning. Which nursing action is most important to complete before the procedure?

- A.** Encourage the client to cough and deep-breathe every 2 hours
- B.** Verify that coagulation studies have been reviewed
- C.** Place the client in high-Fowler position
- D.** Offer a light breakfast 1 hour before the biopsy

86. A nurse is obtaining a telemetry rhythm strip for a client and sees a flat line across the monitor. The client is awake, talking, and has a palpable radial pulse of 82/min. Which action should the nurse take first?

- A.** Begin cardiopulmonary resuscitation immediately
- B.** Check the lead connections and the client's electrodes
- C.** Administer oxygen by face mask
- D.** Document the rhythm as asystole

87. A nurse is collecting a venous blood specimen from an adult client using an evacuated tube system. The blood is flowing steadily into the first tube. To help prevent reflux of tube additives into the client's vein, which action is most appropriate while changing tubes?

- A.** Hold the client's arm below heart level until all tubes are filled
- B.** Keep the tube stopper pointed upward while inserting each tube into the holder

- C.** Turn each filled tube upside down several times before attaching the next tube
- D.** Release the tube from the holder only after removing the needle from the vein

88. A client has a newly inserted nasogastric tube connected to low intermittent suction for gastric decompression. During rounds, the nurse finds that there has been no drainage for 2 hours, and the client reports nausea and increasing abdominal fullness. Which action should the nurse take first?

- A.** Advance the tube several inches to improve drainage
- B.** Irrigate the tube with sterile saline according to the prescription or facility policy
- C.** Increase the suction setting to restore tube function
- D.** Remove the tube and prepare for reinsertion

89. A nurse is caring for a client who is receiving a continuous heparin infusion for treatment of a deep-vein thrombosis. The client suddenly reports a severe headache and says, "My vision looks blurry." Which focused assessment should the nurse perform first?

- A.** Check the client's pupils, level of consciousness, and hand grasps
- B.** Inspect the client's gums for bleeding
- C.** Ask the client to rate the headache on a 0 to 10 scale
- D.** Assess the client's abdomen for bruising

90. A client with angle-closure glaucoma receives IV acetazolamide and prescribed ophthalmic medications. Which finding best indicates the treatment is effective?

- A.** The client reports decreased eye pain and blurred vision
- B.** The client has a heart rate of 104/min
- C.** The client reports a dry mouth
- D.** The client has urine output of 900 mL over 8 hours

91. A client with an indwelling urinary catheter has had no urine output for the past 2 hours. The client reports lower abdominal discomfort, and the nurse notes a firm, distended area over the bladder. Which action should the nurse take first?

- A.** Encourage the client to increase oral fluid intake
- B.** Irrigate the catheter with sterile solution as prescribed
- C.** Check the catheter tubing for kinks or dependent loops
- D.** Remove the catheter and insert a new one

92. A client with long-standing Parkinson disease is receiving carbidopa-levodopa three times daily. During a home visit, the practical nurse sees the client rising quickly from a chair, becoming unsteady, and grabbing the table for support. Which instruction is most appropriate?

- A.** Stop taking carbidopa-levodopa until the prescriber can be contacted

- B.** Rise slowly from sitting or lying positions before walking
- C.** Limit fluid intake during the day to reduce dizziness
- D.** Take the medication only at bedtime to prevent daytime symptoms

93. A client with asthma suddenly develops severe respiratory distress after exposure to cigarette smoke. The nurse notes audible wheezing, use of accessory muscles, and inability to speak more than one-word answers. A few minutes later, the wheezing becomes much quieter, and breath sounds are barely audible bilaterally. Which interpretation is most appropriate?

- A.** The bronchospasm is resolving because airflow is improving
- B.** The client is becoming exhausted and may be progressing to respiratory failure
- C.** The client is having a panic attack rather than an asthma exacerbation
- D.** Secretions are loosening, which commonly makes wheezing less noticeable

94. A nurse is caring for a client with a tracheostomy who has thick, dried secretions around the stoma and on the tracheostomy tube. Which nursing action is most appropriate to help loosen secretions and reduce crusting?

- A.** Apply sterile saline dressings tightly over the stoma opening
- B.** Increase humidification of inspired air as prescribed
- C.** Deflate the tracheostomy cuff for 30 minutes every shift
- D.** Clean the site less often to avoid irritating the skin

95. A client has been mechanically ventilated through an endotracheal tube for 6 hours. Which assessment finding most strongly suggests that the client is developing ventilator-associated pneumonia?

- A.** Blood pressure 88/54 mm Hg after initiation of propofol infusion
- B.** Temperature 38.6°C (101.5°F) with new purulent tracheal secretions
- C.** Heart rate 112/min during endotracheal suctioning
- D.** Oxygen saturation 93% while receiving 40% oxygen

96. A client with a temporary pacemaker is on continuous monitoring. The nurse notes regular pacer spikes on the rhythm strip, but one spike is not followed by a QRS complex. The client then reports feeling dizzy. Which finding does the nurse recognize?

- A.** Failure to sense
- B.** Failure to capture
- C.** Normal pacemaker function
- D.** Oversensing

97. A nurse is caring for a client with a new ostomy. Which finding should the nurse report to the provider immediately?

- A.** The stoma appears pale and dusky
- B.** The stoma is pink and moist
- C.** A small amount of edema is present around the stoma
- D.** The client reports mild tenderness near the incision

98. A nurse is caring for a client who was admitted with acute pyelonephritis and has been receiving IV antibiotics. Which change in condition should the nurse recognize as the priority to report immediately?

- A.** Urine output decreases to 25 mL/hr over 2 hours
- B.** The client reports nausea and has one episode of emesis
- C.** The client's temperature rises from 37.8°C (100.0°F) to 38.3°C (100.9°F)
- D.** The client reports flank pain increasing from 4 to 6 on a 0-to-10 scale

99. A nurse is caring for a client who has been taking digoxin and develops nausea, generalized weakness, and an irregular pulse after 3 days of poor oral intake. Which laboratory value should the nurse review first?

- A.** Serum potassium 2.9 mEq/L
- B.** Serum sodium 146 mEq/L
- C.** Serum calcium 8.8 mg/dL
- D.** Serum magnesium 2.0 mg/dL

100. A client with end-stage kidney disease who receives hemodialysis on Mondays, Wednesdays, and Fridays arrives for a Wednesday treatment. Which finding is most important for the nurse to expect before the client goes to dialysis?

- A.** Weight gain of 2.2 kg (4.8 lb) since Monday
- B.** Heart rate of 58/min after resting in bed
- C.** Complaints of mild itching on the back
- D.** Urine output of 250 mL over the last 24 hours

Simulation Test 21 — Answer Key

1. C	26. B	51. A	76. C
2. A	27. B	52. B	77. C
3. C	28. B	53. B	78. C
4. C	29. A	54. B	79. A
5. B	30. A	55. B	80. B
6. B	31. C	56. B	81. A
7. B	32. A	57. A	82. B
8. C	33. B	58. B	83. B
9. B	34. A	59. C	84. D
10. C	35. B	60. A	85. B
11. C	36. C	61. A	86. B
12. A	37. B	62. A	87. B
13. B	38. B	63. A	88. B
14. C	39. B	64. B	89. A
15. B	40. B	65. B	90. A
16. B	41. C	66. C	91. C
17. C	42. C	67. C	92. B
18. A	43. C	68. A	93. B
19. B	44. C	69. B	94. B
20. C	45. B	70. B	95. B
21. B	46. B	71. C	96. B
22. A	47. A	72. A	97. A
23. B	48. B	73. A	98. A
24. B	49. C	74. A	99. A
25. A	50. C	75. B	100. A

Simulation Test 22

100 questions

1. The practical nurse is organizing care at the start of a shift in a medical unit. Which client should the nurse check first?

- A.** A client with pneumonia whose oxygen saturation has dropped from 95% to 89% on room air
- B.** A client with a healed hip fracture asking for assistance to the bathroom
- C.** A client with chronic anemia who reports feeling tired after physical therapy
- D.** A client with cellulitis who is due for a routine dressing change in 30 minutes

2. A practical nurse in a pediatric office receives several calls just before lunch. Which caller should the nurse respond to first?

- A.** The parent of a 6-year-old with a sore throat who wants to know whether the child can return to school tomorrow
- B.** The parent of a 2-month-old who has had a rectal temperature of 100.8°F (38.2°C) and is difficult to wake for feeding
- C.** The parent of a 10-year-old with poison ivy who says the rash is still itchy after calamine lotion
- D.** The parent of a 14-year-old who forgot this morning's prescribed iron tablet

3. A practical nurse delegates intake and output measurement for a stable client to an assistive personnel (AP). At the end of the shift, the AP says, "I forgot to measure the client's urine after lunch, so I just estimated it in the record." Which action by the practical nurse is best?

- A.** Instruct the AP to leave the estimate because the exact amount is no longer available
- B.** Tell the AP to avoid mentioning the mistake so the team is not delayed
- C.** Document that the AP is unreliable and reassign all tasks for the next shift
- D.** Assess the client, correct the inaccurate documentation according to facility policy, and reinforce proper reporting to the AP

4. A practical nurse is working with an assistive personnel (AP) in a skilled nursing facility. Which task is most appropriate for the practical nurse to delegate to the AP?

- A.** Assess a resident's new complaint of burning with urination
- B.** Change a sterile dressing on a leg ulcer
- C.** Offer toileting assistance to a resident on a scheduled bladder-training program

D. Teach a resident how to use a newly prescribed walker

5. A nurse is caring for an alert adult client who has decision-making capacity. During morning care, the client says, "Do not let my roommate hear anything about my HIV diagnosis." Later, while the curtain is open in the semi-private room, the provider enters and begins speaking loudly about the client's latest viral load results. What should the nurse do first?

- A.** Close the curtain and allow the provider to continue so care is not delayed
- B.** Interrupt and ask the provider to continue the discussion in a private setting
- C.** Wait until the provider finishes, then remind the client about the limits of privacy in shared rooms
- D.** Document the event after rounds and file a privacy report at the end of the shift

6. A nurse is preparing to discuss a newly diagnosed sexually transmitted infection with an adult client in a semi-private room. The client's roommate and the roommate's visitor are present. The client looks uncomfortable and says quietly, "I don't want to talk about this in front of other people." What should the nurse do first?

- A.** Ask the client to speak softly so others will be less likely to hear
- B.** Proceed with the discussion because the client needs timely teaching
- C.** Move the discussion to a private area or ask unauthorized persons to step out if possible
- D.** Give the written teaching materials now and return only if the client has questions later

7. A practical nurse is reviewing the assignment sheet before shift change and notices that a client's diagnosis has been written in full on a paper clipboard that will be carried through public hallways. Which action by the nurse is most appropriate?

- A.** Leave the sheet as written because only staff members use assignment clipboards
- B.** Cover or remove the unnecessary diagnostic information and follow facility policy for protecting client information
- C.** Ask the client whether it is acceptable for the diagnosis to remain visible on the sheet
- D.** Make a second copy of the sheet and keep the original at the nurses' station

8. A practical nurse receives a secure message from a physical therapist asking for a client's most recent pain rating and mobility status before an afternoon session. The therapist is listed on the care team, but the nurse is currently caring for a different client and has not yet verified whether the message was sent through the facility-approved platform. Which action is most appropriate?

- A.** Reply immediately with the information because the therapist is involved in the client's care
- B.** Ignore the message until the end of the shift because privacy questions should be handled by management
- C.** Verify that the communication method is facility-approved and then share only the minimum necessary information relevant to treatment
- D.** Call the client's family to ask whether they consent to sharing the information with therapy

9. An adult client with decision-making capacity asks, "Why does the hospital ask me about advance directives every time I am admitted? I already told another clinic what I want." Which response by the nurse is best?

- A.** Each health care setting should ask whether you have advance directives so your wishes can be identified and included in your current record.
- B.** Advance directives from another health care setting are not valid unless they are rewritten at each admission.
- C.** You only need to answer that question if you expect to have surgery during this stay.
- D.** The hospital asks only so your family can review your choices before treatment begins.

10. A client who is now unable to communicate has both a living will and a health care proxy documented in the chart. The living will addresses mechanical ventilation in a permanent unconscious condition, but the client is currently hospitalized with a severe infection and needs a treatment decision that is not specifically discussed in the document. Which action by the nurse is most appropriate?

- A.** Ask the client's nearest relative to make the decision because the living will does not cover the current situation
- B.** Follow only the living will and withhold all aggressive treatment until the infection resolves
- C.** Recognize that the designated health care proxy should make decisions guided by the client's known wishes and best interests
- D.** Delay treatment until the client regains capacity because advance directives cannot be used for temporary conditions

11. A nurse is asked to witness a client's signature on a consent form for an elective paracentesis. Before signing, the client asks, "Will this procedure cure my liver disease?" Which response by the nurse is most appropriate?

- A.** Explain that the procedure removes fluid but refer the client to the provider for questions about expected benefits and long-term outcomes
- B.** Tell the client that the procedure is minor and usually fixes the problem quickly
- C.** Ask a family member to answer the question so the consent process is not delayed
- D.** Witness the signature first because the provider already discussed the procedure earlier

12. A nurse in the preoperative area is reviewing the chart of a client scheduled for an elective ulnar nerve release. The consent form is signed, the correct procedure is listed, and the client is awake and oriented. Which finding should cause the nurse to question whether the informed consent is valid?

- A.** The client says, "I am nervous, but I still want the surgery."
- B.** The client signed the form in the clinic 2 days ago after discussing the procedure with the surgeon.
- C.** The client received IV midazolam 15 minutes ago and is now drowsy but follows simple commands.
- D.** The client asks whether recovery will require wearing a splint for several weeks.

13. A client is preparing for discharge after hospitalization for a stroke with mild right-sided weakness. The client will return home alone and is able to take medications independently but needs extra time and hands-on assistance for bathing and dressing. The client says, "My neighbor can check on me sometimes, but not every day." Which referral should the nurse prioritize before discharge?

- A.** Home health aide services
- B.** Speech therapy
- C.** Cardiac rehabilitation
- D.** Respiratory therapy

14. A practical nurse is part of a quality improvement team on a clinic unit that wants to reduce specimen-labeling errors. After 1 month, the team finds that most errors happen during the first hour after morning blood draws, especially when labels are printed away from the bedside. Which action is the best next step in the quality improvement process?

- A.** Implement bedside label printing during morning draws on one clinic unit and track labeling errors
- B.** Provide written counseling to staff members who were involved in the documented errors

- C.** Wait 6 more months to collect additional data before making any change
- D.** Report the error rate to staff and expect the problem to improve on its own

15. A practical nurse is caring for a client hospitalized with newly diagnosed type 2 diabetes. During the shift, the nurse learns that the client cannot read the printed teaching materials, says, "I share one phone with my brother," and asks whether insulin can be kept at work because the home power is shut off at times. The client is expected to be discharged in 24 hours. Which action best supports interdisciplinary information sharing?

- A.** Document the findings in the chart and continue routine discharge teaching
- B.** Tell the client to discuss these concerns with the provider during morning rounds
- C.** Promptly communicate the barriers to the registered nurse and case manager so the discharge plan and teaching approach can be adjusted
- D.** Delay discharge teaching until a family member is available to receive the instructions

16. A practical nurse is assigned to care for a postoperative client. Before administering an IV antibiotic, the nurse notices that the client reports a severe allergy to the same medication class, but the prescription is active in the chart and the dose is due now. The RN is temporarily assisting in another room. Which action by the practical nurse is most appropriate?

- A.** Administer the medication because it was prescribed by the provider and is already verified in the chart
- B.** Hold the medication, assess the client, and immediately notify the RN and provider of the allergy concern
- C.** Ask the unlicensed assistive personnel to stay with the client while the nurse gives the medication slowly
- D.** Document that the client refused the medication and move on to the next task

17. A practical nurse in an outpatient dialysis center is assessing a 17-year-old client. During a private conversation, the client says, "I've been staying with my aunt because my stepfather hit me last weekend. Please don't tell anyone because it will make things worse." The client has a fading bruise around the upper arm. Which action by the nurse is most appropriate?

- A.** Respect the client's request for confidentiality because the client disclosed the information in private
- B.** Ask the client to return another day if ready to discuss the situation further
- C.** Report the suspected abuse according to facility policy and state law, and document the client's statement and observed findings
- D.** Wait to report until the nurse can verify the client's statement with the aunt or stepfather

18. A practical nurse asks an experienced assistive personnel (AP) to obtain a set of routine vital signs on assigned clients. Thirty minutes later, the AP reports that one client has a blood pressure of 88/54 mm Hg, a pulse of 118/min, and says, "I laid the client back in bed because she looked pale." Which action by the nurse is best?

- A.** Thank the AP and instruct the AP to repeat the vital signs in 1 hour
- B.** Assess the client immediately and determine whether further intervention is needed
- C.** Ask the AP to give the client water and recheck the blood pressure after lunch
- D.** Document that the AP reported abnormal findings and continue caring for other clients

19. A practical nurse is directing care for clients on a long-term care unit. Which activity is most appropriate to assign to an assistive personnel (AP)?

- A.** Assess a resident's new complaint of burning with urination
- B.** Reinforce teaching about how to use an incentive spirometer after pneumonia
- C.** Measure and record the weight of a resident who has stable chronic heart failure
- D.** Evaluate whether a resident's pain medicine was effective 30 minutes after administration

20. A practical nurse in an outpatient clinic is reinforcing care instructions for a client newly diagnosed with diabetes. The client says, "I know the provider wants me to check my blood sugar, but I live in my car right now and I have nowhere safe to keep my supplies." Which nursing action best promotes equitable access to care?

- A.** Repeat the instructions slowly and ask the client to restate them
- B.** Tell the client to focus on diet changes until stable housing is found
- C.** Notify the care team and connect the client with case management or community resources for housing and supply support
- D.** Advise the client to store the supplies in the clinic and come daily to use them

21. A practical nurse helping with clinic quality improvement reviews follow-up data after hospital discharge for clients with chronic obstructive pulmonary disease. Which finding most strongly suggests an inequitable access problem that should be escalated for system-level action?

- A.** Clients from one neighborhood have lower follow-up rates, and that area has the longest public transit travel time to the clinic

- B.** Several clients say they dislike early morning appointments
- C.** One provider has more cancellations during a week of vacation coverage
- D.** A few clients request to reschedule because of bad weather

22. A nurse enters a semiprivate room to administer a scheduled subcutaneous anticoagulant injection. Both clients are awake, and one says, "The shot is for me." Which action should the nurse take to identify the correct client before giving the medication?

- A.** Ask the client to state their full name and date of birth, then compare both with the medication record and identification band
- B.** Confirm the client's identity using the bed assignment and the roommate's statement
- C.** Ask the client to state the reason for hospitalization before administering the injection
- D.** Compare the medication label with the room number listed on the assignment sheet

23. A nurse is preparing a client for a portable ECG. The client is alert but says, "I lost my voice after surgery, so I can't answer you out loud." The identification band is in place and readable. Which action is best to verify the client's identity before the test?

- A.** Proceed with the ECG because the client is in the correct room and has an identification band
- B.** Ask a family member at the bedside to confirm the client's name and date of birth
- C.** Have the client point to or write their full name and date of birth, then compare that information with the identification band and order
- D.** Use the name printed on the door sign and compare it with the ECG requisition

24. A nurse is preparing to use a sterile irrigation set to flush an indwelling urinary catheter. While gathering supplies, the nurse notices the package containing the irrigation tubing is damp and has a small tear along one sealed edge. The expiration date has not passed. Which action should the nurse take?

- A.** Use the set because the expiration date is still valid
- B.** Dry the package and open it carefully to avoid contaminating the tubing
- C.** Discard the set and obtain a new sterile irrigation package
- D.** Ask another nurse to inspect the package before deciding whether to use it

25. A nurse is preparing to apply a protective skin barrier and ostomy pouch for a client with a new ileostomy. During the assessment, the client states, "I had a severe rash and shortness of breath after using a product with pectin before." The supply package the nurse plans to use lists pectin as an ingredient. What should the nurse do first?

- A.** Apply the pouch because the reaction was limited to a previous product
- B.** Hold the product and obtain a non-pectin alternative before proceeding
- C.** Premedicate the client with an antihistamine and continue with the same supplies
- D.** Apply the barrier to a small area first to see whether a reaction occurs

26. A practical nurse is making evening rounds in a long-term care facility when the nurse notices a space heater plugged into an extension cord in a resident's room. The heater is positioned beside curtains, and the resident says, "I get cold at night, so my family brought it from home." Which action should the nurse take first?

- A.** Turn off and remove the heater from use, then follow facility policy to report the hazard
- B.** Move the heater farther from the curtains and leave it available for the resident
- C.** Ask the family to sign a form stating they accept responsibility for the heater
- D.** Wait until the charge nurse finishes medication pass to ask whether the heater can remain

27. A client with severe anxiety and alcohol withdrawal is pacing, pulling at the cardiac monitor leads, and trying to leave the treatment area. The client briefly follows directions when approached one-on-one and says, "Too many people are talking to me." Which nursing action is most appropriate to support safety while following least-restrictive restraint principles?

- A.** Apply a jacket restraint to keep the client in bed until the provider evaluates the client
- B.** Move the client to a quieter area, reduce stimulation, and assign a staff member to stay with the client for close observation
- C.** Raise all side rails and tell the client to remain in bed or restraints will be needed
- D.** Request an as-needed sedative immediately because medication is less restrictive than physical restraint

28. A nurse preparing a sterile field for a bedside procedure notices that the outer wrapper of a sterile instrument tray is damp from a spill on the counter. The tray has not been opened or used, and no client contact has occurred. Which action is most appropriate?

- A.** Open the tray and use the contents if the inner items appear dry

- B.** Set the tray aside, obtain a new sterile tray, and report the near miss according to facility policy
- C.** Dry the outside wrapper with a towel and continue preparing the field
- D.** Ask another nurse to inspect the tray and decide whether it is still acceptable

29. A nurse is preparing to transfer a client from the bed to a wheelchair. The client is alert, follows directions, and can bear weight on both legs, but the client has a right-sided weakness from a recent stroke. Where should the nurse place the wheelchair before the transfer?

- A.** Parallel to the foot of the bed on the client's weak side
- B.** At a 45-degree angle on the client's strong side
- C.** Directly facing the bed on the client's weak side
- D.** At a 90-degree angle on the client's weak side

30. A practical nurse is assisting with a sterile bedside procedure. After surgical hand antisepsis and donning sterile gloves, the nurse keeps both hands clasped together at waist level while waiting for the provider to begin. Which statement best describes the nurse's technique?

- A.** The technique is correct because sterile gloves remain sterile as long as the hands are touching each other.
- B.** The technique is incorrect because sterile hands should be kept below waist level to avoid blocking the sterile field.
- C.** The technique is incorrect because sterile gloved hands should be kept above waist level and in view.
- D.** The technique is correct because the outside of sterile gloves can touch any part of the sterile gown sleeves.

31. A nurse is preparing to change the dressing on a small, dry postoperative incision for a client who is not on isolation precautions. The old dressing is intact, there is no drainage, and splashing is not expected. Which personal protective equipment is most appropriate?

- A.** Sterile gloves only
- B.** Clean gloves only
- C.** Clean gloves and a gown
- D.** Clean gloves, mask, and goggles

32. A practical nurse is preparing supplies for a sterile dressing change. The nurse picks up a packaged sterile instrument and notices the package is dry and intact, but the expiration date passed 2 weeks ago. Which action should the nurse take?

- A.** Use the instrument because the package is still sealed and dry
- B.** Open the package and inspect the instrument before deciding whether to use it
- C.** Discard the item and obtain a new sterile instrument

- D.** Use the instrument if the charge nurse confirms it looks uncontaminated

33. A practical nurse is caring for a client who has herpes zoster (shingles) with lesions that are localized and completely covered. Which isolation precaution is most appropriate in addition to standard precautions?

- A.** Contact precautions
- B.** Droplet precautions
- C.** Airborne precautions
- D.** Protective (neutropenic) precautions

34. A practical nurse is preparing to assist a client with shaving at the sink. The client has no isolation orders but has several small nicks on the chin from shaving earlier in the day. The nurse expects to handle the razor and washcloth and may come in contact with a small amount of blood. Which action best follows standard precautions?

- A.** Wear clean gloves while assisting with the shave and handling items contaminated with blood
- B.** Wear a sterile gown and sterile gloves because any blood requires sterile protection
- C.** No protective equipment is needed because the client does not have an infection diagnosis
- D.** Apply a mask only, because the main risk is exposure from the client's breathing

35. A nurse is reinforcing teaching with a client who is 12 weeks pregnant about ways to improve iron absorption from a prenatal vitamin. Which instruction should the nurse include?

- A.** Take the vitamin with a glass of orange juice
- B.** Take the vitamin with milk at bedtime
- C.** Take the vitamin with an antacid after meals
- D.** Take the vitamin with a high-fiber cereal

36. A client at 40 weeks' gestation is in active labor and receiving an oxytocin infusion. The nurse notes contractions lasting 95 to 110 seconds and occurring every 1.5 to 2 minutes. The fetal heart rate baseline is 150/min with recurrent late decelerations. Which action should the nurse take first?

- A.** Increase the oxytocin infusion rate to strengthen labor
- B.** Turn the client to a side-lying position
- C.** Perform a vaginal examination to check dilation
- D.** Encourage the client to begin pushing with contractions

37. A nurse is reinforcing teaching for a client who is 36 hours postpartum after an uncomplicated vaginal birth. Which statement by the client indicates correct understanding of lochia changes during recovery?

- A.** My discharge should stay bright red until the end of the first month.

- B. If my flow changes from pink or brown back to bright red after I overdo activity, I should rest and monitor it.
- C. Large clots are expected for the first 2 weeks after birth.
- D. Lochia should stop completely by 3 days after delivery.

38. A nurse is reinforcing teaching with the parent of a 30-month-old child. Which parent statement indicates the child is demonstrating an expected developmental milestone?

- A. "My child can copy a circle with a crayon."
- B. "My child usually speaks in short phrases of 3 to 4 words."
- C. "My child can hop on one foot for several seconds."
- D. "My child draws a person with six body parts."

39. A practical nurse is reinforcing teaching with the family of a 70-year-old client who moved to an assisted-living community 2 weeks ago after the death of a spouse. The family says, "She keeps comparing everything here to her old home and says she does not belong." Which response by the nurse is most appropriate?

- A. "This usually means the move was a mistake and she should return home as soon as possible."
- B. "Adjusting to several major losses at once can take time, and familiar routines and personal items may help her feel more secure."
- C. "If she participates in every activity offered, these feelings should stop within a few days."
- D. "It is best not to talk about her old home, because that will keep her from adapting."

40. A practical nurse is assisting with a health-promotion class for adults. Which activity is the best example of screening rather than prevention?

- A. Teaching clients to use seat belts on every car ride
- B. Checking fasting blood glucose levels at a community fair for adults without symptoms
- C. Administering an influenza vaccine before flu season
- D. Instructing a client with diabetes about daily foot care

41. A nurse is obtaining a health history from an adult client who says, "I haven't had my period for months, but that's probably just stress." Which follow-up question is most important for the nurse to ask first?

- A. "How many pregnancies have you had in the past?"
- B. "When was the first day of your last menstrual period?"
- C. "Do other people in your family have irregular periods?"
- D. "What do you usually do to manage stress?"

42. A practical nurse is admitting an adult client who had a total knee replacement 6 hours ago. Which assessment finding is most important to document now as baseline physical data for future comparison during recovery?

- A. The client states that knee pain is 7 on a 0 to 10 scale
- B. The client prefers to sleep with two pillows at home
- C. The client ate half of breakfast before surgery
- D. The client reports a history of seasonal allergies

43. A nurse is reinforcing teaching with the parent of an 11-year-old child who is scheduled to receive the tetanus, diphtheria, and acellular pertussis (Tdap) vaccine. Which statement by the parent indicates correct understanding?

- A. My child should avoid all physical activity for 1 week after this vaccine.
- B. A sore arm and mild redness at the injection site can happen after this vaccine.
- C. If my child had a mild cold last week, the vaccine must be delayed for a month.
- D. This vaccine can give my child pertussis for a few days after the shot.

44. A client newly diagnosed with multiple sclerosis says to the practical nurse, "Some days I look fine, so people think I'm exaggerating. I feel alone even when others are around." Which response by the nurse best provides emotional support?

- A. You should focus on the fact that many people live full lives with this condition.
- B. It sounds like not feeling understood has been very painful for you.
- C. Try not to worry about what other people think of your illness.
- D. Have you told your provider that your symptoms seem to be getting worse?

45. A client whose spouse died 6 weeks ago tells the nurse, "I can't sleep, I forget things at work, and I feel like I'm falling apart." Which nursing response best promotes effective coping?

- A. "You need to stay busy so you do not dwell on the loss."
- B. "Tell me what part of the day feels hardest for you right now."
- C. "At least you are still able to go to work each day."
- D. "You should avoid talking about your spouse for a while until you feel stronger."

46. A nurse in an inpatient mental health unit is assessing a client who has been increasingly withdrawn since receiving upsetting family news. Which finding should the nurse identify as the strongest indicator of imminent violence?

- A.** The client refuses to join the dayroom group activity
- B.** The client speaks very little and avoids eye contact
- C.** The client suddenly begins shouting, clenches the jaw, and slams a fist into the wall
- D.** The client says, "I just want to be left alone for now"

47. A client who stopped heavy daily alcohol use 2 days ago becomes increasingly anxious and says, "The room looks strange, and I keep hearing things." The client is diaphoretic, disoriented, and has a heart rate of 118/min. Which action should the nurse take first?

- A.** Encourage the client to discuss feelings about stopping alcohol use
- B.** Reduce environmental stimulation and stay with the client while obtaining immediate medical assistance
- C.** Offer oral fluids and ask the client to rest until the symptoms pass
- D.** Explain that the symptoms are expected and will resolve without treatment

48. A client who had a stroke 2 days ago has expressive aphasia and becomes increasingly frustrated during a meal when unable to ask for help. The client hits the overbed table with an open hand, pushes the plate away, and begins to cry. Which action by the nurse is most appropriate?

- A.** Ask a series of yes-or-no questions to identify what the client needs
- B.** Tell the client to calm down and try again when more in control
- C.** Remove the meal tray and return when the client is less upset
- D.** Explain that angry behavior is unacceptable and may delay assistance

49. A client whose surgery has been postponed for the third time says to the nurse, "Why do they keep doing this to me? Maybe they don't think I'm worth bothering with." Which response by the nurse is most therapeutic?

- A.** I'm sure the staff values you and is doing the best they can.
- B.** You shouldn't assume that. Delays happen to many clients.
- C.** It sounds like these repeated delays are making you feel unimportant.
- D.** Let's focus on getting you ready in case they call for you soon.

50. A nurse on a medical unit sees a client become visibly angry after a family visit is ended early. The client is sitting forward in a chair, jaw tight, and speaking in a tense voice: "Everybody keeps controlling me." Which response by the nurse is most appropriate to de-escalate the situation?

- A.** "You need to calm down before we can talk about this."
- B.** "I can see you're upset. Tell me what is bothering you most right now."
- C.** "Your family had to leave, so there is nothing we can do about it now."
- D.** "If you keep talking like that, I will need to call security."

51. A resident with moderate dementia becomes upset during a bath and says, "Stop looking at me." The resident turns away, covers the chest with both arms, and refuses to step into the shower. Which action by the practical nurse is most appropriate?

- A.** Tell the resident that bathing is required and continue with the shower as planned
- B.** Offer a towel or bath blanket for covering and explain each step before providing care
- C.** Remind the resident that staff have seen many clients bathe and there is nothing to be embarrassed about
- D.** Delay all bathing until the resident agrees without hesitation

52. A nurse is assessing a client who is 1 day postoperative. The client is quiet, avoids complaining, and says, "Pain is something to endure," even while grimacing and guarding the incision. Which action by the nurse is most appropriate?

- A.** Document that the client denies pain and continue routine postoperative care
- B.** Explain that pain should always be reported directly and ask the client to rate it again later
- C.** Recognize that cultural beliefs may affect pain expression, assess pain further, and offer prescribed pain relief
- D.** Ask a family member whether the client is actually having pain before taking further action

53. A nurse is preparing a client who identifies as nonbinary for surgery. In the preoperative area, the client says, "I only want my spouse to receive updates. My parents are here too, and they do not know about my gender identity." Which action by the nurse is most appropriate?

- A.** Ask the parents to step out and then review with the client exactly what information may be shared and with whom
- B.** Tell the parents that all family members will receive the same general postoperative information
- C.** Document the request after surgery so the team can focus on the procedure now

D. Advise the client that legal next of kin must be updated first if they ask questions

54. A client who recently began using a hearing aid tells the nurse, "I know it should help, but in the day room all the sounds run together. I get frustrated and leave before anyone notices." Which nursing response is best?

A. "Try wearing the hearing aid only when staff are talking so you do not get overstimulated."

B. "It sounds frustrating to feel overwhelmed by background noise. Let's talk about quieter times or places where you can practice joining conversations."

C. "You need to keep socializing so you do not become isolated."

D. "Most clients adjust quickly to hearing aids, so this feeling should pass soon."

55. A client with metastatic cancer who is receiving end-of-life care says to the nurse, "My family is arguing in the hallway about whether I should keep trying more treatment. I just want a little peace, but I don't want to upset them." Which nursing action is most appropriate?

A. Ask the family to leave immediately so the client can rest

B. Tell the client that the health care provider will need to settle the disagreement

C. Explore with the client what the client wants communicated to the family and help arrange a calm discussion

D. Encourage the client to avoid the topic until the family is less emotional

56. A nurse is preparing to administer a scheduled bolus enteral feeding through a client's mature gastrostomy tube. To help maintain tube patency, which action is most appropriate?

A. Flush the tube with water before and after the feeding

B. Mix the formula with water until it flows more easily

C. Clamp the tube for 30 minutes after the feeding

D. Apply petroleum jelly to the end of the tube before connecting the syringe

57. A nurse is caring for a client who reports nausea after surgery and asks what to order for the first meal when solid foods are allowed again. Which selection is most appropriate?

A. Fried chicken, corn, and chocolate cake

B. Turkey sandwich with lettuce and chips

C. Dry toast, applesauce, and clear tea

D. Chili, coleslaw, and iced coffee

58. A client who is 24 hours postoperative after abdominal surgery reports incisional pain that becomes sharper when moving from lying to sitting. Which nonpharmacologic intervention should the nurse teach to reduce pain during movement?

A. Use a pillow to splint the incision when changing position

B. Massage the skin directly over the incision before moving

C. Keep the client flat in bed to avoid all movement

D. Apply a heating pad to the fresh incision for 30 minutes

59. A nurse is caring for a client with right shoulder weakness after an injury. The client wants to put on a pullover shirt without help. Which instruction should the nurse give to promote independence while reducing strain during this activity of daily living?

A. Put the shirt on over the stronger arm first, then pull it over the weaker arm

B. Insert the weaker arm into the sleeve first, then pull the shirt over the head and stronger arm

C. Ask a family member to dress the client until the shoulder is fully healed

D. Avoid using the affected arm at all while getting dressed

60. A nurse is caring for a client who is on a scheduled bowel program after a spinal cord injury. The client usually has a bowel movement every other morning after breakfast. Today, the nurse needs to help the client achieve the most effective evacuation. Which action is best?

A. Offer a bedpan at bedtime to avoid interrupting the morning routine

B. Administer the bowel intervention at a different time each day to prevent dependence

C. Assist the client to the toilet or commode after breakfast at the usual time

D. Encourage the client to remain in bed during the bowel program to conserve energy

61. A nurse is preparing a teaching plan for a client who reports trouble sleeping at night. Which instruction should the nurse include to help promote sleep?

A. Exercise vigorously within 1 hour of bedtime to become physically tired

B. Use the bed for watching television so it becomes a relaxing place

C. Maintain a consistent bedtime and wake-up time each day

D. Drink a caffeinated beverage in the evening to prevent waking during the night

62. A practical nurse is caring for a client who has a short leg cast for a recent ankle fracture. During the afternoon assessment, the client reports burning under the cast at the heel. Which action by the nurse is most appropriate?

- A.** Insert a padded cotton swab under the cast to relieve the pressure
- B.** Ask the client to shift the leg position and continue to monitor at the next round
- C.** Inspect the edges of the cast and report the burning sensation promptly because it may indicate pressure on the skin
- D.** Apply lotion around the cast opening to reduce dryness and itching

63. A practical nurse is helping a client with diminished hearing in the right ear prepare for teaching about a new walker. Where should the nurse position when speaking to the client?

- A.** Stand on the client's right side and speak loudly
- B.** Stand on the client's left side and face the client
- C.** Stand behind the client to avoid distraction
- D.** Stand at the foot of the bed and gesture only

64. A client with heart failure is on strict intake and output. Which action by the practical nurse is most important to improve the accuracy of the client's output measurement during the shift?

- A.** Estimate urine volume in the toilet based on the amount seen
- B.** Ask the client to void in a calibrated collection hat placed in the toilet
- C.** Record output only at the end of the shift to avoid repeated calculations
- D.** Subtract 50 mL from each void to account for normal measurement variation

65. A practical nurse is caring for a client who has been on prolonged bed rest and is using a trochanter roll. What is the main purpose of this device?

- A.** To prevent external rotation of the hip
- B.** To reduce pressure on the heels
- C.** To keep the knees slightly flexed
- D.** To maintain the client in high-Fowler position

66. A nurse is preparing to administer a scheduled dose of metoprolol to a client. After identifying the client, the nurse checks the latest vital signs and notes the apical pulse is 52/min. The client says, "I feel more tired than usual today." Which action is most appropriate?

- A.** Administer the medication because it is scheduled and document the pulse afterward
- B.** Hold the medication and notify the provider of the client's pulse and symptoms

- C.** Give half the dose now and recheck the pulse in 30 minutes
- D.** Ask another nurse to administer the medication to confirm the finding

67. A practical nurse is preparing a client's 2100 medications. The MAR lists warfarin 5 mg PO nightly. While reviewing the chart, the nurse sees a provider prescription entered at 1830 that states, "Hold warfarin tonight; INR 4.1." The MAR has not been updated, and the previous shift did not give the dose. What should the nurse do?

- A.** Administer the warfarin as listed on the MAR because it is a scheduled nightly medication
- B.** Hold the warfarin and clarify the discrepancy according to facility policy before administration
- C.** Give half the dose because the INR is elevated but the medication is still prescribed
- D.** Ask the client whether warfarin is usually taken at home and follow the client's routine

68. A nurse is administering a medication that is intended to provide continuous systemic absorption over 72 hours through intact skin. Which route of administration should the nurse identify?

- A.** Topical
- B.** Transdermal
- C.** Intradermal
- D.** Subcutaneous

69. A nurse is preparing to administer packed red blood cells to a client through an existing IV line. Which IV fluid is appropriate to infuse with the blood product?

- A.** 0.9% sodium chloride
- B.** 5% dextrose in water
- C.** Lactated Ringer's solution
- D.** 0.45% sodium chloride

70. A nurse must administer 50 mL of IV medication over 25 minutes using gravity tubing with a drop factor of 15 gtt/mL. How many drops per minute should the nurse regulate?

- A.** 20 gtt/min
- B.** 25 gtt/min
- C.** 30 gtt/min
- D.** 45 gtt/min

71. A practical nurse is reinforcing teaching for a client who is being discharged with a new prescription for an opioid pain medication. Which client statement indicates a need for further teaching?

- A.** I should keep this medication in a secure place away from children and visitors.

- B.** If I no longer need the medication, I should ask about a safe take-back option for disposal.
- C.** If my pain gets worse, I can share a few tablets with my brother because he has similar back pain.
- D.** I should take the medication only as prescribed.

72. A nurse is caring for a client who has a transdermal fentanyl patch in place for chronic pain. The client reports increased back pain and says, "I put a heating pad over my patch so the medicine will work faster." Which response by the nurse is best?

- A.** Leave the heating pad in place because warmth improves absorption without increasing risk
- B.** Remove the heating pad because heat can increase fentanyl absorption and cause toxicity
- C.** Apply a second fentanyl patch on a different site while using the heating pad
- D.** Cover the patch with a thick dressing so the medication stays in the skin longer

73. A client with heart failure has been taking digoxin for 1 week. Which new finding should the nurse report to the provider immediately as the most concerning possible adverse response to the medication?

- A.** Decreased appetite with nausea
- B.** Yellow-tinted vision and seeing halos around lights
- C.** Mild fatigue at the end of the day
- D.** Urinating more often during the night

74. A patient with heart failure takes digoxin 0.125 mg daily and furosemide 40 mg daily. Which new assessment finding should the nurse report first as the most likely sign of a clinically significant drug interaction?

- A.** Ringing in the ears after taking the morning medications
- B.** Nausea, decreased appetite, and seeing yellow-green halos around lights
- C.** Dry mouth and increased thirst at bedtime
- D.** Mild headache that improves with rest

75. A client with severe acne is prescribed isotretinoin. At a follow-up visit, the nurse reviews the laboratory results before reinforcing medication teaching. Which finding requires the nurse to notify the provider before the client continues taking the medication?

- A.** Triglycerides 612 mg/dL
- B.** Hemoglobin 13.8 g/dL
- C.** Sodium 138 mEq/L
- D.** WBC count 7,400/mm³

76. A nurse is teaching a client who has a new prescription for an epinephrine auto-injector because of a severe allergy to peanuts. Which instruction should the nurse include?

- A.** Inject the medication through clothing into the outer thigh if severe symptoms occur
- B.** Store the auto-injector in the refrigerator until it is needed
- C.** Use the auto-injector only after taking an oral antihistamine
- D.** Insert the injector into the abdomen for the fastest absorption

77. A nurse is preparing to administer insulin glargine and notices that the vial has been gently shaken and now appears cloudy with small particles. The label and expiration date are intact. Which action should the nurse take?

- A.** Administer the dose because insulin can appear cloudy after mixing
- B.** Warm the vial to room temperature and recheck the appearance
- C.** Discard the vial and obtain a new one
- D.** Roll the vial between the hands until the particles dissolve

78. A nurse is preparing to administer phenytoin 125 mg by mouth. The suspension available is labeled 100 mg per 4 mL. How many milliliters should the nurse administer for one dose?

- A.** 4 mL
- B.** 5 mL
- C.** 6 mL
- D.** 8 mL

79. A practical nurse is collecting data on four clients during a home health visit. Which client should the nurse identify as being at greatest risk for injury related to infection exposure?

- A.** A client receiving chemotherapy who reports avoiding large crowds but has fresh flowers and standing water in the bedroom
- B.** A client with type 2 diabetes who checks blood glucose before meals and stores insulin in the refrigerator
- C.** A client with osteoarthritis who uses a heating pad on the knees for 15 minutes at bedtime
- D.** A client with hypertension who takes a daily walk outdoors and wears supportive shoes

80. A nurse is preparing to collect a venous blood specimen to measure a client's serum potassium level. The client has an IV infusing potassium chloride in the left forearm. Which action should the nurse take?

- A.** Draw the specimen from the left arm below the IV site

- B.** Stop the IV for 5 minutes and draw the specimen from the tubing port
- C.** Obtain the specimen from the right arm if possible
- D.** Draw the specimen from the left hand after flushing the IV
- 81.** A nurse is reviewing laboratory results for a client who has acute pancreatitis and reports numbness around the mouth with hand cramping. Which laboratory finding best explains these manifestations?
- A.** Serum calcium 7.1 mg/dL
- B.** Serum sodium 132 mEq/L
- C.** Serum magnesium 2.0 mEq/L
- D.** Blood glucose 168 mg/dL
- 82.** A client who is 11 weeks pregnant comes to the clinic reporting severe lower abdominal pain on the right side and feeling faint. Which additional assessment finding should the nurse identify as most concerning for a ruptured ectopic pregnancy?
- A.** Scant brown vaginal spotting
- B.** Referred pain at the shoulder
- C.** Urinary frequency without dysuria
- D.** Nausea when smelling food
- 83.** A nurse is preparing to perform a bedside urine dipstick test on a freshly collected specimen from a client with urinary frequency. After dipping the strip, the nurse notices excess urine is still coating the reagent pads. Which action is most appropriate to help prevent an inaccurate point-of-care result?
- A.** Blot the edge of the strip on absorbent material and then compare each pad at the recommended times
- B.** Hold the strip upright for 2 minutes before reading all pads at once
- C.** Redip the strip after 30 seconds to be sure each pad is fully saturated
- D.** Shake the strip vigorously to remove urine before comparing the colors
- 84.** A practical nurse is obtaining vital signs on an adult client who is sleeping. The client does not have a cardiac monitor. Which pulse assessment method will provide the most accurate heart rate?
- A.** Count the radial pulse for 15 seconds and multiply by 4
- B.** Count the apical pulse for 30 seconds and multiply by 2
- C.** Count the radial pulse for 1 full minute
- D.** Use the client's previous pulse reading from the last shift
- 85.** A client is scheduled for a percutaneous renal biopsy in 1 hour. Which finding is most important for the nurse to report to the provider before the procedure?
- A.** The client reports mild anxiety about the procedure
- B.** The client voided 45 minutes ago
- C.** The client received heparin earlier this morning
- D.** The client has been NPO since midnight
- 86.** A nurse is reviewing a client's ECG rhythm strip. The rhythm is regular at 150/min, the QRS complexes are narrow, and P waves are not clearly visible. Which rhythm should the nurse identify?
- A.** Sinus tachycardia
- B.** Supraventricular tachycardia
- C.** Atrial flutter
- D.** Ventricular tachycardia
- 87.** A nurse is obtaining a venous blood specimen from an adult client using a syringe. When the plunger is pulled back quickly, blood flow slows and the vein appears to flatten. Which action is most appropriate?
- A.** Release the tourniquet and remove the needle immediately
- B.** Pull the plunger back more forcefully to maintain blood flow
- C.** Reduce the amount of suction by pulling the plunger back more gently
- D.** Advance the needle farther into the vein
- 88.** A practical nurse is caring for a client on the first day after a total hip arthroplasty. The client says, "I don't want to get out of bed because it hurts." Which nursing response is best?
- A.** Remain on bed rest until the pain is completely relieved
- B.** Limit oral fluids so the client will not need to get up as often
- C.** Encourage prescribed pain control and early ambulation to reduce postoperative complications
- D.** Massage the client's calves before ambulation to improve circulation
- 89.** A nurse is caring for a client 18 hours after a laparoscopic cholecystectomy. The client says, "My right shoulder hurts, and now my belly feels much more swollen than it did this morning." Which focused assessment should the nurse perform first?
- A.** Inspect and palpate the abdomen for distention, firmness, and tenderness
- B.** Ask the client to rate the shoulder pain on a 0 to 10 scale
- C.** Review the client's preoperative consent form
- D.** Check whether the client has passed flatus since surgery
- 90.** A client with benign prostatic hyperplasia begins tamsulosin therapy. Which finding 1 week later best indicates the medication is having the desired therapeutic effect?
- A.** The client reports a stronger urine stream and less hesitancy when starting to void

- B.** The client reports sleeping through the night without waking to urinate
- C.** The client has a urine output of 2400 mL in 24 hours
- D.** The client has a blood pressure of 148/88 mm Hg

91. A client who is 8 hours postpartum suddenly reports a severe headache and blurred vision. The nurse notes blood pressure 168/104 mm Hg, 3+ deep tendon reflexes, and new swelling of the hands and face. Which action should the nurse take first?

- A.** Place the client in a quiet room and notify the provider
- B.** Administer the prescribed magnesium sulfate
- C.** Encourage oral fluids and reassess in 30 minutes
- D.** Assist the client to ambulate to promote circulation

92. A client with long-standing osteoporosis asks the practical nurse how to lower the risk of fractures at home. Which instruction is most appropriate?

- A.** Keep walkways clear and use nonskid footwear when walking
- B.** Limit daily activity to avoid putting stress on the bones
- C.** Bend from the waist when lifting light objects from the floor
- D.** Take calcium supplements only when bone pain occurs

93. A client with chronic kidney disease misses two hemodialysis treatments and now reports weakness and palpitations. The nurse notes muscle cramps and obtains a cardiac monitor strip showing tall, peaked T waves. Which action should the nurse take first?

- A.** Prepare to administer a potassium-lowering intervention as prescribed
- B.** Encourage the client to increase oral fluid intake
- C.** Place the client in a high-Fowler position
- D.** Teach the client about the importance of not missing dialysis

94. A client has a newly inserted percutaneous nephrostomy tube draining urine into a bedside bag. During the assessment, the nurse notes the tubing is kinked under the client's leg, the drainage bag contains 150 mL of cloudy yellow urine from the previous 4 hours, and the client reports new right flank pain rated 7/10. Which action should the nurse take first?

- A.** Straighten the tubing and ensure the drainage bag hangs below kidney level
- B.** Irrigate the nephrostomy tube with sterile saline
- C.** Obtain a urine specimen from the drainage bag for culture
- D.** Administer the prescribed PRN analgesic for flank pain

95. A client receiving mechanical ventilation has an arterial blood gas report showing pH 7.50, PaCO₂ 30 mm Hg, and HCO₃ 24 mEq/L. Which interpretation is most appropriate?

- A.** Respiratory acidosis from hypoventilation
- B.** Respiratory alkalosis from excessive ventilation
- C.** Metabolic acidosis with respiratory compensation
- D.** Metabolic alkalosis caused by bicarbonate retention

96. A client with a permanent pacemaker comes to the clinic 2 weeks after implantation and says, "I almost used a heating pad over the sore area on my chest last night." Which response by the nurse is most appropriate?

- A.** Apply the heating pad for 20 minutes at a low setting to improve circulation
- B.** Avoid placing heat directly over the pacemaker site unless specifically instructed by the provider
- C.** Use the heating pad only if the pacemaker is turned off first
- D.** It is safe to use heat over the site once the incision is closed

97. A nurse is reinforcing teaching for a client with a new ostomy who asks why the opening in the skin barrier should be cut to fit closely around the stoma. Which response by the nurse is best?

- A.** It helps keep stool from contacting the surrounding skin
- B.** It prevents the stoma from swelling during digestion
- C.** It keeps the pouch from filling with gas
- D.** It allows the pouch to stay in place without adhesive

98. A nurse is caring for a client who is 24 hours postoperative following thyroidectomy. Which finding should the nurse recognize as the most urgent change in condition?

- A.** The client reports incisional pain rated 6 on a 0 to 10 scale
- B.** The client has a hoarse voice when speaking
- C.** The client develops tingling around the mouth and in the fingertips
- D.** The client states feeling tired after sleeping poorly overnight

99. A nurse is caring for a client with diabetic ketoacidosis who has started IV insulin therapy. Which finding is most important for the nurse to report immediately as a possible complication of treatment?

- A.** Blood glucose decreases from 420 mg/dL to 280 mg/dL in 2 hours
- B.** Serum potassium decreases from 4.8 mEq/L to 3.1 mEq/L
- C.** Urine output increases to 45 mL/hr after fluid replacement

D. Respirations change from deep and rapid to 20/min and regular

100. A client with end-stage kidney disease who receives hemodialysis says, "I feel weak all the time." Which laboratory value most likely explains this finding?

A. Hemoglobin 8.2 g/dL

B. Sodium 136 mEq/L

C. Platelet count 240,000/mm³

D. White blood cell count 7,500/mm³

Simulation Test 22 — Answer Key

1. A	26. A	51. B	76. A
2. B	27. B	52. C	77. C
3. D	28. B	53. A	78. B
4. C	29. B	54. B	79. A
5. B	30. C	55. C	80. C
6. C	31. B	56. A	81. A
7. B	32. C	57. C	82. B
8. C	33. A	58. A	83. A
9. A	34. A	59. B	84. C
10. C	35. A	60. C	85. C
11. A	36. B	61. C	86. B
12. C	37. B	62. C	87. C
13. A	38. B	63. B	88. C
14. A	39. B	64. B	89. A
15. C	40. B	65. A	90. A
16. B	41. B	66. B	91. B
17. C	42. A	67. B	92. A
18. B	43. B	68. B	93. A
19. C	44. B	69. A	94. A
20. C	45. B	70. C	95. B
21. A	46. C	71. C	96. B
22. A	47. B	72. B	97. A
23. C	48. A	73. B	98. C
24. C	49. C	74. B	99. B
25. B	50. B	75. A	100. A

Simulation Test 23

100 questions

1. At the start of a shift in a surgical unit, the practical nurse reviews these assigned clients. Which client should be assessed first?
 - A. A client 1 day after abdominal surgery who reports nausea after breakfast
 - B. A client 4 hours after receiving a cast who says the fingers feel numb and look pale
 - C. A client scheduled for discharge who needs teaching about wound care
 - D. A client with a urinary catheter whose drainage bag contains 250 mL of cloudy urine
2. A practical nurse in a community clinic has 15 minutes before the provider arrives and receives these requests at the same time. Which task is the best priority for the nurse to address first?
 - A. Call a client whose fasting blood glucose log shows values of 140 to 160 mg/dL this week
 - B. Prepare teaching materials for a client newly prescribed a low-sodium diet
 - C. Assess a walk-in client who is pale, diaphoretic, and reports chest pressure
 - D. Return a message from a parent asking whether a child can receive a routine vaccine next month
3. A practical nurse is caring for several clients with help from an assistive personnel (AP). Which task is safest for the nurse to delegate to the AP?
 - A. Teach a client with newly diagnosed diabetes how to use a glucometer
 - B. Irrigate a client's indwelling urinary catheter that has stopped draining
 - C. Assist a stable client with eating breakfast after the nurse verifies swallowing is safe
 - D. Decide whether a postoperative client's incision is healing normally
4. A practical nurse delegated application of antiembolism stockings to an assistive personnel (AP) for a client recovering from surgery. The AP reports, "The client's toes look bluish and feel cool after I put them on." What should the practical nurse do first?
 - A. Assess the client's circulation and remove or adjust the stockings as needed
 - B. Tell the AP to leave the stockings in place and check again in 1 hour
 - C. Document the AP's observation in the record and continue with other tasks
 - D. Ask the AP to massage the client's feet until the color improves
5. A nurse is caring for an adult client who is alert, oriented, and able to make decisions. The client says, "I don't want my test results discussed while my neighbor is visiting, even though we are close." Later, the neighbor is in the room and says, "You can tell me anything. We share everything." What should the nurse do?
 - A. Review the test results because the neighbor is present with the client
 - B. Ask the client whether the client wants the neighbor to stay for the discussion
 - C. Discuss only general information about the condition, not the test results
 - D. Give the information to the neighbor after the visit ends
6. A nurse is discharging an adult client with a new ostomy. The client is alert, has decision-making capacity, and says, "I understand the teaching, but I can't afford the supplies and I have no transportation to follow-up visits." Which action by the nurse best demonstrates advocacy?
 - A. Emphasize that the client is responsible for obtaining supplies after discharge
 - B. Ask the provider to delay discharge until the client feels more confident about self-care
 - C. Refer the client to case management or social services to help arrange supplies and follow-up resources
 - D. Instruct the client to reuse supplies as long as they appear clean
7. A practical nurse is teaching a client how to use the facility's online patient portal. The client says, "My daughter usually keeps track of everything for me. Can you give her my portal username and temporary password?" The daughter is present in the room, but there is no documented permission in the chart. Which response by the nurse is most appropriate?
 - A. Give the daughter the login information because the client asked in front of the nurse
 - B. Ask the daughter to write down the information so it is not spoken aloud
 - C. Explain that portal access can be shared only after the client's authorization is verified according to facility policy
 - D. Refuse to discuss portal access with either the client or the daughter

8. A practical nurse is caring for a client who had surgery earlier in the day. While documenting in the electronic health record, the nurse realizes that a progress note containing the client's full name, procedure, and postoperative complication was accidentally entered into the chart of another client with a similar name. Which action should the nurse take first?

- A.** Delete the note immediately to remove the incorrect information
- B.** Notify the charge nurse or supervisor and follow facility procedure for correcting the documentation error and privacy breach
- C.** Add a second note in the wrong chart explaining that the first note was entered by mistake, then leave it there
- D.** Wait until the end of the shift to see whether the other client's primary nurse notices the error

9. A nurse is admitting an alert adult client who says, "I do not have an advance directive, and I really do not want to fill out legal papers tonight." Which nursing response is most appropriate?

- A.** You should complete one now because every hospitalized client is required to have one
- B.** That is your choice. I can document that you do not want to complete one now and let you know resources are available if you change your mind
- C.** Your closest family member will need to complete the form for you before treatment can continue
- D.** Without an advance directive, the provider may be unable to treat you during this admission

10. A hospitalized client is now unconscious and unable to make decisions. The chart contains a valid advance directive requesting no resuscitation if cardiac arrest occurs. When the client becomes pulseless, the spouse says, "Ignore that paper and start CPR." Which action by the nurse is most appropriate?

- A.** Begin CPR until the provider arrives and clarifies the plan
- B.** Ask the spouse to sign consent to revoke the advance directive
- C.** Follow the documented directive and facility orders while notifying the appropriate team members
- D.** Delay action until all immediate family members agree on what should be done

11. A nurse is preparing a client for an elective liver biopsy. The provider asks the nurse to obtain informed consent. Which action is within the nurse's role?

- A.** Explain the major risks, benefits, and alternatives to the procedure
- B.** Witness the client's signature and verify the client appears to be signing voluntarily

C. Recommend whether the client should agree to the biopsy

D. Answer detailed questions about how the procedure will change the treatment plan

12. A client is scheduled for an elective cardiac ablation later today. The client received an IV benzodiazepine 20 minutes ago for severe anxiety. The provider now asks the nurse to have the client sign the informed consent form because transport is waiting. The client is drowsy but answers simple questions correctly. Which action should the nurse take?

- A.** Have the client sign because the client is still awake and oriented
- B.** Ask the client's spouse to sign since the client is drowsy
- C.** Delay the signing and notify the provider that consent should be obtained when the client is able to provide it competently
- D.** Witness the signature now and document that sedation was given after consent

13. A client is being discharged after hospitalization for heart failure. The client has a new prescription for a low-sodium diet, daily weights, and a follow-up visit in 1 week. During teaching, the client says, "I don't own a scale, and I usually eat most of my meals from canned or fast-food places." Which nursing action is most appropriate before discharge?

- A.** Ask the client to keep the follow-up appointment and discuss these concerns with the provider then
- B.** Notify the case manager or social worker to help address equipment and food-access barriers before discharge
- C.** Tell the client to weigh at a neighbor's house whenever possible
- D.** Emphasize taking the prescribed medication correctly because diet and weights are less urgent

14. A client is preparing for discharge after a below-the-knee amputation related to peripheral vascular disease. The client lives alone in a rural area and will need dressing care, mobility training, and evaluation for adaptive equipment at home. Which referral should the nurse prioritize to support the client's transition from hospital to home?

- A.** Home health services for nursing and therapy follow-up
- B.** Outpatient dietitian consultation in 1 month
- C.** Local senior center meal program
- D.** Pastoral care for coping support

15. An assisted living facility wants to reduce missed doses of scheduled medications. A 3-week review shows that most missed doses occur when residents leave the unit for activities just before medication pass. The quality improvement team is choosing a measure to track whether a new pre-activity medication check is being used consistently. Which measure is best?

- A.** The percentage of residents who missed a scheduled medication dose each week
- B.** The number of residents who attended activities after receiving medications
- C.** The percentage of eligible residents who had the pre-activity medication check documented before leaving the unit
- D.** The number of activity sessions offered during the week

16. A practical nurse is caring for a client admitted for severe nausea, vomiting, and dehydration. During the shift, the nurse notes the client has had only 100 mL of urine output in 8 hours, dry mucous membranes, and dizziness when standing. The provider plans possible discharge later today if the client tolerates oral fluids. Which information should the nurse communicate to the interdisciplinary team to best support safe care planning?

- A.** The client prefers apple juice instead of broth and says the room is too warm
- B.** The client has persistent signs of dehydration, very low urine output, and orthostatic dizziness despite the planned discharge
- C.** The client asked whether discharge paperwork will take a long time to complete
- D.** The client watched television most of the morning and slept after lunch

17. A practical nurse is preparing to give a prescribed dose of opioid pain medication to an adult client after surgery. Before the medication is administered, the client says, "I changed my mind. I don't want it." The client's spouse replies, "He needs it. Go ahead and give it so he can rest." Which action by the nurse is most appropriate?

- A.** Administer the medication because it was prescribed and the spouse agrees
- B.** Hold the medication, respect the client's refusal, and notify the RN
- C.** Ask the spouse to sign a refusal form for the client
- D.** Tell the client that the medication must be taken because it was already prepared

18. A practical nurse in an urgent care clinic is caring for a 72-year-old client who is brought in by a neighbor. The client is alert and oriented. During the assessment, the nurse notices poor hygiene, dehydration, and that the client's prescription bottles are empty even though they were recently filled. When the neighbor steps out, the client says, "My nephew is supposed to help me, but he hasn't been coming. Please don't report this. I don't want him to get in trouble." Which action by the nurse is most appropriate?

- A.** Respect the client's request and document the findings without making a report
- B.** Ask the client to sign a statement before taking any further action
- C.** Report the suspected neglect according to agency policy and state law
- D.** Wait until additional proof is available before notifying anyone

19. A practical nurse is supervising an assistive personnel (AP) on a medical unit. Which task is appropriate for the nurse to assign to the AP for a client who is being discharged today?

- A.** Teach the client how to use an incentive spirometer at home
- B.** Review the discharge medication schedule with the client
- C.** Transport the client by wheelchair to the hospital entrance
- D.** Assess the client's understanding of follow-up appointments

20. A practical nurse in a clinic is reinforcing care for a client with heart failure. The client says, "I was told to weigh myself every morning, but I live in a rural area with poor internet and no smartphone. The office keeps sending me links for telehealth and app check-ins, but I can't use them." Which action by the nurse best promotes equitable access to care?

- A.** Advise the client to borrow a family member's phone for all follow-up communication
- B.** Ask the client to keep trying the telehealth system because it is the clinic's standard process
- C.** Collaborate with the team to arrange follow-up options the client can actually use, such as phone calls or in-person visits
- D.** Tell the client to call emergency services if symptoms worsen instead of scheduling follow-up care

21. A practical nurse participates in a clinic quality-improvement meeting about follow-up after abnormal cancer screening results. Which finding most strongly suggests an inequitable access barrier that requires system-level action?

- A.** Clients across all insurance groups have similar follow-up completion rates, but several individual clients report forgetting appointments
- B.** Clients who need wheelchair-accessible transportation have a much lower rate of completed follow-up, and the clinic schedules the specialty referrals only at a site not served by accessible transit
- C.** One provider has a slightly higher rescheduling rate than other providers during the same month
- D.** New clients ask more questions than established clients during discharge teaching

22. A nurse is preparing to administer eye drops to a client in an urgent care clinic. The electronic record is open for Elena Morris, date of birth 11/14/1972. The client in the exam room says, "Yes, I'm Elena," when the nurse enters. Which action should the nurse take to verify the client's identity correctly before giving the medication?

- A.** Ask the client to state their full name and date of birth, then compare that information with the medication record and identification band or clinic record
- B.** Confirm the client's identity by asking whether they are waiting for eye drops and checking the room number
- C.** Use the client's first name and the provider's diagnosis to confirm the correct person
- D.** Accept the client's statement that she is Elena because the electronic record is already open

23. A nurse is preparing to obtain a client's weight using a standing scale. Before the client steps on, the nurse notices the scale rocks slightly because one foot is uneven. The client is weak but able to stand with assistance. Which action should the nurse take?

- A.** Have the client stand briefly while the nurse steadies the scale
- B.** Place a folded towel under the uneven foot and continue
- C.** Use a different properly functioning scale or obtain the weight by another safe method
- D.** Ask the client to hold onto the nearby overbed table during the measurement

24. A nurse is preparing to use an electric suction setup for a client with copious oral secretions. The canister and tubing are connected, and the machine turns on. During the pre-use check, the nurse occludes the tubing and notes that suction pressure does not rise to the expected level. The client is breathing comfortably at the moment. Which action is best?

- A.** Use the suction equipment anyway because some suction is better than none
- B.** Troubleshoot the setup and replace the equipment before client use
- C.** Delay further action until the client actively needs suctioning
- D.** Ask assistive personnel to hold the tubing tighter during suctioning

25. A nurse is preparing to administer the first dose of regular insulin from a multidose vial. The client says, "I had a serious reaction after receiving protamine in the past, and my provider told me to avoid it." What is the nurse's priority action?

- A.** Administer the insulin because regular insulin does not contain protamine
- B.** Hold the medication and contact the provider and pharmacist to verify the insulin product before administration
- C.** Ask the client whether the protamine reaction caused only local redness
- D.** Give a reduced dose first to assess the client's response

26. A practical nurse is checking rooms in a skilled nursing facility after a delivery of supplies. In one resident's room, the nurse notes an aerosol can stored on a windowsill in direct afternoon sunlight, a clear path to the bathroom, a wheelchair with locked brakes parked by the bed, and a call light within reach. Which action should the nurse take first?

- A.** Move the aerosol can to an approved storage area away from heat and sunlight
- B.** Reposition the wheelchair closer to the doorway
- C.** Place the call light on the overbed table instead of the bed
- D.** Ask housekeeping to mop the bathroom floor

27. An older adult hospitalized with a urinary tract infection becomes confused and repeatedly tries to slide out of a chair to "go catch the bus." The client is weak, not aggressive, and becomes calmer when the nurse sits at eye level and speaks slowly. The unit clerk suggests tying the client into the chair so the nurse can finish medication pass. Which action by the nurse is most appropriate?

- A.** Apply a vest restraint because the client is confused and at risk for falling

- B.** Ask the provider for an as-needed restraint order to use during busy times
- C.** Move the client closer to the nurses' station and use frequent observation and redirection
- D.** Raise all side rails and leave the client in the chair until medications are completed

28. At the start of the shift, a nurse finds that a client's allergy wristband lists penicillin, but the electronic medication record does not show any allergies. No medications have been administered yet. Which action is most appropriate?

- A.** Remove the wristband because the electronic record is the permanent source of information
- B.** Give scheduled medications that are not antibiotics and update the allergy record later
- C.** Stop and clarify the discrepancy immediately according to facility policy before administering medications
- D.** Ask the client whether the allergy is severe enough to matter before deciding what to do

29. A nurse is preparing to use a full-body mechanical lift to transfer a client from bed to chair. Which action is most important to complete before raising the client off the bed?

- A.** Place the chair directly behind the nurse
- B.** Verify that the sling loops are securely attached to the lift hooks
- C.** Elevate the bed to its highest position
- D.** Ask the client to hold both side rails during the lift

30. A practical nurse is caring for a client who is on droplet precautions for suspected meningococcal meningitis. The client must be transported to the radiology department for a CT scan. Which action is most appropriate to reduce the risk of transmission during transport?

- A.** Place a surgical mask on the client before leaving the room
- B.** Ask transport staff to wear sterile gloves while moving the client
- C.** Delay the CT scan until droplet precautions are discontinued
- D.** Cover the stretcher with a sterile drape before transport

31. A nurse is preparing to administer an intramuscular injection to a client who is not on isolation precautions. The nurse does not anticipate splashing or contact with the client's body fluids other than possible brief contact with a small amount of blood at the injection site. Which personal protective equipment is most appropriate?

- A.** Sterile gloves only
- B.** Clean gloves only
- C.** Clean gloves and a gown

- D.** Clean gloves, mask, and goggles

32. A practical nurse is assisting with a sterile dressing change. The nurse opens a sterile package of forceps and places the forceps onto the sterile field without touching them. During the procedure, the forceps slip and fall to the floor. Which action should the nurse take next?

- A.** Pick up the forceps and wipe them with sterile gauze before using them
- B.** Place the forceps back on the sterile field if the handles did not touch the floor
- C.** Discard the forceps and obtain a new sterile pair
- D.** Rinse the forceps with sterile saline and continue the procedure

33. A practical nurse is caring for a client with active *Clostridioides difficile* infection who is on contact precautions. The nurse notes a sign outside the room that says, "Use alcohol-based hand rub before entering and after leaving." Which action should the nurse take?

- A.** Follow the sign because alcohol-based hand rub is recommended for all clients on contact precautions
- B.** Use alcohol-based hand rub after removing gloves, then wash with soap and water at the end of the shift
- C.** Wash hands with soap and water after removing gloves because alcohol-based hand rub may not be sufficient for this organism
- D.** Keep gloves on until reaching the hallway hand sanitizer dispenser, then remove them and clean hands there

34. A practical nurse is assisting a client with changing a heavily soiled bed pad after an episode of diarrhea. The client has no isolation orders. The nurse expects contact with stool but does not expect splashing. Which action best follows standard precautions?

- A.** Wear gloves while removing the soiled pad and perform hand hygiene after glove removal
- B.** Use a sterile gown and sterile gloves because stool is present
- C.** Skip gloves because the client's skin is intact and there are no isolation orders
- D.** Wear gloves only if the stool has a visible blood streak

35. A client in labor has spontaneous rupture of membranes. The nurse notes a gush of clear fluid and, on the next assessment, sees the fetal heart rate tracing gradually decrease from 140/min to 125/min and return to baseline in a smooth U-shaped pattern with contractions. The client is 6 cm dilated and reports increasing pelvic pressure. How should the nurse interpret this fetal heart rate pattern?

- A.** Umbilical cord compression requiring immediate emergency intervention
- B.** Fetal head compression, an expected finding during labor

- C. Uteroplacental insufficiency indicating fetal hypoxia
- D. Marked fetal distress caused by maternal fever

36. A nurse is reinforcing discharge teaching for a client who is 3 days postpartum after an uncomplicated vaginal birth. The client says, "My breasts feel full and tender today." Which instruction should the nurse give?

- A. Apply warm compresses before breastfeeding and feed the baby frequently
- B. Limit fluid intake for 24 hours to reduce milk production
- C. Wear a tight breast binder continuously until the discomfort stops
- D. Avoid breastfeeding until breast tenderness is completely gone

37. At a preschool screening, the nurse asks a 5-year-old child to perform several tasks. Which action is most consistent with expected development for this age?

- A. Copies a circle and uses mostly one-word answers
- B. Hops on one foot and prints a few letters or numbers
- C. Stacks two blocks and transfers a toy from one hand to the other
- D. Sits without support and begins to show stranger anxiety

38. A practical nurse is reinforcing teaching with the daughter of a 74-year-old client who lost his spouse 4 months ago. The daughter says, "Since moving to a smaller apartment, my father keeps saying he misses the house, forgets where he put things, and does not want to join any social activities." Which response by the nurse is most appropriate?

- A. These behaviors mean your father is no longer able to make decisions for himself.
- B. Adjustment to several losses at once can take time, so keeping familiar routines and encouraging gradual involvement may help.
- C. Avoid talking about the move or your mother's death so your father does not become more upset.
- D. The best way to help is to insist that your father attend group activities every day.

39. A practical nurse is reinforcing teaching with a client who asks, "Why do they ask about my family history before deciding which screening tests I may need?" Which response by the nurse is best?

- A. Family history helps identify health conditions for which a person may have higher risk and may need closer screening.
- B. Family history is mainly collected to diagnose diseases that are already causing symptoms.
- C. If a client has no family history of illness, screening is no longer necessary.

- D. Family history affects treatment choices only and does not change screening recommendations.

40. A nurse is collecting a health history from an adult client. Which client statement should the nurse identify as subjective data?

- A. Blood pressure is 148/92 mm Hg.
- B. Temperature is 38.1°C (100.6°F).
- C. I have had a sore throat for 3 days.
- D. The throat appears red and swollen.

41. A practical nurse is admitting an adult client who has chronic low back pain. To establish a useful baseline for future comparison, which assessment should the nurse document most specifically?

- A. The client reports back pain.
- B. Pain is rated 6/10 in the lower back, described as aching, worsens with walking, and improves with rest.
- C. The client appears uncomfortable when moving.
- D. Pain medication is taken at home as needed.

42. A nurse in a prenatal clinic is reviewing records for several clients who are due for immunization teaching. Which client should the nurse identify as the highest priority to discuss receiving the Tdap vaccine during pregnancy?

- A. A client at 29 weeks of gestation who received Tdap 3 years ago
- B. A client at 10 weeks of gestation who received Tdap 1 year ago
- C. A client at 36 weeks of gestation who received a tetanus booster 2 years ago
- D. A client at 8 weeks of gestation who has never received an influenza vaccine

43. A nurse is reinforcing teaching with the caregiver of an 18-month-old toddler about nutrition. Which statement by the caregiver indicates an understanding of age-appropriate feeding?

- A. I will keep giving my child whole milk instead of switching to skim milk.
- B. I should expect my child to finish three large meals each day.
- C. I need to avoid letting my child try finger foods independently.
- D. I should offer only clear liquids when my child refuses a meal.

44. A client who was told that vision loss from glaucoma is likely to worsen says to the practical nurse, "I used to read every evening. If I lose that too, I don't know what I'll have left." Which response by the nurse best provides emotional support?

- A. You should focus on the treatments, because worrying will only make this harder.

- B.** Many people adjust to vision changes over time, so try not to think the worst.
- C.** It sounds like losing your vision feels tied to losing an important part of your daily life.
- D.** Would you like me to ask the provider to explain the diagnosis again?

45. A client with rheumatoid arthritis tells the nurse, "By late afternoon my joints hurt so much that I snap at everyone. Then I feel guilty and pretend I'm fine instead of asking for help." Which response by the nurse best promotes adaptive coping?

- A.** "Try not to think about the pain so much, and your mood will probably improve."
- B.** "You should apologize to your family and focus on being more patient with them."
- C.** "Let's talk about what signs tell you stress is building and identify one way you could ask for help before you feel overwhelmed."
- D.** "It is best to rest alone in your room until the pain goes away so you do not upset anyone."

46. A nurse in a medical-surgical unit is caring for a client who becomes increasingly agitated after receiving unexpected test results. The client begins pounding a fist into an open hand, speaking more loudly, and saying, "I can't take this anymore." Which action should the nurse take first?

- A.** Ask the client to explain why the test results are upsetting
- B.** Move the client to a quiet area and remain with the client using a calm, nonthreatening approach
- C.** Tell the client that this behavior is unacceptable and must stop immediately
- D.** Administer a PRN sedative medication right away

47. A client is admitted to the unit for treatment of pneumonia. During the admission interview, the client reports drinking heavily every day and says, "If I stop suddenly, I've had seizures before." Which nursing action is most appropriate?

- A.** Encourage the client to rest and reassess for withdrawal symptoms at the next scheduled vital-sign check
- B.** Institute seizure precautions and promptly notify the provider of the client's withdrawal history
- C.** Ask the client to sign a safety contract agreeing to call for help if feeling worse
- D.** Limit fluids until nausea has resolved to reduce the risk of aspiration

48. A client with schizophrenia is sitting in the dayroom staring toward a corner and whispering, "They keep telling me you're trying to poison me." The client suddenly refuses scheduled oral medication and begins breathing faster, but remains seated and does not threaten anyone. Which action by the nurse is most appropriate?

- A.** Tell the client the voices are not real and insist that the medication be taken now
- B.** Ask the client what the voices are saying and whether the client feels able to stay in control
- C.** Leave the client alone in the dayroom to reduce stimulation until the hallucinations stop
- D.** Call security immediately to stand by in case the client becomes violent

49. A client whose partner died 3 months ago says to the nurse, "People keep telling me to stay busy, but when I get home the house feels so quiet that I can't stand it." Which response by the nurse is most therapeutic?

- A.** You should try joining more activities so you are not alone so much.
- B.** At least you had several good years together to remember.
- C.** It sounds like the quiet at home feels especially painful for you.
- D.** Have you thought about getting a pet to keep you company?

50. A nurse in a community health clinic is checking in a client who becomes visibly angry after learning the prescriber is running 45 minutes late. The client steps close to the desk, speaks loudly, and says, "This is a joke. If nobody is going to help me, I'm leaving and making sure everyone hears about this." Which action by the nurse is most appropriate?

- A.** Tell the client that threatening behavior is unacceptable and security will be called if it continues
- B.** Ask the client to sit down and lower their voice before the nurse continues the conversation
- C.** Move to a calm tone, give the client personal space, and say, "I can see you're frustrated. Let's step over here so we can talk about what you need right now."
- D.** Explain that many clients are waiting and the delay is not the staff's fault

51. A resident with moderate dementia becomes anxious at lunchtime and says, "I need to get to work right now or I'll be late." The resident walks toward the exit and tries to open the door. Which response by the practical nurse is most appropriate?

- A.** "You do not work anymore. Please sit down and eat your lunch now."
- B.** "Your job called and said you are off today. Come have lunch with me first."

C. "If you keep trying to leave, I will have to call the provider."

D. "Tell me the full date and where you are before we talk about leaving."

52. A client tells the nurse, "When I get stressed, I skip meals, stay up late trying to catch up, and tell myself I will rest after everything is done." Which client statement indicates an understanding of effective stress management?

A. "I should keep pushing until all of my responsibilities are finished before taking breaks."

B. "I will plan short breaks, eat regular meals, and aim for a consistent bedtime even when I am busy."

C. "I should avoid thinking about my stress so I can stay productive."

D. "I will cut back on sleep for a few days so I have more time to get things done."

53. A client is admitted for treatment of pneumonia. During the assessment, the nurse asks whether there are any personal concerns that could affect the client's comfort in the hospital. The client says, "I am transgender, and I want respectful care, but I do not want everyone involved in my treatment asking me personal questions about my body or transition." Which response by the nurse is best?

A. "I understand. I will make sure questions stay focused on your care needs and explain when sensitive information is clinically necessary."

B. "The staff needs complete details about your transition so we can avoid mistakes."

C. "It may be easier if you wait to discuss your gender identity unless a provider brings it up first."

D. "Once your chart is updated, everyone on the unit should ask the same questions so the information is consistent."

54. A client who recently lost an eye after trauma says to the nurse, "I know the surgery saved me, but I don't want anyone to visit. I don't want them to see me like this." Which response by the nurse is best?

A. "You should try not to worry about what other people think."

B. "Many people feel this way at first, but you will probably adjust soon."

C. "It sounds like you're uncomfortable with the change in your appearance. Tell me more about what worries you most."

D. "Your family will be upset if you keep refusing visitors."

55. A client with end-stage renal disease who is receiving end-of-life care says to the nurse, "I need to apologize to my brother, but I don't know if he would even take my call." Which nursing action is most appropriate?

A. Tell the client that unresolved family issues are common and suggest focusing on comfort instead

B. Ask the client whether they would like help exploring ways to contact the brother

C. Offer to call the brother immediately and explain that the client wants to apologize

D. Reassure the client that the brother probably already understands and there is no need to bring it up now

56. A nurse is caring for a client who has a newly placed gastrostomy tube connected to continuous enteral feeding. Which finding should the nurse report to the provider immediately?

A. A small amount of serosanguineous drainage on the dressing

B. Mild tenderness at the insertion site during the first day after placement

C. A rigid abdomen with severe pain and guarding

D. Scant crusting around the tube at the skin edge

57. A nurse is reinforcing teaching for a client who has left-leg weakness and is learning to walk with a quad cane on a flat surface. Which client action shows correct technique?

A. Moves the left leg forward first, then the cane, then the right leg

B. Moves the cane forward with the right leg, then brings the left leg forward

C. Moves the cane forward with the left leg, then steps through with the right leg

D. Keeps the cane on the left side and advances the right leg before moving the cane

58. A client with a closed fracture of the left forearm in a newly applied cast reports aching pain and says, "It feels a little better when I hold it up." Which nonpharmacologic intervention should the nurse implement?

A. Place a heating pad over the casted arm

B. Elevate the arm on pillows above heart level

C. Massage the fingers of the affected hand vigorously

D. Encourage the client to let the arm hang at the side while walking

59. A nurse is caring for a client who had a right-sided stroke 3 days ago and has left-sided weakness. The client wants to brush their teeth independently while sitting at the sink. Which nursing action is most appropriate?

A. Place the toothbrush in the client's left hand and encourage repeated practice

B. Complete the toothbrushing for the client to prevent fatigue

C. Set up supplies within reach on the client's right side and allow extra time to complete the task

D. Instruct the client to remain on bed rest until strength returns

60. A nurse is caring for a client who has a spinal cord injury and follows a daily bowel program. The client asks why the bowel routine is scheduled at the same time every day instead of whenever it is convenient. Which response by the nurse is best?

A. "A regular schedule helps train the bowel to empty more predictably."

B. "Changing the time each day prevents the bowel from becoming dependent on stimulation."

C. "The bowel program must be done before any food or fluids are taken."

D. "Doing the bowel program at bedtime reduces the risk of autonomic dysreflexia."

61. A nurse is caring for a client who asks why the nurse is encouraging a short walk in the hallway during the day instead of staying in bed to "save energy" for nighttime sleep. Which explanation by the nurse is best?

A. Limiting daytime activity helps the body produce more melatonin before bedtime.

B. Moderate daytime activity can promote better sleep at night by reducing excess daytime sleeping and supporting a normal sleep-wake pattern.

C. Walking in the daytime prevents the need for repositioning during the night.

D. Daytime exercise guarantees the client will sleep through the night without awakening.

62. A practical nurse is caring for a client who has weakness on the right side after a stroke. While transferring the client from bed to chair, the nurse notices the client's right arm is dragging against the bedrail and the skin on the forearm looks fragile. Which action is best to protect the client's skin and tissue integrity during future transfers?

A. Place the client's right arm across the chest and support it throughout the transfer

B. Massage the client's right forearm before each transfer to improve circulation

C. Pull the client quickly to the standing position to reduce transfer time

D. Allow the client to use the bedrail for balance with the weak arm

63. A practical nurse is caring for a client with diabetic neuropathy who cannot feel pain well in the feet. The client asks for a hot water bottle to warm the feet before sleep. Which response by the nurse is most appropriate?

A. Fill the hot water bottle with very warm water and place it under the blanket near the feet

B. Apply the hot water bottle for 10 minutes, then check the skin for redness

C. Explain that a hot water bottle should be avoided and offer warm socks instead

D. Wrap the hot water bottle in a towel so the client can use it safely through the night

64. A practical nurse is helping a client complete a 24-hour urine collection that started at 8:00 a.m. Yesterday, the client's urine at 8:00 a.m. was discarded. Which urine specimen should be included as the final specimen in the collection?

A. The urine voided at 7:45 a.m. today

B. The urine voided at 8:00 a.m. today

C. The urine voided at 8:15 a.m. today

D. Only urine voided before breakfast today

65. A practical nurse is caring for a client who has been immobilized in bed for several days. Which action is most appropriate to help prevent contractures of the lower extremities?

A. Place a pillow under both knees at all times

B. Encourage frequent active or passive range-of-motion exercises

C. Keep the client's feet pointed downward for comfort

D. Limit repositioning to avoid causing fatigue

66. A nurse is preparing to administer a scheduled oral medication to a client. The nurse verifies the prescription, compares the medication with the MAR, and identifies the client using two identifiers. As the nurse opens the package, the client says, "That pill usually looks blue, not white. Are you sure it's mine?" Which action is most appropriate?

A. Tell the client that manufacturers sometimes change pill color and administer the medication as prescribed

B. Withhold the medication until the nurse rechecks the medication against the MAR and pharmacy label

C. Ask another client in the room whether the pill looks familiar before giving it

D. Document that the client is refusing the medication and return the dose to the medication cart

67. A practical nurse is reviewing the MAR before giving a client's 0800 medications. The MAR lists furosemide 40 mg PO daily. In the chart, the nurse sees a provider prescription entered at 0645: "Decrease furosemide to 20 mg PO daily starting today due to dizziness and low blood pressure." The pharmacy has not updated the MAR yet, and the client has not received the morning dose. Which action should the nurse take?

- A.** Administer furosemide 40 mg as listed on the MAR because it is the active medication record
- B.** Hold the medication and document that the client refused the dose
- C.** Clarify the discrepancy and ensure the MAR is updated before administering the medication
- D.** Administer 20 mg based on the new prescription and update the MAR personally after giving it

68. A nurse is preparing to administer a medication to a patient with external ear canal inflammation. The prescription states that the medication should treat the affected area directly. Which route of administration should the nurse use?

- A.** Otic
- B.** Ophthalmic
- C.** Transdermal
- D.** Sublingual

69. A client with a gastrointestinal bleed is receiving a unit of packed red blood cells. After 25 minutes, the client develops itching and scattered hives on the chest and arms. Breath sounds are clear, vital signs are stable, and the client denies chest pain or back pain. Which action should the nurse take first?

- A.** Slow the transfusion rate and continue to monitor the client closely
- B.** Stop the transfusion and maintain IV access with normal saline
- C.** Administer prescribed acetaminophen and restart the transfusion
- D.** Discard the blood tubing and remove the IV catheter immediately

70. A nurse needs to administer 180 mL of IV medication over 1.5 hours using an infusion pump. What rate in mL/hr should the nurse program?

- A.** 90 mL/hr
- B.** 100 mL/hr
- C.** 120 mL/hr
- D.** 135 mL/hr

71. A practical nurse is assigned to administer a prescribed controlled opioid to a client who reports severe incisional pain. Before giving the medication, the nurse notes the client is difficult to awaken, has slurred speech, and has a respiratory rate of 8/min. Which action should the nurse take?

- A.** Administer the opioid as prescribed because the client is reporting severe pain
- B.** Hold the opioid and notify the registered nurse or provider immediately
- C.** Give the opioid with food to reduce adverse effects
- D.** Document the findings and return in 30 minutes to reassess the client

72. A nurse is reinforcing discharge teaching for a client who will continue using a transdermal fentanyl patch at home for chronic pain. Which instruction should the nurse include about disposal of a used patch?

- A.** Fold the patch so the adhesive sides stick together before disposing of it as instructed
- B.** Cut the patch into small pieces before putting it in the trash
- C.** Leave the patch open to the air for several hours so the medication can evaporate
- D.** Place the patch back into the package it came in and save it in case pain worsens

73. A nurse is preparing to administer the next dose of oral diltiazem to a client who has atrial fibrillation. Which pre-administration assessment is most important for the nurse to review?

- A.** Apical pulse rate
- B.** Bowel sounds
- C.** Daily weight
- D.** Oxygen saturation

74. A patient with asthma uses theophylline for long-term control. The patient tells the nurse, "I started taking cimetidine for heartburn last week, and now I feel nauseated and my heart is racing." Which action by the nurse is most appropriate?

- A.** Advise the patient to take both medications with food and continue them as prescribed
- B.** Recognize a possible increase in theophylline levels and notify the provider promptly
- C.** Instruct the patient to stop the cimetidine and double the next dose of theophylline
- D.** Explain that these effects are expected and should resolve after several more days

75. A client with a mechanical heart valve is receiving a continuous unfractionated heparin infusion. The nurse reviews the morning laboratory results and notes a platelet count of 52,000/mm³, decreased from 168,000/mm³ four days ago. Which action is most appropriate?

- A.** Continue the infusion and document the expected anticoagulant effect
- B.** Hold the heparin and notify the provider immediately
- C.** Administer the prescribed heparin bolus and recheck the platelet count in 6 hours
- D.** Reduce the infusion rate and monitor for therapeutic aPTT results

76. A nurse is teaching a client who has a new prescription for oral potassium chloride. Which instruction should the nurse include?

- A.** Take the medication on an empty stomach for best absorption.
- B.** Crush the tablet and mix it with applesauce before taking it.
- C.** Take the medication with food and a full glass of water.
- D.** Stop taking the medication if your urine becomes dark yellow.

77. A nurse removes an unopened bottle of latanoprost eye drops from the medication refrigerator for a client. The label indicates the bottle should be refrigerated before opening and kept at room temperature after first use. What is the most appropriate nursing action?

- A.** Return the bottle to the refrigerator and delay administration until the next dose time
- B.** Administer the medication, then label the bottle for room-temperature storage after opening
- C.** Discard the bottle because medications should not be moved between refrigerated and room-temperature storage
- D.** Warm the bottle in hot water before opening it to prevent eye irritation

78. A nurse is preparing to administer enoxaparin 60 mg subcutaneously. The prefilled syringe is labeled 80 mg/0.8 mL. How many milliliters should the nurse administer?

- A.** 0.4 mL
- B.** 0.5 mL
- C.** 0.6 mL
- D.** 0.75 mL

79. A practical nurse is preparing to assist with care for four clients on a medical unit. Which client should the nurse identify as being at greatest risk for bleeding?

- A.** A client with rheumatoid arthritis who takes ibuprofen for joint pain

- B.** A client with atrial fibrillation who takes warfarin and reports black, tarry stools
- C.** A client with heart failure who takes furosemide and reports leg cramps
- D.** A client with hypothyroidism who takes levothyroxine and reports fatigue

80. A nurse needs to obtain a sterile urine specimen for culture from a client who has a newly inserted indwelling urinary catheter. Which action should the nurse take?

- A.** Disconnect the catheter from the drainage tubing and collect urine from the tubing connection
- B.** Collect urine from the drainage bag using a sterile syringe
- C.** Clamp the tubing briefly, cleanse the sampling port, and withdraw urine from the port with a sterile syringe
- D.** Open the drainage spout and allow urine to drain into a sterile specimen cup

81. A nurse is reviewing morning laboratory results for a client who has heart failure and is taking furosemide. The client now reports leg cramps, weakness, and palpitations. Which laboratory finding should the nurse identify as the priority?

- A.** Potassium 2.8 mEq/L
- B.** Sodium 134 mEq/L
- C.** Blood urea nitrogen (BUN) 24 mg/dL
- D.** Calcium 8.7 mg/dL

82. A client who is 10 weeks pregnant comes to the clinic reporting vaginal spotting and mild cramping for the past 6 hours. Which finding should the nurse identify as most concerning and report to the provider immediately?

- A.** Passage of tissue from the vagina
- B.** Nausea earlier in the morning
- C.** Urinary frequency for 2 days
- D.** Breast tenderness

83. A nurse is preparing to perform a bedside capillary blood glucose test for a client. After cleansing the fingertip with alcohol, the nurse notices the site is still wet. Which action is most appropriate?

- A.** Puncture the site immediately so the sample does not clot
- B.** Wipe away the first drop of blood and use the second drop only
- C.** Allow the fingertip to dry completely before obtaining the sample
- D.** Massage the finger firmly to mix the alcohol with capillary blood

84. A practical nurse is measuring blood pressure manually on an adult client. The nurse inflates the cuff, begins deflating it, and hears the first tapping sound at 146 mm Hg. The tapping sounds become muffled at 88 mm Hg and disappear at 82 mm Hg. Which blood pressure should the nurse document?

- A. 146/88 mm Hg
- B. 146/82 mm Hg
- C. 88/82 mm Hg
- D. 82/146 mm Hg

85. A client is scheduled for a myelogram this afternoon. Which information obtained during the preprocedure assessment should the nurse report to the provider immediately?

- A. The client drank coffee with breakfast 6 hours earlier
- B. The client reports a history of seizures
- C. The client asks whether a headache can occur afterward
- D. The client voided 1 hour before arriving to radiology

86. A nurse is obtaining a 12-lead ECG for a client who is anxious and repeatedly talks while the tracing is being recorded. The monitor display shows wandering baseline artifact. Which instruction by the nurse is most appropriate?

- A. Take several deep breaths during the recording
- B. Hold still and avoid talking until the tracing is complete
- C. Cough each time the nurse applies a lead
- D. Turn onto the left side for better lead contact

87. A nurse is preparing to obtain a venous blood specimen from an adult client. The nurse inserts the needle but no blood enters the collection tube. The client reports no pain, and the needle does not appear to have gone through the vein. Which action should the nurse take first?

- A. Withdraw the needle completely and choose a new site
- B. Advance the needle slightly in case the bevel is not fully in the vein
- C. Probe side to side under the skin until blood flow begins
- D. Apply a second tourniquet above the first one

88. A practical nurse is caring for a client who is scheduled for surgery in 30 minutes. During the final preoperative check, the nurse notes the consent form is signed, but the client states, "I thought this procedure was to remove my whole gallbladder, not just the stones." Which action is most appropriate?

- A. Notify the RN or provider and hold transport to surgery until the client's understanding is clarified
- B. Send the client to surgery because the consent form has already been signed
- C. Explain the procedure in detail to the client and then send the client to surgery

D. Ask a family member to confirm that the client understood the procedure earlier

89. A client with a continuous peripheral IV infusion has redness, warmth, and a palpable cord along the vein above the insertion site. The client reports tenderness when the area is touched. Which action should the nurse take?

- A. Slow the infusion rate and reassess the site in 30 minutes
- B. Discontinue the IV catheter and restart the IV at a different site
- C. Apply a pressure dressing and continue to use the same catheter
- D. Flush the catheter with normal saline to confirm patency

90. A client with acute pyelonephritis has been receiving IV antibiotics for 36 hours. Which finding best indicates the treatment is effective?

- A. Urine output increases from 30 mL/hr to 35 mL/hr
- B. The client reports flank pain decreased from 8/10 to 3/10
- C. Oral temperature decreases from 39.2°C (102.6°F) to 37.6°C (99.7°F)
- D. Blood pressure changes from 138/84 mm Hg to 128/80 mm Hg

91. A client with a history of peptic ulcer disease suddenly reports black, tarry stools and dizziness when standing. The nurse notes cool skin, heart rate 116/min, blood pressure 92/58 mm Hg, and capillary refill of 4 seconds. Which action should the nurse take first?

- A. Place the client flat with legs elevated and apply oxygen
- B. Obtain a stool specimen for occult blood testing
- C. Administer the prescribed oral antacid
- D. Encourage the client to increase oral fluid intake

92. A client with chronic heart failure is prescribed a 2-gram sodium diet. During meal planning, which lunch choice is most appropriate?

- A. Canned tomato soup with crackers and deli turkey slices
- B. Baked chicken breast, brown rice, and steamed green beans
- C. Ham and cheese sandwich with pickles
- D. Instant noodles with a side of cottage cheese

93. A client with a history of deep vein thrombosis suddenly reports shortness of breath and pleuritic chest pain while walking to the bathroom. The nurse notes restlessness, tachycardia, and oxygen saturation of 84% on room air. Which action should the nurse take first?

- A. Place the client in high-Fowler's position and apply oxygen
- B. Obtain a prescription for an anticoagulant

- C. Encourage the client to cough and deep breathe
 - D. Ambulate the client back to bed to improve circulation
- 94.** A client has a ventriculoperitoneal (VP) shunt for hydrocephalus. Which finding should the nurse report to the provider immediately?
- A. The client reports a mild headache after coughing
 - B. The client has redness and tenderness along the shunt tract with a temperature of 38.6°C (101.5°F)
 - C. The client prefers to lie on the nonoperative side
 - D. The dressing has a small area of dried serosanguineous drainage
- 95.** A nurse is teaching a client about ways the body can lose heat. Which situation is the best example of heat loss through radiation?
- A. Sitting near a cold window and losing body heat to the cooler surface without touching it
 - B. Stepping out of a swimming pool and cooling as water dries from the skin
 - C. Holding an ice pack directly against the forearm
 - D. Standing in front of a fan that moves air across moist skin
- 96.** A client with a temporary transcutaneous pacemaker is being stabilized while awaiting transfer to a higher level of care. The monitor shows pacing spikes followed by wide QRS complexes at a rate of 70/min. The client is drowsy, and the nurse notices a palpable pulse of 42/min that does not match the monitor rate. Which interpretation is most appropriate?
- A. The pacemaker is sensing appropriately but firing too slowly
 - B. The monitor leads are probably attached incorrectly
 - C. Electrical capture is present, but mechanical capture is absent
 - D. The pacemaker is oversensing the client's intrinsic rhythm
- 97.** A nurse is caring for a client 4 days after creation of a new colostomy. During assessment, which finding should the nurse recognize as most concerning?
- A. The stoma appears pale grayish-blue
 - B. The stoma is slightly edematous and moist
 - C. A small amount of mucus is noted in the pouch
 - D. The client reports mild tenderness around the incision
- 98.** A nurse is caring for a client with chronic obstructive pulmonary disease who has been stable on 2 L/min oxygen by nasal cannula. The client was alert and talking 30 minutes ago. Which new finding should the nurse recognize as the priority change in condition?
- A. The client requests assistance to sit in a chair for dinner
 - B. The client has a productive cough with thick yellow sputum
 - C. The client is difficult to arouse and has respirations of 8/min
 - D. The client reports dry lips and irritation from the nasal cannula
- 99.** A nurse is caring for a client admitted with severe dehydration after working outdoors in extreme heat. Which finding is most consistent with fluid volume deficit?
- A. Bounding peripheral pulses and jugular vein distention
 - B. Crackles in the lungs and dependent edema
 - C. Flat neck veins and orthostatic hypotension
 - D. Bradycardia and moist mucous membranes
- 100.** A client receiving hemodialysis through a left arm arteriovenous fistula reports numbness and pain in the left hand. The hand is cool and pale compared with the right hand. Which action should the practical nurse take first?
- A. Elevate the left arm on pillows and reassess in 30 minutes
 - B. Apply a warm compress to the left hand to improve circulation
 - C. Report the findings immediately because they may indicate impaired blood flow distal to the fistula
 - D. Encourage the client to squeeze a rubber ball to strengthen the access site

Simulation Test 23 — Answer Key

1. B	26. A	51. B	76. C
2. C	27. C	52. B	77. B
3. C	28. C	53. A	78. C
4. A	29. B	54. C	79. B
5. B	30. A	55. B	80. C
6. C	31. B	56. C	81. A
7. C	32. C	57. C	82. A
8. B	33. C	58. B	83. C
9. B	34. A	59. C	84. B
10. C	35. B	60. A	85. B
11. B	36. A	61. B	86. B
12. C	37. B	62. A	87. B
13. B	38. B	63. C	88. A
14. A	39. A	64. B	89. B
15. C	40. C	65. B	90. C
16. B	41. B	66. B	91. A
17. B	42. A	67. C	92. B
18. C	43. A	68. A	93. A
19. C	44. C	69. B	94. B
20. C	45. C	70. C	95. A
21. B	46. B	71. B	96. C
22. A	47. B	72. A	97. A
23. C	48. B	73. A	98. C
24. B	49. C	74. B	99. C
25. B	50. C	75. B	100. C

Simulation Test 24

100 questions

1. A practical nurse in an outpatient dialysis center is notified of four client updates at the same time. Which client should the nurse assess first?
 - A. A client who reports muscle cramps near the end of treatment
 - B. A client whose blood pressure has dropped to 84/50 mm Hg and who says, "I feel dizzy"
 - C. A client who asks for a blanket because the room feels cold
 - D. A client who wants to know whether today's treatment will finish on time
2. A practical nurse in a rehabilitation center is preparing morning care when these events occur at once. Which action should the nurse take first?
 - A. Check on a client with Parkinson disease who is due for scheduled medication in 20 minutes
 - B. Assist a client with a healed hip fracture who is asking to walk to the bathroom
 - C. Go to a client with diabetes who is pale, diaphoretic, and shaking while waiting for breakfast
 - D. Answer a family member's question about visiting hours for a newly admitted client
3. A practical nurse is assigning tasks for the morning in a long-term care facility. Which task is appropriate to delegate to an experienced assistive personnel (AP)?
 - A. Teach a client how to use a walker safely for the first time
 - B. Perform a skin assessment on a client with redness over the coccyx
 - C. Assist a stable client with eating breakfast
 - D. Evaluate whether a client's PRN pain medication relieved discomfort
4. A practical nurse asks an assistive personnel (AP) to help a stable client transfer from bed to chair after the nurse has already assessed the client as safe for a one-person assist. A few minutes later, the AP says, "The client suddenly became weak and almost fell, so I put the client back in bed." What is the nurse's best action?
 - A. Ask the AP to try the transfer again in 30 minutes
 - B. Document the event as completed and continue with other tasks
 - C. Tell the AP to monitor the client and report if it happens again
 - D. Assess the client immediately for injury and change in condition
5. A nurse is caring for an adult client who is alert, oriented, and has decision-making capacity. The client says, "My partner can stay during my care, but I do not want them making decisions for me or signing anything." Later, when the client is away for imaging, the partner asks to sign the consent for a bedside procedure scheduled for that afternoon, stating, "I know what the client wants." What should the nurse do?
 - A. Allow the partner to sign because they are closely involved in the client's care
 - B. Ask another nurse to witness the partner's signature so the procedure is not delayed
 - C. Explain that the alert client must provide consent personally and notify the provider that the client has questions or is unavailable
 - D. Have the partner sign now and confirm the consent with the client after the procedure
6. A nurse is caring for an adult client who is alert, oriented, and recovering after surgery. The client says, "I want to file a complaint. The staff member in pre-op spoke to me in a way that felt disrespectful, and I do not want this ignored." Which action by the nurse best demonstrates advocacy?
 - A. Encourage the client to wait until discharge so emotions have time to settle
 - B. Tell the client that interpersonal concerns are not part of nursing care unless harm occurred
 - C. Document the client's statement and provide information on how to contact the appropriate patient representative or grievance process
 - D. Offer to speak privately with the staff member instead of involving any formal process
7. A practical nurse is calling a consulting provider from the nurses' station about a client whose condition has changed. Several visitors are seated nearby. Which action best protects the client's privacy while allowing needed care coordination?
 - A. Delay the call until the end of the shift so no visitors can overhear
 - B. Move to a more private area and share only the client information needed for the consultation
 - C. Ask the visitors to keep the information confidential before making the call
 - D. Speak quietly at the desk and avoid using the client's room number

8. A practical nurse is preparing teaching materials for a client newly diagnosed with hepatitis C. The nurse plans to leave the packet at the bedside before the client returns from a procedure. The client is in a semiprivate room, and the roommate's family is visiting. Which action is most appropriate?

- A.** Leave the packet face down on the overbed table so the title is less visible
- B.** Place the packet in the bedside drawer because items in the client's area are automatically private
- C.** Hold the teaching materials until the client returns and provide them directly in a way that limits unintended viewing
- D.** Give the packet to the roommate's family and ask them to place it with the client's belongings

9. An alert adult client preparing for a planned procedure tells the nurse, "I signed an advance directive years ago, but I changed my mind about some of my choices. I haven't updated the paperwork yet." Which response by the nurse is most appropriate?

- A.** "Your original advance directive must still be followed until a provider cancels it."
- B.** "You can review and revise your advance directive while you have decision-making capacity."
- C.** "Your family should decide now which parts of the document still apply."
- D.** "Once a procedure is scheduled, advance directives cannot be changed."

10. A client admitted with acute respiratory failure is intubated and no longer able to make decisions. The chart includes an advance directive, but the copy is partly illegible and the nurse cannot clearly determine the client's stated wishes about prolonged life support. The client's sibling says, "I know exactly what my brother would want," while another relative disagrees. Which action should the nurse take first?

- A.** Ask the relatives to vote on the plan of care and follow the majority decision.
- B.** Delay all communication until the client regains decision-making capacity.
- C.** Notify the provider and follow facility policy to clarify the client's documented wishes and identified decision-maker.
- D.** Use the sibling's statement because family members are the best source when paperwork is unclear.

11. A client who speaks very little English is scheduled for an elective thyroid biopsy. The provider has explained the procedure in English, and the client's bilingual teenage grandson says, "I can translate everything so we can get this done faster." Which action by the nurse is most appropriate?

- A.** Allow the grandson to translate because the procedure is minor and the client appears calm
- B.** Ask the grandson to explain only the risks while the nurse explains the rest
- C.** Use a qualified medical interpreter before the client signs the consent form
- D.** Have the client sign the form if the grandson says the client understood the explanation

12. A nurse is preparing a client for an elective arteriogram. The consent form is signed, and the client is alert and oriented. While verifying the preprocedure checklist, the client says, "The provider told me I would have some bleeding and that the dye could affect my kidneys. I understand that part. I just need to know whether there are any other treatment choices besides this test." Which action should the nurse take?

- A.** Witness the consent because the client already understands the main risks
- B.** Explain the available alternatives so the procedure is not delayed
- C.** Notify the provider that the client has questions about alternatives before proceeding
- D.** Ask the client to sign a new consent form after the nurse reviews the procedure again

13. A client is preparing for discharge after a left-hemispheric stroke. The client walks with a cane and will stay with an adult daughter. During teaching, the client nods and says, "Yes, I understand," but gives unrelated answers when asked how to take the new medications. Which nursing action is the priority before discharge?

- A.** Provide the written instructions to the daughter only and continue with discharge
- B.** Ask the provider to delay all discharge teaching until the follow-up visit
- C.** Use a teach-back method with short, simple instructions and involve the daughter in the education
- D.** Request that the pharmacy place all medications in childproof containers

14. A client is being discharged after treatment for a diabetic foot infection. The client will need daily dressing changes, blood glucose monitoring, and oral antibiotics at home. The client is alert, understands the medication schedule, and says, "My niece can stop by some evenings, but she doesn't know wound care." The nurse observes that the client cannot reach the wound well enough to change the dressing independently. Which action should the nurse take first?

- A.** Arrange a home health referral for wound assessment and dressing care after discharge
- B.** Teach the niece how to perform sterile dressing changes over the phone after discharge
- C.** Tell the client to return to the emergency department each day for dressing changes
- D.** Ask the provider to discontinue the dressing-change prescription

15. A clinic wants to improve the percentage of clients with newly prescribed antibiotics who correctly state how and when to take the medication before leaving. The team is choosing a measure to determine whether the new nurse teach-back script is improving the client outcome. Which measure is best?

- A.** The percentage of nurses who attended the in-service on the teach-back script
- B.** The percentage of eligible discharges in which the teach-back script was documented
- C.** The percentage of clients who can accurately explain the antibiotic instructions before discharge
- D.** The total number of antibiotic prescriptions written each week

16. A unit is trying to reduce urinary catheter-associated infections. Review of recent cases shows that catheters are often inserted without a clear ongoing need and then remain in place for several days. The team wants to test a change next week. Which plan best reflects sound quality improvement practice?

- A.** Require every nurse on all shifts to follow a new catheter removal policy starting tomorrow, then compare infection rates at the end of 6 months
- B.** Begin a small test on one unit using a daily catheter-necessity prompt during rounds, track catheter days and prompt completion, and adjust the process based on what is learned
- C.** Wait to make any change until the infection prevention department can guarantee which intervention will eliminate the infections
- D.** Focus only on clients who already developed infections and provide extra treatment teaching at discharge

17. A practical nurse is caring for a client admitted with community-acquired pneumonia. During the shift, the nurse notes the client is more fatigued, is coughing during meals, and ate only a few bites of lunch. The assistive personnel reports the client needed help sitting fully upright and became short of breath while eating. Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the poor intake at the end of the shift and continue encouraging the client to eat
- B.** Notify the charge nurse or provider and communicate the coughing with meals, shortness of breath while eating, need for upright positioning, and poor intake so the team can evaluate swallowing and nutrition needs
- C.** Ask dietary to send softer foods on the next tray without reporting the client's symptoms
- D.** Tell the client to eat more slowly at the next meal and reassess tomorrow

18. A practical nurse is caring for an alert, oriented adult client who is scheduled for a central line insertion at the bedside. When the provider arrives, the client says, "I signed the form earlier, but now I'm not sure I want this done." What should the nurse do first?

- A.** Ask the client to wait until after the procedure to discuss concerns
- B.** Tell the client that signing the form means the procedure must go forward
- C.** Stop preparation for the procedure and notify the provider that the client is expressing uncertainty
- D.** Explain the risks and benefits of the procedure in detail so the client can decide

19. A practical nurse in a community clinic is caring for a 14-year-old client who came in for treatment of abdominal pain. During the visit, the client quietly says, "My mom's boyfriend hit my little brother with a belt last night. He has marks on his back. Please don't tell anyone because he'll get mad." Which action by the nurse is most appropriate?

- A.** Respect the client's request for confidentiality because the information involves another family member
- B.** Tell the client to have the brother come to the clinic so the nurse can decide whether abuse occurred
- C.** Report the suspected child abuse according to agency policy and document the client's statement objectively
- D.** Wait to report until the nurse can speak directly with the client's mother and confirm the story

20. A practical nurse (PN) is supervising an assistive personnel (AP) on a medical-surgical unit. Which action by the PN best demonstrates appropriate management of the AP during the shift?

- A.** Ask the AP to teach a newly diagnosed client how to use an incentive spirometer

- B.** Assign the AP to obtain routine weights on stable clients and report any unexpected changes
- C.** Have the AP decide which postoperative client is ready for ambulation first
- D.** Tell the AP to notify the health care provider directly if a client reports new chest pain

21. A practical nurse in a wound care clinic is reinforcing the plan of care for a client with a venous leg ulcer. The client says, "I want to come to the weekly dressing checks, but I work at a packing plant. If I miss a shift, I may lose pay, and the clinic only has appointments from 8 a.m. to 4 p.m." Which nursing recommendation best promotes equitable access to care at the system level?

- A.** Tell the client to ask the employer for unpaid time off for each visit
- B.** Encourage the client to go to the emergency department if the wound gets worse between visits
- C.** Recommend adding evening or weekend wound care appointments for clients with daytime work barriers
- D.** Advise the client to keep the current appointment schedule and review the risks of nonadherence

22. A nurse in a community health clinic is preparing to give an intramuscular vaccine to a client who does not speak English fluently. The client's adult daughter says, "This is my mother, Rosa Martinez, born in May 1958," and begins rolling up the client's sleeve. Which action should the nurse take to identify the client correctly before administering the vaccine?

- A.** Accept the daughter's statement because she is a family member accompanying the client
- B.** Ask the client to state her full name and date of birth, using an interpreter if needed, and compare that information with the clinic record
- C.** Confirm the client's identity by checking the name printed on the exam room door
- D.** Use the client's apparent age and the scheduled vaccine appointment time as identifiers

23. A nurse is preparing to place a client in a chair alarm pad after the client was identified as a high fall risk. During setup, the nurse notices the alarm cord does not click securely into the monitor box and disconnects with slight movement. The client is awake, confused, and tries to get up without calling for help. Which action should the nurse take?

- A.** Use the alarm anyway and check the client more often
- B.** Tape the cord in place and document that the alarm is unreliable
- C.** Obtain functioning alarm equipment before leaving the client in the chair
- D.** Ask a family member to watch the client until the next rounding time

24. A nurse is preparing to give a client the first dose of a chewable medication. The client says, "I have a severe allergy to aspartame and once had hives and throat tightness after taking a sugar-free product." The package insert for the medication lists aspartame as an inactive ingredient, and no allergy is documented in the record. Which action should the nurse take first?

- A.** Administer the medication and observe the client closely for 30 minutes
- B.** Hold the medication, document the reported allergy, and notify the provider and pharmacy
- C.** Ask another nurse to verify the ingredient list before giving the medication
- D.** Give half the dose first to test whether the client reacts

25. A nurse is preparing a client for surgery and reviews the preoperative checklist. The client reports a previous anaphylactic reaction to chlorhexidine used during a procedure. The skin-prep applicator placed in the room contains chlorhexidine, and the provider says, "We use this for everyone." Which response by the nurse is most appropriate?

- A.** Proceed with the prep because the reaction happened in the past and may not recur
- B.** Apply a small amount first to check whether the client still reacts
- C.** Remove the chlorhexidine product and request an alternative prep solution before the procedure continues
- D.** Use the chlorhexidine prep but keep emergency medications available at the bedside

26. A practical nurse enters a resident's room in an assisted-living facility and notes several conditions after the resident's family brought personal items from home. Which finding should the nurse address first?

- A.** A decorative throw rug is placed at the bedside
- B.** A portable halogen lamp is positioned with its shade touching the bedspread
- C.** A stack of magazines is stored on the lower shelf of the bedside table
- D.** A cell phone charger is plugged into the wall outlet behind the chair

27. A client with a new tracheostomy is anxious and repeatedly reaches toward the tracheostomy ties during dressing changes. The client can answer simple questions by nodding, becomes calmer when the nurse explains each step, and stops reaching when given a washcloth to hold. Which nursing action is most appropriate?

- A.** Apply soft wrist restraints before the next dressing change to prevent accidental decannulation
- B.** Ask another staff member to hold the client's hands down during all care

- C.** Continue verbal reassurance and provide a hand item to occupy the client's hands during care
- D.** Place the client in a chair with a locked tray to limit access to the tracheostomy

28. A nurse reviewing a client's chart before giving a scheduled dose of digoxin notices that today's documented apical pulse is 52/min, but the medication has not yet been administered and the client has no new symptoms. Which action is most appropriate?

- A.** Administer the medication as prescribed because no harm has occurred yet
- B.** Hold the medication, assess the client, follow agency procedure to notify the appropriate provider, and report the near-miss per facility policy
- C.** Ask another nurse to give the medication so the schedule is not delayed
- D.** Document the medication as refused and continue with the rest of the medication pass

29. A nurse is preparing to transfer a client from a wheelchair to the bed using a gait belt. During the transfer, the client's knees begin to buckle. Which action should the nurse take first?

- A.** Tighten the gait belt and pull the client upright
- B.** Lower the client to the floor while protecting the head
- C.** Pivot quickly and place the client on the bed
- D.** Ask the client to lock both knees and continue the transfer

30. A practical nurse enters a client's room to perform routine morning care. The nurse's hands are not visibly soiled. After adjusting the bed linens, taking the client's temperature, and helping the client open a breakfast tray, the nurse prepares to clean around the client's indwelling urinary catheter. Which action is most appropriate next?

- A.** Apply clean gloves and begin catheter care because the nurse has remained in the same room
- B.** Perform hand hygiene, then apply clean gloves before providing catheter care
- C.** Rinse gloved hands with water and proceed with catheter care
- D.** Ask an assistive personnel staff member to perform the catheter care to avoid cross-contamination

31. A nurse enters the room of a client on contact precautions wearing a gown and gloves. During care, the nurse's glove tears while turning the client. Which action should the nurse take first?

- A.** Finish turning the client, then remove the torn gloves and perform hand hygiene
- B.** Pull a second pair of gloves over the torn glove and continue care

- C.** Stop care when safe, remove the torn gloves, perform hand hygiene, and apply new gloves before continuing
- D.** Remove only the torn glove, put on a new glove, and continue care

32. A practical nurse is setting up a sterile field for a wound procedure. After opening the sterile drape correctly on a dry overbed table, the nurse notices that one area of the drape has become damp from a small amount of sterile solution that seeped underneath the basin. Which action is most appropriate?

- A.** Continue using the field because the solution was sterile
- B.** Place a dry sterile towel over the damp area and continue
- C.** Consider the field contaminated and prepare a new sterile field
- D.** Move the basin to a dry corner of the drape and avoid the wet area

33. A practical nurse is caring for a client who has impetigo with draining skin lesions. Which intervention is most appropriate to prevent transmission of the infection to others?

- A.** Place the client in a negative-pressure room and keep the door closed
- B.** Wear a gown and gloves for direct client care and when touching contaminated items
- C.** Require the client to wear a surgical mask whenever staff enter the room
- D.** Use a fit-tested N95 respirator during routine care

34. A practical nurse is preparing to insert a rectal suppository for a client who has no isolation orders. The nurse expects direct contact with mucous membranes but does not expect splashing of body fluids. Which action best follows standard precautions?

- A.** Wear clean gloves during the procedure and perform hand hygiene before and after client contact
- B.** Use sterile gloves because any contact with mucous membranes requires surgical asepsis
- C.** Wear a gown and face shield because all contact with mucous membranes requires full barrier protection
- D.** No gloves are needed if the client does not have a known infection

35. A nurse is reinforcing discharge teaching for a client who is 4 weeks postpartum after an uncomplicated vaginal birth. Which statement by the client indicates correct understanding of postpartum follow-up care?

- A.** I only need follow-up care if my vaginal bleeding has not stopped by 6 weeks.
- B.** If I feel well, I can skip postpartum follow-up because the birth was uncomplicated.

C. I should keep my postpartum appointment even if I feel fine so my recovery, mood, and family planning needs can be assessed.

D. Postpartum follow-up is mainly for the baby, so I can discuss my own concerns at a later visit.

36. A nurse is evaluating the development of a 9-year-old child during a school health visit. Which finding is most consistent with expected development for this age?

A. Enjoys collecting objects and forming friendships with peers of the same sex

B. Uses magical thinking to explain why events happen

C. Engages mainly in parallel play with other children

D. Has no concept of time and cannot follow game rules

37. A practical nurse is reinforcing teaching with the mother of a 13-year-old client who recently began menstruating. The mother says, "My daughter used to tell me everything, but now she wants more privacy, spends more time with friends, and gets annoyed when I ask too many questions." Which response by the nurse is most appropriate?

A. These behaviors usually suggest the adolescent is becoming depressed and should be evaluated right away.

B. This is a common part of adolescence as the child develops independence and a stronger peer focus.

C. You should set stricter rules so your daughter does not become too influenced by friends.

D. This change is unusual at this age and often means the adolescent is rejecting family relationships.

38. During an intake interview at a family clinic, a practical nurse collects health information from several adult clients. Which client information most strongly suggests the need for individualized screening recommendations rather than relying only on routine age-based screening?

A. A client reports drinking 1 cup of coffee each morning.

B. A client states that both a parent and a sibling were diagnosed with the same type of cancer.

C. A client says they usually sleep 7 hours each night.

D. A client reports walking the dog for 20 minutes most evenings.

39. A nurse is collecting a health history from an adult client who says, "I use some herbal products, but they're natural, so they don't really count as medicine." Which response by the nurse is best?

A. "Natural products are usually safe, so I only need to record prescription medications."

B. "Herbal products can affect your care, so please tell me the names, how much you take, and how often you use them."

C. "You should stop all herbal products immediately before we continue the interview."

D. "I will make a note that you use supplements, but the details are not necessary unless you are having side effects."

40. A practical nurse is admitting an older adult client with Parkinson disease who lives at home with a spouse. Which assessment finding is most important to document as baseline physical data to help evaluate the client's risk for aspiration during the hospital stay?

A. Whether the client wears glasses for reading

B. The client's usual swallowing ability with food, liquids, and pills

C. How many hours the client sleeps each night at home

D. The client's preferred time to take a daily walk

41. A nurse in a pediatric clinic is reviewing vaccine plans for several children. Which child should the nurse identify as having a contraindication to receiving a live vaccine today?

A. A 12-month-old who is taking amoxicillin for otitis media and has a temperature of 37.6°C (99.7°F)

B. A 5-year-old who had a mild local swelling after the last dose of diphtheria-tetanus-acellular pertussis vaccine

C. A 3-year-old who is receiving chemotherapy for leukemia

D. A 6-year-old who has a history of egg sensitivity causing mild nausea after eating scrambled eggs

42. A nurse is reinforcing teaching with the family of an older adult client who recently retired and moved into a smaller home. Which statement by a family member best reflects an expected developmental task of this life stage?

A. He should be focused on establishing independence from his parents.

B. He should be working to form a strong sense of identity separate from peers.

C. He should be adjusting to changes in roles, physical abilities, and living arrangements.

D. He should be concentrating on learning initiative through imaginative play.

43. A nurse is preparing to teach a client about wound care before discharge. The client winces, clutches the incision site, and says, "Can we do this later? It really hurts right now." Which action by the nurse is best to reduce this communication barrier?

A. Continue the teaching now so the client receives all instructions before discharge

B. Ask a family member to listen instead and explain the instructions to the client later

C. Administer prescribed pain medication if available and return when the client is more comfortable

D. Speak more loudly and repeat each step slowly while the client rests in bed

44. A client who recently started hemodialysis says to the practical nurse, "My whole week revolves around treatments now. I feel like my life isn't mine anymore." Which response by the nurse best provides emotional support?

- A.** "You should try to stay positive because many people get used to dialysis."
- B.** "It sounds like this change has made you feel a loss of control over your life."
- C.** "At least dialysis is helping keep you healthy while you adjust."
- D.** "You need to focus on following the treatment plan so you do not get worse."

45. A client with heart failure tells the nurse, "Since I left the hospital, I keep thinking I'll do something wrong and end up back here. So I stopped weighing myself and haven't opened the diet handouts." Which nursing response is most appropriate to promote effective coping?

- A.** "You need to follow the instructions exactly or your condition will probably worsen."
- B.** "Try not to think about being readmitted. Staying positive is the most important thing right now."
- C.** "It sounds like fear is making this feel unmanageable. Let's break it into one small step you can start today, like checking your weight each morning."
- D.** "Your family should take over your care until you feel less anxious about it."

46. A nurse in an emergency department is assessing a client who has been waiting for several hours. The client abruptly becomes silent, pulls a hood over the head, and keeps one hand inside a jacket pocket while staring at the triage desk. Which interpretation should the nurse make?

- A.** The client is showing decreased anxiety because verbal complaints have stopped
- B.** The client may be escalating toward violence and requires immediate intervention
- C.** The client is most likely trying to avoid embarrassment about becoming upset
- D.** The client is demonstrating a normal coping response to a stressful delay

47. A client admitted for treatment of a fractured wrist tells the nurse, "I take alprazolam every day for my nerves, but I ran out 2 days ago." Which finding should the nurse recognize as the highest priority to report to the provider?

- A.** Mild drowsiness and slowed speech
- B.** Increasing anxiety, tremors, and a new generalized seizure
- C.** Watery eyes, yawning, and muscle aches
- D.** Depressed mood and increased appetite

48. A client who was admitted after a head injury becomes increasingly restless during the night shift. The client repeatedly tries to get out of bed, pulls off the pulse oximeter, and says, "I have to go to work right now." The client is difficult to redirect but is not attempting to strike staff. Which nursing intervention is most appropriate?

- A.** Apply wrist restraints so the client cannot remove equipment
- B.** Turn on the television to distract the client
- C.** Provide a calm, low-stimulation environment and use short, simple reorientation statements
- D.** Firmly explain that the behavior is unacceptable and will delay discharge

49. A client who was told today that they will need long-term oxygen therapy says to the nurse, "I don't want anyone from church to visit me and see me wearing that tubing." Which response by the nurse is most therapeutic?

- A.** You should let your church friends visit because they will want to support you.
- B.** Many people adjust to oxygen therapy after a while, so try not to worry about it now.
- C.** It sounds like you are worried that oxygen therapy may change how others see you.
- D.** If you do not want visitors, I can tell them to come back after you are discharged.

50. A nurse on a rehabilitation unit is speaking with a client whose therapy session was canceled because the therapist became ill. The client suddenly crumples the schedule sheet, begins pacing near the window, and says loudly, "This whole day is ruined. Just leave me alone." Which action by the nurse is most appropriate?

- A.** Remain a few feet away, speak calmly, and say, "I can see you're upset. I'll give you some space, and we can talk when you're ready."
- B.** Step closer to the client and explain all of the reasons the session had to be canceled
- C.** Tell the client that pacing is unacceptable and must stop immediately
- D.** Touch the client's arm and encourage the client to sit down right away

51. A resident with moderate dementia paces in the hallway for 20 minutes before dinner and repeatedly says, "I need to catch the bus home." The resident is not threatening anyone but becomes more restless when staff tell the resident, "You live here now." Which nursing action is most appropriate?

- A.** Explain that the resident is confused and firmly direct the resident back to the room
- B.** Ask the resident why going home is so important right now

- C.** Walk with the resident and say, "You seem worried about getting home. Let's sit down and have dinner first"
- D.** Ignore the statement to avoid reinforcing the resident's confusion

52. A client tells the nurse, "When I start feeling stressed, I complain to everyone around me, but I never actually solve what is bothering me." Which nursing recommendation is most appropriate?

- A.** Set aside time to identify the stressor and make a simple plan to address it
- B.** Avoid thinking about the problem until the stress goes away on its own
- C.** Release tension by blaming the people who seem to be causing the stress
- D.** Keep busy with unrelated tasks so the stressful situation can be ignored

53. A client receiving hospice care says to the practical nurse, "I want to face death in peace, but my sons keep arguing at the bedside about what treatments I should still have. I just want someone to help them understand what matters to me." Which action by the nurse is most appropriate?

- A.** Ask the client to choose one son to make all future decisions so conflict will stop
- B.** Tell the sons that family disagreements are common and they should discuss them outside the room
- C.** Explore the client's wishes and notify the registered nurse so the care team can support a family discussion focused on the client's values
- D.** Encourage the client to avoid talking about end-of-life wishes because it may increase emotional distress

54. A client who recently had a laryngectomy writes, "People keep finishing my sentences or talking for me. It's easier not to try." Which nursing action is best to support this client's psychosocial adaptation?

- A.** Encourage family members to answer quickly for the client to reduce frustration
- B.** Provide a communication method such as a writing board and allow extra time for the client to express needs
- C.** Limit conversations to yes-or-no questions until the client feels less upset
- D.** Explain that frustration is expected and that communication will improve after discharge

55. A client with advanced ovarian cancer who is receiving end-of-life care says to the nurse, "Everyone keeps asking me what I want for the funeral, but I can't even think straight right now. I just want one person at a time and a little quiet." Which nursing action is most appropriate?

- A.** Tell the family they should postpone all discussions until the client is no longer alert

- B.** Ask the client which person they would like to speak with first and help limit visitors to support rest
- C.** Encourage the client to make all final decisions now before the opportunity is lost

D. Explain to the client that the family is grieving and needs to discuss plans together at the bedside

56. A nurse is caring for a client who has a mature gastrostomy tube and is receiving intermittent enteral feedings. The prescribed formula is not available, so a newly opened can of the same formula is obtained from refrigeration. Which action is most appropriate before administering the feeding?

- A.** Warm the formula to room temperature before administration
- B.** Dilute the formula with sterile water to reduce gastric irritation
- C.** Shake the formula vigorously and administer it while still cold
- D.** Add prescribed medications to the formula to limit tube access

57. A nurse is assisting a client who has left-sided weakness after a stroke to move from a lying position to a sitting position at the side of the bed. Which action is most appropriate to promote safe mobility?

- A.** Lift the client's shoulders straight up while the client keeps both legs on the bed
- B.** Raise the head of the bed, help the client turn onto the right side, and assist the legs over the edge of the bed
- C.** Pull the client forward by both arms until the client is upright
- D.** Ask the client to stand as soon as the feet touch the floor

58. A nurse is reinforcing teaching for a client who needs to follow a full liquid diet for the next 24 hours. Which menu item should the nurse recommend?

- A.** Strained cream soup
- B.** Scrambled eggs
- C.** Mashed potatoes
- D.** Toast with jelly

59. A nurse is caring for a client with multiple sclerosis who has hand tremors and wants to feed themselves at mealtime. Which intervention is most appropriate to promote independence with this activity of daily living?

- A.** Feed the client to prevent spilling and reduce frustration
- B.** Provide weighted utensils and allow extra time for eating
- C.** Offer finger foods only and avoid all utensils
- D.** Ask the client to wait until the tremors stop before eating

60. A nurse is caring for a client who has reflex bowel function after a thoracic spinal cord injury. To stimulate defecation during the client's scheduled bowel program, which action should the nurse take?

- A.** Offer the client a large glass of cold water at bedtime
- B.** Insert a lubricated suppository and wait the prescribed time before assisting with evacuation
- C.** Encourage the client to remain in bed in a supine position during the bowel routine
- D.** Limit fluid intake on the morning of the bowel program

61. A nurse is teaching a client who has worked rotating shifts for several months and reports, "On my days off, I stay up late with friends and sleep until noon so I can catch up." Which response by the nurse is best?

- A.** "Try to keep your sleep and wake times as consistent as possible, even on days off."
- B.** "It is best to stay in bed longer on off days so your body can recover lost sleep."
- C.** "Switching between very different sleep schedules helps your body adjust to shift work."
- D.** "Schedule several long daytime naps on off days instead of sleeping at night."

62. A practical nurse is caring for a thin older adult who has had a poor appetite for several weeks and now has a slow-healing sacral wound. Which meal selection should the nurse encourage to best support skin and tissue healing?

- A.** Toast, gelatin, and black coffee
- B.** Chicken, eggs, milk, and steamed vegetables
- C.** Applesauce, tea, and clear broth
- D.** Pancakes with syrup and fruit juice

63. A practical nurse is caring for a client who has decreased sensation in the fingertips from peripheral neuropathy. The client wants to help style their hair before discharge. Which instruction should the nurse reinforce to reduce the risk of injury?

- A.** Test the curling iron on the fingertips before using it on the hair
- B.** Use the highest heat setting so styling can be finished quickly
- C.** Choose a low-heat styling tool and rely on the mirror rather than touch to judge temperature
- D.** Wrap the hand tightly around the heated tool to improve grip while styling

64. A practical nurse is reinforcing instructions for a client who must save urine for a 24-hour collection at home. Which client statement indicates correct understanding of the teaching?

- A.** I will save the first urine when I wake up to start the collection.

B. I will keep the urine container refrigerated or on ice during the collection period.

C. If I forget to save one void, I can keep collecting and still send the specimen.

D. I should stop the collection exactly 24 hours after the last specimen is added.

65. A practical nurse is caring for a client who has been immobile for several days. Which action is most appropriate to help prevent renal calculi related to immobility?

- A.** Encourage increased fluid intake if not contraindicated
- B.** Limit the client's intake of dairy products
- C.** Keep the client in a semi-Fowler's position after meals
- D.** Provide a bedpan at the same time each day

66. A nurse is preparing to administer a scheduled oral medication to a client. The nurse verifies the prescription, checks the MAR, and identifies the client using two identifiers. The client states, "My daughter brought in a bottle of this same medicine from home, and I already took one about an hour ago." Which action is most appropriate?

- A.** Administer the scheduled dose because the MAR shows it is due
- B.** Hold the medication and clarify the situation according to facility policy before giving any more of the drug
- C.** Ask the client's daughter how much medication was taken, then give half of the scheduled dose
- D.** Document the home dose on the MAR and administer the scheduled dose later in the shift

67. A practical nurse is admitting a client for dehydration and completing medication reconciliation. The client says, "I take my heart pill and water pill every day. I also use an inhaler when I get short of breath, a medicated skin cream for a rash, and vitamins from the grocery store." The current admission medication list includes only the two prescription pills. Which action should the nurse take?

- A.** Document only the prescribed oral medications because nonprescription products are not part of medication reconciliation
- B.** Add all medications the client reports, including the inhaler, topical cream, and vitamins, and communicate the complete list for provider review
- C.** Wait until discharge planning to update the list because home medications can be reviewed later
- D.** Ask the client to bring the items from home before documenting any of them

68. A nurse is preparing to administer a medication to a patient with severe dehydration and very little subcutaneous tissue. The provider wants predictable systemic absorption and the medication is available for oral, subcutaneous, intramuscular, and intravenous use. Which route should the nurse expect to be most reliable?

- A. Oral
- B. Subcutaneous
- C. Intramuscular
- D. Intravenous

69. A nurse has just begun infusing packed red blood cells for a client with postoperative blood loss. Which finding most strongly suggests an acute hemolytic transfusion reaction rather than a mild allergic reaction?

- A. Flank pain with fever and hypotension
- B. Itching with a few raised hives
- C. Facial flushing without other symptoms
- D. Mild anxiety during the first few minutes

70. A nurse needs to administer 84 mL of IV antibiotic over 40 minutes using gravity tubing calibrated at 20 gtt/mL. What flow rate should the nurse regulate in drops per minute?

- A. 28 gtt/min
- B. 36 gtt/min
- C. 42 gtt/min
- D. 56 gtt/min

71. A practical nurse administers a prescribed controlled opioid to a client at 1000. At 1030, the nurse finds the client nauseated and sees an unopened tablet of the opioid on the overbed table. The client says, "I saved it in case the pain gets worse later." Which action should the nurse take first?

- A. Instruct the client to take the tablet now so the dose matches the MAR
- B. Secure the tablet and follow facility policy for handling an unadministered controlled substance
- C. Document that the client refused the medication and leave the tablet with the client
- D. Ask another nurse to witness the client discarding the tablet in the trash

72. A nurse is reinforcing teaching for a postoperative client who will receive pain medication through a patient-controlled analgesia (PCA) pump. The client's adult child asks, "If my mother falls asleep and starts hurting, can I press the button for her?" Which response by the nurse is best?

- A. Yes, if you press it only when she has severe pain
- B. Yes, because the pump is programmed to prevent overdose

C. No, only the client should press the PCA button

D. No, because family members are never allowed to touch the pump equipment

73. A nurse is preparing to administer the first dose of lisinopril to a client with hypertension. Which pre-administration data is most important for the nurse to review?

- A. The client's most recent serum potassium level
- B. The client's bowel movement pattern for the past 24 hours
- C. The client's hemoglobin A1c from 3 months ago
- D. The client's daily fluid intake goal

74. A client with newly diagnosed epilepsy has been taking phenytoin for 1 month. Which finding best indicates the medication is achieving the intended therapeutic effect?

- A. The client reports no seizure episodes since starting the medication
- B. The client has mild gingival swelling when brushing teeth
- C. The serum phenytoin level is 24 mcg/mL
- D. The client reports feeling drowsy for 2 hours after each dose

75. A client with a seizure disorder is prescribed phenytoin 100 mg by mouth three times daily. Before administering the next dose, the nurse reviews the laboratory results and notes a serum phenytoin level of 24 mcg/mL. Which action is most appropriate?

- A. Administer the dose as prescribed because the level is expected with long-term therapy
- B. Hold the medication and notify the provider of the elevated drug level
- C. Give the medication with food and recheck the level after the dose
- D. Reduce the dose by half and document the change in the medication record

76. A nurse is teaching a client who has a new prescription for a scopolamine transdermal patch to prevent motion sickness during an upcoming cruise. Which instruction should the nurse include?

- A. Apply the patch only after nausea begins
- B. Place the patch behind the ear at least several hours before travel
- C. Cut the patch in half if dry mouth occurs
- D. Remove the patch each night and reapply the same patch in the morning

- 77.** A nurse is preparing to administer several morning medications in a long-term care facility. Which medication should the nurse question if it has been crushed and mixed together with the client's applesauce in the same medication cup 30 minutes before administration?
- A.** Enteric-coated aspirin
 - B.** Docusate sodium capsule contents
 - C.** Immediate-release acetaminophen tablet
 - D.** Vitamin C chewable tablet
- 78.** A nurse is preparing to administer azithromycin 0.25 g by mouth. The suspension available is labeled 100 mg/5 mL. How many milliliters should the nurse administer for one dose?
- A.** 8.5 mL
 - B.** 10 mL
 - C.** 12.5 mL
 - D.** 15 mL
- 79.** A practical nurse is collecting data in a rehabilitation unit. Which client should the nurse identify as being at greatest risk for aspiration?
- A.** A client with a below-the-knee amputation who reports incisional pain rated 6/10
 - B.** A client with Parkinson's disease who coughs while drinking water and has a soft, gurgling voice after swallowing
 - C.** A client with osteoarthritis who needs help transferring from bed to chair
 - D.** A client with heart failure who has bilateral ankle edema at the end of the day
- 80.** A nurse is preparing to collect a sputum specimen for culture from a client who is coughing thick secretions. Which instruction should the nurse give the client to obtain the best specimen?
- A.** Collect the specimen after drinking a full glass of water
 - B.** Spit saliva into the container as soon as you wake up
 - C.** Cough deeply and expectorate sputum from the lungs into the sterile container
 - D.** Use mouthwash immediately before coughing into the container
- 81.** A nurse is reviewing laboratory results for a client who has chest pain and is being evaluated for a possible myocardial infarction. Which finding is most suggestive of acute cardiac muscle injury?
- A.** Troponin I level above the reference range
 - B.** Hemoglobin A1c of 8.1%
 - C.** Blood urea nitrogen of 24 mg/dL
 - D.** White blood cell count of 11,000/mm³
- 82.** A client who is 24 weeks pregnant comes to the prenatal clinic reporting sudden blurred vision and a severe headache that started 1 hour ago. Which question by the nurse is most important to ask next?
- A.** "Have you noticed pain in the upper right part of your abdomen?"
 - B.** "Have you had heartburn after meals recently?"
 - C.** "Did you drink any caffeinated beverages today?"
 - D.** "Have you been sleeping well this week?"
- 83.** A nurse is performing a bedside capillary blood glucose test for a client who is receiving an IV infusion containing dextrose through the hand being used for sampling. Which action is most appropriate to help prevent an inaccurate point-of-care result?
- A.** Obtain the capillary sample from a different finger on the same hand
 - B.** Turn off the IV for 5 minutes, then collect the sample from the same hand
 - C.** Use a finger on the hand without the dextrose infusion for the capillary sample
 - D.** Wipe away the first drop of blood and use the second drop from the same hand
- 84.** A practical nurse is obtaining a tympanic temperature on an adult client. The reading is 95.9°F (35.5°C), but the client is warm, alert, and has no symptoms of hypothermia. Which action should the nurse take next?
- A.** Document the temperature as normal for a tympanic reading
 - B.** Recheck the temperature, making sure the probe is positioned correctly in the ear canal
 - C.** Apply warm blankets and notify the provider immediately
 - D.** Switch to an oral temperature after giving the client ice chips
- 85.** A client is scheduled for a colonoscopy at 1000. During the preprocedure assessment, the client tells the nurse, "I drank the bowel prep last night, but my stools are still dark brown and formed." Which action should the nurse take first?
- A.** Proceed with the procedure as scheduled
 - B.** Administer a cleansing enema immediately
 - C.** Notify the provider that the bowel preparation may be inadequate
 - D.** Instruct the client to drink clear liquids until the procedure

86. A nurse is reviewing a client's ECG rhythm strip and notes an underlying regular rhythm of 80/min. An early beat appears with a widened, bizarre QRS complex and no visible P wave before it. This is followed by a brief compensatory pause. Which interpretation is most appropriate?

- A. Premature atrial contraction (PAC)
- B. Premature ventricular contraction (PVC)
- C. Sinus tachycardia
- D. First-degree AV block

87. A nurse is preparing to obtain a venous blood specimen from an adult client. The client states, "I had a right-sided mastectomy last year," and the nurse notes a functioning arteriovenous fistula in the left forearm for hemodialysis. Which action is most appropriate?

- A. Obtain the specimen from the right arm because the left arm contains the fistula
- B. Obtain the specimen from the left hand distal to the fistula
- C. Hold the venipuncture and clarify an alternate site or method with the provider or facility protocol
- D. Apply a blood pressure cuff to the left upper arm and collect above the fistula

88. A practical nurse is caring for a client who is 24 hours postoperative after abdominal surgery. The client reports calf tenderness when walking to the bathroom. Which action should the nurse take first?

- A. Massage the calf to reduce discomfort
- B. Assist the client back to bed and notify the RN
- C. Encourage the client to continue ambulating to improve circulation
- D. Apply a heating pad to the painful area

89. A client with a newly inserted chest tube has continuous bubbling in the water-seal chamber of the drainage system several hours after insertion. The client is resting comfortably, and the dressing is dry and intact. Which action should the nurse take first?

- A. Increase the wall suction setting
- B. Clamp the chest tube near the client's chest to locate the leak
- C. Assess the drainage system and tubing connections for an air leak
- D. Document the finding as an expected effect of chest tube drainage

90. A nurse is caring for a client with a plaster cast applied earlier today for a tibial fracture. The client reports increasing pain that is not relieved by the prescribed analgesic and says, "My toes feel tingly." Which focused assessment should the nurse perform first?

- A. Assess color, temperature, movement, sensation, and pulses in the affected extremity
- B. Ask the client to rate the pain again on a 0 to 10 scale
- C. Check the client's temperature for possible infection
- D. Inspect the cast for areas of moisture or cracking

91. A client 12 hours after abdominal surgery suddenly becomes restless and says, "I feel like something is very wrong." The nurse notes cool, clammy skin; heart rate 128/min; respirations 30/min; blood pressure 78/46 mm Hg; and urine output 15 mL in the past hour. Which condition should the nurse suspect?

- A. Neurogenic shock
- B. Hypovolemic shock
- C. Increased intracranial pressure
- D. Acute renal failure

92. A client with chronic heart failure is being seen in the clinic 1 week after starting daily home weights. Which statement by the client should the practical nurse report to the nurse immediately?

- A. I weigh myself after breakfast so I can remember to do it every day.
- B. My weight was 2 pounds higher this morning than it was yesterday.
- C. I use a different scale when I stay at my daughter's house on weekends.
- D. I write my weight down only when it changes a lot.

93. A client with a history of heart failure suddenly reports severe shortness of breath while lying in bed. The nurse notes pink, frothy sputum, diffuse crackles in both lungs, cool clammy skin, and oxygen saturation of 82% on room air. Which action should the nurse take first?

- A. Place the client in high-Fowler position and apply oxygen
- B. Obtain a prescription for IV diuretics
- C. Encourage the client to cough and deep-breathe
- D. Auscultate bowel sounds and assess abdominal distention

94. A nurse is caring for a client who has a Jackson-Pratt drain after abdominal surgery. Which action by the nurse is appropriate to maintain the drain's function?

- A. Compress the bulb before closing the stopper after emptying it
- B. Irrigate the drain with sterile saline every 4 hours

C. Keep the bulb positioned above the incision to promote drainage

D. Strip the tubing vigorously whenever drainage slows

95. A nurse is caring for a client who has been walking outside on a very hot, humid day. The client reports weakness and nausea. Assessment findings include temperature 40.2°C (104.4°F), pulse 128/min, dry flushed skin, and confusion. Which action should the nurse take first?

A. Offer the client cool oral fluids and allow the client to rest

B. Move the client to a cooler environment and begin rapid cooling measures

C. Cover the client with a light sheet to reduce stimulation

D. Encourage the client to walk slowly to improve circulation

96. A nurse is changing the dressing on a postoperative abdominal wound when the client suddenly says, "It feels like something just gave way." The nurse notes that the incision edges have separated and a loop of bowel is visible. Which action should the nurse take first?

A. Apply sterile saline-moistened dressings over the wound

B. Reinforce the dressing with dry sterile gauze

C. Place the client in high-Fowler's position

D. Attempt to gently reinsert the protruding bowel

97. A nurse is reinforcing discharge teaching for a client with a new colostomy. Which statement by the client indicates correct understanding of when to contact the provider?

A. I should call if the stoma looks pale, dark red, or bluish.

B. I should call if the stoma bleeds a little when I clean it.

C. I should call if the stoma is slightly swollen for the first several weeks.

D. I should call if the stoma is moist and shiny.

98. A nurse is caring for a client with a bowel obstruction who has a nasogastric tube connected to low intermittent suction. The client has been alert and stable throughout the morning. Which new finding should the nurse recognize as the priority change in condition?

A. The client reports a sore throat and dry lips

B. The drainage in the suction canister changes from green to dark red

C. The client asks for pain medication 1 hour earlier than scheduled

D. The client has absent bowel sounds in all four quadrants

99. A nurse is caring for a client who has had watery diarrhea for 2 days. The client is weak and reports palpitations. Which ECG finding should the nurse recognize as most consistent with hypokalemia?

A. Peaked T waves

B. Prominent U waves

C. ST-segment elevation

D. Widened QRS complexes with absent P waves

100. A client with end-stage kidney disease is scheduled to receive the usual morning medications before leaving for hemodialysis. Which prescription should the practical nurse question first?

A. Folic acid tablet

B. Docusate sodium

C. Lisinopril

D. Renal multivitamin

Simulation Test 24 — Answer Key

1. B	26. B	51. C	76. B
2. C	27. C	52. A	77. A
3. C	28. B	53. C	78. C
4. D	29. B	54. B	79. B
5. C	30. B	55. B	80. C
6. C	31. C	56. A	81. A
7. B	32. C	57. B	82. A
8. C	33. B	58. A	83. C
9. B	34. A	59. B	84. B
10. C	35. C	60. B	85. C
11. C	36. A	61. A	86. B
12. C	37. B	62. B	87. C
13. C	38. B	63. C	88. B
14. A	39. B	64. B	89. C
15. C	40. B	65. A	90. A
16. B	41. C	66. B	91. B
17. B	42. C	67. B	92. B
18. C	43. C	68. D	93. A
19. C	44. B	69. A	94. A
20. B	45. C	70. C	95. B
21. C	46. B	71. B	96. A
22. B	47. B	72. C	97. A
23. C	48. C	73. A	98. B
24. B	49. C	74. A	99. B
25. C	50. A	75. B	100. C

Simulation Test 25

100 questions

1. A practical nurse in a psychiatric inpatient unit is notified of four client situations during evening rounds. Which client should the nurse attend to first?
 - A. A client with major depression who refuses the dinner tray and says, "I'm not hungry."
 - B. A client with generalized anxiety who asks for a scheduled bedtime medication 20 minutes early.
 - C. A client with bipolar disorder who is pacing in the hallway, shouting, and clenching fists near other clients.
 - D. A client with schizophrenia who reports that the television is too loud in the day room.
2. A practical nurse is supervising an assistive personnel (AP) on a telemetry unit. Which situation requires the nurse to intervene before delegation?
 - A. The AP is asked to obtain a finger-stick blood glucose level for a client with diabetes who is alert and scheduled for a routine premeal check
 - B. The AP is asked to record oral intake for a stable client who is being monitored for hydration status
 - C. The AP is asked to assist a client to the bathroom after the nurse has confirmed the client transfers with one-person assistance
 - D. The AP is asked to take routine vital signs on a client who returned from surgery 15 minutes ago
3. A practical nurse is assigning tasks at the start of a clinic day. Which task is most appropriate to delegate to an assistive personnel (AP)?
 - A. Obtain a clean-catch urine specimen from a stable client who has already received instructions
 - B. Teach a client how to use a newly prescribed peak flow meter
 - C. Decide whether a client with ankle swelling needs same-day provider follow-up
 - D. Call a client to explain abnormal laboratory results and answer questions
4. A nurse is preparing an adult client for a provider visit. The client is alert, oriented, and says, "I want to ask about all my treatment options, but I get flustered and forget my questions once the provider starts talking." Which nursing action best supports the client's right to participate in care decisions?
 - A. Ask the provider to shorten the visit so the client does not become overwhelmed
 - B. Tell the client to wait and see whether the provider brings up the same concerns
 - C. Help the client write down questions and keep the list available during the visit
 - D. Request that a family member answer questions for the client during the discussion
5. A nurse is caring for an adult client who is alert, oriented, and has decision-making capacity. Before a scheduled cardiac catheterization, the provider obtains consent and leaves. The client then tells the nurse, "I feel pressured. I still do not understand the alternative options if I say no." The transport team has arrived to take the client to the procedure area. Which action by the nurse is best?
 - A. Send the client to the procedure because the signed consent is already in the chart
 - B. Explain the alternatives in detail so the procedure is not delayed
 - C. Notify the provider that the client has unresolved questions and pause transport until the client receives clarification
 - D. Ask the transport team to wait while the nurse asks the client to sign a new consent form
6. A practical nurse is speaking with a client who says, "I don't want anyone except my husband to get updates." Later that day, the client's adult daughter calls the unit, correctly provides the client's date of birth, and asks for the latest test results. What should the nurse do?
 - A. Give the daughter a brief update because she verified identifying information
 - B. Ask the daughter to call the husband for information
 - C. Tell the daughter that no information can be shared without the client's permission
 - D. Share only the test results but no treatment plan
7. A practical nurse is caring for a client who is well known in the community. During lunch in the hospital cafeteria, the nurse hears two off-duty staff members discussing the client's diagnosis and surgery outcome in a conversation that can be overheard by visitors. What is the nurse's best action?
 - A. Join the conversation to make sure the details being discussed are accurate
 - B. Immediately tell the staff members to stop discussing client information in a public area
 - C. Wait until returning to the unit, then document the client's diagnosis more carefully
 - D. Ignore the conversation because the staff members are off duty
8. A nurse is admitting an alert adult client who says, "I never completed any advance directive forms because I thought only older adults or people who are dying need them." Which response by the nurse is most appropriate?
 - A. Advance directives are mainly for clients with terminal illnesses, so you can wait until your condition becomes more serious.

- B.** Advance directives are only used after a provider determines that curative treatment is no longer helpful.
- C.** Any competent adult can complete an advance directive so future care decisions reflect that person's wishes if decision-making capacity is lost.
- D.** Advance directives are required before a client can refuse any recommended treatment.

9. A client who is alert and has decision-making capacity tells the nurse, "My brother keeps insisting that I should sign an advance directive tonight, but I do not want to make that decision right now." Which action by the nurse is best?

- A.** Ask the brother to stay and help the client complete the form so the admission process is not delayed.
- B.** Tell the client that signing an advance directive is recommended for all admitted adults and should be done before discharge.
- C.** Document the client's preference not to complete an advance directive at this time and support the client's right to decide later.
- D.** Notify the provider that the client is refusing a required legal document.

10. A client is being prepared for elective surgery. The client asks the nurse, "What exactly are you confirming when you witness my signature on this consent form?" Which response by the nurse is best?

- A.** That the provider fully explained the procedure, risks, benefits, and alternatives
- B.** That the client is signing the form voluntarily and appears to be the person giving consent
- C.** That the surgery is necessary and is the best treatment option
- D.** That the client understands the procedure better than the family does

11. A client is scheduled for an elective bone marrow biopsy. The consent form was signed earlier in the day after the provider discussed the procedure. When the nurse arrives to take the client for the biopsy, the client says, "I'm not saying no, but I don't really understand why this test is needed now instead of waiting a few weeks." Which action should the nurse take first?

- A.** Transport the client because the form has already been signed
- B.** Explain that delaying the test could affect treatment and ask the client to proceed
- C.** Hold the procedure and notify the provider that the client has further questions
- D.** Ask a family member to encourage the client to continue with the biopsy

12. A client is being discharged after treatment for major depression. The client denies suicidal thoughts and will start a new antidepressant at home. During discharge teaching, the client says, "I don't think I'll be able to afford counseling visits until my next paycheck in 3 weeks." Which action by the nurse is most appropriate before discharge?

- A.** Advise the client to stop the antidepressant until counseling can be scheduled
- B.** Ask the provider to keep the client hospitalized until counseling is affordable
- C.** Arrange a referral to social services or case management to explore low-cost mental health resources
- D.** Tell the client to call the emergency department only if the medication causes nausea

13. A client is preparing for discharge after abdominal surgery. The client has a new prescription for a temporary low-residue diet, oral pain medication, and a follow-up visit in 1 week. The client speaks limited English, and the adult grandchild has been translating during the hospitalization. When the nurse begins discharge teaching, the grandchild says, "I'll explain everything later at home." Which action should the nurse take first?

- A.** Proceed with discharge because a family member is available to translate the instructions
- B.** Request a qualified medical interpreter to provide and verify discharge teaching
- C.** Give the written instructions to the grandchild and ask the grandchild to sign that teaching was completed
- D.** Skip the teaching now and review the instructions at the follow-up appointment instead

14. A long-term care unit wants to reduce skin tears during evening care. Before changing supplies or routines, the team reviews incident reports from the past 3 months and notices that many skin tears occur when adhesive dressings are removed. Which action is the best next step in the quality improvement process?

- A.** Implement a facility-wide policy requiring daily wound specialist rounds
- B.** Test a gentler adhesive-removal technique with a small group of residents and track skin tear rates
- C.** Counsel the staff members who reported the highest number of skin tears
- D.** Stop using all adhesive dressings on the unit immediately

15. A dialysis clinic is trying to reduce treatment delays caused by incomplete pre-treatment weights. The team starts a new workflow in which a designated staff member checks the scale area every 15 minutes and reminds arriving clients to be weighed before chair assignment. After 3 weeks, the percentage of clients with documented pre-treatment weights rises from 70% to 97%, but the average treatment start delay changes very little. Which interpretation best reflects sound quality improvement reasoning?

- A.** The project was successful because the process measure improved, so no further review is needed
- B.** The delays are probably unrelated to pre-treatment weights, so the team should stop collecting data
- C.** The workflow change improved the process, but the team should investigate other causes of treatment delays affecting the outcome
- D.** The staff member should remind clients more often because increasing the process measure always improves the outcome measure

16. A practical nurse is transferring a client from the emergency department to a medical-surgical unit. Which information is most important to communicate directly to the receiving nurse before transport?

- A.** The client prefers ice water instead of juice with medications
- B.** The client has a hearing aid in the left ear
- C.** The client received IV morphine 10 minutes ago and is now drowsy
- D.** The client's family plans to visit later this evening

17. A practical nurse is preparing to call the provider about a client who had abdominal surgery yesterday. Which report best supports effective interdisciplinary information sharing?

- A.** The client is not doing well after surgery and probably needs a different plan.
- B.** The client reports incisional pain of 6/10 and says the dressing feels tight.
- C.** I am calling about a client who is restless and has a fever. Please come evaluate.
- D.** I am calling about Ms. Lee, postoperative day 1 after abdominal surgery. Over the last 2 hours, her temperature increased from 37.4°C to 38.3°C, heart rate is 118/min, the incision dressing has new drainage measuring 4 cm by 6 cm, and urine output was 20 mL in the last hour.

18. A practical nurse is caring for an adult client who is alert and oriented. While preparing the client for a minor bedside procedure, the nurse notices that the consent form in the chart is blank. The provider says, "It's a routine procedure. Have the client sign this now so we can get started." Which action by the practical nurse is most appropriate?

- A.** Ask the client to sign the form because the procedure is routine and already prescribed
- B.** Explain the risks and benefits of the procedure to the client, then obtain the signature
- C.** Tell the provider that the client needs the procedure explanation from the provider before signing
- D.** Witness the client's signature first and ask the provider to answer questions afterward

19. A practical nurse in a home health agency visits an 81-year-old client who lives with an adult grandson. The client is alert and oriented. During the visit, the nurse notes that the client has lost weight, has no food in the kitchen, and says, "My grandson uses my pension check for bills, so I just eat when he brings something home." The client adds, "Please don't report this. He gets angry when I complain." Which action by the nurse is most appropriate?

- A.** Respect the client's wishes because the client is alert and oriented and document the concern only
- B.** Confront the grandson directly and demand that he return the client's money
- C.** Report the suspected abuse or exploitation according to agency policy and state law
- D.** Wait to report until another team member can confirm the client's statement

20. A practical nurse is supervising an assistive personnel (AP) on a postoperative unit. Which situation best shows that the nurse needs to give clearer direction to the AP before delegation occurs?

- A.** The nurse asks the AP to record routine intake and output for a stable client and report urine output less than 30 mL/hr.
- B.** The nurse tells the AP to ambulate a client after lunch and to stop and report if the client becomes lightheaded or short of breath.
- C.** The nurse asks the AP to help a client with a bed bath and reminds the AP not to massage the client's reddened heels.
- D.** The nurse tells the AP to keep an eye on a client who returned from surgery this morning and let the nurse know if anything seems wrong.

21. A practical nurse in an outpatient clinic is reinforcing discharge teaching for a client with newly prescribed medications after hospitalization for asthma. The client says, "I understand what to do, but I am Deaf and the pharmacy usually calls out names and gives spoken instructions. Last time I missed important information." Which action by the nurse best promotes equitable access to care?

- A.** Ask the client's family member to accompany the client to the pharmacy and explain the instructions
- B.** Tell the client to request written medication information after arriving at the pharmacy
- C.** Coordinate with the pharmacy before discharge to communicate the client's preferred method for accessible counseling and pickup instructions
- D.** Suggest that the client transfer prescriptions only if a problem occurs again

22. A nurse in the emergency department is preparing to administer an IV pain medication to a client who was just moved from triage to a treatment room. The client is alert, and the nurse has the medication and the electronic medication record open. Which action best verifies the correct client before administration?

- A.** Ask the client, "Are you Ms. Lopez?" and proceed if the client says yes
- B.** Check the room assignment on the tracking board and compare it with the medication record
- C.** Ask the client to state their full name and date of birth, and compare both with the identification band and medication record
- D.** Confirm the client's identity with the triage nurse who brought the client to the room

23. A nurse is preparing to use a floor-based sit-to-stand lift to move a client from the bed to a chair. The client can bear some weight and follow directions. During setup, the nurse sees that the client's nonskid socks are on, but the lift's wheel brakes are locked. Which action should the nurse take?

- A.** Leave the brakes locked to keep the lift from moving during the transfer
- B.** Unlock the lift's wheel brakes before raising the client
- C.** Proceed with the transfer only if a second nurse holds the lift frame steady
- D.** Remove the client's nonskid socks so the feet can pivot more easily on the lift platform

24. A nurse is preparing to send a client for a contrast-enhanced CT scan. The client says, "The last time I got contrast dye, I broke out in hives and felt my throat getting tight." The provider has already entered the procedure order, and the allergy is not documented in the chart. Which action should the nurse take first?

- A.** Document the client's statement and notify the provider before the scan proceeds
- B.** Send the client to radiology and advise the staff there about the reaction
- C.** Administer the prescribed preprocedure sedative and reassess the client afterward
- D.** Ask the client whether the reaction resolved without treatment before taking further action

25. A practical nurse enters a resident's room in a long-term care facility and notes these conditions: a portable oxygen concentrator is running beside the bed, petroleum-based lip ointment is on the overbed table, the bed is in the low position, and the resident's eyeglasses are within reach. Which finding should the nurse address first?

- A.** Portable oxygen concentrator running beside the bed
- B.** Petroleum-based lip ointment on the overbed table
- C.** Bed in the low position
- D.** Eyeglasses within reach

26. A practical nurse is assisting with triage in the lobby of an urgent care center when a small truck strikes a utility pole outside, causing a downed power line to fall across the parking area. Several clients are moving toward the front doors to leave. Which action should the nurse take first to promote environmental safety?

- A.** Instruct clients and staff to stay away from the parking area and block access to the exit facing the downed line
- B.** Go outside to check whether anyone in the parking lot is injured
- C.** Move vehicles away from the area beneath the line
- D.** Open the front doors to warn people in the parking lot to keep back

27. A client 6 hours after abdominal surgery becomes confused and repeatedly grabs at a newly inserted urinary catheter, saying, "This hurts." The client is lying still between episodes and follows simple directions when the nurse speaks calmly. Which action should the nurse take first to follow least-restrictive restraint principles?

- A.** Apply soft mitt restraints to prevent the client from pulling on the catheter
- B.** Assess the client for pain and catheter discomfort, then provide relief and reorient the client
- C.** Raise all side rails and move the client farther from the nurses' station

D. Request an order for wrist restraints because the catheter must remain in place

28. A nurse enters a client's room to administer a scheduled subcutaneous anticoagulant injection and notices that the syringe in the medication tray is labeled with the correct medication and dose but a different client's name. The injection has not been given, and the nurse removes the syringe from the room. Which action is most appropriate next?

- A.** Administer the medication after verifying the dose with another nurse because no client exposure occurred
- B.** Discard the syringe, obtain the correct medication for the client, and report the event according to facility policy as a near miss
- C.** Relabel the syringe with the current client's information and continue with administration
- D.** Document the event only in the current client's medical record because the medication error was prevented

29. A nurse and an assistive personnel (AP) are preparing to use a friction-reducing transfer sheet to move a client up in bed. Which action by the nurse best reduces the risk of injury to staff during the move?

- A.** Keep the client's bed flat so the client does not slide during the transfer
- B.** Have the AP pull from the far side of the bed while the nurse pushes from the near side
- C.** Raise the bed to working height and move the client with a coordinated weight shift instead of twisting
- D.** Place a pillow under the client's shoulders before the move to decrease friction

30. A practical nurse is performing tracheostomy care for a client with thick secretions. After removing the soiled dressing and cleaning around the stoma with clean gloves, the nurse removes the gloves, performs hand hygiene, and opens a sterile package containing a new tracheostomy dressing. While placing the dressing, the nurse accidentally touches the sterile dressing with the bare fingertips of the nondominant hand. Which action should the nurse take?

- A.** Place the dressing anyway because the nurse performed hand hygiene just before opening the package
- B.** Ask another nurse to place the dressing while the original nurse secures the tracheostomy ties
- C.** Discard the contaminated dressing and obtain a new sterile dressing
- D.** Apply clean gloves and continue using the same dressing

31. A nurse is preparing to assist a client with frequent vomiting. The client is not on isolation precautions, but the nurse expects possible splashing of emesis onto the face and uniform while helping the client reposition and use an emesis basin. Which personal protective equipment should the nurse wear?

- A.** Gloves only
- B.** Gloves and gown only
- C.** Gloves, gown, mask, and eye protection
- D.** Mask and eye protection only

32. A practical nurse is assisting with a sterile wound procedure. After removing the cap from a bottle of sterile normal saline, the nurse places the cap on the bedside table with the inside surface facing up so it can be replaced later. Which action best maintains asepsis?

- A.** Use the bottle as planned because the inside of the cap is facing upward
- B.** Wipe the cap with sterile gauze before replacing it on the bottle
- C.** Discard the bottle and obtain a new sterile solution container
- D.** Ask another nurse to recap the bottle using sterile gloves

33. A practical nurse is caring for a client who has a large, uncontained draining abscess on the thigh. The provider has prescribed contact precautions. Which instruction is most appropriate to give the client's family members who want to visit?

- A.** They should wear a gown and gloves when they may have contact with the client or contaminated items in the room.
- B.** They should wear an N95 respirator whenever they enter the room.
- C.** They should avoid visiting until the drainage has completely stopped.
- D.** They should wear a sterile gown and sterile gloves before touching any item in the room.

34. A practical nurse is assigned to transport a client with no isolation orders to radiology. Before leaving the room, the client begins to have an episode of vomiting, and the nurse expects possible splashing while helping the client sit up and use an emesis basin. Which action best reflects standard precautions?

- A.** Put on gloves and a mask with eye protection before assisting the client
- B.** Apply sterile gloves before touching the emesis basin
- C.** Transport the client first and clean the vomit after returning
- D.** Use no personal protective equipment because the client has no isolation orders

35. A nurse in a pediatric clinic is observing several children in the waiting room. Which behavior is most appropriate for a 3-year-old child?

- A.** Copies a triangle on paper
- B.** Rides a two-wheel bicycle without training wheels
- C.** Builds a tower of more than six blocks
- D.** Prints first and last name clearly

36. A practical nurse is reinforcing teaching with the caregiver of an 82-year-old client who moved in with family 1 month ago after a hospitalization. The caregiver says, "He gets frustrated when I help him dress, and yesterday he said, 'I can still do things for myself.'" Which response by the nurse is most appropriate?

- A.** "It would be best to complete all of his care quickly so he does not become overtired."
- B.** "Loss of independence can be stressful, so encourage him to do the parts of self-care he can still manage safely."
- C.** "This behavior usually means he is developing confusion related to aging."
- D.** "You should correct him each time so he understands his current limitations."

37. At a senior center, a practical nurse is helping with a fall-prevention program for older adults. Which action is the best example of secondary prevention?

- A.** Teaching clients to remove loose rugs and improve home lighting
- B.** Checking clients for balance problems and asking about recent falls
- C.** Assisting a client with gait training after a hip fracture
- D.** Encouraging daily calcium intake to support bone health

38. A nurse is obtaining a health history from an adult client who says, "I take a few pills, but I don't remember all their names." Which response by the nurse is best?

- A.** "That is fine. We can skip the medication history for now."
- B.** "Just tell me the ones you take most often."
- C.** "Please describe what you take, including why you take it and any vitamins, over-the-counter drugs, or supplements."
- D.** "I only need to know about medications that were prescribed this year."

39. A practical nurse is admitting an adult client with severe scoliosis. Which documentation best establishes a baseline respiratory assessment for future comparison during the stay?

- A.** Client denies shortness of breath at this time
- B.** Respirations 20/min, diminished breath sounds at bilateral bases, uses 2 pillows to sleep at home, oxygen saturation 93% on room air
- C.** Chest x-ray completed in the emergency department

D. Breathing appears comfortable after admission

40. A nurse in a community clinic is reinforcing teaching for the parent of a 6-month-old infant who has a mild cold with clear nasal drainage and no fever. The parent asks whether the scheduled routine immunizations should be postponed until the infant is completely well. Which response by the nurse is best?

- A.** The immunizations should be delayed because any current illness increases the risk for vaccine reactions.
- B.** The immunizations can usually be given as scheduled because a mild illness without fever is not generally a reason to postpone them.
- C.** Only live vaccines should be delayed, but inactivated vaccines should still be given.
- D.** The immunizations should be delayed until the infant has been symptom-free for at least 1 week.

41. A nurse is reinforcing teaching with the parent of a 6-month-old infant about expected developmental progress. Which statement by the parent indicates understanding?

- A.** My baby should be able to sit steadily without support for long periods.
- B.** My baby may begin to transfer a toy from one hand to the other.
- C.** My baby should be able to speak several clear words.
- D.** My baby will likely be ready for daytime toilet training.

42. A nurse is obtaining a health history from a client who keeps glancing toward the door and answering with short, distracted responses. After a few minutes, the client says, "My son was supposed to pick up my children an hour ago, and I can't focus until I know they're safe." Which action by the nurse is best to reduce this communication barrier?

- A.** Continue the interview quickly so the nurse can finish before the client becomes more upset
- B.** Ask the client to set aside personal concerns and focus on the questions being asked
- C.** Help the client contact the son or arrange for someone to check on the children before continuing
- D.** Rephrase each question using simpler words and a louder speaking voice

43. A nurse is teaching a client who was newly prescribed warfarin about home management. During the session, the client says, "I don't care what the paper says. My brother took this medicine and he was fine doing it his way." Which response by the nurse is best?

- A.** "You need to follow the written instructions exactly, even if your brother did something different."
- B.** "Tell me what your brother did, and we can compare that with what is safest for you."

- C. "Your provider would not have prescribed this unless it was the only correct plan."
- D. "Let's skip the teaching for now since you do not seem interested."

44. A client who had a myocardial infarction 2 days ago tells the practical nurse, "Everyone keeps asking how I'm doing, but I don't even know what to say. I feel shaky inside." Which response by the nurse best provides emotional support?

- A. You need to stay positive so your recovery will go better.
- B. Many people feel this way after a heart attack, so try not to worry too much.
- C. It sounds like this has been overwhelming for you. Do you want to talk more about what feels most frightening right now?
- D. Let's focus on your discharge instructions so you will know what to do at home.

45. A client with a new ileostomy tells the nurse, "I get nervous when it's time to empty the pouch, so I ask my partner to do it every time. I know I need to learn, but I keep putting it off." Which nursing response best promotes effective coping?

- A. "It sounds like this feels stressful right now. Let's break the task into small steps, and you can try the first part while I stay with you."
- B. "Your partner should continue doing the pouch care until you feel completely confident."
- C. "You need to start managing the pouch on your own so you do not become dependent on others."
- D. "Try not to think about the ileostomy so much, and the anxiety will probably fade on its own."

46. A nurse in an urgent care clinic is speaking with a client who is upset about a denied prescription refill. The client has been arguing at the desk, is now pacing, and says in a loud voice, "This is ridiculous." Which nurse response is best to help reduce the risk of violence?

- A. "You need to lower your voice and calm down right now."
- B. "Come with me to a quiet room so we can talk about what is upsetting you."
- C. "If you keep acting like this, security will be called."
- D. "There is nothing I can do until the provider is available."

47. A client is admitted to a mental health unit after stopping heavy daily alcohol use 24 hours ago. The client is anxious, tremulous, and pacing. Which statement by the client should the nurse report immediately to the provider as the strongest indicator of possible progression to severe alcohol withdrawal?

- A. "I feel sick to my stomach and don't want to eat."

- B. "My heart feels like it's racing, and I can't sit still."
- C. "I keep seeing shadows moving, even when no one is there."
- D. "I have a pounding headache and feel very irritable."

48. A client with borderline personality disorder becomes upset after a family member leaves and says to the nurse, "You are the only one here who actually cares. Don't go help anyone else right now." The client starts crying loudly and blocks the doorway when the nurse tries to leave the room. Which response by the nurse is most appropriate?

- A. Stay in the room until the client stops crying so the client does not feel abandoned
- B. Tell the client that the behavior is inappropriate and leave the room immediately without further discussion
- C. Say, "I can stay with you for 5 minutes, and then I need to care for other clients. Let's talk about what you're feeling right now."
- D. Ask another staff member to take over all care for this client for the rest of the shift

49. A client newly admitted after a house fire says to the nurse, "I keep hearing the smoke alarm in my head. If I had gotten everyone out faster, maybe my dog would still be alive." Which response by the nurse is most therapeutic?

- A. You need to stop blaming yourself because accidents happen.
- B. At least the rest of your family made it out safely.
- C. It sounds as though you're feeling guilty and overwhelmed by what happened.
- D. Try to focus on getting some rest so you can think more clearly.

50. A nurse is caring for a client 1 day after surgery who becomes increasingly agitated when told the provider has not yet written discharge orders. The client clenches the bed linens, speaks loudly, and says, "You people keep changing the plan. I'm done listening." Which nursing response is most appropriate to de-escalate the situation?

- A. "You need to lower your voice so we can talk about this appropriately."
- B. "I can see this is frustrating. Let's talk about what changed and what I can clarify for you."
- C. "The provider is busy, so you'll need to be patient until orders are entered."
- D. "If you keep talking like that, I will have to call security."

51. A resident with moderate dementia is sitting in the day room folding and unfolding a washcloth. The resident asks the practical nurse, "What time is the train leaving? I need to meet my husband." The resident appears calm and is not trying to leave the unit. Which response by the practical nurse is most appropriate?

- A.** "There is no train. You live here now, and your husband died years ago."
- B.** "Your husband knows where you are. Tell me about where you used to meet him."
- C.** "You already asked that several times, so try to remember what I told you."
- D.** "If you stop talking about the train, I will help you find a snack."

52. A client says to the nurse, "When I get stressed, I keep adding more things to my schedule because staying busy helps me avoid thinking about it." Which response by the nurse is most appropriate?

- A.** Keeping busy is a good long-term solution as long as you do not miss any deadlines.
- B.** Try to ignore the stress until your schedule becomes less demanding.
- C.** Let's identify what is causing your stress and plan one realistic coping action you can use each day.
- D.** You should wait to make any changes until you feel more in control emotionally.

53. A client hospitalized for heart failure tells the practical nurse, "I am fasting today for a religious observance, so I do not want breakfast or lunch." The client is prescribed a morning diuretic and potassium supplement to be taken with food. Which action by the nurse is most appropriate?

- A.** Tell the client that hospital treatment must take priority and insist that the client eat before taking the medications
- B.** Hold the medications without notifying anyone, since the client's spiritual practice should be respected
- C.** Explore the client's wishes and notify the supervising RN and provider that the prescribed medications are ordered with food while the client plans to fast
- D.** Ask the client's family to persuade the client to stop fasting until after discharge

54. A nurse is planning care for a hospitalized client who speaks fluent English. Before taking an afternoon medication, the client says, "I need to know whether this capsule contains pork-based gelatin, because I do not use pork products for religious reasons." Which action by the nurse is most appropriate?

- A.** Tell the client that the medication is prescribed and should be taken as ordered
- B.** Ask the pharmacist to verify the capsule ingredients and discuss acceptable alternatives if needed

- C.** Advise the client to skip the medication until discharge and restart it at home
- D.** Explain that capsule ingredients are not important because the medication itself is therapeutic

55. A client with advanced pancreatic cancer is receiving end-of-life care. During a quiet moment, the client tells the nurse, "I keep thinking about all the mistakes I made. Maybe I deserve this." Which response by the nurse is most therapeutic?

- A.** "You should try not to think about that right now and focus on staying positive."
- B.** "Many people feel this way near the end of life. Would you like to talk more about what is weighing on you?"
- C.** "You do not deserve this, and you should not blame yourself for being sick."
- D.** "If you would like, I can ask your family to come so they can reassure you."

56. A nurse is caring for a client who has a small-bore nasoenteric tube for continuous enteral feeding. The pump alarm indicates occlusion, and the nurse is unable to flush the tube easily with water. Which action should the nurse take next?

- A.** Use a 30-mL syringe to apply gentle back-and-forth pressure with warm water
- B.** Advance the tube 2 to 3 cm and attempt the flush again
- C.** Instill carbonated soda into the tube to dissolve the clog
- D.** Increase the pump rate to force the formula through the tube

57. A nurse is caring for a client who had abdominal surgery 8 hours ago and is reluctant to turn in bed because of incisional pain. Which nursing action is most appropriate to promote mobility while protecting the incision?

- A.** Place the client flat in bed and limit movement until the pain is gone
- B.** Teach the client to splint the incision with a pillow while turning
- C.** Encourage the client to pull on the side rail quickly to turn independently
- D.** Massage the incision site before repositioning the client

58. A nurse is reinforcing nutrition teaching for a client who has lactose intolerance. Which food should the nurse recommend as the best source of calcium that is least likely to trigger symptoms?

- A.** Cheddar cheese
- B.** Vanilla ice cream
- C.** Calcium-fortified soy milk
- D.** Cream of mushroom soup

- 59.** A client with a newly inserted nasogastric tube reports throat and nasal discomfort and says, "I know the tube is needed, but it really bothers me." Which nonpharmacologic nursing intervention is most appropriate?
- A.** Offer frequent mouth care and lubricate the lips
 - B.** Apply a heating pad over the nose and cheeks
 - C.** Encourage the client to drink warm liquids
 - D.** Reposition the tube slightly without a prescription
- 60.** A nurse is caring for a client who has a spinal cord injury and an established bowel program. The client reports hard stools and difficulty with evacuation during the scheduled routine. Which action should the nurse take first?
- A.** Increase the client's fluid and fiber intake as prescribed
 - B.** Administer a PRN antidiarrheal medication
 - C.** Delay the bowel program until the client feels the urge to defecate
 - D.** Limit activity until bowel elimination improves
- 61.** A nurse is reinforcing teaching for a client who reports, "I usually take a 3-hour nap in the late afternoon because I'm tired, but then I can't stay asleep at night." Which instruction should the nurse provide?
- A.** Take the nap closer to bedtime so you are more relaxed for sleep
 - B.** Limit daytime naps to short periods earlier in the day
 - C.** Drink a caffeinated beverage before the nap so you wake up refreshed
 - D.** Stay in bed longer each morning to make up for lost sleep
- 62.** A practical nurse is caring for a client who is receiving continuous enteral feedings and is restrained because of repeated attempts to pull at the tubing. During assessment, the nurse notes erythema and a small open area around the client's nostril where the nasogastric tube is taped. Which action is most appropriate?
- A.** Apply extra adhesive tape to secure the tube more tightly
 - B.** Reposition the tube and retape it to relieve pressure on the nares
 - C.** Remove the tube and leave it out until the skin heals completely
 - D.** Clean the area once daily and continue monitoring without changing the setup
- 63.** A practical nurse is preparing to transfer a client with bilateral cataracts from the bed to a wheelchair for the first time since admission. Which action is most appropriate to promote safety during the transfer?
- A.** Move the client quickly so the client does not become anxious
 - B.** Explain each step of the transfer before and during the movement
 - C.** Stand behind the client and push at the shoulders to guide direction
 - D.** Ask the client to keep the eyes closed until seated in the wheelchair
- 64.** A practical nurse is documenting a client's output for the day shift. The client voided 250 mL in a urinal, had 150 mL of emesis, and 75 mL of liquid stool. The client also had one medium formed bowel movement. How much total output should the nurse record?
- A.** 325 mL
 - B.** 400 mL
 - C.** 475 mL
 - D.** 550 mL
- 65.** A practical nurse is caring for a client who has been on bed rest for 5 days. Which finding should the nurse identify as most concerning for a complication of immobility that requires prompt reporting?
- A.** Decreased appetite at breakfast
 - B.** Mild stiffness in the knees on awakening
 - C.** A sudden increase in respiratory rate and shortness of breath
 - D.** Difficulty falling asleep during the night
- 66.** A nurse is preparing to administer a scheduled dose of acetaminophen 650 mg orally to a client. After verifying the prescription, checking the MAR, and identifying the client with two identifiers, the nurse discovers that another nurse documented the same dose as given 20 minutes earlier. The client says, "I don't think I took it, but I'm not sure." Which action is most appropriate?
- A.** Administer the medication because the client still has a prescription for it
 - B.** Ask the client to decide whether to take the medication based on pain level
 - C.** Withhold the medication and clarify whether the dose was already administered before giving any acetaminophen
 - D.** Give half the dose now and the remaining half later if the client still needs it

67. A practical nurse is preparing to give a client's 1000 medications and sees that the MAR shows insulin lispro 6 units subcutaneously before meals. The breakfast dose at 0730 is documented as given by the night nurse. The client says, "I was off the unit for a test this morning and did not eat breakfast. I don't remember getting any insulin." What is the nurse's best action?

- A. Administer 6 units of insulin lispro now because the MAR shows the dose was due before breakfast
- B. Document that the client refused the breakfast insulin dose
- C. Check the client's current blood glucose and clarify the documented administration before giving any additional insulin
- D. Ask another nurse to co-sign the breakfast insulin entry and continue with the next medication

68. A nurse is preparing to administer a medication to a patient with chronic dry mouth caused by radiation therapy. The tablet is formulated to dissolve slowly in the mouth and provide absorption through the oral mucosa over time. Which route of administration should the nurse identify?

- A. Buccal
- B. Sublingual
- C. Enteral
- D. Intramuscular

69. A nurse is reviewing prescriptions for several clients. Which client is the best candidate to receive cryoprecipitate?

- A. A client with a hemoglobin of 6.9 g/dL and fatigue from iron-deficiency anemia
- B. A client with an INR of 3.1 due to warfarin therapy who needs rapid replacement of multiple clotting factors
- C. A client with thrombocytopenia and petechiae whose platelet count is 18,000/mm³
- D. A client with hypofibrinogenemia and diffuse oozing from venipuncture sites

70. A nurse needs to administer 300 mL of IV solution over 2 hr using gravity tubing with a drop factor of 12 gtt/mL. What flow rate should the nurse regulate in drops per minute?

- A. 20 gtt/min
- B. 25 gtt/min
- C. 30 gtt/min
- D. 36 gtt/min

71. A practical nurse receives a verbal report that a client has a new prescription for a controlled opioid after surgery. Before removing the medication from secure storage, which action is most appropriate?

- A. Ask the unlicensed assistive personnel to bring the medication to the room
- B. Review the medication administration record and verify the prescription and time due
- C. Remove the medication now so it is ready if the client reports pain
- D. Wait until the end of the shift to give the first dose with the next scheduled medications

72. A nurse is reinforcing teaching for a client who will go home with a patient-controlled intrathecal pain pump for chronic pain. Which client statement indicates correct understanding of pump safety?

- A. I should increase the dose myself if my pain is worse than usual.
- B. I should call the provider if I notice redness, swelling, or drainage at the pump site.
- C. I can let a family member press the dose button when I am resting.
- D. I should stop the pump for a few hours if I feel nauseated.

73. A nurse is preparing to administer the scheduled morning dose of oral phenytoin to a client with a seizure disorder. Which pre-administration data is most important for the nurse to review?

- A. The client's most recent serum phenytoin level
- B. The client's bowel movement pattern over the last 24 hours
- C. The client's daily fluid intake record
- D. The client's fasting blood glucose level

74. A client with open-angle glaucoma began using prescribed timolol eye drops 2 weeks ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A. Intraocular pressure has decreased from baseline
- B. Pupils are equal and reactive to light
- C. Eye drainage is clear and without odor
- D. Visual acuity is unchanged from the previous visit

75. A client with newly diagnosed hyperthyroidism has been taking propylthiouracil for 10 days. Which new finding should the nurse report to the provider immediately as a possible serious adverse response to this medication?

- A. Sore throat and fever
- B. Mild weight gain
- C. Decreased sweating
- D. Sleeping longer at night

76. A nurse is reinforcing teaching for a client who is starting levothyroxine for hypothyroidism. Which instruction should the nurse include?

- A.** Take the medication at bedtime with a snack to reduce stomach upset.
- B.** Stop taking the medication once your energy level improves.
- C.** Take the medication at the same time each morning on an empty stomach.
- D.** Expect the medication to relieve symptoms within a few hours of the first dose.

77. A nurse is preparing to administer an intramuscular dose of hepatitis B vaccine in a clinic. The nurse notices that the vaccine vial was left on a countertop for several hours after being removed from the refrigerator. The vial is unopened, and the printed expiration date has not passed. Which action should the nurse take?

- A.** Administer the vaccine because the vial is unopened and not expired
- B.** Return the vial to the refrigerator and use it later in the day
- C.** Check the vaccine storage guidance and follow facility policy before using the vial
- D.** Shake the vial well and administer it if the solution looks normal

78. A nurse is preparing to administer 0.125 mg of levothyroxine by mouth. The available tablets are labeled 50 mcg each. How many tablets should the nurse administer?

- A.** 1 tablet
- B.** 2 tablets
- C.** 2.5 tablets
- D.** 4 tablets

79. A nurse is obtaining a stool specimen from a client for ova and parasite testing. Which instruction should the nurse give the client?

- A.** Collect the specimen from toilet water after the bowel movement
- B.** Place urine and stool in the same collection container
- C.** Avoid mixing the stool specimen with urine or toilet paper
- D.** Refrigerate the specimen overnight before sending it to the laboratory

80. A nurse is reviewing laboratory results for a client who has acute hepatitis. Which finding should the nurse expect?

- A.** Elevated alanine aminotransferase (ALT)
- B.** Decreased serum bilirubin
- C.** Low ammonia level
- D.** Decreased alkaline phosphatase

81. A client who is 36 weeks pregnant calls the clinic and says, "For the past hour, I have had constant upper right belly pain and I just vomited. My blood pressure was high at my last visit 2 days ago." Which instruction should the nurse give first?

- A.** Lie on the left side, drink water, and call back in 2 hours if the pain continues
- B.** Go to the hospital now for immediate evaluation
- C.** Take an antacid because this is most likely heartburn from late pregnancy
- D.** Schedule the next available office appointment for later today

82. A nurse is preparing to perform a bedside urine dipstick test on a specimen that was collected 1 hour ago and left uncovered on the counter. The client is waiting for results related to possible urinary tract irritation. Which action is most appropriate?

- A.** Perform the test because urine remains acceptable for point-of-care testing for up to 2 hours at room temperature
- B.** Stir the specimen well and perform the test immediately to reduce settling
- C.** Discard the specimen and obtain a fresh urine sample before testing
- D.** Test the specimen only for leukocytes and nitrites, since those are least affected by standing

83. A practical nurse is obtaining vital signs on an adult client who has an irregular heartbeat. To obtain the most accurate pulse rate, which action should the nurse take?

- A.** Count the radial pulse for 15 seconds and multiply by 4
- B.** Count the radial pulse for 30 seconds and multiply by 2
- C.** Count the apical pulse for 1 full minute
- D.** Use the pulse oximeter reading instead of palpating a pulse

84. A client is scheduled for an upper gastrointestinal (GI) series with barium this morning. Which statement by the client requires the nurse to notify the provider before the procedure?

- A.** I took my usual blood pressure medicine with a sip of water when I woke up.
- B.** I chewed sugar-free gum on the way to the hospital.
- C.** I had scrambled eggs and toast about 2 hours ago.
- D.** I removed my dentures before coming in.

85. A nurse is obtaining a 12-lead ECG for a client and notices that one lead shows a tracing that is much smaller than the others and intermittently cuts out. The client denies symptoms, and the other leads appear stable. Which action should the nurse take first?

- A.** Document the finding as a likely cardiac abnormality
- B.** Reposition the client into high-Fowler's position

C. Check the electrode and lead-wire connection for that lead

D. Ask the client to take slow, deep breaths during the tracing

86. A nurse is obtaining a venous blood specimen from an adult client. Immediately after the needle is removed, the client says, "I take warfarin every day." Which action is most appropriate?

A. Apply firm pressure to the site longer than usual and inspect for continued bleeding before applying a dressing

B. Massage the puncture site to help the vein seal and prevent bruising

C. Ask the client to bend the arm tightly at the elbow for several minutes

D. Apply a warm compress to the site right away to reduce bleeding

87. A practical nurse is caring for a client who is returning from surgery after receiving spinal anesthesia for a hernia repair. Which assessment finding is most important for the nurse to report to the RN immediately?

A. Blood pressure 138/84 mm Hg and heart rate 88/min

B. Inability to move the legs 30 minutes after arrival

C. Complaint of a severe headache when sitting upright

D. Pain rating of 4 on a 0-to-10 scale at the incision site

88. A client with a newly inserted peripheral IV catheter is receiving an antibiotic infusion. A few minutes after the infusion begins, the client reports itching and burning at the site. The nurse notes swelling and a small amount of fluid leaking around the insertion site, but the skin is warm rather than cool. Which complication is most likely occurring?

A. Infiltration

B. Phlebitis

C. Local allergic reaction to the dressing

D. Extravasation

89. A nurse is caring for a client 6 hours after a tonsillectomy. The client is swallowing frequently and reports nausea. Which focused assessment should the nurse perform first?

A. Inspect the back of the throat for active bleeding

B. Ask the client to rate the throat pain on a 0 to 10 scale

C. Auscultate bowel sounds in all four quadrants

D. Check the client's temperature for possible infection

90. A client with type 1 diabetes is being treated for diabetic ketoacidosis with IV regular insulin and fluids. Which finding best indicates the treatment is effective?

A. Blood glucose decreases from 420 mg/dL to 240 mg/dL

B. Urine output increases from 20 mL/hr to 35 mL/hr

C. Serum potassium decreases from 5.6 mEq/L to 4.2 mEq/L

D. Respirations change from deep and rapid to even and unlabored

91. A client 36 hours after a thyroidectomy becomes restless and reports tingling around the mouth and in the fingertips. The nurse notes muscle twitching and a harsh inspiratory sound when the client breathes. Which action should the nurse take first?

A. Place the client in high-Fowler's position and encourage coughing

B. Prepare to administer IV calcium gluconate as prescribed

C. Ask the client to describe the pain using a 0 to 10 scale

D. Offer clear liquids to reduce throat irritation

92. A client with chronic kidney disease who is not on dialysis reports increasing muscle weakness. Which assessment finding should the practical nurse report to the nurse immediately?

A. Serum potassium level of 6.2 mEq/L

B. Blood pressure of 148/88 mm Hg

C. Dependent ankle edema at the end of the day

D. Reports of decreased appetite for 1 week

93. A client with a severe allergy to peanuts accidentally eats a cookie containing peanut butter. Within minutes, the client develops hoarseness, widespread hives, inspiratory stridor, and a blood pressure of 84/50 mm Hg. Which action should the nurse take first?

A. Administer intramuscular epinephrine

B. Apply a warm blanket and recheck vital signs in 15 minutes

C. Obtain a prescription for an antihistamine

D. Start oxygen at 2 L/min by nasal cannula

94. A client is 12 hours postoperative after abdominal surgery and has a Jackson-Pratt drain. When assessing the drain, the nurse finds the bulb expanded and half full of serosanguineous fluid. Which action should the nurse take?

A. Irrigate the drain with sterile saline to restore flow

B. Compress the bulb after emptying it to reestablish suction

C. Clamp the tubing for 30 minutes after emptying the bulb

D. Raise the drain above the incision to improve drainage

95. A nurse is caring for a client with heat exhaustion after several hours of yard work in hot weather. Which finding best indicates that the client's thermoregulatory mechanisms are still functioning?

A. Hot, dry skin with altered mental status

B. Profuse sweating with cool, moist skin

- C. Absence of thirst despite prolonged heat exposure
- D. Core temperature continuing to rise above 40.5°C (104.9°F)

96. A nurse is caring for a client with a pressure injury on the heel. The wound bed appears beefy red, moist, and bumpy. Which description should the nurse document?

- A. Eschar
- B. Slough
- C. Granulation tissue
- D. Maceration

97. A nurse responds to an adult client who suddenly becomes unresponsive in the dialysis unit. The scene is safe. The nurse checks for responsiveness, calls for help, and assesses breathing and pulse for no more than 10 seconds. The client has no pulse. A second nurse arrives with an AED while CPR is in progress. Which action is most appropriate when the AED says, "Analyzing rhythm"?

- A. Continue chest compressions until the AED announces whether a shock is advised
- B. Stop touching the client and clear the area until the AED finishes analyzing
- C. Give 2 rescue breaths before allowing the AED to continue
- D. Check the carotid pulse again to confirm the AED finding

98. A nurse is caring for a client 8 hours after an open cholecystectomy. Earlier in the shift, the client was awake, oriented, and reporting incisional pain of 6/10. The client now appears anxious and says, "Something feels wrong." Which finding should the nurse recognize as the priority change in condition?

- A. Blood pressure 84/48 mm Hg and heart rate 124/min
- B. Temperature 38.1°C (100.6°F)
- C. Pain at the incision rated 7/10 with movement
- D. Absent bowel sounds in all quadrants

99. A nurse is caring for a client who has chronic kidney disease and missed the last 2 scheduled dialysis treatments. Which finding should the nurse identify as the priority to report immediately as a possible manifestation of hyperkalemia?

- A. Muscle weakness and peaked T waves on the ECG
- B. Dry mucous membranes and increased thirst
- C. Hyperactive bowel sounds and abdominal cramping
- D. Perioral numbness and a positive Chvostek sign

100. A client receiving peritoneal dialysis has had 1,500 mL of dialysate infused. After the prescribed dwell time, only 900 mL has drained after the first few minutes of outflow. The client denies pain, and the abdomen is soft. Which action should the practical nurse take first?

- A. Reposition the client and check for tubing kinks
- B. Warm a new bag of dialysate and restart the infusion
- C. Add heparin to the dialysate solution
- D. Stop the procedure and notify the provider immediately

Simulation Test 25 — Answer Key

1. C	26. A	51. B	76. C
2. D	27. B	52. C	77. C
3. A	28. B	53. C	78. C
4. C	29. C	54. B	79. C
5. C	30. C	55. B	80. A
6. C	31. C	56. A	81. B
7. B	32. C	57. B	82. C
8. C	33. A	58. C	83. C
9. C	34. A	59. A	84. C
10. B	35. C	60. A	85. C
11. C	36. B	61. B	86. A
12. C	37. B	62. B	87. C
13. B	38. C	63. B	88. D
14. B	39. B	64. C	89. A
15. C	40. B	65. C	90. D
16. C	41. B	66. C	91. B
17. D	42. C	67. C	92. A
18. C	43. B	68. A	93. A
19. C	44. C	69. D	94. B
20. D	45. A	70. C	95. B
21. C	46. B	71. B	96. C
22. C	47. C	72. B	97. B
23. B	48. C	73. A	98. A
24. A	49. C	74. A	99. A
25. B	50. B	75. A	100. A

Simulation Test 26

100 questions

1. While covering another nurse's assignment for 20 minutes, a practical nurse receives these reports from an assistive personnel staff member. Which client should the nurse check first?
 - A. A client with pneumonia who is suddenly restless and has an oxygen saturation drop from 95% to 89%
 - B. A client with a hip fracture who asks for help back to bed after sitting in a chair for 30 minutes
 - C. A client with heartburn who says the prescribed antacid did not relieve burning after lunch
 - D. A client scheduled for discharge who wants written instructions reviewed again with family
2. An assistive personnel (AP) tells the practical nurse, "I just got a finger-stick glucose of 48 mg/dL on Mr. Lane before lunch." Which instruction by the practical nurse is most appropriate?
 - A. "Give him his lunch tray now and chart the result afterward."
 - B. "Repeat the glucose in 2 hours to see if it improves on its own."
 - C. "Stay with the client while I assess him and manage this immediately."
 - D. "Call dietary and ask them to bring orange juice to the room."
3. A nurse is caring for an adult client who is alert, oriented, and has decision-making capacity. Before a dressing change, the client says, "Please close the door. I do not want the student nurse or anyone else watching my wound unless I agree first." What is the nurse's best action?
 - A. Tell the client that students may observe because this is a teaching facility
 - B. Ask the client whether the student nurse may stay, and honor the client's decision
 - C. Have the student nurse remain in the room but stand behind the privacy curtain
 - D. Explain that the dressing change is routine and does not require special permission for observers
4. A nurse is admitting an adult client who is deaf and uses American Sign Language. The client is scheduled for a procedure later that day. The client's friend says, "I know sign language well enough. I can interpret so we can move faster." The client looks uncertain. Which action by the nurse best advocates for the client's rights?
 - A. Accept the friend's help because the procedure is scheduled soon
 - B. Proceed with teaching using written instructions only to avoid delay
 - C. Request a qualified interpreter and postpone further consent discussion until communication is adequate
 - D. Ask the friend to interpret only the consent form while the nurse explains the rest
5. A practical nurse is giving bedside care when the client asks that a neighbor who drove them to the hospital be allowed to stay in the room during the discussion of the treatment plan. What should the nurse do first?
 - A. Ask the client directly whether they want the neighbor present for the discussion
 - B. Allow the neighbor to stay because the client brought the neighbor to the hospital
 - C. Tell the neighbor to leave because nonfamily members can never hear medical information
 - D. Discuss the plan quietly so the neighbor hears only part of the conversation
6. A practical nurse is updating a client's electronic chart at the end of the shift. Another nurse asks to use the same workstation immediately to review medications for a different client. The first nurse is still logged in. Which action is most appropriate?
 - A. Stay at the workstation and let the other nurse review the second client's chart under the first nurse's login
 - B. Log off or lock the workstation before stepping away so the other nurse accesses the record using their own credentials
 - C. Open the second client's chart for the other nurse, then leave once the chart is displayed
 - D. Give the other nurse a quick verbal summary instead of allowing chart access
7. A nurse is admitting an adult client who is alert, oriented, and able to make decisions. The client says, "My daughter brought me a copy of my advance directive, but I left the original at home. Does this copy still matter?" Which response by the nurse is most appropriate?
 - A. A copy can be placed in the chart so the health care team can review the client's stated wishes
 - B. Only the original document can be used to guide care decisions
 - C. The client should complete a new advance directive during this admission because copies are not acceptable
 - D. The copy is useful only after the client loses decision-making capacity and cannot be reviewed now

8. A nurse is caring for an adult client who is no longer able to make decisions. The chart includes a valid advance directive stating refusal of prolonged mechanical ventilation in an irreversible condition. The provider tells the nurse, "The family is divided, so we will continue full treatment until they agree." Which action by the nurse is most appropriate?

- A.** Support continuing treatment because family agreement is required before the directive can be followed
- B.** Ask the family to vote so the majority decision can guide the plan of care
- C.** Advocate for the client's documented wishes and notify the charge nurse or appropriate supervisor of the conflict
- D.** Delay any discussion of the directive until the client regains decision-making capacity

9. A competent adult client is scheduled for an elective renal biopsy. The consent form has already been signed after the provider discussed the procedure. As the nurse prepares the client, the client asks, "If I sign this, can I still change my mind later?" Which response by the nurse is best?

- A.** No, once the form is signed, the procedure must be completed as planned.
- B.** Yes, you may withdraw consent at any time before the procedure.
- C.** Only the provider can cancel the procedure after the form is signed.
- D.** Yes, but only if a family member agrees with your decision.

10. A nurse is asked to witness a client's signature for consent to an elective angioplasty. The client is alert and says, "The cardiologist explained the benefits and risks, and I understand those. But I haven't decided because I want to know what could happen if I do nothing for now." Which action should the nurse take?

- A.** Witness the signature because the client understands the procedure's risks and benefits.
- B.** Explain the possible consequences of refusing treatment so the consent can be completed.
- C.** Hold the consent process and notify the provider that the client needs more information before consenting.
- D.** Ask the client's spouse to help the client make the decision and then sign the consent.

11. A client is preparing for discharge after hospitalization for a new diagnosis of heart failure. The client is prescribed several medications, a low-sodium diet, and a follow-up visit in 1 week. During discharge teaching, the client says, "I can read the instructions later, but I don't have a ride to the clinic and the bus stop is three blocks from my house." Which action should the nurse take first?

- A.** Ask the client to arrange transportation with a family member after discharge
- B.** Notify the case manager or social worker about the transportation barrier before discharge
- C.** Document that the client received the written discharge instructions
- D.** Tell the client to call the clinic to reschedule if the appointment is missed

12. A client is being discharged after treatment for a venous stasis ulcer. The provider prescribes compression therapy, dressing changes three times each week, and a follow-up wound clinic visit. The client lives alone, has limited vision, and says, "I can probably feel my way through the dressing change, but I can't read the labels on the supplies." The client's daughter visits only on weekends. Which discharge plan is most appropriate?

- A.** Arrange home health nursing to assist with wound care and reinforce teaching at home
- B.** Delay discharge until the daughter can take over all dressing changes
- C.** Provide extra written instructions in large print and discharge the client without referrals
- D.** Advise the client to remove compression wraps if they feel too tight and reapply them later

13. A long-term care unit is testing a new bedside brief to reduce omitted information during shift report. For the first 2 weeks, the team plans to try the brief on one hallway only and review the results before expanding it. What is the best rationale for this approach?

- A.** It allows the team to test the change on a small scale and adjust it before wider use
- B.** It ensures the results will apply equally to every unit in the facility
- C.** It prevents staff from needing any training before the new process begins
- D.** It proves that the new bedside brief is better than all other handoff methods

- 14.** A clinic is trying to reduce delays in calling clients with critical laboratory results. A review shows that delays are most common on days when the ordering provider is away from the clinic and staff are unsure who should be notified next. Which quality improvement action is most appropriate to test first?
- A.** Remind staff to work faster when laboratory reports arrive
 - B.** Create a clear backup notification pathway for critical results when the primary provider is unavailable
 - C.** Begin tracking client satisfaction scores related to clinic visits
 - D.** Discontinue calling clients until the provider returns and can give instructions personally
- 15.** A practical nurse is giving a change-of-shift report about a client who has an indwelling urinary catheter. Which information is most important to communicate to the oncoming nurse?
- A.** The client prefers to watch television before bedtime
 - B.** The urine has become cloudy and foul-smelling during the shift, and the client reports burning discomfort
 - C.** The client drank two cups of tea with dinner
 - D.** The family visited for 1 hour this afternoon
- 16.** A practical nurse is updating the interdisciplinary team about a client recovering from hip fracture repair. Which report best supports coordinated care planning?
- A.** The client is doing better today and will probably be ready for discharge soon.
 - B.** The client says the pain medicine is not strong enough and wants to go home tomorrow.
 - C.** The client required two staff members to transfer to the chair, became dizzy when standing, and states there are three steps to enter the home and no one is available during the day.
 - D.** The client had breakfast late because transport took the client to x-ray first.
- 17.** A practical nurse is caring for an adult client who is alert and oriented. During medication administration, the client quietly tells the nurse, "Please don't tell my partner about my new diagnosis. I want to talk about it myself first." A short time later, the partner asks the nurse, "What did the test show?" Which response by the nurse is most appropriate?
- A.** Explain the diagnosis because the partner is involved in the client's care
 - B.** Tell the partner that information cannot be shared without the client's permission
 - C.** Ask the partner to return later, then provide the information when the client is asleep
 - D.** Give a general update without naming the diagnosis
- 18.** A practical nurse on a medical-surgical unit is asked by an experienced UAP to insert an indwelling urinary catheter for a newly admitted client because "I do it all the time at my old facility." The nurse knows that catheter insertion is not within the UAP's role at this facility. Staffing is short, and the RN is busy with another client. Which action by the practical nurse is most appropriate?
- A.** Allow the UAP to perform the procedure since the UAP has prior experience
 - B.** Ask the UAP to perform the procedure while the nurse remains available nearby
 - C.** Decline to delegate the procedure and follow facility policy for who may perform it
 - D.** Let the UAP begin preparation, then decide whether to stop based on the client's condition
- 19.** A practical nurse in a community health clinic is caring for a 9-year-old child with frequent school absences. During the visit, the nurse notices the child is very thin, wears clothing with a strong urine odor, and says, "Sometimes I stay home to take care of my baby sister when my mom is gone." The caregiver states, "We're just having a hard time right now," and asks the nurse not to involve anyone else. Which action by the nurse is most appropriate?
- A.** Document the findings and wait for additional evidence before taking further action
 - B.** Teach the caregiver about community resources and reassess the family at the next visit
 - C.** Report the suspected neglect according to agency policy and state law
 - D.** Ask the school nurse to monitor the child before deciding whether a report is needed
- 20.** At the beginning of the shift, a practical nurse asks an assistive personnel (AP) to reposition a bedridden client every 2 hours and apply a moisture barrier after each episode of incontinence. Which outcome best indicates the nurse managed the AP appropriately?
- A.** The AP decides to skip one repositioning because the client is asleep comfortably
 - B.** The AP documents skin redness over the coccyx and reports it to the nurse promptly
 - C.** The AP asks the client's family to help turn the client during visiting hours
 - D.** The AP reapplies the moisture barrier more often than ordered to prevent breakdown

21. A practical nurse in a primary care office is helping plan follow-up for clients with newly diagnosed sleep apnea. Review of scheduling records shows that clients who rely on county paratransit complete far fewer sleep-study appointments than other clients because the testing center starts most studies after the paratransit service stops running. Which recommendation by the nurse best promotes equitable access to care?

- A.** Advise clients to ask family or friends for rides to evening appointments
- B.** Provide written teaching that stresses the importance of keeping the scheduled study time
- C.** Recommend adding appointment options that align with paratransit operating hours or arranging an accessible alternative testing pathway
- D.** Schedule the clients with the fewest missed visits first because they are most likely to complete testing

22. A nurse is preparing to administer a prescribed nebulizer treatment in a skilled nursing facility. The client is sitting in the day room with several residents. When the nurse approaches, the client smiles and says, "I'm ready for my breathing treatment." Which action should the nurse take to identify the client correctly before administering the treatment?

- A.** Ask the client to state their full name and date of birth, and compare the information with the identification band and medication record
- B.** Confirm the client's identity by asking a staff member who usually cares for the client
- C.** Use the client's room number and usual treatment time to verify identity
- D.** Ask the client whether they are waiting for a breathing treatment and proceed if the answer is yes

23. A nurse is preparing to ambulate a client who had hip surgery yesterday using a gait belt. As the nurse inspects the belt, the buckle closes, but the fabric near the buckle is frayed and partially torn. The client is eager to walk and says, "It will probably hold for one trip." Which action should the nurse take?

- A.** Use the gait belt carefully and ask a second staff member to walk alongside the client
- B.** Place the torn area away from the buckle and proceed with the transfer
- C.** Do not use the gait belt; obtain intact equipment before ambulating the client
- D.** Skip the gait belt and assist the client by holding the client's underarms during ambulation

24. A nurse is preparing to administer the first dose of enteral feeding to a client through a nasogastric tube. The client states, "I have a severe soy allergy. The last time I ate something with soy, my throat started to close." The formula label lists soy protein isolate as an ingredient, and no allergy is documented in the record. Which action should the nurse take first?

- A.** Administer the feeding at a slower rate while observing for a reaction
- B.** Hold the feeding and notify the provider and pharmacy about the reported soy allergy
- C.** Give the feeding because the allergy is not yet documented in the chart
- D.** Ask the client to sign a refusal form before withholding the feeding

25. A practical nurse is doing a safety check in a home health client's apartment. Which finding should the nurse address first to reduce immediate environmental injury risk?

- A.** A throw rug is loose at the edge near the bathroom doorway
- B.** A lamp cord is placed under the living room rug
- C.** A space heater is operating less than 1 foot from bedding
- D.** A hallway night-light is burned out

26. A client 1 day after a stroke becomes restless and repeatedly tries to remove the nasal cannula. The client is not violent, can follow simple commands, and becomes calmer when the nurse reduces noise in the room and explains the oxygen equipment. Which action should the nurse take next to follow least-restrictive restraint principles?

- A.** Apply soft wrist restraints to prevent removal of the oxygen tubing
- B.** Use a sheet to secure the client's hands at the sides
- C.** Continue nonrestraint measures such as close observation, reorientation, and redirection
- D.** Raise all 4 side rails and leave the room

27. A client with delirium has a temporary central venous catheter and repeatedly reaches for the dressing during episodes of agitation. The practical nurse has already tried reorientation, pain assessment, toileting, repositioning, moving the line out of the client's view, and having a staff member stay at the bedside, but the client continues trying to pull at the catheter. The provider prescribes soft wrist restraints. Which action is most appropriate?

- A.** Apply the restraints as prescribed and discontinue them as soon as the client can be managed safely with less-restrictive measures
- B.** Apply the restraints and keep them on until the catheter is removed, even if the client later settles down

- C.** Refuse to apply the restraints because restraints are never appropriate for confused clients
- D.** Ask a family member to hold the client's arms down instead of using restraints

28. A nurse is preparing to apply sequential compression devices for a postoperative client and notices that the sleeves brought to the room are labeled for another client. The sleeves have not touched the client, and no treatment has been started. Which action is most appropriate?

- A.** Apply the sleeves because the equipment is clean and the error was caught before use
- B.** Obtain the correct sleeves for the client and report the event according to facility policy
- C.** Place the sleeves in the room for later use after confirming the compression settings
- D.** Document the sleeves were unavailable and delay therapy until the next shift

29. A nurse is transferring a client from the bed to a wheelchair using a transfer board. Which client statement requires the nurse to stop and choose a different transfer method?

- A.** I can support my head and sit up without help.
- B.** I have a pressure ulcer on my sacrum.
- C.** I can follow directions and move my arms well.
- D.** I had pain medicine 30 minutes ago, but I feel alert.

30. A practical nurse is preparing to perform finger-stick blood glucose testing for several residents during a morning medication pass. Which action is most appropriate to prevent transmission of infection between residents?

- A.** Use the same lancet device for all residents after wiping it with alcohol between uses
- B.** Wear one pair of gloves for the entire medication pass if the gloves remain clean
- C.** Use a new single-use lancet for each resident and perform hand hygiene with glove changes between residents
- D.** Place the glucometer on each resident's bed while obtaining the sample to avoid carrying it by hand

31. A nurse is caring for a client on contact precautions for a draining skin infection. After completing wound care, which sequence for removing personal protective equipment before leaving the room is most appropriate?

- A.** Remove gown, remove gloves, perform hand hygiene, leave the room
- B.** Remove gloves, perform hand hygiene, remove gown, perform hand hygiene
- C.** Remove gloves, remove gown, perform hand hygiene
- D.** Perform hand hygiene over gloved hands, remove gown, remove gloves

32. A practical nurse is assisting with a sterile dressing change and needs to pour sterile normal saline into a sterile basin on the field. Which action best maintains asepsis while pouring the solution?

- A.** Hold the bottle 1 inch above the basin and pour without letting the bottle touch the basin
- B.** Rest the lip of the bottle on the edge of the sterile basin to prevent splashing
- C.** Pour a small amount of solution into the basin, then recap the bottle and place it inside the sterile field
- D.** Ask an assistant wearing clean gloves to hold the sterile basin while the nurse pours the solution

33. A practical nurse is caring for a client who has a productive cough, fever, and a chest x-ray suggestive of pulmonary tuberculosis. The client arrives on the unit before test results are confirmed. Which action should the nurse take first?

- A.** Place the client in a negative-pressure room and keep the door closed
- B.** Apply contact precautions until the sputum culture report is available
- C.** Ask the client to wear a surgical mask only when leaving the room
- D.** Wait to begin isolation precautions until the provider documents the diagnosis

34. A practical nurse is preparing to obtain a urine specimen from the sampling port of an indwelling urinary catheter for a client who has no isolation orders. The nurse expects possible contact with urine but does not expect splashing. Which action best follows standard precautions?

- A.** Wear clean gloves while handling the sampling port and specimen container
- B.** Wear a sterile gown because the catheter is an invasive device
- C.** Use a mask and face shield because all urine is considered high risk for airborne spread
- D.** No personal protective equipment is needed if the client has no known infection

35. A nurse is reinforcing teaching with a client who is 24 weeks pregnant about nutrition. Which client statement indicates correct understanding of foods to avoid during pregnancy?

- A.** I can eat deli turkey as long as it is served cold and looks fresh.
- B.** I should avoid shark and swordfish because they can contain high levels of mercury.
- C.** Soft scrambled eggs are safe if the yolks are still a little runny.
- D.** Unpasteurized fruit juice is safe if I drink it the same day I buy it.

36. A practical nurse is reinforcing teaching with the parent of an 18-year-old who recently started college and moved into a dormitory 1 week ago. The parent says, "My child calls every night saying they miss home, feel overwhelmed by classes, and are not sure they made the right decision." Which response by the nurse is most appropriate?

- A.** "This usually means your child is not emotionally ready for college and should come home for a while."
- B.** "These feelings can occur during a normal transition, so encourage your child to use campus supports while adjusting to the new routine."
- C.** "Tell your child to avoid meeting new people until the stress of classes decreases."
- D.** "This is a sign that the course load is too difficult, so dropping several classes is the best first step."

37. A practical nurse is reinforcing teaching at a wellness visit. Which client statement best shows an understanding of the difference between screening and prevention?

- A.** "Screening and prevention are the same because both are used to stop disease before it starts."
- B.** "Screening looks for possible disease early, while prevention focuses on lowering the chance that disease will occur."
- C.** "Screening is only needed when symptoms appear, but prevention is used when a person feels well."
- D.** "Prevention is mainly follow-up care after an abnormal test result, and screening is treatment."

38. A nurse is obtaining a health history from an adult client who says, "I haven't been sleeping well lately." Which follow-up question best helps the nurse assess factors that worsen this problem?

- A.** "How many hours of sleep do you usually get each night?"
- B.** "What do you do when you cannot fall asleep?"
- C.** "When did you first notice the sleep problem?"
- D.** "Does anything seem to make your sleep worse?"

39. A practical nurse is admitting an adult client with long-standing left-sided weakness after a previous stroke. Which documentation best establishes baseline musculoskeletal status for future comparison during the stay?

- A.** Client needs assistance with meals and prefers the call light within reach
- B.** Left hand grasp weak; left leg strength 2/5; requires one-person assist to transfer from bed to chair
- C.** Client states, "My left side has been weak since my stroke 2 years ago"
- D.** Range of motion exercises performed to both lower extremities after admission

40. A nurse in a family clinic is reviewing immunization plans for several clients. Which client should the nurse identify as appropriate to receive a live vaccine today?

- A.** A 7-year-old child who is taking high-dose oral prednisone for nephrotic syndrome
- B.** A 28-year-old client who is pregnant and due for prenatal teaching
- C.** A 15-month-old toddler who has a temperature of 37.4° C (99.3° F) and clear rhinorrhea
- D.** A 10-year-old child who received another live vaccine 1 week ago

41. A nurse is reinforcing teaching with the caregiver of a 4-year-old child about normal growth and development. Which activity is most appropriate for this child?

- A.** Putting together a 50-piece jigsaw puzzle independently
- B.** Playing dress-up and pretending to be a firefighter
- C.** Keeping a detailed diary about daily events
- D.** Joining a competitive peer-led sports league with formal rules

42. A nurse is explaining a low-sodium diet to a client. The client says, "I can read your lips a little, but if more than one person talks, I get lost." Which action by the nurse is best to reduce this communication barrier?

- A.** Speak very loudly while standing at the side of the client
- B.** Face the client directly and speak one sentence at a time in a quiet setting
- C.** Repeat the teaching several times without changing the environment
- D.** Provide the client with printed instructions and postpone all verbal teaching

43. A nurse is preparing to teach a client how to perform range-of-motion exercises after knee surgery. The client says, "I want to learn, but my pain is an 8 out of 10 right now, and I can't pay attention to anything." Which nursing action is best?

- A.** Ask the client to sign the teaching form now and repeat the instructions later
- B.** Provide the teaching quickly before the next dose of pain medication is given
- C.** Delay the teaching until after treating the client's pain and the client can focus
- D.** Give the client written instructions to read independently despite the pain

44. A client whose house was destroyed in a fire is admitted for treatment of minor smoke inhalation. The client says, "I keep thinking about everything we lost. I can't stop replaying it in my head." Which response by the practical nurse best provides emotional support?

- A.** "You should try not to think about it so you can focus on getting better."

B. "At least you and your family got out safely, and that is what matters most."

C. "This sounds overwhelming. Would you like to talk more about what has been hardest for you?"

D. "I know exactly how you feel, because I went through something similar once."

45. A client who is starting hemodialysis says to the nurse, "I get so overwhelmed before each treatment that I just scroll on my phone and try not to think about it. Then I forget the questions I wanted to ask." Which nursing response is most appropriate to promote effective coping?

A. "Try not to focus on the treatment, because thinking about it will only make you more upset."

B. "Let's make a short list of your questions now so you can bring it to each treatment."

C. "You need to accept that dialysis is part of your life now and stop avoiding it."

D. "It would be better to wait until you feel less anxious before talking about your concerns."

46. A nurse in a rehabilitation unit is caring for a client who has become increasingly tense after a phone call. The client is pacing near the nurses' station, jaw is clenched, and speech is short and loud. Which action by the nurse is most appropriate to reduce the client's risk for violence?

A. Stand directly in front of the client and explain unit rules firmly

B. Ask several detailed questions about what happened during the phone call

C. Speak calmly, maintain personal space, and offer to move with the client to a quiet area

D. Tell the client that the behavior is inappropriate and will not be tolerated

47. A client admitted for dehydration tells the nurse, "I drink heavily every day, and the last time I tried to stop, I had a seizure." The client is calm, oriented, and has no tremor at this time. Which nursing action is most appropriate?

A. Provide fluids and continue routine assessments because the client has no current withdrawal symptoms

B. Ask the client to notify staff if shaking or nausea begins later

C. Report the history to the provider promptly because the client is at high risk for severe withdrawal

D. Encourage the client to rest in a quiet room until cravings subside

48. A client with acute confusion after surgery becomes upset when staff enter the room to check vital signs. The client shouts, "Stop bothering me!" and throws a pillow toward the doorway. The client remains in bed and has not attempted to leave the room. Which action by the nurse is most appropriate?

A. Approach the client with two staff members and quickly complete the vital signs check

B. Tell the client that this behavior is unacceptable and that privileges will be restricted

C. Reduce stimulation, speak calmly in short simple statements, and try again in a few minutes

D. Apply wrist restraints so the client can be assessed without interruption

49. A client who had a below-the-knee amputation 2 days ago says to the nurse, "I don't even want to talk about rehab. What's the point? Nothing is ever going to be normal again." Which response by the nurse is most therapeutic?

A. You need to stay positive so you can recover more quickly.

B. Many people adjust well after an amputation and go back to their usual routines.

C. It sounds like you are feeling overwhelmed by how much your life has changed.

D. You should focus on learning to use your prosthesis when the time comes.

50. A nurse is caring for a client who has been awake most of the night and is now becoming increasingly agitated during a crowded shift change. The client is sitting on the side of the bed, speaking loudly, and saying, "Too many people are talking. Just get away from me." The client is not attempting to get up or strike anyone. Which action by the nurse is most appropriate?

A. Stand in the doorway and explain that the client must lower their voice before staff can help

B. Move closer to the client and quickly ask several questions to identify the cause of the anger

C. Reduce nearby stimulation, keep a calm nonthreatening distance, and speak briefly in a low voice

D. Tell the client that the behavior is disruptive and that security will be called if it continues

51. A practical nurse is caring for a resident with moderate dementia who becomes frustrated while getting dressed. The resident picks up a shirt, then shoes, then socks, and says, "I can't do this anymore." Which action by the practical nurse is most appropriate?

A. Lay out the clothing in the order it should be put on and give one simple direction at a time

B. Ask the resident to choose a complete outfit independently to maintain decision-making ability

C. Quickly dress the resident to prevent further frustration and save time

D. Explain each step in detail so the resident understands the full dressing process

52. A client says, "When I get stressed, I stay up late trying to fix everything at once. Then I am exhausted the next day and make even more mistakes." Which nursing teaching is most appropriate?

A. Set a specific stopping time for the evening and continue the remaining tasks the next day

B. Work until every task is completed so the client can sleep without worry

C. Avoid thinking about unfinished tasks by keeping busy with television until falling asleep

D. Use caffeine at night to maintain focus and finish more work

53. A client hospitalized for complications of diabetes tells the practical nurse, "My illness makes me feel ashamed. I believe I need to ask my spiritual leader for forgiveness, but I do not want my family to know about this." Which action by the nurse is most appropriate?

A. Tell the client that feelings of guilt are common and encourage focusing on discharge teaching instead

B. Offer to contact the client's spiritual leader privately, following facility policy and the client's wishes

C. Ask a family member to arrange the visit because family usually helps with spiritual needs

D. Explain that spiritual concerns should be discussed after the client's medical condition is stabilized

54. A nurse is reinforcing discharge teaching for a client prescribed warfarin. The client says, "I also take a traditional ginseng supplement every day because my family believes it keeps the blood strong." Which response by the nurse is most appropriate?

A. Stop taking the supplement now because prescribed medicines should always replace traditional practices.

B. Tell me more about how often you take it, and let's review it with the provider and pharmacist for possible interactions.

C. It is safe to continue the supplement as long as you take warfarin at the same time each day.

D. Use the supplement only on days when you do not take warfarin.

55. A nurse is caring for a client admitted for IV antibiotics. During conversation, the client says, "I identify as genderqueer. I don't mind telling you that, but I really do not want students coming in to ask questions about it." Which response by the nurse is best?

A. I will let the students know they should avoid speaking with you today.

B. I understand. Your gender identity is personal, and I will help make sure your care stays focused on what is necessary and respectful.

C. Students need to learn about different identities, so brief questions would still be helpful.

D. You should explain your identity to the team so they can provide appropriate care.

56. A nurse is caring for a client who is receiving continuous enteral feeding through a nasogastric tube. The client becomes restless, starts coughing, and has an oxygen saturation drop from 96% to 88%. Which action should the nurse take first?

A. Slow the feeding rate and reassess the client in 30 minutes

B. Stop the feeding and place the client in an upright position

C. Irrigate the tube with 30 mL of water

D. Check the formula expiration date

57. A nurse is teaching a client who has a right-leg injury to use crutches with a three-point gait. Which client action demonstrates correct technique?

A. Moves both crutches and the right leg forward together, then shifts weight to the hands and swings the left leg through

B. Moves the left leg forward first, then advances both crutches and the right leg

C. Moves the right crutch forward with the left leg, then the left crutch forward with the right leg

D. Advances both legs forward together, then moves both crutches forward

58. A nurse is reinforcing teaching for an older adult who has a poor appetite and needs to increase daily calorie and protein intake. Which snack choice is most appropriate?

A. Celery sticks and black coffee

B. Apple slices and plain tea

C. Peanut butter on whole-grain crackers

D. Sugar-free gelatin dessert

59. A client with osteomyelitis of the lower leg reports throbbing pain that becomes worse when the leg hangs down while sitting in a chair. Which nonpharmacologic intervention should the nurse implement first?

A. Encourage the client to ambulate in the hallway every 2 hours

B. Elevate the affected leg on pillows while the client is resting

C. Massage the painful leg for 10 minutes every 4 hours

D. Apply a heating pad to the lower leg continuously

- 60.** A nurse is caring for a client with early-stage amyotrophic lateral sclerosis (ALS) who wants to continue eating independently. The client has increasing weakness in both hands and says, "I keep dropping my fork, and it's frustrating." Which nursing intervention is most appropriate to support the client's independence with this activity of daily living?
- A.** Feed the client all meals to prevent fatigue
 - B.** Provide built-up utensil handles and allow extra time for meals
 - C.** Ask the provider for a prescription for wrist restraints during meals
 - D.** Encourage the client to eat only finger foods from this point forward
- 61.** A nurse is caring for a client who reports, "I watch television in bed until I finally fall asleep, but lately I've been lying awake for a long time." Which instruction should the nurse provide to promote better sleep?
- A.** Stay in bed and keep the television on until sleep occurs
 - B.** Get out of bed if unable to sleep and return when sleepy
 - C.** Try to make up for lost sleep by sleeping later each morning
 - D.** Use the bed during the day for reading and phone calls to relax
- 62.** A practical nurse is caring for a client who is confined to bed and slides down in semi-Fowler's position several times during the shift. Which skin-related problem is the client at greatest risk for from this repeated movement?
- A.** Shearing of tissue over the sacrum
 - B.** Maceration from excessive moisture
 - C.** Allergic dermatitis from bed linens
 - D.** Bruising from blood pressure cuffs
- 63.** A practical nurse is caring for a client who has impaired sensation in both feet and is preparing for bedtime. Which action by the nurse is most appropriate to help prevent injury?
- A.** Place an extra blanket directly over the client's feet
 - B.** Check the temperature of bath water with a thermometer before the client soaks the feet
 - C.** Massage the client's feet vigorously to improve circulation
 - D.** Encourage the client to wear tight socks so the feet stay warm
- 64.** A client is on strict intake and output. The practical nurse empties the urinary hat at 7:00 a.m. By 11:00 a.m., the client has voided 210 mL in the hat and has had 95 mL of emesis. The client also reports one episode of diarrhea in the toilet that was not measured. Which action should the nurse take when documenting output for this time period?
- A.** Record 305 mL of output and note the diarrhea as unmeasured
 - B.** Estimate the diarrhea as 100 mL and record 405 mL of output
 - C.** Record only the 210 mL of urine because emesis is not counted as output
 - D.** Wait until the end of the shift to document any output
- 65.** A practical nurse is caring for a client who has been immobile in bed for 6 days. Which intervention is most appropriate to help prevent constipation related to immobility?
- A.** Encourage increased fluid intake if not contraindicated and offer foods high in fiber
 - B.** Limit oral fluids to reduce abdominal distention
 - C.** Keep the client flat in bed after meals
 - D.** Avoid toileting at regular times to prevent fatigue
- 66.** A nurse is preparing to administer a scheduled liquid anticonvulsant to a client. The medication label reads 250 mg/5 mL, and the MAR lists the prescribed dose as 375 mg by mouth. Which action best supports the client's medication rights?
- A.** Administer 5 mL because the concentration on the label matches the medication name on the MAR
 - B.** Administer 7.5 mL after calculating the ordered dose from the available concentration
 - C.** Hold the medication and ask the pharmacy to repackage it in a different concentration
 - D.** Administer 10 mL to avoid underdosing the client
- 67.** A practical nurse is preparing a client's 0800 medications. The MAR lists potassium chloride 20 mEq PO daily. While reviewing the chart, the nurse sees that the client's potassium level from 0630 is 5.6 mEq/L, and there is no new provider prescription about the medication. The client is alert and asks for the morning pills. What is the nurse's best action?
- A.** Administer the potassium because it is still listed on the MAR
 - B.** Hold the potassium dose and notify the provider of the elevated laboratory result
 - C.** Ask another nurse to give the medication so the schedule is not delayed
 - D.** Document that the client refused the medication and return later

68. A nurse is reinforcing teaching for a patient who has been prescribed a medication by metered-dose inhaler to open the airways during episodes of wheezing. Which statement by the patient indicates correct understanding of this route of administration?

- A.** I should swallow the medication quickly after spraying it into my mouth.
- B.** I should inhale the medication deeply so it reaches my lungs.
- C.** I should place the medication under my tongue and let it dissolve.
- D.** I should apply the medication to my skin at the same time each day.

69. The nurse is preparing to start a transfusion of packed red blood cells for a client with postoperative anemia. Which action is most appropriate to complete immediately before initiating the transfusion?

- A.** Verify the blood product and client identification with another licensed nurse
- B.** Add dextrose 5% in water to the blood tubing to maintain line patency
- C.** Warm the blood at the bedside by placing the unit in hot water
- D.** Hang the unit and infuse it over 6 hours to reduce the risk of fluid overload

70. A nurse needs to administer 250 mL of IV solution over 1 hr 40 min using gravity tubing with a drop factor of 15 gtt/mL. What flow rate should the nurse regulate in drops per minute?

- A.** 25 gtt/min
- B.** 38 gtt/min
- C.** 40 gtt/min
- D.** 63 gtt/min

71. A practical nurse is reviewing a newly admitted client's belongings and finds a bottle of oxycodone tablets prescribed by another provider. The client says, "I keep these with me in case my back starts hurting." What should the nurse do first?

- A.** Place the bottle in the client's bedside drawer so it is available if pain occurs
- B.** Send the medication home with the family without documenting it
- C.** Secure the medication according to facility policy and notify the charge nurse or provider
- D.** Administer one tablet if the client rates the pain greater than 6 out of 10

72. A nurse is caring for a client 12 hours after surgery who is receiving pain medication through a patient-controlled analgesia (PCA) pump. The client says, "I don't want to bother anyone, so I've been pressing the button every few minutes even when my pain is only mild." Which response by the nurse is best?

- A.** Keep pressing the button often so the medication can build up in your system.
- B.** Use the PCA when you start to feel pain increase, rather than pressing it repeatedly when your pain is already controlled.
- C.** Ask a family member to remind you to press the button on a schedule.
- D.** Wait until the pain becomes severe before using the PCA so the doses are not wasted.

73. A nurse is preparing to administer the scheduled morning dose of insulin glargine to a client with type 2 diabetes mellitus. Which pre-administration data is most important for the nurse to review?

- A.** The client's bedtime snack intake from the previous evening
- B.** The client's most recent blood glucose level
- C.** The client's bowel movement pattern for the last 24 hours
- D.** The client's daily fluid intake record

74. A client with depression started taking sertraline 5 weeks ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client reports improved sleep, appetite, and interest in daily activities
- B.** The client has a blood pressure of 108/68 mm Hg
- C.** The client reports dry mouth after each dose
- D.** The client is requesting the medication be taken only as needed

75. A client with heart failure is receiving furosemide 40 mg IV twice daily. Which new finding should the nurse identify as the most important adverse response to report to the provider?

- A.** Ringing in the ears after the morning dose
- B.** Increased urine output for 2 hours after administration
- C.** Mild thirst before lunch
- D.** Blood pressure of 138/84 mm Hg

76. A patient with chronic kidney disease takes spironolactone for heart failure. The provider prescribes lisinopril. Which finding after the new medication is started should the nurse recognize as the highest priority possible sign of a clinically significant drug interaction?

- A.** Serum potassium of 6.1 mEq/L
- B.** Dry cough at night

C. Blood pressure of 102/64 mm Hg

D. Mild nausea after breakfast

77. A nurse is preparing to administer an IV dose of phenytoin to a client who has a continuous infusion of 5% dextrose in water (D5W) running through a peripheral line. After reviewing the medication information, the nurse notes that phenytoin is incompatible with D5W. Which action is most appropriate?

A. Add the phenytoin directly to the D5W bag and infuse it slowly

B. Stop the D5W, flush the line with 0.9% sodium chloride, administer the phenytoin as prescribed, and flush again per protocol

C. Administer the phenytoin through the same line without interruption because the D5W is infusing at a low rate

D. Mix the phenytoin with sterile water in a syringe to reduce the incompatibility

78. A nurse is preparing to administer digoxin 62.5 mcg by mouth. The available oral solution is labeled 0.05 mg/mL. How many milliliters should the nurse administer for one dose?

A. 0.8 mL

B. 1.25 mL

C. 1.5 mL

D. 2 mL

79. A practical nurse is collecting data on four clients at the start of the shift. Which client should the nurse identify as being at greatest risk for an adverse reaction to iodinated contrast media before a scheduled CT scan?

A. A client who reports a shellfish allergy and had a normal creatinine level yesterday

B. A client who takes metformin and has a history of chronic kidney disease

C. A client who is anxious about needles and asks if the scan will hurt

D. A client who had nausea after drinking oral contrast 2 years ago

80. A nurse is reviewing arterial blood gas results for a client who has had repeated vomiting for 2 days. The results are pH 7.49, PaCO₂ 46 mm Hg, and HCO₃⁻ 34 mEq/L. How should the nurse interpret these findings?

A. Respiratory acidosis with no compensation

B. Metabolic alkalosis with partial respiratory compensation

C. Metabolic acidosis with full respiratory compensation

D. Respiratory alkalosis with partial metabolic compensation

81. A client who is 29 weeks pregnant comes to the prenatal clinic and says, "I feel burning when I urinate, and since last night I've had a fever and pain in my lower back on one side." Which complication should the nurse suspect first?

A. Round ligament pain

B. Pyelonephritis

C. Placenta previa

D. Hyperemesis gravidarum

82. A nurse performs a bedside urine dipstick test on a client with dysuria. The specimen appears cloudy and dark orange. The dipstick shows a positive leukocyte esterase result but a negative nitrite result. Which interpretation is most appropriate?

A. A urinary tract infection is ruled out because the nitrite result is negative

B. The specimen is invalid because cloudy urine cannot be tested with a dipstick

C. The findings may still be consistent with urinary tract irritation or infection and should be correlated with the client's symptoms

D. The positive leukocyte esterase result confirms contamination from vaginal secretions

83. A practical nurse is preparing to measure an adult client's blood pressure manually. The client has been sitting with legs crossed and talking on the phone. Which action should the nurse take to obtain the most accurate reading?

A. Measure the blood pressure immediately using the current position

B. Ask the client to uncross the legs, rest quietly, and then measure the blood pressure

C. Have the client stand up for the measurement to improve circulation

D. Place the cuff over the sleeve to avoid interrupting the client

84. A client is scheduled for pulmonary function testing this morning. Which client statement should the nurse follow up on before the test?

A. I used my albuterol inhaler about 20 minutes ago because I felt short of breath.

B. I wore a loose shirt so I can breathe comfortably during the test.

C. I brought my eyeglasses in case I need to read instructions.

D. I used the restroom before coming to the testing area.

85. A nurse is obtaining a 12-lead ECG for a client who has coarse chest hair. Several electrodes keep loosening, and the tracing is inconsistent. Which action should the nurse take?

- A. Apply electrodes over the hair and secure them with extra tape
- B. Shave or clip small areas of hair where the electrodes will be placed
- C. Moisten the electrodes with alcohol before placement
- D. Place the electrodes on the client's gown to avoid skin irritation

86. A nurse is collecting a venous blood specimen from an adult client. After puncturing the vein, swelling develops around the insertion site and the client reports tenderness. Which action should the nurse take first?

- A. Remove the tourniquet and continue collecting the specimen from the same site
- B. Withdraw the needle and apply direct pressure to the site
- C. Advance the needle slightly until blood flow resumes
- D. Flush the vein with normal saline to confirm patency

87. A practical nurse is preparing a client for surgery. Which preoperative finding should the nurse report to the RN immediately?

- A. The client removed nail polish from both hands
- B. The client states an allergy to latex
- C. The client voided 30 minutes ago
- D. The client is wearing a hospital gown without underwear

88. A client has a newly inserted subclavian central venous catheter and is receiving IV fluids. A few minutes later, the client becomes restless and reports sudden shortness of breath and sharp chest pain. Breath sounds are decreased on the affected side. Which complication should the nurse suspect first?

- A. Air embolism
- B. Pneumothorax
- C. Catheter occlusion
- D. Fluid overload

89. A nurse is caring for a client with preeclampsia who suddenly reports a severe frontal headache and says, "The lights are bothering my eyes." Which focused assessment should the nurse perform first?

- A. Assess for clonus and check deep tendon reflexes
- B. Ask the client about sleep quality during the previous night
- C. Measure fundal height
- D. Review the client's prenatal weight gain pattern

90. A client with acute pain from renal colic receives prescribed IV morphine 30 minutes ago. Which finding best indicates the treatment has been effective?

- A. Blood pressure decreases from 148/88 mm Hg to 138/84 mm Hg
- B. The client reports pain decreased from 9/10 to 3/10
- C. Urinary output increases from 25 mL/hr to 35 mL/hr
- D. The client appears sleepy and is difficult to arouse

91. A client 4 hours after a total knee replacement suddenly becomes restless and short of breath. The nurse notes a respiratory rate of 32/min, heart rate of 118/min, and oxygen saturation of 87% on room air. Which finding would most strongly support the nurse's suspicion of a pulmonary embolism?

- A. Crackles heard at both lung bases
- B. Productive cough with thick yellow sputum
- C. Unilateral calf swelling and tenderness
- D. Fever of 38.6°C (101.5°F)

92. A client with chronic peripheral arterial disease tells the practical nurse, "My legs feel better when I sit in the recliner with them up on the footrest." Which response by the practical nurse is best?

- A. "Keep your legs elevated above the level of your heart as much as possible."
- B. "Try lowering your legs to a dependent position when the pain occurs."
- C. "Massage your calves firmly whenever your legs start to ache."
- D. "Apply a heating pad directly to your lower legs for comfort."

93. A client with acute pancreatitis suddenly becomes confused and restless. Assessment findings include cool, mottled skin; respiratory rate 30/min; heart rate 136/min; blood pressure 82/48 mm Hg; and urine output of 15 mL over the last hour. Which complication should the nurse suspect?

- A. Hypovolemic shock
- B. Paralytic ileus
- C. Acute cholecystitis
- D. Hyperglycemia

94. A client has a chest tube connected to a water-seal drainage system after thoracic surgery. During the assessment, the nurse notices continuous bubbling in the water-seal chamber. The client is more short of breath than 1 hour ago. Which action should the nurse take first?

- A. Increase the wall suction to restore drainage
- B. Briefly clamp the chest tube near the insertion site to locate the leak
- C. Check the tubing and connections for an air leak

D. Empty the drainage collection chamber

95. A nurse is caring for a client after a prolonged surgical procedure. The client is pale and shivering. The room is cool, and the nurse notes the client's arms and legs feel cool to the touch while the trunk remains warmer. Which physiologic response is most likely causing this finding?

- A.** Peripheral vasoconstriction that reduces heat loss from the skin
- B.** Peripheral vasodilation that increases heat transfer to the environment
- C.** Decreased muscle activity that lowers metabolic heat production
- D.** Increased sweating that promotes evaporative cooling

96. A nurse is changing the dressing on a client's surgical wound. Which action is most appropriate to reduce the risk of transferring microorganisms from the wound back onto the surrounding skin?

- A.** Cleanse from the outer edges of the wound toward the center with each stroke
- B.** Use the same gauze pad for the entire wound to avoid disturbing tissue
- C.** Cleanse from the least contaminated area toward the most contaminated area
- D.** Scrub the wound bed briskly until all visible drainage is removed

97. A nurse arrives in a hospital parking lot and sees an adult lying motionless next to a car. Before approaching, the nurse notices the vehicle is still running and the area smells strongly of exhaust. Which action should the nurse take first?

- A.** Begin chest compressions immediately
- B.** Open the car door to turn off the engine, then check for a pulse
- C.** Move the client away from the vehicle and then assess responsiveness
- D.** Stay at a safe distance and activate emergency response for a hazardous scene

98. A client with bronchiectasis has copious, foul-smelling sputum and is scheduled for chest physiotherapy with postural drainage. The client asks why the nurse wants the treatment done before breakfast. Which response by the nurse is most appropriate?

- A.** It helps prevent nausea and vomiting during drainage positions.
- B.** It increases oxygen absorption from food after the treatment.
- C.** It reduces the risk of aspiration by drying airway secretions.

D. It allows the client to rest longer after meals without coughing.

99. A nurse is caring for a client who has a serum sodium level of 118 mEq/L. The client becomes restless, develops a headache, and is now confused. Which action should the nurse take first?

- A.** Restrict the client's oral fluid intake
- B.** Place the client on seizure precautions
- C.** Encourage foods high in sodium
- D.** Recheck the sodium level in 4 hours

100. A client on continuous ambulatory peritoneal dialysis tells the practical nurse, "I gained 3 pounds since yesterday, and my ankles look more swollen." Which finding would best support that the client is retaining too much fluid from the dialysis process?

- A.** The dialysate outflow is consistently less than the amount infused
- B.** The client reports feeling hungry after each exchange
- C.** The dialysate is pale yellow and clear
- D.** The abdomen feels full during the dwell period

Simulation Test 26 — Answer Key

1. A	26. C	51. A	76. A
2. C	27. A	52. A	77. B
3. B	28. B	53. B	78. B
4. C	29. B	54. B	79. B
5. A	30. C	55. B	80. B
6. B	31. C	56. B	81. B
7. A	32. A	57. A	82. C
8. C	33. A	58. C	83. B
9. B	34. A	59. B	84. A
10. C	35. B	60. B	85. B
11. B	36. B	61. B	86. B
12. A	37. B	62. A	87. B
13. A	38. D	63. B	88. B
14. B	39. B	64. A	89. A
15. B	40. C	65. A	90. B
16. C	41. B	66. B	91. C
17. B	42. B	67. B	92. B
18. C	43. C	68. B	93. A
19. C	44. C	69. A	94. C
20. B	45. B	70. B	95. A
21. C	46. C	71. C	96. C
22. A	47. C	72. B	97. D
23. C	48. C	73. B	98. A
24. B	49. C	74. A	99. B
25. C	50. C	75. A	100. A

Simulation Test 27

100 questions

1. A practical nurse in an assisted-living facility is preparing evening medications when four new situations are reported. Which client should the nurse assess first?

- A.** A client with Parkinson disease who says the next dose of carbidopa-levodopa is 20 minutes late
- B.** A client with heart failure who has gained 2 lb since yesterday and has mild ankle edema
- C.** A client with type 1 diabetes who is awake, diaphoretic, and trembling before dinner
- D.** A client with chronic arthritis who requests a PRN analgesic for hand pain rated 5/10

2. A practical nurse is coordinating care with an assistive personnel (AP) during a busy morning shift. Which task is most appropriate for the nurse to assign to the AP?

- A.** Teach a client how to use a walker before discharge
- B.** Check on a newly admitted client's response to pain medication
- C.** Perform oral care for a stable client who has no swallowing problems
- D.** Decide whether a client's surgical dressing needs to be changed

3. A nurse is caring for an adult client who is alert, oriented, and has decision-making capacity. The provider recommends a blood transfusion for symptomatic anemia. The client says, "I understand the reason for it, but I do not want blood for personal reasons." The client's adult child becomes upset and says, "You have to convince my parent to agree." Which action by the nurse is best?

- A.** Ask the adult child to sign the consent form because the client is making an unsafe decision
- B.** Tell the client that refusing the transfusion means the health care team may not be able to continue treatment
- C.** Support the client's decision, notify the provider of the refusal, and continue to monitor the client closely
- D.** Delay responding until the family has had more time to discuss the decision privately

4. A practical nurse is caring for a client in a semiprivate room. The client's coworker arrives and says, "I brought flowers and just need to know whether she is still having chest pain before I go." The client is away for testing and has not given any documented permission to share information. What is the nurse's best response?

- A.** Tell the coworker only that the client is stable but cannot discuss details

B. Ask the coworker to wait in the room until the client returns

C. Explain that no information can be shared without the client's permission

D. Give a brief update because the coworker already knows the reason for admission

5. A practical nurse finishes documenting care for a client on a workstation in the medication room. The nurse is called away urgently to assist with another client and notices that the electronic chart is still open on the screen. An unlicensed assistive personnel member is restocking supplies nearby. Which action should the nurse take first?

A. Ask the assistive personnel to watch the workstation until the nurse returns

B. Log off or secure the screen before leaving the workstation

C. Leave the chart open because the medication room is not open to visitors

D. Return later and close the chart after helping the other client

6. A client asks, "If I complete an advance directive, does that mean I will be labeled DNR and the staff will stop treating me?" Which response by the nurse is best?

A. Yes, an advance directive automatically means no resuscitation and no aggressive treatment

B. No, an advance directive lets you state your wishes for future care if you cannot speak for yourself

C. Yes, once it is signed, the health care team must withhold any life-sustaining treatment immediately

D. No, an advance directive is only used to decide who will pay for your care

7. An adult client is brought to the emergency department after a stroke and is unable to speak or follow commands. The chart contains no advance directive. The client's adult son says he is the health care proxy, but he does not have documentation with him. The client suddenly becomes apneic. Which action should the nurse take first?

A. Withhold emergency treatment until the son provides legal proof of authority

B. Ask the son to make an immediate decision about resuscitation

C. Initiate emergency care while the team clarifies the client's wishes and decision-maker

D. Delay action until the provider determines whether the client ever completed a living will

8. A client is brought to the emergency department with a ruptured ectopic pregnancy and severe internal bleeding. The client is unconscious, and no family members or legal surrogate can be reached. The surgeon says the client needs immediate surgery to prevent death. Which action by the nurse is most appropriate?

- A.** Delay the procedure until a family member can be contacted for consent
- B.** Ask two nurses to sign the consent form instead of the client
- C.** Prepare the client for surgery under the emergency exception to informed consent
- D.** Wait until the client regains consciousness to obtain informed consent personally

9. A nurse is preparing a cognitively intact adult client for a scheduled blood transfusion. The provider discussed the transfusion with the client earlier and asks the nurse to bring the consent form for signature. The client states, "I understand I need blood, but my faith does not allow transfusions." Which action should the nurse take first?

- A.** Explain that the transfusion is necessary and ask the client to sign anyway
- B.** Notify the provider that the client is refusing the transfusion
- C.** Ask the client's family to persuade the client to accept the transfusion
- D.** Request that another nurse witness the refusal so the transfusion can proceed

10. A client is being discharged after treatment for a deep vein thrombosis and has a new prescription for warfarin. During discharge teaching, the client says, "I usually go to a free community meal program, so I eat whatever vegetables they serve that day." Which response by the nurse is most appropriate?

- A.** Tell the client to avoid all green vegetables while taking warfarin
- B.** Instruct the client to keep vitamin K intake as consistent as possible and arrange follow-up monitoring
- C.** Advise the client to skip warfarin on days when green vegetables are served
- D.** Suggest replacing warfarin with aspirin if diet choices are unpredictable

11. A client is preparing for discharge after surgical repair of a fractured wrist. The client will need help with meal preparation, bathing, and transportation for the next 2 weeks. The adult child says, "I can help in the evenings, but no one will be there during the day." The client is alert, ambulates independently, and does not require skilled wound care. Which referral should the nurse anticipate as most appropriate?

- A.** Home health aide or personal care services

- B.** Hospice care
- C.** Inpatient rehabilitation facility
- D.** Long-term care placement

12. A rehabilitation unit wants to improve the accuracy of handoff communication during client transfers to another department. The team introduces a brief transfer form that includes diagnosis, allergies, current oxygen use, and fall risk. Which measure is the best process measure for this project?

- A.** The percentage of transfers in which the new form is completed before the client leaves the unit
- B.** The number of clients who return from the other department within 24 hours
- C.** The rate of transfer-related complications over the next 6 months
- D.** The percentage of clients who require readmission after discharge

13. A clinic is trying to reduce delays in starting appointments for clients who need interpreter services. Staff members believe the delay happens because interpreters are requested too late, but no data have been collected yet. The team can test only one action this week. Which action is best?

- A.** Require all staff to begin using a new interpreter-request protocol for every client immediately
- B.** Collect baseline data for several days on when interpreter need is identified, when the request is placed, and how long the client waits
- C.** Counsel staff members individually about the importance of culturally responsive care
- D.** Compare this clinic's delays with national benchmarks before examining the local process

14. A practical nurse is preparing a client with a new colostomy for discharge. During care, the client says, "My hands shake too much to cut the pouch right," and the spouse says, "I have trouble seeing the instructions." Which action best demonstrates effective interdisciplinary information sharing?

- A.** Document the comments in the chart and continue routine discharge teaching
- B.** Notify the charge nurse that discharge teaching was completed
- C.** Share the client's and spouse's functional concerns with the RN and request ostomy/wound care and case management follow-up before discharge
- D.** Tell the spouse to ask the pharmacist for larger-print written instructions after discharge

15. A practical nurse is caring for a client receiving IV antibiotics for cellulitis. At 1000, the nurse notes swelling and coolness at the IV site, stops the infusion, and removes the catheter. The next antibiotic dose is due at 1030, and the client says, "I don't want to be stuck again unless I really have to." Which communication best supports interdisciplinary information sharing?

- A.** Tell the next shift that the client lost IV access and may need another line later
- B.** Document the infiltration and wait to see whether the provider rounds before the next dose is due
- C.** Report the discontinued IV, the condition of the site, the scheduled antibiotic time, and the client's reluctance to the RN or appropriate licensed team member promptly so a plan can be made
- D.** Ask the unlicensed assistive personnel to encourage the client to accept a new IV site

16. A practical nurse is reinforcing discharge teaching with a client who has a new colostomy. The client asks, "Can you show my neighbor how to change the pouch? She'll be helping me at home." Which action by the nurse is most appropriate?

- A.** Teach the neighbor only after the client gives permission for the teaching to be shared
- B.** Refuse to teach the neighbor because home care tasks can be taught only to family members
- C.** Teach the neighbor because the client will need help after discharge
- D.** Ask the neighbor to return after discharge for teaching at the outpatient clinic

17. A practical nurse is caring for a client who speaks limited English and is scheduled for a provider to obtain informed consent for a procedure later that morning. The client's adult daughter says, "I can translate everything. We do this all the time." The unit has access to a qualified medical interpreter by video. Which action by the nurse is most appropriate?

- A.** Allow the daughter to interpret because she knows the client's usual preferences
- B.** Use the qualified medical interpreter to support communication during the consent discussion
- C.** Ask another bilingual staff member from housekeeping to explain the procedure
- D.** Have the daughter explain the procedure, then ask the client to sign if the client nods

18. A practical nurse in a community clinic is caring for a 7-year-old child with abdominal pain. During the exam, the nurse notices round bruises on the upper arms and a healing cut above the lip. The caregiver says, "He is just clumsy," but the child becomes silent and pulls away when the caregiver moves closer. What is the nurse's best action?

- A.** Ask the caregiver to sign a statement explaining the injuries before taking further action
- B.** Report the suspicion of possible abuse according to facility policy and state law
- C.** Wait until the child confirms abuse before notifying anyone
- D.** Discharge the child and schedule a follow-up visit in 1 week

19. A practical nurse in an outpatient wound clinic is caring for a 29-year-old client with repeated untreated injuries. The client appears fearful, avoids eye contact, and says privately, "My partner did this, but please do not call anyone. I am afraid it will get worse." Which action by the nurse is most appropriate?

- A.** Respect the client's request and omit the concern from the record because the client is an adult
- B.** Immediately call the partner to verify what happened before documenting findings
- C.** Document objective findings, ensure privacy and safety, and follow facility policy and applicable reporting requirements
- D.** Tell the client that leaving the relationship is required before any help can be offered

20. A practical nurse is coordinating care with an assistive personnel (AP) on a busy clinic day. Which delegated task requires the nurse to give the AP the clearest report-back instructions?

- A.** Refill the water pitcher for a client waiting for transportation home
- B.** Help a stable client to the bathroom before discharge
- C.** Measure and record the urine output of a client taking a diuretic
- D.** Transport a client in a wheelchair to the x-ray department

21. A practical nurse in a primary care clinic is reinforcing follow-up instructions for a client with a new diagnosis of colon cancer. The client says, "I am a member of a Tribal community, and I usually speak with our traditional healer before I make major health decisions. The surgery office told me to decide by tomorrow." Which action by the nurse best promotes equitable access to care?

- A.** Tell the client that delaying the decision could interfere with treatment and that the surgical plan should be followed immediately

B. Ask whether the client would like help coordinating a discussion that includes the provider and the traditional healer, if available

C. Explain that traditional healing practices should be kept separate from medical treatment to avoid confusion

D. Document that the client is refusing prompt treatment and notify the provider that the client is noncompliant

22. A nurse arrives to administer a prescribed oral antibiotic to a client on a medical unit. The client is alert and oriented and says, "I'm Carla Benson, and I take this pill every evening." The medication administration record lists Carla Benson, date of birth 04/22/1968. What is the best action by the nurse to verify the client's identity before giving the medication?

A. Ask the client to state her full name and date of birth, then compare them with the medication administration record and identification band

B. Confirm the client's identity by asking what medication she usually takes at home

C. Use the client's room number and the fact that she recognizes the medication as the identifiers

D. Accept the client's stated name because she is alert and oriented

23. A nurse is preparing to use sequential compression devices (SCDs) for a postoperative client. After applying the sleeves, the nurse notices that the tubing from one leg sleeve is kinked sharply under the side rail and that sleeve is not inflating. The client is resting comfortably. Which action should the nurse take?

A. Leave the device in place and recheck it at the next routine round

B. Remove both sleeves and document that the client refused treatment

C. Straighten the tubing, confirm both sleeves inflate properly, and continue use

D. Increase the machine pressure setting until the affected sleeve inflates

24. A nurse is preparing to administer a prescribed dose of codeine to a client whose chart lists "allergy: morphine." When asked about the reaction, the client states, "Morphine made me very nauseated and sleepy, but I did not have a rash or trouble breathing." Which action by the nurse is most appropriate?

A. Hold the codeine and document a severe opioid allergy

B. Administer the codeine because nausea and sleepiness are expected side effects, but continue to monitor the client

C. Ask the provider to prescribe morphine instead because it is already listed in the allergy record

D. Remove the morphine allergy from the chart before giving the codeine

25. A practical nurse is completing a home safety visit for an older adult who uses oxygen by nasal cannula. Which finding should the nurse address first?

A. A small rug is loose near the bathroom doorway

B. A candle and matches are on the bedside table next to the oxygen tubing

C. The client keeps the thermostat set at 78°F (25.6°C)

D. Several magazines are stacked on a chair in the corner of the room

26. A client admitted with pneumonia becomes increasingly restless at night and repeatedly pulls off the pulse oximeter probe, saying, "This thing bothers me." The client is not violent, answers simple questions appropriately, and keeps the oxygen cannula in place when the nurse explains its purpose. Which action by the nurse best follows least-restrictive restraint principles?

A. Apply soft wrist restraints so the pulse oximeter can stay in place continuously

B. Cover the client's hands with mitt restraints to prevent removal of monitoring equipment

C. Assess for discomfort, reposition the probe or move it to another site, and re-explain the need for monitoring

D. Ask the provider for a sedative so the client will tolerate the equipment better

27. A nurse preparing to insert an indwelling urinary catheter opens the kit and notices that the sterile drape has a small tear in the package seal. The catheterization has not started, and the client has not been touched. Which action is most appropriate?

A. Use the kit because the inner supplies still appear clean

B. Replace the kit with a new sterile kit and report the near miss according to facility policy

C. Continue with the procedure after cleaning the outside of the package

D. Ask another nurse to inspect the kit and proceed if they agree it is usable

28. A nurse disconnects a client's intermittent enteral feeding tubing and notices that the tubing was labeled with a time from the previous day, indicating it remained in use longer than allowed by facility policy. The client received formula through the tubing but has no current signs of distress. Which action is most appropriate?

A. Replace the tubing, assess the client, and complete required incident reporting per facility policy

B. Discard the tubing and continue care because no harm is currently apparent

C. Document only that the tubing was changed and monitor at the next routine assessment

D. Flush the tubing thoroughly and relabel it with the current date and time

29. A nurse is preparing to transfer a client from the bed to a wheelchair. The client is alert, follows directions, and can bear weight, but says, "My left leg is very weak today." Which placement of the wheelchair is safest for the transfer?

- A.** Place the wheelchair on the client's left side at a 90-degree angle to the bed
- B.** Place the wheelchair on the client's right side at a 45-degree angle to the bed
- C.** Place the wheelchair at the foot of the bed facing the client
- D.** Place the wheelchair parallel to the bed on the client's left side

30. A practical nurse is preparing to suction a client with a tracheostomy using sterile technique. After opening the sterile catheter kit, the nurse puts on sterile gloves. While connecting the suction tubing, the nurse accidentally brushes the sterile catheter against the bed sheet. Which action should the nurse take?

- A.** Continue the procedure because the catheter has not entered the client's airway yet
- B.** Rinse the catheter with sterile saline and use it for suctioning
- C.** Discard the contaminated catheter and obtain a new sterile suction catheter
- D.** Ask an assistant to hold the catheter while the nurse completes the setup

31. A nurse is preparing to irrigate an indwelling urinary catheter of a client who is not on isolation precautions. The nurse expects urine to drain during the procedure and understands that fluid could splash while disconnecting the tubing. Which personal protective equipment is most appropriate?

- A.** Clean gloves only
- B.** Clean gloves and a gown
- C.** Clean gloves, a gown, and face protection
- D.** A surgical mask only

32. A practical nurse is opening a sterile dressing kit on an overbed table. After placing the sterile drape, the nurse notices that the client's bedside fan is blowing directly across the field. Which action is best?

- A.** Proceed if no one has touched the sterile field
- B.** Turn off or redirect the fan and set up a new sterile field
- C.** Cover the field with a clean towel until the procedure begins
- D.** Move only the sterile instruments to another area of the table

33. A practical nurse is caring for a client newly diagnosed with active pulmonary tuberculosis who is placed in an airborne infection isolation room. Which action by the nurse requires intervention?

- A.** Keeping the room door closed except when entering and leaving
- B.** Wearing a fit-tested N95 respirator before entering the room
- C.** Removing the respirator immediately after leaving the client's room
- D.** Transporting the client to physical therapy without a mask if the trip is brief

34. A practical nurse is preparing to draw blood from a client who has no isolation orders. The client becomes anxious and says, "You don't need gloves. I know I'm not sick." Which response by the nurse best reflects standard precautions?

- A.** Gloves are only needed when a client is known to have a bloodborne infection.
- B.** Gloves are worn because blood and certain body fluids are treated as potentially infectious for every client.
- C.** Gloves are required only because venipuncture is an invasive procedure.
- D.** Gloves are mainly used to keep the client from being exposed to germs on the nurse's hands.

35. A nurse is reinforcing teaching with a client who is 11 weeks pregnant about measures to promote comfort and circulation during pregnancy. Which instruction should the nurse include?

- A.** Wear knee-high support stockings before getting out of bed in the morning
- B.** Apply a heating pad to the calves for 30 minutes at bedtime
- C.** Sit with the legs crossed at the knees whenever possible
- D.** Limit walking to prevent strain on the leg veins

36. A client in active labor has a spontaneous rupture of membranes. The nurse notes thick green amniotic fluid and a fetal heart rate of 144/min with moderate variability. Which action is most appropriate next?

- A.** Document the finding as expected near term and continue routine care
- B.** Prepare for immediate vaginal delivery
- C.** Notify the provider and continue close fetal monitoring
- D.** Place the client supine to improve uteroplacental perfusion

37. A practical nurse is reinforcing teaching with a 41-year-old client who says, "No one in my family has diabetes, so I do not need to be screened for it." Which response by the nurse is best?

- A.** Screening is only needed when a person has symptoms of high blood sugar.

- B.** A family history is just one risk factor, so screening recommendations can still apply even without it.
- C.** People without a family history are very unlikely to develop diabetes.
- D.** If your blood pressure is normal, diabetes screening is not necessary.

38. A nurse is obtaining a health history from an adult client who says, "I use an inhaler sometimes when my breathing acts up." Which follow-up question is best to clarify the client's current medication use?

- A.** What do you think usually causes your breathing to act up?
- B.** How long have you had trouble with your breathing?
- C.** What is the name of the inhaler, how often do you use it, and when was your last dose?
- D.** Does anyone in your family have asthma or other lung disease?

39. A practical nurse is admitting an adult client with long-standing psoriasis. Which assessment finding is most important to document in detail as baseline integumentary data for comparison during the client's stay?

- A.** The client states that skin itching is sometimes worse at night
- B.** Several well-demarcated, scaly plaques are present on both elbows and knees, with no drainage noted
- C.** The client reports using an over-the-counter moisturizing cream at home
- D.** The client says psoriasis flare-ups tend to occur during times of stress

40. A nurse is reinforcing teaching for the parent of a toddler who will receive routine vaccines at today's clinic visit. The parent says, "My child had a temperature of 100.1°F (37.8°C) last night and is taking amoxicillin for an ear infection, so I think the vaccines should wait." Which response by the nurse is best?

- A.** Vaccines should be delayed until the antibiotic course is completed.
- B.** Routine vaccines are usually still given when a child has a mild illness and a low-grade fever.
- C.** Only live vaccines can be given when a child is taking antibiotics.
- D.** Vaccines should be postponed because a recent fever increases the risk for an allergic reaction.

41. A nurse is reinforcing teaching with the parent of a 3-year-old child about expected language development. Which statement by the parent indicates understanding?

- A.** My child should be able to speak in short sentences of about 3 to 4 words.

- B.** My child should mostly communicate by pointing to objects and grunting.
- C.** My child should be able to read simple picture books independently.
- D.** My child should use about 2 or 3 words total during the day.

42. A nurse is discussing a new antihypertensive medication with a client. The client says very little, keeps folded arms across the chest, and frequently glances at a family member before answering. The family member then says, "In our family, I usually speak for her." Which action by the nurse is best to reduce this communication barrier?

- A.** Continue the teaching with the family member and ask the client to interrupt if anything is incorrect
- B.** Ask the family member to leave so the nurse can speak with the client alone
- C.** Acknowledge the family member's support and ask the client how she prefers to receive information and answer questions
- D.** Speak more slowly and loudly so the client can better understand the instructions

43. A nurse is preparing to teach a client who was newly prescribed iron tablets for anemia. The client says, "I never finished school, and forms like this are hard for me to understand." Which action by the nurse is best?

- A.** Ask the client's family member to review the written instructions at home
- B.** Provide a detailed pamphlet with medical terms so the client can study it later
- C.** Use plain language, review one point at a time, and ask the client to explain the instructions back
- D.** Speak more loudly and repeat the same instructions several times

44. A client with newly diagnosed epilepsy tells the nurse, "Since the seizures started, I keep canceling outings with friends because I'm embarrassed and worried something might happen in public." Which response by the nurse best promotes effective coping?

- A.** You should avoid social situations until your treatment is fully established.
- B.** Try not to think about the seizures so you do not make yourself more anxious.
- C.** Let's talk about specific situations that worry you and plan ways to stay involved safely.
- D.** Your friends probably understand, so there is no reason to feel upset.

- 45.** A nurse in an inpatient mental health unit is assessing a client who has become increasingly angry after a telephone call. Which behavior is the strongest indicator that the client is at risk for imminent violence?
- A.** The client asks to be left alone for a few minutes
 - B.** The client paces in the hallway and avoids conversation
 - C.** The client suddenly becomes quiet, fixes attention on one staff member, and clenches both fists
 - D.** The client states, "I'm upset," and begins to cry
- 46.** A client admitted for treatment of pyelonephritis tells the nurse, "I take diazepam every day, but I haven't had any for 3 days because I ran out." Which assessment finding is most important for the nurse to report immediately?
- A.** Decreased appetite and fatigue
 - B.** Fine hand tremors and increasing anxiety
 - C.** New onset confusion with a generalized seizure
 - D.** Headache and sensitivity to light
- 47.** A client with chronic schizophrenia is shouting in the hallway, "You're all trying to control me!" The client is pacing quickly, but stays several feet away from others and has not made physical threats. Which nursing response is most appropriate?
- A.** "If you do not calm down right now, you will lose your privileges."
 - B.** "Come with me to a quieter area so we can talk about what is upsetting you."
 - C.** "There is nothing to be upset about, so you need to stop yelling."
 - D.** "Why do you think the staff are trying to control you?"
- 48.** A client who was told today that infertility treatment was unsuccessful says to the nurse, "Everyone keeps telling me to relax, but they don't get it. Maybe this is never going to happen for us." Which response by the nurse is most therapeutic?
- A.** You should try not to think so negatively.
 - B.** Many couples are able to conceive after several treatment cycles.
 - C.** It sounds like you are feeling discouraged and overwhelmed right now.
 - D.** At least you know you tried everything you could.
- 49.** A nurse is caring for a client who becomes increasingly agitated after receiving bad news from a family member. The client is sitting in a chair with clenched fists, speaking loudly, and saying, "Just stop. I can't deal with anyone right now." Which action by the nurse is most appropriate?
- A.** Remain several feet away, speak calmly, and offer the client a quiet space to talk when ready
 - B.** Move closer to the client and place a hand on the client's shoulder for reassurance
 - C.** Tell the client that yelling is unacceptable and will lead to consequences
 - D.** Ask the client to explain exactly what the family member said so the nurse can assess the situation
- 50.** A resident with moderate dementia becomes upset while trying to choose clothes for the day. The resident opens several drawers, drops clothing on the bed, and says, "There are too many things. I can't do this." Which action by the practical nurse is most appropriate?
- A.** Lay out one complete outfit in the order it should be put on
 - B.** Ask the resident which clothes were usually worn to work years ago
 - C.** Tell the resident to sit quietly until feeling less frustrated
 - D.** Place all available clothing on the bed so the resident can compare options
- 51.** A client tells the nurse, "My stress gets worse because I never know what actually sets me off. It just seems to hit me out of nowhere." Which nursing recommendation is most appropriate?
- A.** Keep a brief daily log of stressful events, physical sensations, and reactions to identify patterns
 - B.** Avoid thinking about stressful situations until they have completely passed
 - C.** Wait to address stress until symptoms become severe enough to notice clearly
 - D.** Focus on solving other people's problems first to stay distracted from personal stress
- 52.** A client who has just been told that chemotherapy is no longer effective says to the practical nurse, "I do not want to talk to a chaplain. I just need someone to sit here quietly while I make sense of this." Which response by the nurse is most appropriate?
- A.** "You should speak with the chaplain because this is exactly what chaplains are trained to handle."
 - B.** "Of course. I can sit with you for a while, and if you want to talk later, I will listen."
 - C.** "Try to stay positive. Many clients find that negative thoughts make things harder."
 - D.** "Let me ask your family to come now so you will not have to be alone."

53. A nurse is reinforcing preoperative teaching for a client scheduled for surgery later that morning. When the nurse explains that the client will need to remove a small necklace before going to the operating room, the client says, "This necklace is a sacred item, and I am afraid something bad will happen if it is taken off." Which response by the nurse is most appropriate?

- A.** "You must remove it now because hospital policy does not allow exceptions."
- B.** "Tell me more about what the necklace means to you so we can look for a safe way to respect your beliefs."
- C.** "You should give the necklace to a family member so you will not have to think about it anymore."
- D.** "Most clients feel better once surgery starts, so this should not worry you for long."

54. A nurse is caring for a client admitted for dehydration. While the nurse is in the room, an assistive personnel member laughs and says, "I never know what to call this client anyway." The client looks away and becomes silent. Which action by the nurse is best?

- A.** Tell the client that the staff member probably did not mean any harm and continue the assessment
- B.** Ask the client privately later whether they want to file a complaint about the staff member
- C.** Address the staff member immediately, stop the disrespectful behavior, and then refocus on the client's preferred name and pronouns
- D.** Report the incident to the charge nurse at the end of the shift without discussing it in front of the client

55. A client who recently lost the ability to read printed text because of worsening diabetic retinopathy says, "My family keeps leaving me magazines and crossword books like before. I just stack them in the drawer so nobody sees I can't use them." Which nursing response is best?

- A.** "You should ask your family to stop bringing those items until your vision improves."
- B.** "It sounds upsetting to hide this change. Let's talk about other ways you can still enjoy reading and stay involved with your family."
- C.** "You need to accept that some hobbies are no longer possible and focus on safer activities."
- D.** "Your family is only trying to help, so it is best not to mention that the magazines bother you."

56. A nurse is caring for a client with a mature gastrostomy tube who receives intermittent enteral feedings. Before giving the prescribed formula, the nurse notes that the client reports feeling full and nauseated, and 180 mL of undigested formula is aspirated from the tube. Which action is most appropriate?

- A.** Administer the feeding as prescribed because the tube is in place

- B.** Discard the aspirated contents and give the feeding more slowly

- C.** Hold the feeding and reassess the client according to facility policy

- D.** Flush the tube with carbonated beverage and restart the feeding

57. A nurse is reinforcing teaching for a client who has weakness in the left leg and is learning to use a cane for walking on level ground. Which client action indicates correct understanding?

- A.** Holds the cane in the left hand and moves the cane forward with the left leg

- B.** Holds the cane in the right hand and moves the cane forward with the left leg

- C.** Holds the cane in the right hand and moves the right leg forward first, then the cane

- D.** Holds the cane in the left hand and advances the stronger right leg with the cane

58. A nurse is reinforcing dietary teaching for a client with chronic constipation. Which food choice should the nurse recommend to help promote regular bowel movements?

- A.** White toast with jelly

- B.** Baked chicken with mashed potatoes

- C.** Oatmeal topped with fresh pears

- D.** Cheese and crackers

59. A client with cellulitis of the left lower leg reports aching pain and says, "It hurts less when I keep my leg up." Which nonpharmacologic intervention should the nurse implement?

- A.** Place the leg in a dependent position while the client is sitting

- B.** Elevate the affected leg on pillows above heart level

- C.** Massage the reddened area of the lower leg

- D.** Encourage the client to walk frequently to distract from the pain

60. A nurse is caring for a client with mild dementia who can wash and comb their hair independently after setup but becomes frustrated when given several directions at once. Which nursing action best promotes the client's ability to perform activities of daily living?

- A.** Complete the task for the client to avoid agitation

- B.** Give one simple step at a time and allow extra time for completion

- C.** Provide all instructions at once before the client begins

- D.** Ask a family member to take over the client's morning care each day

61. A nurse is caring for a client who has a reflex bowel after a spinal cord injury. The client is on a bowel program that includes a bisacodyl suppository. Which action should the nurse take to promote the most effective bowel evacuation?

- A.** Insert the suppository and keep the client lying flat for 30 minutes
- B.** Place the suppository against the stool in the rectal vault
- C.** Insert the suppository so it contacts the rectal mucosa
- D.** Lubricate the suppository with mineral oil before insertion

62. A practical nurse is caring for a client who is thin, weak, and unable to reposition independently. During rounds, the nurse finds the client lying directly on the right hip with the knees and ankles rubbing together. Which action is best to help protect the client's skin and tissue integrity?

- A.** Massage the client's hip and knees to increase circulation
- B.** Place pillows between the knees and ankles and reposition the client slightly off the hip
- C.** Raise the head of the bed to high-Fowler's position
- D.** Apply powder to the legs and keep the client in the same position to promote rest

63. A practical nurse is caring for a client with decreased sensation after a recent stroke. While assisting with a shower, which action is most important to help prevent injury?

- A.** Ask the client to test the water temperature with the affected arm first
- B.** Use a bath thermometer or test the water with an unaffected body area before the client enters the shower
- C.** Keep the water as hot as the client usually prefers to promote comfort
- D.** Encourage the client to stand under running water to judge the temperature gradually

64. A practical nurse is calculating a client's intake for a shift. The client drank 120 mL of coffee, received 500 mL of IV fluids, ate 1 cup of ice chips, and received 30 mL of liquid medication by mouth. How much total intake should the nurse document?

- A.** 770 mL
- B.** 890 mL
- C.** 900 mL
- D.** 1,010 mL

65. A practical nurse is caring for a client who has been immobile in bed for 5 days. Which finding most strongly suggests atelectasis related to immobility?

- A.** Diminished breath sounds at the lung bases with a low-grade fever

- B.** Bilateral dependent ankle edema at the end of the day
- C.** Urine specific gravity of 1.030
- D.** A reddened area on the heel that blanches with pressure

66. A nurse is preparing to administer a scheduled subcutaneous dose of enoxaparin to a client. After verifying the prescription, checking the MAR, and identifying the client with two identifiers, the nurse notices bruising and firmness at the intended abdominal injection site. Which action is most appropriate?

- A.** Administer the medication into the bruised area because abdominal tissue is the preferred site
- B.** Withhold the medication and document that the client refused the dose
- C.** Select a different appropriate injection site and administer the medication as prescribed
- D.** Massage the bruised area before and after the injection to improve absorption

67. A practical nurse is preparing a client's 1400 medications and notices the MAR shows amoxicillin 500 mg PO due at 1400. The same antibiotic is also listed on the MAR as given at 1300 by the previous nurse. The prescription is for amoxicillin 500 mg PO every 8 hours. The client is stable and says, "I already took a capsule not long ago." What is the nurse's best action?

- A.** Administer the 1400 dose because it is listed as due on the MAR
- B.** Hold the 1400 dose and clarify the discrepancy before administering
- C.** Ask the client whether they want to take the medication now or later
- D.** Document the 1400 dose as refused and move on to the next medication

68. A nurse is caring for a patient who has a painful mouth ulcer and a prescription for a medication intended to treat the area directly with minimal systemic absorption. Which route of administration should the nurse use?

- A.** Topical oral mucosal route
- B.** Sublingual route
- C.** Enteral route
- D.** Intramuscular route

69. A nurse is preparing to administer packed red blood cells to a client with symptomatic anemia. The blood is available from the blood bank, but the client has a 20-gauge IV infusing lactated Ringer's solution and a prescribed IV antibiotic that is due now. Which action should the nurse take?

- A.** Y-site the packed red blood cells with lactated Ringer's solution and give the antibiotic after the transfusion starts

- B.** Stop the lactated Ringer's solution, flush the IV, and infuse the packed red blood cells through new tubing primed with 0.9% sodium chloride
- C.** Mix the prescribed antibiotic into the blood tubing so both can infuse through the same line
- D.** Delay the transfusion until a central line can be inserted for blood administration

70. An IV infusion contains 500 mL of 0.9% sodium chloride and is ordered to infuse over 4 hours using tubing with a drop factor of 15 gtt/mL. What is the correct drip rate in drops per minute?

- A.** 21 gtt/min
- B.** 28 gtt/min
- C.** 31 gtt/min
- D.** 36 gtt/min

71. A practical nurse is reinforcing teaching for a hospitalized client who is prescribed an extended-release controlled opioid for chronic pain. Which client statement indicates correct understanding of safe use of this medication?

- A.** I should crush the tablet if it feels too large to swallow.
- B.** I can take an extra dose whenever my pain suddenly gets worse.
- C.** I should swallow the tablet whole and take it only as prescribed.
- D.** I should stop taking the medication as soon as I feel better that day.

72. A nurse is checking a postoperative client's patient-controlled analgesia (PCA) pump at the end of the shift. The total volume remaining is much lower than expected, and the client says, "My sister pressed the button a few times while I was asleep because I looked uncomfortable." Which action by the nurse is most appropriate?

- A.** Tell the sister that pressing the button helped prevent the client's pain from getting worse
- B.** Instruct the sister that only the client should press the PCA button and notify the charge nurse or provider per facility policy
- C.** Turn off the PCA pump and withhold all opioid medication until the next shift
- D.** Increase the PCA dose because the client likely needs more medication for adequate pain control

73. A nurse is preparing to administer the scheduled dose of sublingual nitroglycerin to a client who reports chest pressure. Which pre-administration information is most important for the nurse to review?

- A.** The client's blood pressure
- B.** The client's last bowel movement
- C.** The client's daily fluid intake

D. The client's serum calcium level

74. A client with hypothyroidism started levothyroxine 6 weeks ago. Which finding best indicates the medication is producing the intended therapeutic response?

- A.** The client reports improved energy and less sensitivity to cold
- B.** The client reports increased appetite with a 4-lb weight gain
- C.** The client has a heart rate of 112/min
- D.** The client has developed a fine hand tremor

75. A client with bipolar disorder began lithium therapy 1 week ago. Which new finding should the nurse report to the provider immediately as the most concerning possible adverse response to the medication?

- A.** Mild thirst and increased urination
- B.** Fine hand tremor
- C.** Slurred speech and unsteady gait
- D.** Occasional nausea after meals

76. A patient takes simvastatin each evening for hyperlipidemia. During a clinic visit, the patient says, "I've also been drinking a large glass of grapefruit juice every morning because I heard it's healthy." Which response by the nurse is best?

- A.** Continue both because grapefruit juice does not affect cholesterol medications
- B.** Take the simvastatin with the grapefruit juice to improve absorption
- C.** Avoid grapefruit juice because it can raise simvastatin levels and increase the risk of muscle injury
- D.** Switch to drinking orange juice only at night so it will not interact with simvastatin

77. A client with heart failure is prescribed digoxin 0.125 mg by mouth daily. Before administering the morning dose, the nurse reviews the laboratory results and notes a serum potassium level of 2.8 mEq/L. Which action is most appropriate?

- A.** Administer the digoxin as prescribed and recheck the potassium level later
- B.** Hold the digoxin and notify the provider of the potassium result
- C.** Give the digoxin with food to reduce gastrointestinal adverse effects
- D.** Document the finding as expected for digoxin therapy

78. A nurse is preparing to administer furosemide 30 mg by mouth. The available oral solution is labeled 10 mg/mL. How many milliliters should the nurse administer for one dose?

- A.** 2 mL
- B.** 2.5 mL

C. 3 mL

D. 5 mL

79. A practical nurse is collecting data on four clients in an outpatient setting. Which client should the nurse identify as being at greatest risk for developing deep vein thrombosis?

A. A client 2 days after knee arthroscopy who reports walking in the hallway several times a day

B. A client taking an oral antibiotic for sinusitis who reports mild nausea after meals

C. A client who returned yesterday from a 12-hour international flight and takes oral contraceptives

D. A client with seasonal allergies who reports nasal congestion and watery eyes

80. A nurse is preparing to collect a stool specimen for fecal occult blood testing from a hospitalized client. Which action should the nurse take to help ensure the specimen is valid?

A. Collect the specimen from stool mixed with urine in the toilet hat

B. Obtain the specimen from stool that is free of toilet paper and urine

C. Use a specimen taken from the toilet bowl after the client flushes

D. Add tap water to the stool sample if it appears dry

81. A client who is 20 weeks pregnant calls the prenatal clinic and says, "I've had severe nausea and vomiting for 2 days, and today I feel dizzy when I stand up. I can barely keep any fluids down." Which instruction should the nurse give first?

A. Come to the clinic later this week for a weight check

B. Sip ginger ale and eat dry crackers, then call back tomorrow

C. Go to the emergency department or provider now for evaluation

D. Lie down for 30 minutes and then try to drink a large glass of water

82. A nurse is preparing to perform a bedside fecal occult blood test. Which client statement should cause the nurse to postpone testing and notify the provider that the result may be unreliable?

A. I ate a turkey sandwich for lunch today.

B. I used a laxative suppository this morning because I was constipated.

C. I drank two cups of coffee before breakfast.

D. I had plain yogurt with my medications.

83. A practical nurse is obtaining morning vital signs on an adult client. The radial pulse feels weak and uneven. Which action should the nurse take to obtain the most accurate heart rate?

A. Count the radial pulse for 15 seconds and multiply by 4

B. Count the apical pulse for 1 full minute

C. Use the pulse oximeter reading as the heart rate

D. Ask the client to take several deep breaths, then recount the radial pulse

84. A client is scheduled for a percutaneous endoscopic gastrostomy (PEG) tube placement with moderate sedation. Which finding is most important for the nurse to report to the provider before the procedure?

A. The client has not had anything to eat since midnight

B. The client takes warfarin for atrial fibrillation

C. The client reports mild anxiety about the procedure

D. The client voided 1 hour before transport

85. A nurse is preparing a client for continuous ECG monitoring. The client has oily skin and dried lotion on the chest. Which action should the nurse take to help obtain the clearest tracing?

A. Apply the electrodes directly over the lotion to avoid skin irritation

B. Clean and dry the skin before attaching the electrodes

C. Place the electrodes over bony areas so they stay in place better

D. Use extra electrode gel without removing the oils on the skin

86. A nurse is preparing to obtain a venous blood specimen from an adult client. After cleansing the site with antiseptic, the nurse palpates the vein again with an ungloved finger just before inserting the needle. Which action should the nurse take next?

A. Insert the needle because the site was already cleaned

B. Cleanse the site again with antiseptic and allow it to dry

C. Apply the tourniquet more tightly before inserting the needle

D. Ask the client to make a fist repeatedly while the needle is inserted

87. A practical nurse is completing the preoperative checklist for a client scheduled for cataract surgery. Which finding should the nurse report to the RN immediately before the client is taken to the operating room?

A. The client drank 8 oz of orange juice 2 hours ago

B. The client removed dentures and hearing aids

C. The client voided 30 minutes ago

D. The client is wearing a cotton hospital gown

88. A client receiving enteral feedings through a small-bore nasogastric tube suddenly begins coughing during the feeding and has an oxygen saturation drop from 96% to 90%. Which action should the nurse take first?

- A.** Slow the feeding rate and continue to monitor the client
- B.** Stop the feeding and check tube placement
- C.** Irrigate the tube with 30 mL of water
- D.** Place the client flat to reduce abdominal cramping

89. A nurse is caring for a client 4 hours after a subtotal gastrectomy. The client becomes restless and says, "I feel full in my stomach, and something is wrong." Which focused assessment should the nurse perform first?

- A.** Inspect the abdominal dressing for increased drainage
- B.** Check the client's bowel sounds in all four quadrants
- C.** Palpate the calves for warmth and tenderness
- D.** Ask the client to rate incisional pain on a 0 to 10 scale

90. A client with pernicious anemia began weekly intramuscular vitamin B12 injections 1 month ago. Which finding best indicates the treatment is effective?

- A.** The client reports less fatigue and has a rising hemoglobin level
- B.** The client has increased appetite and a 2-lb weight gain
- C.** The client reports mild soreness at the injection site after each dose
- D.** The client has pale conjunctiva but denies shortness of breath

91. A client 24 hours after a hysterectomy suddenly becomes very short of breath and coughs up pink, frothy sputum. The nurse notes crackles throughout both lungs, severe anxiety, heart rate 132/min, respiratory rate 34/min, and oxygen saturation 82% on room air. Which action should the nurse take first?

- A.** Place the client in high-Fowler position and apply oxygen
- B.** Obtain a prescription for IV furosemide
- C.** Auscultate bowel sounds and inspect the abdominal incision
- D.** Encourage the client to cough and use the incentive spirometer

92. A client with type 2 diabetes mellitus is preparing for discharge. Which statement by the client shows correct understanding of how to treat mild hypoglycemia at home?

- A.** I should drink a glass of water and lie down until the shaky feeling passes.
- B.** I should eat a candy bar and recheck my blood sugar in 2 hours.
- C.** I should take 15 grams of fast-acting carbohydrate, then recheck my blood sugar in 15 minutes.

D. I should give myself the next dose of insulin early because low blood sugar means my diabetes is getting worse.

93. A client 3 days after a long-bone fracture suddenly becomes restless and confused. The nurse notes a temperature of 38.5°C (101.3°F), heart rate of 118/min, respiratory rate of 30/min, oxygen saturation of 88% on room air, and a fine petechial rash over the upper chest. Which complication should the nurse suspect?

- A.** Fat embolism syndrome
- B.** Neurogenic shock
- C.** Pulmonary edema
- D.** Acute pericarditis

94. A client is postoperative after biliary surgery and has a T-tube draining bile by gravity. Before assisting the client to a chair, which action is most appropriate?

- A.** Clamp the T-tube for 30 minutes to prevent bile loss during movement
- B.** Keep the drainage bag below the level of the abdomen during transfer
- C.** Irrigate the T-tube with 30 mL of sterile saline to maintain patency
- D.** Place the drainage bag on the bed beside the client to avoid pulling on the tube

95. A nurse is caring for a client who has a high fever. The nurse encourages the client to drink cool fluids and notes beads of sweat forming on the skin. Which statement best explains how sweating helps lower body temperature?

- A.** Sweat removes heat as it evaporates from the skin surface.
- B.** Sweat warms the skin so heat moves inward toward the core.
- C.** Sweat decreases blood flow to the skin to prevent further heat gain.
- D.** Sweat causes muscles to contract rapidly to generate cooling.

96. A nurse is reinforcing home care instructions for a client with a healing surgical incision. Which client statement indicates a need for further teaching?

- A.** I will wash my hands before I touch the dressing area.
- B.** I will look at the incision every day for redness or swelling.
- C.** I will clean the incision with hydrogen peroxide each day to keep it sterile.
- D.** I will contact the provider if the wound starts to open.

97. A nurse is the first responder to an adult client who suddenly becomes unresponsive in a day room. The scene is safe. The nurse checks for responsiveness, calls for help, and finds no normal breathing and no pulse within 10 seconds. Chest compressions are started. Which action is most important to improve the client's chance of survival during the next few minutes?

- A.** Delay use of the AED until 5 minutes of CPR have been completed
- B.** Apply the AED as soon as it is available and follow its prompts
- C.** Reassess the carotid pulse every 30 seconds during compressions
- D.** Give two breaths after every 10 compressions

98. A postoperative client is receiving oxygen at 4 L/min by nasal cannula. The nurse finds the client restless and anxious, with an oxygen saturation decrease from 95% to 88%. Breath sounds are diminished at both lung bases, and the client is taking shallow breaths because the incision "hurts too much." Which nursing action is most appropriate?

- A.** Increase the oxygen flow rate to 8 L/min by nasal cannula
- B.** Assist the client to cough and deep breathe, using splinting of the incision
- C.** Place the client flat in bed to reduce movement-related pain
- D.** Remove the nasal cannula and apply a simple face mask at 2 L/min

99. A nurse is caring for a client 6 hours after a tracheostomy was placed. While assessing the client, the nurse notes bright red bleeding from the tracheostomy site and blood-tinged secretions filling the dressing. The client is restless and has a heart rate of 118/min. Which action should the nurse take first?

- A.** Remove the soiled dressing and continue to monitor the amount of drainage
- B.** Suction the tracheostomy to clear the blood-tinged secretions
- C.** Apply oxygen and notify the provider immediately
- D.** Reposition the client flat to reduce movement at the tracheostomy site

100. A client with end-stage kidney disease has just had a new arteriovenous fistula created for future hemodialysis access. Which finding best indicates that the fistula is patent?

- A.** The client reports mild tenderness at the surgical site
- B.** A bruit is heard and a thrill is palpated over the fistula
- C.** The radial pulse is 2+ distal to the fistula
- D.** The skin over the fistula is slightly bruised

Simulation Test 27 — Answer Key

1. C	26. C	51. A	76. C
2. C	27. B	52. B	77. B
3. C	28. A	53. B	78. C
4. C	29. B	54. C	79. C
5. B	30. C	55. B	80. B
6. B	31. C	56. C	81. C
7. C	32. B	57. B	82. B
8. C	33. D	58. C	83. B
9. B	34. B	59. B	84. B
10. B	35. A	60. B	85. B
11. A	36. C	61. C	86. B
12. A	37. B	62. B	87. A
13. B	38. C	63. B	88. B
14. C	39. B	64. A	89. A
15. C	40. B	65. A	90. A
16. A	41. A	66. C	91. A
17. B	42. C	67. B	92. C
18. B	43. C	68. A	93. A
19. C	44. C	69. B	94. B
20. C	45. C	70. C	95. A
21. B	46. C	71. C	96. C
22. A	47. B	72. B	97. B
23. C	48. C	73. A	98. B
24. B	49. A	74. A	99. C
25. B	50. A	75. C	100. B